



Test Bank for Fundamentals of Nursing: Active Learning for Collaborative Practice, 3rd Edi

Nursing
Chamberlain College of Nursing
432 pag.

**Test Bank for Fundamentals of Nursing: Active Learning for Collaborative
Practice 3rd Edition by Barbara L. Yoost, Lynne R. Crawford
Chapter 1-42 Latest Version**

Table of Content

- Chapter 01: Nursing, Theory, and Professional Practice
- Chapter 02: Values, Beliefs, and Caring
- Chapter 03: Communication
- Chapter 04: Clinical Judgment in Nursing (NEW!)
- Chapter 05: Introduction to the Nursing Process
- Chapter 06: Assessment
- Chapter 07: Data Analysis/Nursing Diagnosis
- Chapter 08: Planning
- Chapter 09: Implementation and Evaluation
- Chapter 10: Documentation, Electronic Health Records, and Reporting
- Chapter 11: Ethical and Legal Considerations
- Chapter 12: Leadership and Management
- Chapter 13: Evidence-Based Practice and Nursing Research
- Chapter 14: Health Literacy and Patient Education
- Chapter 15: Nursing Informatics
- Chapter 16: Health and Wellness
- Chapter 17: Human Development: Conception Through Adolescence
- Chapter 18: Human Development: Young Adult Through Older Adult
- Chapter 19: Vital Signs
- Chapter 20: Health History and Physical Assessment
- Chapter 21: Ethnicity and Cultural Assessment
- Chapter 22: Spiritual Health

Chapter 23: Public Health, Community-Based, and Home Health Care

Chapter 24: Human Sexuality

Chapter 25: Safety

Chapter 26: Asepsis and Infection control

Chapter 27: Hygiene and Personal Care

Chapter 28: Activity, Immobility, and Safe Movement

Chapter 29: Skin Integrity and Wound Care

Chapter 30: Nutrition

Chapter 31: Cognitive and Sensory Alterations

Chapter 32: Stress and Coping

Chapter 33: Sleep

Chapter 34: Diagnostic Testing

Chapter 35: Medication Administration

Chapter 36: Pain Management

Chapter 37: Perioperative Nursing Care

Chapter 38: Oxygenation and Tissue Perfusion

Chapter 39: Fluid, Electrolytes, and Acid-Base Balance

Chapter 40: Bowel Elimination

Chapter 41: Urinary Elimination

Chapter 42: Death and Loss

Chapter 01: Nursing, Theory, and Professional Practice

Yoost & Crawford: Fundamentals of Nursing: Active Learning for Collaborative Practice, 3rd Edition

MULTIPLE CHOICE

1. A group of nursing students are discussing the impact of nonnursing theories in clinical practice. The students would be correct if they chose which theory to prioritize patient care?
- Erikson's Psychosocial Theory
 - Paul's Critical-Thinking Theory
 - Maslow's Hierarchy of Needs
 - Rosenstock's Health Belief Model

ANS: C

Maslow's hierarchy of needs specifies the psychological and physiologic factors that affect each person's physical and mental health. The nurse's understanding of these factors helps with formulating Nursing diagnoses that address the patient's needs and values to prioritize care. Erikson's Psychosocial Theory of Development and Socialization is based on individuals' interacting and learning about their world. Nurses use concepts of developmental theory to critically think in providing care for their patients at various stages of their lives. Rosenstock (1974) developed the psychological Health Belief Model. The model addresses possible reasons for why a patient may not comply with recommended health promotion behaviors. This model is especially useful to nurses as they educate patients.

DIF: Remembering

OBJ: 1.5

TOP: Planning

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT: Concepts: Care Coordination

2. A nursing student is preparing study notes from a recent lecture in nursing history. The student would credit Florence Nightingale for which definition of nursing?
- The imbalance between the patient and the environment decreases the capacity for health.
 - The nurse needs to focus on interpersonal processes between nurse and patient.
 - The nurse assists the patient with essential functions toward independence.
 - Human beings are interacting in continuous motion as energy fields.

ANS: A

Florence Nightingale's (1860) concept of the environment emphasized prevention and clean air, water, and housing. This theory states that the imbalance between the patient and the environment decreases the capacity for health and does not allow for conservation of energy. Hildegard Peplau (1952) focused on the roles played by the nurse and the interpersonal process between a nurse and a patient. Virginia Henderson described the nurse's role as substitutive (doing for the person), supplementary (helping the person), or complementary (working with the person), with the goal of independence for the patient. Martha Rogers (1970) developed the Science of Unitary Human Beings. She stated that human beings and their environments are interacting in continuous motion as infinite energy fields.

DIF: Understanding

OBJ: 1.4

TOP: Planning

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT: Concepts: Health Promotion

3. The nurse identifies which nurse established the American Red Cross during the Civil War?
- Dorothea Dix
 - Linda Richards
 - Lena Higbee
 - Clara Barton

ANS: D

Clara Barton practiced nursing in the Civil War and established the American Red Cross. Dorothea Dix was the head of the U.S. Sanitary Commission, which was a forerunner of the Army Nurse Corps. Linda Richards was America's first trained nurse, graduating from Boston's Women's Hospital in 1873, and Lena Higbee, superintendent of the U.S. Navy Nurse Corps, was awarded the Navy Cross in 1918.

DIF: Remembering

OBJ: 1.3

TOP: Assessment

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance

NOT: Concepts: Professionalism

4. The nursing instructor is researching the five proficiencies regarded as essential for students and professionals. The nursing instructor identifies which organization would be found to have added safety as a sixth competency?
- Quality and Safety Education for Nurses (QSEN)
 - Institute of Medicine (IOM)
 - American Association of Colleges of Nursing (AACN)
 - National League for Nursing (NLN)

ANS: A

The Institute of Medicine report, *Health Professions Education: A Bridge to Quality* (2003), outlines five core competencies. These include patient-centered care, interdisciplinary teamwork, use of evidence-based medicine, quality improvement, and use of information technology. QSEN added safety as a sixth competency. *The Essentials of Baccalaureate Education for Professional Nursing Practice* are provided and updated by the American Association of Colleges of Nursing (AACN) (2008). The document offers a framework for the education of professional nurses with outcomes for students to meet. The National League for Nursing (NLN) outlines and updates competencies for practical, associate, baccalaureate, and graduate nursing education programs.

DIF: Remembering

OBJ: 1.1

TOP: Planning

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

5. The nurse manager is interviewing graduate nurses to fill existing staffing vacancies. When hiring graduate nurses, the nurse manager realizes that they will probably not be considered —competent until they complete which task?
- They graduate and pass NCLEX.
 - They have worked 2 to 3 years.
 - Their last year of nursing school.
 - They are actually hired.

ANS: B

Benner's model identifies five levels of proficiency: novice, advanced beginner, competent, proficient, and expert. The student nurse progresses from novice to advanced beginner during nursing school and attains the competent level after approximately 2 to 3 years of work experience after graduation. To obtain the RN credential, a person must graduate from an approved school of nursing and pass a state licensing examination called the National Council Licensure Examination for Registered Nurses (NCLEX-RN) usually taken soon after completion of an approved nursing program.

DIF: Remembering

OBJ: 1.7

TOP: Planning

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

6. The prospective student is considering options for beginning a career in nursing. Which degree would best match the student's desire to conduct research at the university level?
- Associate Degree in Nursing (ADN)
 - Bachelor of Science in Nursing (BSN)
 - Doctor of Nursing Practice (DNP)
 - Doctor of Philosophy in Nursing (PhD)

ANS: D

Doctoral nursing education can result in a Doctor of Philosophy (PhD) degree. This degree prepares nurses for leadership roles in research, teaching, and administration that are essential to advancing nursing as a profession. Associate Degree in Nursing (ADN) programs usually are conducted in a community college setting. The nursing curriculum focuses on adult acute and chronic disease; maternal/child health; pediatrics; and psychiatric/mental health nursing. ADN RNs may return to school to earn a bachelor's degree or higher in an RN-to-BSN or RN-to-MSN program. Bachelor's degree programs include community health and management courses beyond those provided in an associate degree program. A newer practice-focused doctoral degree is the Doctor of Nursing practice (DNP), which concentrates on the clinical aspects of nursing. DNP specialties include the four advanced practice roles of NP, CNS, CNM, and CRNA.

DIF: Remembering

OBJ: 1.9

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

7. During a staff meeting, the nurse manager announces that the hospital will be seeking Magnet status. To explain the requirements for this award, the nurse manager will contact which organization?
- American Nurses Association (ANA)
 - American Nurses Credentialing Center (ANCC)
 - National League for Nursing (NLN)
 - Joint Commission

ANS: B

The American Nurses Credentialing Center (ANCC) awards Magnet Recognition to hospitals that have shown excellence and innovation in nursing. The ANA is a professional organization that provides standards of nursing practice. The National League for Nursing (NLN) outlines and updates competencies for practical, associate, baccalaureate, and graduate nursing education programs. The Joint Commission is the accrediting organization for health care facilities in the United States.

DIF: Remembering

OBJ: 1.1

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

8. The nurse is caring for a patient who refuses two units of packed red blood cells. When the nurse notifies the health care provider of the patient's decision, the nurse is acting in which role?
- Manager
 - Change agent
 - Advocate
 - Educator

ANS: C

As the patient's advocate, the nurse interprets information and provides the necessary education. The nurse then accepts and respects the patient's decisions even if they are different from the nurse's own beliefs. The nurse supports the patient's wishes and communicates them to other health care providers. A nurse manages all of the activities and treatments for patients. In the role of change agent, the nurse works with patients to address their health concerns and with staff members to address change in an organization or within a community. The nurse ensures that the patient receives sufficient information on which to base consent for care and related treatment. Education becomes a major focus of discharge planning so that patients will be prepared to handle their own needs at home.

DIF: Applying

OBJ: 1.2

TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

NURSINGTB.COM

9. The nursing student develops a plan of care based on a recently published article describing the effects of bed rest on a patient's calcium blood levels. When creating the plan of care, the nursing student has the obligation to consider which action?
- Critically appraise the evidence and determine validity.
 - Ensure that the plan of care does not alter current practice.
 - Change the process even when there is no problem identified.
 - Maintain the plan of care regardless of initial outcome.

ANS: A

Evidence-based practice (EBP) is an integration of the best-available research evidence with clinical judgment about a specific patient situation. The nurse assesses current and past research, clinical guidelines, and other resources to identify relevant literature. The application of EBP includes critically appraising the evidence to assess its validity, designing a change for practice, assessing the need for change and identifying a problem, and integrating and maintaining change while monitoring process and outcomes by reevaluating the application of evidence and assessing areas for improvement.

DIF: Applying

OBJ: 1.2

TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

10. The nurse is delegating frequent blood pressure (BP) measurements for a patient admitted with a gunshot wound to a licensed practical nurse (LPN). When delegating, the nurse understands which fact?
- He/she may assume that the LPN is able to perform this task appropriately.
 - The LPN is ultimately responsible for the patient findings and assessment.
 - The LPN may perform the tasks assigned without further supervision.
 - He/she retains ultimate responsibility for patient care and supervision is needed.

ANS: D

The RN retains ultimate responsibility for patient care, which requires supervision of those to whom patient care is delegated. In the process of collaboration, the nurse delegates certain activities to other health care personnel. The RN needs to *know* the scope of practice or capabilities of each health care member for delegation to be effective and safe.

DIF: Understanding

OBJ: 1.2

TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

11. The nurse is preparing to discharge a patient admitted with fever of unknown origin. The patient states, —I never got past the fifth grade in school. Don't read much. Never saw much sense in it. But I do OK. I can read most stuff. But my doctor explains things good and doesn't think that my sickness is serious. Considering this patient response, what action should the nurse carry out?
- Provide discharge medication information from a professional source to provide the most information.
 - Expect that the patient may return to the hospital if the discharge process is poorly done.
 - Assume that the physician and the patient have a good rapport and that the physician will clarify everything.
 - Defer offering the patient the opportunity to sign up for wellness classes due to the low literacy rate.

ANS: B

Low health literacy is associated with increased hospitalization, greater emergency care use, lower use of mammography, and lower receipt of influenza vaccine. A goal of patient education by the nurse is to inform patients and deliver information that is understandable by examining their level of health literacy. The more understandable health information is for patients, the closer the care is coordinated with need.

DIF: Applying

OBJ: 1.2

TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Health Promotion

12. A nurse is caring for a patient who lost a large amount of blood during childbirth. The nurse provides the opportunity for the patient to maintain her activity level while providing adequate periods of rest and encouragement. Which nursing theory would the nurse most likely choose as a framework for addressing the fatigue associated with the low blood count?
- Watson Human Caring Theory
 - Parse's Theory of Human Becoming
 - Roy's Adaptation Model
 - Rogers' Science of Unitary Human Beings

ANS: C

Roy's Adaptation Model is based on the human being as an adaptive open system. The person adapts by meeting physiologic-physical needs, developing a positive self-concept-group identity, performing social role functions, and balancing dependence and independence.

Stressors result in illness by disrupting the equilibrium. Nursing care is directed at altering stimuli that are stressors to the patient. The nurse helps patients strengthen their abilities to adapt to their illnesses or helps them to develop adaptive behaviors. Watson's theory is based on caring, with nurses dedicated to health and healing. The nurse functions to preserve the dignity and wholeness of humans in health or while peacefully dying. Parse's theory is called the Human Becoming School of Thought. Parse formulated the Theory of Human Becoming by combining concepts from Martha Rogers' Science of Unitary Human Beings with existential-phenomenologic thought. This theory looks at the person as a constantly changing being, and at nursing as a human science. Martha Rogers (1970) developed the Science of Unitary Human Beings. She stated that human beings and their environments are interacting in continuous motion as infinite energy fields.

DIF: Applying OBJ: 1.4 TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT: Concepts: Care Coordination

13. The nurse recognizes which nursing theorist who described the relationship between the nurse and the patient as an interpersonal and therapeutic process?
- Virginia Henderson
 - Betty Neuman
 - Imogene King
 - Hildegard Peplau

ANS: D

NURSINGTB.COM

Hildegard Peplau focused on the roles played by the nurse and the interpersonal process between a nurse and a patient. The interpersonal process occurs in overlapping phases: (1) orientation, (2) working, consisting of two subphases: identification and exploitation, and (3) resolution. Betty Neuman's Systems Model includes a holistic concept and an open-system approach. The model identifies energy resources that provide for basic survival, with lines of resistance that are activated when a stressor invades the system. Virginia Henderson described the nurse's role as substitutive (doing for the person), supplementary (helping the person), or complementary (working with the person), with the ultimate goal of independence for the patient. Imogene King developed a general systems framework that incorporates three levels of systems: (1) individual or personal, (2) group or interpersonal, and (3) society or social.

The theory of goal attainment discusses the importance of interaction, perception, communication, transaction, self, role, stress, growth and development, time, and personal space. In this theory, both the nurse and the patient work together to achieve the goals in the continuous adjustment to stressors.

DIF: Remembering OBJ: 1.4 TOP: Assessment MSC:

NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT: Concepts: Health Promotion

14. When a nursing class volunteers to serve hot meals at a local homeless shelter on a Saturday afternoon, which term identifies this focus on serving the community?
- Altruism

- b. Accountability
- c. Autonomy
- d. Advocate

ANS: A

A profession provides services needed by society. Additionally, practitioners' motivation is public service over personal gain (altruism). Service to the public requires intellectual activities, which include responsibility. This accountability has legal, ethical, and professional implications. Members of a profession have autonomy in decision making and practice and are self-regulating in that they develop their own policies in collaboration with one another.

As the patient's advocate, the nurse interprets information and provides the necessary education. The nurse then accepts and respects the patient's decisions even if they are different from the nurse's own beliefs.

DIF: Understanding

OBJ: 1.6

TOP: Assessment

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance

NOT: Concepts: Health Promotion

15. A patient is being discharged from the hospital with wound care dressing changes. The nurse recommends a referral for home health nursing care. The nurse is using which standard of practice?
- a. Assessment
 - b. Diagnosis
 - c. Planning
 - d. Implementation

ANS: C

As a care provider, the nurse follows the nursing process to assess patient data, prioritize Nursing diagnoses, plan the care of the patient, implement the appropriate interventions, and evaluate care in an ongoing cycle. In recommending a referral, the nurse is, in effect, planning care.

DIF: Applying

OBJ: 1.2

TOP: Planning

MSC: NCLEX Client Needs Category: Physiological Integrity: Basic Care and Comfort NOT:

Concepts: Care Coordination

16. The nurse administers a medication to the patient and then realizes that the medication had been discontinued. The error is immediately reported to the physician. The nurse recognizes which term that identifies complying with the standards of professional performance?
- a. Ethics
 - b. Socialization
 - c. Altruism
 - d. Autonomy

ANS: A

Guiding the nurse's professional practice are ethical behaviors. Ethics is the standards of right and wrong behavior. The main concepts in nursing ethics are accountability, advocacy, autonomy (be independent and self-motivated), beneficence (act in the best interest of the patient), confidentiality, fidelity (keep promises), justice (relate to others with fairness and equality), nonmaleficence (do no harm), responsibility, and veracity (be truthful). Ethical guidelines direct the nurse's decision making in routine situations and in ethical dilemmas.

Socialization to professional nursing is a process that involves learning the theory and skills necessary for the role of nurse. A profession provides services needed by society.

Additionally, practitioners' motivation is public service over personal gain (altruism).

Members of a profession have autonomy in decision making and practice and are

self-regulating in that they develop their own policies in collaboration with one another.

DIF: Applying

OBJ: 1.6

TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Safety and Infection Control

NOT: Concepts: Ethics

17. A newly licensed registered nurse is curious about the scope of care that he or she has in caring for patients undergoing conscious sedation. Which would be the best source of information for this nurse?

- a. National Student Nurses Association
- b. Nurse Practice Act
- c. ANA Standards of Professional Performance
- d. National League for Nursing

ANS: B

Nurse practice acts provide the scope of practice defined by each state or jurisdiction and set forth the legal limits of nursing practice. Nursing organizations enable the nurse to have access to current

information and resources as well as a voice in the profession. Nursing organizations include the ANA, the NLN, the ICN, Sigma Theta Tau International Honor Society of Nursing, and the National Student Nurses Association (NSNA).

DIF: Remembering

OBJ: 1.7

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Health Care Law

18. The nursing student is writing a paper about the direct patient care role of advanced practice nurses. Which advanced practice role would the student include in the report?

- a. Nurse Administrator
- b. Clinical Nurse Leader
- c. Clinical Nurse Specialist
- d. Nurse Educator

ANS: C

There are four specialties in which nurses provide direct patient care in advanced practice roles: certified nurse midwife (CNM), nurse practitioner (NP), clinical nurse specialist (CNS), and certified registered nurse anesthetist (CRNA). Four additional advanced practice roles that do not always involve direct patient care are clinical nurse leader (CNL), nurse educator, nurse researcher, and nurse administrator.

DIF: Remembering

OBJ: 1.9

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care

NOT: Concepts: Health Care Law

19. The nurse is determining the patient care assignments for a nursing unit. The nurse knows which responsibility may be delegated to the licensed practical nurse?
- Initiating the nursing care plans
 - Formulating Nursing diagnoses
 - Assessing a newly admitted patient
 - Administering oral medications

ANS: D

LPNs, or LVNs in California and Texas, are not RNs. They complete an educational program consisting of 12 to 18 months of training, and then they must pass the National Council Licensure Examination for Practical Nurses (NCLEX-PN) to practice as an LPN/LVN. They are under the supervision of an RN in most institutions and are able to collect data but cannot perform an assessment requiring decision making, cannot formulate a Nursing diagnosis, and cannot initiate a care plan. They may update care plans and administer medications except for certain IV medications.

DIF: Applying OBJ: 1.9 TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT: Concepts: Health Care Law

20. The nursing student is taking a class in Nursing Research. In class the student has learned which term that identifies the most abstract level of knowledge?
- Metaparadigm
 - Philosophy
 - Conceptual framework
 - Nursing theory

NURSINGTB.COM

ANS: A

A metaparadigm, as the most abstract level of knowledge, is defined as a global set of concepts that identify and describe the central phenomena of the discipline and explain the relationship between those concepts. For example, the metaparadigm for nursing focuses on the concepts of person, environment, health, and nursing. The next level of knowledge is a philosophy, which is a statement about the beliefs and values of nursing in relation to a specific phenomenon such as health. The third level of knowledge is a nursing conceptual framework, or model, which is a collection of interrelated concepts that provides direction for nursing practice, research, and education. The fourth level of nursing knowledge is a nursing theory, which represents a group of concepts that can be tested in practice and can be derived from a conceptual model.

DIF: Remembering OBJ: 1.4 TOP: Assessment

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance

NOT: Concepts: Professionalism

MULTIPLE RESPONSE

1. The nurse recognizes which statements contribute to the understanding that nursing is considered a profession? (*Select all that apply.*)
- Nursing requires specialized training.
 - Nursing has a specialized body of knowledge.

- c. The ANA regulates nursing practice.
- d. Nurses make independent decisions within their scope of practice.
- e. Once licensure is complete, no further education is required.

ANS: A, B, D

A profession is an occupation that requires at a minimum specialized training and a specialized body of knowledge. Nursing meets these minimum requirements. Thus nursing is considered to be a profession. Members of a profession have autonomy in decision making and practice and are self-regulating in that they develop their own policies in collaboration with one another. Nursing professionals make independent decisions within their scope of practice and are responsible for the results and consequences of those decisions. A profession is committed to competence and has a legally recognized license. Members are accountable for continuing their education. The ANA is a professional organization that provides standards (not regulation) of nursing practice.

DIF: Remembering

OBJ: 1.6

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Professionalism

2. The Institute of Medicine (IOM) Report identified several goals for nursing in the United States. The nurse identifies that the IOM offered which suggestions? (*Select all that apply.*)
- a. Nurses should practice to the full extent of their education.
 - b. Nursing education should demonstrate seamless progression.
 - c. Nurses should continue to be subservient to physicians in the hospital setting.
 - d. Policy making requires better data collection and information infrastructure.
 - e. Higher levels of education will not be needed by practicing nurses.

ANS: A, B, D

NURSINGTB.COM

The Future of Nursing: Leading Change, Advancing Health (IOM, 2011) identified several goals for nursing in the United States: nurses should practice to the full extent of their education and training; Nurses should achieve higher levels of education and training through an improved education system that promotes seamless academic progression; Nurses should be full partners with physicians and other health care professionals in redesigning health care in the United States; and Effective workforce planning and policy making require better data collection and an improved information infrastructure.

DIF: Remembering

OBJ: 1.1

TOP: Assessment

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance

NOT: Concepts: Professionalism

3. The nurse is caring for a patient admitted for the removal of an infected appendix. Which actions by the nurse would indicate an understanding of the 2018 hospital safety goals? (*Select all that apply.*)
- a. Places an identification band on the right arm.
 - b. Marks the surgical site with a black-felt pen.
 - c. Checks medications three times before administration.
 - d. Washes hands between patients and/or when soiled.
 - e. Removes allergy bands prior to transfer to surgery.

ANS: A, B, C, D

The Joint Commission identifies each category and has specific elements of performance that are required for the health care worker to meet the goals. As new problems in patient care emerge, the safety goals are reassessed and revised. The 2018 hospital goals include the following broad categories: improve the accuracy of patient identification, improve the effectiveness of communication among caregivers, improve the safety of using medications, reduce the harm associated with clinical alarm systems, reduce the risk of health care– associated infections. The organization identifies safety risks inherent in its patient population. Improve the accuracy of patient identification. (Placing an ID band on the right are), improve the safety of using medications (check medications three times before administration), reduce the risk of health care– associated infections. (Washing hands), and the organization identifies safety risks inherent in its patient population. (Mark the surgical site with a black-felt pen) are all examples of actions that comply with the 2018 safety goals.

Removing allergy bands would prevent identification of that patient's safety risk.

DIF: Applying OBJ: 1.1 TOP: Implementation
MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Safety and Infection Control
NOT: Concepts: Care Coordination

4. The nurse is conducting a health assessment on a patient from a foreign country. Which concepts should be addressed by the nurse during the interview? (*Select all that apply.*)
- a. Food preferences
 - b. Religious practices
 - c. Health beliefs
 - d. Family orientation
 - e. Politics

ANS: A, B, C, D

Culture is the integrated patterns of human behavior that include the language, thoughts, communications, actions, customs, beliefs, values, and institutions of racial, ethnic, religious, or social groups.

DIF: Applying OBJ: 1.7 TOP: Implementation
MSC: NCLEX Client Needs Category: Psychosocial Integrity NOT: Concepts: Care Coordination

5. The nurse documents that patient laboratory results often take 4 hours to populate into the electronic medical record. The lengthy time frame has contributed to delayed antibiotic administration. From this point, what should the nurse do to produce change using Evidence-Based practice? (*Select all that apply.*)
- a. Identify a problem affecting patient care.
 - b. Realize the facility resources may influence the decision.
 - c. Review pertinent journal articles from the literature search.
 - d. Apply the findings to clinical practice considering patient preferences.
 - e. Using the process recommended by the best clinical article.

ANS: A, B, C, D

The process of using evidence-based practice (EBP) starts with the identification of a problem. The nurse then conducts a literature search to find the best evidence pertaining to the problem. Facility resources may impact the ability to implement the chosen decision. Patient preferences need to be incorporated into the use of evidence from the literature combined with clinical expertise. The nurse would not use just one clinical article to determine a solution to the issue.

DIF: Applying OBJ: 1.2 TOP: Implementation
MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
Concepts: Evidence

NURSINGTB.COM

Chapter 02: Values, Beliefs, and Caring

Yoost & Crawford: Fundamentals of Nursing: Active Learning for Collaborative Practice, 3rd Edition

MULTIPLE CHOICE

1. The nurse identifies the concept of enduring ideas about what a person considers desirable or has worth in life is known by which term?
 - a. Values
 - b. First-order belief
 - c. Higher-order belief
 - d. Stereotype

ANS: A

Values are enduring ideas about what a person considers is the good, the best, and the —right thing to do and their opposites—the bad, worst, and wrong things to do—and about what is desirable or has worth in life. First-order beliefs serve as the foundation or the basis of an individual's belief system. Higher-order beliefs are ideas derived from a person's first-order beliefs through inductive or deductive reasoning. A stereotype is a belief about a person, a group, or an event that is thought to be typical of all others in that category.

DIF: Remembering

OBJ: 2.1

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT: Concepts: Professionalism

2. A group of nursing students are discussing the history of nursing to a staff nurse. When a student states, —Yeah, nurses used to be called the doctors' handmaidens, the staff nurse recognizes that this comment is identified by which term?
 - a. Prejudice
 - b. Generalization
 - c. Stereotype
 - d. Belief

ANS: C

A stereotype is a belief about a person, a group, or an event that is thought to be typical of all others in that category. A prejudice is a preformed opinion, usually an unfavorable one, about an entire group of people that is based on insufficient knowledge, irrational feelings, or inaccurate stereotypes. In the process of learning, people form generalizations (general statements or ideas about people or things) to relate new information to what is already known and to categorize the new information, making it easier to remember or understand. A belief is a mental representation of reality or a person's perceptions about what is right (correct), true, or real, or what the person expects to happen in a given situation.

DIF: Understanding

OBJ: 2.1

TOP: Evaluation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT: Concepts: Professionalism

3. A values system is a set of somewhat consistent values and measures that are organized hierarchically into a belief system on a continuum of relative importance. The nurse knows that a value system is also identified by which concept?

- a. It is culturally based.
- b. It is unique to each individual.
- c. It is a poor basis for making decisions.
- d. It is rigid and uniform within a culture.

ANS: A

Anthropologists and social scientists have noted that in every culture, a particular value system prevails and consists of culturally defined moral and ethical principles and rules that are learned in childhood. Everyone possesses a relatively small number of values and may share the same values with others, but to different degrees. A value system helps the person choose between alternatives, resolve values conflicts, and make decisions. Within every culture, however, values vary widely among subcultural groups and even between individuals on the basis of the person's gender, personal experiences, personality, education, and many other variables.

DIF: Remembering

OBJ: 2.1

TOP: Assessment

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Professionalism

4. The nurse is caring for a patient who is under arrest for murder and is attempting to perform nursing care duties while, at the same time, feeling a sense of repugnance toward the patient. The nurse recognizes this situation is identified by which term?
- a. Value clarification
 - b. Value conflict
 - c. First-order beliefs
 - d. Higher-order beliefs

ANS: B

A values conflict occurs when a person's values are inconsistent with his or her behaviors or when the person's values are not consistent with the choices that are available. Providing care for a convicted murderer may elicit troubling feelings for a nurse, resulting in a values conflict between the nurse's commitment to care for all people and a personal repugnance for the act of murder. First-order beliefs serve as the foundation or the basis of an individual's belief system. Higher-order beliefs are ideas derived from a person's first-order beliefs, inductive, or syllogistic reasoning.

DIF: Understanding

OBJ: 2.1

TOP: Diagnosis

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Professionalism

5. While helping patients with values clarification and care decisions, the nurse should complete which action?
- a. Convince the patient to do what the nurse believes is best.
 - b. Give advice about what the nurse would do.
 - c. Tell the patient what the right thing to do is.
 - d. Provide information so the patient can make informed decisions.

ANS: D

While helping patients with values clarification and care decisions, nurses must be aware of the potential influence of their professional nursing role on patient decision making. Nurses should be careful to assist patients to clarify their own values in reaching informed decisions. Providing information to patients so that they can make informed decisions is a critical nursing role. Giving advice or telling patients what to do in difficult circumstances is both unethical and ill-advised.

DIF: Applying OBJ: 2.2 TOP: Implementation

MSC: NCLEX Client Needs Category: Psychosocial Integrity NOT: Concepts: Professionalism

6. A patient with terminal cancer says to the nurse, —I just don't know if I should allow CPR in the event I quit breathing. What do you think? Which statement by the nurse would be most beneficial to the patient?
- If it were me, I would want to live no matter what. ||
 - Don't worry. You have plenty of time to decide that later on. ||
 - It's totally up to you. Have you discussed this with your family? ||
 - Let's talk about what CPR means to you. ||

ANS: D

The use of the value clarification process is helpful when assisting patients in making health care decisions regarding end-of-life care. Giving advice or telling patients what to do is unethical and not recommended. Ignoring a patient concern or changing the subject is inappropriate. Patients should be given factual information in order for them to make their own decisions.

DIF: Applying OBJ: 2.2 TOP: Implementation

MSC: NCLEX Client Needs Category: Psychosocial Integrity NOT: Concepts: Professionalism

NURSINGTB.COM

7. The nurse is observed sitting at the bedside of a patient discussing the nursing care plan for the shift. The nurse identifies which theory or model most accurately reflects this nurse-patient relationship?
- Swanson's Theory of Caring
 - Travelbee's human-to-human relationship model
 - Watson's Theory of Caring
 - Leininger Cultural Care Theory

ANS: A

Swanson's five caring processes include being with and enabling. Sitting at the bedside and sharing information are activities that exemplify these behaviors. Travelbee's model describes steps toward compassionate and empathetic care. Watson's Theory of Caring impacts both the person and the universe and is built upon 10 caritas processes. Leininger describes patient care and its relationship to cultural diversity.

DIF: Understanding

OBJ: 2.4

TOP: Diagnosis

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Professionalism

8. The student nurse is planning care for a patient who believes that Western medicine is effective but not always accurate and recognizes which nursing theory would best explain the patient's health practices?
- Nursing: Human Science and Human Care
 - Theory of Cultural Care Diversity and Universality

- c. Theory of Nursing as Caring
- d. Five caring processes

ANS: B

Leininger describes patient care and its relationship to cultural diversity. Swanson's five caring processes include maintaining belief, knowing, being with, doing for, and enabling. In the Theory of Nursing as Caring, Boykin & Schoenhofer, note that caring is defined as —the intentional and authentic presence of the nurse with another who is recognized as person living caring and growing in caring. Watson's Theory of Human Science and Human Care impacts both the person and the universe and is built upon 10 caritas processes.

DIF: Understanding

OBJ: 2.4

TOP: Diagnosis

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Professionalism

9. The nurse identifies which nursing theorist/theorists who describes/describe the nurse–patient relationship as a situation in which the nurse and patient share the lived experience of caring?
- a. Kristen Swanson
 - b. Jean Watson
 - c. Madeleine Leininger
 - d. Anne Boykin & Savina Schoenhofer

ANS: D

In the Theory of Nursing as Caring (Boykin & Schoenhofer, 2015), caring is defined as —the intentional and authentic presence of the nurse with another who is recognized as person living caring and growing in caring (Boykin & Schoenhofer, 2001, p. 13), and —the general intention of nursing as a practiced discipline is nurturing persons living caring and growing in caring (Boykin & Schoenhofer, 2015, p. 343). One of the major concepts of the theory is the nursing situation in which the nurse and patient share the lived experience of caring. It is in this nursing situation that nursing is created and can best be understood. The model has been used in a variety of settings to guide practice, education, and research. Leininger describes patient care and its relationship to cultural diversity. Swanson's five caring processes include maintaining belief, knowing, being with, doing for, and enabling. Watson's Theory of Human Science and Human Care impacts both the person and the universe and is built upon 10 caritas processes.

DIF: Remembering

OBJ: 2.4

TOP: Assessment

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Professionalism

10. The nurse on a busy medical–surgical floor contacts a social worker requesting a home care referral prior to a patient's discharge. This action is best illustrated by which of Swanson's five caring processes?
- a. Enabling
 - b. Knowing
 - c. Doing for
 - d. Being with
 - e. Maintaining belief

ANS: A

Advocating for a patient's post-hospitalization care is an enabling process. Enabling also includes informing, anticipating, and preparing for the future. Swanson's five caring processes also include maintaining belief, knowing, being with, and doing for.

DIF: Remembering

OBJ: 2.4

TOP: Assessment

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Professionalism

11. Which action observed by a nurse manager is not indicative of the qualities and behaviors of caring?
- A staff nurse orders extra desserts for a patient diagnosed with morbid obesity.
 - A medication nurse administers scheduled pain medication to patients as ordered.
 - A respiratory therapist teaches a patient's spouse how to adjust an oxygen mask.
 - A nursing assistant encourages a patient to assist with the morning bath.

ANS: A

Caring includes demonstrating to the patient and significant others —authentic concern. Giving extra dessert for a morbidly obese patient, even if the patient is asking for them, does not show authentic concern for the patient, the patient's conditions, and the possible consequences of the condition. Giving pain medications on time, teaching a spouse how to help provide care, and encouraging self-care all demonstrate this authentic concern.

DIF: Evaluating

OBJ: 2.6

TOP: Evaluation

MSC: NCLEX Client Needs Category: Physiological Integrity: Basic Care and Comfort NOT:

Concepts: Caregiving

12. The nurse recognizes that when developing a nursing practice, it is important for the nurse to: carry out which action?
- Be exposed to negative as well as positive role models.
 - Avoid negative role models as much as possible.
 - Understand that caring and compassion are taught in class.
 - Consider another profession if she is not naturally compassionate.

ANS: A

Nurses develop caring skills through life experiences, educational activities, observation of both positive and negative role models, and interaction with strong professional mentors. Although there has been disagreement in the past about whether or not it is possible to teach values—specifically caring, recent research suggests that care, compassion, and empathy can be taught.

DIF: Applying

OBJ: 2.5

TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Caregiving

13. The nurse is discussing the use of a values clarification tool with a patient. The patient asks, —What is the goal of the values clarification tool? Which is the best response by the nurse?
- The tool will help change your value system so that you can make the right decision.
 - The tool will dispel your current beliefs and formulate brand new ones.
 - The tool will assist you in prioritizing your value preferences and help you make decisions.
 - The tool allows you to make decisions without the need of self-awareness.

ANS: C

Values clarification is a process used to help people reflect on, clarify, and prioritize personal values to increase self-awareness or to make decisions. Nurses can use values clarification to help patients identify the nature of a conflict and reach a decision based on their values.

DIF: Understanding

OBJ: 2.2

TOP: Evaluation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Caregiving

14. The nurse knows providing care that is consistent and predictable can make the health care experience less intimidating for the patient. What additional action can the nurse take to enhance this experience?
- Explaining what is going to take place beforehand
 - Never making promises to patients
 - Assuring the patient that his/her requests will get done eventually
 - Protecting the patient from knowing why things are happening

ANS: A

Care should be delivered in a way that conveys competence. Patients become alarmed when they detect that their nurse is unfamiliar with a procedure. It is best to seek assistance with any procedure or skill that the nurse cannot safely accomplish alone. Every task-oriented procedure should be explained to a patient, followed by feedback indicating patient understanding, before care is initiated. The remaining three actions do not reduce patients' feelings of intimidation.

DIF: Applying

OBJ: 2.6

TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Caregiving

NURSINGTB.COM

15. When planning to change a dressing on an anxious patient, the nurse recognizes which to be the best approach?
- Ask another staff member to perform the task.
 - Tell the patient the dressing change will take 30 minutes.
 - Schedule a time in collaboration with the patient.
 - Review the physician's order prior to the procedure.

ANS: C

Setting up a schedule to perform tasks helps to relieve patient anxiety and promotes a sense of security. Explaining the procedure and reviewing physician orders should be completed after establishing a schedule. Asking another staff member to change the dressing may increase patient anxiety.

DIF: Applying

OBJ: 2.6

TOP: Implementation

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Caregiving

16. Collaborating effectively with patients to find treatment methods that are congruent with the patients' belief systems and that promote healthy outcomes is an approach that requires the nurse to include which activity?
- Focus on patient values only and disregard family desires in setting goals.
 - Rely more and more on their scientific background.
 - Listen carefully to how the patient's beliefs impact their health beliefs.
 - Understand that the nurse's beliefs are the most important.

ANS: C

Nurses must collaborate effectively with patients to find treatment methods that are congruent with the patients' belief systems and that promote healthy outcomes. This approach requires excellent assessment skills and a willingness to listen carefully to determine how patients' personal beliefs impact their health beliefs. Failure to consider the patient's belief systems may result in ineffective implementation of the plan of care.

DIF: Applying OBJ: 2.3 TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Caregiving

17. The nurse is caring for a patient scheduled for heart surgery. Which statement made by the patient requires further discussion?
- My friend died on the operating table several months ago.||
 - The surgeon has a great reputation in the community.||
 - I believe that this surgery is going to make me better.||
 - Yesterday I asked my pastor to visit me after the procedure.||

ANS: A

Personal beliefs are one of the most important factors in determining how a person responds to a health problem and its treatment. The patient has a concern about the possibility of dying during the surgery based on prior experiences. The nurse should further explore the concern and determine the patient's true meaning of the statement. Failure to consider the patient's belief systems may result in ineffective implementation of the plan of care. Belief in the surgeon's reputation, the success of the surgery, and the patient's ability to visit after the surgery indicates a positive belief.

DIF: Analyzing OBJ: 2.3 TOP: Analysis

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Coping

18. The nurse recognizes that a vital aspect of providing effective and appropriate nursing care is being able to actively listen to a patient and then demonstrates this concept when carrying out which activity?
- Pays attention as if in a social conversation with the patient.
 - Practices and develops this skill over many years.
 - Focuses on what the patient is saying.
 - Passively listens with the ears.

ANS: B

A vital aspect of providing effective and appropriate nursing care is being able to actively listen to a patient in a way that conveys understanding, sensitivity, and compassion. Caring involves interpersonal relationships and communication skills that require paying more attention to the details of communication than would be necessary in a social conversation. This type of listening is a highly developed skill that usually takes a great deal of time and many years of experience to acquire. It can be learned with practice and enhanced with sensitivity and attention to the feedback that is received during each interaction. In a caring nurse–patient relationship, the nurse takes responsibility for establishing trust, making sure that the lines of communication are open and that the nurse accurately understands not only what the patient is saying, but also that the nurse is clearly understood. Active listening means paying careful attention and using all of the senses to listen rather than just passively listening with the ears. It requires energy and concentration and involves hearing the entire message— what the patient means as well as what the patient says. This type of listening focuses solely on the patient and conveys respect and interest.

DIF: Applying OBJ: 2.6 TOP: Implementation
MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
Concepts: Caregiving

19. The nurse is caring for a patient with lung disease. The patient tells the nurse that the most important thing to do during the shift is to walk down to the nurses' station and back without having shortness of breath. The patient's request is an example of which nursing theory?
- a. Leininger's Cultural Care Theory
 - b. Boykin & Schoenhofer's Theory of Nursing as Caring
 - c. Swanson's Theory of Caring
 - d. Watson's Human Science and Human Care Theory

ANS: C

[NURSINGTB.COM](https://www.nursingtb.com)

Swanson's Theory of Caring is composed of five interrelated caring processes: having faith in the ability of others to have meaningful lives; striving to understand the meaning of events in other's lives; being emotionally present to the other person; doing for others what they would do if possible and facilitating or enabling the capacity of others to help themselves and their families. The patient's goal to walk without breathing problems is an example of the enabling process. Leininger's Cultural Care Theory centers on cultural practices that influence patient care. Boykin & Schoenhofer's theory focuses on the intentional and authentic presence of the nurse with another who is recognized as a person living caring and growing in caring. Watson's theory describes holistic care and focuses on caritas processes such as instilling faith and hope, promoting and accepting positive and negative feelings, and developing a helping-trust relationship.

DIF: Remembering OBJ: 2.4 TOP: Assessment MSC:
NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:
Concepts: Caregiving

20. When the nurse is dealing with the concept of beliefs and values, the nurse recognizes which type is based in the unconscious?
- a. Zero-order beliefs
 - b. First-order beliefs
 - c. Higher-order beliefs
 - d. Prejudices

ANS: A

Three types of beliefs are recognized: zero-order beliefs, most of which are unconscious, such as object permanence; first-order beliefs, which are conscious, typically based on direct experiences; and higher-order beliefs, which are generalizations or ideas that are derived from first-order beliefs and reasoning. A prejudice is a preformed opinion, usually an unfavorable one, about an entire group of people that is based on insufficient knowledge, irrational feelings, or inaccurate stereotypes.

DIF: Remembering

OBJ: 2.1

TOP: Assessment

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Cognition

MULTIPLE RESPONSE

1. A nurse working in a dermatology clinic observes that a patient of Mexican-American descent typically arrives 10 to 15 minutes late to every appointment. Based on an understanding of first-order beliefs, what characteristics can the nurse associate with this level of beliefs? (*Select all that apply.*)
 - a. First-order beliefs serve as the basis of a person's belief system.
 - b. First-order beliefs begin to develop in early adolescence.
 - c. First-order beliefs are completely formed in childhood.
 - d. People seldom question their first-order beliefs.
 - e. Challenging a patient's first-order beliefs may cause cognitive upset.

ANS: A, D, E

First-order beliefs serve as the foundation or the basis of an individual's belief system. People begin developing first-order beliefs about what is correct, real, and true in early childhood

directly through experiences and indirectly from information shared by authority figures such as parents or teachers. People continue to develop first-order beliefs into adulthood through both direct experiences and the acquisition of knowledge from a vast number of sources with various degrees of expertise and levels of influence. People seldom question their first-order beliefs and rarely replace one, because to do so would require a great deal of rethinking about both that belief and similar or closely associated beliefs. Remember that presenting information to patients that challenges their first-order beliefs may cause a great deal of emotional or cognitive upset.

DIF: Remembering

OBJ: 2.2

TOP: Assessment

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Professionalism

2. When dealing with patient who has a values conflict in which substance abuse or an addiction is involved, the nurse should conduct an assessment interview and use which techniques that will make the interview most effective? (*Select all that apply.*)
 - a. Listen for subtle signs of denial.
 - b. Directly confront the patient about his drug abuse.
 - c. Use a matter-of-fact approach to inform the patient.
 - d. Provide straightforward information.
 - e. Avoid direct confrontation.

ANS: A, C, D, E

The most effective approach for dealing with a values conflict in which substance abuse or an addiction is involved is to begin with an assessment interview, during which the nurse should: listen for the subtle signs of denial, avoid direct confrontation, use a matter-of-fact approach to inform the patient of the reality of the consequences of the harmful behavior, and provide straightforward information about the effects of the substance abuse.

DIF: Applying

OBJ: 2.2

TOP: Implementation

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Professionalism

3. Caring, according to the American Nurses Association (ANA) Code of Ethics (2015), is having concern or regard for that which affects the welfare of another. The nurse recognizes that as a profession, nursing can trace its earliest beginnings to what types of nurturing activities that demonstrate care? (*Select all that apply.*)
- a. Active listening
 - b. Advocating for the vulnerable
 - c. Valuing all individuals
 - d. Separating healing from spirit
 - e. Attempting to relieve pain

ANS: A, B, C, E

Caring, according to the American Nurses Association (ANA) Code of Ethics, is having concern or regard for that which affects the welfare of another. As a profession, nursing can trace its earliest beginnings to the types of nurturing activities that demonstrate care, such as taking time to be with a suffering person, actively listening, advocating for the vulnerable, valuing and respecting all individuals, attempting to relieve pain, and making the healing process an act of the body, mind, and spirit.

DIF: Remembering

NURSINGTB.COM

OBJ: 2.6

TOP: Assessment

MSC: NCLEX Client Needs Category: Physiological Integrity: Basic Care and Comfort NOT:

Concepts: Caregiving

4. Touch is the intentional physical contact between two or more people and it is deemed to be an essential and universal component of nursing care. The nurse knows that task-oriented touch occurs during which activities? (*Select all that apply.*)
- a. Holding the patient's hand during a painful procedure
 - b. Giving the patient an injection to treat discomfort
 - c. Starting an intravenous (IV) line for fluid administration
 - d. Inserting a nasogastric tube to decompress the patient's stomach
 - e. Shaking the patient's hand in order to establish rapport

ANS: B, C, D

Task-oriented touch includes performing nursing interventions such as giving treatments, changing dressings, suctioning an endotracheal tube, giving an injection, starting an IV line, or inserting an NG tube. Task-oriented touch should be done gently, skillfully, and in a way that conveys competence. Patients become alarmed when they detect that their nurse is unfamiliar with a procedure. It is best to seek assistance with any procedure or skill that the nurse cannot safely accomplish alone. Every task-oriented procedure should be explained to a patient, followed by feedback indicating patient understanding, before care is initiated. Caring touch is considered by most people to be a valuable means of nonverbal communication. In today's highly technical world of nursing, caring touch is an essential aspect of patient-centered care. Caring touch can be used to soothe, comfort, establish rapport, and create a bond between the nurse and the patient. Care may be conveyed by holding the hand of a patient during a painful or frightening procedure or when delivering bad news. This is an important way nurses let patients know that they are not alone and that another human being cares.

DIF: Applying OBJ: 2.6 TOP: Implementation

MSC: NCLEX Client Needs Category: Psychosocial Integrity NOT: Concepts: Caregiving

5. The nurse recognizes that after several years of work in the emergency room, compassion fatigue has developed. What symptoms associated with this condition would the nurse be experiencing? (*Select all that apply.*)
- a. Chronic depression
 - b. Sleeping all the time
 - c. Anorexia
 - d. Poor concentration
 - e. Feeling detached from patients
 - f. Euphoria

NURSINGTB.COM

ANS: A, D, E

Compassion fatigue is an extreme state of distress experienced as the progressive and cumulative result of exposure to stress in the therapeutic use of self in caring for others. Compassion fatigue involves the nurse experiencing a feeling of being unable to meet the needs of patients arising from the inability to alleviate suffering. Compassion fatigue may result in feelings of vulnerability, anxiety, depression, and anger. Left unrecognized, compassion fatigue can produce physical and mental exhaustion manifested by difficulty sleeping, poor concentration, and low morale; and it can lead to compulsive behaviors, such as substance abuse. Nurses experiencing compassion fatigue often detach themselves from patients, have a higher risk of making errors, exercise poor judgment, and experience difficulty in maintaining interprofessional relationships.

DIF: Understanding OBJ: 2.7 TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT: Concepts: Caregiving

Chapter 03: Communication

Yoost & Crawford: Fundamentals of Nursing: Active Learning for Collaborative Practice, 3rd Edition

MULTIPLE CHOICE

1. The nurse is caring for an adult patient with a recent below-the-knee amputation. During shift report, the nurse reports that the patient has urinated in the bed multiple times since the surgery. The nurse knows which defense mechanism best describes this behavior?
 - a. Compensation
 - b. Denial
 - c. Rationalization
 - d. Regression

ANS: D

Regression is the return to an earlier developmental stage as a means of avoiding unpleasant or unacceptable thoughts. The adult patient recently lost a limb and reverted to bedwetting as a coping mechanism. Compensation refers to a strategy that uses a personal strength to counterbalance a weakness or a feeling of inadequacy. Refusing to accept a fact or reality as truth is termed denial. Rationalization is the act of suggesting a different explanation for one that is painful, negative, or unacceptable.

DIF: Understanding

OBJ: 3.8

TOP: Assessment

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Coping

2. A female patient is admitted to the emergency department after being raped by a neighbor. The patient refuses to discuss the circumstances surrounding the event with the sexual assault nurse examiner. The nurse identifies that the patient is utilizing which defense mechanism?
 - a. Suppression
 - b. Sublimation
 - c. Displacement
 - d. Rationalization

ANS: A

Suppression is the conscious decision to conceal unacceptable or painful thoughts. The patient refuses to talk about the rape possibly because of the emotional and physical pain associated with the act. Sublimation is the rechanneling of unacceptable impulses into socially acceptable activities. Displacement is an unconscious defense mechanism used to avoid conflict and anxiety by transferring emotions from one object to another object that produces less anxiety. Rationalization is the act of suggesting a different explanation for one that is painful, negative, or unacceptable.

DIF: Understanding

OBJ: 3.8

TOP: Assessment

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Coping

3. A patient calls the nurse to report the smell of cigarette smoke in the bathroom. The nurse recognizes that this component of the communication process is identified by which term?
 - a. Channel
 - b. Referent
 - c. Message

d. Feedback

ANS: B

The elements of the communication process include a referent (i.e., event or thought initiating the communication), a sender (i.e., person who initiates and encodes the communication), a receiver (i.e., person who receives and decodes, or interprets, the communication), the message (i.e., information that is communicated), the channel (i.e., method of communication), and feedback (i.e., response of the receiver).

DIF: Understanding

OBJ: 3.1

TOP: Assessment

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Communication

4. The nurse manager sends an e-mail to the nursing staff as a reminder for a scheduled monthly meeting. In doing so, the nurse manager understands that e-mail could result in which issue?
- It is usually slower than other methods of communication.
 - It has the potential for miscommunication.
 - It cannot be used to deliver vital information.
 - It is especially effective because of the absence of nonverbal cues.

ANS: B

A message is the content transmitted during communication. Messages are transmitted through all forms of communication, including spoken, written, and nonverbal modalities. Electronic communication in the form of information referencing, e-mail, social networking, and blogging can quickly contribute to a person's knowledge, providing patients and health care professionals with vital information. However, the potential for miscommunication exists, in part because nonverbal cues are not apparent.

DIF: Understanding

OBJ: 3.1

TOP: Assessment

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Communication

5. The nursing student has been assigned to help feed patients at lunch time. Which nursing intervention would be most effective when assisting a blind patient to eat a meal?
- Speak loudly to ensure that the patient understands.
 - Describe the food arrangement using the numbers on a clock.
 - Tell the patient what is on the plate since he has lost the sense of smell.
 - Encourage the patient to eat faster so that the task will be done.

ANS: B

An important factor to remember when caring for visually impaired or blind patients is that they are rarely hearing impaired. Typically, blind patients have heightened auditory and olfactory senses. Communication with blind patients can be characterized as anticipatory in nature, meaning that the nurse should alert visually impaired patients of potential hazards or object locations to provide necessary information and safe care. For example, the nurse may inform the visually impaired patient that the meat entrée is in the 6 o'clock position and the coffee cup is at 2 o'clock on the tray. This system may be helpful in orienting blind patients to their hospital rooms or informing them of where their food is on a plate or tray.

DIF: Applying

OBJ: 3.9

TOP: Implementation

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Caregiving

6. The nurse observes a confused patient pacing back and forth in the dining room. The patient yells, —The doctor is going to make us all drink poison!! The most appropriate intervention by the nurse at this time would be to take what action?
- Ask the patient why he would say something like that.
 - Change the subject to disrupt the patient's thought process.
 - Tell the patient that he should probably think of something else.
 - Quietly ask the patient to explain the statement.

ANS: D

Seeking clarification encourages the patient to expand on a topic that may be confusing or that seems contradictory. Asking —why! questions implies criticism, may make the patient defensive, tends to limit conversation, requires justification of actions, and focuses on a problem rather than a possible solution. Changing the subject avoids exploration of the topic raised by the patient and demonstrates the nurse's discomfort with the topic introduced by the patient. Giving advice implies a lack of confidence in the patient to make a healthy decision.

DIF: Applying OBJ: 3.7 TOP: Implementation
MSC: NCLEX Client Needs Category: Psychosocial Integrity NOT: Concepts: Communication

7. A patient with an inoperable brain tumor says to the nurse, —I just want to die now. It's going to happen soon anyway. Which would be the most appropriate response?
- Don't worry about that right now. It'll be OK.
 - I disagree with what you just said!
 - Honey, now don't you talk like that.
 - Tell me why you are saying that.

ANS: D

Using open-ended questions or comments gives the patient the opportunity to share freely on a subject, avoids interjection of feelings or assumptions by the nurse, and provides for patient elaboration on important topics when the nurse wants to collect a breadth of information. Giving false reassurance discounts the patient's feelings, cuts off conversation about legitimate concerns of the patient, and demonstrates a need by the nurse to —fix something that the patient just wants to discuss. Showing agreement or disagreement discontinues patient reflection on an introduced topic, and implies a lack of value for the thoughts, feelings, or concerns of patients. Using personal terms of endearment, such as —Honey, demonstrates disrespect for the individual, diminishes the dignity of a unique patient, and may indicate that the nurse did not take the time or care enough to learn or remember the patient's name.

DIF: Applying OBJ: 3.7 TOP: Implementation
MSC: NCLEX Client Needs Category: Psychosocial Integrity NOT: Concepts: Communication

8. The nurse is caring for a patient with chronic lung disease. When the patient demands a cigarette after eating breakfast, the nurse responds, —If that was me, I wouldn't be asking for a cigarette. That is what has made you so sick in the first place. This nontherapeutic response is an example of what communication technique?
- Changing the subject
 - Giving advice
 - A stereotypical response
 - Defensiveness

ANS: B

Giving advice implies that the patient cannot make his or her own decisions and the nurse accepts the responsibility for the action. Changing the subject ignores the patient's concerns. Stereotypical or generalized responses such as, —Don't cry over spilled milk may be judgmental. A defensive response such as, —The nurses work very hard to take care of you moves the focus of the conversation from the patient and limits further discussion.

DIF: Remembering

OBJ: 3.7

TOP: Assessment

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Communication

9. The nurse identifies what measurement to be an acceptable personal space distance for most English-speaking persons?
- 14 inches
 - 18 inches
 - 21 inches
 - 24 inches

ANS: B

Proxemics refers to the amount of space or distance acceptable to two or more individuals based on cultural standards and personal preferences. Most English-speaking persons consider 18 inches to be an acceptable distance for communication. In general, intimate space is 0 to 1.5 feet; personal space is 1.5 to 4 feet; social space is 4 to 12 feet; and public space is 12 to 25 feet or more.

DIF: Remembering

OBJ: 3.2

TOP: Assessment

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Communication

10. The nurse understands that based on a patient's perception of professional competence and caring, the nurse should wear which item?
- Large, dangling, hoop earrings
 - Bright, multicolored acrylic fingernails
 - Clean, neatly pressed uniform
 - Offensive tattoos that cannot be covered

ANS: C

Professional symbolic expressions often communicate self-worth and pride. A clean uniform demonstrates a competent and caring demeanor. Patients consistently judge health care professionals by their appearance. The use of large amounts of jewelry, fake fingernails, and visible body markings, including body piercings, are generally not considered appropriate attire in the nursing profession.

DIF: Applying

OBJ: 3.2

TOP: Implementation

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Communication

11. The nurse is admitting a patient with a foul smelling leg wound. Which behavior by the nurse indicates an understanding of appropriate body language?
- Using hand gestures to enhance verbal communication
 - Standing at the end of the bed with arms crossed
 - Facial grimacing at the sight of the wound
 - Gentle touching of the patient's shoulder

ANS: D

Therapeutic touch, such as holding the patient's hand or touching the patient's shoulder, can provide comfort and may alleviate pain. This is especially true when a patient is undergoing a painful or stressful procedure. Making inappropriate facial expressions may be offensive and hurtful to patients or their family members. The nurse must control his or her facial expressions to avoid communicating disdain or judgmental attitudes in challenging patient care situations. Maintaining a neutral facial expression establishes an environment of caring and openness in which the patient and family members can feel safe to share their innermost concerns. The use of gestures may be challenging to nurses practicing in a multicultural environment. Although they may enhance verbal communication, gestures may be viewed as inappropriate by patients of various cultures. Standing with crossed arms may be indicating a lack of openness or acceptance.

DIF: Applying OBJ: 3.2 TOP: Implementation

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Communication

12. Several nurses on a medical-surgical unit have been asked by the nurse manager to form a group and gather data regarding patient complaints of late meals. When the nurses meet and establish ground rules, this would be what phase of group development?
- Forming
 - Storming
 - Norming
 - Performing

ANS: A

Tuckerman's model of group performance includes forming, storming, norming, and performing. In the forming phase, there is little agreement on team goals other than those received from the leader, and there is a high dependence on the leader for guidance and direction. Ground rules are established and trust among the members begins to develop. There is unrest in the storming phase as the individual team members struggle for power and form cliques. Decisions do not come easily at this stage. In the norming phase, the leader plays a facilitating and enabling role as the team begins to agree and engage in group decisions. Both commitment and unity are strong. The team, in the performing phase, has a shared vision and works together to achieve the goals.

DIF: Remembering

OBJ: 3.3

TOP: Assessment

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Communication

13. A nurse has been working with a patient for the entire shift. Which action by the nurse is unacceptable?
- Sharing a personal mobile phone number
 - Touching the patient's hand during a painful procedure
 - Standing 6 feet away from the patient when conversing
 - Using the SBAR method of hand-off communication

ANS: A

Professional role boundaries define the limits and responsibilities of nurses within a specific setting. It is unprofessional and unethical to share personal phone numbers or meet with patients outside of the health care setting. Therapeutic touch, such as holding the patient's hand or touching the patient's shoulder, can provide comfort and may alleviate pain. This is especially true when a patient is undergoing a painful or stressful procedure. Conversing 6 feet away is appropriate because it falls in the realm of social space; intimate space is 0 to 1.5 feet, personal space is 1.5 to 4 feet, and public space is 12 feet or more. One method of interpersonal communication that has been adopted to increase interprofessional and hand-off communication is the SBAR model (situation, background, action/assessment/awareness, and recommendation).

DIF: Applying OBJ: 3.3 TOP: Implementation
MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
Concepts: Ethics

14. During a shift report, the nurse briefly describes the history of a patient admitted with chronic gastrointestinal bleeding. In which SBAR topical area would this information be presented?
- Situation
 - Background
 - Assessment
 - Recommendation

ANS: B

The —B in SBAR stands for —Background, or what led up to the current situation. The —S stands for Situation or what is happening right now. The —A stands for —Assessment, or what is the identified problem, concern, or need. The —R stands for —Recommendation, or what actions or interventions should be initiated to alleviate the problem.

DIF: Remembering OBJ: 3.3 TOP: Assessment
MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
Concepts: Communication

15. The nurse is performing an abdominal assessment on a postoperative surgical patient. The nurse notes that the dressing needs to be changed twice a day and discusses when the patient would like to have it done. The nurse then plans to change the dressing at that time. In which phase of the nurse–patient helping relationship would this process occur?
- Introductory phase
 - Orientation phase
 - Working phase
 - Termination phase

ANS: C

In the working phase, there is the development of a contract or plan of care to achieve identified patient goals; implementation of the care plan or contract; collaborative work among the nurse, patient, and other health care providers, as needed; enhancement of trust and rapport between the nurse and the patient; reflection by the patient on emotional aspects of illness; and use of therapeutic communication by the nurse to keep interactions focused on the patient. In the orientation phase or introductory phase, introductions are made, establishing professional role boundaries (formally or informally) and expectations, and clarifying the role of the nurse. Identifying the needs and resources of the patient through observing, interviewing, and assessing the patient, followed by validation of perceptions also occur in this phase. Termination involves alerting the patient to impending closure of the relationship, evaluating the outcomes achieved during the interaction, and concluding the relationship and transitioning patient care to another caregiver, as needed.

DIF: Understanding

OBJ: 3.5

TOP: Evaluation MSC:

NCLEX Client Needs Category: Physiological Integrity: Basic Care and Comfort NOT:

Concepts: Communication

16. The nurse is collaborating with a patient to determine interventions to ensure compliance with medication administration after the pending discharge. The nurse understands that the goals and nursing interventions would be agreed upon in which phase of the nurse–patient relationship?
- Preinteraction phase
 - Orientation phase
 - Working phase
 - Termination phase

ANS: D

NRIGBCM
USNT O

Termination phase involves alerting the patient to impending closure of the relationship, evaluating the outcomes achieved during the interaction, and concluding the relationship and transitioning patient care to another caregiver, as needed. In this case, the —new caregiver is the patient. The working phase involves the development of a contract or plan of care to achieve identified patient goals; implementation of the care plan or contract; collaborative work among the nurse, patient, and other health care providers, as needed; enhancement of trust and rapport between the nurse and the patient; reflection by the patient on emotional aspects of illness; and use of therapeutic communication by the nurse to keep interactions focused on the patient. In the orientation phase or introductory phase, introductions are made, establishing professional role boundaries (formally or informally) and expectations, and clarifying the role of the nurse. Identifying the needs and resources of the patient through observing, interviewing, and assessing the patient, followed by validation of perceptions also occur in this phase.

DIF: Understanding

OBJ: 3.5

TOP: Evaluation

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Caregiving

17. A patient complains that several staff members entered the room during the morning bath without knocking. Which component of professional nursing communication has been violated in this scenario?
- Collaboration
 - Advocacy
 - Assertiveness

d. Respect

ANS: D

Respect for the patient includes providing privacy during procedures such as a bath. It is considered respectful to knock on a patient's door prior to entering the room. Assertive communication allows for the expressions of feelings and ideas without hurting or judging. Collaboration refers to the interactions with patients and health care workers to accomplish mutually acceptable goals. Advocacy involves defending the rights of others, especially those who are vulnerable or unable to make decisions independently.

DIF: Understanding

OBJ: 3.6

TOP: Evaluation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Caregiving

18. The nurse is caring for a patient who is unable to take oral medications because of persistent nausea and vomiting. When the nurse decides to call the primary care physician and ask for a different medication administration route, this is a demonstration of what act?
- Collaboration
 - Delegation
 - Assertiveness
 - Advocacy

ANS: D

The nurse acts as a patient advocate by promoting what is best for the patient and ensuring that the patient's needs are met. Since the patient is unable to take medications by mouth, it is the nurse's responsibility to inform the physician and obtain alternative medication routes, as appropriate. Assertive communication allows for the expressions of feelings and ideas without hurting or judging. Collaboration refers to the interactions with patients and health care workers to accomplish mutually acceptable goals. Delegation is the art of transferring responsibility of an assigned task to another while at the same time retaining accountability for the outcome.

DIF: Understanding

OBJ: 3.6

TOP: Evaluation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Caregiving

19. The nurse is assisting a co-worker who is preparing to change a deep wound dressing on a patient's abdomen. Several of the patient's out-of-town friends are at the bedside watching a football game. Which action is most appropriate for the nurse to consider prior to the dressing change?
- Ask the friends to leave the room.
 - Pull the curtain around the bed.
 - Allow visitors to stay in the room during the procedure.
 - Ask the patient to turn up the volume on the television.

ANS: A

It is appropriate for the nurse to ask visitors to leave a patient's room for a few minutes. Several factors affect the location appropriate for communication with patients. Privacy and confidentiality are critical during the interviewing and assessment process. Simply pulling a cubicle curtain around a patient's bed does not prevent the transmission of sound beyond the curtain. Make every effort to talk with patients in an environment with as few interruptions and distractions as possible. Ask the patient to turn off competing technology and to focus on the nurse-patient interaction as needed.

DIF: Applying OBJ: 3.4 TOP: Implementation
MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
Concepts: Caregiving

20. The nurse is conducting a presurgical screening interview with a patient at a local surgical center. When performing a health assessment, the nurse identifies which source should be the primary source of information?
- Spouse
 - Medical record
 - Close relative
 - Patient

ANS: D

The primary source from which data are collected is the patient. A secondary source would include a significant other, family members, caregivers, other members of the health team, and medical records.

DIF: Remembering OBJ: 3.4 TOP: Assessment
MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
Concepts: Caregiving [NURSINGTB.COM](https://www.nursingtb.com)

21. A mother of a young child kicks a trashcan in anger and says to the nurse, —You just don't understand! Why can't the doctor find out what is wrong with my child?‖ The nurse understands that this behavior is most likely an example of which defense mechanism?
- Suppression
 - Sublimation
 - Displacement
 - Rationalization

ANS: C

Displacement is an unconscious defense mechanism used to avoid conflict and anxiety by transferring emotions from one object to another object that is less anxiety-producing. The mother is upset that the health care team is not able to determine the cause of her child's illness and expresses her anger by kicking the trashcan. Suppression is the conscious decision to conceal unacceptable or painful thoughts. Sublimation is the rechanneling of unacceptable impulses into socially acceptable activities. Rationalization is the act of suggesting a different explanation for one that is painful, negative, or unacceptable.

DIF: Understanding OBJ: 3.8 TOP: Assessment
MSC: NCLEX Client Needs Category: Psychosocial Integrity NOT: Concepts: Coping

22. The nurse is caring for a patient scheduled for a partial mastectomy resulting from advanced cancer. The patient tells the nurse, —I'm sure when the surgeon operates on me, he will not find any cancer in my breast. It looks just fine.‖ The nurse recognizes that the patient is using which defense mechanism to cope with the medical diagnosis?
- Suppression
 - Sublimation
 - Displacement
 - Denial

ANS: D

The patient is refusing to admit that the breast has to be removed because of cancer. This inability to accept the truth is termed *denial*. Displacement is an unconscious defense mechanism used to avoid conflict and anxiety by transferring emotions from one object to another object that is less anxiety producing. Suppression is the conscious decision to conceal unacceptable or painful thoughts. Sublimation is the rechanneling of unacceptable impulses into socially acceptable activities.

DIF: Understanding

OBJ: 3.8

TOP: Assessment

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Coping

MULTIPLE RESPONSE

1. The nurse understands that the nurse–patient relationship focuses on which areas? (*Select all that apply.*)
- Building trust
 - Demonstrating sympathy
 - Tearing down boundaries
 - Developing a plan of care
 - Applying cultural generalities

ANS: A, C, D

A helping relationship develops through ongoing, purposeful interaction between a nurse and a patient. The focal point of the nurse–patient helping relationship is the patient and the patient's needs and concerns. Nurse–patient relationships focus on five areas: (1) building trust, (2) demonstrating empathy, (3) establishing boundaries, (4) recognizing and respecting cultural influences, and (5) developing a comprehensive plan of care.

DIF: Understanding

OBJ: 3.4

TOP: Assessment

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Communication

2. When administering a bath to a hearing-impaired patient, what actions should the nurse carry out? (*Select all that apply.*)
- Speak very loudly into the patient's right ear.
 - Control background noise as much as possible.
 - Turn away when responding to a question.
 - Adjust the lighting in the room.
 - Be wary of consistent affirmative answers.

ANS: B, D, E

When communicating with a hearing-impaired patient, the nurse should make sure that the area is well lit with as little background noise as possible. Hearing aids amplify all sounds, making noisy environments confusing and frustrating. Raising the voice level slightly, speaking clearly, and making sure that the patient can see the nurse's face helps to facilitate communication. Adequate lighting enhances the patient's ability to see the speaker's mouth and face and interpret nonverbal communication. Consistent affirmative answers to the nurse's questions may be an indication that the patient is not hearing the information being shared. Care should be taken to verify that patients truly understand the content of verbal interaction. Extra patience may be required by the nurse to demonstrate caring while communicating with hearing-impaired patients.

DIF: Applying OBJ: 3.9 TOP: Implementation

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Communication

3. The nursing student is writing a report on the use of nonverbal techniques to encourage therapeutic communication. Which examples would be included in the report? (*Select all that apply.*)
- a. Providing a backrub
 - b. Remaining silent
 - c. Refraining from distracting body movements
 - d. Facing the patient
 - e. Avoiding eye contact

ANS: A, B, C, D

Providing a backrub is considered therapeutic touch; additional examples include holding a patient's hand and gently touching a patient's arm. Silence refers to being present with a patient without verbal communication. Facing the patient and refraining from unusual body movements are active listening techniques. Avoiding eye contact does not facilitate communication.

DIF: Remembering

OBJ: 3.2

TOP: Assessment

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Communication

Chapter 04: Critical Thinking in Nursing

Yoost & Crawford: Fundamentals of Nursing: Active Learning for Collaborative Practice, 3rd Edition

MULTIPLE CHOICE

1. The patient is complaining of severe incisional pain 2 days after surgery. The patient has Morphine ordered intravenously or by mouth. When the nurse chooses to give the medication orally, this is an example of which thought process?
 - a. Decision making
 - b. Reasoning
 - c. Problem solving
 - d. Judgment

ANS: A

Decision making requires choosing a solution to a problem. Reasoning is the process by which a nurse is able to focus and filter information and determine what is most important to consider. A systematic, analytic approach in finding solutions is termed *problem solving*, and judgment is the process of forming an opinion by comparing solutions through reasoning.

DIF: Remembering

OBJ: 4.1

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Safety and Infection Control

NOT: Concepts: Clinical Judgment

2. The nurse is reviewing the last 3 days of a patient's pain history and notes that the pain level has remained constant. The nurse validates the pain level with the patient and decides to contact the provider for further orders. In this scenario, which process is the nurse using?
 - a. Decision making
 - b. Reasoning
 - c. Problem solving
 - d. Judgment

ANS: D

Processes dependent on critical thinking include problem solving, decision making, reasoning, and judgment. Judgment is the process of forming an opinion by comparing solutions through reasoning. The nurse observes that the patient's pain level is not decreasing and further assesses the pain level through discussions with the patient. The nurse concludes that the patient's pain should be further addressed and contacts the provider. Decision making requires choosing a solution to a problem. The student is making a decision by reviewing two pertinent resources related to the removal of staples. Reasoning is the process by which a nurse links thoughts, ideas and facts together in a logical way. A systematic approach in finding solutions is termed *problem solving*.

DIF: Remembering

OBJ: 4.1

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Safety and Infection Control

NOT: Concepts: Clinical Judgment

3. The nurse has been hired for a first job and is nervous about making errors in clinical judgment. It is important for the nurse to realize that clinical reasoning and the ability to make decisions in a clinical setting occurs at which time?

- a. When it has been instilled in the content covered in nursing school.
- b. When it is solely based in clinical experience.
- c. When it develops over time with increased knowledge and expertise.
- d. When it is an expectation of all nurses regardless of experience.

ANS: C

Clinical reasoning uses critical thinking, knowledge, and experience to develop solutions to problems and make decisions in a clinical setting. A nurse's clinical-reasoning skills develop over time with increased knowledge and expertise.

DIF: Understanding

OBJ: 4.1

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Safety and Infection Control

NOT: Concepts: Clinical Judgment

4. The nurse is taking an advanced cardiac life support (ACLS) recertification class. As part of that class, the nurse and other nurses in the group rotate responsibilities during multiple mock code exercises simulating cardiac arrest scenarios. The nurse recognizes what process is assigning the nurses to these different responsibilities?
- a. Concept mapping
 - b. Simulation
 - c. Role playing
 - d. Literature review

ANS: C

A role-play strategy involves assigning learners to different roles based on expected outcomes in a particular setting. Other learners and facilitators observe the role playing, and then all are involved in the debriefing or discussion of the scenario. As with simulation, this approach

allows learners to interact in a safe, controlled environment. The concept map is a way to organize and visualize data to identify relationships and solve problems. Simulated experiences enable the student to apply previously learned content in a safe and realistic environment that allows time for questioning, clarifying, and feedback. Students develop confidence in providing direct nursing care. Because critical thinking cannot occur about subjects that are unknown, a review of literature may foster this type of thinking by addressing knowledge deficits.

DIF: Understanding

OBJ: 4.7

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

5. The nurse is preparing to administer an anticoagulant when the patient says, —Why do I have these bruises on my arms? The nurse reviews the patient's blood tests and notes an abnormal bleeding time. When the nurse then decides to hold the medication and notify the health care provider, the nurse recognizes this to be an example of which action?
- a. Thinking aloud
 - b. Reviewing the literature
 - c. Applying knowledge
 - d. Role playing

ANS: C

Nursing practice is based on the application of knowledge to address patient problems. In this case, the nurse is able to connect the medication, physical signs and laboratory data to determine a course of action. Nurses may —think aloud as an inner dialogue to examine their thinking. The literature review is used to address knowledge gaps through the review of scholarly journals. A role-play strategy involves assigning learners to different roles based on expected outcomes in a particular setting. Other learners and facilitators observe the role playing, and then all are involved in the debriefing or discussion of the scenario.

DIF: Understanding

OBJ: 4.5

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

6. The nurse is preparing to restart a patient's intravenous line and discovers that the patient has no usable veins in either arm. When working to solve this problem, the nurse should carry out which action?
- Discuss the problem with the nurse in charge.
 - Not start the intravenous line.
 - Conduct an Internet search for infusion journal articles.
 - Contact the provider and report the concern.

ANS: A

Whether in an academic setting or in the clinical area, discussion of a problem, issue, or situation with colleagues may improve critical thinking. Through dialogue with others who have expertise or experience with the issue being faced, knowledge gaps can be filled, erroneous assumptions exposed, and unconscious biases addressed. Not starting the intravenous line is not an option at this point. A literature review to gain published information about intravenous complications may be appropriate after the patient's concern has been addressed. Initially contacting the provider without fully exploring the options for alternate insertion sites is neither wise nor recommended.

DIF: Applying

OBJ: 4.7

TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

7. The nurse has finished a shift and is on the way home. During the shift, one of the patients attempted to climb out of bed and fell. When the nurse is returning home and is thinking about what could have been done differently to prevent the fall, this would be an example of what concept?
- Evidence
 - Standards
 - Attributes or traits
 - Reflection

ANS: D

Reflection is an effective tool that enables students and nurses to think about how best to improve their future caregiving in similar situations. The results of deliberate thinking are used to guide further thinking. Identification and use of evidence is necessary to guide analysis of situations and decision making. Nursing practice must be based on evidence gained through research and review of findings. Some personal characteristics are associated with critical thinking. Critical thinking needs to be assessed and evaluated according to standards to ensure the quality of thinking. Nursing practice is based on standards established by the American Nurses Association in areas such as the nursing process, ethics, education, research, communication, leadership, and collaboration.

DIF: Applying OBJ: 4.2 TOP: Implementation
MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
Concepts: Care Coordination

8. When working on the ability to critically think, the nurse needs to develop a critical-thinking character that includes which quality?
- Developing honesty and confidence
 - Learning from experiences
 - Enhancing self-reliance
 - Growing a —thick skin to withstand criticism

ANS: A

To develop critical thinking, the nurse needs to develop a critical-thinking character, which includes maintaining high standards and developing critical-thinking qualities such as honesty, fair-mindedness, creativity, patience, persistence, and confidence. The next step in the development of critical thinking includes taking responsibility for personal learning and seeking needed experiences that can provide the necessary knowledge on which to base the thinking. Fostering interpersonal skills, such as teamwork, conflict management, and advocacy, is important in the development of critical thinking. Self-evaluation and having thinking evaluated by others require the ability to accept and use constructive criticism.

DIF: Applying OBJ: 4.2 TOP: Implementation
MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
Concepts: Care Coordination

9. The nurse is caring for a patient scheduled for a heart catheterization. During shift report, the nurse describes an overheard telephone conversation regarding the patient's HIV-positive son-in-law. The nurse recognizes that this information should be evaluated for which characteristic?
- Accuracy
 - Depth
 - Breadth
 - Relevance

ANS: D

Relevance is focusing on facts and ideas directly related and pertinent to a topic—how is this related to the question? The son-in-law's HIV status has no bearing on the patient's care. Accuracy involves representing something in a true and correct way. Depth is getting beneath the surface of the topic or problem to identify and manage related complexities, whereas breadth involves considering a topic, problem, or issue from every relevant viewpoint.

DIF: Applying OBJ: 4.3 TOP: Implementation
MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
Concepts: Care Coordination

10. A patient arrives at the urgent care clinic and complains of vague pains in the legs and the nurse asks the patient to describe this pain in greater depth. The nurse knows this is a critical-thinking skill and can be developed in which context?
- Critical thinking is used to avoid repetition in providing care.
 - Critical thinking can be enhanced through practice.
 - Critical thinking should be based in thought and not spontaneity.
 - Critical-thinking skills become dull when used routinely.

ANS: B

The ultimate goal is for these questions to become so spontaneous in thinking that they form a natural part of our inner voice, guiding us to better reasoning. As with any skill, critical thinking can be enhanced through practice. The routine use of these questions should promote critical thought.

DIF: Understanding OBJ: 4.3 TOP: Evaluation
MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
Concepts: Care Coordination

11. The nurse is planning care for a group of patients and recognizes which activity may be delegated to unlicensed assistive personnel?
- Analysis of the patient's physical condition
 - Morning vital signs, height, and weight
 - Evaluation of whether colostomy drainage is normal
 - Determining patient readiness for post-surgical learning

ANS: B

The nurse often works with unlicensed assistive personnel (UAP) to collect relevant data on height and weight, intake and output, and vital signs. The registered nurse uses critical thinking to guide decisions related to delegation of assignments and tasks. Before delegation of a task, the nurse must be knowledgeable about the role, scope of practice, and competency of the recipient of the delegated task. Analysis and evaluation of patient conditions and readiness for teaching require critical thinking and are nursing functions.

DIF: Understanding OBJ: 4.5 TOP: Evaluation
MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
Concepts: Care Coordination

12. The nurse is caring for a patient who is suspected of having early stages of dementia and observes mild confusion, short-term memory loss, and restlessness. When the nurse conducts a mini-mental status exam, the nurse is using which component of critical thinking?
- Validation
 - Interpretation
 - Intuition
 - Reasoning

ANS: A

Validation is the process of gathering information to determine whether the information or data collected are factual and true. Examining how information is organized and given meaning guides the interpretation of the information. Interpretations must be differentiated from facts and evidence because they are based on personal conceptions, experiences, and perspective. Intuition is the feeling that you know something without specific evidence. The terms *thinking* and *reasoning* are often used synonymously, although reasoning is more formal because it usually is aimed at finding answers, providing explanations, and forming conclusions.

DIF: Understanding

OBJ: 4.4

TOP: Evaluation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

13. The nursing student is admitting a patient with abdominal distention and severe nausea. The provider orders the insertion of a nasogastric tube. The student reviews the procedure, gathers the supplies, and tells the instructor, —I'm ready to begin. Which critical-thinking trait suggests that the student is prepared for the task?
- Risk taking
 - Curiosity
 - Confidence
 - Perseverance

ANS: C

Confidence is feeling certain about one's ability to accomplish a goal. The student stating —I'm ready indicates this. Risk taking involves being willing to try new ideas. Curiosity is being motivated to achieve and asking why. Perseverance is staying determined to work until

the goal is achieved.

NURSINGTB.COM

DIF: Understanding

OBJ: 4.4

TOP: Evaluation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

14. A patient has been instructed in self-administration of insulin injections. The nurse observes the patient attempting to recap the needle and realizes that further teaching is needed. The nurse is applying which critical-thinking skill of the nursing process?
- Interpretation
 - Analysis
 - Inference
 - Evaluation

ANS: D

Evaluation occurs when information, including the reliability, credibility, and bias of the source, is assessed. Nurses also evaluate when determining whether the desired outcome for an intervention was achieved. Recapping the needle is not part of the desired outcome.

Analysis includes investigating plans of action on the basis of examination of subjective and objective data. *Interpretation* is used to understand and explain the meaning of data. *Inference* leads to accurate conclusions that are based on sound reasoning.

DIF: Understanding

OBJ: 4.5

TOP: Evaluation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

15. The nurse completes the health interview and physical exam on a patient admitted with an infection of the gallbladder. The nurse reviews the medical record and compares the abnormal lab results to the normal standards. Which critical-thinking skill is the nurse using in this part of the nursing process?
- Interpretation
 - Analysis
 - Evaluation
 - Inference

ANS: B

Analysis includes investigating plans of action on the basis of examination of subjective and objective data is an example of nursing analysis. *Interpretation* is used to understand and explain the meaning of data. *Evaluation* occurs when information, including the reliability, credibility, and bias of the source, is assessed. Nurses also evaluate when determining whether the desired outcome for an intervention was achieved. Recapping the needle is not part of the desired outcome.

Inference leads to accurate conclusions that are based on sound reasoning.

DIF: Understanding

OBJ: 4.5

TOP: Evaluation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

16. A patient, frequently admitted to the hospital for chronic back pain, asks the medication nurse for additional pain medication. The nurse has seen patients like this before, and —knows that the only reason that these people come to the hospital is to get their pain medication. In this scenario, the nurse is demonstrating which concept?
- Illogical thinking
 - Bias
 - Closed-mindedness
 - Erroneous assumption

NURSINGTB.COM

ANS: B

Decisions may be unduly influenced by bias, which is an inclination or tendency to favoritism or partiality. Bias may be related to a preconceived notion or prejudice such as believing that —these people seek their medication. It is important for nurses to examine personal biases because they can negatively impact care. Illogical thinking is characterized by a failure to follow rational, systematic processes when approaching an issue or problem. Often making hasty generalizations and assumptions that do not consider the evidence, the illogical thinker may jump to conclusions. Errors in thinking and decision making can result from intentionally overlooking alternatives suggested by others. When relevant information from patients or experts is ignored because of closed-mindedness, nursing care can be compromised.

Closed-minded individuals often believe that their way is the best and preferred way. Assumptions are beliefs that are taken for granted and assumed to be true. Assumptions can be unjustified or justified, depending on whether there are good reasons for them. Erroneous assumptions can lead to safety issues in the clinical setting.

DIF: Understanding

OBJ: 4.6

TOP: Evaluation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

17. The nurse has received advanced orders for a patient who is being admitted from the emergency department (ED). The patient's name is Mr. Herman Goldstein. Trying to get ahead, of tasks, the nurse changes the patient's diet from —Regular to —Kosher. When the patient reaches the unit, the nurse discovers that the patient is Catholic even though his father is Jewish. The nurse is guilty of giving in to which concept?
- Illogical thinking
 - Bias
 - Closed-mindedness
 - Erroneous assumption

ANS: D

Assumptions are beliefs that are taken for granted and assumed to be true. Assumptions can be unjustified or justified, depending on whether there are good reasons for them. Erroneous assumptions can lead to safety issues in the clinical setting. Illogical thinking is characterized by a failure to follow rational, systematic processes when approaching an issue or problem. Often making hasty generalizations and assumptions that do not consider the evidence, the illogical thinker may jump to conclusions. Decisions may be unduly influenced by bias, which is an inclination or tendency to favoritism or partiality. Bias may be related to a preconceived notion or prejudice against a group of people. It is important for nurses to examine personal biases because they can negatively impact care. Errors in thinking and decision making can result from intentionally overlooking alternatives suggested by others. When relevant information from patients or experts is ignored due to closed-mindedness, nursing care can be compromised. Closed-minded individuals often believe that their way is the best and preferred way.

DIF: Understanding

OBJ: 4.6

TOP: Evaluation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

18. The nursing student is observing a staff nurse demonstrating a subcutaneous injection during a skills competency fair. The student tells the nurse that nursing textbooks indicate that aspirating for blood is not necessary. The nurse replies, —I prefer to check for blood, just in case. This is the way I learned to give shots and it works for me. The nurse's response is most likely related to which concept?
- Illogical thinking
 - Bias
 - Closed-mindedness
 - Erroneous assumption

ANS: C

When relevant information from patients or experts is ignored due to closed-mindedness, nursing care can be compromised. Closed-minded individuals often believe that their way is the best and preferred way. Illogical thinking is characterized by a failure to follow rational, systematic processes when approaching an issue or problem. Often making hasty generalizations and assumptions that do not consider the evidence, the illogical thinker may jump to conclusions. Decisions may be unduly influenced by bias, which is an inclination or tendency to favoritism or partiality. Bias may be related to a preconceived notion or prejudice against a group of people is important for nurses to examine personal biases because they can negatively impact care. Errors in thinking and decision making can result from intentionally overlooking alternatives suggested by others. Assumptions are beliefs that are taken for granted and assumed to be true. Assumptions can be unjustified or justified, depending on whether there are good reasons for them. Erroneous assumptions can lead to safety issues in the clinical setting.

DIF: Understanding

OBJ: 4.6

TOP: Evaluation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

19. The nurse is preparing to teach indwelling urinary catheter insertion techniques to a group of graduate nurses. Which teaching-learning strategy would the nurse find most useful in teaching this skill?
- Concept mapping
 - Simulation
 - Role playing
 - Literature review

ANS: B

NRIGBCM
USNT O

Simulated experiences enable the student to apply previously learned content in a safe and realistic environment that allows time for questioning, clarifying, and feedback. Students develop confidence in providing direct nursing care. The concept map as a way to organize and visualize data to identify relationships and solve problems. Role-play strategies involve assigning learners to different roles based on expected outcomes in a particular setting. Other learners and facilitators observe the role playing, and then all are involved in the debriefing or discussion of the scenario. As with simulation, this approach allows learners to interact in a safe, controlled environment. Because critical thinking cannot occur about subjects that are unknown, a review of literature may foster this type of thinking by addressing knowledge deficits.

DIF: Applying

OBJ: 4.7

TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

20. The nurse is administering medications to a patient with high blood pressure. The patient states, —This pill made me so sick yesterday. Are you sure I have to take it now? What action does the nurse take next?
- Give the medication because no one gets sick on this pill.
 - Hold the medication and check the order since there may be a lack of information.
 - Give the medication since he/she is the nurse and knows what should be done.
 - Give the medication since the nurse did not see the provider come so the order is valid.

ANS: B

Errors in thinking can lead to errors in nursing care. It is essential for the nurse to assess and validate the patient's concerns before proceeding with a planned action. This helps avoid decisions being made on personal biases caused by preconceived notions. The nurse should hold the medication and investigate further. Believing that —no one gets sick on this pill is a preconceived bias. Giving the medication because —I know best is an example of close-mindedness. Assuming the order is valid simply because the nurse did not see the provider is illogical thinking.

DIF: Applying OBJ: 4.6 TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

21. A patient is admitted to a skilled nursing facility with a closed head injury. The nurse believes that the patient has been pocketing food in his cheeks during the noon meal although she has not found any food pocketed. The nurse refers the patient to the speech therapist for a swallowing evaluation. The nurse is using which critical-thinking component in making this decision?
- Inference
 - Deductive reasoning
 - Intuition
 - Inductive reasoning

ANS: C

Intuition is the feeling that you know something without specific evidence. Inferences are intellectual acts that involve a conclusion being made on the basis of something. The accuracy of an inference is directly related to the accuracy of what the inference is based on. Deductive reasoning

involves generating facts or details from a major theory, generalization, or premise (i.e., from general to specific). Inductive reasoning uses specific facts or details to make conclusions and generalizations; it proceeds from specific to general.

DIF: Understanding

OBJ: 4.4

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Safety

22. The nurse observes that a patient who recently had an indwelling urinary catheter removed complains of burning on urination and that the urine is cloudy and foul smelling. Based on this assessment, the nurse may reason that the patient has a urinary tract infection (UTI). The nurse comes to this conclusion using which reasoning concept?
- Inductive reasoning
 - Deductive reasoning
 - Intellectual thought processes
 - Intuition

ANS: A

Inductive reasoning uses specific facts or details to make conclusions and generalizations; it proceeds from specific to general. Deductive reasoning involves generating facts or details from a major theory, generalization, or premise (i.e., from general to specific). Intellectual standards that are essential to critical thinking include clarity, accuracy, precision, relevance, depth, breadth, logic, significance, and fairness. Intuition is the feeling that you know something without specific evidence.

DIF: Understanding

OBJ: 4.5

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Safety

MULTIPLE RESPONSE

1. The nurse recognizes that when a patient is initially interviewed and assessed, the nurse must complete which tasks? (*Select all that apply.*)
 - a. Analyze the patient's psychomotor status.
 - b. Take the patient's vital signs.
 - c. Weigh the patient using a bed scale.
 - d. Evaluate the patient's emotional and spiritual needs.
 - e. Ensure the coordination of the patient's care.

ANS: A, D, E

When a patient is initially interviewed and assessed, the nurse must complete a thorough analysis of the patient's physical, emotional, spiritual, and psychomotor status. The nurse often works with unlicensed assistive personnel (UAP) to collect relevant data on height and weight, intake and output, and vital signs. Nurses collaborate with other health care professionals to coordinate care. Interdisciplinary clinical rounds, which include physicians, registered nurses, physical therapists, occupational therapists, and dietitians, are often undertaken to identify priorities of care, discuss overlapping areas of treatment, and ensure coordination of care.

DIF: Applying

OBJ: 4.2

TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

2. The nurse knows that professional nursing requires a commitment to which reasons for lifelong learning? (*Select all that apply.*)
 - a. Treatment modalities and technology continue to advance.
 - b. There are always new things to memorize and store in memory.
 - c. Nurses are expected to update and maintain competency.
 - d. Critical thinking is essential in nursing.
 - e. Nursing school gives the nurse all one needs to be competent.

ANS: A, C, D

Professional nursing requires a commitment to lifelong learning. Nurses must possess critical-thinking skills to maintain pace with ever-changing treatment modalities and technological advances. Outdated learning strategies that focus on remembering content must be replaced by a focus on understanding the rationales and outcomes. It is an expectation of professional practice that nurses update and maintain their competency and knowledge base. The increasing complexity of health care and information technology make critical thinking essential in nursing. No longer is rote memorization and recall of content sufficient for the complex decisions and judgment required in professional nursing practice. Because knowledge and technology continue to expand for nursing professionals, the content learned in nursing school is not sufficient to maintain competence in nursing practice.

DIF: Understanding

OBJ: 4.5

TOP: Evaluation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care

NOT: Concepts: Care Coordination

3. The nurse has been practicing for several years and has become an unofficial leader, with newer nurses asking for advice about patient care. They are amazed at how much the older nurse —thinks like a nurse. To —think like a nurse, the nurse must carry out which actions? (*Select all that apply.*)
- a. Be a nurse for several years.
 - b. Be able to apply knowledge in making clinical decisions.
 - c. Actively participate in the process.
 - d. Accept procedures that have been in place for years as right.
 - e. Develop a questioning attitude.

ANS: B, C, E

Because nursing requires the application of knowledge to make clinical decisions and guide care, it involves active participation by the nurse. The application of knowledge requires development of a questioning attitude. This process is sometimes referred to as *thinking like a nurse*. —Several years is vague, and nurses develop critical thinking abilities at different rates. A questioning attitude does not accept doing things because they have been done that way for a long time.

DIF: Understanding

OBJ: 4.5

TOP: Evaluation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

NURSINGTB.COM

Chapter 05: Introduction to the Nursing Process

Yoost & Crawford: Fundamentals of Nursing: Active Learning for Collaborative Practice, 3rd Edition

MULTIPLE CHOICE

1. The nurse identifies the nursing process as the foundation of professional nursing practice and can define it in which appropriate terms?
 - a. The framework that nurses use to provide care.
 - b. A complex process during which nurses think about their thinking.
 - c. The process that allows nurses to collect essential data.
 - d. Thinking like a nurse in developing plans of care.

ANS: A

The nursing process is the foundation of professional nursing practice. It is the framework within which nurses provide care to patients in an organized and effective manner. Paul describes critical thinking as a complex process during which individuals think about their thinking to provide clarity and increase precision and relevance in a specific situation while attempting to be fair and consistent. Critical thinking using the nursing process allows nurses to collect essential patient data, articulate the specific needs of individual patients, and effectively communicate those needs, realistic goals, and customized interventions with members of the health care team. *Thinking like a nurse* is facilitated by nurses using the nursing process in the development of individualized patient plans of care.

DIF: Remembering

OBJ: 5.1

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

2. The term *nursing process* was first used in 1955. In 1973, the American Nurses Association identified five specific steps of the process. The nurse knows which essential step was added in 1991?
 - a. Assessment
 - b. Diagnosis
 - c. Outcome identification
 - d. Evaluation

ANS: C

The term *nursing process* was first used by Lydia Hall in 1955. In 1973, the American Nurses Association (ANA) identified five specific steps of the nursing process in its *Standards of Clinical Practice* (1991). These five steps—assessment, diagnosis, planning, implementation, and evaluation—define how professional nursing practice is conducted. Outcome identification was added as an essential aspect of the nursing process by the ANA in 1991. Most nursing professionals and educators recognize outcome identification as part of the planning step of the traditional five-step nursing process.

DIF: Remembering

OBJ: 5.2

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

3. Since the nursing process is cyclic rather than linear, the nurse knows that as an individual patient's condition changes the nurse should anticipate what concept?
- The nurse's thought processes do not have to vary.
 - Plans of care are easier to use and do not need modification.
 - The accuracy and effectiveness of thought processes must be considered.
 - Reflective thought is not necessary since issues tend to be repetitive.

ANS: C

The nursing process is cyclic rather than linear. As an individual patient's condition changes, so does the way a professional nurse thinks about that patient's needs, forcing modification of earlier plans of care. At each step of the nursing process, nurses must consider the accuracy and effectiveness of their thought process. This form of reflective thought is an essential aspect of critical thinking. The evolutionary nature of the nursing process allows nurses to adjust to changing patient needs. Plans of care must evolve as patients' needs change.

DIF: Understanding

OBJ: 5.3

TOP: Planning

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

4. The charge nurse is discussing a patient's care plan during a team meeting. The team determines that the patient has not met the goal of —ambulating to the nurse's station twice a day—and decides to revise the plan. The nurse recognizes which characteristic of the nursing process most represents this decision?
- Organization
 - Dynamics
 - Adaptability
 - Outcome orientation

ANS: D

NURSINGTB.COM

Patient care plans are developed to meet each patient's goals, not the goals of standardized patients or members of the health care team, including the nurse. Decisions regarding which nursing interventions and medical treatments to implement are made on the basis of safety and their effectiveness in meeting a patient's identified needs and desired outcomes. The dynamic, responsive nature of the nursing process allows it to be used effectively with patients in any setting and at every level of care. The plan of care is individualized for the patient on the basis of assessment findings, changing needs, setting, and timing of interaction, not just outcomes. Following the steps of the nursing process ensures that patient care is well organized and thorough. The nursing process is adaptable for developing plans of care for individuals who are hospitalized or are receiving care in an outpatient, long-term care, or home setting. It is an equally useful method for addressing the needs of a specific population.

DIF: Understanding

OBJ: 5.3

TOP: Planning

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

5. The nurse is caring for a patient who will be discharged home following surgical repair of a broken shoulder. The patient tells the nurse, —I don't have anyone at home who can help me cook my meals. Is there something you can do?— When demonstrating the adaptability of the nursing process, the nurse should carry out which task?
- Adjust the patient's care plan so that nursing goals can be met.
 - Consult the care provider about extending the patient's hospitalization.

- c. Abandon the plan of care as not able to be done.
- d. Contact the social worker about community services.

ANS: D

The nursing process is adaptable for developing plans of care for individuals who are hospitalized or are receiving care in an outpatient, long-term care, or home setting. The nurse would adjust planning to contact the social worker for community resources so the patient can maintain as much independence as possible. The care plan focuses on the patient's goals. The provider may or may not be able to extend the hospital stay, but even if that were possible, the patient would not be able to stay until all function returned. The nurse does not simply abandon the care plan; the nurse looks for options and adaptations.

DIF: Applying OBJ: 5.3 TOP: Implementation
MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
Concepts: Care Coordination

6. The community health nurse is applying the nursing process to the care of patients with coronary artery disease. The nurse determines that most of the patients eat high-fat meals from local fast-food restaurants and plans a nutrition workshop. The nurse is applying which characteristic of the nursing process?
- a. Organization
 - b. Dynamics
 - c. Adaptability
 - d. Collaboration

ANS: C

The nursing process is adaptable for developing plans of care for individuals who are hospitalized or are receiving care in an outpatient, long-term care, or home setting. The nurse has planned actions based on the needs of this specific population. Organization is another key concept, however; there is no information in the stem on organization. A care plan should be dynamic, changing over time to meet changing needs. The nurse may or may not have to collaborate with other providers in planning and conducting the seminar, but that is another characteristic of a good nursing care plan.

DIF: Applying OBJ: 5.3 TOP: Implementation
MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
Concepts: Care Coordination

7. The nursing student is caring for a patient admitted with severe anemia. The patient receives two units of packed red blood cells and tells the student, —I am feeling so much better. I'm not so tired anymore and can bathe myself. The student reviews the patient goal —report an increase in activity tolerance and concludes that the patient's goal has been met and adjusts the patient's plan of care. The nurse knows this is applying which characteristic of the nursing process?
- a. Organization
 - b. Dynamics
 - c. Adaptability
 - d. Collaboration

ANS: B

The nursing process is dynamic, reflecting changing conditions and needs of patients. Adjusting the plan of care after an outcome has been met is an example of this. Care plans should be organized. Care plans are adaptable, in that they are useful in multiple settings and with either individual or groups as the patient. Collaboration is a key component of meeting patient outcomes.

DIF: Understanding

OBJ: 5.3

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

8. The nurse is completing the health history on a patient admitted for cardiac rehabilitation. The nurse recognizes that the health history is conducted in which step of the nursing process?
- Assessment
 - Diagnosis
 - Implementation
 - Evaluation

ANS: A

During the assessment step, patient care data are gathered through observation, interviews, and physical assessment. In the diagnosis step, patient data are analyzed, validated, and clustered to identify patient problems. Each problem is then stated in standardized language as a specific Nursing diagnosis to provide greater clarity and universal understanding by all care providers. The implementation step includes initiating specific nursing interventions and treatments designed to help the patient achieve established goals or outcomes. In the evaluation step, the nurse determines whether the patient's goals are met, examines the effectiveness of interventions, and decides whether the plan of care should be discontinued,

continued, or revised.

NURSINGTB.COM

DIF: Understanding

OBJ: 5.4

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

9. The nurse is assisting a patient to bed when the patient says, —My chest hurts and my left arm feels numb. What's wrong with me?|| What is the type and source of data obtained from the patient's complaint?
- Objective data from a primary source
 - Objective data from a secondary source
 - Subjective data from a primary source
 - Subjective data from a secondary source

ANS: C

Objective data consist of observable information that the nurse gathers on the basis of what can be seen, measured, or tested. Subjective data are spoken. Primary data consist of information obtained directly from a patient. Secondary data are collected from family members, friends, other health care professionals, or written sources such as medical records and test results.

DIF: Understanding

OBJ: 5.4

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

10. The nurse identifies which statement to be a correctly written Nursing diagnosis appropriate for a patient's plan of care?
- Ineffective airway clearance related to excessive secretions as evidenced by diminished breath sounds.
 - Imbalanced nutrition: less than body requirements.
 - Impaired physical mobility related to contractures.
 - Risk for suffocation related to smoking in bed as evidenced by absent breath sounds.

ANS: A

There are three types of diagnoses: actual, risk, and opportunities for improvement. Actual diagnoses have three parts: problem, etiology, and signs/symptoms. Risk diagnoses include only the identified need and the risk factors. The Nursing diagnosis, imbalanced nutrition: less than body requirements, is missing the problem, etiology, and signs and symptoms. Impaired physical mobility is missing the evidence. Risk for suffocation should have only two parts: the potential problem and etiology. There are no signs and symptoms if the patient is at risk.

DIF: Analyzing

OBJ: 5.4

TOP: Evaluation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

11. The nurse is admitting a patient experiencing chest discomfort and shortness of breath, who has a history of stroke. When the nurse documents the Nursing diagnosis —Risk for impaired mobility related to history of stroke, the nurse knows which condition to be the risk factor?
- Stroke
 - History of stroke
 - Chest discomfort
 - Shortness of breath

NURSINGTB.COM

ANS: B

A two-part risk, Nursing diagnostic statement contains only: (1) the patient's identified need or problem (i.e., NANDA-I Nursing diagnostic label) and (2) factors indicating vulnerability (i.e., risk factors). The risk factor is the history of stroke. The chest discomfort and shortness of breath are symptoms of the current problems and would not be documented as potential or —risk issues. —Stroke would be the identified potential problem.

DIF: Understanding

OBJ: 5.4

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

12. A patient with a congenital heart defect is admitted for further testing. The nurse observes the patient has increased shortness of breath and is restless. The nurse is demonstrating which phase of the nursing process?

d. Evaluation

ANS: A

- a. Assessment
- b. Planning
- c. Implementation

d. Evaluation

ANS: A

During the assessment step, patient care data are gathered through observation, interviews, and physical assessment. During the planning step of the nursing process, the nurse prioritizes the Nursing diagnoses and identifies short- and long-term goals that are realistic, measurable, and patient focused, with specific outcome identification for evaluation purposes. The implementation step includes initiating specific nursing interventions and treatments designed to help the patient achieve established goals or outcomes. In the evaluation step, the nurse determines whether the patient's goals are met, examines the effectiveness of interventions, and decides whether the plan of care should be discontinued, continued, or revised.

DIF: Understanding

OBJ: 5.4

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

13. While the nurse is assisting with morning care, the patient has a tonic clonic seizure. The nurse immediately turns the patient to a side-lying position. The nurse is demonstrating which phase of the nursing process?
- Assessment
 - Planning
 - Implementation
 - Evaluation

ANS: C

The implementation step includes initiating specific nursing interventions and treatments designed to help the patient achieve established goals or outcomes. During the assessment step, patient care data are gathered through observation, interviews, and physical assessment. During the planning step of the nursing process, the nurse prioritizes the Nursing diagnoses and identifies short- and long-term goals that are realistic, measurable, and patient focused, with specific outcome identification for evaluation purposes. In the evaluation step, the nurse determines whether the patient's goals are met, examines the effectiveness of interventions, and decides whether the plan of care should be discontinued, continued, or revised.

DIF: Understanding

OBJ: 5.4

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

14. The nurse is caring for a patient diagnosed with Lyme disease. The patient tells the nurse, —My heart seems to be skipping some beats. My doctor told me to let the nurse know if this happens since it might be a complication of my disease.‖ The nurse auscultates the heart and confirms the palpitations. Which step of the nursing process does the nurse's action demonstrate?
- Assessment
 - Planning
 - Implementation
 - Evaluation

ANS: A

During the assessment step, patient care data are gathered through observation, interviews, and physical assessment. During the planning step of the nursing process, the nurse prioritizes the Nursing diagnoses and identifies short- and long-term goals that are realistic, measurable, and patient focused, with specific outcome identification for evaluation purposes. The implementation step includes initiating specific nursing interventions and treatments designed to help the patient achieve established goals or outcomes. In the evaluation step, the nurse

c. Implementation

d. Evaluation

ANS: A

During the assessment step, patient care data are gathered through observation, interviews, and physical assessment. During the planning step of the nursing process, the nurse prioritizes the Nursing diagnoses and identifies short- and long-term goals that are realistic, measurable, and patient focused, with specific outcome identification for evaluation purposes. The implementation step includes initiating specific nursing interventions and treatments designed to help the patient achieve established goals or outcomes. In the evaluation step, the nurse determines whether the patient's goals are met, examines the effectiveness of interventions, and decides whether the plan of care should be discontinued, continued, or revised.

DIF: Understanding

OBJ: 5.4

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

15. In which step of the nursing process does the nurse prioritize the Nursing diagnoses and identify interventions to address the patient goals?
- Assessment
 - Planning
 - Implementation
 - Evaluation

ANS: B

During the planning step of the nursing process, the nurse prioritizes the Nursing diagnoses and identifies short- and long-term goals that are realistic, measurable, and patient focused, with specific outcome identification for evaluation purposes. During the assessment step, patient care data are gathered through observation, interviews, and physical assessment. The implementation step includes initiating specific nursing interventions and treatments designed to help the patient achieve established goals or outcomes. In the evaluation step, the nurse determines whether the patient's goals are met, examines the effectiveness of interventions, and decides whether the plan of care should be discontinued, continued, or revised.

DIF: Understanding

OBJ: 5.4

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

16. The nurse writes a short-term goal for a patient scheduled for surgery in the morning and identifies which goal that contains all the necessary elements?
- The patient will walk to the bathroom within 48 hours after surgery.
 - The patient will walk to the bathroom without experiencing shortness of breath within 48 hours after surgery.
 - The patient will walk to the bathroom without experiencing shortness of breath.
 - The patient will walk to the bathroom without experiencing shortness of breath after surgery.

ANS: B

All short- and long-term goals must be: (1) patient focused, (2) realistic, and (3) measurable. For example, a patient-focused, realistic, and measurable short-term goal may be written for a patient with the Nursing diagnosis of *Activity intolerance*: The patient walks to the bathroom without experiencing shortness of breath within 48 hours after surgery.

DIF: Understanding

OBJ: 5.4

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care

NOT: Concepts: Care Coordination

17. A new community health nurse observes that a patient has generalized itching and a red rash after touching a latex glove. When the nurse asks the manager if there is a document written by the physician for this type of reaction, the nurse is referring to which concept?
- Protocol
 - Clinical pathway
 - Standing order
 - Care map

ANS: C

Standing orders are written by physicians and list specific actions to be taken by a nurse or other health care provider when access to a physician is not possible or when care is common to a certain type of situation, such as what to do if a patient experiences chest pain or what actions to take after a colonoscopy. Protocols are written plans that can be generalized to groups of patients with the same or similar clinical needs that do not require a physician's order. Health care agencies have established protocols outlining procedures for admitting patients or handling routine care situations. Clinical pathways, sometimes referred to as care pathways, care maps, or critical pathways, are multidisciplinary resources designed to guide patient care.

DIF: Remembering

OBJ: 5.4

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

18. All nursing interventions that are implemented for patients must be documented or charted. The nurse knows that proper documentation of interventions leads to what positive outcome?
- Proper documentation facilitates communication with all members of the health care team.
 - Proper documentation is only considered —legal if documented in the paper chart.
 - Proper documentation prevents errors of omission and repetition of care.
 - Proper documentation does not directly measure goal achievement or outcomes.

ANS: A

All nursing interventions that are implemented for patients must be documented or charted. In some cases, this may involve checking off an intervention in the patient's EMR designed to track the effectiveness of specific interventions. Many health care agencies have special requirements for documenting interventions such as the use of physical restraints or pain protocols. Proper documentation of interventions facilitates communication with all members of the health care team and provides an essential legal record. Accurate charting helps to alleviate omissions and repetition of care although it cannot prevent them. Documentation also allows nurses to evaluate the effectiveness of nursing interventions in meeting patient goals and outcomes, which is the final step in the nursing process.

DIF: Understanding

OBJ: 5.4

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

19. The nurse makes the following entry on the patient's care plan: —Goal not met. Patient refuses to walk and states, "I'm afraid of falling." The nurse should complete which next action?
- Ignore the patient's concern in evaluating goal attainment.

- b. Document the patient's unwillingness to continue the plan of care.
- c. Continue the plan of care as originally agreed upon.
- d. Modify the care plan in response to the patient's condition and wishes.

ANS: D

Evaluation focuses on the patient and the patient's response to nursing interventions and goal or outcome attainment. If a goal was not met, the care plan needs to be modified to avoid simply repeating the same actions. Ignoring the patient is not a therapeutic response. The nurse should respect the patient's fear and assess further without simply documenting that the patient is unwilling.

DIF: Applying OBJ: 5.4 TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

20. The nurse is demonstrating how to correctly perform deep breathing and coughing exercises to a patient scheduled for back surgery. Which step of the nursing process is the nurse addressing?
- a. Assessment
 - b. Diagnosis
 - c. Implementation
 - d. Evaluation

ANS: C

The implementation step includes initiating specific nursing interventions and treatments designed to help the patient achieve established goals or outcomes. During the assessment step, patient care data are gathered through observation, interviews, and physical assessment.

In the diagnosis step, patient data are analyzed, validated, and clustered to identify patient problems. Each problem is then stated in standardized language as a specific Nursing diagnosis to provide greater clarity and universal understanding by all care providers. In the evaluation step, the nurse determines whether the patient's goals are met, examines the effectiveness of interventions, and decides whether the plan of care should be discontinued, continued, or revised.

DIF: Remembering

OBJ: 5.4

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

21. The nurse develops a list of Nursing diagnoses for a patient receiving intravenous chemotherapy for breast cancer. The patient tells the nurse, —I understand that I will lose most of my hair. Will it grow back?‖ The nurse identifies which diagnosis will have the highest priority?
- a. Disturbed body image
 - b. Nausea
 - c. Risk for bleeding
 - d. Imbalanced nutrition: less than body requirements

ANS: A

Priority of Nursing diagnoses is determined by the patient's preference as well as the severity of the symptoms. The patient is concerned about the loss of hair because this will affect body image. For the patient, this is a prime focus. It is possible that the patient may experience nausea as a result of the chemotherapy drugs. The patient will not be able to eat properly if the nausea is not controlled thus decreasing nutritional intake. There is a potential for bleeding as a result of the low platelet count created by the drugs. All of these must be addressed, but the primary diagnosis, in this case, would be body image.

DIF: Analyzing OBJ: 5.4

TOP: Evaluation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

22. The nurse is gathering data on a patient with acute bacterial pneumonia. The nurse recognizes that this is an example of which step of the nursing process?

- a. Assessment
- b. Planning
- c. Implementation
- d. Evaluation

ANS: A

During the assessment step, patient care data are gathered through observation, interviews, and physical assessment. In the diagnosis step, patient data are analyzed, validated, and clustered to identify patient problems. Each problem is then stated in standardized language as a specific Nursing diagnosis to provide greater clarity and universal understanding by all care providers. The implementation step includes initiating specific nursing interventions and treatments designed to help the patient achieve established goals or outcomes. In the evaluation step, the nurse determines whether the patient's goals are met, examines the effectiveness of interventions, and decides whether the plan of care should be discontinued, continued, or revised.

DIF: Remembering

OBJ: 5.4

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

MULTIPLE RESPONSE

1. The nurse knows which statements would be considered objective data? (*Select all that apply.*)

- a. —I'm short of breath.¶
- b. —Blood pressure 90/68, apical pulse 102, skin pale and moist.¶
- c. —Lung sounds clear bilaterally, diminished in right lower lobe.¶
- d. —I feel weak all over when I exert myself.¶
- e. —My pain level is down to 2. It was 8.¶

ANS: B, C

Data collected from medical records, laboratory, and diagnostic test results, or physical assessments are objective. Objective data (i.e., signs) consist of observable information that the nurse gathers on the basis of what can be seen, measured, or tested. Subjective data (i.e., symptoms) are spoken. Patients' feelings about a situation or comments about how they are feeling are examples of subjective data. Data shared by a source verbally are considered subjective. Subjective data may be difficult to validate because they cannot be independently and objectively measured.

DIF: Understanding

OBJ: 5.4

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

2. The nurse is attempting to develop Nursing diagnoses for a patient. The nurse understands that Nursing diagnoses have which characteristics? (*Select all that apply.*)
- Nursing diagnoses identify actual or potential problems as well as responses to a problem.
 - Nursing diagnoses require naming patient problems using Nursing diagnostic labels.
 - Nursing diagnoses utilize objective data since subjective data are often inaccurate.
 - Nursing diagnoses include unvalidated data to determine an accurate and thorough diagnosis.
 - Nursing diagnoses are similar to medical diagnoses since they both are labels for diseases.

ANS: A, B

The Nursing diagnosis identifies an actual or potential problem or response to a problem. Accurate identification of Nursing diagnoses for patients results from carefully analyzing, validating, and

clustering related patient subjective (symptoms) and objective (signs) data. If data collection includes inaccurate or inadequate information or if data are not validated or clustered with related information, a patient may be misdiagnosed. Diagnosis in the nursing process requires naming patient problems using Nursing diagnostic labels. Medical diagnoses are labels for diseases, whereas Nursing diagnoses describe a response to an actual or potential problem or life process.

DIF: Understanding

OBJ: 5.4

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

3. The nurse recognizes that establishing short- and long-term goals to address Nursing diagnoses involve which actions? (*Select all that apply.*)
- Discussion with the patient
 - Exclusion of family with making patient decisions
 - Collaboration with other members of health care team
 - Making the health care provider as the central figure
 - Coordination of care as collaborative care

ANS: A, C, E

Establishing short- and long-term goals to address Nursing diagnoses involves discussion with the patient and often requires collaboration with family members and other members of the health care team. Coordinated, team-based patient care is called *collaborative care*. The patient's health care team members may include several nurses: the primary care provider; medical or surgical specialists; respiratory therapists; a dietitian; a physical therapist; occupational, music, or art therapists; a spiritual adviser; and social workers. The patient's primary nurse is often the central figure in coordinating collaborative care.

DIF: Understanding

OBJ: 5.4

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

NURSINGTB.COM

Chapter 06: Assessment

Yoost & Crawford: Fundamentals of Nursing: Active Learning for Collaborative Practice, 3rd Edition

MULTIPLE CHOICE

1. The nurse is caring for a patient with pneumonia, who is a retired soldier who served in World War II. With this information in mind, what should the nurse do in regarding this patient?
 - a. Shake the patient's hand and allow the patient time to —warm up.¶
 - b. Expect the patient to be optimistic and question everything.
 - c. Allow the patient to multitask and talk in short —sound bites.¶
 - d. Understand that the patient is probably technologically literate.

ANS: A

Establishing rapport is paramount to gaining the trust of the patient. The nurse should consider the patient's generational cohort, which may influence behavior, and willingness to share personal information during the interview process. Veterans (born before 1945) respect authority; are detail oriented; communicate in a discrete, formal, respectful way; may be slow to warm up; value family and community; and accept physical touch as an effective form of therapeutic communication. Baby Boomers (born 1946 to 1964) are optimistic, relationship oriented, and communicate by using open or direct speech, using body language, and answering questions thoroughly. They expect detailed information, question everything, and value success. Generation X members (born 1965 to 1976) are informal; are technology immigrants; multitask; communicate in a blunt or direct, factual, and informal style; may talk in short sound bites; share information frequently; and value time. Millennials, also called

Generation Y (born 1977 to 1994) are flexible; are technologically literate or are technology natives; multitask; communicate by using action verbs and humor; may be brief in the form of texting or e-mail exchanges; like personal attention; and value individuality. Individuals from Generation Z (born 1995 to 2012) are digitally connected, value group work, want immediate feedback, are accepting of others, value honesty and family, and are entrepreneurial.

DIF: Applying OBJ: 6.1

TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Communication

2. The patient interview consists of three phases: orientation (introductory), working, and termination. Each phase contributes to the development of trust and engagement between the nurse and the patient. During the orientation phase of a patient interview, the nurse carries out what action?
 - a. Obtain demographic data using open-ended questions.
 - b. Establish the name by which the patient prefers to be addressed.
 - c. Gather general information using closed-ended questions.
 - d. Stand by the bedside to ask the needed questions.

ANS: B

The patient interview consists of three phases: orientation (introductory), working, and termination. Each phase contributes to the development of trust and engagement between the nurse and the patient. During the orientation phase of the interview, the nurse should establish the name by which the patient prefers to be addressed. Some individuals prefer formal titles of respect (e.g., Dr., Mr., Ms., Professor) and the use of surnames, whereas others are comfortable with less formality. How a patient is addressed is the patient's choice.

Demographic data should be collected by asking focused or closed-ended questions. More general information can be gathered by open-ended communication techniques. When feasible, the nurse and the patient should be seated at eye level with each other. In this way, the interaction between the nurse and the patient is horizontal instead of vertical. Standing over someone implies control, power, and authority. The implication of power can result in less-than-optimal data collection and a potential conflict as the patient strives to regain control over the situation.

DIF: Applying OBJ: 6.1 TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT: Concepts: Communication

3. A nurse is conducting a health interview on a newly admitted patient. To establish a trusting relationship with the patient, the nurse carries out which action?
- Avoid eye contact to appear less threatening.
 - Demonstrate professionalism by not smiling.
 - Sit close and leans in slightly toward the patient.
 - Speaks in a slow rate of speech and low tone.

ANS: C

Nonverbal behaviors of the nurse can influence the information obtained from the patient.

Negative nonverbal cues such as distracting gestures (e.g., tapping a pen, swinging a foot, looking at a watch), inappropriate facial expressions, and lack of eye contact communicate disinterest. To establish a trusting relationship with the patient before the physical examination is conducted, the nurse should communicate professionally, sit close and lean in slightly toward the patient, listen attentively and demonstrate appropriate eye contact, smile, and use a moderate rate of speech and tone of voice.

DIF: Applying OBJ: 6.1 TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT: Concepts: Communication

4. The nurse is assigned the admission health history and physical for a patient diagnosed with a fever of unknown etiology. The patient tells the nurse, —I just don't feel good. I'm so hot and I feel sick to my stomach. Can you ask me those questions later? What would be the best response by the nurse?
- It will not take too long. I can hurry.
 - We need the information to complete your admission paperwork.
 - I will come back in a few minutes and we can start over.
 - Let me see if you can have something for the nausea and then talk later.

ANS: D

If a patient being admitted to the hospital is too ill to interact for an extended period, the interview can be broken into smaller segments. Interviews with patients already hospitalized or established in the health care system are less extensive and more focused on newly identified patient concerns or problems. Ensuring that the patient is comfortable and relaxed is a priority and often takes prior thought and planning by the nurse.

DIF: Applying OBJ: 6.1 TOP: Implementation
MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
Concepts: Communication

5. The nurse is using a stethoscope to assess a patient's cardiac status. Which assessment technique is the nurse using?
- Inspection
 - Percussion
 - Palpation
 - Auscultation

ANS: D

Auscultation is a technique of listening with the assistance of a stethoscope to sounds made by organs or systems such as the heart, blood vessels, lungs, and abdominal cavity. Inspection involves the use of vision, hearing, and smell to closely scrutinize physical characteristics of a whole person and individual body systems. Percussion involves tapping the patient's skin with short, sharp strokes that cause a vibration to travel through the skin and to the upper layers of the underlying structures. Vibration is reflected by the tissues, and the character of the sound heard depends on the density of the structures that reflect the sound. Palpation uses touch to assess body organs and skin texture, temperature, moisture, turgor, tenderness, and thickness.

NURSINGTB.COM

DIF: Remembering OBJ: 6.2 TOP: Assessment
MSC: NCLEX Client Needs Category: Physiological Integrity: Basic Care and Comfort NOT:
Concepts: Caregiving

6. The nurse is performing an assessment of a patient's right kidney. The nurse bluntly strikes the area of the costovertebral angle while observing the patient's reaction. Which assessment technique is the nurse using?
- Inspection
 - Percussion
 - Palpation
 - Auscultation

ANS: B

Percussion involves tapping the patient's skin with short, sharp strokes that cause a vibration to travel through the skin and to the upper layers of the underlying structures. Inspection involves the use of vision, hearing, and smell to closely scrutinize physical characteristics of a whole person and individual body systems. Palpation uses touch to assess body organs and skin texture, temperature, moisture, turgor, tenderness, and thickness. Auscultation is a technique of listening with the assistance of a stethoscope to sounds made by organs or systems such as the heart, blood vessels, lungs, and abdominal cavity. Vibration is reflected by the tissues, and the character of the sound heard depends on the density of the structures that reflect the sound.

DIF: Remembering OBJ: 6.2 TOP: Assessment

MSC: NCLEX Client Needs Category: Physiological Integrity: Basic Care and Comfort NOT:
Concepts: Caregiving

7. The nurse is performing a physical exam on a patient diagnosed with liver failure resulting from chronic alcoholism. The nurse notes that the abdomen is swollen and decides to assess for abdominal skin tenderness and temperature. Which technique would the nurse use to collect this data?
- Inspection
 - Percussion
 - Palpation
 - Auscultation

ANS: C

Palpation uses touch to assess body organs and skin texture, temperature, moisture, turgor, tenderness, and thickness. Inspection involves the use of vision, hearing, and smell to closely scrutinize physical characteristics of a whole person and individual body systems. Percussion involves tapping the patient's skin with short, sharp strokes that cause a vibration to travel through the skin and to the upper layers of the underlying structures. Auscultation is a technique of listening with the assistance of a stethoscope to sounds made by organs or systems such as the heart, blood vessels, lungs, and abdominal cavity. Vibration is reflected by the tissues, and the character of the sound heard depends on the density of the structures that reflect the sound.

DIF: Remembering OBJ: 6.2 TOP: Assessment
MSC: NCLEX Client Needs Category: Physiological Integrity: Basic Care and Comfort NOT:
Concepts: Caregiving

8. The triage nurse in a hospital emergency department is determining the order of care for several patients. Which patient would the nurse consider as having the highest priority?
- A 68-year-old patient suffering from dehydration and disorientation
 - A 14-year-old patient having respiratory distress and increasing anxiety
 - A 46-year-old patient with multiple cuts and abrasions to the upper extremities
 - A 38-year-old patient with a broken right hip and in severe pain

ANS: B

Triage, a form of emergency assessment, is the classification of patients according to treatment priority. Patients are categorized by the urgency of their condition. Most emergency departments use a five-tier triage system. The five-tier system classifies patients by levels numbered 1 through 5. Level 1 is considered critical: life-threatening conditions require immediate and continuous care such as severe trauma, cardiac arrest, respiratory distress, seizure, or shock. Level 2 emergencies can be imminently life-threatening conditions requiring care within 30 minutes, such as chest pain or major fractures, with severe pain.

Level 3 is considered urgent: potentially life-threatening conditions that require care within 30 to 60 minutes, such as minor fractures, lacerations, and dehydration. Level 4 is considered semi-urgent, stable health conditions that require care within 60 to 120 minutes, such as a twisted ankle. Level 5 conditions are non-urgent and lower risk such as cold symptoms.

DIF: Analyzing OBJ: 6.3 TOP: Assessment
MSC: NCLEX Client Needs Category: Physiological Integrity: Basic Care and Comfort NOT:
Concepts: Caregiving

9. The morning nurse is assigned to care for a patient admitted during the night with rectal bleeding. When making rounds, the nurse observes that the patient's face is ashen in color and the skin is cool and clammy. The nurse auscultates the patient's heart and lungs. Which category of physical assessment is the basis for the nurse's response?
- Emergency assessment
 - Focused assessment
 - Complete assessment
 - Initial comprehensive

ANS: A

Emergency assessment is a physical examination done when time is a factor, treatment must begin immediately, or priorities for care need to be established in a few seconds or minutes. Attention is paid to the patient's airway, breathing, and circulation. Other concerns in the emergent setting are noticeable deformities such as compound fractures, contusions, abrasions, puncture wounds, burns, tenderness, lacerations, bleeding, and swelling. During an emergency, the nurse may never have time to do a complete assessment and may work to stabilize one body system at a time. A focused or clinical assessment is a brief individualized physical examination conducted at the beginning of an acute care setting work shift to establish current patient status or during ongoing patient encounters in response to a specific patient concern. A focused assessment may be conducted when signs indicate a change in a patient's condition or the development of a new complication. A comprehensive or complete assessment includes a thorough interview, health history, review of systems, and extensive physical head-to-toe assessment, including evaluation of cranial nerves and sensory organs, such as with sight and hearing testing. A complete physical examination may be conducted on admission to a hospital, during an annual physical at the office of a physician or nurse practitioner, or on initial interaction with a specialist.

NURsINGTB.COM

DIF: Remembering

OBJ: 6.3

TOP: Assessment

MSC: NCLEX Client Needs Category: Physiological Integrity: Basic Care and Comfort NOT:

Concepts: Caregiving

10. The nurse is performing her initial assessment of the day when she notices that the patient has a facial droop that was not present yesterday and that was not reported in the hand-off report from the night nurse. The nurse proceeds to assess the neurologic status of the patient and knows this to be which type of assessment?
- Emergency assessment
 - Focused assessment
 - Complete physical examination
 - Comprehensive assessment

ANS: B

A focused or clinical assessment is a brief individualized physical examination conducted at the beginning of an acute care setting work shift to establish current patient status or during ongoing patient encounters in response to a specific patient concern. A focused assessment may be conducted when signs indicate a change in a patient's condition or the development of a new complication. Emergency assessment is a physical examination done when time is a factor, treatment must begin immediately, or priorities for care need to be established in a few seconds or minutes. Attention is paid to the patient's airway, breathing, and circulation. Other concerns in the emergent setting are noticeable deformities such as compound fractures, contusions, abrasions, puncture wounds, burns, tenderness, lacerations, bleeding, and swelling. A comprehensive or complete assessment includes a thorough interview, health history, review of systems, and extensive physical head-to-toe assessment, including evaluation of cranial nerves and sensory organs, such as with sight and hearing testing. A complete physical examination may be conducted on admission to a hospital, during an annual physical at the office of a physician or nurse practitioner, or on initial interaction with a specialist.

DIF: Remembering

OBJ: 6.3

TOP: Assessment

MSC: NCLEX Client Needs Category: Physiological Integrity: Basic Care and Comfort NOT:

Concepts: Caregiving

11. The nurse is documenting data collected during a health assessment interview. Which statement by the nurse indicates subjective data?
- a. —My last bowel movement was 4 days ago.¶
 - b. Abdomen distended; firm and tender.
 - c. Dark colored; hard pellet-shaped stool.
 - d. Color pink. Skin warm and dry. No sign of discomfort.

ANS: A

NURSINGTB.COM

Subjective data are spoken information or symptoms that cannot be authenticated. Subjective data usually are gathered during the interview process if patients are well enough to describe their symptoms. Objective data, also referred to as signs, can be measured or observed. The nurse's senses of sight, hearing, touch, and smell are used to collect objective data. Objective assessment data are acquired through observation, physical examination, and analysis of laboratory and diagnostic test results.

DIF: Analyzing

OBJ: 6.4

TOP: Evaluation

MSC: NCLEX Client Needs Category: Physiological Integrity: Basic Care and Comfort NOT:

Concepts: Caregiving

12. A patient is transported to the emergency department from a local skilled nursing facility and admitted for a bacterial blood infection. The nurse reviews the transferring physician notes, which indicate that the patient has dementia. The nurse contacts the patient's son for additional health history information. Information provided by the son would be considered which type of data?
- a. Primary, objective data
 - b. Primary, subjective data
 - c. Secondary, objective data
 - d. Secondary, subjective data

ANS: D

Subjective data are spoken information or symptoms that cannot be authenticated. Subjective data usually are gathered during the interview process if patients are well enough to describe their symptoms. Family members, friends, and other members of the health care team can contribute valid secondary, subjective data. Objective data, also referred to as signs, can be measured or observed. The nurse's senses of sight, hearing, touch, and smell are used to collect objective data. Objective assessment data are acquired through observation, physical examination, and analysis of laboratory and diagnostic test results. Primary data come directly from the patient.

DIF: Remembering

OBJ: 6.4

TOP: Assessment

MSC: NCLEX Client Needs Category: Physiological Integrity: Basic Care and Comfort NOT:

Concepts: Caregiving

13. The nurse is monitoring the blood sugar results of a patient receiving an intravenous nutritional supplement. The patient tells the nurse, —I have never had sugar problems before. My doctor says it is because I am getting this IV.‖ These types of data are considered to be which type?
- Primary, objective data
 - Primary, subjective data
 - Secondary, objective data
 - Secondary, subjective data

ANS: B

Primary data come directly from the patient. Subjective data are spoken information or symptoms that cannot be authenticated. Subjective data usually are gathered during the interview process if patients are well enough to describe their symptoms. Family members, friends, and other members of the health care team can contribute valid secondary, subjective data. Objective data, also referred to as signs, can be measured or observed. The nurse's senses of sight, hearing, touch, and smell are used to collect objective data. Objective assessment data are acquired through observation, physical examination, and analysis of laboratory and diagnostic test results.

DIF: Remembering

OBJ: 6.4

TOP: Assessment

MSC: NCLEX Client Needs Category: Physiological Integrity: Basic Care and Comfort NOT:

Concepts: Caregiving

14. The unlicensed nursing assistive person (UAP) reports to the nurse that a patient is crying during a comedy show on television. What would be the best response by the nurse?
- Maybe the patient doesn't think the show is funny.‖
 - Don't worry about it. The patient's daughter says this is normal.‖
 - I will go visit her right away and see what is going on.‖
 - Just document what you observe in your notes.‖

ANS: C

Validating data is making sure that the data are accurate. As patient information is collected, consistency between subjective and objective data must be confirmed. Confirming the validity of collected data often requires verbally checking with the patient to see whether assumptions or conclusions at which the nurse arrived are correct. Crying, a disheveled appearance, and lack of eye contact may be cues of depression. However, conclusions about the underlying cause of the patient's actions cannot be assumed. All cues need to be interpreted and validated to verify the data's accuracy. The nurse cannot assume that this is normal behavior nor ignore the problem by making a joke. The nurse has the responsibility to attempt to determine the real reason for the crying episode.

DIF: Applying OBJ: 6.5 TOP: Implementation
MSC: NCLEX Client Needs Category: Physiological Integrity: Basic Care and Comfort NOT:
Concepts: Caregiving

15. A patient with moderate lower back pain tells the nurse, —My urine smells awful and is as dark as my glass of tea. Which action by the nurse will assist in validating the patient's concern?
- Ask the patient to describe the back pain.
 - Review the lab results of the most recent urinalysis.
 - Request the nursing assistant to obtain a set of vital signs.
 - Check the patient's history for urinary tract infections.

ANS: B

As patient information is collected, consistency between subjective and objective data must be confirmed. Sometimes, the nurse can use laboratory and diagnostic test results to validate the subjective data. In this case, checking the urinalysis for congruency with the patient's subjective data will validate the patient's statements. Obtaining a set of vital signs, reviewing the patient's history, and exploring the patient's pain are appropriate actions but cannot validate the current problem. **USNT R I G B C M O**

DIF: Applying OBJ: 6.5 TOP: Implementation
MSC: NCLEX Client Needs Category: Physiological Integrity: Basic Care and Comfort NOT:
Concepts: Caregiving

16. The nurse is attempting to get the patient to sign the operative consent. When asked if the health care provider explained the procedure to the patient, the patient replies —Not much. What action will the nurse take next?
- Develop a comprehensive teaching plan related to the surgical procedure.
 - Ask the patient what information the surgeon has explained about the surgery.
 - Contact the surgeon to clarify information given to the patient.
 - Focus on post-operative exercises and home-care following surgery.

ANS: B

Careful observation and attention to detail help the nurse to notice subtle cues and recognize how best to validate and interpret patient data. The nurse must be careful not to make false assumptions or generalizations regarding the patient's responses to the health concern. The nurse is correct to ask the patient about the upcoming surgical procedure instead of assuming that the patient has limited knowledge. This is the nurse's best action to determine what the surgeon said to the patient. Developing a comprehensive teaching plan is not necessary until further clarification is obtained. Focusing on postoperative treatment plans is important but not the priority at this time. It is not appropriate to contact the surgeon unless the patient demonstrates an actual knowledge deficit.

DIF: Applying OBJ: 6.5 TOP: Implementation
MSC: NCLEX Client Needs Category: Physiological Integrity: Basic Care and Comfort NOT:
Concepts: Caregiving

17. After the patient's data are collected, validated, and interpreted, the nurse organizes the information in a framework (format) that facilitates access by all members of the health care team. What is the framework that provides the most holistic view of the patient's condition?
- Head-to-toe pattern
 - Functional Health Patterns
 - Cephalic-caudal pattern
 - Body systems model

ANS: B

Marjory Gordon developed the Functional Health Patterns to help nurses focus on patient strengths and related but sometimes overlooked data relationships. This method of organizing patient data is a more holistic approach than the others because it includes data such as values, beliefs, and roles in addition to physical data. Organizing assessment data in a head-to-toe (cephalic-caudal) pattern ensures that all areas of concern are addressed as the nurse performs an assessment covering the entire body. The body systems model organizes data on the basis of each system of the body: integumentary, respiratory, cardiovascular, nervous, reproductive, musculoskeletal, gastrointestinal, genitourinary, and immune systems. It follows a sequence similar to the medical model for physical examination. The body systems model for data organization tends to focus on the physical aspects of a patient's condition rather than a more holistic view.

DIF: Remembering OBJ: 6.6 TOP: Assessment
MSC: NCLEX Client Needs Category: Physiological Integrity: Basic Care and Comfort NOT:
Concepts: Caregiving

18. The wound care nurse is assessing a non-healing leg wound on a patient recently admitted for uncontrolled diabetes. The nurse organizes the data using which Gordon's Functional Health Pattern?
- Nutrition and metabolism
 - Activity and exercise
 - Sleep and rest
 - Elimination

ANS: A

Health assessment data is organized in frameworks that provide a comprehensive view of a patient's health. Gordon's Functional Health Pattern focuses on patient's strengths and relationships of the data collected. The focus of nutrition and metabolism is tissue integrity. Data collected during a wound assessment would be classified in this health pattern. Activities of daily living and musculoskeletal information are the focus of the activity and exercise pattern. Sleep and rest includes sleep patterns, relaxation activities, and levels of fatigue. Bowel and urinary concerns are the focus of the elimination pattern.

DIF: Remembering OBJ: 6.6 TOP: Assessment
MSC: NCLEX Client Needs Category: Physiological Integrity: Basic Care and Comfort NOT:
Concepts: Caregiving

19. During the health history interview, the patient tells the nurse, —Just walking to the mailbox and back makes my calves ache. Is this normal? Which framework would the nurse most likely choose to document this data?
- Head-to-toe model
 - Gordon's Functional Health Patterns
 - Body systems model
 - Cephalic-caudal model

ANS: C

The body systems model organizes data on the basis of each system of the body. As this patient report is confined to the patient's leg pain, the nurse would document the data according to this model. Organizing assessment data in a head-to-toe (cephalic-caudal) pattern ensures that all areas of the body are assessed, including vital signs and other data not pertinent to this report by the patient. Gordon's Health Patterns allow the nurse to organize data in a holistic manner and reveals relationships between data. The cephalic-caudal model allows for a head to toe assessment.

DIF: Remembering

OBJ: 6.6

TOP: Assessment

MSC: NCLEX Client Needs Category: Physiological Integrity: Basic Care and Comfort NOT:

Concepts: Caregiving

MULTIPLE RESPONSE

1. The nurse knows what should be included in an in-depth health history? (*Select all that apply.*)
- Demographic data
 - Patient's allergies
 - Family history of diseases
 - Patient's health promotion practices
 - Patient's history of illness and surgery

ANS: A, B, C, D, E

An in-depth health history includes all pertinent information that can guide the development of a patient-centered plan of care. The health history includes demographic data, which are collected during the orientation phase of the interview; a patient's chief complaint or reason for seeking health care; history of current and past illnesses and surgery; allergies; medications; adverse reactions to medications; medical history; family and social history; and health promotion practices. Because a patient's health history is continuously evolving, the data collection is ongoing, progressive, and methodical.

DIF: Remembering

OBJ: 6.1

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Communication

2. The nurse is admitting a patient for uncontrolled diabetes mellitus. The nurse suspects that the patient could benefit from diabetic teaching. What actions by the nurse will assist in validating this suspicion? (*Select all that apply.*)
- Determine the patient's cognitive ability and potential language barriers.
 - Gather information about what the patient already knows about diabetes.
 - Have the patient demonstrate checking a blood glucose level.

- d. Formulate the patient's plan of care using a standard protocol.
- e. Prepare to teach the patient using materials written at a third-grade level.

ANS: B, C

Data that would validate the nurse's suspicion that the patient needs further education include determining what the patient already knows about diabetes and having the patient demonstrate the technique of blood glucose monitoring. If the nurse is correct, further education is needed. Before further education can occur however; the nurse should determine if the patient has cognitive difficulties or a language barrier which would all contribute to an individualized plan of care. Reading material should typically be written at a fifth-grade level, but the nurse should not assume the patient needs third-grade level material.

DIF: Remembering

OBJ: 6.1

TOP: Assessment

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance

NOT: Concepts: Health Promotion

3. The nurse is preparing to begin a physical examination for a patient with open lesions on the lower extremities. Which would the nurse evaluate during the physical assessment? (*Select all that apply.*)
- a. Blood test results
 - b. X-ray results
 - c. Recent vital signs
 - d. Patient's health history
 - e. Subjective data

ANS: A, B, C

On completion of the patient interview, health history, and review of systems, the nurse begins the physical assessment. During the physical assessment, the nurse collects objective data. If diagnostic tests, such as blood tests or x-rays, were ordered before the patient was seen, the results are reviewed by the nurse. Vital signs are taken and recorded at the beginning of the physical examination.

DIF: Applying

OBJ: 6.2

TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

4. The charge nurse is planning vital sign assignments for the unlicensed assistive personnel (UAP) on a busy medical-surgical unit. Which patients are appropriate for the UAP to obtain vital signs? (*Select all that apply.*)
- a. A 28-year old patient scheduled to be discharged home today
 - b. A 49-year-old patient with stable chronic lung disease
 - c. A 78-year-old patient with recent onset of rectal bleeding
 - d. A 35-year-old patient waiting for transfer to a rehabilitation center
 - e. A 40-year-old patient being admitted from the emergency department

ANS: A, B, D

Routine assessment of vital signs of a patient who is stable may be delegated to licensed practical or licensed vocational nurses (LPNs/LVNs) or qualified UAP. Initial and ongoing assessment of patients requiring critical care or who are unstable cannot be delegated to UAPs. The patient with rectal bleeding may need critical care, and a new admission needs to be assessed by an RN. Stable patients such as the patient with stable lung disease or awaiting discharge or transfer can be delegated to UAP.

DIF: Analyzing OBJ: 6.3 TOP: Analysis
MSC: NCLEX Client Needs Category: Physiological Integrity: Basic Care and Comfort NOT:
Concepts: Caregiving

5. The nurse identifies which examples listed indicate objective data? (*Select all that apply.*)
- a. Respirations—24 breaths/min
 - b. Platelet count—350,000 mm³
 - c. Wound size—3 cm 2 cm
 - d. Temperature—98.4 °F (36.8 °C)
 - e. Reports severe abdominal pain

ANS: A, B, C, D

Objective data, also referred to as *signs*, can be measured or observed. The nurse's senses of sight, hearing, touch, and smell are used to collect objective data. Objective assessment data are acquired through observation, physical examination, and analysis of laboratory and diagnostic test results. Subjective data are spoken information or symptoms that cannot be authenticated. Subjective data usually are gathered during the interview process if patients are well enough to describe their symptoms.

DIF: Remembering OBJ: 6.4 TOP: Assessment
MSC: NCLEX Client Needs Category: Physiological Integrity: Basic Care and Comfort NOT:
Concepts: Caregiving

6. Patient-centered care requires the nurse to complete which actions? (*Select all that apply.*)
- a. Have an understanding of patient preferences.
 - b. Be aware of family values.
 - c. Recognize the patient's expectations.
 - d. Base conclusions on the nurse's personal experiences.
 - e. Provide care in a standardized manner.

ANS: A, B, C

Patient-centered care requires the nurse to understand patient and family preferences and values. Nurses must recognize patients' expectations for care and provide care with respect for the diversity of human experience. While interpreting data, the nurse must be careful to avoid inaccurate inferences (i.e., conclusions) based on the nurse's personal preferences, past experiences, generalizations, or outdated and inaccurate health care information.

DIF: Applying OBJ: 6.6 TOP: Implementation
MSC: NCLEX Client Needs Category: Physiological Integrity: Basic Care and Comfort NOT:
Concepts: Caregiving

Chapter 07: Nursing Diagnosis

Yoost & Crawford: Fundamentals of Nursing: Active Learning for Collaborative Practice, 3rd Edition

MULTIPLE CHOICE

1. The nurse completes a health and physical assessment on a patient admitted with a fractured pelvis. Which task would the nurse do next?
 - a. Analyze and cluster the assessment information.
 - b. Formulate a Nursing diagnosis addressing actual issues.
 - c. Determine the need for potential Nursing diagnoses.
 - d. Create health promotion diagnoses for the patient.

ANS: A

Nursing diagnosis is the second step of the nursing process. Formulation of nursing diagnoses follows patient data collection and involves the analysis and clustering of related assessment information. Actual nursing diagnoses identify existing problems or concerns of a patient.

Risk nursing diagnoses apply when there is an increased potential or vulnerability for a patient to develop a problem or complication. Health-promotion nursing diagnoses are used in situations in which patients express interest in improving their health status through a positive change in behavior. The analysis of information is required to determine nursing diagnoses.

DIF: Applying OBJ: 7.1 TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT: Concepts: Care Coordination

2. A group of patients in a community center attend a nursing-led information session on the risks of contracting tuberculosis. After the presentation, several patients ask the nurse for additional web-based resources regarding the lung disease. Which type of nursing diagnosis would the nurse choose for the community care plan?
 - a. Risk
 - b. Actual
 - c. Health-promotion
 - d. Potential

ANS: C

Health-promotion nursing diagnoses are used in situations in which patients express interest in improving their health status through a positive change in behavior. Although most nursing diagnoses are used for individual patients, nursing diagnosis taxonomy can be applied to families, groups of individuals, and communities. Actual nursing diagnoses identify existing problems or concerns of a patient. Risk (potential) nursing diagnoses apply when there is an increased potential or vulnerability for a patient to develop a problem or complication.

DIF: Applying OBJ: 7.1 TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT: Concepts: Care Coordination

3. A nurse completes a care plan for an assigned patient diagnosed with an inflammation of the pericardium. Which diagnosis written on the plan indicates a need for further instruction on using the nursing process?

- a. Pericarditis
- b. Acute pain
- c. Anxiety
- d. Activity intolerance

ANS: A

Whereas medical diagnoses identify and label medical (physical and psychological) illnesses, nursing diagnoses are much broader in and consider a patient's response to medical diagnoses and life situations. The underlying etiology, or cause of a patient's concern or situation, rather than a medical diagnosis, should be used as a related factor when writing an ICNP nursing diagnosis statement and can be included in the list of supporting data in the EMR. Pericarditis is a medical diagnosis defined as an inflammation of the pericardium. Pain, anxiety and intolerance to activity are all possible patient responses to the medical condition of pericarditis.

DIF: Analyzing OBJ: 7.1

TOP: Evaluation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

4. A patient is receiving an experimental drug for leukemia. The nurse is worried that the drug may cause a reduction in platelets leading to intestinal tract bleeding. Which type of nursing diagnosis does the nurse use to address this concern?
 - a. Risk
 - b. Actual
 - c. Health-promotion
 - d. Medical diagnosis

ANS: A

NURSINGTB.COM

The three types of nursing diagnostic statements are actual, risk, and health promotion. Determining which type is needed for each patient can be challenging. Risk (potential) Nursing diagnoses apply when there is an increased potential or vulnerability for a patient to develop a problem or complication. Actual Nursing diagnoses identify existing problems or concerns of a patient. Health-promotion Nursing diagnoses are used in situations in which patients express interest in improving their health status through a positive change in behavior. Although most nursing diagnoses are used for individual patients, nursing diagnosis taxonomy can be applied to families, groups of individuals, and communities. Medical diagnoses identify and label medical (physical and psychological) illnesses.

DIF: Remembering

OBJ: 7.3

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

5. The nurse is writing the care plan for a patient admitted to the hospital for complications associated with muscular dystrophy. Which Nursing diagnoses written on the care plan indicate a need for further instruction in constructing the diagnostic statement?
 - a. Constipation related to immobility as manifested patient passing hard, dry stool with difficulty
 - b. Activity intolerance related to weakness as evidenced by verbal report of fatigue.
 - c. Impaired self feeding related to fatigue as manifested by inability to open containers and bring food to the mouth.
 - d. Impaired airway clearance related to muscle weakness.

ANS: D

Each type of Nursing diagnostic statement contains sections or parts. Actual Nursing diagnostic statements are written with three parts: a diagnosis label, related factors, and defining characteristics. Risk Nursing diagnoses have two segments: a diagnosis label and risk factors. Health-promotion Nursing diagnoses are written with only two sections: the diagnosis label and defining characteristics. The impaired airway clearance label is missing the defining characteristics.

DIF: Remembering

OBJ: 7.4

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

6. Nursing students are analyzing the following Nursing diagnostic statement during a study group session. *Acute pain related to pressure on lumbar spinal nerves as evidenced by a pain level of 9, patient verbalizations of pain, and grimacing when walking.* The students would be correct if they stated which response to be the etiology of the patient's problem?
- Patient verbalizations of pain
 - Acute pain
 - Pressure on lumbar spinal nerves
 - Grimacing when walking

ANS: C

The second part of the Nursing diagnosis consists of related factors (for actual Nursing diagnoses) and risk factors (for risk Nursing diagnoses). Related factors are the underlying cause or etiology of a patient's problem. Risk factors are environmental, physical, psychological, or situational concerns that increase a patient's vulnerability to a potential

problem or concern. In this case, the acute pain is being caused by pressure on the lumbar spinal nerves.

DIF: Understanding

OBJ: 7.4

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

7. The nurse is caring for a patient diagnosed with blood clots in the right lower extremity. The admitting provider orders bed rest. The patient tells the nurse, —I usually exercise three times a week. It helps me go to the bathroom.‖ The nurse determines that the patient may have difficulty with bowel movements. Which Nursing diagnosis statement accurately reflects the nurse's concern?
- Constipation related to bed rest as manifested by hard, dry stools.
 - Constipation resulting from reduced peripheral circulation manifested by patient's anxiety.
 - Risk for constipation related to immobility as manifested by verbal complaint.
 - Risk for constipation related to insufficient physical activity.

ANS: D

NURSINGTB.COM

Each type of Nursing diagnostic statement contains sections or parts. Actual Nursing diagnostic statements are written with three parts: a diagnosis label, related factors, and defining characteristics. Risk Nursing diagnoses have two segments: a diagnosis label and risk factors. Health-promotion Nursing diagnoses are written with only two sections: the diagnosis label and defining characteristics. There are no data suggesting the patient is constipated at this time.

DIF: Remembering

OBJ: 7.4

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

8. The nursing student is reviewing the components of a Nursing diagnosis. Which statement made by the student indicates correct understanding of a health-promotion diagnostic statement?
- The defining characteristics will include the patient's willingness to get better.||
 - The risk factors are only psychological in nature, not physical.||
 - The health-promotion diagnostic statement is composed of three parts.||
 - An example of a health-promotion label is ineffective community coping.||

ANS: A

The three types of Nursing diagnostic statements are actual, risk, and health promotion. Determining which type is needed for each patient can be challenging. Health-promotion Nursing diagnoses are used in situations in which patients express interest in improving their health status through a positive change in behavior. The second part of the Nursing diagnosis consists of related factors (for actual Nursing diagnoses) and risk factors (for risk Nursing diagnoses). Related factors are the underlying cause or etiology of a patient's problem. Risk factors are environmental, physical, psychological, or situational. Health-promotion Nursing diagnoses are written with only two sections: the diagnosis label and defining characteristics. Actual diagnoses describe the person, family, or community's response to a health condition or life process that already has occurred. —Ineffective community coping|| would be an example of an actual problem.

DIF: Understanding

OBJ: 7.4

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

9. The nurse is reviewing assessment findings on a patient admitted with an extremely slow heart rate in preparation to write a care plan. The patient complains of dizziness, shortness of breath, chest pain, and fainting spells. Vital signs are blood pressure of 98/60 mm Hg and pulse of 52 beats/min. Oxygen saturation is 88%. Which action does the nurse perform next?
- Exclude all subjective data in favor of objective data.
 - Focus on data gathered during the physical assessment.
 - Evaluate the data looking for patterns and related data.
 - Dismiss family members input as —hearsay.||

ANS: C

After collecting and reviewing all of the assessment data, the nurse looks for patterns and related data to support specific Nursing diagnoses. This process is referred to as *clustering data*. Clustering involves organizing patient assessment data into groupings with similar underlying causes. All patient information should be considered as potentially contributing to the identification of diagnostic labels. This information includes subjective and objective data collected through physical assessment of the patient, interview of the patient and family members, and laboratory and diagnostic test results, including x-rays, physicians' orders, and documentation from health care providers. Verifying specific Nursing diagnoses for a patient or situation follows accurate analysis and clustering of data.

DIF: Applying OBJ: 7.5 TOP: Implementation
 MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
 Concepts: Care Coordination

10. The nurse is admitting a patient with severe dehydration. Assessment data reveal a decreased blood pressure, an increased pulse rate, and a low circulating blood volume. The student observes that the patient is confused and restless. Which patient information would the nurse consider as a contributing factor when choosing the Nursing diagnostic label?
- Blood pressure, pulse rate
 - Blood pressure, pulse rate, blood volume
 - Blood pressure, pulse rate, blood volume, mental status
 - Blood pressure, pulse rate, blood volume, mental status, dehydration

ANS: D

All patient information should be considered as potentially contributing to the identification of diagnostic labels. This information includes subjective and objective data collected through physical assessment of the patient, interview of the patient and family members, and laboratory and diagnostic test results, including x-rays, physicians' orders, and documentation from health care providers. Verifying specific Nursing diagnoses for a particular patient or situation follows accurate analysis and clustering of data.

DIF: Applying OBJ: 7.5 TOP: Implementation
 MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
 Concepts: Care Coordination

11. The nurse is reviewing data obtained through the health history interview and physical assessment of an assigned patient. Data collected include dry skin, brittle nails, weight gain, thinning hair, constipation, prolonged menstruation, and the patient's complaints of feeling tired and cold. The nurse recognizes which statement represents an appropriate data cluster?
- Prolonged menstruation, constipation
 - Dry skin, brittle nails, weight gain
 - Tired, cold, thinning hair
 - Constipation, weight gain

ANS: D

Clustering involves organizing patient assessment data into groupings with similar underlying causes. The nurse looks for cues among the data that support the diagnosis of a problem. One patient may have several problems simultaneously, requiring the nurse to understand the potential relatedness of signs and symptoms from various body systems. The nurse combines an understanding of pathophysiology, normal structure and function, disease processes, and symptomatology to accurately cluster data. A person who has not had a bowel movement may experience weight gain. Skin, nails, and hair are components of the integumentary system. The subjective feelings of tired and cold are related and prolonged menstruation, as part of the reproductive system, is in a group by itself.

DIF: Understanding

OBJ: 7.5

TOP: Planning

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

12. The nurse in an outpatient clinic obtains a blood pressure of 190/88 mm Hg on a patient diagnosed with high blood pressure. The patient tells the nurse, —My blood pressure medicine is really expensive. Do you think I really need it? The nurse assumes the patient is not taking the medication based on the blood pressure result and the patient's statement and chooses lack of knowledge as a diagnostic label. The nurse identifies the action taken is an example of what concept of Nursing diagnosis formation?
- Clustering unrelated data in the diagnostic statement
 - Selecting erroneous data for use in the diagnostic statement
 - Using medical diagnoses in the diagnostic statement
 - Identifying multiple problems within one diagnostic statement

ANS: A

A variety of errors in identification, statement structure, and statement content may occur when formulating Nursing diagnoses. These include clustering unrelated data, accepting erroneous data, missing the true underlying etiology of a problem, using medical diagnoses as related factors in a NANDA-I Nursing diagnostic statement, and identifying multiple Nursing diagnosis labels in one NANDA-I Nursing diagnostic statement. Clustering unrelated data most often occurs when the nurse has not completed a thorough review of the patient's assessment information or is missing important data. The nurse assumes the patient is not taking the blood pressure medication because of the cost and chooses the diagnosis of noncompliance. The nurse fails to ask the patient if the medication is being taken as ordered. Errors in data collection (e.g., omitting key information) or an incomplete understanding or knowledge of assessment techniques or a patient's condition may lead to the inclusion of erroneous data in a Nursing diagnostic statement. When writing Nursing diagnoses, the nurse should avoid inclusion of more than one label in the statement. Regardless of the type of Nursing diagnosis being written, only one label should be used in each statement. The nurse does not commit this error here. —Lack of knowledge is not a medical diagnosis.

DIF: Understanding

OBJ: 7.6

TOP: Planning

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

13. The nurse is developing a plan of care for a patient with gastritis and an inflammation of the intestines. The patient is complaining of severe abdominal discomfort and nausea. The patient also reports having restless leg syndrome and an inability to urinate. What should the nurse write as a problem statement for the Nursing diagnosis?

- a. Gastritis related to inflammation.
- b. Alterations in comfort and ability to void.
- c. Abdominal pain and nausea related to inflammation.
- d. Alteration in comfort related to restless leg syndrome and inflammation.

ANS: C

One patient may have several problems simultaneously, requiring the nurse to understand the potential relatedness of signs and symptoms from various body systems. The nurse combines an understanding of pathophysiology, normal structure and function, disease processes, and symptomatology to accurately cluster data. Abdominal pain, nausea, and inflammation (of the intestines) are clustered together.

DIF: Understanding

OBJ: 7.6

TOP: Planning

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT: Concepts: Care Coordination

14. The nursing student submits a care plan to the nursing instructor for a review prior to implementing the nursing interventions. The instructor identifies which Nursing diagnostic statement that is written incorrectly?
- a. Difficulty coping related to inadequate support systems as evidenced by patient's verbalization, —I don't have any friends or family in town. I just moved here a week ago.
 - b. Activity intolerance related to immobility as manifested by shortness of breath and patient's verbalization of fatigue.
 - c. Impaired sleep and lack of knowledge related to stress as evidenced by patient report of difficulty sleeping and lack of energy.
 - d. Impaired self feeding related to upper extremity weakness as manifested by inability to get food onto spoon

ANS: C

To correctly formulate a nursing diagnostic statement, the student needs to cluster related data and choose one diagnosis per statement. In the incorrect example, two nursing diagnoses were combined in one statement.

DIF: Understanding

OBJ: 7.6

TOP: Planning

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT: Concepts: Care Coordination

15. When creating a Nursing diagnosis, the nurse knows the related factor is based on what premise?
- a. It should be based on the medical diagnosis.
 - b. It is unrelated to the pathophysiology causing the problem.
 - c. It is the underlying etiology of the patient's situation.
 - d. It does not reflect the nurse's understanding of pathophysiology.

ANS: C

The underlying etiology, or cause of a patient's concern or situation, rather than a medical diagnosis, should be used as a related factor when writing a Nursing diagnosis. By doing so, the nurse articulates an understanding of the pathophysiology or situation with which the patient is faced.

DIF: Understanding

OBJ: 7.6

TOP: Planning

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
Concepts: Care Coordination

16. The nurse is caring for a complex patient needing physical and emotional support. As the primary caregiver, the nurse has which responsibility?
- The nurse is ultimately responsible for assessment of patient needs and progress.
 - The nurse delegates to people who know what they are doing and operate independently.
 - The nurse provides total care to the patient after getting direction from other disciplines.
 - The nurse understands that the patient is ultimately responsible for failure or success.

ANS: A

Even though collaboration and delegation may occur, the nurse is ultimately responsible for the continued assessment of patient needs and progress. As delegator, the nurse must supervise other disciplines to make sure that the patient needs are being met. Detection of additional problems or lack of progress with the patient should prompt the nurse to reconsider the nursing process steps.

DIF: Understanding

OBJ: 7.7

TOP: Planning

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
Concepts: Care Coordination

17. The nurse has identified several problems for a patient scheduled for a bone marrow transplant. When formulating the Nursing diagnosis, the nurse includes which key concept?
- The nurse realizes that changes in patient condition do not have to change diagnoses.
 - The nurse uses a language that is difficult to interpret by legislators.
 - The nurse can communicate with other nurses but not other disciplines.
 - The nurse facilitates communication of patient needs and promotes accountability.

ANS: D

The use of Nursing diagnosis labels facilitates clear communication of patient needs and promotes professional accountability and autonomy by defining and describing the independent area of nursing practice. Nursing diagnostic statements clearly communicate to legislators, consumers, and insurance providers the unique care nurses deliver and the specific nature of the health conditions they treat. Use of a unified language classification system, or taxonomy, is an effective vehicle for communication among nurses and other health care professionals.

DIF: Understanding

OBJ: 7.7

TOP: Planning

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
Concepts: Care Coordination

18. The nurse is developing a plan of care for a patient who had a stroke. Assessment findings include weakness in right upper and lower extremities, numbness in face, slurred speech, difficulty with walking and balance, and headache. The nurse identifies which response would best represent the etiology of the patient's gait and balance problems?
- Lack of muscle motor movement
 - Decreased sensation to touch

- c. Inability to speak clearly
- d. Pain in back of head

ANS: A

The related factor in an actual Nursing diagnosis needs to address the underlying etiology of the patient's problem expressed by the Nursing diagnostic label rather than listing data that are defining characteristics. The decreased sensation to touch, inability to speak clearly, and pain in the back of the head are only reiterations of the defining characteristics (numbness in face, slurred speech, and headache).

DIF: Understanding

OBJ: 7.4

TOP: Planning

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

19. The nurse is caring for a patient admitted to the intensive care unit with malnutrition. The patient is unable to walk and has developed a pressure ulcer from lying in bed constantly without changing positions. The family believes that the patient is depressed and that is why getting out of bed has stopped. When planning this patient's care, the nurse will include which key concept?
- a. Develop multiple Nursing diagnoses.
 - b. Develop only one Nursing diagnosis to aid in focusing.
 - c. Focus on the physical issues facing this patient.
 - d. Deal primarily with the patient's psychological needs.

ANS: A

Analysis of patient assessment data may yield several clusters of related data or cues. It is common to apply several Nursing diagnostic statements to one patient. This is especially true for acutely ill patients with multiple problems related to complex physical or psychological needs.

DIF: Applying

OBJ: 7.7

TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

MULTIPLE RESPONSE

1. The nurse is creating a care plan for a patient admitted with severe bone pain related to an infected leg wound. Which diagnosis written on the plan indicates an understanding of the components of a Nursing diagnosis? (*Select all that apply.*)
- a. Acute pain
 - b. Risk for impaired walking
 - c. Ineffective bone tissue perfusion
 - d. Osteomyelitis
 - e. Infection

ANS: A, B, C

Whereas medical diagnoses identify and label medical (physical and psychological) illnesses, Nursing diagnoses are much broader in focus. Nursing diagnoses consider a patient's response to medical diagnoses and life situations in addition to making clinical judgments based on a patient's actual medical diagnoses and conditions. Pain, potential inability to ambulate, and decreased blood flow to the bone are a patient's response to the medical condition of osteomyelitis. Medical diagnoses identify the specific physical or psychological condition. Osteomyelitis and infection are medical diagnoses defined as inflammation and an infection of the bone usually caused by bacteria.

DIF: Analyzing OBJ: 7.1 TOP: Evaluation
 MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
 Concepts: Care Coordination

2. The nurse is caring for a patient admitted to the psychiatric unit as a result of an overdose of cocaine. Which Nursing diagnosis indicates an understanding of a Nursing diagnostic statement? (*Select all that apply.*)
- a. Impaired breathing related to drug effect on the respiratory center
 - b. Risk for injury related to hallucinations
 - c. Insomnia
 - d. Impaired socialization related to excessive stimulation of nervous system as evidenced by unintelligible speech.
 - e. Powerlessness

ANS: B, D

Each type of Nursing diagnostic statement contains sections or parts. Actual Nursing diagnostic statements are written with three parts: a diagnosis label, related factors, and defining characteristics. Risk Nursing diagnoses have two segments: a diagnosis label and risk factors. Health-promotion Nursing diagnoses are written with only two sections: the diagnosis label and defining characteristics. The first statement needs defining characteristics. Insomnia is a medical diagnosis. The last statement needs etiology and manifestations.

DIF: Remembering OBJ: 7.4 TOP: Assessment
 MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
 Concepts: Care Coordination

3. A patient is admitted to the Emergency Department after experiencing severe chest pain and difficulty in taking deep breaths. The patient anxiously tells the nurse, —My father died suddenly of a heart attack at the age of 52. I'm so scared.‖ Which Nursing diagnoses are appropriate for this situation? (*Select all that apply.*)
- a. Acute pain
 - b. Fear
 - c. Risk for aspiration
 - d. Risk for infection
 - e. Impaired role performance

ANS: A, B

One patient may have several problems simultaneously, requiring the nurse to understand the potential relatedness of signs and symptoms from various body systems. The nurse combines an understanding of pathophysiology, normal structure and function, disease processes, and symptomatology to accurately cluster data. The patient is reporting severe chest pain with an inability to take deep breaths. The Nursing diagnostic label of acute pain is appropriate. Being scared is a defining characteristic of the Nursing diagnosis of fear. The patient is not at risk for aspiration or infection based on the data presented.

DIF: Understanding

OBJ: 7.5

TOP: Planning

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

4. A group of nursing students is discussing the importance of accurately selecting Nursing diagnoses. Which ideas offered in the students' discussion are reasons for choosing the diagnoses carefully? (*Select all that apply.*)
- a. Patient satisfaction
 - b. Positive patient outcomes
 - c. Quality patient care
 - d. Help develop standardized care plans
 - e. Determine appropriate interventions

ANS: A, B, C, E

Ultimately, nurses are accountable for formulating accurate Nursing diagnoses and intervening appropriately. By collecting accurate and complete assessment data and articulating concise Nursing diagnoses for each patient, the professional nurse has a significant impact on patient care outcomes, the quality of patient care, and patient satisfaction. By identifying and writing clear Nursing diagnostic statements, the nurse enables accurate development of individualized patient plans of care. Nursing diagnoses and patient outcomes, which are established during the planning step, help the nurse to determine appropriate interventions for patient care.

DIF: Understanding

OBJ: 7.7

TOP: Planning

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

5. The nurse has requested an order to place a patient on suicide watch. Which data noted in the health assessment led the nurse to this conclusion? (*Select all that apply.*)
- a. Threats of killing oneself
 - b. Chronic pain
 - c. History of prior suicide attempt
 - d. Loneliness
 - e. Stable heart rhythm

ANS: A, B, C, D

Risk factors may be environmental, physical, psychological, or situational concerns. The nurse is concerned that the patient may be at risk for suicide. Verbal statements by the patient, physical illness such as chronic pain, prior attempts to commit suicide, and a lack of social interaction are potential causes for the act of suicide. A stable heart rhythm would not be a safety concern.

DIF: Analyzing

OBJ: 7.4

TOP: Planning

MSC: NCLEX Client Needs Category: Physiological Integrity: Reduction of Risk Potential NOT:
Concepts: Safety

NURSINGTB.COM

Chapter 08: Planning**Yoost & Crawford: Fundamentals of Nursing: Active Learning for Collaborative Practice, 3rd Edition****MULTIPLE CHOICE**

1. The nurse is caring for a patient who has undergone abdominal surgery. The patient stated prior to surgery that —I don't think I'll be able to handle this if I get a colostomy. I wouldn't know how to manage it. The patient is complaining of severe surgical pain and has an order for morphine sulfate. The nurse is correct when addressing which Nursing diagnosis first?
 - a. Pain
 - b. Alteration in body image
 - c. Knowledge deficit
 - d. Risk for falls

ANS: A

Using Maslow's hierarchy of needs helps to organize the most-urgent to less-urgent needs. This framework organizes patient data according to basic human needs common to all individuals. Maslow's theory suggests that basic needs, such as physiologic needs, must be met before higher needs, such as self-esteem. The nurse also realizes that an actual problem takes priority over a potential problem. By using the nursing process appropriately, the nurse correctly chooses the actual, physiological problem first: pain. Once the patient has the morphine, the risk for falls becomes a higher priority than knowledge deficit or alteration in body image because the morphine might confuse the patient, cause dizziness or faintness, and lead to a fall.

DIF: Applying OBJ: 8.2 TOP: Planning

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
Concepts: Care Coordination

2. Setting priorities among identified Nursing diagnoses is the first step in the planning process. The nurse knows this prioritization includes which action?
 - a. Monitoring patient responses
 - b. Carrying out the health care provider's plan of care
 - c. Providing all interventions
 - d. Collaborating with other disciplines

ANS: A

Setting priorities among identified Nursing diagnoses is the first step in the planning process. The nurse is responsible for monitoring patient responses, making decisions culminating in a plan of care, and implementing interventions, including interdisciplinary collaboration and referral, as needed. The nurse is significantly accountable for achieving the desired outcomes.

DIF: Remembering OBJ: 8.1 TOP: Planning

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
Concepts: Care Coordination

3. Which assessment made by the nurse should be addressed first?
 - a. Reddened area to coccyx
 - b. Decreased urinary output

- c. Shortness of breath
- d. Drainage from surgical incision

ANS: C

It is essential that the nurse identify life-threatening concerns and patient situations that need to be addressed most quickly. The ABCs—airway, breathing, and circulation—are a valuable tool for directing the nurse's thought process. Depending on the severity of a problem, the steps of the nursing process may be performed in a matter of seconds.

For instance, if a patient is in respiratory arrest, the most critical goal is for the patient to begin breathing. The reddened coccyx, decreased urinary output, and surgical incision drainage are not immediately life threatening.

DIF: Understanding

OBJ: 8.2

TOP: Planning

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

4. Which patient issue should the nurse address first?
- a. Pain
 - b. Hunger
 - c. Decreased self-esteem
 - d. Absence of pulse

ANS: D

It is essential that the nurse identify life-threatening concerns and patient situations that need to be addressed most quickly. The ABCs—airway, breathing, and circulation—are a valuable tool for directing the nurse's thought process. Depending on the severity of a problem, the steps of the nursing process may be performed in a matter of seconds. In this situation, the patient needs CPR

immediately due to the absence of a pulse. Pain, hunger, and decreased self-esteem are not life-threatening issues. Although the nurse must address them, pulselessness is the priority.

DIF: Understanding

OBJ: 8.2

TOP: Planning

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

5. The nurse demonstrates a thorough understanding of the planning phase of the nursing process when making which statement?
- a. —Patients should be included in the planning process.¶
 - b. —Patient families should not interfere in the planning process.¶
 - c. —The planning process should focus on short-term goals only.¶
 - d. —Planning is the first phase of the nursing process.¶

ANS: A

Planning is the third step of the nursing process. During the planning phase, the professional nurse prioritizes the patient's Nursing diagnoses, determines short- and long-term goals, identifies outcome indicators, and lists nursing interventions for patient-centered care. Patients should be included in the planning process. Involving patients in planning their care helps them to: (1) be aware of identified needs, (2) accept realistic and measurable goals, and (3) embrace interventions to best achieve the mutually agreed-on goals. Inclusion of patients in the planning process tends to improve goal attainment and patient cooperation with interventions. Depending on the patient's condition or circumstances, it may be advantageous to include members of the patient's support system (i.e., family, friends, and caregivers) in the planning phase.

DIF: Understanding

OBJ: 8.3

TOP: Planning

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

6. The nurse recognizes that patient goals include which characteristic?
- They are considered short-term if achieved within a month of identification.
 - They always have established time parameters, such as —long-term or —short-term.
 - They are mutually acceptable to the nurse, patient, and family.
 - They can be vague to facilitate flexibility when evaluating achievement.

ANS: C

Goals are broad statements of purpose that describe the aim of nursing care. Goals represent short- or long-term objectives that are determined during the planning step. Some sources establish time parameters for short- and long-term goals, whereas others do not. According to Carpenito, goals that are achievable in less than a week are short-term goals, and goals that take weeks or months to achieve are long-term goals. Useful and effective goals have certain characteristics. They are mutually acceptable to the nurse, patient, and family. They are appropriate in terms of nursing and medical diagnoses and therapy. The goals are realistic in terms of the patient's capabilities, time, energy, and resources, and they are specific enough to be understood clearly by the patient and other nurses. They can be measured to facilitate evaluation.

DIF: Understanding

OBJ: 8.3

TOP: Planning

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

7. When developing the nursing care plan, the nurse includes which concept when creating goals?
- Develops the goals with the patient and possibly the family.
 - Creates the goals that the nurse wants the patient to achieve.
 - Includes the actions that are needed to accomplish the goal.
 - Focus on goals that are aggressive to ensure success.

ANS: A

The nurse creates goals with the patient and possibly with the family by discussing the patient's current condition, the condition to which the patient wants to progress, and the actions the patient and nurse undertake to accomplish the goal. If the goals are simply what the nurse wants the patient may have other goals in mind to which he or she gives more attention and effort. The goal does not include the interventions or actions needed; however they must be discussed so the patient understands the care he or she is to receive and what part the patient plays in achieving the goals. The nurse works with the patient to develop a plan of care that is appropriately challenging and promotes patient success in attaining goals.

DIF: Applying OBJ: 8.5 TOP: Implementation
 MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
 Concepts: Care Coordination

8. Which statement by the nurse is correct regarding diversity considerations?
- The male gender may struggle less with health care terminology.
 - High numbers of minority populations do not understand health teachings.
 - Older adults understand health teaching easily because of life experience.
 - Disabilities have no impact on the development of patient care goals.

ANS: B

Minority patients globally and those with low English proficiency in the United States are especially vulnerable to low health literacy. Patients of both genders, including those who are well educated and highly literate but have limited health care experience, may struggle with the complexity of health care terminology and procedures. Older adults have also been identified as a group experiencing low health literacy. Before implementing teaching strategies to support goal attainment, the nurse must explore a patient's disabilities and the effects they may have on achieving specific goals. Successful accommodation of a patient's

disabilities should yield attainable goals that lead to positive outcomes.

DIF: Understanding OBJ: 8.3 TOP: Assessment
 MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
 Concepts: Care Coordination

9. The nurse recognizes which is a correctly written example of a short-term goal?
- The patient will lose 50 lb in 1 year.
 - Patient will ambulate 1 mile without shortness of breath.
 - Patient will be able to change the colostomy bag in 6 weeks.
 - The patient will eat 75% of all meals for the next three days.

ANS: D

According to Carpenito, goals that are achievable in less than a week are short-term goals, and goals that take weeks or months to achieve are long-term goals. Eating 75% of all meals for the next three days is an example of a short-term goal. A goal set for 6 weeks or a year in the future is a long-term goal. The ambulation goal has no time determination; therefore it can't be labeled as either short- or long-term and as such, is incorrectly written.

DIF: Analyzing OBJ: 8.4 TOP: Evaluation
 MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
 Concepts: Care Coordination

10. The nurse identifies which goal is written correctly for the Nursing diagnosis of activity intolerance related to imbalance between oxygen supply and demand?
- Patient will walk 1 mile without shortness of breath.
 - Patient will ambulate 100 feet with no shortness of breath on third day after treatment.
 - Patient will climb stairs without shortness of breath by day 2 of hospital stay.
 - Patient will tolerate activity.

ANS: B

Useful and effective goals have certain characteristics. They are appropriate in terms of nursing and medical diagnoses and therapy. The goals are realistic in terms of the patient's capabilities, time, energy, and resources, and they are specific enough to be understood clearly by the patient and other nurses. They can be measured to facilitate evaluation. In option A, there is no time frame to gauge expectations, so the diagnosis is not measurable. In option C, the number of stairs is not specified and so is not measurable. In option D, the type of activity is not mentioned, so it is not specific and there is no measurable criterion.

DIF: Analyzing

OBJ: 8.4

TOP: Evaluation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

11. The nurse recognizes which response as a barrier to achieving goals?
- The effects of pain and/or clinical depression
 - Patient involvement in setting patient goals
 - Family involvement in setting patient goals
 - Realistic expectations of the patient's capabilities

ANS: A

NURSINGTB.COM

Pain and depression both can lead the patient away from cooperation and motivation in working towards goal achievement. The nurse must address either problem in order to have the best chance of success for the patient in meeting goals. Patient and family involvement help create —buy-in and cooperation. Realistic expectations lead to reasonable and achievable goals.

DIF: Understanding

OBJ: 8.4

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

12. The nurse is caring for a patient who has had abdominal surgery and has developed a slight temperature. The nurse identifies which statement to be a patient-centered goal?
- The patient's temperature will return to normal within 24 hours.
 - The nurse will medicate the patient for elevated temperature every 4 hours as needed.
 - Skin integrity will be maintained until the patient is ambulatory.
 - The patient will ambulate 10 feet by postoperative day 2.

ANS: D

Patient-centered goals are written specifically for the patient. The goal should specify the activity the patient is to exhibit or demonstrate to indicate goal attainment. These goals are written to reflect patient, not nursing, activities. Instead of focusing on the patient, the incorrect answers focus on the patient's temperature, the nurse medicating the patient, and the patient's skin integrity. Only option D focuses on the patient.

DIF: Understanding

OBJ: 8.5

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

13. The nurse knows which response to be an example of a measurable goal?
- The patient will be able to lift 10 lb. by the end of week one.||
 - The patient will be able to lift weights by the end of the week.||
 - The patient will be able to lift his normal weight amount.||
 - The patient will be able to lift an acceptable amount of weight by week one.||

ANS: A

Measurable goals are specific, with numeric parameters or other concrete methods of judging whether the goal was met. When writing a goal statement with a patient, the nurse needs to clearly identify how achievement of the goal will be evaluated. When terms such as *acceptable* or *normal* are used in a goal statement, goal attainment is difficult to judge because they are not measurable terms, unless they refer to laboratory values or diagnostic test findings. The amount of weight a patient will lift at the end of the week is not specified.

—Normal and —acceptable weight have not been defined.

DIF: Analyzing

OBJ: 8.3

TOP: Evaluation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

14. The nurse is formulating the patient's care plan. In determining when to evaluate the patient's progress, the nurse is aware that evaluations should be carried out within which parameters?
- They must be done at the end of every shift.
 - They should be done at least every 24 hours.
 - They depend on intervention and patient condition.
 - They are always done at time of discharge.

ANS: C

In most cases, goal statements need to include a time for evaluation. The time depends on the intervention and the patient's condition. Some goals may need to be evaluated daily or weekly, and others may be evaluated monthly. The health care setting affects the time of evaluation. If the goal is set during hospitalization, the goal may need to be evaluated within days, whereas a goal set for home care may be evaluated weekly or monthly. At the time of evaluation, the goal is assessed for goal attainment, and new goals are set or a new evaluation date for the same goal may be chosen if the goal is still applicable for the patient care plan.

DIF: Remembering

OBJ: 8.4

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

15. The nurse knows that standardized care plans may be available and are utilized under which circumstance?

- a. They need to be individualized for each patient.
- b. They are implemented without adjustment.
- c. They remove the need for nurse involvement.
- d. They do not require the use of Nursing diagnoses.

ANS: A

There are multiple formats in which to develop individualized care plans for patients, families, and communities. Each health care agency has its own form, including electronic formats, to facilitate the documentation of patient goals and individualized patient-centered plans of care. All formats contain areas in which the nurse identifies key assessment data, Nursing diagnostic statements, goals, interventions for care, and evaluation of outcomes. In many agencies and specialty units, standardized care plans that must be individualized for each patient are available to guide nurses in the planning process.

DIF: Remembering

OBJ: 8.5

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

16. The nurse recognizes which term identifies nursing interventions that originate from the health care provider orders?
- a. Dependent
 - b. Independent
 - c. Collaborative
 - d. Nursing interventions classifications

ANS: A

Interventions originating from a provider's order are dependent nursing interventions. Independent nursing interventions are originated by the nurse based on expertise in meeting patient needs or preventing complications. Interventions that include collaboration with other providers, such as physical therapy, are collaborative interventions. Nursing Interventions Classification (NIC) is a research-based, standardized collection of interventions and associated activities.

DIF: Remembering

OBJ: 8.6

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

17. The nurse identifies medication administration to be what type of nursing intervention?
- a. Independent
 - b. Dependent
 - c. Collaborative
 - d. Interdisciplinary

ANS: B

Interventions originating from a provider's order are dependent nursing interventions. Independent nursing interventions are originated by the nurse based on expertise in meeting patient needs or preventing complications. Interventions that include collaboration with other providers, such as physical therapy, are collaborative interventions. Collaborative interventions require cooperation among a few or many members of the interdisciplinary health care team.

DIF: Remembering

OBJ: 8.6

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
Concepts: Care Coordination

18. The nurse recognizes which action to be a dependent nursing intervention?
- Utilizing heel protectors
 - Preadmission teaching
 - Medication reconciliation
 - Oxygen administration via mask

ANS: D

Dependent nursing interventions originate from health care provider orders. These interventions include orders for oxygen administration, dietary requirements, medications and diagnostic tests. The nurse incorporates these orders into the patient's overall care. Independent interventions have expanded in recent years, allowing nurses to initiate care that they recognize as essential in meeting patient needs or preventing complications. Utilizing heel protectors for patients susceptible to skin breakdown and initiating preventive measures (e.g., activity regimens, consultations with social workers, preadmission teaching) are often independent, nurse-initiated interventions. Collaborative interventions require cooperation among several health care professionals and unlicensed assistive personnel (UAP).

Collaborative interventions include activities such as physical therapy, home health care, personal care, spiritual counseling, medication reconciliation, and palliative or hospice care.

DIF: Remembering

OBJ: 8.6

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
Concepts: Care Coordination

19. The nurse recognizes that physical therapy, speech therapy, home health care, and personal care are examples of which type of interventions?
- Collaborative interventions
 - Dependent nursing interventions
 - Independent nursing interventions
 - Assessment interventions

ANS: A

Interventions that include collaboration with other providers, such as physical therapy, are collaborative interventions. Interventions originating from a provider's order are dependent nursing interventions. Independent nursing interventions are originated by the nurse based on expertise in meeting patient needs or preventing complications. An assessment is done to gather data.

DIF: Remembering

OBJ: 8.6

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
Concepts: Care Coordination

20. The nurse understands that discharge planning begins at what point in the patient's hospitalization?
- The day before discharge
 - Upon admission
 - Prior to admission
 - Day of discharge

ANS: B

Discharge planning plays an important role in the success of a patient's transition to the home setting after hospitalization. Because most patients are in the hospital for only a short time, nurses must begin discharge planning on admission and continue until a patient is dismissed.

DIF: Remembering

OBJ: 8.7

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

21. The nurse identifies which statement to be accurate regarding discharge planning?
- It may decrease the incidence of patients who need to return to the hospital.||
 - It increases complications and readmissions in most cases.||
 - It adapts to the situation as the patient's conditions changes.||
 - It should begin as soon as the patient is discharged home.||

ANS: A

Research shows that comprehensive discharge planning reduces complications and readmissions. Home care planning adapts to the situation as the patient's condition improves or deteriorates as a result of advancing disease. Because most patients are in the hospital for only a short time, nurses must begin discharge planning on admission and continue until a patient is dismissed.

DIF: Remembering

OBJ: 8.7

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

MULTIPLE RESPONSE

1. Since in the planning phase, the significance of developing organized plans of care for patients is important, the nurse must take seriously which of these responsibilities? (*Select all that apply.*)
- Prioritizing patient needs
 - Developing mutually agreed-on goals
 - Determining outcome criteria
 - Identifying interventions
 - Implementation of the patient's plan of care

ANS: A, B, C, D

The significance of developing organized plans of care for patients cannot be stressed enough. The nurse must take seriously the responsibility of prioritizing patient needs, developing mutually agreed-on goals, determining outcome criteria, and identifying interventions that can help patients to achieve positive outcomes. After these actions are completed in the planning phase of the nursing process, it is time for implementation of the patient's plan of care (implementation phase).

DIF: Understanding

OBJ: 8.7

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

2. The nurse is formulating a plan of care for a patient. In this phase of the nursing process, the nurse should complete which actions? (*Select all that apply.*)
- Prioritize Nursing diagnoses.

- b. Determine short- and long-term goals.
- c. Identify outcome indicators.
- d. List nursing interventions.
- e. Gather assessment data.

ANS: A, B, C, D

Planning is the third step of the nursing process. During the planning phase, the professional nurse prioritizes the patient's Nursing diagnoses, determines short- and long-term goals, identifies outcome indicators, and lists nursing interventions for patient-centered care. Each of these actions requires careful consideration of assessment data (collected earlier) and a thorough understanding of the relationship among Nursing diagnoses, goals, and evidence-based interventions.

DIF: Applying OBJ: 8.1 TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT: Concepts: Care Coordination

3. The nurse recognizes that by involving the patient in planning care, which patient results occur? (*Select all that apply.*)
- a. Being aware of identified needs
 - b. Accepting that not all goals are measurable
 - c. Embracing mutually agreed-on goals
 - d. Feeling a sense of empowerment
 - e. Overcoming unrealistic goals

ANS: A, C, D

Patients should be included in the planning process. Involving patients in planning their care helps them to: (1) be aware of identified needs, (2) accept realistic and measurable goals, and (3) embrace interventions to best achieve the mutually agreed-on goals. Inclusion of patients in the planning process tends to improve goal attainment and patient cooperation with interventions. By accepting guidance and input from patients during the planning process, the nurse provides them with a greater sense of empowerment and control.

DIF: Remembering OBJ: 8.3 TOP: Planning

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT: Concepts: Care Coordination

4. The nurse recognizes measurable goal to have which characteristics? (*Select all that apply.*)
- a. Specific
 - b. Concrete
 - c. Vague
 - d. Easy to judge
 - e. Nonspecific

ANS: A, B, D

Measurable goals are specific, with numeric parameters or other concrete methods of judging whether the goal was met. When writing a goal statement with a patient, the nurse needs to clearly identify how achievement of the goal will be evaluated. When terms such as *acceptable* or *normal* are used in a goal statement, goal attainment is difficult to judge because they are not measurable terms, unless they refer to laboratory values or diagnostic test findings.

DIF: Remembering

OBJ: 8.5

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

NURSINGTB.COM

Chapter 09: Implementation and Evaluation**Yoost & Crawford: Fundamentals of Nursing: Active Learning for Collaborative Practice, 3rd Edition****MULTIPLE CHOICE**

1. The nurse identifies which action as a direct-care intervention?
 - a. Administration of an injection
 - b. Making the change-of-shift report
 - c. Collaborating with members of the health care team
 - d. Ensuring availability of needed equipment

ANS: A

Direct care refers to interventions that are carried out by having personal contact with patients. For example, direct-care interventions include cleaning an incision, administering an injection, ambulating with a patient, and completing patient teaching at the bedside. Indirect care includes nursing interventions that are performed to benefit patients but do not involve face-to-face contact with patients. Examples of indirect care include making the change-of-shift report, communicating and collaborating with members of the interdisciplinary health care team, and ensuring availability of needed equipment.

DIF: Applying OBJ: 9.2 TOP: Implementation
 MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
 Concepts: Caregiving

2. The nurse manager is creating the patient assignment for today. She has five registered nurses (RNs), two licensed practical nurses (LPNs), and five nurse technicians (NAs) scheduled. When making the assignment, the nurse manager needs to remember which fact of delegation?
 - a. RNs are responsible for all care delegated to unlicensed nursing personnel.
 - b. Delegation is considered direct intervention for patient care.
 - c. LPNs operate independently and may delegate patient care.
 - d. Nursing practice is clearly delineated and is standard across the country.

ANS: A

Delegation is the transfer of responsibility for performing a task to another person while the nurse who delegated the task remains accountable. Delegation is an indirect intervention based on assessment findings and established care priorities. Nurses must be familiar with the nurse practice act in their practice jurisdiction to ensure legal delegation. The nursing process cannot be delegated. In most jurisdictions, LPNs function in a dependent role and may not delegate.

DIF: Applying OBJ: 9.2 TOP: Implementation
 MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Safety and Infection Control
 NOT: Concepts: Care Coordination

3. The nurse is preparing to administer medications to a patient. When the patient reports new shortness of breath, which action by the nurse is most appropriate?
 - a. Provide the patient with oxygen since it does not require a provider order.
 - b. Complete at least three checks to ensure that the proper medication is given.

- c. Check the provider orders for all forms of prescription medications.
- d. Document that the 6 rights of medication administration were followed.

ANS: C

All forms of prescription medication (i.e., oral, topical, and parenteral) require an order before administration, as does providing oxygen to a patient. The nurse would check for an as needed order for oxygen. Nurses must complete three checks, follow the six rights of medication administration, and document appropriately when administering medications, but those actions are not the priority due to the change in the patient's condition. The nurse must first address the patient's shortness of breath.

DIF: Applying OBJ: 9.2 TOP: Implementation
 MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
 Concepts: Care Coordination

4. After completing a patient's initial assessment and developing a plan of care, what action by the nurse is most appropriate?
 - a. Continuously reassess the patient.
 - b. Restrict changes to the care interventions.
 - c. Reassess the patient at the start of each shift.
 - d. Evaluate patient goal attainment at intervals.

ANS: A

After the nurse completes a patient's initial assessment and develops a plan of care, continual reassessment of the patient detects noticeable changes in the patient's condition, requiring adjustments to interventions outlined in the plan of care. The need for continual patient reassessment underscores the dynamic nature of the nursing process and is crucial to providing essential care.

NURSINGTB.COM

DIF: Evaluating OBJ: 9.8 TOP: Evaluation
 MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
 Concepts: Care Coordination

5. The male nurse is caring for a female patient who needs a complete bed bath. The patient requests that a female nurse bathe her. The male nurse recognizes this request as an example of what type of diversity?
 - a. Gender diversity involving generational norms.
 - b. Life span diversity
 - c. Disability diversity
 - d. Morphology diversity

ANS: A

The nurse must perform the procedures competently and safely, taking into consideration any special needs of the patient. Gender diversity occurs with the identification of gender roles that may affect care delivery. Some patients may prefer care from nurses of the same gender. This preference may stem from generational norms, personal comfort, or cultural considerations. With life span diversity, interventions must always be age or developmental level appropriate. Disability diversity requires that interventions be individualized for each patient and adapted for any limitations. The nurse must ensure safe practice in relation to patient body size (morphology diversity) and should seek additional support or equipment when necessary.

DIF: Remembering

OBJ: 9.2

TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
Concepts: Care Coordination

6. The nurse is providing care for a patient of the Jehovah's Witness faith. Based on the nurse's knowledge of the patient's religious beliefs, the nurse would question which order?
- Obtain vital signs every shift.
 - Regular diet as tolerated.
 - Activity as tolerated.
 - Infuse 1 unit packed red blood cells.

ANS: D

Some interventions may be declined because of religious affiliation (e.g., blood transfusion for a Jehovah's Witness, pork-based insulin for a Muslim patient).

DIF: Evaluating

OBJ: 9.2

TOP: Evaluation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
Concepts: Care Coordination

7. The nurse is caring for a patient who is blind. When reviewing the care plan, the nurse would modify which goal?
- The patient will report any drainage from the wound with a foul odor to the primary care provider after discharge.
 - The patient will agree to report pain promptly while hospitalized.
 - The patient will obtain no injuries while in the hospital.
 - The patient will report any purulent wound drainage to the primary care provider after discharge.

NURSINGTB.COM

ANS: D

Interventions must be individualized for each patient and adapted for any limitations (e.g., amputation, learning disability, blindness, deafness). The patient would be able to detect a foul odor, report pain, and remain injury free, but would not be able to tell if drainage is purulent.

DIF: Analyzing

OBJ: 9.2

TOP: Evaluation

MSC: NCLEX Client Needs Category: Physiological Integrity: Basic Care and Comfort NOT:
Concepts: Care Coordination

8. The registered nurse is providing an independent nursing intervention when completing which action?
- Administering oral medications
 - Administering oxygen
 - Providing emotional support
 - Administering intravenous medication

ANS: C

Independent nursing interventions are tasks within the nursing scope of practice that the nurse may undertake without a physician or PCP order. Repositioning a patient in bed, performing oral hygiene, and providing emotional support through active listening are examples of independent nursing interventions. Dependent nursing interventions are tasks the nurse undertakes that are within the nursing scope of practice but require the order of a primary care provider to be implemented. Administering patient medications or administering oxygen to a patient are examples of common dependent nursing interventions that require clinical judgment before implementation. These interventions are based on a collaborative effort of the nurse and the physician to provide care to patients.

DIF: Evaluating OBJ: 9.4 TOP: Implementation
 MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
 Concepts: Care Coordination

9. The nurse recognizes which topic is appropriate teaching content for the patient who is returning from surgery?
- Signs and symptoms of infection
 - Use of patient-controlled analgesia
 - Activity limitations upon discharge
 - Physical therapy

ANS: B

Readiness to learn is an important consideration. For example, when a patient returns from surgery, it is essential that some information be reviewed (e.g., how to use the patient-controlled analgesia pump and incentive spirometer) but completing all discharge teaching at this time would not be effective. At other times, teaching is more formalized, such as discharge teaching, signs of infection, and physical therapy.

DIF: Analyzing OBJ: 9.2 TOP: Implementation

MSC: NCLEX Client Needs Category: Physiological Integrity: Health Promotion and Maintenance NOT:
 Concepts: Care Coordination

10. The nurse is learning to identify readiness to learn in patients. Which patient would the nurse identify correctly as ready to learn?
- The patient requesting pain medication for treatment of severe discomfort
 - The patient reporting nausea and vomiting
 - The patient who was just told the diagnosis of cancer of the pancreas
 - The patient who was recently diagnosed with diabetes mellitus and is scheduled to be discharged in 2 days

ANS: D

Choosing opportunities when the patient's condition and environment are most conducive to learning is recommended when attempting to teach patients. Patients who are in pain, are nauseated, or who have been given recent traumatic diagnoses are not psychologically able to retain information.

DIF: Evaluating OBJ: 9.4 TOP: Assessment
 MSC: NCLEX Client Needs Category: Physiological Integrity: Health Promotion and Maintenance NOT:
 Concepts: Care Coordination

11. The nurse asks the patient for permission to involve the patient's family members in the teaching plan for the patient. Which response is the best rationale to support this involvement?
- Involving the family empowers the patients and their support system.
 - Teaching family members decreases the number of questions they may ask.
 - Educated family members ensure the patient will comply with the treatment plan.
 - The family members may be interested in the information.

ANS: A

With the patient's permission, the nurse should share instructions with the people who may assist with care. Nurses empower patients and their support systems through effective teaching. When nurses provide patients and their families with opportunities to ask questions and comprehend health care information, they become an integral part of the health care process. The family members may ask fewer questions but that is not a reason to involve them. Nothing will ensure patient compliance other than the patient deciding to do so. Family members may be interested in the information, but that is not the main reason to include them.

DIF: Analyzing OBJ: 9.2 TOP: Planning
MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:
Concepts: Care Coordination

12. The nurse identifies change-of-shift report, collaboration with other health care members, and ensuring availability of needed equipment are examples of which term?
- Indirect care
 - Direct care
 - Referrals
 - Delegation

ANS: A

NURSINGTB.COM

Indirect care includes nursing interventions that are performed to benefit patients but do not involve face-to-face contact with patients. Examples of indirect care include making the change-of-shift report, communicating and collaborating with members of the interdisciplinary health care team, and ensuring availability of needed equipment. Direct care refers to interventions that are carried out by having personal contact with patients. For example, direct-care interventions include cleaning an incision, administering an injection, ambulating with a patient, and completing patient teaching at the bedside. Referrals in health care involve sending a patient to another member of the interdisciplinary health care team for a consultation or other services. Delegation is the transfer of responsibility for performing a task to another person while the nurse who delegated the task remains accountable.

DIF: Remembering OBJ: 9.3 TOP: Implementation
MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
Concepts: Care Coordination

13. The nurse correctly identifies which referral as an inappropriate nursing referral?
- Music therapist
 - Community agencies
 - Adaptive care services
 - Dermatologist

ANS: D

A primary care provider (PCP) may refer a patient to a medical or surgical specialist for further assessment, testing, or treatment. Nurses are often instrumental in initiating these types of referrals but do not complete the actual referral. Referral to a community agency is usually a collaborative action. Obtaining adaptive services and music therapy are independent nursing actions.

DIF: Applying OBJ: 9.3 TOP: Implementation
 MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
 Concepts: Care Coordination

14. When implementing research-based interventions, the nurse realizes which concept?
- Implementing evidence-based care is unique to the nursing profession.
 - Evidence-based practice is based entirely in nursing research.
 - Evidence-based care is focused on practices and not outcomes.
 - Nurses must read recent literature and remain current in practice.

ANS: D

To implement research-based interventions, nurses must read recent literature and remain current in practice. Implementation of evidence-based care is not unique to nursing; it involves interventions provided by all members of the interdisciplinary health care team. The best methods for treating patients with a variety of signs and symptoms are researched by nurses with input from the research findings of other disciplines. Nursing care continues to evolve as nursing research provides new knowledge and recognizes best practices to improve patient care and outcomes. Evidence-based practice guidelines and updated information must be included in plans of care.

DIF: Remembering OBJ: 9.3 TOP: Implementation
 MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
 Concepts: Care Coordination

15. When the nurse is supportive and works on behalf of patients, this role is identified by which term?
- Advocate
 - Primary care provider
 - Collaborator
 - Delegator

ANS: A

Nurses advocate by supporting and working on behalf of patients or persons for whom they have concern. Nurses advocate for patients by coordinating care and supporting the changes necessary to improve conditions and outcomes. Effective communication and collaboration regarding patient care are essential for patient safety and positive patient outcomes. The change-of-shift reports are an example. A PCP is usually a physician or advance practice nurse. Delegation is the transfer of responsibility for performing a task to another person while the nurse who delegated the task remains accountable.

DIF: Remembering OBJ: 9.3 TOP: Implementation
 MSC: NCLEX Client Needs Category: Health Promotion and Maintenance
 NOT: Concepts: Care Coordination

16. The nurse recognizes which task that cannot be delegated?

- a. Obtaining vital signs
- b. Assessing lung sounds
- c. Bathing a patient
- d. Ambulating a patient

ANS: B

Delegation is the transfer of responsibility for performing a task to another person while the nurse who delegated the task remains accountable. Obtaining vital signs, bathing, and ambulating are all tasks associated with the assessment part of the nursing process. The nursing process cannot be delegated.

DIF: Analyzing OBJ: 9.3 TOP: Implementation
 MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
 Concepts: Care Coordination

17. The nurse identifies repositioning a patient, providing hygiene, and active listening as examples of what concept?
- a. Dependent interventions
 - b. Independent nursing interventions
 - c. Standing orders
 - d. Counseling

ANS: B

Independent nursing interventions are tasks within the nurse's scope of practice and do not require an order from a physician. Dependent nursing interventions are tasks the nurse undertakes that are within the nursing scope of practice but require the order of a primary care provider to be implemented. Some physician orders are received through a preapproved standardized order set known as **standing orders**. Counseling is the process through which individuals use professional guidance to address personal conflicts or emotional problems.

DIF: Understanding OBJ: 9.4 TOP: Implementation
 MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
 Concepts: Caregiving

18. The patient has an order for morphine sulfate 2 mg intravenously prn (as needed) every 2 hours. When the nurse administers this medication, which concept is being provided?
- a. Independent nursing intervention
 - b. Dependent nursing intervention
 - c. Referral
 - d. Indirect care procedure

ANS: B

Dependent nursing interventions are tasks that require an order from a physician or primary care provider (PCP). Independent nursing interventions are tasks within the nursing scope of practice that the nurse may undertake without a physician or PCP order. Referrals in health care involve sending a patient to another member of the interdisciplinary health care team for a consultation or other services. Indirect care includes nursing interventions that are performed to benefit patients but do not involve face-to-face contact with patients.

DIF: Applying OBJ: 9.5 TOP: Implementation
 MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
 Concepts: Care Coordination

19. The nurse understands which essential fact regarding documentation?
- It should be completed accurately and in a timely manner.
 - It should not be computerized (EHR) because of disclosure risks.
 - It is not a legal document although they can be helpful in lawsuits.
 - It cannot be used in determining billing and reimbursement issues.

ANS: A

All documentation entries should be completed in a timely, accurate, and professional manner. Documentation most often is charted in the patient's EHR and standardized flow sheets according to agency policy. Patient health records are legal documents. Within the Health Insurance Portability and Accountability Act (HIPAA) guidelines, patient documentation is provided to insurance companies and others for billing and reimbursement.

DIF: Applying OBJ: 9.7 TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

20. The nurse knows what fact to be the focus of evaluation, the final phase of the nursing process?
- The focus is recording the care that was implemented.
 - The focus is medical and nursing goals for the welfare of the patient.
 - The focus is long-term goals only.
 - The focus is patient responses to interventions and outcomes.

ANS: D

Evaluation is the final step in the nursing process. Evaluation focuses on the patient and the patient's response to nursing interventions and outcome attainment. Evaluation is not a record of care that was implemented. Patient outcomes serve as the criteria against which the success of a nursing intervention is judged. During the evaluation phase, nurses use critical thinking to determine whether a patient's short- and long-term goals were met and whether desired outcomes were achieved. Monitoring whether the patient's goals were attained is collaborative, involving the patient in the decision-making process.

DIF: Remembering OBJ: 9.8 TOP: Evaluation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Care Coordination

21. When the nurse realizes that the patient's short-term goals have not been met, the nurse should carry out which task?
- Revise or adapt the plan of care.
 - Assume that the patient did not want to achieve his goals.
 - Understand that a plan of care is almost never changed.
 - Reassess plans of care only after major patient-nurse interactions.

ANS: A

When a patient goal is unmet or only partially met, the plan of care may need to be revised or adapted to support goal attainment. There are many reasons why goals are not met, including changes in the patient condition, unrealistic goals, or inappropriate interventions that do not help meet the goal. It is common for plans of care to change to meet evolving needs. Reassessment occurs with each patient–nurse interaction. As changes in a patient’s condition occur, the plan of care should be revised.

DIF: Applying OBJ: 9.8 TOP: Evaluation
 MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
 Concepts: Care Coordination

22. The nurse identifies that the nursing process is an attempt to meet patient needs, including which concept?
- Nursing process is linear in nature.
 - Nursing process is dynamic and cyclic.
 - Nursing process requires occasional care plan re-evaluation.
 - Nursing process does not allow care plan modification.

ANS: B

The nursing process is ongoing in an attempt to meet patient needs. The nursing process is not linear in nature but is dynamic and cyclic, constantly adapting to a patient’s health status. Care plan modifications may be necessitated due to deterioration or improvement of a patient’s condition. The Joint Commission requires patient care plans to be evaluated on a continual basis.

DIF: Understanding OBJ: 9.9 TOP: Evaluation
 MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
 Concepts: Care Coordination

MULTIPLE RESPONSE

1. The nurse recognizes which interventions to be prevention oriented? (*Select all that apply.*)
- Immunization programs
 - Cleansing an incision
 - Cardiac risk factor modification
 - Placing infants prone when they sleep
 - Teaching patients to ask their providers to wash their hands

ANS: A, B, C, D, E

Some interventions prevent illness or complications and promote healthy activities or lifestyles. Interventions such as patient education and immunization programs are prevention oriented. Cleansing an incision is a nursing intervention that can help prevent infection. Educating a patient about risk-factor modification for cardiovascular disease may prevent a future myocardial infarction. Placing infants on their backs to sleep may reduce the risk of sudden infant death syndrome. Patients should be instructed to ask their care providers to wash their hands if they have not observed them doing so.

DIF: Remembering OBJ: 9.3 TOP: Implementation
 MSC: NCLEX Client Needs Category: Health Promotion and Maintenance
 NOT: Concepts: Caregiving

2. The nurse considers which skills to be invasive procedures? (*Select all that apply.*)
 - a. Administering oral medications
 - b. Starting an intravenous (IV) line
 - c. Repositioning the patient
 - d. Inserting a urinary catheter

ANS: B, D

Many interventions focus on physical care that is performed when treating patients. These interventions may include invasive procedures, such as starting an intravenous line or inserting a catheter, or they may be noninvasive, such as administering oral medications and repositioning.

DIF: Evaluating OBJ: 9.2 TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Safety and Infection Control

NOT: Concepts: Caregiving

3. The nurse understands that the five rights of delegation include which components? (*Select all that apply.*)
 - a. Right patient
 - b. Right time
 - c. Right person
 - d. Right supervision
 - e. Right task

ANS: C, D, E

Delegation principles focus on the appropriate intervention (task) being performed under the correct circumstances, by the correct personnel, and with the correct direction and supervision.

The right patient and the right time refer to components of the —6 Rights of medication administration.

DIF: Understanding OBJ: 9.3 TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Caregiving

Chapter 10: Documentation, Electronic Health Records, and Reporting**Yoost & Crawford: Fundamentals of Nursing: Active Learning for Collaborative Practice, 3rd Edition****MULTIPLE CHOICE**

1. The nurse understands the need for accurate documentation due to which fact?
 - a. Accurate documentation is needed for proper reimbursement.
 - b. Accurate documentation must be electronically generated.
 - c. Accurate documentation does not include e-mails or faxes.
 - d. Accurate documentation is only accepted in court if written by hand.

ANS: A

Accurate documentation is necessary for hospitals to be reimbursed according to diagnostic-related groups (DRGs). DRGs are a system used to classify hospital admissions.

Health care documentation is any written or electronically generated information about a patient that describes the patient, the patient's health, and the care and services provided, including the dates of care. These records may be paper or electronic documents, such as electronic medical records, faxes, e-mails, audiotapes, videotapes, and images. All such records are considered legal documentation and may be used in court.

DIF: Remembering

OBJ: 10.1

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT: Concepts: Communication

2. The nurse identifies which statement to be true regarding nursing documentation?
 - a. Standards for documentation are established by a national commission.
 - b. Medical records should be accessible to everyone.
 - c. Documentation should not include the patient's diagnosis.
 - d. High-quality nursing documentation reflects the nursing process.

ANS: D

The ANA's model for high-quality nursing documentation reflects the nursing process and includes accessibility, accuracy, relevance, auditability, thoughtfulness, timeliness, and retrievability. Standards for documentation are established by each health care organization's policies and procedures. They should be in agreement with The Joint Commission's standards and elements of performance, including having a medical record for each patient that is accessed only by authorized personnel. General principles of medical record documentation from the Centers for Medicare and Medicaid Services (2017) include the need for completeness and legibility; the reasons for each patient encounter, including assessments and diagnosis; and the plan of care, the patient's progress, and any changes in diagnosis and treatment.

DIF: Understanding

OBJ: 10.1

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT: Concepts: Communication

3. The nurse identifies which true statement regarding the medical record?
 - a. It serves as a major communication tool but is not a legal document.
 - b. It cannot be used to assess quality of care issues.

- c. It is not used to determine reimbursement claims.
- d. It can be used as a tool for biomedical research and provide education.

ANS: D

The medical record promotes continuity of care and ensures that patients receive appropriate health care services. The record can be used to assess quality-of-care measures, determine the medical necessity of health care services, support reimbursement claims, and protect health care providers, patients, and others in legal matters. It is a clinical data archive. The medical record serves as a tool for biomedical research and provider education, collection of statistical data for government and other agencies, maintenance of compliance with external regulatory bodies, and establishment of policies and regulations for standards of care. The record serves as the major communication tool between staff members and as a single data access point for everyone involved in the patient's care. It is a legal document that must meet guidelines for completeness, accuracy, timeliness, accessibility, and authenticity. The record can be used to assess quality-of-care measures, determine the medical necessity of health care services, support reimbursement claims, and protect health care providers, patients, and others in legal matters.

DIF: Understanding

OBJ: 10.2

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT: Concepts: Communication

4. The nurse knows that paper records are being replaced by other forms of record keeping for what reason?
- a. Paper is fragile and susceptible to damage.
 - b. Paper records are always available to multiple people at a time.
 - c. Paper records can be stored without difficulty and are easily retrievable.
 - d. Paper records are permanent and last indefinitely.

ANS: A

Paper records have several potential problems. Paper is fragile, susceptible to damage, and can degrade over time. It may be difficult to locate a particular chart because it is being used by someone else, it is in a different department, or it is misfiled. Storage and control of paper records can be a major problem.

DIF: Evaluating

OBJ: 10.2

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT: Concepts: Communication

5. When the nurse is charting in the paper medical record, what action does the nurse carry out?
- a. Print his/her name since signatures are often not readable.
 - b. Omit nursing credentials since only the nurses chart
 - c. Skip a line between entries so that it looks neat.
 - d. Use black ink unless the facility allows a different color.

ANS: D

Entries into paper medical records are traditionally made with black ink to enable copying or scanning, unless a facility requires or allows a different color. The date, time, and signature, with credentials of the person writing the entry, are included in the entry. No blank spaces are left between entries because they could allow someone to add a note out of sequence.

DIF: Remembering

OBJ: 10.3

TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
Concepts: Communication

6. The nurse is admitting a patient who has had several previous admissions. To obtain a knowledge base about the patient's medical history, the nurse would access which document?
- Electronic medical record (EMR)
 - The computerized provider order entry (CPOE)
 - Electronic health record (EHR)
 - Primary provider's office notes

ANS: C

The EHR is a longitudinal record of health that includes the information from inpatient and outpatient episodes of health care from one or more care settings. The EMR is a record of one episode of care, such as an inpatient stay or an outpatient appointment. CPOE allows clinicians to enter orders in a computer that are sent directly to the appropriate department. It does not provide historical data. The primary provider's office notes may not include all the patient's information if the patient has other providers.

DIF: Applying OBJ: 10.2 TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
Concepts: Communication

7. The nurse understands which statement about the use of electronic health records is true?
- They improve patient health status.
 - They require a keyboard to enter data.
 - They have not reduced medication errors.
 - They require increased storage space.

ANS: A

Adoption of an EHR system produces major cost savings through gains in productivity and error reduction, which ultimately improves patient health status. The most common benefits of electronic records are increased delivery of guideline-based care, better monitoring, reduced medication errors, and decreased use of care. Use of EHRs can reduce storage space, allow simultaneous access by multiple users, facilitate easy duplication for sharing or backup, and increase portability in environments using wireless systems and hand-held devices. Although data are often entered by keyboard, they can also be entered by means of dictated voice recordings, light pens, or handwriting and pattern recognition systems.

DIF: Remembering OBJ: 10.2 TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
Concepts: Communication

8. The nurse is caring for patients on a unit that uses electronic health records (EHRs). What action by the nurse protects personal health information?
- The nurse should allow only nurses that he/she knows and trusts to use his/her verification code.
 - The nurse should not worry about mistakes since the information cannot be tracked.
 - The nurse should never share any password with anyone.
 - The nurse should be aware that the EHR is sophisticated and immune to failure.

ANS: C

Access to an EHR is controlled through assignment of individual passwords and verification codes that identify people who have the right to enter the record. Passwords and verification codes should never be shared with anyone. Health care information systems have the ability to track who uses the system and which records are accessed. These organizational tools contribute to the protection of personal health information. Disadvantages of use of computers for documentation include computer and software failure and problems if there is a power outage.

DIF: Applying OBJ: 10.4 TOP: Implementation
 MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
 Concepts: Communication

9. The nurse recognizes which statement to be accurate regarding what should be documented?
- Document facts and subjective data from the patient.
 - Document how he/she feels about the care being provided.
 - Document in a —blockl fashion once per shift.
 - Double document as often as possible in order to not miss anything.

ANS: A

Nursing documentation is an important part of effective communication among nurses and with other health care providers. Documentation should be factual and nonjudgmental, with proper spelling and grammar. Subjective data from the patient should be included. Events should be reported in the order they happened, and documentation should occur as soon as possible after assessment, interventions, condition changes, or evaluation. Each entry includes the date, time, and signature with credentials of the person documenting. Double documentation of data should be avoided because legal issues can arise as a result of conflicting data. [NURSINGTB.COM](https://www.nursingtestbank.com)

DIF: Remembering OBJ: 10.3 TOP: Assessment
 MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
 Concepts: Communication

10. The nurse recognizes that nursing documentation is guided by what process?
- The nursing process
 - NANDA-I, nursing diagnoses
 - Nursing interventions classification*
 - Nursing Outcomes Classification

ANS: A

Nursing documentation is guided by the five steps of the nursing process: assessment, diagnosis, planning, implementation, and evaluation. Standardized nursing terminologies such as the North American Nursing Diagnosis Association–International (NANDA-I) Nursing Diagnoses, *nursing interventions classification (NIC)*, and Nursing Outcomes Classification (NOC) may be used in the documentation process.

DIF: Remembering OBJ: 10.3 TOP: Assessment
 MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
 Concepts: Communication

11. What fact does the nurse know applies to PIE, APIE, SOAP, and SOAPIE documentation?
- They are chronologic.

- b. They are examples of problem-oriented charting.
- c. They are narrative charting.
- d. They are forms of —charting by exception.¶

ANS: B

The nurse's notes may be in a narrative format or in a problem-oriented structure such as the PIE, APIE, SOAP, SOAPIE, SOAPIER, DAR, or CBE format. Narrative charting is chronologic, charting by exception (CBE) is documentation that records only abnormal or significant data.

DIF: Remembering

OBJ: 10.3

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Communication

12. The nursing instructor teaching students about charting explains that this type of charting records only abnormal or significant data?
- a. PIE
 - b. SOAP
 - c. Narrative
 - d. Charting by exception

ANS: D

Charting by exception (CBE) is documentation that records only abnormal or significant data. A PIE note is used to document problem (*P*), intervention (*I*), and evaluation (*E*). A SOAP note is used to chart the subjective data (*S*), objective data (*O*), assessment (*A*), and plan (*P*). Narrative charting is chronologic, with a baseline recorded on a shift-by-shift basis. Data are recorded in the progress notes, often without an organizing framework. Narrative charting may stand alone, or it may be complemented by other tools.

DIF: Remembering

OBJ: 10.3

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Communication

13. Prior to preparing to administer medications to the patient, the nurse should compare the provider orders with what document?
- a. Flow sheet
 - b. Kardex
 - c. MAR
 - d. Admission summary

ANS: C

A medication administration record (MAR) is a list of ordered medications, along with dosages and times of administration, on which the nurse initials medications given or not given. A paper MAR usually includes a signature section in which the nurse is identified by linking the initials used with a full signature. The EHR includes an electronic medication administration record (eMAR). Flow sheets and checklists may be used to document routine care and observations that are recorded on a regular basis, such as vital signs, and intake and output measurements. Data collected on flow sheets may be converted to a graph, which pictorially reflects patient data. Originally, the Kardex was a nonpermanent filing system for nursing records, orders, and patient information that was held centrally on the unit. Although computerization of records may mean that the Kardex system is no longer active, the term *kardex* continues to be used generically for certain patient information held at the nurses' station. An admission summary includes the patient's history.

DIF: Applying OBJ: 10.3 TOP: Implementation
 MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
 Concepts: Communication

14. The nurse is caring for a patient for the first time and needs background information such as history, medications taken at home, etc. What is the best central location for the nurse to obtain this information?
- Admission summary
 - Discharge summary
 - Flow sheet
 - Kardex

ANS: A

An admission summary includes the patient's history, a medication reconciliation, and an initial assessment that addresses the patient's problems, including identification of needs pertinent to discharge planning and formulation of a plan of care based on those needs. The discharge summary addresses the patient's hospital course and plans for follow-up, and it documents the patient's status at discharge. It includes information on medication and treatment, discharge placement, patient education, follow-up appointments, and referrals. Flow sheets and checklists may be used to document routine care and observations that are recorded on a regular basis, such as vital signs, medications, and intake and output measurements. Although computerization of records may mean that the Kardex system is no longer active, the term *kardex* continues to be used generically for certain patient information held at the nurses' station.

DIF: Applying OBJ: 10.3 TOP: Implementation
 MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
 Concepts: Communication

15. What fact is the nurse aware of when charting using paper nursing notes?
- Use red ink so the nursing entries stand out.
 - When mistakes are made in documentation, the nurse should white out the entry.
 - Only one nurse should document on a sheet so that it can be removed in case of error.
 - The medical record, in any format, is the most reliable source of information in a legal action.

ANS: D

The medical record is seen as the most reliable source of information in any legal action related to care. When legal counsel is sought because of a negative outcome of care, the first action taken by an attorney is to acquire a copy of the medical record. Ink color is usually black, blue or other as designated by the facility. Notes should never be altered or obliterated. Documentation mistakes must be acknowledged. If an error is made in paper documentation, a line is drawn through the error and the word *error* is placed above or after the entry, along with the nurse's initials and followed by the correct entry.

DIF: Applying OBJ: 10.3 TOP: Implementation
 MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
 Concepts: Communication

16. What fact is the nurse aware of when charting using electronic documentation?
- Errors can be corrected and totally removed from the record in the screen view.
 - Log-on access to the electronic record identifies the person charting.
 - Each entry requires the nurse to sign her/his name and credentials.
 - Documenting significant changes in the electronic record ends the nurse's responsibility.

ANS: B

Log-on access to the electronic record identifies the person charting or making a change. If an error is made in electronic documentation, it can be corrected on the screen view but the error and correction process remain in the permanent electronic record. Any correction in documentation that indicates a significant change in patient status should include notification of the primary care provider.

DIF: Understanding OBJ: 10.3 TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
 Concepts: Communication

17. What action should the nurse take to correct an error in paper charting?
- Remove the sheet with the error and replace it with a new sheet with the correct entry.
 - Scribble out the error and rewrite the entry correctly.
 - Draw a single line through the error write —error above or after the entry, along with the nurse's initials.
 - Leave the entry as is and tell the charge nurse.

ANS: C

Documentation mistakes must be acknowledged. If an error is made in paper documentation, a line is drawn through the error and the word *error* is placed above or after the entry, along with the nurse's initials and followed by the correct entry. Notes should never be altered or obliterated. Documentation mistakes must be acknowledged.

DIF: Applying OBJ: 10.3 TOP: Implementation
 MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
 Concepts: Communication

18. If a verbal or phone order is necessary in an emergency, the nurse knows what action needs to be completed?
- The order must be taken by an RN or LPN.

- b. The order must be repeated verbatim to confirm accuracy.
- c. The order is documented as a written order.
- d. The order does not need further verification by the provider.

ANS: B

If a verbal or phone order is necessary in an emergency, the order must be taken by a registered nurse (RN) who repeats the order verbatim to confirm accuracy and then enters the order into the paper or electronic system, documenting it as a verbal or phone order and including the date, time, physician's name, and RN's signature. Most facility policies require the physician to co-sign a verbal or telephone order within a defined time period.

DIF: Understanding

OBJ: 10.6

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT: Concepts: Communication

19. The nurse identifies which statement to be accurate regarding the process of making a change-of-shift report (handoff)?
- a. Handoff is an uncommon occurrence of little importance.
 - b. Handoff occurs only at change of shift and only to oncoming nurses.
 - c. Handoff can lead to patient death if done incorrectly.
 - d. Handoff does not allow for collaboration or problem solving.

ANS: C

An ineffective handoff may lead to wrong treatments, wrong medications, or other life-threatening events, increasing the length of stay and causing patient injury or death. Improvement in the hand-off process can increase patient safety and promote positive patient outcomes. The hand-off process can be an opportunity for collaborative problem solving.

During an average hospital stay of approximately 4 days, as many as 24 handoffs can occur for just one patient because shifts change every 8 to 12 hours and many individuals are responsible for care.

DIF: Understanding

OBJ: 10.5

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT: Concepts: Communication

20. When the patient has had a fall while trying to climb out of bed, the nurse must carry out which task?
- a. Complete an incident report as a risk management document.
 - b. Complete an incident report and add it to the medical record.
 - c. Document that an incident report was completed in the medical record.
 - d. Say nothing about the incident in the medical record.

ANS: A

Incident reports are objective, nonjudgmental, factual reports of the occurrence and its consequences. The incident report is not part of a medical record but is considered a risk management or quality-improvement document. The fact that an incident report was completed is not recorded in the patient's medical record; however, the details of a patient incident are documented.

DIF: Applying

OBJ: 10.7

TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT: Concepts: Communication

MULTIPLE RESPONSE

1. The nurse identifies which components to be expected nursing documentation? (*Select all that apply.*)
 - a. Nursing assessment
 - b. The care plan
 - c. Critique of the physician's care
 - d. Interventions
 - e. Patient responses to care

ANS: A, B, D, E

Expected nursing documentation includes a nursing assessment, the care plan, interventions, the patient's outcomes or response to care, and assessment of the patient's ability to manage after discharge. Documentation should be factual and nonjudgmental.

DIF: Remembering

OBJ: 10.3

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT: Concepts: Communication

2. The Joint Commission has compiled a list of do-not-use abbreviations, acronyms, and symbols to avoid the possibility of errors that may be life threatening. The nurse identifies which abbreviations to be unacceptable? (*Select all that apply.*)
 - a. prn
 - b. QD
 - c. qod
 - d. o.X mg
 - e. X mg

NURSINGTB.COM

ANS: B, C

Nurses must be aware of the danger of using abbreviations that may be misunderstood and compromise patient safety. The Joint Commission (2018) has compiled a list of do-not-use abbreviations, acronyms, and symbols to avoid the possibility of errors that may be life threatening. QD, Q.D., qd, q.d. (daily), QOD, Q.O.D., qod, and q.o.d. (every other day) can be mistaken for each other. Periods after *Q* can be mistaken for *I*, and the *O* mistaken for *I*. Write *daily* or *every other day*. Trailing zero (X.o mg) or a lack of leading zero (.X mg) can be confusing. Write as *X mg* or *o.X mg*.

DIF: Applying

OBJ: 10.3

TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT: Concepts: Communication

3. The nurse understands the use of standardized language in care planning is beneficial for what reasons? (*Select all that apply.*)
 - a. Standardized language provides consistency.
 - b. Standardized language improves communication among nurses.
 - c. Standardized language increases the visibility of nursing interventions.
 - d. Standardized language enhances data collection. Standardized language supports adherence to care standards.

ANS: A, B, C, D

Standardized nursing terminologies such as the North American Nursing Diagnosis Association–International (NANDA-I) Nursing diagnoses, Nursing Interventions Classification (NIC), and Nursing Outcomes Classification (NOC) may be used in the documentation process. Use of standardized language provides consistency, improves communication among nurses and with other health care providers, increases the visibility of nursing interventions, improves patient care, enhances data collection to evaluate nursing care outcomes, and supports adherence to care standards.

DIF: Remembering

OBJ: 10.3

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Communication

4. When charting is done using the DAR charting format, the nurse documents which components? (*Select all that apply.*)
- The patient problems
 - Subjective data
 - Any actions initiated
 - Objective data
 - The patient's response to interventions

ANS: A, C, E

A DAR note is used to chart the data (*D*) collected about the patient problems, the action (*A*) initiated, and the patient's response (*R*) to the actions. A SOAP note is used to chart the subjective data (*S*), objective data (*O*), assessment (*A*), and plan (*P*).

DIF: Remembering

OBJ: 10.3

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Communication

5. The nurse knows that the Health Insurance Portability and Accountability Act (HIPAA) allows health information to be shared in which circumstances? (*Select all that apply.*)
- To provide treatment for the patient
 - To determine billing and payment issues
 - To enhance health care operations related to the patient
 - In public areas such as the cafeteria or elevator
 - Over the telephone with any family member

ANS: A, B, C

The Health Insurance Portability and Accountability Act (HIPAA), originally passed in 1996, created standards for the protection of personal health information, whether conveyed orally or recorded in any form or medium. The act clearly mandates that protected health information may be used only for treatment, payment, or health care operations. HIPAA privacy standards should be applied during phone, fax, e-mail, or Internet transmission of protected patient information.

DIF: Understanding

OBJ: 10.4

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Communication

6. The nursing student is learning about SBAR reporting. What statements about the patient are matched with the correct part of the report? (*Select all that apply.*)

- a. Patient is an 84-year-old female with a history of hypertension: S
- b. Patient's blood pressure has dropped from 142/92 to 98/48 mmHg: S
- c. Patient is hemorrhaging with four saturated dressings in an hour: A
- d. The patient took an overdose of antidepressants three days ago: B
- e. By policy, the patient needs transferred to the ICU; please come write the orders: R

ANS: B, C, D, E

SBAR stands for situation (what is happening the current time), background (circumstances leading up to this situation), assessment (what the nurse thinks the problem is), and recommendation (what needs to be done to correct the situation). A history of hypertension would be background (if it were related to the current issue).

DIF: Understanding

OBJ: 10.5

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Communication

NURSINGTB.COM

Chapter 11: Ethical and Legal Considerations**Yoost & Crawford: Fundamentals of Nursing: Active Learning for Collaborative Practice, 3rd Edition****MULTIPLE CHOICE**

1. The student nurse learning about ethics expresses good knowledge when making which appropriate statement?
 - a. —Ethics are internal values developed outside the influence of societal norms.||
 - b. —Ethics are influenced by many variables including family and friends.||
 - c. —Ethics are societal in nature and do not involve personal influences.||
 - d. —Ethics are totally independent from a person's character.||

ANS: B

Family, friends, beliefs, education, culture, and socioeconomic status influence the development of ethical behavior. The study of ethics considers the standards of moral conduct in a society. Personal ethics are influenced by values, societal norms, and practices. Behaviors that are judged as ethical or unethical, right or wrong, reflect a person's character.

DIF: Remembering

OBJ: 11.2

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Ethics

2. The nurse is providing patient care and pays special attention to meeting the needs of the patient while maintaining the patient's right to privacy, confidentiality, autonomy, and dignity. This nurse is applying what ethical theory?
 - a. Deontology
 - b. Utilitarianism
 - c. Autonomy
 - d. Accountability

ANS: A

Deontology is an ethical theory that stresses the rightness or wrongness of individual behaviors, duties, and obligations without concern for the consequences of specific actions. Meeting the needs of patients while maintaining their right to privacy, confidentiality, autonomy, and dignity is consistent with the tenets of deontology. Compared with deontology, utilitarianism is on the opposite end of the ethical theory continuum. Utilitarianism maintains that behaviors are determined to be right or wrong solely based on their consequences.

Autonomy, or self-determination, is the freedom to make decisions supported by knowledge and self-confidence. Accountability is the willingness to accept responsibility for one's actions.

DIF: Remembering

OBJ: 11.1

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Ethics

3. The nurse is caring for a patient recently diagnosed with cancer that is being asked to participate in a new chemotherapy trial. How would the nurse respond if working under the ethical principle of utilitarianism?
 - a. —The patient should be allowed to decide.||

- b. —As your nurse, I'll support your right to refuse.¶
- c. —You should do this because many could benefit from it.¶
- d. —If this is against your beliefs, you should not do it.¶

ANS: C

Compared with deontology, utilitarianism is on the opposite end of the ethical theory continuum. Utilitarianism maintains that behaviors are determined to be right or wrong solely based on their consequences. Deontology is an ethical theory that stresses the rightness or wrongness of individual behaviors, duties, and obligations without concern for the consequences of specific actions. Meeting the needs of patients while maintaining their right to privacy, confidentiality, autonomy, and dignity is consistent with the tenets of deontology. Autonomy, or self-determination, is the freedom to make decisions supported by knowledge and self-confidence. The remaining responses are examples of either deontology or autonomy.

DIF: Applying OBJ: 11.1 TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Ethics

4. The nurse realizes that a medication error has been made. The nurse then reports the error and takes responsibility to ensure patient safety despite personal consequences. This nurse has exhibited what ethical concept?
 - a. Autonomy
 - b. Accountability
 - c. Justice
 - d. Advocacy

ANS: B

Accountability is the willingness to accept responsibility for one's actions. Autonomy, or self-determination, is the freedom to make decisions supported by knowledge and self-confidence. Supporting or promoting the interests of others or doing so for a cause greater than oneself defines advocacy. To do justice is to act fairly and equitably.

DIF: Remembering

OBJ: 11.2

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Ethics

5. The nurse is providing care for a patient who has had a stroke recently and has multiple self-care deficits. The nurse is coordinating care with in-home agencies and arranging for the delivery of needed equipment. Which ethical concept is the nurse applying?
 - a. Advocacy
 - b. Confidentiality
 - c. Autonomy
 - d. Accountability

ANS: A

Supporting or promoting the interests of others or doing so for a cause greater than ourselves defines advocacy. Confidentiality is the ethical concept that limits sharing private patient information. Autonomy, or self-determination, is the freedom to make decisions supported by knowledge and self-confidence. Accountability is the willingness to accept responsibility for one's actions.

DIF: Remembering

OBJ: 11.2

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
Concepts: Ethics

6. A nurse has been asked to care for a patient who is an inmate from a nearby prison. During shift report, the nurse asks, —Why was the man convicted and imprisoned? Another nurse responds that this is not important since nurses are required to provide compassionate care for all people in all circumstances. The responding nurse has displayed what concept?
- Beneficence
 - Advocacy
 - Confidentiality
 - Autonomy

ANS: A

In its simplest form, beneficence can be defined as doing good. Nurses demonstrate beneficence by acting on behalf of others and placing a priority on the needs of others rather than on personal thoughts and feelings. The ethical concept of beneficence necessitates providing care for the prisoner without reproach and provide compassionate care for all people in all circumstances. Supporting or promoting the interests of others or doing so for a cause greater than ourselves defines advocacy. Confidentiality is the ethical concept that limits sharing private patient information. Autonomy, or self-determination, is the freedom to make decisions supported by knowledge and self-confidence.

DIF: Remembering

OBJ: 11.2

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
Concepts: Ethics

7. The nurse is providing care to a patient experiencing pain. The nurse assesses the pain and promptly administers the ordered analgesics as promised to the patient. This nurse has applied what concept?
- Autonomy
 - Accountability
 - Confidentiality
 - Fidelity

ANS: D

Keeping promises or agreements made with others constitutes fidelity. In nursing, fidelity is essential for building trusting relationships with patients and their families. Following through on promises is a critical factor in establishing strong professional relationships with patients and their families. Autonomy, or self-determination, is the freedom to make decisions supported by knowledge and self-confidence. Accountability is the willingness to accept responsibility for one's actions. Confidentiality is the ethical concept that limits sharing private patient information.

DIF: Remembering

OBJ: 11.2

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
Concepts: Ethics

8. The nurse understands —First, do no harm defines what ethical principle?
- Beneficence
 - Justice
 - Fidelity

d. Nonmaleficence

ANS: D

First, do no harm is the colloquial definition of nonmaleficence. Unlike beneficence, which requires actively doing good, nonmaleficence requires only the avoidance of harm. In its simplest form, beneficence can be defined as doing good. To do justice is to act fairly and equitably. Keeping promises or agreements made with others constitutes fidelity.

DIF: Remembering

OBJ: 11.2

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Ethics

9. The nurse is caring for a patient whose family does not want the patient to be told about the new diagnosis of cancer because of the poor prognosis. Keeping this secret from the patient is in direct conflict with which ethical concepts?

- a. Autonomy and veracity
- b. Veracity and advocacy
- c. Justice and nonmaleficence
- d. Confidentiality and justice

ANS: A

Autonomy, or self-determination, is the freedom to make decisions supported by knowledge and self-confidence. Truthfulness defines the ethical concept of veracity. Supporting or promoting the interests of others or to do so for a cause greater than ourselves defines advocacy. To do justice is to act fairly and equitably. *First, do no harm* is the colloquial definition of nonmaleficence. Unlike beneficence, which requires actively doing good, nonmaleficence requires only the avoidance of harm. Confidentiality is the ethical concept that limits sharing private patient information.

DIF: Remembering

OBJ: 11.2

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Ethics

10. The nurse identifies that The Code of Ethics for Nurses is defined in which terms?

- a. Like the Constitution and not revisable
- b. A succinct statement of ethical obligations
- c. Required by entry level nurses only
- d. A negotiable document dependent on individual conscience

ANS: B

The current nursing code, the Code of Ethics for Nurses with interpretive statements, was published in 2015. The Code of Ethics for Nurses is —a succinct statement of the ethical obligations and duties of every individual who enters the nursing profession, the profession's —nonnegotiable ethical standard, and —an expression of nursing's own understanding of its commitment to society.

DIF: Understanding

OBJ: 11.3

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Ethics

11. The nurse knows which statement indicates an appropriate understanding of ethical practice by the student nurse?

- a. —I will be held to the same ethical standards as professional nurses.||
- b. —I will not be held ethically accountable until I graduate.||
- c. —My nurse educators are responsible for my ethical standards.||
- d. —Ethics are not important as a student.||

ANS: A

The Code of Ethics for Nurses is —a succinct statement of the ethical obligations and duties of every individual (not just nurse educators) who enters the nursing profession,|| the profession's —nonnegotiable ethical standard,|| and —an expression of nursing's own understanding of its commitment to society.|| This is a powerful mandate for all nurses to communicate and act professionally to prevent inflicting physical or emotional pain on others while pursuing nursing education and engaging in nursing practice.

DIF: Understanding

OBJ: 11.3

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Ethics

12. The nurse is caring for a patient who has been belligerent and is in 4-point —leather|| restraints. When the patient continues to be verbally abusive and still tries to kick and punch staff even though he is restrained, the nurse should carry out which action?
- a. Do not attempt to meet patient needs until the patient has calmed down.
 - b. Only provide care while security is in the room.
 - c. Continue to attempt to meet the patient's needs.
 - d. Inform the patient the police will be called if the patient's behavior does not stop.

ANS: C

Provision 1.5 (of the Nursing Code of Ethics) states, —The principle of respect for persons extends to all individuals with whom the nurse interacts. The nurse maintains compassionate and caring relationships with colleagues and others with a commitment to the fair treatment of individuals, to integrity-preserving compromise, and to resolving conflict. The nurse should make all attempts to provide for the patient's needs. It is unrealistic to only provide care if security is present. Telling the patient that the police will be called is threatening.

DIF: Applying

OBJ: 11.3

TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Ethics

13. The nurse recognizes which action by the nursing student would be considered uncivil?
- a. Prompt arrival to class
 - b. Texting during class
 - c. Attentive listening
 - d. Active participation in class

ANS: B

Civility (i.e., acting politely) is essential in all interactions among faculty and nursing students. Respectful interaction between students and faculty members establishes professional communication patterns and affects the way in which students interact with patients. Texting in class is disrespectful and is an example of incivility. Arriving on time, listening attentively, and participating in class all show respect and civility.

DIF: Remembering

OBJ: 11.3

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care

NOT: Concepts: Ethics

14. When providing end-of-life care, the nurse knows it is essential to carry out which action?
- Tell the patient what he might like to hear to relieve anxiety.
 - Begin making health care decisions for the patient.
 - Provide the patient with the nurse's personal opinions.
 - Offer unconditional support for the patient and family.

ANS: D

Two major roles of a nurse caring for a dying patient are: (1) providing accurate information regarding the disease process and treatment options and (2) offering support for the patient and family without interjecting personal opinions. An essential ethical concept is autonomy, which underscores the importance of allowing patients to make their own health care decisions. Limiting information to what will relieve anxiety, providing personal opinions, and making decisions for the patient do not demonstrate respect for patient autonomy.

DIF: Applying OBJ: 11.4 TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT: Concepts: Ethics

15. The nurse frequently cares for patients who are nearing the end of life. The nurse identifies what strategy that is designed to prolong the time of death rather than restoring life?
- Establishing a do-not-resuscitate (DNR) order
 - Adherence to living will requests
 - Removal of extraordinary measures already in place
 - Continuance of futile care

ANS: D

NURSINGTB.COM

Ethical dilemmas in end-of-life care exist regarding the establishment of do-not-resuscitate (DNR) orders, adherence to living will and organ donation requests, removal of extraordinary measures already initiated, and continuance of futile care (i.e., care that is useless and prolongs the time until death rather than restoring life).

DIF: Understanding OBJ: 11.4 TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT: Concepts: Caregiving

16. Which statement by the nurse indicates comprehension of ethical issues?
- Ethical issues are rare occurrences but take a great deal of time to resolve.
 - Ethical issues have required The Joint Commission to mandate ethics committees.
 - Ethical issues most frequently lead to legal intervention in patient care matters.
 - Ethical issues lead to ethics committees made up entirely by nurses.

ANS: B

All nurses are faced with ethical decisions each day in practice, and some choose to obtain further education and experience in the field of bioethics and participate on institutional ethics committees along with physicians, ethicists, attorneys, and academicians. Ethics committees are required by The Joint Commission to respond to ethical challenges related to patient care requiring consultation. The work of the ethics committees in health care institutions helps to prevent unnecessary legal intervention in patient care matters. Ethics committee members come from all areas of health care, not just nursing. If acceptable resolutions are not achieved through consultation with the ethics committee, patients, families, and health care providers, the legal system may become involved.

DIF: Understanding

OBJ: 11.4

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Caregiving

17. After studying legal issues important to nursing, the student shows appropriate understanding with which statement?
- Laws change often, creating liability issues for nurses.
 - Licensure laws are devised to protect the nurse.
 - The nurse is not responsible for other disciplines' mistakes.
 - Keeping current with changing laws can protect the nurse.

ANS: D

Laws delineate acceptable nursing practice, provide a basis on which many health care decisions are determined, and protect nurses from liability in cases in which safe practice is maintained. Each state has a nurse practice act that establishes the standards of care required for legal nursing practice. Licensure, laws, rules, and regulations governing nursing practice are enforced to protect the public from harm. In many cases, the nurse is the last line of defense to prevent an error in medication administration or other types of patient care.

Keeping current with changing laws related to nursing practice and technology can ensure safety for nurses and their patients.

DIF: Understanding

OBJ: 11.6

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Caregiving

18. The nurse knows practicing nursing without a license is what wrongdoing?
- Misdemeanor
 - Statute
 - Felony
 - Tort

ANS: C

Practicing nursing without a license is a felony. A misdemeanor is a minor crime, such as stealing an item from a patient that does not have much value. A statute is a law created by legislative bodies. Torts are crimes committed against another person. An intentional tort example is assault and battery. Negligence and malpractice are examples of unintentional torts.

DIF: Remembering

OBJ: 11.7

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Health Care Law

19. The nurse recognizes that starting an intravenous (IV) infusion line on a patient against his will may be classified as which wrongdoing?
- Assault
 - Battery
 - Felony
 - Misdemeanor

ANS: B

Actual physical harm caused to another person is battery. Battery may involve angry, forceful touching of people, their clothes, or anything attached to them. Performing a surgical procedure without informed consent is an example of battery. Actions much more subtle, such as inserting an intravenous catheter or urinary catheter against the will of a patient, also may be classified as battery. Assault is a threat of bodily harm or violence caused by a demonstration of force by the perpetrator. A feeling of imminent harm or feeling of immediate danger must exist for assault to be claimed. A misdemeanor is a crime of lesser consequence that is punishable by a fine or incarceration in a local or county jail for up to 1 year. A felony is a more serious crime that results in the perpetrator's being imprisoned in a state or federal facility for more than 1 year.

DIF: Remembering

OBJ: 11.7

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Health Care Law

20. The nurse is caring for a patient who has had many admissions and readmissions. The nurse believes that the patient keeps coming to the hospital because the patient —wants his drugs, and is —non-compliant at home with diabetic therapy. To reduce the risk of slander against this

patient, the nurse should carry out which action?

- Write opinions in the medical record only.
- Never share observations.
- Make judgmental statements in private.
- Avoid making judgmental statements.

ANS: D

Defamation of character occurs when a public statement is made that is false and injurious to another person. Oral defamation of character is slander. Slander is spoken information that is untrue, causing prejudice against someone or jeopardizing that person's reputation. The nurse should not make opinionated, slanderous comments about patients, orally or in writing.

Written forms of defamation of character are considered libel.

DIF: Applying

OBJ: 11.7

TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Health Care Law

21. The nurse is providing care for a patient who demands discharge from the hospital against the physician's orders. What action by the nurse is most appropriate?
- Have the patient sign an —Against medical advice form.
 - Follow the guidelines as presented in the code of Academic and Clinical Conduct.
 - Review the ANA's Nursing Code of Ethics for guidance.
 - Permit the patient to leave after an informed consent form is signed.

ANS: A

Preventing patients from leaving a health care facility at their request may be considered false imprisonment. To prevent health care providers and institutions from being held liable if a patient chooses to leave a facility when physicians and nurses think that it is in the patient's best interest to remain hospitalized, the patient is asked to sign an *against medical advice* (AMA) form. A signed AMA form documents that the patient has chosen to leave the facility when leaving could jeopardize the patient's condition. The National Student Nurses Association adopted the Code of Academic and Clinical Conduct, in which students agree to —promote the highest level of moral and ethical principles and —promote an environment that respects human rights, values, and choice of cultural and spiritual beliefs. This document does not apply to the issue at hand. The Code of Ethics for Nurses is —a succinct statement of the ethical obligations and duties of every individual who enters the nursing profession. While this is resource for nurses the described situation requires nurses to follow facility policy. Informed consent is permission granted by a patient after discussing each of the following topics with the physician, surgeon, or advanced practice nurse who will perform the surgery or procedure: (1) exact details of the treatment, (2) necessity of the treatment, (3) all known benefits and risks involved, (4) available alternatives, and (5) risks of treatment refusal. This does not apply to the stated situation.

DIF: Applying OBJ: 11.7 TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Health Care Law

22. The nurse has been involved sexually with a patient. The nurse manages becomes aware of this situation and tells the nurse this behavior is a which type of crime?
- Malpractice
 - Libel
 - Slander
 - Battery

NURSINGTB.COM

ANS: A

Malpractice may occur when a professional such as nurse acts unethically, demonstrates deficient skills, or fails to meet standards of care required for safe practice. Examples of these types of malpractice include engaging in sexual activity with a patient and administering penicillin to a patient with a documented penicillin allergy, resulting in the patient's death from a severe allergic (anaphylactic) reaction. Written forms of defamation of character are considered libel. Broadcasting or reading statements aloud that have the potential to hurt the reputation of another person is considered libel. Oral defamation of character is slander.

Actual physical harm caused to another person is battery.

DIF: Remembering OBJ: 11.8 TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Health Care Law

23. The nurse understands state legislatures give authority to administrative bodies, such as state boards of nursing, to carry out what action?
- Create statutory laws.
 - Establish regulatory laws.
 - Try case law cases.
 - Create laws based on social mores

ANS: B

Statutory law is created by legislative bodies such as the U.S. Congress and state legislatures. Statutory laws are often referred to as *statutes*. State legislatures give authority to administrative bodies, such as state boards of nursing, to establish regulatory law, which outlines how the requirements of statutory law will be met. Judicial decisions from individual court cases determine case law. Case law was historically referred to as *common law* because it originally was determined by customs or social mores that were common at the time.

DIF: Understanding

OBJ: 11.6

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Health Care Law

24. Which nurse has committed a serious documentation error?

- a. The nurse who documents all medications for assigned patients prior to administration.
- b. The nurse who documents medication administration as the medications are given.
- c. The nurse who documents assessments as soon as they are completed.
- d. The nurse who documents meal intake as meal trays are picked up.

ANS: A

Documentation must be accurate to provide a realistic view of a patient's condition. Serious documentation errors include: (1) omitting documentation from patient records, (2) recording assessment findings obtained by another nurse or unlicensed assistive personnel (UAP), and (3) recording care not yet provided. Nurses sometimes document that a patient has received medication before its administration; this is a serious violation of the law and becomes a medication error of omission if the nurse is distracted before administering the patient's

medication.

DIF: Understanding

NURSINGTB.COM

OBJ: 11.8

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Health Care Law

25. The nurse understands who is ultimately responsible for explaining the content of the informed consent?

- a. The registered nurse
- b. The hospital social worker
- c. Educated family members
- d. The provider of the procedure

ANS: D

Informed consent is permission granted by a patient after discussing each of the following topics with the physician, surgeon, or advanced practice nurse who will perform the surgery or procedure: (1) exact details of the treatment, (2) necessity of the treatment, (3) all known benefits and risks involved, (4) available alternatives, and (5) risks of treatment refusal.

DIF: Understanding

OBJ: 11.9

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Health Care Law

26. The nurse knows which law protects health care professionals from charges of negligence when providing emergency care at the scene of an accident?

- a. Good Samaritan Act
- b. HIPPA
- c. Licensure
- d. Living wills

ANS: A

All 50 states have enacted Good Samaritan laws offering protection for physicians and other health care professionals who provide emergency care at the scene of a disaster, emergency, or accident. Good Samaritan laws protect health care professionals from charges of negligence in providing emergency care if: (1) the care is within the professional's scope of knowledge and standards of care and (2) no fee is received or charged for services. The Health Insurance Portability and Accountability Act (HIPAA) was enacted in 1996 to protect the privacy of health care information. Licensure and certification of nurses seek to ensure professional competence. The laws of each state require graduates of accredited nursing schools and colleges pass the National Council Licensure Examination (NCLEX) before beginning professional practice. A living will specifies the treatment a person wants to receive when he/she is unconscious or no longer capable of making decisions independently.

DIF: Understanding

OBJ: 11.10

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Health Care Law

MULTIPLE RESPONSE

1. In addition to maintaining current professional practice knowledge, competent practice skills, and professional relationships with patients and their families, what additional actions should the nurse take to practice within the law? (Select all that apply.)
 - a. Maintain confidentiality.
 - b. Follow legal guidelines for sharing information.
 - c. Block document once per shift.
 - d. Change nursing procedures according to latest journal articles.
 - e. Meet licensure and continuing education requirements.

ANS: A, B, E

In addition to maintaining current professional practice knowledge, competent practice skills, and professional relationships with patients and their families, nurses should follow guidelines to practice legally and avoid charges of malpractice, maintain confidentiality, follow legal and ethical guidelines when sharing information, document punctually and accurately, adhere to established institutional policies governing safety and procedures, comply with legal requirements for handling and disposing of controlled substances, meet licensure and continuing education requirements, and practice responsibly within the scope of personal capabilities, professional experience, and education.

DIF: Applying

OBJ: 11.6

TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Caregiving

2. Which statements by the nurse are correct regarding informed consent and someone who requires an interpreter? (Select all that apply.)
 - a. A professional interpreter is needed.

- b. A family member may interpret when convenient.
- c. Detailed medical information remains a priority.
- d. Professional interpreters are not effective in providing medical information.
- e. If necessary, family members can make decisions regarding informed consent.

ANS: A, C, E

If a patient is illiterate or requires an interpreter, the method of obtaining informed consent must be adapted appropriately. Use of a professional interpreter rather than a family member is essential to provide detailed medical information accurately. A patient whose culture prefers to allow other family members to make final health care decisions is inconsistent with nursing's ethical belief in autonomy. However, in this situation, the method of obtaining informed consent may need to be adapted to meet the patient's beliefs within the scope of the law.

DIF: Understanding

OBJ: 11.9

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
Concepts: Health Care Law

3. Health care providers are required to supply patients with written information regarding their rights to make medical decisions and implement advance directives, which consist of three documents. The nurse knows which items are considered —advanced directives!? (*Select all that apply.*)
- a. Living will
 - b. Durable power of attorney
 - c. Health care proxy
 - d. Patient's Bill of Rights
 - e. The Uniform Anatomical Gift Act

GB.CM
NUSNT O

ANS: A, B, C

Advance directives consist of three documents: (1) living will, (2) durable power of attorney, and (3) health care proxy, commonly referred to as a *durable power of attorney for health care*. The Patient's Bill of Rights informs consumers of health care about specific privileges of which they should be aware. Patients should expect: (1) excellent care, (2) a safe environment, (3) participation in planning their care, (4) privacy, (5) help with discharge arrangements, and (6) assistance with fulfilling financial responsibilities. The Uniform Anatomical Gift Act was approved to allow people over the age of 18 to donate their bodies or body parts after death for transplantation, deposit in tissue banks, or research.

DIF: Understanding

OBJ: 11.9

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
Concepts: Health Care Law

Chapter 12: Leadership and Management**Yoost & Crawford: Fundamentals of Nursing: Active Learning for Collaborative Practice, 3rd Edition****MULTIPLE CHOICE**

1. The nurse knows while leadership behaviors and management skills often complement each other they differ in which way?
 - a. Managers focus on relationships.
 - b. A manager may not possess leadership traits.
 - c. Leadership focuses on coordinating and directing others.
 - d. A manager is a visionary who sets the direction for a group.

ANS: B

A manager may not possess leadership traits, and a leader may lack management skills. Management is the process of coordinating others and directing them toward a common goal. Leadership focuses on relationships, using interpersonal skills to persuade others to work toward a common goal. Leaders are visionaries who set the overall direction for a group or organization.

DIF: Remembering

OBJ: 12.1

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT: Concepts: Leadership

2. The nurse correctly defines leadership when making which statement?
 - a. —Leadership is coordinating others toward a common goal.
 - b. —Leadership is the ability to influence others.
 - c. —Leadership focuses on the task at hand.
 - d. —Leadership is based in formal authority.

ANS: B

Leaders have the ability influence and motivate others while maintaining relationships to accomplish a goal. Management is the process of coordinating others and directing them toward a common goal. Management is focused on the task at hand. A manager holds a formal position of authority in an organization; that position includes accountability and responsibility for accomplishing the tasks within the work environment. Managers demonstrate accountability when they are answerable for their own actions and the actions of those under their direction.

DIF: Understanding

OBJ: 12.1

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT: Concepts: Leadership

3. The nurse is acting as a leader in the role of charge nurse and notes that the unlicensed assistive personnel (UAP) on the floor are stressed related to their increased workload. The nurse changes the original planned approach based on the presenting situation. Which theory of leadership is the nurse implementing with this action?
 - a. Situational
 - b. Transactional
 - c. Transformational

d. Autocratic

ANS: A

Situational theories suggest that leaders change their approach depending on the situation. Transactional leaders use reward and punishment to gain the cooperation of followers. Transformational leaders use methods that inspire people to follow their lead. Transformational leaders work toward transforming an organization with the help of others. The authoritarian or autocratic leader exercises strong control over subordinates.

DIF: Understanding

OBJ: 12.2

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Leadership

4. The unit charge nurse uses reward and punishment to gain the cooperation of the nurses assigned to the unit. What type of leader is this charge nurse?
- Transformation
 - Autocratic
 - Transactional
 - Situational

ANS: C

Transactional leaders use reward and punishment to gain the cooperation of followers. Transformational leaders use methods that inspire people to follow their lead. Transformational leaders work toward transforming an organization with the help of others. The authoritarian or autocratic leader exercises strong control over subordinates. Situational theories suggest that leaders change their approach depending on the situation.

DIF: Understanding

OBJ: 12.2

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Leadership

5. The nurse manager of a unit is sharing the most recent results of a patient satisfaction survey to motivate staff. This approach is a characteristic of what type of nursing leader?
- Transformational
 - Transactional
 - Situational
 - Autocratic

ANS: A

Transformational leaders use methods that inspire people to follow their lead. Transformational leaders work toward transforming an organization with the help of others sharing survey results may work to inspire staff. Transactional leaders use reward and punishment to gain the cooperation of followers. The authoritarian or autocratic leader exercises strong control over subordinates. Situational theories suggest that leaders change their approach depending on the situation.

DIF: Understanding

OBJ: 12.2

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Leadership

6. The nurse manager is considered a —great communicator. She can be found on the unit talking with staff, keeping them informed and asking their opinions. She believes that nurses are motivated by internal means and that they want to participate in making decisions about the unit although the final decision always rests with her. The nurses recognize that this nurse manager is what type of leader?
- Autocratic
 - Democratic
 - Bureaucratic
 - Laissez-faire

ANS: B

The participative or democratic leader believes that employees are motivated by internal means and want to participate in decision making. The primary function of the leader in this situation is to foster communication and develop relationships with followers. The authoritarian or autocratic leader exercises strong control over subordinates. Like the autocratic leader, the bureaucratic leader assumes that employees are motivated by external forces. This type of leader relies on policies and procedures to direct goals and work processes. The nurse using bureaucratic leadership tends to relate impersonally to staff and exercises power on the basis of established rules. Like the democratic leader, the permissive or laissez-faire leader thinks that employees are motivated by their own desire to do well. The laissez-faire leader provides little or no direction to followers, who develop their own goals and make their own decisions.

DIF: Understanding

OBJ: 12.2

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Leadership

7. Upon entering a patient's room, the nurse notes that the patient is unresponsive. The nurse takes control and begins to direct other members of the health care team during this crisis. The nurse is demonstrating characteristics of which type of nursing leadership?
- Autocratic
 - Democratic
 - Laissez-faire
 - Bureaucratic

ANS: A

The authoritarian or autocratic leader exercises strong control over subordinates. In this scenario, the nurse takes charge and gives directions that others will follow. The participative or democratic leader believes that employees are motivated by internal means and want to participate in decision making. The primary function of the leader in this situation is to foster communication and develop relationships with followers. Like the democratic leader, the permissive or laissez-faire leader thinks that employees are motivated by their own desire to do well. The laissez-faire leader provides little or no direction to followers, who develop their own goals and make their own. Like the autocratic leader, the bureaucratic leader assumes that employees are motivated by external forces. This type of leader relies on policies and procedures to direct goals and work processes. The nurse using bureaucratic leadership tends to relate impersonally to staff and exercises power on the basis of established rules.

DIF: Understanding

OBJ: 12.2

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Leadership

8. A patient is found unresponsive and pulseless. The nurse begins cardiopulmonary resuscitation (CPR) and calls for help. When help arrives, the nurse should take on which role?
- Autocratic leader
 - Democratic leader
 - Laissez-faire leader
 - Bureaucratic leader

ANS: A

Although autocratic leadership is a strict form of leadership, it is useful in crisis situations. A nurse may act as an autocratic leader when taking charge after a patient is found unresponsive. In this situation, it is helpful to have a leader who takes control and directs other members of the health care team. Democratic leaders may see themselves as equals with other team members and may consult with other nurses, exhibiting a democratic form of leadership. This style of leadership can be used in unit council meetings where nurses collaborate to identify solutions to common problems. A nurse in a leadership position who uses the laissez-faire style of leadership assigns patient care and expects all team members to set goals for the day and manage their time to complete the assignment. Successful implementation of this leadership style in nursing requires a highly efficient and reliable staff, such as seen in some specialized OR nursing teams with a history of working together on a set type of cases. The bureaucratic leader relies on policies and procedures to direct goals and work processes. The nurse using bureaucratic leadership tends to relate impersonally to staff and exercises power on the basis of established rules.

DIF: Applying OBJ: 12.2 TOP: Implementation
 MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
 Concepts: Leadership

9. The nurse has made patient care assignments and expects all team members to set their own goals for the day and manage their time to meet their goals. The nurse is implementing what style of leadership?
- Autocratic
 - Democratic
 - Bureaucratic
 - Laissez-faire

ANS: D

Like the democratic leader, the permissive or laissez-faire leader thinks that employees are motivated by their own desire to do well. The laissez-faire leader provides little or no direction to followers, who develop their own goals and make their own decisions. The authoritarian or autocratic leader exercises strong control over subordinates. The participative or democratic leader believes that employees are motivated by internal means and want to participate in decision making. The primary function of the leader in this situation is to foster communication and develop relationships with followers. Like the autocratic leader, the bureaucratic leader assumes that employees are motivated by external forces. This type of leader relies on policies and procedures to direct goals and work processes. The nurse using bureaucratic leadership tends to relate impersonally to staff and exercises power on the basis of on established rules.

DIF: Applying OBJ: 12.2 TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
 Concepts: Leadership

10. The nurse manager of the emergency room believes that efficiency is the expected standard for the department and believes that efficiency lies in following established rules, policies, and guidelines. The only way to change procedures is to changes rules, policies, and guidelines. To run the emergency room with this philosophy, the nurse manager must take on which role?
- Laissez-faire leader
 - Democratic leader
 - Bureaucratic leader
 - Autocratic leader

ANS: C

Like the autocratic leader, the bureaucratic leader assumes that employees are motivated by external forces. This type of leader relies on policies and procedures to direct goals and work processes. The nurse using bureaucratic leadership tends to relate impersonally to staff and exercises power on the basis of established rules. The permissive or laissez-faire leader thinks that employees are motivated by their own desire to do well. The laissez-faire leader provides little or no direction to followers, who develop their own goals and make their own. The participative or democratic leader believes that employees are motivated by internal means and want to participate in decision making. The primary function of the leader in this situation is to foster communication and develop relationships with followers. The authoritarian or autocratic leader exercises strong control over subordinates.

DIF: Applying OBJ: 12.2 TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
 Concepts: Leadership

NURSINGTB.COM

11. The manager of the intensive care unit is accepting an award for excellence and efficiency in the provision of patient care. The manager accepts the award for the unit and cites the contributions of the staff since, without their expertise and dedication, the award may not have been achieved. The staff nurse recognizes the nurse manager is demonstrating which quality?
- Dedication
 - Openness
 - Magnanimity
 - Creativity

ANS: C

Magnanimity means giving credit where credit is due. Good leaders reflect the work and success of accomplishing a goal by crediting those who helped reach it. Dedication is the ability to spend the time necessary to accomplish a task. Effective leaders persist in working toward accomplishment of a goal even when doing so is difficult. Openness refers to the leader's ability to listen to other points of view without prejudging or discouraging them. An effective leader considers others' opinions with an open mind because a wider variety of solutions to problems is offered. Openness by the nurse leader encourages creative solutions by providing an environment in which people feel comfortable —thinking outside the box. Creativity is the ability to think differently. A creative leader examines all possible solutions to a problem even if at first glance they appear to be unrealistic or outside the norm. This ability allows the nurse leader to inspire followers to consider broader visions and goals.

DIF: Remembering

OBJ: 12.3

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
Concepts: Leadership

12. The nurse leader is conducting a staff meeting. During the meeting, staff members have verbalized dissatisfaction with the staffing pattern created by the nurse leader. The nurse listens intently as the staff come up with other options. The staff members recognize that the nurse leader is demonstrating which quality?
- Openness
 - Integrity
 - Dedication
 - Magnanimity

ANS: A

Openness refers to the leader's ability to listen to other points of view without prejudging or discouraging them. An effective leader considers others' opinions with an open mind because a wider variety of solutions to problems is offered. Openness by the nurse leader encourages creative solutions by providing an environment in which people feel comfortable —thinking outside the box. Integrity is the quality of having clear ethical principles and aligning one's actions with the stated values. Dedication is the ability to spend the time necessary to accomplish a task. Magnanimity means giving credit where credit is due. Good leaders reflect the work and success of accomplishing a goal by crediting those who helped reach it.

DIF: Remembering

OBJ: 12.3

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
Concepts: Leadership

13. The nurse is acting in the planning function as a manager. The nurse knows which stage should be completed first? **NURSINGTB.COM**
- Set the plan.
 - Assess the situation and future trends.
 - Convert plan into action statement.
 - Set the goals.

ANS: D

The planning function of a manager is comparable to the assessment, diagnosis, and planning portions of the nursing process. It includes four stages: (1) setting goals, (2) assessing the current situation and future trends, (3) setting the plan, and (4) converting the plan into an action statement.

DIF: Remembering

OBJ: 12.3

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
Concepts: Leadership

14. According to Fayol, controlling is a function of management. The nurse understands controlling compares to what phase of the nursing process?
- Evaluation
 - Diagnosis
 - Assessment
 - Implementation

ANS: A

The act of controlling involves comparing expected results of the planned work with the actual results. In the nursing process, evaluation is comparable to controlling. The planning function of a manager is comparable to the assessment, diagnosis, and planning portions of the nursing process.

DIF: Remembering

OBJ: 12.3

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Leadership

15. The nurse leader recognizes that to deliver quality care, focus needs to be placed on which participant?
- Patient
 - Self
 - Other staff members
 - Health care provider

ANS: A

It is important for nurse leaders to be focused on the patients rather than themselves to deliver good patient-focused care. Nurses must desire to improve the status quo to provide higher levels of quality in the care delivered. These qualities are also discussed in other works concerning effective managers (Delgado & Mitchell, 2016; Feather, Ebright, & Bakas, 2015).

DIF: Applying

OBJ: 12.3

TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Leadership

16. When explaining delegation to student nurses, what statement by the nurse educator aligns to the ANA regarding delegation?
- A transfer of authority to a less-qualified individual
 - The nurse transferring accountability to the delegate
 - The transfer of tasks by the nurse while retaining accountability
 - Transferring responsibility for assessments and planning

ANS: C

For patient care to be completed in a safe and timely manner, it is sometimes necessary for the nurse to delegate tasks to other health care providers. The National Council of State Boards of Nursing (NCSBN) offers support in this process. In their joint statement (ANA and NCSBN, 2005), the ANA describes delegation as the transfer of responsibility, and the NCSBN calls it a transfer of authority. This transfer gives a competent individual the authority to perform a selected nursing task in a selected situation. The nurse retains accountability for the delegation. Any significant findings during the care such as alterations in skin integrity, shortness of breath, or changes in a patient's condition should be reported to the nurse. The nurse is then responsible for assessing the alterations and addressing them in the plan of care.

DIF: Understanding

OBJ: 12.5

TOP: Evaluation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Leadership

17. The nurse manager would counsel the staff nurse for delegating which task to the UAP?
- Personal hygiene
 - Assistance with eating breakfast

- c. Assistance with toileting
- d. Interpretation of abnormal vital signs

ANS: D

The RN must remember to delegate tasks that do not require nursing judgment. Interpretation of abnormal vital signs requires assessment skills possessed by the RN only. Only tasks that are routine and do not require variation from a standardized procedure, such as providing hygiene, assisting with eating, and toileting, should be delegated.

DIF: Evaluating OBJ: 12.5 TOP: Evaluation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT: Concepts: Leadership

18. Which delegation of tasks would require the nurse manager to intervene?
- a. The UAP re-delegates vital signs to the student nurse.
 - b. The RN delegates assistance with bathing to the student nurse.
 - c. The RN delegates monitoring of intake and output to the UAP.
 - d. The RN delegates assistance with mobility to the UAP.

ANS: A

The person to whom the assignment was delegated cannot delegate that assignment to someone else. If the person cannot carry out the assignment, the individual needs to notify the delegating RN so that the task may be reassigned or completed by the RN. The RN must remember to delegate tasks that do not require nursing judgment. Only tasks that are routine such as bathing, monitoring intake and output, and assisting with mobility, and do not require variation from a standardized procedure should be delegated.

DIF: Evaluating OBJ: 12.5 TOP: Evaluation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT: Concepts: Leadership

19. The nurse recognizes which leadership theory that assumes that leaders are born with certain leadership skill that few people possess?
- a. Trait theory
 - b. Behavioral theory
 - c. Situational theory
 - d. Transformational theory

ANS: A

Trait theories assume that leaders are born with the personality traits necessary for leadership, which few people are thought to possess. Behavioral theories assume that leaders learn certain behaviors. These theories focus on what leaders do, rather than on what characteristics they innately possess. Situational theories suggest that leaders change their approach depending on the situation. Transformational leaders use methods that inspire people to follow their lead. Transformational leaders work toward transforming an organization with the help of others.

DIF: Remembering OBJ: 12.3 TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT: Concepts: Leadership

20. The nurse who plans, organizes, delivers, and evaluates nursing care for patients is functioning in what role?

- a. Patient care provider
- b. Patient advocate
- c. Case manager
- d. Clinical nurse leader

ANS: A

A nurse does not have to be a manager to be a leader. Even at the bedside, nurses use leadership skills, although possibly in different ways than a nurse manager. The patient care provider must be able to plan, organize, deliver, and evaluate nursing care for patients. An advocate is someone who supports and promotes the interests of others. The RN acts as a patient advocate during treatment. Although many health care organizations have case managers to aid in moving the patient through the health care system, the bedside nurse also acts as a case manager. One important way a nurse can do this is by beginning discharge

planning on admission. The clinical nurse leader (CNL) has a master's degree and certification from the American Association of Colleges of Nursing (AACN) Commission on Nurse Certification.

DIF: Remembering

OBJ: 12.4

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Leadership

MULTIPLE RESPONSE

1. The nurse recognizes which skills that are needed to be an effective manager? (*Select all that apply.*)
 - a. Understand the concepts of budgeting.
 - b. Run a unit efficiently without regard to cost.
 - c. Be able to staff the unit effectively.
 - d. Be adept at information management.
 - e. Achieve desired outcomes in any way possible.

ANS: A, C, D

An effective manager must have business skills and a business sense. Part of quality care is ensuring that the care the patient receives is cost effective. The nurse manager must understand concepts of budgeting, staffing, marketing, and information management. An understanding of human resource management is equally important. The skillful nurse manager understands the way these elements interact and their influence in achieving expected outcomes in an economically responsible manner.

DIF: Understanding

OBJ: 12.4

TOP: Evaluation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Leadership

2. When assigning tasks to other health care providers, the nurse understands that each task must be delegated using which guidelines? (*Select all that apply.*)
 - a. The task must be within the scope of the person to whom it is being delegated.
 - b. The task is one that can be delegated to other health care providers.
 - c. The task can be delegated whenever assessments are required.
 - d. The task may be re-delegated by the person to whom it was first delegated.
 - e. The task may require the nurse to procure resources to complete the task.

ANS: A, B, E

Through quality improvement, the nurse appreciates the value of what each team member can do to improve patient care. When delegating to other health care providers, the nurse understands that the task must be within the scope and abilities of the person to whom it is being delegated. The nurse must know if the task is something that can be delegated. The RN is responsible for assessment of patients even if certain tasks are delegated to others. The person to whom the assignment was delegated cannot delegate that assignment to someone else. Adequate resources must be made available to the delegatee to complete the task.

DIF: Understanding

OBJ: 12.5

TOP: Evaluation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Leadership

3. The nurse has a question regarding scope of practice and delegation. Where should the nurse seek clarification? (*Select all that apply.*)
- a. The state's nurse practice act
 - b. Theory X management
 - c. Nurse's Code of Ethics
 - d. The NCSBN website
 - e. NCSBN journal articles

ANS: A, D, E

Nurses must have knowledge of the nurse practice act in the state where they are licensed. Each state's nurse practice act defines the RN scope of practice and discusses appropriate delegation. A second resource in delegation is the use of the organization's policy and procedure manual. Employers must have job descriptions for each job class that outline the responsibilities and limitations of each position. The National Council of State Boards of

Nursing (NCSBN) website and journal articles are other resources for understanding delegation. Nurses are expected to follow personal and professional ethics, as outlined in the American Nurses Association (ANA) Code of Ethics for Nurses to maintain integrity. Theory X—style managers believe that the average person dislikes work and will avoid it if given the opportunity to do so.

DIF: Applying

OBJ: 12.5

TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Leadership

4. The nurse identifies what decisional roles that are included in Mintzberg's description of management in terms of behavior? (*Select all that apply.*)
- a. Figurehead
 - b. Spokesperson
 - c. Entrepreneur
 - d. Resource allocator
 - e. Negotiator

ANS: C, D, E

Mintzberg described management in terms of behaviors. Underlying his descriptions were two assumptions: much of a manager's time is spent in human relations, and managers are more reactive than proactive. These assumptions provided the basis for three categories of behaviors: interpersonal roles, informational roles, and decisional roles. Mintzberg described three interpersonal roles: figurehead, leader, and liaison. The three informational roles he described are monitor, disseminator, and spokesperson. The third category of Mintzberg's behavioral roles comprises the four decisional roles: entrepreneur, disturbance handler, resource allocator, and negotiator.

DIF: Remembering

OBJ: 12.4

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Leadership

NURSINGTB.COM

Chapter 13: Evidence-Based Practice and Nursing Research**Yoost & Crawford: Fundamentals of Nursing: Active Learning for Collaborative Practice, 3rd Edition****MULTIPLE CHOICE**

1. The nurse knows testing the application of theories in different situations with different populations is what type of research?
 - a. Applied research
 - b. Clinical research
 - c. Basic research
 - d. Quantitative research

ANS: A

Research conducted to generate theories is basic research. These theories help to provide explanations for phenomena. Testing theories in different situations with different populations is applied research. Clinical research is conducted to test theories about the effectiveness of interventions. Each type of research contributes to the theoretical base for the practice of nursing. Quantitative research usually produces data in the form of numbers.

DIF: Remembering

OBJ: 13.1

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT: Concepts: Technology and Informatics

2. The American Nurses Association (ANA) standards of professional performance require nurses to use research findings in practice. How do these standards impact nurses in the workplace?
 - a. Nurses need to regulate their practice according to the latest journal articles.
 - b. Nurses need to use the best available evidence to guide practice decisions.
 - c. Nurses only need to participate in research while in advanced practice.
 - d. Nurses may use evidence-based practice to develop procedures but not policies.

ANS: B

The American Nurses Association (ANA) standards of professional performance require nurses to use research findings in practice. Two criteria are measured. The first criterion is that nurses need to use the best available evidence, which includes research findings, to guide their practice decisions (ANA, 2015). The second criterion is that nurses participate in research activities that are appropriate for their position and level of education. Activities may include identifying problems in the clinical setting that may be researched; participating in data collection; participating as a member of a research committee or a research program; sharing research findings the nurse has found with others; conducting research; critiquing research that may be used in practice; using research findings to develop policies, procedures, and standards for patient care at health care facilities; and incorporating research as part of ongoing learning as a nurse. Nurses may participate in one or more of these activities during their careers. The ANA (2017) Research Toolkit was developed to help nurses to provide evidence-based care that improves patient outcomes. The ANA (2017) Research Toolkit was developed to help nurses to provide evidence-based care that improves patient outcomes.

DIF: Understanding

OBJ: 13.1

TOP: Assessment

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance

NOT: Concepts: Technology and Informatics

3. The nurse is reviewing a research study that includes data in the form of numbers and recognizes that this is likely to be what type of study?
- Qualitative
 - Experimental
 - Quasi-experimental
 - Quantitative

ANS: D

Quantitative research usually produces data in the form of numbers. Experimental research explores the causal relationships between variables. Experimental research examines whether one variable has a cause-and-effect relationship with another. Quasi-experimental research examines a causal relationship between variables, but it may not meet the strict guidelines of experimental research. Qualitative research is based on a constructivist philosophy, which assumes that reality is composed of multiple socially constructed realities of each person or group and is therefore value laden, focusing on personal beliefs, thoughts, and feelings.

DIF: Remembering

OBJ: 13.2

TOP: Assessment

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance

NOT: Concepts: Technology and Informatics

4. In practice, the nurse has identified an observable phenomenon and wants to conduct research to generate a hypothesis through observation of the situation. The nurse knows what approach to be the best way for the nurse to conduct this type of investigation?
- Correlational research study
 - Experimental research study
 - Descriptive research study
 - Quasi-experimental research study

ANS: C

Descriptive research identifies data and characteristics about the population or phenomenon. Correlational research is used to explore a relationship between two variables. Experimental research explores the causal relationships between variables. Experimental research examines whether one variable has a cause-and-effect relationship with another. Quasi-experimental research examines a causal relationship between variables, but it may not meet the strict guidelines of experimental research.

DIF: Applying

OBJ: 13.2

TOP: Implementation

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance

NOT: Concepts: Technology and Informatics

5. The nurse recognizes that when conducting a qualitative research study, what concept is a basis of this type of research?
- Qualitative research is based on a constructivist philosophy.
 - Qualitative research assumes that reality is the same for everyone.
 - Qualitative research is deductive in nature and approach.
 - Qualitative research proceeds from specific facts to generalizations.

ANS: A

Qualitative research is based on a constructivist philosophy, which assumes that reality is composed of multiple socially constructed realities of each person or group and is therefore value laden, focusing on personal beliefs, thoughts, and feelings. Constructivism assumes an approach that is inductive (Creswell, 2014). Inductive reasoning generalizes from specific facts. Qualitative research usually results in data expressed in words, often in the form of a narrative.

DIF: Understanding

OBJ: 13.2

TOP: Diagnosis

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance

NOT: Concepts: Technology and Informatics

6. When the nurse is conducting a quantitative research study, what concept is implemented?
- Quantitative research assumes that reality is fixed and stable.
 - Quantitative research is based on an inductive approach.
 - Quantitative research seeks to gain knowledge through observation.
 - Quantitative research usually produces data in narrative format.

ANS: A

Quantitative research is based on a postpositivist philosophy, which assumes that reality is objective, fixed, stable, observable, measurable, and value free. Positivism assumes that the approach is deductive in nature, and it seeks to gain knowledge through scientific and experimental research. Quantitative research usually produces data in the form of numbers.

DIF: Understanding

OBJ: 13.2

TOP: Diagnosis

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance

NOT: Concepts: Technology and Informatics

7. The nurse is preparing to conduct a research study and is interested in exploring the lived experiences of nurses responsible for approaching patients and family members about the donation of organs. Which type of research would this be considered?
- Grounded theory
 - Ethnography
 - Historical
 - Phenomenologic

ANS: D

Phenomenologic research explores the reactions of a specific group of people who experienced a similar event in their lives. Grounded theory research derives theories from the data collected in studies. Ethnography focuses on the sociology of meaning through close field observation of a sociocultural phenomenon. The term *ethnography* is sometimes applied to the field notes or case studies produced from ethnographic research. Historical research studies historical documents to determine an accurate picture of a past event or time period.

DIF: Understanding

OBJ: 13.2

TOP: Diagnosis

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance

NOT: Concepts: Technology and Informatics

8. The nurse has identified a research problem and knows what to be. What is the next step with this problem?
- Conduct a literature review.
 - Address ethical procedures.

- c. Collect data.
- d. Analyze data.

ANS: A

The research process involves many different components. The literature review is conducted after a research problem is identified. Ethical procedures must be addressed before the study begins. Data are then collected and analyzed before discussion of the research results.

DIF: Understanding OBJ: 13.3 TOP: Diagnosis
 MSC: NCLEX Client Needs Category: Health Promotion and Maintenance
 NOT: Concepts: Technology and Informatics

9. The nurse is conducting a literature review to determine the statistical results of all related studies and identifies this to be what type of review?
- a. A meta-analysis
 - b. An integrative literature review
 - c. A systematic review
 - d. Grounded theory research

ANS: A

Literature reviews include scholarly analyses of research. A meta-analysis merges statistical results from related studies to discover similarities and differences in their findings. An integrative literature review synthesizes research findings and formulates ideas about future research. A systematic review of the literature provides a comprehensive, unbiased analysis using a strict scientific design to select and assess each of the studies. Grounded theory research derives theories from the data collected in studies.

DIF: Remembering OBJ: 13.3 TOP: Assessment
 MSC: NCLEX Client Needs Category: Health Promotion and Maintenance
 NOT: Concepts: Technology and Informatics

10. In researching the effectiveness of an antihypertensive medication, the nurse knows that the medication would be what type of variable?
- a. Dependent
 - b. Independent
 - c. Treatment
 - d. Controlled

ANS: B

In experimental research, the independent variable is referred to as an *experimental variable* or *treatment variable*. An independent variable is a concept or idea whose value determines the value of other (dependent) variables. In research, the independent variable comprises the experimental treatment or intervention, and it is manipulated by the researcher to yield various outcomes. The dependent variable is the outcome that is affected by manipulation of the independent variable. For example, in researching the effectiveness of an antihypertensive medication, the medication is the independent variable and the person's blood pressure is the dependent variable. In a controlled study, some of the participants are assigned to the treatment group, and others are assigned to the control group by a random process. The control group does not receive the treatment. In the clinical trial of a medication, the control group receives a placebo. The purpose of a control group is to prevent bias and ensure that the outcome results from the treatment rather than some other factor.

DIF: Remembering

OBJ: 13.3

TOP: Assessment

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance

NOT: Concepts: Technology and Informatics

11. While conducting a controlled research study, the nurse wants greater assurance that the result is due to treatment itself and not another factor. For this purpose, the researcher should include what other component?
- A treatment group
 - An independent variable
 - A dependent variable
 - A control group

ANS: D

In a controlled study, some of the participants are assigned to the treatment group, and others are assigned to the control group by a random process. The control group does not receive the treatment. In the clinical trial of a medication, the control group receives a placebo. The purpose of a control group is to prevent bias and ensure that the outcome results from the treatment rather than some other factor. An independent variable is a concept or idea whose value determines the value of other (dependent) variables. In research, the independent variable comprises the experimental treatment or intervention, and it is manipulated by the researcher to yield various outcomes. The dependent variable is the outcome that is affected by manipulation of the independent variable.

DIF: Applying

OBJ: 13.3

TOP: Implementation

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:

Concepts: Technology and Informatics

NURSINGTB.COM

12. The nurse understands an institutional review board (IRB) is a review committee established to carry out what task?
- Approve research involving animal subjects.
 - Approve research that is not government funded.
 - Function differently than scholarly journals do.
 - Protect the rights of human research subjects.

ANS: D

An institutional review board (IRB) is a review committee established to help protect the rights and welfare of human research subjects. Regulations require IRB review and approval for research involving human subjects if it is funded or regulated by the federal government. Most research institutions, professional organizations, and scholarly journals apply the same requirements to all human research. The IRB must approve the research and procedure for data collection from human subjects.

DIF: Understanding

OBJ: 13.3

TOP: Assessment

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance

NOT: Concepts: Technology and Informatics

13. The nurse is preparing to conduct a study involving the —postprandial|| blood sugars in patients who have received intensive diabetic rehabilitation versus diabetics undergoing —usual care.|| For the consent to be valid, the nurse would have to carry out which action?
- Change the language of the consent.

- b. Keep explanations to a minimum to reduce stress.
- c. Keep potential risks undisclosed.
- d. Insist that the participant sign the consent right away.

ANS: A

Research participants require an explanation of the study in which they are subjects. Any information provided needs to be in a language that is understandable to them. Procedures and the purpose of the study need to be explained. The way subject anonymity and confidentiality will be protected needs to be explained. Any potential harm, including physical or mental discomfort, and possible benefits from participation should be explained. Questions should be answered so that participants fully understand the research and their part in the process. All subjects need to be given time to decide about participation. Study participants are voluntary, may withdraw at any time, or may choose not to complete tasks.

DIF: Applying OBJ: 13.3 TOP: Implementation
 MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:
 Concepts: Technology and Informatics

14. The nurse researcher audiotaped interviews with subjects and would like to play these tapes during dissemination. The nurse identifies what steps that may be required to play the tapes?
- a. Inform the participants that they cannot hear the tapes beforehand.
 - b. None, if the tape is of a group, since there is no expectation of anonymity.
 - c. None, since the tape is a direct —quote and voice recognition is not controllable.
 - d. A release will need to be obtained from the subjects.

ANS: D

Any videos, photos, or audiotapes require releases if they are to be shown in the dissemination of research findings. The participants have the right to review these tapes before allowing them to be used for research. If subjects are involved in a group, they should be reminded that their exchange of information and identities must remain confidential. Participants' permission is needed if the data include quotations or can reveal the subject's identity.

DIF: Applying OBJ: 13.3 TOP: Implementation
 MSC: NCLEX Client Needs Category: Psychosocial Integrity NOT:
 Concepts: Technology and Informatics

15. The nurse researcher understands a human subject is defined as a living individual about whom an investigator conducting research obtains what information?
- a. Data without direct or indirect interaction or intervention
 - b. Information that is not expected to be made public
 - c. No diagnostic information and does not manipulate the subject environment
 - d. Information without any communication/contact during the research

ANS: B

A human subject is defined as—a living individual about whom an investigator conducting research obtains: (1) data through intervention or interaction with the individual or (2) identifiable private information (Office of Human Research Protection (OHRP), 2016). *Interventions* may include procedures such as gathering diagnostic information or manipulating the subject's environment. *Interaction* refers to any communication or contact during the research. *Private information* includes anything not expected to be made public, such as a medical record.

DIF: Understanding

OBJ: 13.3

TOP: Evaluation

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Technology and Informatics

16. The nurse is ready to analyze the data obtained through a qualitative study. What approach to data analysis should the nurse use?
- Content analysis
 - Statistical analysis
 - Coding of themes
 - Dissemination

ANS: A

Data analysis techniques are procedures used to summarize words or numbers and create a meaningful result for interpretation. Qualitative analysis involves content analysis. The qualitative data may contain quotations and require their interpretation. Quotations from study participants support the evidence that is provided by the study. Quantitative analysis involves statistical analysis. Many types of statistical analyses may be performed on the data, and the appropriate technique needs to be applied. This process requires coding of themes and analysis of the narrative content. Dissemination is communication and distribution of the information.

DIF: Remembering

OBJ: 13.3

TOP: Assessment

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Technology and Informatics

17. The nurse correctly devises a dissemination plan at what point during the research process?
- Conclusion of the study
 - After the literature review
 - The beginning of the research process
 - While conducting research

ANS: C

A dissemination plan should be devised at the beginning of the research.

DIF: Understanding

OBJ: 13.3

TOP: Assessment

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Technology and Informatics

18. When applying research to practice, the nurse finds what information?
- It is usually easy to access information at the bedside.
 - Research articles are clear in defining nursing practice.
 - Bedside care is not directly related to research.
 - Nursing research should be used to improve care.

ANS: D

One obstacle to applying research to practice is the difficulty in bedside access to information by nurses. Nurses often lack the time to participate in research-related activities. By reading research articles, the nurse may notice discrepancies in what is recommended in current practice and what is found in the literature. Nurses often feel that their bedside nursing care is removed from the research process. However, nurses participate every day in the care of patients, which is based on the nursing process. Nurses should use research to improve the quality of patient care and should understand the research base before initiating nursing interventions.

DIF: Understanding

OBJ: 13.3

TOP: Assessment

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Technology and Informatics

19. The acronym PICO assists in remembering the steps to constructing a good research question and the nurse identifies that the —O— in the acronym refers to what term?
- Objectivity
 - Ordinal approach
 - Outcome
 - Observer

ANS: C

One method of formulating a research question is identified by the acronym PICO (i.e., patient, population, or problem; intervention; comparison intervention; and outcomes).

DIF: Remembering

OBJ: 13.5

TOP: Assessment

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Technology and Informatics

NURSINGTB.COM

20. The nurse knows that the third phase of evidence-based research involves what action?
- Searching for evidence and evaluating
 - Assessing the problem
 - Developing a question
 - Performing a critical appraisal

ANS: A

Searching the database for the evidence begins the third phase. The nurse may need to consult three categories of information resources, which are reviewed in sequential order depending on need and applicability. The categories are general information (background) resources, filtered resources, and unfiltered resources. The second phase of evidence-based research consists of formulating a specific research question so that the nurse can effectively search the literature databases. The first phase of evidence-based research consists of assessing the need for change in practice by identifying a problem. After identifying an article or systematic review resource that seems appropriate to the question, the nurse must critically appraise the information.

DIF: Remembering

OBJ: 13.5

TOP: Assessment

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Technology and Informatics

21. If the nurse is trying to determine the best treatment or course of action and wants to incorporate the most reliable evidence into the decision, the nurse will use what filtered resource?
- Cochrane Reviews
 - UpToDate
 - STAT!Ref
 - MD Consult

ANS: A

When trying to determine the best treatment or course of action and wanting to incorporate the most reliable evidence into the decision, the nurse can use a filtered resource such as the Cochrane Reviews or the Joanna Briggs Institute Library of Systematic Reviews. The filtered resource provides the best available evidence. In filtered resources, clinical and subject experts have asked a question and then synthesized evidence to establish conclusions based on the research. This pre-evaluation process is already completed for nurses and allows the resources to be used while caring for patients. The conclusions from filtered resources still need to be evaluated by clinicians in terms of a specific patient. Filtered resources produce systematic reviews of the literature. Nurses may encounter conditions outside their specialty area and need an overview. Background resources provide detailed information. If the nurse is looking for a presentation of information or types of therapies, the best source is a background resource. Background resources include UpToDate, STAT!Ref, and MD Consult, which are web-based databases. Another source of background information is a current nursing textbook.

DIF: Remembering

OBJ: 13.5

TOP: Assessment

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Technology and Informatics

N R I G B C M
USNT O

MULTIPLE RESPONSE

1. The nurse understands that Florence Nightingale is noted to have provided the initial basis for evidence-based practice (EBP) by doing which action? (*Select all that apply.*)
- Basing her work in trial and error as well as observation
 - Using statistical data as a basis for improvements
 - Applying statistical methods such as —pie charting to display results
 - Focusing on bedside care and ignoring nursing education
 - Publishing the first EBP journal

ANS: A, B, C

Florence Nightingale, in her *Notes on Nursing* in 1859, outlined basic principles of nursing science. Nightingale's method of nursing included rigorous monitoring of the effectiveness of interventions and treatments. This provided the initial basis for EBP. Her work was based on trial and error, careful observation, discussion with patients, and clinical experience. She used statistical data to improve sanitation, health, nursing education, and health administration. Nightingale applied a statistical approach to the study of public health and mortality data and used a pie chart to display research findings. However, nursing did not publish its first EBP journal, *Evidence-Based Nursing*, until 1998.

DIF: Remembering

OBJ: 13.4

TOP: Assessment

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Technology and Informatics

2. The nurse recognizes barriers to the use of evidence-based practice (EBP) include what points? (*Select all that apply.*)
- a. Nurses critiquing research
 - b. Difficulty communicating how to conduct EBP
 - c. Copious amount of literature available
 - d. Short time between research and practice
 - e. Reluctance of organizations to fund research

ANS: B, C, E

To adequately integrate EBP into patient care, nurses must critique research to differentiate between opinion and evidence and must regularly read current professional journals. Some barriers are common to research use and EBP, including the difficulty of communicating how to conduct EBP and the individual nurse's skills in determining the quality of research available for review. Another limitation is the reluctance of organizations to fund research and subsequently make potentially costly practice changes based on the best evidence. Because of the copious amount of literature on a specific topic, it is difficult to analyze the literature in an efficient and effective manner. Health care literature with clinically applicable findings is published at a rate that is impossible for individual health care professionals to keep up with. There are delays of approximately 17 years for implementation of clinical research into practice.

DIF: Understanding

OBJ: 13.4

TOP: Assessment

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Technology and Informatics

3. Nurses use new information in their practice. In the process of implementing EBP, the nurse carries out which actions? (*Select all that apply.*)
- a. Develops clinical questions.
 - b. Creates workshops and in-services.
 - c. Seeks answers to support the clinical decision.
 - d. Applies finding to patients.
 - e. Publishes a bulletin.

ANS: A, C, D

Workshops and in-service or focused training may be necessary if the plan involves a comprehensive change in care or it affects the entire health care agency or community. A bulletin can be provided that lists several safe practice concerns with rationales in the form of a safe practice alert. The fifth phase in the EBP process requires implementation of the change by applying the evidence. Nurses use the new information in their practice. In the process of implementing EBP, the nurse develops a clinical question, seeks answers to verify and support a clinical decision, and ultimately applies the findings to patients.

DIF: Applying

OBJ: 13.6

TOP: Implementation

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance

NOT: Concepts: Caregiving

4. The nurse manager knows a Magnet hospital is characterized by which? (*Select all that apply.*)
- a. Excellent medical outcomes

- b. A high level of nursing job satisfaction
- c. A low number of grievances
- d. Nursing care leading excellent patient outcomes
- e. Evidence-based environment support

ANS: B, D, E

A Magnet hospital is characterized by excellent patient outcomes resulting from nursing, a high level of nursing job satisfaction with a low nurse turnover rate, and appropriate resolution of any grievances. The Magnet Recognition Program supports an evidence-based environment, which includes the nurses' autonomy to improve quality of care through research utilization (ANCC, 2018). Research and EBP must, therefore, become a part of the nurses' care of the patients.

DIF: Understanding

OBJ: 13.7

TOP: Assessment

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance

NOT: Concepts: Caregiving

NURSINGTB.COM

Chapter 14: Health Literacy and Patient Education**Yoost & Crawford: Fundamentals of Nursing: Active Learning for Collaborative Practice, 3rd Edition**

MULTIPLE CHOICE

1. The nurse understands the unique ability of the patient to understand and integrate health-related knowledge is known by which term?
 - a. Health literacy
 - b. Formal patient education
 - c. Informal patient education
 - d. Primary education

ANS: A

The unique ability of a patient to understand and integrate health-related knowledge is known as health literacy. Formal patient education is delivered throughout the community in the form of media, in a variety of educational and group settings, or in a planned, goal-directed, one-on-one session with a patient in the acute care setting. Informal education is usually learner or patient directed. Many health care consumers begin receiving information as children through their primary education. Handwashing, proper dental care, and nutrition are examples of early instructions.

DIF: Remembering

OBJ: 14.1

TOP: Assessment

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance

NOT: Concepts: Patient Education

- NursingTB.COM
2. The patient is reportedly well educated and employed as an engineer but is struggling to comprehend terms found in health-related literature given to explain his disease process. The nurse recognizes that this is evidence of what issue?
 - a. Low literacy
 - b. Psychomotor dysfunction
 - c. Affective domain deficiency
 - d. Low health literacy

ANS: D

Although *low literacy* and *low health literacy* are related terms, they are not interchangeable. Low health literacy is content specific, meaning that the individual may not have difficulty reading and writing outside the health care arena. These patients may struggle to comprehend the complicated, unfamiliar terms and ideas found in health-related materials or instructions. The psychomotor domain incorporates physical movement and the use of motor skills in learning. Teaching the newly diagnosed diabetic how to check blood sugar is an example of a psychomotor skill. Affective domain learning recognizes the emotional component of integrating new knowledge. Successful education in this domain takes into account the patient's feelings, values, motivations, and attitudes.

DIF: Remembering

OBJ: 14.1

TOP: Assessment

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance

NOT: Concepts: Patient Education

3. To teach effectively, nurses must recognize which concept?

- a. Age and socioeconomic status play a large role in understanding.
- b. 90% of Americans possess rudimentary literary skills.
- c. The ability to comprehend is a very new concept in health care.
- d. Most health care teaching is effective and understood.

ANS: B

To teach effectively, nurses must recognize that patients of all ages come from diverse cultural and socioeconomic backgrounds. Each has a different ability to comprehend health care information. Results of the NAAL research indicate that among American adults, 30 million (14%) had below basic health literacy in English and 47 million (22%) had basic health literacy. This means that 77 million (36%) American adults possessed very rudimentary literacy skills that allowed them to read only short, simple printed and written materials.

Although discussion of Nightingale's work often focuses on her efforts to distinguish nursing as a profession and address the impact of sanitation on health, she advocated exploring all aspects of the patient. She thought that patients needed care that is —delicate and decent¹ and that demonstrates —the power of giving real interests to the patient.¹ Exploring patients' interests and abilities was an early acknowledgment that nurses must be aware of patients' ability to comprehend the health care information provided. Often, health care professionals assume that the explanations and instructions given to patients and families are readily understood. In reality, research has shown that these instructions are frequently misunderstood, sometimes resulting in serious errors.

DIF: Understanding

OBJ: 14.2

TOP: Assessment

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance

NOT: Concepts: Patient Education

4. The nurse understands that as the health care community explores the concept of health literacy, many organizations recognize what concept?
- a. Consumers need to understand has no governmental support.
 - b. Improvements are dependent on developing operational definitions.
 - c. *Low literacy* and *low health literacy* are interchangeable terms.
 - d. Interest in effective patient education is unique to the United States.

ANS: B

As the health care community explores the concept of health literacy, many organizations recognize that before improvements can be made, operational definitions are imperative. The realization that consumers need to be able to understand the medical information delivered by health care providers has gained recognition at many governmental levels. The *Healthy People 2020* publication describes a national movement that addresses the priorities of prevention and public health in the United States. Health literacy with its impact on this initiative is being recognized and has become a key component of the project. Although *low literacy* and *low health literacy* are related terms, they are not interchangeable. Low health literacy is content-specific, meaning that the individual may not have difficulty reading and writing outside the health care arena. Interest in effective patient education is not a phenomenon unique to the United States. The Institute of Medicine Roundtable on Health Literacy held a workshop in 2012 focused on international health literacy.

DIF: Understanding

OBJ: 14.2

TOP: Assessment

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance

NOT: Concepts: Patient Education

5. The nurse is preparing to discharge a patient home. In providing instruction about the patient's medications, the nurse should make which statement?
- Before taking Metoprolol, you need to take your BP and rate.¶
 - MS should be taken only when needed for pain.¶
 - Take 1 baby aspirin by mouth every morning.¶
 - Take your water pill bid and you should be fine.¶

ANS: C

Do not use abbreviations or medical terminology when providing patients with instructions.

DIF: Applying OBJ: 14.2 TOP: Implementation
 MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:
 Concepts: Patient Education

6. The nurse has established a teaching plan including goals and identifies this type of education is termed by what term?
- Formal teaching
 - Informal teaching
 - Psychomotor teaching
 - Affective teaching

ANS: A

Formal patient education is delivered throughout the community in the form of media, in a variety of educational and group settings, or in a planned, goal-directed, one-on-one session with a patient in the acute care setting. Informal education is usually learner or patient directed. The psychomotor domain incorporates physical movement and the use of motor skills in learning. Teaching the newly diagnosed diabetic how to check blood sugar is an example of a psychomotor

skill. Affective domain learning recognizes the emotional component of integrating new knowledge. Successful education in this domain takes into account the patient's feelings, values, motivations, and attitudes.

DIF: Remembering OBJ: 14.3 TOP: Assessment
 MSC: NCLEX Client Needs Category: Health Promotion and Maintenance
 NOT: Concepts: Patient Education

7. The nurse is admitting a patient who has cystic fibrosis. During the admission interview, it is apparent that the patient is well versed in most aspects of his illness. When asked about where he learned so much, the patient responds, —I learned most of it myself. I looked things up on the Internet and read books. You have to know what's wrong with you to be sure that you're being treated right.¶ The nurse knows this is an example of what type of education/learning?
- Formal education
 - Psychomotor learning
 - Informal education
 - Affective learning

ANS: C

Informal education is usually learner or patient directed. Formal patient education is delivered throughout the community in the form of media, in a variety of educational and group settings, or in a planned, goal-directed, one-on-one session with a patient in the acute care setting. The psychomotor domain incorporates physical movement and the use of motor skills in learning. Teaching the newly diagnosed diabetic how to check blood sugar is an example of a psychomotor skill. Affective domain learning recognizes the emotional component of integrating new knowledge. Successful education in this domain takes into account the patient's feelings, values, motivations, and attitudes.

DIF: Understanding

OBJ: 14.3

TOP: Assessment

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance

NOT: Concepts: Patient Education

8. During patient teaching led by the nurse with goals established through cooperation of the nurse and patient, the patient asks questions as needed and the nurse answers. The nurse understands that this is what type of teaching?
- Formal teaching
 - Informal teaching
 - Both formal and informal teaching
 - Psychomotor teaching

ANS: C

Some patient education sessions have formal and informal elements, because the nurse and patient may set goals together before the nurse formulates and implements the plan of care, and the patient is free to ask questions that may direct the session. The health care information is considered informal because it is situation and patient specific. Formal patient education is delivered throughout the community in the form of media, in a variety of educational and group settings, or in a planned, goal-directed, one-on-one session with a patient in the acute care setting. Informal education is usually learner or patient directed. The psychomotor domain incorporates physical movement and the use of motor skills in learning. Teaching the newly diagnosed diabetic how to check blood sugar is an example of a psychomotor skill.

DIF: Understanding

OBJ: 14.3

TOP: Assessment

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance

NOT: Concepts: Patient Education

9. The nurse is implementing a patient teaching plan regarding diabetes mellitus. One of the short-term goals of the plan is that the patient will be able to verbalize three symptoms of hypoglycemia. The nurse recognizes that this is what type of teaching?
- Psychomotor teaching
 - Cognitive teaching
 - Affective teaching
 - VARK teaching

ANS: B

Learners in the cognitive domain integrate new knowledge through first learning and then recalling the information. They then categorize and evaluate, making comparisons with previous knowledge that result in conclusions related to the new content. The psychomotor domain incorporates physical movement and the use of motor skills in learning. Teaching the newly diagnosed diabetic how to check blood sugar is an example of a psychomotor skill.

Affective domain learning recognizes the emotional component of integrating new knowledge. Successful education in this domain takes into account the patient's feelings, values, motivations, and attitudes. Tools have been developed to help health care workers evaluate the health literacy of their patients. One such tool is the VARK (verbal, aural, read/write, kinesthetic) assessment of learning styles of people who are having difficulty learning.

DIF: Remembering

OBJ: 14.4

TOP: Assessment

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance

NOT: Concepts: Patient Education

10. The nurse is working with a diabetic patient and is attempting to teach psychomotor skills. This is occurring when the nurse has the patient complete what action?
- Verbally describe his feelings about diabetes.
 - Answer three of five true-or-false questions about diabetes.
 - Identify three positive lifestyle changes to manage blood sugar.
 - Draw up and self-inject insulin correctly.

ANS: D

The psychomotor domain incorporates physical movement and the use of motor skills in learning. Teaching the newly diagnosed diabetic how to check blood sugar is an example of a psychomotor skill. Learners in the cognitive domain integrate new knowledge through first learning and then recalling the information. They then categorize and evaluate, making comparisons with previous knowledge that result in conclusions related to the new content. Affective domain learning recognizes the emotional component of integrating new knowledge. Successful education in this domain takes into account the patient's feelings, values, motivations, and attitudes.

DIF: Applying

OBJ: 14.4

TOP: Implementation

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:

Concepts: Patient Education

11. The nurse is preparing to teach a patient for the first time and needs to evaluate the health literacy of the patient. The nurse uses the VARK assessment to gather what information?
- Assess the learning styles of the patient.
 - Find the one method that the patient uses to learn.
 - Be sure that the patient is a unimodal learner.
 - Reduce the need for creating a collaborative learning plan.

ANS: A

Tools have been developed to help health care workers evaluate the health literacy of their patients. One such tool is the VARK (verbal, aural, read/write, kinesthetic) assessment of learning styles of people who are having difficulty learning. Individuals typically learn through more than one method. For example, a patient's VARK assessment may indicate learning through VAR or ARK. When the use of more than one style facilitates learning, the individual is considered a multimodal learner, meaning that the person does best when more than one teaching strategy is used or that the person is able to adapt to a variety of teaching strategies on the basis of what is being presented. Understanding how patients learn best makes collaborative learning plans most effective. It is good practice to provide multiple means of learning, because most individuals learn through more than one style and repetition enhances learning.

DIF: Applying OBJ: 14.5 TOP: Implementation
 MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:
 Concepts: Patient Education

12. The nurse is preparing to teach a 90-year-old patient. In teaching an elderly patient, the nurse realizes what information?
- Most elderly patients are highly literate.
 - Cognitive abilities always decline with age.
 - Sensory alterations often occur with aging.
 - Teaching methods are the same as for the middle aged.

ANS: C

Teaching should be tailored to elderly patients. Reports indicate that two-thirds of U.S. adults 66 years old and older have inadequate or marginal literacy skills, and 81% of patients 60 years old and older at a public hospital could not read or understand basic materials such as prescription labels.

Although each patient must be assessed individually, cognitive and sensory alterations often occur with aging, and the teaching materials should be adjusted accordingly.

DIF: Understanding OBJ: 14.6 TOP: Assessment
 MSC: NCLEX Client Needs Category: Health Promotion and Maintenance
 NOT: Concepts: Patient Education

13. The nurse identifies which patient would most likely need to have adjustments made to the education plan for discharge because of role function?
- A 67-year-old married female who lives with her retired husband
 - A 32-year-old single mother of a toddler following hysterectomy
 - A 13-year-old who lives at home with his parents after appendectomy
 - A 50-year-old married mother with two children in college and teenager at home

ANS: B

Exploration of the patient's roles is an important task that must be done before development of a patient education plan. For example, a 32-year-old, single mother of five young children who has just undergone a hysterectomy may require a different perspective in her discharge instructions than that in the instructions of a 67-year-old female living with her husband who recently retired after 35 years as a family practice physician. The first patient may have less support and less flexibility regarding rest, lifting limitations, and cost of prescriptions than the second. It is important not to stereotype and assign roles but rather to develop a plan in collaboration with the individual. The patient's support system should be taken into consideration when the nurse plans patient education.

DIF: Evaluating OBJ: 14.6 TOP: Evaluation
 MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:
 Concepts: Patient Education

14. When the nurse is preparing to provide preoperative teaching to a deaf patient, what action by the nurse is best?
- Use printed materials.
 - Provide recorded materials.
 - Use a family member to interpret.
 - Provide an interpreter.

ANS: D

Patients who are deaf or have low English proficiency are entitled to professional interpretation by federal law. Printed material may be helpful but not if the patient has low literacy/low health literacy. Recorded material may be an option if the patient has some hearing and the recordings are amplified. Family members are not used as interpreters.

DIF: Applying OBJ: 14.7 TOP: Implementation
 MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:
 Concepts: Patient Education

15. When the nurse is preparing to teach a 5-year-old child postoperative care that will be anticipated after a tonsillectomy, the nurse would incorporate what concept?
- Use pictures and simple words to describe care to the patient.
 - Teach the parents alone to reduce fear in the patient.
 - Exclude the parents to reduce parental anxiety.
 - Use clear simple explanations to convey information.

ANS: A

Patient education provided for children should be age specific. Use pictures and simple words for young children. Use clear, simple explanations for school-age children. The patient's age directly affects the instructional methods and materials used. Effective patient education involving a child requires the presence of a parent or caregiver, who is likely the target of teaching. Children should not be excluded from the learning session unless exclusion is deemed appropriate by the parent or caregiver; a presentation using an age-appropriate strategy may complement the instructions reviewed with the adult. The stages of development should be explored as the foundation for the choice of educational materials.

DIF: Applying OBJ: 14.7 TOP: Implementation
 MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:
 Concepts: Patient Education

16. The nurse is preparing a teaching plan and is applying evidence-based practice. To promote involvement, the nurse must include which concept?
- Provide the latest professional literature to the patient.
 - Ensure that the patient understands relevant information.
 - Use only one teaching method to reduce confusion.
 - Not review previously learned information.

ANS: B

To promote involvement, nurses must ensure that patients understand the information relevant to their care. Nurses need to provide patients with easy-to-understand information and speak in a clear, distinct voice, using short sentences and understandable terminology. Multiple teaching methods should be used to meet the needs of all types of learners. Patient education sessions should be reassessed after two to three key points to ensure that the patient is still engaged in learning and ready to assimilate more information. Information taught at previous sessions can be reviewed before proceeding with new key points.

DIF: Applying OBJ: 14.7 TOP: Implementation
 MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:
 Concepts: Patient Education

17. In determining patient goals, the nurse should complete which action?
- Allow patients to identify what is most important to them.
 - Take the lead and determine what is best for the patient.
 - Focus on health promotion and staying healthy.
 - Explain the importance of avoiding complications.

ANS: A

As health care educators, nurses should allow patients to identify what is most important to them. If a newly diagnosed diabetic patient is interested in learning techniques of care that will allow discharge to home rather than to an extended care facility, the patient is more likely to be receptive to learning about self-monitoring blood sugar levels. After the learning goals related to the issues that the patient feels are a priority have been met, the patient may then be able to focus on health promotion and avoiding complications.

DIF: Applying OBJ: 14.9 TOP: Implementation
 MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:
 Concepts: Patient Education

18. The nurse understands ongoing evaluation of patient education occurs by which team member?
- Each member of the health care team who provides teaching
 - The nurse who evaluates the patient's physical abilities
 - The patient stating that he understands the instruction
 - Not allowing review from the provider so the focus remains forward

ANS: A

Ongoing evaluation of patient education occurs by each member of the health care team who provides teaching according to the patient's teaching plan. Having the learner repeat what has been learned can help the nurse evaluate the teaching plan and adjust the plan for future patient education sessions. Future sessions should review what was learned previously and continue to add to what has been taught. Health care team members can view documentation on the electronic health record (EHR) before beginning an education session to determine the patient's progress in meeting educational goals.

DIF: Understanding OBJ: 14.10 TOP: Evaluation
 MSC: NCLEX Client Needs Category: Health Promotion and Maintenance
 NOT: Concepts: Patient Education

MULTIPLE RESPONSE

1. In addressing patient education, the nurse recognizes that patient education is a process involving what components? (*Select all that apply.*)
 - a. Assessment
 - b. Diagnosis
 - c. Planning
 - d. Implementation and evaluation
 - e. Reliance on evidence-based practice (EBP)

ANS: A, B, C, D

Assessment of health literacy occurs with each patient encounter. On completion of assessment, a nursing diagnosis relevant to the educational needs of the patient or caregiver can be determined. After working with the patient or caregiver to determine the appropriate nursing diagnosis, the next step is developing the patient education plan. In all patient education situations, a return demonstration by the patient (i.e., repeating what has been taught) helps the nurse to assess the level of learning that has taken place. Although evidence-based practice is important, it is sometimes insufficient when making patient care decisions.

DIF: Understanding

OBJ: 14.2

TOP: Assessment

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance

NOT: Concepts: Patient Education

2. According to the *Healthy People 2020* initiative, health information and the associated access issues have become more complicated. There are many considerations when determining whether an individual has proficient health literacy. The nurse acknowledges that the patient should be able to do what actions? (*Select all that apply.*)
 - a. Read and identify credible health information.
 - b. Recognize abnormalities on an x-ray.
 - c. Navigate complex insurance programs.
 - d. Evaluate EKG findings.
 - e. Advocate for appropriate care.

ANS: A, C, E

The patient should be able to exhibit certain competencies such as reading and identifying credible health information, understanding numbers in the context of the patient's health care, making appointments, filling out forms, gathering health records and asking appropriate questions of physicians, advocating for appropriate care, navigating complex insurance programs (Medicare or Medicaid, and other financial assistance programs), and using technology to access information and services. Interpreting EKGs and X-rays is beyond this scope.

DIF: Applying

OBJ: 14.2

TOP: Implementation

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:

Concepts: Patient Education

3. In preparing to teach the patient, the nurse must consider which concepts? (*Select all that apply.*)
 - a. Background
 - b. Race
 - c. Pain level

- d. Emotional status
- e. Readiness to learn

ANS: A, C, D, E

Consideration must be given to the patient's background, readiness to learn, and current condition before education can occur. A patient's ability to read, write, and comprehend health care materials enhances health literacy. Race, by itself, is not a factor.

DIF: Applying OBJ: 14.3 TOP: Implementation
 MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:
 Concepts: Patient Education

4. The nurse is to teach an 84-year-old Spanish-speaking patient newly diagnosed with diabetes how to self-administer insulin. The patient has hearing and visual impairments. To be effective as a teacher, the nurse should carry out which tasks? (*Select all that apply.*)
- a. Assess reading level and learning style.
 - b. Determine readiness to learn.
 - c. Use family members as interpreters.
 - d. Provide written instruction in English.
 - e. Place the patient in group classes.

ANS: A, B

Before health care teaching sessions for adults, assess reading level, learning styles, and readiness to learn. Family members should not be used as interpreters of specific medical information to maintain the patient's right to privacy and to avoid possible misinterpretation of medical terminology. Access to interpretation or translation for deaf and limited English proficiency (LEP) patients is required by Title VI of the Civil Rights Act of 1964, which mandates equal rights for people regardless of race, color, or national origin. Use photos, drawings, or video to enhance understanding. A patient whose cultural beliefs and values are considered is more likely to demonstrate compliance. Patients with learning disabilities or cognitive alterations need individualized instruction geared to their special needs.

DIF: Applying OBJ: 14.6 TOP: Implementation
 MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:
 Concepts: Patient Education

5. When teaching children, the nurse should include which concepts? (*Select all that apply.*)
- a. Exclude the children from teaching.
 - b. Encourage parents or caregivers to be present.
 - c. Use age-specific strategies.
 - d. Consider the stages of development.
 - e. Remember that parents are not the targets of the teaching.

ANS: B, C, D

Patient education provided for children should be age specific. Effective patient education involving a child requires the presence of a parent or caregiver, who is likely the target of teaching. Children should not be excluded from the learning session unless exclusion is deemed appropriate by the parent or caregiver; a presentation using an age-appropriate strategy may complement the instructions reviewed with the adult. The stages of development should be explored as the foundation for the choice of educational materials.

DIF: Applying OBJ: 14.6 TOP: Implementation

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:
 Concepts: Patient Education

6. The nurse must provide patient education to a patient who has just been given the diagnosis of stage III cancer. The patient is complaining of chest and bone discomfort. Before providing the needed education, the nurse will complete which tasks? (*Select all that apply.*)
- Draw the curtain in the semi-private room.
 - Medicate the patient to ease the pain.
 - Place the patient in a private room if possible.
 - Wait until later in the day.
 - Attend to any other personal needs first.

ANS: B, C, D, E

The location of patient education influences the outcome. The setting should be quiet, and the session should have minimal interruptions. Providing privacy is difficult in settings such as emergency rooms, outpatient surgery centers, and semi-private inpatient rooms, but the nurse should make every effort to ensure confidentiality. Environmental considerations such as good lighting and the availability of resources should be explored to enhance the outcome of patient education. The nurse should examine the patient's situation and comfort level before beginning teaching. For example, a postoperative patient who is rating pain at 7 of 10 will be much more receptive to learning after being medicated for pain. A patient who just received a diagnosis of metastatic cancer will learn and assimilate more information later in the day or perhaps the next day. The nurse must also take care of any other personal needs first, such as the need to use the bathroom.

DIF: Applying OBJ: 14.7 TOP: Implementation
 MSC: NCLEX Client Needs Category: Health Promotion and Maintenance
 NOT: Concepts: Patient Education

7. On completion of assessment, a nursing diagnosis relevant to the educational needs of the patient or caregiver can be determined. The nurse recognizes that diagnoses specifically related to patient education include which responses? (*Select all that apply.*)
- Deficient knowledge
 - Readiness for enhanced knowledge
 - Noncompliance
 - Pain
 - Alteration in elimination

ANS: A, B, C

On completion of assessment, a nursing diagnosis relevant to the educational needs of the patient or caregiver can be determined. Diagnoses specifically related to patient education include deficient knowledge, readiness for enhanced knowledge, and noncompliance.

DIF: Remembering OBJ: 14.8 TOP: Assessment
 MSC: NCLEX Client Needs Category: Health Promotion and Maintenance
 NOT: Concepts: Patient Education

Chapter 15: Nursing Informatics**Yoost & Crawford: Fundamentals of Nursing: Active Learning for Collaborative Practice, 3rd Edition****MULTIPLE CHOICE**

1. The nurse knows the integration of nursing, computers, and information science for the management and communication of data, information, knowledge, and wisdom is identified by which term?
 - a. Nursing informatics
 - b. Computer science
 - c. Medical informatics
 - d. Informatics

ANS: A

Informatics is a broad academic field encompassing artificial intelligence, cognitive science, computer science, information science, and social science. Medical informatics refers to informatics related to health care and describes a distinct specialty in the discipline of medicine. Nursing informatics is a specialty area of informatics that addresses the use of health information systems to support nursing practice. The American Nurses Association (ANA, 2015) states that the specialty of nursing informatics integrates nursing computer and information science for the management and communication of data, information, knowledge, and wisdom.

DIF: Remembering

OBJ: 15.1

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Technology and Informatics

2. The hospital has recently implemented computer charting. The nurse knows the computerization of nursing practice has what impact?
 - a. It enhances and increases the time spent on documentation.
 - b. It makes patient data immediately available to the health care team.
 - c. It makes retrieval of data more difficult but safer.
 - d. It is enhanced by limiting the use of point-of-care technology.

ANS: B

Patient data collected by a nurse and recorded electronically are immediately available to all members of the health care team. The computerization of nursing practice data enables capture, storage, retrieval, organization, processing, and analysis of information. The information can be used to make a diagnosis, plan for care, provide nursing decision support, enhance documentation, and identify nursing care trends and costs. Systems that support data collection at the point of care can directly enhance patient care by decreasing the time spent on documentation, reducing the potential for errors, and supporting improved assessment and data communication. Computers, tablets, or pocket devices used at the bedside for documentation are examples of point-of-care technology.

DIF: Understanding

OBJ: 15.1

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Technology and Informatics

3. Nurses working surrounded by computers and mobile IT must develop skills in the use of all available technology. At the same time, it is important for nurses to recognize what fact?
- The technology in use today will be the same tomorrow.
 - Cell phones are not usually allowed in the acute care setting.
 - Most forms of mobile technology are in violation of HIPAA guidelines.
 - The technology supports bedside and remote charting.

ANS: D

Nurses working surrounded by computers and mobile IT must develop skills in the use of all available technology. At the same time, it is important to recognize that the rapid advancement of IT means that the technology in use today may be entirely different tomorrow. Some facilities have computer access at every bedside, and others have mobile computers, sometimes called *workstations on wheels* (WOWs), that can be taken to each bedside. Nurses using technology as part of patient care need to work within facility policy and HIPAA guidelines. The technology supports bedside and remote charting. Nurses may use a portable device such as a smartphone or tablet computer to access reference materials, including medical information and vast amounts of drug information. Some facilities issue these devices to staff.

DIF: Understanding

OBJ: 15.2

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Technology and Informatics

4. The home health nurse provides care for a patient with congestive heart failure. Daily the patient weighs himself and takes his own temperature, pulse, respirations, and blood pressure. That information is sent as electronic data to the patient's physician and nurse daily to adjust the plan of care as indicated. The nurse understands this is an example of which concept?
- Telehealth nursing
 - Computerized decision support system (DSS)
 - Computerized provider order entry (CPOE)
 - Point-of-care technology

ANS: A

Telehealth nursing is the transmission by a nurse of electronic data, images, or audio from a patient's bedside or home to other health providers for the purpose of providing care and improving outcomes. Patients may have telehealth hardware in their homes to provide in-home monitoring and direct reporting to their health care providers. Computerized decision support systems (DSSs) include safe practice alerts and reminders that improve the quality of care. Some DSSs assist in determining a correct diagnosis and choosing an appropriate medication. Computerized provider order entry (CPOE) allows orders to be directly communicated to the appropriate department—diet orders to dietary, medication orders to the pharmacy, laboratory orders to the laboratory. Computers, tablets, or pocket devices used at the bedside for documentation are examples of point-of-care technology. Patient data collected by a nurse and recorded electronically are immediately available to all members of the health care team.

DIF: Understanding

OBJ: 15.2

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Technology and Informatics

5. The nurse realizes that information technology (IT) can be used to increase patient safety in what way?
- By creating redundancy in orders making them safer
 - By removing the need for verification by the nurse
 - By analyzing errors to develop prevention strategies
 - By eliminating the need for bar codes in medication administration

ANS: C

IT can be used to increase patient safety. Errors are analyzed to develop strategies for prevention. Diagnostic test results are available faster to support treatment decisions and avoid redundancy in orders. When technology such as a bar-code medication administration (BCMA) system is used as part of the process of medication administration, fewer errors are made. After signing into the system or scanning his/her identification (ID) badge, the nurse electronically scans the bar codes of the patient ID, the medication administration record (MAR), and the drug to determine that the right patient is getting the right drug and dose at the right time. An alert signals a potential error, and it is the nurse's responsibility to verify all information before administration.

DIF: Applying OBJ: 15.3 TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Technology and Informatics

6. When technology such as a bar-code medication administration (BCMA) system is used as part of the process of medication administration, fewer errors are made. The nurse knows that the proper procedure when using the BCMA includes which action?
- Signing into the system using the patient's ID number
 - Typing in the patient's name and room number
 - Scanning the patient's ID, MAR, and medication
 - Discontinuing the medication if the system signals an error

ANS: C

When technology such as a BCMA system is used as part of the process of medication administration, fewer errors are made. After signing into the system or scanning his/her ID badge, the nurse electronically scans the bar codes of the patient ID, the MAR, and the drug to determine that the right patient is getting the right drug and dose at the right time. An alert signals a potential error, and it is the nurse's responsibility to verify all information before administration.

DIF: Applying OBJ: 15.3 TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Technology and Informatics

7. The nurse understands that computerized provider order entry (CPOE) is beneficial for what reason?
- CPOE decreases the number of transcribing errors.
 - CPOE enhances provider acceptance because of new technology.
 - CPOE decreases workflow issues in general.
 - CPOE reduces dependence on technology and computers.

ANS: A

Computerized provider order entry (CPOE) allows orders to be directly communicated to the appropriate department. CPOE systems ensure legible orders and have the potential to reduce ordering and transcribing errors. Disadvantages of CPOE include workflow issues, provider resistance to new technology, and overdependence on technology (AHRQ, 2012).

DIF: Understanding

OBJ: 15.3

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Technology and Informatics

8. Which statement does the nurse recognize as accurate regarding the use of electronic medical records (EMR)?
- EMR holds the documentation of a single episode of care.
 - EMR is a longitudinal record of care for each patient.
 - EMR is widely used for individual health care encounters.
 - EMR includes progress notes for all disciplines.

ANS: A

The electronic medical record (EMR), which is the documentation of a single episode of care (i.e., outpatient visit or inpatient stay), becomes a part of the electronic health record (EHR), which is a longitudinal record of care. EHRs are becoming widely used for individual health care encounters and for maintaining patients' health records over long periods. As EHRs become fully implemented, they include provider order entries, progress notes for all disciplines, computerized medication profiles, access to diagnostic test results on a timely basis, decision support systems, and online clinical reminders and alerts.

DIF: Understanding

OBJ: 15.3

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Technology and Informatics

9. The nurse knows that computerized provider order entry (CPOE) has which outcome?
- CPOE allows orders to be communicated to the appropriate department.
 - CPOE creates an intermediary for order transcription.
 - CPOE slows documentation and provider communication.
 - CPOE may lead to increased ordering and transcription errors.

ANS: A

Computerized provider order entry (CPOE) allows orders to be directly communicated to the appropriate department—diet orders to dietary, medication orders to the pharmacy, laboratory orders to the laboratory. Elimination of an intermediary for order transcription decreases the potential for errors related to the ambiguity of handwritten orders and allows quicker responses by appropriate departments. Legibility and availability of computerized documentation improve provider communication. The Agency for Healthcare Research and Quality (AHRQ) recommends CPOE as one of the safe practices for better health care. CPOE systems ensure legible orders and have the potential to reduce ordering and transcribing errors. Disadvantages of CPOE include workflow issues, provider resistance to new technology, and overdependence on technology (AHRQ, 2017).

DIF: Understanding

OBJ: 15.3

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Technology and Informatics

10. The nurse is providing care to a patient newly diagnosed with multiple sclerosis. When the patient expresses the desire to communicate with other people living with the disorder, the nurse refers the patient to which source?
- An e-mail list with the patient's contacts
 - A social media blog
 - A listserv concerning multiple sclerosis
 - Facebook, Twitter, and LinkedIn

ANS: C

Listserve can be used in health care to connect groups of patients with common problems or to send updated information to large groups. E-mail has become a common means of communication but would not be focused on the patient's issues. Social media include online technologies such as Facebook, Twitter, and LinkedIn that allow people to communicate easily by the Internet to share information and resources, but they are more general than listserve. These technologies enable a potentially massive community of participants to collaborate, providing a mechanism for tapping into collective power in ways previously unachievable. A blog is a social medium that is usually maintained by an individual and has regular entries of commentary, descriptions of events, or other material such as graphics or videos. Most blogs are interactive, allowing visitors to leave comments and message each other. Many blogs focus on health care issues.

DIF: Understanding

OBJ: 15.3

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Technology and Informatics

11. The nurse can see data relationships, can make judgments based on trends and patterns in the data, is skilled in information management and the use of computer technology, and is able to suggest areas for IT system improvement. The nurse's level of informatics competency can be described by which term?
- Beginner
 - Experienced
 - Specialist
 - Innovator

ANS: B

Descriptions of nursing informatics competencies often focus on levels that include beginner, experienced, specialist, and innovator. Beginner skills include computer, information, and web literacy; fundamental skills in information management and computer technology; and the ability to identify and collect relevant data. The nurse at the beginning level may have keyboarding skills, can document in the EHR, and look up medications and other health information on reputable Internet reference sites. The nurse at the experienced level of informatics competencies understands data relationships and makes judgments based on trends and patterns in data, demonstrates skill in information management and the use of computer technology, suggests areas for Internet technology system improvement, relates data posted by others to the nursing assessment, bases the nursing process and clinical decisions on the data, and devises better ways of using data from the EHR. The nurse at the specialist level of competency focuses on information needs for the practice of nursing; integrates and applies information science, computer science, and nursing science; and applies skills in critical thinking, data management, processing, and system development. At the specialist level of competency, the nurse may conduct research based on information trends or patient data, devise applications for computer technology in nursing, or develop new software to enhance nursing care. Nursing informatics innovators conduct research and generate theory. They develop solutions and understand the interdependence of systems, disciplines, and outcomes.

DIF: Analyzing OBJ: 15.4 TOP: Evaluation
 MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
 Concepts: Technology and Informatics

12. When the nurse manager on a medical-surgical floor has met education and experience requirements in nursing informatics, hospital administration may request the nurse to pursue what additional verification of competence in this area?
- a. Technical competencies
 - b. Utility competencies
 - c. Certification from ANCC
 - d. Leadership competencies

ANS: C

After meeting the educational and experience requirements, the nurse can receive certification in nursing informatics from the Health Care Information and Management Systems Society (HIMSS) and through the American Nurses Credentialing Center (ANCC). Technical competencies pertain to the use of computers and other technological equipment and the use of a variety of software programs for word processing, spreadsheet and database development, presentation, referencing, and e-mail. Utility competencies address critical thinking and evidence-based practice applications. Nurses who have a utility competency recognize the relevance of nursing data for improving practice and can access multiple information sources for gathering evidence for clinical decision making. Leadership competencies address the ethical and management issues related to using IT in nursing practice, education, research, and administration. Specific leadership competencies include the application of accountability, maintenance of privacy and confidentiality, and quality assurance. Technical, utility, and leadership competencies can be achieved without certification.

NOT: Concepts: Technology and Informatics

Fundamentals of Nursing 2nd Edition Yoost Test Bank

DIF: Understanding

OBJ: 15.4

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care

NOT: Concepts: Technology and Informatics

13. Patients frequently seek sources for health information online, and nurses, as advocates, need to be prepared to help patients evaluate online sources. To do this, the nurse asks who sponsors the site, is the author listed, and the author's credentials. The nurse is evaluating what concept?
- Purpose
 - Coverage
 - Currency
 - Authority

ANS: D

Authority involves knowing who the sponsor or publisher is. Is this a personal page? Where does it come from? Is the author or organization listed? What are the author's credentials? Purpose is determining to focus of the site. Does the site inform? Explain? Share? Disclose? Sell? What is the intended audience? Coverage tries to determine if citations are correct. Is there a balance of text and images? Currency refers to when the site was created. How often is it updated?

DIF: Analyzing OBJ: 15.6 TOP: Evaluation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Technology and Informatics

14. The patient asks the nurse about how to evaluate websites and standards used to evaluate Internet health sites. The nurse appropriately refers the patient to which agency?
- The Computer Ethics Commission
 - The U.S. Food and Drug Administration
 - The Health on the Net Foundation
 - The U.S. Federal Trade Commission

ANS: C

The Health on the Net Foundation focuses on the promotion and use of reliable online health information. The Computer Ethics Institute (CEI) was founded in 1985 to serve as a forum and resource for identifying, assessing, and responding to ethical issues associated with the advancement of information technologies and to facilitate the recognition of ethics in the development and use of computer technologies. The World Health Organization, the U.S. Food and Drug Administration, and the U.S. Federal Trade Commission are organizations that are consulted on efforts to promote credible online health care information and combat online health fraud.

DIF: Analyzing OBJ: 15.6 TOP: Evaluation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Technology and Informatics

15. One classification system for nursing informatics competencies uses technical, utility, and leadership categories. The nurse recognizes leadership competencies involve which concept?
- Maintaining privacy and confidentiality
 - Using computers and other technological equipment
 - Using a variety of software programs
 - Addressing critical thinking applications

ANS: A

One classification system for nursing informatics competencies uses technical, utility, and leadership categories. Leadership competencies address the ethical and management issues related to using IT in nursing practice, education, research, and administration. Technical competencies pertain to the use of computers and other technologic equipment and the use of a variety of software programs for word processing, spreadsheet and database development, presentation, referencing, and e-mail. Utility competencies address critical thinking and evidence-based practice applications. Nurses who have a utility competency recognize the relevance of nursing data for improving practice and can access multiple information sources for gathering evidence for clinical decision making.

DIF: Applying OBJ: 15.7 TOP: Implementation
 MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
 Concepts: Technology and Informatics

16. The Computer Ethics Institute has developed guidelines for ethics in the development and use of computer technologies. The nurse knows these guidelines are identified by which term?
- The Ten Commandments of Computer Ethics
 - The eHealth Code of Ethics
 - HIPAA guidelines
 - The Health on the Net Foundation

ANS: A

The Computer Ethics Institute (CEI) was founded in 1985 to serve as a forum and resource for identifying, assessing, and responding to ethical issues associated with the advancement of information technologies and to facilitate the recognition of ethics in the development and use of computer technologies. CEI developed the Ten Commandments of Computer Ethics. The eHealth Code of Ethics, developed by Health Informatics Europe (2005), is —to ensure that people worldwide can confidently and with full understanding of known risks realize the potential of the Internet in managing their own health and the health of those in their care.¶ The Health Insurance Portability and Accountability Act (HIPAA) of 1996 sets the standards on how security and confidentiality of health care information must be maintained. The act also sets the penalties for any breach in security of health care data. The Health on the Net Foundation promotes the use of reliable internet health sites.

DIF: Remembering OBJ: 15.7 TOP: Assessment
 MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
 Concepts: Technology and Informatics

17. The nurse understands that the focus of nursing informatics is which concept?
- Direct patient care
 - Increasing documentation time
 - The introduction of different EHRs
 - How patient care can be improved

ANS: D

The focus of nursing informatics is not on direct patient care but on how the process of patient care can be improved and patient safety ensured. Documentation time has decreased by using informatics. Different EHRs are used in different facilities.

DIF: Remembering OBJ: 15.8 TOP: Assessment
 MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care

NOT: Concepts: Technology and Informatics

18. While adopting new technology to enhance patient care and safety, nurses can continue to provide what service?
- Compassionate care
 - Consumer empowerment
 - Self-management of wellness
 - Education about health care

ANS: A

While adopting new technology that enhances patient care and safety, it is the nurse's responsibility to continue to provide compassionate care. The spread of IT supports consumer empowerment and self-management of wellness and disease. Consumers can electronically access their own health information, communicate with their health care providers, and seek needed education about health care.

DIF: Applying OBJ: 15.8 TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT: Concepts: Technology and Informatics

MULTIPLE RESPONSE

1. The nurse recognizes the use of telemonitoring offers the opportunity to complete which tasks? (*Select all that apply.*)
- Reduce cost of health care.
 - Improve patient satisfaction.
 - Increase duplicate orders.
 - Improve patient outcomes.
 - Improve organization.

ANS: A, B, D, E

The use of telemonitoring offers the opportunity to reduce the cost of health care while improving outcomes and patient satisfaction. Use of health care IT has improved organization, communication, and decision making; reduced duplicate orders, charting time, and paperwork; made medication administration safer; and enhanced information access and administrative functions.

DIF: Understanding OBJ: 15.2 TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT: Concepts: Technology and Informatics

2. The Technology Informatics Guiding Education Reform (TIGER) initiative identified a set of skills needed by all nurses practicing in the 21st century. The nurse identifies the TIGER Vision Pillars include which concepts? (*Select all that apply.*)
- Management and leadership
 - Certification by HIMSS
 - Communication and Collaboration
 - Informatics design
 - IT policy and culture

ANS: A, C, D, E

The Technology Informatics Guiding Education Reform (TIGER) initiative (2018) identified a set of skills needed by all nurses practicing in the 21st century. The TIGER Vision Pillars include management and leadership, education, communication and collaboration, informatics design, and IT policy and culture. Certification is not one of the pillars.

DIF: Remembering

OBJ: 15.2

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Technology and Informatics

3. In reviewing a patient's written chart, the nurse notes the use of the terms —bedsore, —decubitus ulcer, and —pressure ulcer. The nurse knows to reach maximum potential in computerized charting and data analysis that a standardized nursing terminology must be utilized. The nurse knows what concepts are associated with standardizing nursing terminology? (*Select all that apply.*)
- The Nursing Minimum Data Set (NMDS) was the first attempt to do so.
 - The focus was to provide a shared understanding of patient problem labels.
 - The NMDS data was completed and is the definitive source of patient labels.
 - The ICNP was developed to provide a standard for international nurses.
 - Standardized terminology can lead to better utilization of resources.

ANS: A, B, E

A standardized nursing terminology is a structured vocabulary that provides a common means of communication among nurses. A standardized language ensures that when a nurse talks about a specific patient problem, another nurse fully understands the problem. An example is the choice between *pressure ulcer*, *decubitus ulcer*, and *bedsore*. Do all nurses in all settings have a shared understanding of these labels for a patient problem? The Nursing Minimum Data Set (NMDS) represents the first attempt to standardize the collection of essential nursing data. These core data, used on a regular basis by most nurses in the delivery of care across settings, provide accurate descriptions of the nursing diagnoses, nursing care, outcomes of care, and nursing resources used. Collected on an ongoing basis, the NMDS enables nurses to compare data across populations, settings, geographic areas, and time. The International Classification for Nursing Practice (ICNP) (2015), developed under the auspices of the International Council of Nurses (ICN), is a standard terminology that provides a dictionary to describe and report nursing practice in a systematic way. This information supports care and decision making to inform nursing education, research, and health policy.

DIF: Understanding

OBJ: 15.5

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Technology and Informatics

4. The nurse recognizes which statements to be accurate regarding The Health Insurance Portability and Accountability Act (HIPAA) of 1996? (*Select all that apply.*)
- Requires the user to have verification codes.
 - Ensures access to information without fear of audits.
 - Sets the standards on how information is maintained.
 - Sets the penalties for any breach in security of health data.
 - Has no legal authority relative to security issues.

ANS: A, C, D

Access to electronic records requires a user to have system access and verification codes as a measure of security and protection of the patient's privacy. The codes leave an electronic trail of authorized users that can be audited. HIPAA sets the standards on how security and confidentiality of health care information must be maintained. The act also sets the penalties for any breach in security of health care data.

DIF: Understanding

OBJ: 15.7

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Technology and Informatics

NURSINGTB.COM

Chapter 16: Health and Wellness**Yoost & Crawford: Fundamentals of Nursing: Active Learning for Collaborative Practice, 3rd Edition**

MULTIPLE CHOICE

1. The nurse knows the World Health Organization defines health in which of the following terms?
 - a. The absence of disease
 - b. The lack of infirmity
 - c. Complete well-being
 - d. Being independent of fiscal responsibility

ANS: C

The World Health Organization offers a definition for health: —a state of complete physical, mental, and social well-being and not merely the absence of disease or infirmity. Nurses are responsible for helping patients reach their optimal levels of physiologic and mental health, but they also must provide health care in a system that requires cost containment and fiscal responsibility.

DIF: Remembering

OBJ: 16.1

TOP: Assessment

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance

NOT: Concepts: Health Promotion

2. Several models exist that describe the relationship between health and wellness. Which model is used to understand the interrelationship between elements of basic requirements for survival and the desires that drive personal growth and development and is represented as a pyramid?
 - a. Maslow's hierarchy of needs
 - b. Health Belief Model
 - c. Health Promotion Model
 - d. Holistic Health Model

ANS: A

Maslow's hierarchy of needs describes the relationships between the basic requirements for survival and the desires that drive personal growth and development. The model is most often presented as a pyramid consisting of five levels. The lowest level is related to physiologic needs, and the uppermost level is associated with self-actualization needs, specifically those related to purpose and identity. The Health Belief Model was developed by psychologists Hochbaum, Rosenstock, and Kegels. It explores how patients' attitudes and beliefs predict health behavior. The Health Promotion Model, developed by Pender and colleagues, defines health as a positive, dynamic state of well-being rather than the absence of disease in the physiologic state. Holistic health models in nursing care are based on the philosophy that a synergistic relationship exists between the body and the environment. Holistic care is an approach to applying healing therapies. Holistic models focus on the interrelatedness of body and mind.

DIF: Remembering

OBJ: 16.2

TOP: Assessment

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance

NOT: Concepts: Health Promotion

3. The nurse is developing a plan of care for a patient with a hip fracture. Which model would the nurse use to prioritize the patient's care?
- The Health Belief Model
 - Pender's Health Promotion Model
 - Maslow's hierarchy of needs
 - The Holistic Health Model

ANS: C

Maslow's hierarchy of needs describes the relationships between the basic requirements for survival and the desires that drive personal growth and development. The model is most often presented as a pyramid consisting of five levels. The lowest level is related to physiologic needs, and the uppermost level is associated with self-actualization needs, specifically those related to purpose and identity. The Health Belief Model was developed by psychologists Hochbaum, Rosenstock, and Kegels. It explores how patients' attitudes and beliefs predict health behavior. The Health Promotion Model, developed by Pender and colleagues, defines health as a positive, dynamic state of well-being rather than the absence of disease in the physiologic state. Holistic Health Models in nursing care are based on the philosophy that a synergistic relationship exists between the body and the environment. Holistic care is an approach to applying healing therapies. Holistic models focus on the interrelatedness of body and mind.

DIF: Applying OBJ: 16.2 TOP: Implementation
 MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:
 Concepts: Health Promotion

4. The nurse is preparing a patient teaching plan and is seeking a way to determine the patient's readiness and motivation to act regarding lifestyle changes to best manage diabetes mellitus. Which model would be useful for this nurse?
- Maslow's hierarchy of needs
 - Holistic Health Model
 - Health Promotion Model
 - Health Belief Model

ANS: D

The Health Belief Model was developed by psychologists Hochbaum, Rosenstock, and Kegels. It explores how patients' attitudes and beliefs predict health behavior. Maslow's hierarchy of needs describes the relationships between the basic requirements for survival and the desires that drive personal growth and development. The model is most often presented as a pyramid consisting of five levels. The lowest level is related to physiologic needs, and the uppermost level is associated with self-actualization needs, specifically those related to purpose and identity. Holistic Health Models in nursing care are based on the philosophy that a synergistic relationship exists between the body and the environment. Holistic care is an approach to applying healing therapies. Holistic models focus on the interrelatedness of body and mind. The Health Promotion Model, developed by Pender and colleagues, defines health as a positive, dynamic state of well-being rather than the absence of disease in the physiologic state.

DIF: Understanding OBJ: 16.2 TOP: Evaluation
 MSC: NCLEX Client Needs Category: Health Promotion and Maintenance
 NOT: Concepts: Health Promotion

5. According to the Health Belief Model, which of the following patients would be most likely to change health behavior?
- The person who perceives that he is at risk for colon cancer
 - The person who recognizes that colon cancer is easily cured
 - The person who believes that behavior can change outcomes
 - The patient who faces multiple social barriers

ANS: A

In the three primary components of the Health Belief Model, six main constructs influence an individual's decision to take action about disease prevention, screening, and controlling illness. The model suggests that individuals are motivated to take action if they believe that they are susceptible to the condition (i.e., perceived susceptibility), that the condition has serious consequences (i.e., perceived severity), that taking action would reduce the susceptibility or severity of the condition (i.e., perceived benefit), that the costs of taking action (i.e., perceived barriers) are outweighed by the benefits, that those who are exposed to factors (e.g., media campaigns, postcard reminders, and advice from others) will be prompted to action (i.e., cues to action), and that those who have confidence in their ability to perform an action will do so (i.e., perceived self-efficacy).

DIF: Analyzing

OBJ: 16.2

TOP: Evaluation

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:

Concepts: Health Promotion

6. The nurse recognizes that intentional behaviors to circumvent illness, detect it early, and maintain the best possible level of mental and physiologic function within the boundaries of illness is the definition of which term?
- Health promotion
 - Self-actualization
 - Health protection
 - Self-transcendence

NURSINGTB.COM

ANS: C

Health protection includes intentional behaviors aimed at circumventing illness, detecting it early, and maintaining the best possible level of mental and physiologic function within the boundaries of illness. Health promotion is behavior motivated by the desire to increase well-being and optimize health status. Maslow considered self-actualization the highest level of optimal functioning and involves the integration of cognition, consciousness, and physiologic utility in a single entity. In later years, Maslow described a level above self-actualization called *self-transcendence*. He refers to self-transcendence as a peak experience, in which analysis of reality or thought changes a person's view of the world and his or her position in the greater structure of life.

DIF: Remembering

OBJ: 16.2

TOP: Assessment

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance

NOT: Concepts: Health Promotion

7. The nurse caring for a patient with chronic pain uses guided imagery, therapeutic touch, and relaxation techniques as interventions for pain. The nurse is using what type of approach?
- Holistic
 - Eastern holistic
 - Risk factor reduction

d. Health protection

ANS: A

Nurses participate in holistic care through the use of natural healing remedies and complementary interventions. These include the use of art and guided imagery, therapeutic touch, music therapy, relaxation techniques, and reminiscence. Eastern holistic therapists have been using techniques such as acupuncture, yoga, and tai chi for thousands of years as methods of healing and, more recently, in conjunction with modern allopathic medical therapies. Risk factor reduction is step-by-step improvement of individual health factors.

These combined improvements lower the likelihood of developing a disease. Health protection includes intentional behaviors aimed at circumventing illness, detecting it early, and maintaining the best possible level of mental and physiologic function within the boundaries of illness.

DIF: Remembering

OBJ: 16.2

TOP: Assessment

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance

NOT: Concepts: Health Promotion

8. An overweight, sedentary middle-aged smoker with a family history of cardiac disease has noticed a steady rise in resting blood pressure over a 3- to 4-year period. The patient is concerned about his slightly elevated blood pressure and begins walking 20 to 30 minutes in the evenings with his wife and reduces his pack-a-day cigarette habit to ten cigarettes a day. The nurse identifies these actions are the initial step of which behavior?
- Risk factor reduction
 - Self-actualization
 - Self-transcendence
 - Health promotion

ANS: A

NURSINGTB.COM

Risk factor reduction is step-by-step improvement of individual health factors. These combined improvements lower the likelihood of developing a disease. Maslow considered self-actualization the highest level of optimal functioning and involves the integration of cognition, consciousness, and physiologic utility in a single entity. In later years, Maslow described a level above self-actualization called *self-transcendence*. He refers to self-transcendence as a peak experience, in which analysis of reality or thought changes a person's view of the world and his/her position in the greater structure of life. Health promotion is behavior motivated by the desire to increase well-being (as opposed to preventing illness) and optimize health status.

DIF: Remembering

OBJ: 16.3

TOP: Assessment

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance

NOT: Concepts: Health Promotion

9. The nurse recognizes the nursing goal for individuals and families seeking preventative care is to have those groups carry out which action?
- Take responsibility for their health and wellness.
 - Abandon the use of electronic educational media.
 - Make lifestyle changes after diseases occur.
 - Use temporary changes until the danger has passed.

ANS: A

Nursing goals for all individuals and their families seeking preventive care are improvement of quality of life through positive lifestyle choices and taking responsibility for health and wellness. Nurses can refer patients to a variety of personal health quizzes, located in the online version of *Healthy People 2020*, for risk assessments of their health status and lifestyle. The quizzes allow people to track their health and wellness status over a period of years and identify trends in disease risk factors that can be modified through lifestyle interventions or preventive measures before the disease occurs. The *Healthy People 2020* initiative helps nurses provide educational materials for individuals, families, and communities, enabling them to lead healthier lifestyles and to make permanent changes in wellness habits.

DIF: Understanding

OBJ: 16.3

TOP: Assessment

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance

NOT: Concepts: Health Promotion

10. The nurse knows that use of seatbelts and airbags in automobiles is an example of which term?
- Secondary prevention
 - Tertiary prevention
 - Holistic care
 - Primary prevention

ANS: D

Primary prevention is instituted before disease becomes established by removing the causes or increasing resistance. Examples include the use of seatbelts and airbags in automobiles, helmet use when riding bicycles or motorcycles, and the occupational use of mechanical devices when lifting heavy objects. Secondary prevention is undertaken in cases of latent (hidden) disease. Although the patient may be asymptomatic, the disease process can be

detected by medical tests. Nurses may use screening tests to assess for latent disease in vulnerable populations. Examples of screening tests used as secondary prevention strategies include the purified protein derivative (PPD) skin test for tuberculosis, fecal occult blood test for colorectal cancer, and mammograms for breast cancer. Tertiary prevention, also known as the treatment or rehabilitation stage of preventive care, is implemented when a condition or illness is permanent and irreversible. The aim of care is to reduce the number and impact of complications and disabilities resulting from a disease or medical condition. Interventions are intended to reduce suffering caused by poor health and assist the patients in adjusting to incurable conditions.

Nursing care is focused on rehabilitation efforts in the tertiary stage of prevention. Holistic care is an approach to applying healing therapies. Nurses participate in holistic care through the use of natural healing remedies and complementary interventions.

These include the use of art and guided imagery, therapeutic touch, music therapy, relaxation techniques, and reminiscence.

DIF: Understanding

OBJ: 16.4

TOP: Assessment

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance

NOT: Concepts: Health Promotion

11. A 40-year-old patient presents to her provider for a yearly physical. The provider notes a family history of breast cancer in the patient's mother. The provider schedules the patient for a mammogram. The nurse recognizes this as what level of prevention?
- Tertiary
 - Primary

- c. Secondary
- d. Holistic

ANS: C

Secondary prevention is undertaken in cases of latent (hidden) disease. Although the patient may be asymptomatic, the disease process can be detected by medical tests. Nurses may use screening tests to assess for latent disease in vulnerable populations. Examples of screening tests used as secondary prevention strategies include the purified protein derivative (PPD) skin test for tuberculosis, fecal occult blood test for colorectal cancer, and mammograms for breast cancer. Primary prevention is instituted before disease becomes established by removing the causes or increasing resistance. Examples include the use of seatbelts and airbags in automobiles, helmet use when riding bicycles or motorcycles, and the occupational use of mechanical devices when lifting heavy objects. Tertiary prevention, also known as the treatment or rehabilitation stage of preventive care, is implemented when a condition or illness is permanent and irreversible. The aim of care is to reduce the number and impact of complications and disabilities resulting from a disease or medical condition. Interventions are intended to reduce suffering caused by poor health and assist the patients in adjusting to incurable conditions. Nursing care is focused on rehabilitation efforts in the tertiary stage of prevention. Holistic care is an approach to applying healing therapies. Nurses participate in holistic care through the use of natural healing remedies and complementary interventions.

These include the use of art and guided imagery, therapeutic touch, music therapy, relaxation techniques, and reminiscence.

DIF: Understanding

OBJ: 16.4

TOP: Assessment

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance

NOT: Concepts: Health Promotion

12. The patient asks the nurse to explain collaborative health care partnerships. The nurse gives a correct description when making which statement regarding collaborative care?
- a. Does not require participation of the patient.
 - b. Is individual and cannot be mandated or legislated.
 - c. Education needs are delegated to assistive personnel.
 - d. Is designed to provide care to the patient as a whole.

ANS: D

Collaborative health care partnerships are designed to deliver well-balanced care to the patient as a whole, rather than rendering fragmented care involving a single element of a disease process.

Prevention is not solely the responsibility of the nurse; it involves active participation by the individual and the combined services of practitioners in a spectrum of health care disciplines as varied as nutrition, physical therapy, exercise physiology, and pharmacy.

Collaborative preventive care can be mandated in the form of health care legislation, with rates for reimbursement of practitioners determined by the individual provider's ability to collaborate and develop innovative methods for delivering high-quality, cost-effective health care services. The role of the professional nurse is to collaborate and communicate health education to the patient and family, care provider, or surrogate. Patient education responsibilities are not delegated to assistive personnel or other members of the health care team and are considered a cornerstone of nursing care.

DIF: Understanding

OBJ: 16.4

TOP: Assessment

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance

NOT: Concepts: Health Promotion

13. A patient is diagnosed with pneumonia after an abrupt onset of fever, cough, and malaise. The patient is started on antibiotic therapy and is expected to improve in 2 to 3 weeks. Which statement by the nurse correctly identifies this illness?
- Acute
 - Chronic
 - Remission
 - Exacerbation

ANS: A

Acute illness is typically characterized by an abrupt onset and short duration (<6 months). Clinical manifestations of acute illness appear quickly. They may be severe or lethal, or they may soon resolve because they respond to treatment or are self-limiting. Chronic illness is characterized by a loss or abnormality of body function that lasts longer than 6 months and requires ongoing long-term care. Chronic health conditions may be controlled with lifestyle management or drug therapy, but they are considered to be irreversible. Chronic illness may be characterized by periods of wellness (i.e., remission) and exacerbation (worsening) of clinical manifestations, which can be life threatening. Individuals learn to adjust their lifestyles accordingly.

DIF: Understanding

OBJ: 16.5

TOP: Assessment

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance

NOT: Concepts: Health Promotion

14. The nurse needs to consider which approach when caring for patients with chronic illness?
- Help the patient face the reality that he will not get better.
 - Emphasize to the patient that the illness is not his fault.
 - Focus on improving quality of life through preventive behaviors.
 - Acknowledge the limitations placed on the patient by his suffering.

ANS: C

Nurses can help patients establish a daily routine of care by educating them about how to manage their care and the symptoms associated with the condition, including emergency or life-threatening situations. Emphasis is on improving quality of life through preventive behaviors. The attitude of being a victim, suffering with, or being afflicted by a chronic illness is viewed by nurses as a counterproductive behavior that needs positive intervention. Nurses can assist patients with strategies that help them cope with their chronic conditions and associated feelings of anger, frustration, and depression. Encouragement and positive support from a professional nurse can help individuals gain control over the alternating periods of health and illness and improve their quality of life.

DIF: Applying

OBJ: 16.5

TOP: Implementation

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance

NOT: Concepts: Health Promotion

15. A patient presents to the clinic for illness, and the sick role is legitimized by the provider. The nurse recognizes this as what stage of illness according to Suchman's Model?
- I
 - II
 - III
 - IV

ANS: C

In stage III (Medical care contact), professional advice from health care providers is sought by the individual. A professional health care provider identifies and validates the illness and legitimizes the sick role. During stage II (Assumption of the sick role), the person decides that the illness is genuine and that care is necessary. This stage gives an individual permission to act sick and to be excused temporarily from typical social and personal obligations. During stage I (Symptom experience), a clinical manifestation of disease is experienced, and the person acknowledges that something is wrong and seeks a cure. The outcome of stage I is that the person accepts the reality of symptoms and decides to take action in seeking care. During stage IV (Dependent patient role), the person, who is designated as a patient, usually undergoes treatment. During this stage, patients often feel dependent on others and may experience ambivalent or fearful thoughts that cause them to reject treatment, the advice of health care providers, and the illness.

DIF: Remembering

OBJ: 16.5

TOP: Assessment

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance

NOT: Concepts: Health Promotion

16. When considering factors influencing health and the impact of illness, specifically age, the nurse would correctly identify which patient as having the greatest risk?
- 10-year-old girl
 - 23-year-old woman
 - 47-year-old man
 - 85-year-old woman

ANS: D

Assessment of the patient begins with risk factors that take into account the person's age and the associated level of immune system function. The very young, especially neonates and infants born prematurely, are more susceptible to infections because of the immaturity of their immune systems. Likewise, older adults have decreased immune system function because of the aging process. Older patients are at risk for opportunistic infections resulting from harmless organisms that become pathogenic and illness from the spread of community-acquired disease. Complications from comorbidities of chronic disease may also increase suffering in the aged population.

DIF: Understanding

OBJ: 16.6

TOP: Assessment

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance

NOT: Concepts: Health Promotion

17. When discussing immunizations for infants and children with new parents, the nurse should focus on which approach?
- Providing scientific evidence to parents
 - Stressing that nonimmunization is a crime
 - Acknowledging that immunizations are not needed
 - Informing the parents that they have no choice

ANS: A

Parents need to have scientific, evidence-based information about immunizations and their consequences before choosing to accept or reject immunizations for their children. The parent's ability to make an informed decision is the primary goal for nurses educating people about childhood immunizations.

DIF: Applying OBJ: 16.6 TOP: Implementation
 MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:
 Concepts: Health Promotion

18. The nurse recognizes which concept that correctly completes the definition of the genetic vulnerability of an organism (risk of disease expression based on genotype)?
- It is involuntarily passed from biologic parents to offspring.
 - It is totally unrelated to environmental factors.
 - It is nonresponsive to alteration by way of lifestyle modification.
 - It is not a factor in mental illness because it is behavioral.

ANS: A

The genetic vulnerability of an organism, or risk of disease expression based on genotype, is involuntarily passed from biologic parents to their offspring. Societal attitudes about testing and management of high-risk populations depend on the potential for expression of genetic disorders that may be triggered by environmental factors. Controlling factors that place stress on physiologic function can reduce pathologic genetic expression and susceptibility to disease. For example, a person with a family history of hyperlipidemia and atherosclerosis is at risk for developing cardiovascular disease later in life. Lifestyle-modifying factors, such as weight reduction, daily exercise, and balanced nutritional intake, can help reduce the likelihood that the genetic risk factor for heart disease will be expressed. Diabetes, cancer, mental illness, and renal disease also have genetic components and are amenable to interventions that reduce risk.

DIF: Understanding OBJ: 16.6 TOP: Assessment
 MSC: NCLEX Client Needs Category: Health Promotion and Maintenance
 NOT: Concepts: Health Promotion

19. The patient is asking about using the Internet for resources regarding lifestyle behaviors and benefits of modification. What is the best response that the nurse should provide the patient?
- Information on lifestyle behaviors is not available on the Internet.
 - The patient should use websites that are easy to understand.
 - Most websites are designed for health care providers only.
 - Only negative outcomes are evaluated on the Internet.

ANS: B

Information on lifestyle behaviors that lead to disease is available at research-sponsored websites that have peer-reviewed material and expert analyses. Website content should be easy to read and understandable for the general population. Most sites that discuss the latest information about health risks, lifestyle behaviors, and outcomes have separate information specifically for health care providers. Research that evaluates positive and negative lifestyle-behavior outcomes is constantly evolving as discoveries are made about the physiologic changes bodies experience with disease and illness.

DIF: Applying OBJ: 16.6 TOP: Implementation
 MSC: NCLEX Client Needs Category: Health Promotion and Maintenance

NOT: Concepts: Health Promotion

20. The nurse is assessing a patient's environment and its impact on outdoor activity and notes that the child rarely plays outside. Which is true regarding the indoor environment?
- Indoor environments protect the patient from toxic chemicals.
 - Indoor activity is sometimes a result of unsafe outdoor conditions.
 - Indoor activity decreases the risk of respiratory illness.
 - Indoor lifestyles reduce the risk for sedentary behaviors.

ANS: B

Outdoor environments affect individual health in the areas of sanitation and waste disposal, water quality, air quality, and safety. Children living in areas where there are safety issues related to gang activity, sexual predators, or heavy traffic are less likely to engage in outdoor play activities. Their limited access to safe outdoor play space increases their risk for sedentary behaviors, excessive calorie intake, and obesity. Indoor environments may harbor toxic household cleaning agents, chemicals (e.g., radon, carbon monoxide, unused drugs), tobacco smoke, and energy sources (e.g., microwave ovens). Exposure to mold, household pests (e.g., dust mites, spiders), and unsanitary living conditions in an enclosed space increases the likelihood of respiratory illness and skin disorders.

DIF: Remembering

OBJ: 16.6

TOP: Assessment

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance

NOT: Concepts: Health Promotion

21. The nurse correctly recognizes which one of the following illnesses to trigger the broadest range of emotional and behavioral responses?
- Ear infection
 - Mild concussion
 - Rheumatoid arthritis
 - Influenza

NURSINGTB.COM

ANS: C

Chronic, debilitating disease such as rheumatoid arthritis and severe illness can produce a broad range of emotional or behavioral responses in patients and their families. A short-term, self-limited illness that is not life threatening does not evoke emotions or actions that cause fundamental changes in daily lifestyle. More often, illnesses such as the flu, ear infections, and sore throats are viewed as minor irritations or inconveniences. They usually require a short-term adjustment in daily routines, and treatment of symptoms is the priority so that the individual can continue with normal activities. The emotional and behavioral changes associated with non-life-threatening illness are usually minimal, and the individual quickly returns to the previous baseline level of emotional functioning.

DIF: Understanding

OBJ: 16.6

TOP: Assessment

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance

NOT: Concepts: Health Promotion

22. Self-concept refers to the way in which individuals perceive unchanging aspects of themselves, such as social character, cognitive abilities, physical appearance, and body image. Which additional point does the nurse recognize as part of the definition of self-concept?
- If negative, self-concept will allow the patient to compensate for weaknesses.

- b. If positive, self-concept will cause the patient to see challenges as devastating.
- c. Self-concept is a concept that is derived from the patient internally.
- d. Self-concept depends on relationships with family and friends.

ANS: D

Self-concept refers to the way in which individuals perceive unchanging aspects of themselves, such as social character, cognitive abilities, physical appearance, and body image. It is a mental image of self in relation to others and the surroundings. If the image is positive, the person will develop strengths, compensate for weaknesses, and experience life in a healthy way. If the image is negative (e.g., frail), the person will find life's challenges devastating and sometimes insurmountable. The impact of illness on the self-concept of a patient and the patient's family members depends on how secure the parties' relationships are with one another.

DIF: Understanding

OBJ: 16.6

TOP: Assessment

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance

NOT: Concepts: Health Promotion

MULTIPLE RESPONSE

1. Which recommendations would the nurse identify as appropriate screening guidelines? (*Select all that apply.*)
 - a. Women ages 21 to 29 should have a Pap test every 3 years.
 - b. Self-breast exams should be addressed with male and female patients.
 - c. Adolescent males should perform monthly self-testicular exams.
 - d. Women ages 30 to 65 should receive Pap tests every 10 years.
 - e. After a total hysterectomy, Pap testing should be more frequent.

ANS: A, B, C

All women should begin cervical cancer screening at the age of 21 years. Women between the ages of 21 and 29 years should have a Papanicolaou (Pap) test every 3 years. A priority assessment task for nurses in a variety of care settings is to ask female and male patients about breast self-examination. An adolescent male should be assessed for testicular self-examination habits, and older males should have an annual prostate examination. Women between the ages of 30 and 65 years should have a Pap test plus a human papillomavirus (HPV) test (i.e., co-testing) every 5 years. Women 65 years of age or older who have had normal results for previous Pap tests should no longer be screened. Women who have had a total hysterectomy (i.e., removal of the uterus and cervix) should not be tested, unless the surgery was done as a treatment for cervical cancer or pre-cancer.

DIF: Understanding

OBJ: 16.6

TOP: Assessment

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance

NOT: Concepts: Health Promotion

2. The nurse recognizes which of the following to be a benefit of regular physical exercise? (*Select all that apply.*)
 - a. Enhances the immune system.
 - b. Decreases bone density.
 - c. Limits joint mobility.
 - d. Improves mental health.

- e. Helps to prevent type 2 diabetes.

ANS: A, D, E

Exercise is essential for the prevention of illness and promotion of wellness. Physical exercise is any bodily activity or movement that enhances or maintains physical fitness levels and overall health. Exercise strengthens muscles, improves cardiovascular performance, hones athletic skills and endurance, and reduces or maintains weight, and it is performed for enjoyment (Powers and Howley, 2012). Regular physical exercise enhances the immune system, builds and maintains healthy bone density, increases joint mobility, and helps to prevent cardiovascular disease, type 2 diabetes, and obesity. Exercise also improves mental health and helps to prevent depression through the release of endorphins and other neurotransmitters that are responsible for exercise-induced euphoria (Powers and Howley, 2012).

DIF: Remembering

OBJ: 16.6

TOP: Assessment

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance

NOT: Concepts: Health Promotion

3. The economic stability of individuals or families can determine whether they are willing to seek preventive care or screening examinations. The nurse knows which statements about screening examinations to be true? (*Select all that apply.*)
- a. Free or low-cost screening ensures patient screening.
 - b. People may not screen due to fear of testing positive.
 - c. Early screening ensures minimal treatment costs.
 - d. Employment stability is enhanced by early screening.
 - e. Treatment of disorders often means lost wages.

ANS: B, E

NURSINGTB.COM

The economic stability of individuals or families can determine whether they are willing to seek preventive care or screening examinations. Even if screening is free or low cost, the patient or family members may decline because of the potential for testing positive for a disease. Treatment of a disorder often requires time spent away from work, lost wages, and expensive drug therapies and diagnostic tests. The financial impact can be devastating to families or individuals who have a limited or fixed income and fear that employment stability may be compromised.

DIF: Understanding

OBJ: 16.6

TOP: Assessment

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance

NOT: Concepts: Health Promotions

Chapter 17: Human Development: Conception Through Adolescence**Yoost & Crawford: Fundamentals of Nursing: Active Learning for Collaborative Practice, 3rd Edition****MULTIPLE CHOICE**

1. The nurse is asked by the parent of a pediatric patient to explain the difference between growth and development. Which response by the nurse is best?
 - a. —Growth is physical while development relates to physical, emotional, and cognitive function.||
 - b. —There is no difference between the two since they occur simultaneously.||
 - c. —Development refers to musculoskeletal and nervous system abilities and growth is a change in height and weight.||
 - d. —Both refer to an increase in abilities and functions of the child that occur sequentially over time.||

ANS: A

Growth relates to physical changes in height and weight. Development refers to changes in ability across several dimensions such as physical, emotional, and cognitive. Stating that the two are not different does not show understanding of this difference. Development is not related strictly to changes in specific body systems. Although both refer to increases in abilities and functioning over time, this answer is too vague to give the parent useful information.

DIF: Understanding

OBJ: 17.1

TOP: Teaching/Learning MSC:

NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Development

2. The pediatric nurse is treating a patient who has questions about safer sexual practices. The patient states, —I think I should wait until marriage to be sexually active because I'm not sure sex is OK outside of marriage.|| The nurse understands the student is acting with which component of Freud's theory?
 - a. Id
 - b. Ego
 - c. Superego
 - d. Anal

ANS: C

The superego is the structure that houses the moral branch of personality. The Id acts strictly on instinct without consideration of reality. The Ego is partly conscious but does not consider right from wrong. Freud's theory contains the —anal phase.||

DIF: Understanding

OBJ: 17.1

TOP: Assessment

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Development

3. The nurse is collecting a history from the parents of a 4-year-old female at a well-child visit. The parents express concern that they often find their daughter performing what appears to be masturbation. The nurse offers reassurance by explaining which stage of development according to Freud?
 - a. Oral
 - b. Phallic

- c. Anal
- d. Latency

ANS: B

The phallic stage occurs between the ages of 3 and 6 years, and pleasure centers on the child's discovery that self-stimulation is enjoyable. The oral stage is seen in infants where pleasure centers around the mouth and putting things in the mouth. The Anal stage occurs between 18 months and 3 years of age and is when tension and release of tension occur through anal elimination. The latency stage occurs between the ages of 6 years and puberty during which interest in sexuality is repressed.

DIF: Understanding

OBJ: 17.1

TOP: Teaching/Learning MSC:

NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Development

4. A nurse is providing anticipatory guidance to a new mother about the Erikson stage of trust versus mistrust. What education should the nurse provide to the mother to help her child successfully master this stage?
- a. Consistently provide your child with food and attention.
 - b. Ensure someone is able to feed your child on a schedule.
 - c. Allow unrestricted crawling and exploring as the child develops.
 - d. Provide firm guidelines for behavior and activities.

ANS: A

The most important item needed for a child to master this stage of development is a consistent caregiver who provides food and attention. If the caregiver is inconsistent or unable to meet these needs, the child will develop mistrust of those around him. Ensuring that someone feeds the child is not providing consistency. Allowing exploration within limits (setting boundaries) is important to master initiative versus shame and doubt.

DIF: Understanding

OBJ: 17.1

TOP: Teaching/Learning MSC:

NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Development

5. The nurse is caring for a patient that is actively trying to conceive a child but continues to drink alcohol. The patient states that she'll stop drinking once she is pregnant. What is the most appropriate response by the nurse?
- a. —Abstaining is best since most fetal development occurs before you realize you are pregnant.¶
 - b. —Small amounts of alcohol are safe at any time during pregnancy.¶
 - c. —Things will be okay if you quit drinking alcohol once you know you are pregnant.¶
 - d. —Alcohol use should be avoided early in pregnancy but is acceptable past week 20.¶

ANS: A

Rapid development occurs before many women know that they are pregnant, making alcohol consumption unsafe at any time during pregnancy.

DIF: Applying

OBJ: 17.2

TOP: Teaching/Learning MSC:

NCLEX Client Needs Category: Health Promotion and Maintenance NOT:

Concepts: Development

6. The perinatal clinic nurse is going to teach a woman from a culture unfamiliar to the nurse about child-rearing practices. What action by the nurse is best before planning the education?
- Ensure the availability of written material to give the woman.
 - Assess what practices are important to her cultural group.
 - Determine if the woman is the primary family decision maker.
 - Refer the woman to a prenatal educational class.

ANS: B

The nurse must ensure he/she has a solid understanding of important child-rearing concepts in the woman's culture or risk that any teaching done will be irrelevant and perhaps in opposition to important beliefs. Since the nurse is unfamiliar with this culture, the first step is to assess. Written material is helpful if the patient can read and comprehend it. It would be important to determine if the woman is the decision maker, but this is not as much of a priority as learning about the culture. Referring the woman to an educational group may or may not be helpful.

DIF: Applying OBJ: 17.1 TOP: Assessment

MSC: NCLEX Client Needs Category: Psychosocial Integrity NOT: Concepts: Development

7. A home health care nurse is making a well-baby visit to the home of a new mother who has an infant. What assessment finding leads the nurse to provide further anticipatory guidance and teaching to the mother?
- Mother states she does not breastfeed but uses a recommended formula.
 - Crib has colorful blankets and pillows for the baby to cuddle.
 - A mobile is hanging well above the crib playing soft music.
 - Several rattles and plush toys are available in different textures.

ANS: B

NURSINGTB.COM

Objects such as pillows and blankets pose a suffocation hazard to infants and should be kept out of cribs. The other items are appropriate for a newborn.

DIF: Applying OBJ: 17.4 TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment NOT: Concepts: Development

8. To help a hospitalized infant master the tasks in Erikson's stage of Trust versus Mistrust, which action by the nurse is best?
- Provide calming music during quiet time so the infant can sleep.
 - Give the family food vouchers for the hospital cafeteria.
 - Arrange to have a cot or small bed placed in the infant's room.
 - Do not allow unlicensed assistive personnel to care for the infant.

ANS: C

Caregiver consistency is vital to accomplishing this task. The nurse should provide the parent(s) a comfortable place to stay in the infant's room. Giving food vouchers is also a good intervention, but not as important as ensuring the parent(s) can stay with the child. Calming music is appropriate for a child this age but does not help the child master tasks in this phase. Sleep is important for any hospitalized patient but is unrelated to mastering the tasks in this phase.

DIF: Applying OBJ: 17.1 TOP: Implementation

MSC: NCLEX Client Needs Category: Psychosocial Integrity NOT: Concepts: Development

9. The home health care nurse is visiting a family with a 3 year old to observe a meal. The parent gives the child a plate with 1/2 cup of pureed meat. What action by the nurse is best?
- Document how well the child eats the serving of meat.
 - Inquire if the child still drinks from a bottle between meals.
 - Ask the parents what they serve the child for snacks.
 - Provide teaching on the appropriate serving size for this child.

ANS: D

An appropriate serving size is 1 tablespoon per year of age, so an appropriate amount of meat for this child is 3 tablespoons, not 1/2 cup (which is 8 tablespoons). The nurse should provide more education to the family. The other options are appropriate but are not directly related to the serving size of meat.

DIF: Applying OBJ: 17.5 TOP: Implementation
MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:
Concepts: Development

10. A preschool-aged child got into the cookie jar and ate several cookies before dinner. When confronted by the parent, the child responds, —My pet horse ate them. What does the nurse teach the parents about this response?
- It is normal for children to have imaginary friends at this age.
 - This vivid imagination will lead the child to misbehave later on.
 - Lying is disobedient and should be punished consistently.
 - The child is obviously afraid of the parents' response.

ANS: A

It is common for toddlers to have imaginary friends. They are especially important in allowing the child to express something unpleasant. The other responses are not appropriate.

DIF: Applying OBJ: 17.6 TOP: Teaching/Learning MSC:
NCLEX Client Needs Category: Health Promotion and Maintenance NOT:
Concepts: Development

11. A toddler has been hospitalized. The parents become upset when the toddler starts wetting his bed, saying that he has been potty trained for some time now. What response by the nurse is best?
- Don't worry, this behavior will stop when he gets home.
 - Maybe he has a urinary tract infection; I'll get a urine sample.
 - I can call the Child Life Specialist for diversionary activities.
 - It is common for kids in the hospital to regress to earlier behaviors.

ANS: D

The stress of hospitalization often causes toddlers to regress in their behaviors, and the nurse should provide this information to the parents. Stating that the behavior will stop, although accurate, does not provide an explanation. There is no need for a urine sample. Using Child Life is always a good idea for hospitalized children but is not related to the question.

DIF: Understanding OBJ: 17.5 TOP: Teaching/Learning
MSC: NCLEX Client Needs Category: Health Promotion and Maintenance
NOT: Concepts: Development

12. The nurse is conducting a home visit on a newborn. What observation would require the nurse to provide further education?
- The caregiver warms the bottle and tests heat on the inside of the wrist.
 - The parents state the infant is sleeping with them until they buy a crib.
 - One parent states that when the child gets frustrating, the other parent takes over.
 - Caregivers consistently wash their hands before holding the baby.

ANS: B

Infants should not sleep in the same bed as their parents because of the risk of suffocation. The other actions are appropriate.

DIF: Applying OBJ: 17.3 TOP: Assessment
MSC: NCLEX Client Needs Category: Safe and Effective Care Environment NOT:
Concepts: Development

13. The parents of a 4 year old express concern that the child is wearing the same size clothing as she did last year. What action by the nurse is most appropriate?
- Weigh and measure the child and compare with last visit.
 - Reassure parents that their child is growing normally.
 - Assess the child's eating and activity patterns.
 - Encourage the parents to provide the child a multivitamin.

ANS: A

Physical growth slows during the preschool years, with most children only gaining about 5 lb and 2 1/2 to 3 inches a year. The nurse should weigh and measure the child and compare the readings to those taken at the last visit. Showing the parents these results and educating them on expected growth will reassure them. Simply telling the parents their child is normal does not provide objective information and is dismissive of their concern. The nurse should assess each child's eating and activity habits. The child may or may not need a vitamin. This can be discussed with the provider.

DIF: Applying OBJ: 17.5 TOP: Implementation
MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:
Concepts: Development

14. A nurse is conducting a preschool screening in the community. Which child would the nurse refer for further assessment?
- A 4 year old who throws a ball over-handed but better under-handed.
 - A 4 year old who can skip across the room after being shown how.
 - A 5 year old who is able to ride a bicycle with training wheels.
 - A 5 year old who is unable to ride a tricycle without falling.

ANS: D

A 3 year old should be able to ride a tricycle, so a 5 year old unable to perform this task needs further assessment. The other activities are appropriate for each child's age.

DIF: Applying OBJ: 17.6 TOP: Assessment
MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:
Concepts: Development

15. A father expresses frustration that his school-aged child is suddenly —sick all the time. What action by the nurse is best?

- a. Encourage the father to give the child a multivitamin each day.
- b. Explain that illness is frequent in this age-group because of exposure to others.
- c. Encourage the father to discuss testing the child's immunity with the provider.
- d. Make sure the parents are washing their hands frequently in the home.

ANS: B

Children in this age-group tend to have a higher incidence of minor illnesses because of exposure to others. The nurse can reassure the father by explaining this. No other action is needed at this point.

DIF: Applying OBJ: 17.7 TOP: Teaching/Learning MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT: Concepts: Development

16. A school-aged child is scheduled for a minor procedure and is very nervous. What response by the nurse is best?
- a. Reassure the child the procedure is too minor to worry about.
 - b. Read the child a pamphlet about what to expect during the procedure.
 - c. Tell the child you will have the provider —put her to sleep during the procedure.
 - d. Explain the procedure and what to expect in simple terms.

ANS: D

School-aged children benefit from simple explanations they can understand. Just telling the child not to worry is dismissive of the child's concerns. A school-aged child may not be able to read and/or understand a written pamphlet. Using phrases such as —put you to sleep should be avoided since they can be misinterpreted.

DIF: Applying OBJ: 17.7 TOP: Teaching/Learning MSC: NCLEX Client Needs Category: Psychosocial Integrity NOT: Concepts: Development

17. A nurse is assessing an adolescent female who began menstruating 2 years ago. She has grown 1/2 inch in the last 2 years but has not gained any weight. What action by the nurse is most appropriate?
- a. Ask the teen to provide a 24-hour diet recall.
 - b. Talk to the teen about healthy dietary practices.
 - c. Reassure the teen she will have a growth spurt soon.
 - d. Collaborate with the provider for endocrine testing.

ANS: B

During the adolescent growth spurt, teens achieve approximately 20% to 25% of their final height. This occurs during the time span ending about 2 years after the onset of menses. Since this teen has already reached that mark with little growth, the nurse should assess the teen's knowledge and practice of healthy eating. Poor eating habits are common with this age-group. A 24-hour diet recall can be utilized but the nurse's assessment should encompass more than just the recall. The teen most likely will not have another growth spurt later. Endocrine testing is not warranted at this point.

DIF: Applying OBJ: 17.8 TOP: Assessment MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT: Concepts: Development

18. A parent is concerned that her 16 year old is spending most of his time away from the family in his room and does not want to be involved in family activities he used to enjoy. What action by the nurse is best?
- Reassure the parent the teen is exerting independence.
 - Ask the parent about the teen's friends and activities.
 - Assess the teen for depression and possible suicide risk.
 - Refer the family to the community depression support group.

ANS: C

Teens typically begin to withdraw from the family to maintain privacy and exert independence, so this alone is not concerning. However, since the teen is not participating in activities he once enjoyed, the nurse should conduct a depression assessment. If the teen is depressed, the nurse should assess his suicide risk. If these screenings are normal, the nurse can reassure the parent. The teen himself is the best source of information about friends and activities, although the parent can be a good secondary source. A referral is not warranted without further assessment.

DIF: Applying OBJ: 17.8 TOP: Assessment

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:

Concepts: Development

19. A nurse assesses a 4-month-old infant and notes the baby does not follow a moving object with her eyes. What action by the nurse is best?
- Document the findings and continue the assessment.
 - Refer the child and parent to a pediatric neurologist.
 - Assess the child for other age-appropriate behaviors.
 - Assess the child for signs of child abuse or neglect.

ANS: C

A 3-month-old child should be able to follow a moving object with his or her eyes. However, one single abnormal assessment finding does not necessarily mean that the child has a growth and developmental delay. The nurse should assess for other age-appropriate behaviors.

Documentation should occur but is not the priority action at this point. A referral is not warranted nor is assessing for child abuse based on the data.

DIF: Applying OBJ: 17.4 TOP: Assessment

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:

Concepts: Development

20. A home health care nurse notes a parent becoming irritated when his toddler repeatedly throws his rattle from the high chair to the floor. What action by the nurse is most appropriate?
- Teach the parent about age-appropriate discipline.
 - Educate the parent on age-appropriate behaviors.
 - Tell the parent to stop giving the rattle back to the child.
 - Assess the child for signs of abuse or neglect.

ANS: B

Throwing an object down to watch someone else pick it up is a typical behavior for this age-group. The nurse should teach the parent about how this behavior relates to toddler growth and development. The other actions are not appropriate in this situation.

DIF: Applying OBJ: 17.5 TOP: Teaching/Learning MSC:
 NCLEX Client Needs Category: Health Promotion and Maintenance NOT:
 Concepts: Development

MULTIPLE RESPONSE

1. The nurse is teaching parents about actions to assist in developing a critical skill in the concrete operations phase of Piaget's developmental theory. What activities does the nurse suggest the parents participate with their child in? (*Select all that apply.*)
 - a. Separating a collection of toy horses into functions each type performs.
 - b. Exploring a space and astronomy museum and planetarium together.
 - c. Making a scrapbook of leaves sorted by color or type of tree.
 - d. Having the child explore how common objects can be used for different purposes.
 - e. Asking the child to describe an event from several different points of view.

ANS: A, C

In the concrete operational stage of Piaget's theory, seriation is an important task. This task includes separating or sorting objects using specific criteria. Separating toy horses by functions and arranging a leaf album by color or tree type are examples of seriation. Exploring museums does not contribute to seriation. Learning how objects can be used for unusual purposes and describing other points of view are part of the formal operations stage.

DIF: Applying OBJ: 17.1 TOP: Teaching/Learning
 MSC: NCLEX Client Needs Category: Psychosocial Integrity NOT: Concepts: Development

2. A nurse is assessing a 12 month old at a well-baby visit. For what developmental milestones does the nurse assess this child? (*Select all that apply.*)
 - a. Attempting to walk with help
 - b. Transferring objects from one hand to the other
 - c. Ability to roll around on the floor holding a bottle independently
 - d. Searching for objects that are out of sight
 - e. Moving from lying on abdomen to sitting unassisted

ANS: A, D, E

A 12 month old should be attempting to walk with help, hold a bottle independently and move from lying on abdomen to sitting up unassisted. Transferring objects from one hand to the other and rolling from front to back are milestones seen around 7 months of age and holding a bottle independently occurs at 4 to 6 months.

DIF: Remembering OBJ: 17.4 TOP: Assessment
 MSC: NCLEX Client Needs Category: Health Promotion and Maintenance
 NOT: Concepts: Development

3. A nurse is planning a community education event for parents on the topic of school-aged children and the risks of too much social media time. What topics should the nurse plan to include? (*Select all that apply.*)
 - a. Increased bullying
 - b. Decreased physical activity
 - c. Decreased understanding of spatial relationships
 - d. Weight loss and malnutrition

- e. Increased aggressiveness

ANS: A, B, E

Some of the risks associated with social media include bullying, decreased physical activity with resultant obesity, and aggressiveness.

DIF: Understanding

OBJ: 17.7

TOP: Teaching/Learning

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance

NOT: Concepts: Development

4. A high-school nurse is planning an educational presentation for juniors. What activities are most appropriate for the nurse's plan to include? (*Select all that apply.*)
- a. Video showing the aftermath of a drunk driving car crash
 - b. Confidential depression and suicide risk assessment
 - c. Same-age speaker sharing her story about the impact of HIV disease
 - d. Charts and graphs showing the physical changes of puberty
 - e. Bicycle helmet fitting station to see if child has outgrown the helmet

ANS: A, B, C, D

Adolescents need education on drinking and driving, suicide and depression, safer sexual practices, and physical changes that occur during puberty. A bicycle helmet fitting station would not be a priority for this age-group.

DIF: Applying

OBJ: 17.8

TOP: Teaching/Learning

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance

NOT: Concepts: Development

5. A pregnant woman in her second trimester is scheduled for quad testing. What conditions does the nurse explain are screened for in this assessment? (*Select all that apply.*)
- a. Blood clotting abnormalities
 - b. Neural tube defects
 - c. Heart abnormalities
 - d. Trisomy 18
 - e. Trisomy 21

ANS: B, D, E

Quad testing includes assessing for neural tube defects, trisomy 18, and trisomy 21 (Down syndrome). It does not screen for heart or blood-clotting problems.

DIF: Remembering

OBJ: 17.2

TOP: Teaching/Learning

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance

NOT: Concepts: Development

Chapter 18: Human Development: Young Adult Through Older Adult**Yoost & Crawford: Fundamentals of Nursing: Active Learning for Collaborative Practice, 3rd Edition****MULTIPLE CHOICE**

1. The nurse working in long-term care knows that there are multiple theories regarding aging. The one the nurse most identifies with proposes that the body's cells are leading to damaged organs and organ systems. This description is congruent with which theory?
 - a. Cross-linking theory of aging
 - b. Wear-and-tear theory
 - c. Gould's theory on adult development
 - d. Senescence theory of aging

ANS: B

The wear-and-tear theory states that body cells are damaged from years of hard use. The cross-linking theory relates changes of aging to cross-linked and connected cells and systems become hardened over time, decreasing function. Gould's theory is a psychosocial one looking at tasks the adult completes, not physical changes. —Senescence means biologic aging; there is no senescence theory of aging.

DIF: Remembering

OBJ: 18.1

TOP: Teaching/Learning

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance

NOT: Concepts: Development

2. A nurse is obtaining a history from a 37-year-old patient. What statement by the patient indicates that he has met the age-appropriate developmental task according to Gould?
 - a. Patient describes moving out of his parents' house into an apartment.
 - b. Patient reminisces about past life events and accomplishments.
 - c. Patient questions his life choices such as profession and decision not to marry.
 - d. Patient expresses satisfaction in having his own family and successful career.

ANS: D

According to Gould, this patient is in the midlife decade, which occurs after the upheaval of entering the adult world and questioning one's decisions, but prior to reconciling one's life and becoming stabilized. The patient who has moved out of his parents' house is demonstrating activities seen in the early adulthood stage in which leaving the parents' world is paramount. Reminiscing about the past life occurs as part of the reconciliation stage, seen in an older person. Questioning and reexamining are typically seen in the stage for 28- to 34-year olds.

DIF: Evaluating

OBJ: 18.1

TOP: Evaluation

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:

Concepts: Development

3. The nurse plans to develop a comprehensive screening tool to use with young adults, assessing their lifestyles and healthy living habits. What barrier must the nurse plan to overcome to make this screening successful?
 - a. Young adults may not see a health provider regularly.
 - b. Young adults are so diversified that a screening tool may not be appropriate.

- c. Young adults have too many risky lifestyle behaviors to make education relevant.
- d. Young adults are too busy with their lives to see a health care provider regularly.

ANS: A

Since young adults are at the peak of their physical development and abilities, they may not see a health care provider on a regular basis. Screening tools can be used with any population. When riskier behaviors are demonstrated, the more education is needed. Time constraints are generally not the main reason young adults do not have regular medical care.

DIF: Analyzing OBJ: 18.3 TOP: Analysis
 MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:
 Concepts: Development

4. A nurse who uses Havighurst's theory of development is assessing a young adult. What question does the nurse ask to provide the most relevant information about this person's successful negotiation of this developmental stage?
- a. —Do you find yourself doing familiar tasks in new ways to accomplish them?||
 - b. —Please count backwards from 100 by 7s, such as 100, 93, and so on.||
 - c. —What occupation have you chosen for your life's work?||
 - d. —Do you still have a good relationship with your parents and siblings?||

ANS: C

According to Havighurst, one of the tasks of the young adult is to select an occupation. Changing the approach one performs familiar tasks is seen in the middle adult stage. Seriation activities test fluid intelligence and are often used in middle adulthood. Although for Havighurst, achieving emotional independence from parents is important, that does not preclude having a good relationship with them.

NURSINGTB.COM

DIF: Applying OBJ: 18.1 TOP: Assessment
 MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:
 Concepts: Development

5. A nurse reads on a patient's chart that she has sarcopenia. What assessment does the nurse perform to confirm this?
- a. Mini-mental state exam
 - b. Tests of muscle strength
 - c. Gait and balance
 - d. Vision and hearing

ANS: B

—Sarcopenia|| means loss of tissue. Muscle tissue and muscle mass both tend to decrease starting in the 30s. The nurse assesses muscle strength to get information about possible sarcopenia in this patient. Tests of cognition, gait and balance, and sensory perception are not related.

DIF: Applying OBJ: 18.2 TOP: Assessment
 MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:
 Concepts: Development

6. A nurse is assessing a middle-aged adult for cognitive skills. The patient has difficulty with seriation tests. What action by the nurse is most appropriate?
- a. Document the findings and continue the assessment.

- b. Perform another test for fluid intelligence.
- c. Consult with the provider about dementia screening.
- d. Ask the patient about family medical history.

ANS: B

Middle-aged adults (especially younger ones) often have trouble with math processing (fluid intelligence) because of the prevalence of calculators and computers and dependence upon them to do work formerly done by the individual. The nurse should conduct other tests for fluid intelligence. The findings should be documented, but this is not the only action needed. Dementia screening is not indicated with one test result. Family medical history should be part of all screenings but is not directly related to this issue.

DIF: Applying OBJ: 18.5 TOP: Assessment

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:

Concepts: Development

7. A nurse working with a middle-aged adult is concerned that the adult is not meeting developmental tasks associated with Erikson's theory. What question by the nurse is most appropriate?
- a. Are there community organizations you would like to volunteer with?
 - b. Do your children come to see you on a regular basis?
 - c. Do you get at least 30 minutes of exercise most days of the week?
 - d. How do you feel about reading for a leisure time activity?

ANS: A

According to Erikson, this adult is in the Generativity versus Stagnation phase. Successful completion of the tasks associated with this stage includes reaching out to others beyond the nuclear family to community groups and society at large. Volunteering with an organization would be one way to meet the task. The other questions are related to individual-oriented behaviors.

DIF: Applying OBJ: 18.5 TOP: Assessment

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:

Concepts: Development

8. A nurse is planning a community event in which participants will be assessed for their risk of having a stroke. Which site does the nurse choose to access the highest risk population?
- a. Community elder center
 - b. African American church
 - c. Synagogue in a rural area
 - d. Asian American grocery store

ANS: B

African Americans have a higher rate of stroke death than do white Americans, even at younger ages. The nurse chooses the African American church for stroke screening. The other places will not have as large of a high-risk population.

DIF: Applying OBJ: 18.8 TOP: Implementation

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:

Concepts: Development

9. An adult caregiver for an older adult reports the adult is doing well other than sleeping more frequently and for longer periods. What response by the nurse is best?
- Assess the older adult for exercise habits.
 - Perform a screening for depression.
 - Reassure the caregiver that this is normal.
 - Ask the older adult to provide a sleep diary.

ANS: B

Depression is common in the older adult population and is frequently overlooked or misdiagnosed. People may think withdrawal and excessive sleeping are normal age-related changes, but they are not. The nurse should assess the older adult for depression. Other assessments can follow because they are not the priority for this patient.

DIF: Applying OBJ: 18.8 TOP: Assessment

MSC: NCLEX Client Needs Category: Psychosocial Integrity NOT: Concepts: Development

10. A community nurse is working with a family that consists of a middle-aged adult, an older parent with dementia, and two school-aged children. Which assessment by the nurse is most important for this family?
- Stress-relieving methods
 - Child care arrangements
 - Functional ability of the older adult
 - Knowledge of health screening needs

ANS: A

Burnout can occur when caring for an older adult with dementia because their needs are great without lessening over time. Caring for both an older adult and school-aged children (often called the sandwich phenomenon or generation) adds even more stress. The priority assessment for this family is methods used to reduce stress.

DIF: Applying OBJ: 18.8 TOP: Assessment

MSC: NCLEX Client Needs Category: Psychosocial Integrity NOT: Concepts: Development

11. A young nursing student is assessing an older patient. The student nurse questions if a sexual history needs to be taken. What response by the faculty is best?
- Since procreation is not an issue, you do not need to discuss this.
 - Only discuss this topic if you are comfortable in doing so.
 - Ask the patient if he/she wants to talk about sexuality.
 - Sexuality is a basic human need and needs to be assessed.

ANS: D

Sexuality is a basic human need. The faculty should tell the student to complete the assessment. Procreation is not an issue currently; however, this does not eliminate the need to discuss sexual issues. Asking permission may be an important part of taking a sexual history, but that response is implying the student can —get out of the assessment if the patient is agreeable. The student needs practice to improve his/her comfort with this assessment.

DIF: Applying OBJ: 18.7 TOP: Teaching/Learning

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT: Concepts: Development

12. The nurse working with older adults encourages them to stay healthy. What instruction by the nurse takes priority?
- Eat at least seven servings of produce a day.
 - Get at least 8 hours of sleep a night.
 - Get some exercise at least most days of the week.
 - Stay away from people who are ill.

ANS: D

One normal age-related change seen in the older adult is decreased immune function. The older adult should place high priority on avoiding illness by staying away from people who are sick and avoiding large crowds during peak communicable illness periods. The other instructions are also relevant but do not address this age-related change.

DIF: Applying OBJ: 18.8 TOP: Teaching/Learning MSC:
NCLEX Client Needs Category: Health Promotion and Maintenance NOT:
Concepts: Development

13. The nurse is performing wellness checks at a community center for older adults. Which person would the nurse evaluate as having the highest risk of stroke?
- Caucasian, 55 years of age, BP 148/92 mm Hg
 - African American, 70 years of age, BP 150/100 mm Hg
 - Asian American, 40 years of age, BP 146/78 mm Hg
 - Caucasian, 74 years of age, BP 150/82 mm Hg

ANS: B

African Americans have a higher rate of stroke than whites at any age. Hypertension is also a risk factor for stroke. Stroke risk also increases with age overall. Therefore, the person with the highest risk of stroke is the older hypertensive African American.

DIF: Analyzing OBJ: 18.8 TOP: Analysis
MSC: NCLEX Client Needs Category: Physiological Integrity NOT: Concepts: Perfusion

14. The nurse working with an adult population knows that many age-related declines in function begin occurring at what age?
- 20
 - 30
 - 50
 - 70

ANS: B

Many age-related functions peak before age 30 and begin to decline after that.

DIF: Remembering OBJ: 18.2 TOP: Assessment
MSC: NCLEX Client Needs Category: Health Promotion and Maintenance
NOT: Concepts: Development

15. A young adult asks the nurse why she should participate in health screening and educational events. What response by the nurse is best?
- Your choices now affect your future health.¶
 - It's free and full of good information.¶
 - Wouldn't you want to know if you had a problem?¶
 - You can change bad habits now if you know about them.¶

ANS: A

Health behaviors entrenched in the young adult stage impact future health and well-being. While these events are free and full of information and bad habits can be changed if the person has knowledge and motivation, those responses do not give the person useful information. Asking if the person wants to know about health problems sounds accusatory.

DIF: Understanding

OBJ: 18.4

TOP: Teaching/Learning

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance

NOT: Concepts: Development

16. A young adult tells the nurse he has quit smoking cigarettes and now —vapes (uses electronic cigarettes [e-cigarettes]). What response by the nurse is best?
- Excellent! That is so much better for you than tobacco.
 - The health consequences of e-cigarettes are not known.
 - Using e-cigarettes actually is much worse for your health.
 - Tobacco or e-cigarettes ... doesn't matter. You need to quit.

ANS: B

The health consequences of using e-cigarettes are not yet known because they are new products. The nurse educates the young adult to this fact.

DIF: Applying

OBJ: 18.4

TOP: Teaching/Learning

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance

NOT: Concepts: Health Promotion

17. A nurse notes an older adult puts excessive amounts of salt on her food. What intervention by the nurse is best?
- Teach the adult how salt intake relates to hypertension.
 - Ask the older adult why she puts so much salt on food.
 - Encourage the older adult to use less salt on her food.
 - Explore other herbs and flavor enhancers with the adult.

ANS: D

Older adults tend to lose their sense of taste and smell. Food becomes less attractive to them and they may respond by adding salt. The nurse who understands this concept will help the older adult explore other flavor enhancers for food. Teaching about the relationship of sodium to hypertension is important but does not address the problem. Encouraging the adult to use less salt does not give her a strategy to do so. Asking —why questions is a communication barrier that often causes people to become defensive.

DIF: Applying

OBJ: 18.7

TOP: Implementation

MSC: NCLEX Client Needs Category: Health Promotion and Development

NOT: Concepts: Development

18. A nurse in the family practice clinic is assessing an older adult who has dementia. The adult daughter/caregiver expresses concern that the parent should no longer be left alone while the daughter is at work. What response by the nurse is best?
- Refer the family to a social worker.
 - Encourage the daughter to look into nursing homes.
 - Tell the daughter there are medications for dementia.
 - Help the daughter explore adult day care options.

ANS: D

Adult day care facilities offer care of the older person during the working hours. This might be a good option for the family. A social worker can help, but the nurse should be active in problem solving with the daughter. Medications are available for dementia, but dementia remains a progressive disorder, so this does not help solve the problem. A nursing home may not yet be needed.

DIF: Applying OBJ: 18.8 TOP: Communication and Delegation
 MSC: NCLEX Client Needs Category: Safe and Effective Care Environment NOT:
 Concepts: Safety

19. An adult child brings his father to the emergency department and describes the sudden onset of a panic attack and aggressiveness. After ruling out an infectious process, what action by the nurse is best?
- Assess the patient for mental illness.
 - Perform a mini-mental state exam on the patient.
 - Ask about risk factors for delirium.
 - Assess the patient for illicit drug use.

ANS: D

Abuse of illicit drugs can cause many symptoms, including panic attacks and aggressive behavior. After assessing for an infectious process, the nurse should determine if the patient has used any recreational drugs. The other assessments are not as important and can be completed later.

DIF: Applying OBJ: 18.8 TOP: Assessment
 MSC: NCLEX Client Needs Category: Physiological Integrity NOT: Concepts: Safety

NURSINGTB.COM
 USNT O

MULTIPLE RESPONSE

1. The nurse knows that which attributes are characteristics of the young adult age-group? *(Select all that apply.)*
- The number of high school graduates going to college is decreasing.
 - More than 88% of people aged 25 to 34 have completed high school.
 - More males aged 20 to 24 were married than females in the same age-group.
 - A significant percentage of those aged 25 to 34 has advanced degrees.
 - Adult roles for the young adult are more diverse than for other age-groups.

ANS: B, E

More than 88% of people aged 25 to 34 have completed high school. Adult roles, which are influenced by many factors, are diverse and are not normed for this age-group. The number of high school graduates going to college is increasing. More females than males aged 20 to 24 are married. Only about 9% of those 25 to 34 have advanced degrees.

DIF: Remembering OBJ: 18.3 TOP: Teaching/Learning
 MSC: NCLEX Client Needs Category: Health Promotion and Maintenance
 NOT: Concepts: Development

2. The nurse is planning an educational workshop on health risks for the young adult. What topics does the nurse plan to include as priorities? *(Select all that apply.)*

- a. Sexually transmitted diseases
- b. Falling
- c. Responsible alcohol use
- d. Intimate partner and sexual violence
- e. Distracted driving

ANS: A, C, D, E

Health risks for this population include sexually transmitted diseases, alcohol and illicit drug use, violence, and distracted driving. Fall prevention is more appropriately directed toward an older audience.

DIF: Remembering

OBJ: 18.4

TOP: Teaching/Learning

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance

NOT: Concepts: Development

3. The nurse working in a family practice clinic has very limited time to assess patients for health concerns. When working with middle-aged patients, which problems does the nurse assess for as the priorities? (*Select all that apply.*)
- a. Heart disease
 - b. Cancer
 - c. Sexually transmitted diseases
 - d. Stroke
 - e. Functional abilities

ANS: A, B, D

Specific health concerns for this age-group include cardiovascular disease and cancer. The nurse should assess for heart disease, stroke, and cancer. Sexually transmitted diseases can occur in any group but is more a priority for the young adult. Functional abilities are more a priority for the older adult.

DIF: Understanding

OBJ: 18.6

TOP: Assessment

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance

NOT: Concepts: Development

4. The nurse tells the student that which disorders are related to the presence of free radicals? (*Select all that apply.*)
- a. Cancer
 - b. Cataracts
 - c. Glaucoma
 - d. Arthritis
 - e. Liver disease

ANS: A, B, D

Free radicals are naturally occurring chemicals that can cause cellular damage. They are implicated in such diseases as cancer, cataracts, and arthritis. They are not implicated as a causative factor in glaucoma and liver disease.

DIF: Remembering

OBJ: 18.2

TOP: Teaching/Learning

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance

NOT: Concepts: Inflammation

5. The student of adult development learns that cognitive abilities improve during the young adult stage because of the influence of which experiences? *(Select all that apply.)*
- a. Physical growth of the brain
 - b. Formal education
 - c. Occupational training
 - d. Overall life experiences
 - e. Specific profession chosen

ANS: B, C, D

Formal education, occupational training, and overall life experiences contribute to refining cognitive skills such as rational thinking and problem solving. Physical growth of the brain and specific profession chosen are not as directly related.

DIF: Remembering

OBJ: 18.3

TOP: Teaching/Learning

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance

NOT: Concepts: Development

6. The nurse is assessing hospitalized older adults for risk factors that could lead to delirium. For which patients does the nurse plan extra care to prevent delirium? *(Select all that apply.)*
- a. A 95 year old
 - b. On multiple pain medications
 - c. Is blind
 - d. 2 days postoperative
 - e. Intractable pain

ANS: A, B, D, E

There are several risk factors for developing delirium, including advanced age, polypharmacy, pain, surgery, and hospitalization. Being blind is not a risk factor.

DIF: Applying

OBJ: 18.8

TOP: Assessment

MSC: NCLEX Client Needs Category: Physiological Integrity

NOT: Concepts: Cognition

Chapter 19: Vital Signs**Yoost & Crawford: Fundamentals of Nursing: Active Learning for Collaborative Practice, 3rd Edition****MULTIPLE CHOICE**

1. A nurse notes a patient has abnormal vital signs. What action by the nurse is best?
 - a. Document the findings.
 - b. Notify the provider.
 - c. Compare with prior readings.
 - d. Retake the vital signs in 15 minutes.

ANS: C

Individual vital signs are not as important as the trends. For instance, a patient may have a blood pressure higher than —normal that is normal for the patient. Trends give more useful information than a single reading. Documentation is important, but the nurse needs to do more. If the readings are significantly abnormal, the provider should be notified. The nurse may retake the vital signs if he/she is not confident of the first set of measurements, but should not wait for time to pass.

DIF: Applying OBJ: 19.1 TOP: Assessment
 MSC: NCLEX Client Needs Category: Physiological Integrity: Reduction of Risk Potential NOT:
 Concepts: Perfusion, Gas Exchange

2. A patient returned from a procedure and has vital sign measurements ordered every hour. The patient's blood pressure has dropped from 132/82 mm Hg an hour ago to 90/66 mm Hg. What priority action by the nurse is most appropriate?
 - a. Take the vital signs again in another hour.
 - b. Document the findings in the patient's chart.
 - c. Have another nurse recheck the vital signs.
 - d. Plan to take the vital signs more often.

ANS: D

The nurse uses clinical judgment to determine how often the patient's vital signs should be checked when there is a change in patient condition. The nurse should plan to assess vital signs more often in this patient. Since this is a significant change, the nurse should not wait another hour even though this is what the provider prescribed. It is not necessary for another nurse to double-check the vital signs. Documentation needs to occur, but the priority is to plan to take the vitals more often.

DIF: Applying OBJ: 19.1 TOP: Assessment
 MSC: NCLEX Client Needs Category: Physiological Integrity: Reduction of Risk Potential NOT:
 Concepts: Perfusion, Gas Exchange

3. A nurse is told in the hand-off report that a patient is afebrile. What assessment finding correlates with this statement?
 - a. Blood pressure 152/98 mm Hg
 - b. Temperature 98.4 °F (36.8 °C)
 - c. Apical pulse 82 beats/min
 - d. Respirations 16 breaths/min

ANS: B

A temperature of 98.4 °F is normal. —Afebrile means having a normal temperature. The other readings are not related to this term.

DIF: Remembering

OBJ: 19.2

TOP: Assessment MSC:

NCLEX Client Needs Category: Physiological Integrity: Reduction of Risk Potential NOT:

Concepts: Thermoregulation

4. A nurse is caring for a patient who has an elevated temperature. The nurse plans to help the patient regain a normal temperature through conduction. What technique does the nurse use?
- Placing a cooling fan in the patient's room
 - Putting ice packs in the patient's axillae
 - Spraying the patient with a fine mist of water
 - Turning the temperature down in the room

ANS: B

Conduction is the transfer of heat through direct contact with another object, such as an ice pack. A cooling fan would help lower temperature by convection. Spraying the patient with a mist of water would lead to evaporative cooling. Turning the temperature down is an example of radiation.

DIF: Applying

OBJ: 19.2

TOP: Implementation

MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:

Concepts: Thermoregulation

5. A nurse is going to take an oral temperature on a patient who has just consumed a cup of coffee. What action by the nurse is best?
- Have the patient drink room temperature water.
 - Return in 30 minutes to take the patient's temperature.
 - Take the patient's temperature rectally instead.
 - Document that temperature is unable to be obtained.

ANS: B

Oral temperatures will be inaccurate if the patient has been drinking or eating hot or cold foods. The nurse instructs the patient not to continue drinking the coffee and returns in 30 minutes to take the temperature. Drinking room temperature water will not —even out the patient's mouth temperature. The rectal route is not preferred by patients and should not be used in this situation. The nurse needs a temperature and so should not document that it was not obtained.

DIF: Applying

OBJ: 19.2

TOP: Assessment

MSC: NCLEX Client Needs Category: Health Promotion and Management NOT:

Concepts: Thermoregulation

6. A nurse observes a student taking an adult patient's tympanic temperature. What action by the student requires the nurse to intervene?
- Student washes hands prior to patient contact.
 - Student pulls the pinna of the patient's ear down and back.
 - Student explains the procedure to the patient.
 - Student pulls the pinna of the patient's ear up and back.

ANS: B

For an adult, the correct procedure for taking a tympanic temperature includes pulling the pinna of the patient's ear up and back. Children's pinnae are pulled down and back. Washing hands and explaining the procedure are appropriate.

DIF: Applying OBJ: 19.2 TOP: Implementation
 MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:
 Concepts: Thermoregulation

7. A nurse assesses a patient's radial pulse rate to be 110 beats/min and regular. What action by the nurse is best?
- Assess the patient for causes of tachycardia.
 - Take an apical heart rate and compare the two.
 - Document the findings in the patient's chart.
 - Notify the patient's health care provider.

ANS: A

Tachycardia (rapid heart rate) is often caused by factors such as pain, anxiety, fever, or fluid volume alterations. The nurse should assess the patient thoroughly for possible causative factors. Since the pulse is regular, there is no reason to take an apical pulse. The findings should be documented, but the nurse needs to do more. The provider may or may not need to be notified, depending on the outcome of the nurse's assessment.

DIF: Applying OBJ: 19.2 TOP: Assessment
 MSC: NCLEX Client Needs Category: Physiological Integrity: Reduction of Risk Potential NOT:
 Concepts: Perfusion

8. The student nurse is assessing a patient's pulses. What action by the student requires the nurse to intervene?
- Assessing apical pulse between the fifth and sixth intercostal spaces
 - Assessing the dorsalis pedis pulse by palpating behind the patient's knee
 - Assessing the radial pulse on the patient's wrist
 - Assessing the brachial pulse on the patient's inner elbow

ANS: B

The dorsalis pedis pulse is palpated on the top of the foot. The other assessment locations and pulses are correct.

DIF: Applying OBJ: 19.3 TOP: Assessment
 MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:
 Concepts: Perfusion

9. The nurse assesses a patient's pulse and finds it hard to obliterate with palpation. What action by the nurse is the most appropriate?
- Assess the patient for fluid volume overload.
 - Assess the patient for fluid volume deficit.
 - Assess the patient's apical heart rate.
 - Assess the patient's pulse deficit.

ANS: A

A pulse that is hard to obliterate (a bounding pulse) can be caused by fluid volume overload, or overhydration. The nurse should assess for this situation. The other actions are not necessary.

DIF: Applying OBJ: 19.3 TOP: Assessment
 MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:
 Concepts: Perfusion

10. The nursing faculty member is observing a student taking a patient's carotid pulse. What action by the student requires intervention by the faculty member?
- Counts pulse for 30 seconds and multiplies by two.
 - Performs hand hygiene prior to patient contact.
 - Compares pulses in both carotid arteries at the same time.
 - Assesses pulse on one side then assesses the other side.

ANS: C

The carotid arteries are the main supply route of blood to the brain. Compressing both sides of the carotid arteries at the same time can lead to ischemia. The other actions are appropriate.

DIF: Applying OBJ: 19.3 TOP: Assessment
 MSC: NCLEX Client Needs Category: Physiological Integrity: Reduction of Risk Potential NOT:
 Concepts: Perfusion

11. A nurse is caring for a patient who has orthopnea. What action by the nurse is most appropriate?
- Encourage deep breathing and coughing.
 - Medicate the patient for pain as needed.
 - Keep the head of the bed elevated.
 - Monitor the length of time the patient doesn't breathe.

ANS: C

Orthopnea is difficulty breathing in positions other than sitting up. To assist the patient who has orthopnea, the nurse keeps the head of the bed elevated to ease breathing.

DIF: Applying OBJ: 19.4 TOP: Implementation
 MSC: NCLEX Client Needs Category: Physiological Integrity: Basic Care and Comfort NOT:
 Concepts: Gas Exchange

12. The nurse has applied a pulse oximeter to the finger of a patient who is hypothermic. The pulse oximeter does not provide a good reading. What action by the nurse is best?
- Move the oximeter probe to another finger.
 - Assess the fingers for good circulation.
 - Document that the reading cannot be obtained.
 - Remove any fingernail polish present on the fingernail.

ANS: B

A patient who is hypothermic may not have good circulation to the extremities. The nurse should assess the patient's circulation, and if it is poor to the extremities, choose another spot at which to measure the oxygen saturation. Moving the probe to another finger or removing nail polish will not help if the problem is poor circulation. The nurse should document appropriately but needs to do more than just charting that the reading could not be obtained.

DIF: Applying OBJ: 19.4 TOP: Implementation
 MSC: NCLEX Client Needs Category: Physiological Integrity: Reduction of Risk Potential NOT:
 Concepts: Perfusion

13. A patient's blood pressure is 142/76 mm Hg. What does the nurse chart as the pulse pressure?
- 28
 - 42
 - 58
 - 66

ANS: D

The pulse pressure is the difference between the systolic and diastolic blood pressure readings. In this case, $142 - 76 = 66$.

DIF: Applying OBJ: 19.5 TOP: Assessment
 MSC: NCLEX Client Needs Category: Physiological Integrity: Reduction of Risk Potential NOT:
 Concepts: Perfusion

14. A nurse performs orthostatic blood pressure readings on a patient with the following results: lying 148/76 mm Hg, standing 110/60 mm Hg. What action by the nurse is best?
- Instruct the patient not to get up without help.
 - Document the findings and continue to monitor.
 - Reassure the patient that these findings are normal.
 - Reassess the blood pressures in 1 hour.

ANS: A

This patient has orthostatic hypotension, which is a drop of 20 mm Hg in systolic reading and 10 mm Hg in diastolic reading when the patient stands up from a sitting or lying position. The patient's cardiovascular system does not compensate for this, so the patient is at risk of becoming dizzy and fainting. The nurse instructs the patient to call for assistance before getting up to prevent a fall. The nurse should document the findings but needs to do more. These findings are not normal, so the nurse should not tell the patient that they are. The patient may need to be assessed sooner than 1 hour.

DIF: Analyzing OBJ: 19.5 TOP: Implementation
 MSC: NCLEX Client Needs Category: Physiological Integrity: Reduction of Risk Potential NOT:
 Concepts: Perfusion, Safety

15. The nurse is caring for a woman who had a right-sided mastectomy 2 years ago. What action by the nurse is most appropriate?
- Place a sign above the bed: —No blood pressures on the right arm.||
 - Place a sign above the bed: —No continuous blood pressures on the right arm.||
 - Place a sign above the bed: —Blood pressures in legs only.||
 - No specific action is needed for this situation.

ANS: A

After a mastectomy or after lymph nodes have been removed, the patient should not have blood pressures taken on the operative side. Doing so can cause lymphedema. The nurse communicates this to all staff with a sign stating that no blood pressures are to be taken on the right side. The other actions are not warranted.

DIF: Applying OBJ: 19.5

TOP: Integrated Process: Communication and Documentation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Safety and Infection Control

NOT: Concepts: Perfusion

16. Which patient assessment result would require the nurse to assess that patient further?
- A 40-year-old woman with a radial pulse of 68
 - A 65-year-old man with a respiratory rate of 10
 - A 12-year-old with a pulse of 92 after ambulating in the hallway
 - A 50-year-old man with a BP of 112/60 upon awakening in the morning

ANS: B

The normal respiratory rate is 12 to 20 breaths/min for an adult, so a rate of 10 would require further assessment. The other options are all within normal limits.

DIF: Analyzing OBJ: 19.4 TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT: Concepts: Perfusion

17. The nurse receives a hand-off report on four patients. Which patient finding should the nurse assess first?
- Pulse oximetry 96%
 - Blood pressure 102/62 mm Hg
 - Pulse 42 beats/min
 - Respiratory rate 18 breaths/min

ANS: C

A pulse of 42 beats/min is considered bradycardia and the patient should be assessed first because perfusion could be compromised. The blood pressure, pulse oximetry, and respiratory rate are normal.

DIF: Analyzing OBJ: 19.1 TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT: Concepts: Perfusion

18. A nursing student is caring for a patient with metabolic acidosis. The student asks the registered nurse why the patient's respiratory rate is so high. What response by the nurse is best?
- The patient's metabolic rate is increased from being ill.¶
 - The lungs are trying to rid the body of extra carbon dioxide.¶
 - The patient is trying to reduce his temperature through panting.¶
 - Patients who are acutely ill often have abnormal vital signs.¶

ANS: B

The body tries to compensate for excess carbon dioxide (seen in acidosis) by increasing the rate and depth of respirations to —blow off¶ the carbon dioxide.

DIF: Understanding OBJ: 19.4

TOP: Integrated Process: Teaching/Learning

MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT: Concepts: Gas Exchange

MULTIPLE RESPONSE

1. The nursing student learns that the purpose of measuring a patient's vital signs includes which of the following rationale? (*Select all that apply.*)
 - a. Monitor body systems functioning.
 - b. Identify early signs of problems.
 - c. Evaluate effectiveness of interventions.
 - d. Determine if a cure has been obtained.
 - e. Provide a baseline to compare against.

ANS: A, B, C, E

Vital signs give information on the functioning of body systems, can lead the nurse to identify early signs of problems, can be used to evaluate the effectiveness of interventions, and provide a baseline to compare against subsequent readings. They are not used to solely determine if a disease has been cured.

DIF: Remembering

OBJ: 19.1

TOP: Assessment MSC:

NCLEX Client Needs Category: Physiological Integrity: Reduction of Risk Potential NOT:

Concepts: Perfusion, Gas Exchange

2. The nurse is delegating taking vital signs to an unlicensed assistive personnel (UAP). What instructions does the nurse provide the UAP? (*Select all that apply.*)
 - a. —Let me know if Mr. Smith's blood pressure is low.||
 - b. —Take Mrs. Jones' blood pressure every 15 minutes.||
 - c. —Call me if Ms. Walsh's systolic blood pressure drops to under 100 mm Hg.||
 - d. —Do you want me to demonstrate using the electronic blood pressure cuff?||
 - e. —I'll take Mr. Derby's blood pressure since he is not stable.||

ANS: B, C, D, E

NURSINGTB.COM

The nurse can delegate measuring vital signs to UAPs if the patient is stable. The nurse must ensure the UAP knows the proper technique for taking vital signs and knows which readings must be reported. Telling the UAP to report a blood pressure that is —too low|| is too vague.

DIF: Applying OBJ: 19.1

TOP: Integrated Process: Communication and Documentation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Clinical Judgment

3. The nurse understands that which factors can increase blood pressure? (*Select all that apply.*)
 - a. Head injury
 - b. Decreased fluid volume
 - c. Increasing age
 - d. Recent food intake
 - e. Pain

ANS: A, C, D, E

Head injury, increasing age, recent food intake, pain, and increased (not decreased) fluid volume all can increase blood pressure.

DIF: Remembering

OBJ: 19.5

TOP: Assessment MSC:

NCLEX Client Needs Category: Physiological Integrity: Reduction of Risk Potential NOT:

Concepts: Perfusion

4. The nurse assessing respirations understands that problems in what organs can directly affect the process of respiration? (*Select all that apply.*)
- a. Brain
 - b. Lungs
 - c. Heart
 - d. Liver
 - e. Skeletal muscle

ANS: A, B, C

Problems in the brain, heart, and lungs can directly lead to changes in respiratory rate and effort. Problems in the liver and skeletal muscle do not affect respirations directly.

DIF: Remembering

OBJ: 19.4

TOP: Assessment MSC:

NCLEX Client Needs Category: Physiological Integrity: Reduction of Risk Potential NOT:

Concepts: Gas Exchange

5. A nurse is teaching a patient and the patient's family about self-care measures for hypertension. Which topics should the nurse include? (*Select all that apply.*)
- a. Increase exercise on most days.
 - b. Maintain a normal body weight.
 - c. Abstain from any alcohol.
 - d. Reduce dietary sodium to 2.4 g/day.
 - e. Follow the DASH diet.

ANS: B, D, E

Self-care measures for hypertension include 30 minutes of aerobic exercise on most days of the week, maintaining a normal body weight, limiting alcohol to two drinks/day for men and one drink/day for women, reducing sodium intake to 2.4 g/day, and following the DASH diet.

DIF: Understanding

OBJ: 19.5

TOP: Integrated Process: Teaching/Learning

MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:

Concepts: Perfusion

6. Which patient-specific factors does the nurse include when assessing pulse? (*Select all that apply.*)
- a. Age
 - b. Gender
 - c. Religion
 - d. Exercise
 - e. Medications

ANS: A, B, D, E

The nurse should consider several patient-specific factors when assessing the pulse, age, gender, exercise, presence of fever, medications, fluid volume status, stress, and underlying disease processes. Religion is not an appropriate response.

DIF: Remembering

OBJ: 19.3

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Perfusion

7. The nurse is performing a morning assessment and notes the patient to be experiencing dyspnea. Which patient assessment findings would most indicate this respiratory condition? (*Select all that apply.*)
- a. Occasional productive cough
 - b. Pulse oximetry 89%
 - c. Patient in orthopneic position
 - d. Respirations 26 & shallow
 - e. Temperature 100.1 °F

ANS: B, C, D

Dyspnea is difficult, labored breathing, usually with a rapid, shallow pattern, that may be painful. Anxiety usually is present as well. Accessory muscles in the chest and neck are used in dyspneic breathing. Many patients experiencing dyspnea find it easier to breathe in an upright position. Difficulty breathing experienced in positions other than sitting or standing is termed orthopnea. Occasional productive cough and slight temperature elevation are not indicators of dyspnea.

DIF: Applying OBJ: 19.4 TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Gas Exchange

NURSINGTB.COM

Chapter 20: Health History and Physical Assessment**Yoost & Crawford: Fundamentals of Nursing: Active Learning for Collaborative Practice, 3rd Edition**

MULTIPLE CHOICE

1. A nurse is conducting a physical assessment in a clinic with a partly undressed patient. What action by the nurse is most appropriate?
- Offer the patient a small pillow for under his/her head.
 - Provide a method for ensuring the patient stays warm.
 - Raise the head of the bed to about 30 degrees.
 - Ensure there is enough lighting for an adequate examination.

ANS: B

The important fact in this question is that the patient is partly undressed, and the nurse provides a means to keep the patient warm. All answers are appropriate for any examination but keeping the patient warm is specific to this situation.

DIF: Applying OBJ: 20.2 TOP: Implementation
MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:
Concepts: Communication

2. A patient wishes to review his medical record. What response by the nurse is best?
- I'm sorry, we don't allow you to look at your chart.||
 - Let me check to see if we can allow you to do that.||
 - Yes, I can sit with you while you look at it, so you can ask questions.||
 - Yes, all patients can review their charts any time they wish.||

ANS: C

Patients have the right to look at their records. It is best if a health care provider is present to answer any questions the patient may have or to help interpret any information found within the record.

DIF: Applying OBJ: 20.2
TOP: Integrated Process: Communication and Documentation
MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
Concepts: Communication

3. A clinic nurse is examining an older, confused patient on an examination table and realizes a piece of needed equipment was left outside in the hall. What action by the nurse is best?
- Tell the patient to lie still and go get the equipment.
 - Call for another staff member to bring the equipment.
 - Have the patient get into a chair and get the equipment.
 - Finish the rest of the exam, get the equipment, and use it.

ANS: B

For patient safety, some patients should never be left alone on an examination table: infants; small children; older adults who are confused, combative, or uncooperative, and people who are physically or chemically restrained. The nurse calls for another staff member to get the missing equipment. Getting up and down off the table is inconvenient and may be difficult. Finishing the exam and then retrieving the piece of equipment also involves the patient changing locations and is inconvenient for the patient.

DIF: Applying OBJ: 20.2 TOP: Implementation
 MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Safety and Infection Control
 NOT: Concepts: Safety, Clinical Judgment

4. A student nurse is preparing to auscultate a patient's lungs. What action by the student leads the instructor to intervene?
- Student asks to turn the television volume down.
 - Student warms the bell of the stethoscope before use.
 - Student uses the stethoscope bell to listen to bowel sounds.
 - Student places the stethoscope diaphragm on the patient's skin.

ANS: C

The diaphragm is used to listen to bowel sounds. The other actions are appropriate.

DIF: Applying OBJ: 20.3 TOP: Assessment
 MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:
 Concepts: Clinical Judgment

5. The nurse is assessing a patient's alcohol intake. What question is most appropriate?
- Do you drink alcohol at all?
 - You don't drink much, do you?
 - When was your last drink?
 - How much alcohol do you drink daily?

ANS: D

The nurse needs to be nonjudgmental when inquiring about topics that might be sensitive, such as alcohol or drug use. The nurse asks a neutral, objective question such as —How much alcohol do you drink daily? that allows the patient to quantify the intake. Avoid yes/no questions because they are closed ended and do not lead to further discussion or disclosure. Avoid a negatively charged question such as, —You don't drink much, do you?; this demonstrates the nurse's displeasure with drinking. Asking when the last drink was is not as important in a general survey as quantifying the amount of intake.

DIF: Applying OBJ: 20.4 TOP: Assessment
 MSC: NCLEX Client Needs Category: Psychosocial Integrity NOT: Concepts: Communication

6. The nurse is planning to educate four patients on preventing skin cancer and early warning signs. Which patient is the priority for this education?
- Adolescent who uses a tanning bed
 - Middle-aged adult who walks for fitness
 - Older woman who sits in the sun for 10 minutes daily
 - Person who works indoors under fluorescent lights

ANS: A

Research indicates that indoor tanning before the age of 35 increases a person's risk for the deadliest form of skin cancer, melanoma, by 59% with each exposure. The adolescent who tans is the highest priority for this education. The others do not have as high a risk.

DIF: Understanding OBJ: 20.5
 TOP: Integrated Process: Teaching/Learning
 MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:
 Concepts: Tissue Integrity

7. A nurse has assessed a patient's capillary refill, which was 5 seconds. What action by the nurse is most appropriate?
- Document the findings and continue the examination.
 - Ask the patient about the use of artificial nails.
 - Ask the patient about his/her occupation.
 - Assess the patient for signs of hypoxia.

ANS: D

Normal capillary refill is 2 to 3 seconds. Prolonged capillary refill can indicate hypoxia, anemia, or circulatory insufficiency. The nurse should document the findings, but further action is not needed. Asking about artificial nails and occupation are not warranted.

DIF: Applying OBJ: 20.5 TOP: Assessment
 MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:
 Concepts: Clinical Judgment

8. The student nurse asks if it matters whether a healthy eye or a diseased eye should be examined first. What response by the faculty is best?
- Diseased eye first because it is the priority
 - Healthy eye first to prevent spread of disease
 - It does not matter if both eyes are examined
 - Start with the eye the patient wants you to start with

ANS: B

To prevent cross contamination, the healthy eye is examined before the diseased eye.

DIF: Understanding OBJ: 20.5 TOP: Teaching/Learning
 MSC: NCLEX Client Needs Category: Health Promotion and Maintenance
 NOT: Concepts: Infection

9. A nurse observes a patient sitting up in bed, leaning forward with the arms braced against the over-the-bed table. What action by the nurse is best?
- Assess the patient for a barrel-chest appearance.
 - Palpate the patient's abdomen for tenderness.
 - Inspect the patient's spine for deformities.
 - Ask the patient if he/she is experiencing dizziness.

ANS: A

This patient is sitting in a tripod position, often seen in patients with chronic obstructive pulmonary disease. These patients also often have a barrel-chest appearance, so the nurse assesses for this finding. The other actions are not related to a tripod position.

DIF: Applying OBJ: 20.5 TOP: Assessment

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:
Concepts: Gas Exchange

10. The nurse is assessing a patient whose chart indicates a Grade 3 heart murmur. What action is best to hear the murmur?
- Ensure that the room is extremely quiet.
 - Use a specialized stethoscope with amplification.
 - Auscultate the patient's chest with a stethoscope.
 - Place the stethoscope diaphragm on the patient's back.

ANS: C

A Grade 3 murmur should be readily heard with a regular stethoscope. Although the room should be quiet for all auscultation tasks, an —extremely quiet room and an amplification stethoscope should not be necessary. The bell of the stethoscope is usually used to listen to heart murmurs, but the stethoscope needs to be on the patient's chest.

DIF: Applying OBJ: 20.5 TOP: Assessment

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:
Concepts: Perfusion, Clinical Judgment

11. A nurse has conducted an Allen's test on a patient and the result was 8 seconds. What action by the nurse is best?
- Document the findings and continue the assessment.
 - Notify the health care provider immediately.
 - Elevate the patient's arm above the level of the heart.
 - Assess the patient for other signs of circulatory problems.

ANS: A

After the hand blanches in an Allen's test, when the nurse releases the pressure, normal color should return within 10 seconds. This patient's findings were normal, so the nurse should document the results and continue with the assessment. The other actions are not needed.

DIF: Applying OBJ: 20.5 TOP: Assessment

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:
Concepts: Perfusion, Clinical Judgment

12. A hospitalized patient complains of bilateral leg pain and asks the nurse to massage her legs. One calf is noticeably larger than the other and is warm and slightly reddened. What action by the nurse is best?
- Only massage the leg with normal assessment findings.
 - Massage the front of both legs and avoid the posterior surfaces.
 - Perform a Homan's test to both legs prior to massaging either of them.
 - Educate the patient on why a massage would be contraindicated.

ANS: D

This patient has manifestations of a deep vein thrombosis, and the nurse should not massage the patient's legs. The nurse should inform the patient of why this is contraindicated. The other actions are not warranted.

DIF: Applying OBJ: 20.5 TOP: Integrated Process: Teaching/Learning

MSC: NCLEX Client Needs Category: Physiological Integrity: Reduction of Risk Potential NOT:
Concepts: Perfusion

13. A nurse is told in hand-off report that a patient opens eye spontaneously, is confused but able to answer questions, and demonstrates purposeful movement to painful stimuli. What does the nurse calculate the patient's Glasgow Coma Scale to be?
- 7
 - 9
 - 11
 - 13

ANS: D

This patient's eye opening would be scored 4, verbal response would be 4, and motor response would be scored at 5; this equals a score of 13.

DIF: Analyzing OBJ: 20.5 TOP: Assessment

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:
Concepts: Cognitive Function

14. A nurse is assessing a patient's abdomen and hears bowel sounds every 20 to 25 seconds. What action by the nurse is best?
- Avoid palpating this patient's abdomen.
 - Document the findings in the patient's chart.
 - Have another nurse verify the findings.
 - Ask the patient when the last food intake was.

ANS: B

These findings are normal; it may take up to 30 seconds of listening to hear bowel sounds. The nurse documents the findings; no other action is needed.

DIF: Applying OBJ: 20.3 TOP: Assessment

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:
Concepts: Elimination

15. A nurse is assisting a patient who is having an examination of the female genitalia. What action by the nurse is best?
- Get the provider; assist patient into lithotomy position.
 - Assist the patient into lithotomy position; get the provider.
 - Get the provider; assist patient into Sims position.
 - Assist the patient into Sims position; get the provider.

ANS: A

The lithotomy position is used to examine female genitalia. It is an uncomfortable and embarrassing position, so the nurse ensures time spent in that position is limited. The nurse gets the provider, then assists the patient into the position. The Sims position is used to examine the rectal and perineal areas.

DIF: Applying OBJ: 20.5 TOP: Implementation

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:
Concepts: Clinical Judgment

16. A nurse assesses a patient's lungs and notes the presence of low-pitched snoring sounds that clear with coughing. What action by the nurse is best?
- Prepare to treat the patient for asthma.

- b. Prepare to treat the patient for pneumonia.
- c. Teach the parent how to prevent croup.
- d. Assess the patient for heart failure.

ANS: B

The patient has rhonchi. Rhonchi are caused by increased secretions in large airways and can be seen in pneumonia or in other conditions, leading to increased mucus production. The nurse prepares to treat the patient for pneumonia. Asthma would manifest with wheezing, croup with stridor, and heart failure with rales or crackles.

DIF: Analyzing OBJ: 20.5 TOP: Assessment

MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:

Concepts: Gas Exchange

17. The nurse is assessing a patient's cranial nerve III. What technique is best?
- a. Have patient identify a common scent with closed eyes.
 - b. Shine a light into the patient's eyes to assess pupil response.
 - c. Have the patient read a newspaper or use the Snellen chart.
 - d. Assess if patient can hear both spoken and whispered words.

ANS: B

Cranial nerve III (oculomotor nerve) is assessed by observing the patient's pupil size and reaction to light and the direction of gaze. Identifying a common scent would test cranial nerve I. Assessing the patient's visual acuity tests cranial nerve II. Assessing hearing is cranial nerve VIII.

DIF: Applying OBJ: 20.5 TOP: Assessment

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:
Concepts: Clinical Judgment

18. A nurse is assessing a patient's cranial nerves and notes an abnormal response to testing cranial nerve VI. What action by the nurse is best?
- a. Ask the patient about recent facial trauma.
 - b. Inform the provider immediately.
 - c. Document findings in the patient's chart.
 - d. Have the patient frown and lift the eyebrows.

ANS: A

Cranial nerve VI (abducens) is responsible for outward gaze of the eyes. Abnormal findings could indicate a fracture of the orbit or a brain tumor. The nurse asks the patient questions related to these two conditions. The provider needs to be informed and the nurse must document, but first the nurse conducts a thorough assessment. Frowning and lifting the eyes assesses cranial nerve VII.

DIF: Applying OBJ: 20.5 TOP: Assessment

MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:

Concepts: Sensory Perception

19. The nurse reads in a chart that a patient has a paronychia. What assessment technique is most appropriate?
- a. Auscultate the patient's bowel sounds.
 - b. Test the cranial nerves for sensory function.

- c. Inspect the patient's nails and surrounding skin.
- d. Inspect the skin using the ABCDE mnemonic.

ANS: C

A paronychia is inflammation at the base of the nail, so the nurse assesses the patient's nails and the surrounding skin. The other assessments are not related to this diagnosis.

DIF: Applying OBJ: 20.5 TOP: Assessment
 MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:
 Concepts: Clinical Judgment

MULTIPLE RESPONSE

1. A new nurse is conducting a patient interview. What behaviors observed by the experienced nurse require education on this process? (*Select all that apply.*)
 - a. Typing intently on a keyboard when asking questions
 - b. Allowing family to accompany the patient as requested
 - c. Using gestures and eye contact to demonstrate interest
 - d. Closing the door to the room to ensure privacy
 - e. Providing nonverbal cues to negative thoughts

ANS: A, E

During the interview process, the nurse needs to demonstrate interest in the patient by leaning slightly toward him/her, allowing requested family or friends to accompany the patient, and closing the door to the room to ensure privacy. Typing intently when the patient is talking can be interpreted as lack of interest. Providing nonverbal cues to the nurse's negative thoughts (such as scowling when the patient mentions something negative) does not promote comfort

or trust.

NURSINGTB.CO

DIF: Remembering OBJ: 20.1
 TOP: Integrated Process: Communication and Documentation
 MSC: NCLEX Client Needs Category: Psychosocial Integrity NOT: Concepts: Communication

2. A nurse is conducting a physical examination using palpation. Which assessments might the nurse note? (*Select all that apply.*)
 - a. Rebound tenderness
 - b. Crepitation
 - c. Guarding
 - d. Turgor
 - e. Consistency

ANS: A, B, C, D, E

Crepitation is crackling or rubbing felt (and perhaps heard) during palpation. Turgor is the amount of tension in body tissues caused by fluid content. Consistency compares organs for their location and size related to the norms. Rebound tenderness occurs after the stimulation is discontinued but is elicited with palpation. Guarding is positioning to prevent movement of a painful body part. In this scenario, the patient would guard to prevent the nurse from palpating a painful area.

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance

NOT: Concepts: Clinical Judgment

3. A nurse conducting the general survey of a patient includes which items? (*Select all that apply.*)
- a. Hygiene and grooming
 - b. Affect and mood
 - c. Sex and gender orientation
 - d. Sexual preferences and practices
 - e. Age

ANS: A, B, C, E

Components of the general survey include age, race, hygiene and grooming, affect and mood, clothing, sex and gender orientation, age, and safety. Sexual preferences and practices are not included.

DIF: Remembering

OBJ: 20.4

TOP: Assessment

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance

NOT: Concepts: Clinical Judgment

4. The nurse examining a patient's skin correlates which conditions with which underlying pathology? (*Select all that apply.*)
- a. Albinism: Full-thickness burns
 - b. Peripheral cyanosis: poor circulation
 - c. Purpura: clotting disorders
 - d. Jaundice: liver disease
 - e. Vitiligo: skin infestation

ANS: B, C, D

NURSINGTB.COM

Peripheral cyanosis can result from poor circulation. Purpura can be seen in patients with clotting disorders. Jaundice often indicates liver disorders such as liver failure. Albinism is genetically determined. Vitiligo is thought to be an autoimmune response.

DIF: Remembering

OBJ: 20.5

TOP: Assessment MSC:

NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:

Concepts: Clinical Judgment

5. A nurse is educating women on breast cancer risk reduction. What topics does the nurse include in the presentation? (*Select all that apply.*)
- a. Exercise
 - b. Limiting alcohol
 - c. Low-fat diet
 - d. Breast self-exams
 - e. Milk intake

ANS: A, B, C

Reducing breast cancer risk can be accomplished by getting regular exercise, limiting alcohol, and eating a low-fat diet. Performing breast self-examinations will not reduce risk but may help women find abnormalities early. Milk intake is not related.

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance

NOT: Concepts: Health Promotion, Patient Education

6. A nurse has finished examining a patient. What actions does the nurse take next? (*Select all that apply.*)
- a. Document all findings.
 - b. Provide privacy for dressing.
 - c. Provide any hygiene material needed.
 - d. Tells the patient he/she can leave.
 - e. Cleans the room after the patient leaves.

ANS: A, B, C, E

After finishing the exam, the nurse provides the patient with privacy for changing back into street clothes and any needed hygiene material. The nurse also documents the findings and cleans the room before the next patient is seen. The nurse does not simply tell the patient he/she may leave. The nurse should indicate what will happen next before the patient leaves (i.e., providing written material summarizing the visit, scheduling the next appointment).

DIF: Applying OBJ: 20.6 TOP: Implementation

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:

Concepts: Patient Education, Health Promotion

NURSINGTB.COM

Chapter 21: Ethnicity and Cultural Assessment**Yoost & Crawford: Fundamentals of Nursing: Active Learning for Collaborative Practice, 3rd Edition****MULTIPLE CHOICE**

1. A nursing faculty member is contrasting culture and ethnicity to students. Which statement is most accurate?
 - a. Culture is biologically determined; ethnicity is chosen.
 - b. Culture is socially transmitted; ethnicity is identification with a group.
 - c. Culture is a chosen identity whereas ethnicity is biologically based.
 - d. Culture and ethnicity are similar constructs used interchangeably.

ANS: B

Culture refers to the learned, shared, and transmitted knowledge of values, beliefs, and ways of life of a group that generally are transmitted from one generation to another and influence the individual person's thinking, decisions, and actions in patterned or certain ways. Ethnicity is the person's identification with or membership in a racial, national, or cultural group and observation of the group's customs, beliefs, and language. The words may be used interchangeably by some people, but this is not correct.

DIF: Understanding

OBJ: 21.1

TOP: Integrated Process: Teaching/Learning

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Culture

2. A nursing student wants to observe enculturation practices of an ethnic minority community. What action by the student is best?
 - a. Attend a community dance.
 - b. Learn to cook an ethnic meal.
 - c. Visit the group's worship service.
 - d. Observe a grandmother teaching a child.

ANS: D

Enculturation is the process of passing a culture down from generation to generation. Culture can be taught directly, for instance, with the grandmother teaching the child. Culture can also be taught indirectly as when a child observes a role in the community. The student observing the grandmother teaching a child is the best example of enculturation.

DIF: Applying

OBJ: 21.1

TOP: Assessment

MSC: NCLEX Client Needs Category: Psychosocial Integrity NOT: Concepts: Culture

3. The student nurse learns that which item is the most important symbolic aspect of culture?
 - a. Flags
 - b. Language
 - c. Art
 - d. Music

ANS: B

Language is the most extensively used set of symbols in a culture. The other items are important symbols but are not as important as language because words are used to represent objects and ideas.

DIF: Remembering

OBJ: 21.1

TOP: Integrated Process: Teaching/Learning

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Culture

4. A charge nurse works on an inpatient unit in a diverse city. To provide culturally congruent care to the patient, which action by the nurse would be most appropriate?
- Using puns and sarcasm to help draw the patient into sharing information
 - Working to understand the socioeconomic status of the patient so teaching is culturally sensitive and appropriate
 - Assuming a patient from a minority population does not have the economic means to pay for home care follow-up
 - Admonishing a Hispanic patient for showing up for a preoperative teaching class 15 minutes late

ANS: B

Nurses need to be cognizant of the impact of a patient's socioeconomic status to health care practices. The use of puns, sarcasm, and colloquialisms are not easily comprehended or interpreted by those who speak a different primary language. While the level of poverty in minority populations within all cultures is disproportionately higher, it is inappropriate to base an action on an assumption. According to research, some Hispanics believe that time is flexible and events will begin when they arrive. However, admonishment is not the best approach to dealing with this behavior.

DIF: Applying

OBJ: 21.2

TOP: Integrated Process: Caring

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Culture

5. A nurse has been told he has many obvious stereotypes about a specific cultural group. What action by the nurse is best?
- Ask to not care for members of this cultural group.
 - Ask to take care of as many members of this group as possible.
 - Begin to educate himself on aspects of this cultural group.
 - Vow to not allow his stereotypes to show when providing care.

ANS: C

Stereotypes are fixed ideas, often unfavorable, about groups of people. They occur because of being unwilling to gather all the information needed to make fair determinations. The nurse would benefit most from beginning to learn about this cultural group. Caring or not caring for members of this group will not help him obtain new information. The nurse should not let stereotypes show, but this is not the best option.

DIF: Applying

OBJ: 21.2

TOP: Integrated Process: Teaching/Learning

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Culture

6. A nurse is caring for a homeless patient and tells the manager, —I will make sure he doesn't steal food from our nourishment center. What action by the manager is best?
- Tell the nurse she is right to monitor the patient's activity.
 - Inform the nurse that not all homeless people will steal.

- c. Educate the nurse that hunger might make the patient steal.
- d. Remind the nurse to initiate a social work consultation.

ANS: B

This nurse is guilty of being prejudiced against the patient, who is a member of the homeless culture. Although hunger might drive a homeless person to steal, prejudice leads the nurse to believe that all homeless people steal. The manager informs the nurse of this information, gently pointing out the nurse's bias. A social work consultation may be a good idea for the patient but does not address the prejudiced nurse.

DIF: Applying OBJ: 21.2

TOP: Integrated Process: Communication and Documentation

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Culture

7. The new nurse tells the preceptor that since she is not prejudiced against ethnic minorities, they will not be discriminated against while in the hospital. What statement by the preceptor is most appropriate?
- a. Discrimination can occur at the societal level.
 - b. The hospital needs more nurses like her.
 - c. Prejudice and discrimination are not the same thing.
 - d. There is always some discrimination against minorities.

ANS: A

Discrimination can occur at the societal level, so even though this nurse is not prejudiced, patients from ethnic and cultural minorities can still suffer from discrimination. The other answers do not explain how discrimination can occur.

NURSINGTB.COM
OBJ: 21.2 M
U S N

DIF: Understanding

TOP: Integrated Process: Communication and Documentation

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Culture

8. What does the nursing student learn about race?
- a. It is biologically based.
 - b. It is a social construct.
 - c. It is chosen by the person.
 - d. It helps establish superiority.

ANS: B

Race is often thought to be inherited and biologically based, but this is not true. Race is a social construct that is used to group people together based on common physical characteristics, heredity, or common descent. People are placed into racial categories by the larger society. One race is not superior to any other.

DIF: Remembering

OBJ: 21.2

TOP: Integrated Process: Teaching/Learning

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Culture

9. The nurse is caring for a patient from a culture that is unfamiliar. The patient nodded her head—yes! when asked if she will take her prescriptions as ordered, but the nurse discovers the patient does not take the medication but uses herbs for treatment. What action by the nurse is best?
- a. Warn the patient of the consequences on noncompliance.

- b. Tell the patient how the medication will help the condition.
- c. Ask the patient why herbal preparations are preferred.
- d. Ask the patient to explain the meaning of the herbal products.

ANS: D

Ethnocentrism is the belief that one's cultural beliefs are superior to others. To avoid practicing in an ethnocentric manner, the nurse needs to understand the meaning of the herbal preparation to the patient. Warning the patient of bad outcomes will not achieve the desired results if the herbs are culturally important and meaningful to her. Patient education is always important but is not the best answer because it does not allow the nurse to learn from the patient. Asking —why? question is a communication barrier likely to put the patient on the defensive.

DIF: Applying OBJ: 21.5 TOP: Assessment

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Culture

10. A nurse is caring for a refugee patient who wants the community shaman to perform a healing ritual at the bedside. What action by the nurse is best?
- a. Work with the patient to allow the shaman to perform the ritual.
 - b. Investigate whether the ritual will harm the patient.
 - c. Check to see if the ritual breaks laws or policies.
 - d. Offer to call the hospital chaplain instead.

ANS: A

Rituals are deeply powerful and have great meaning for individuals who practice them. The nurse should work with the patient to facilitate the ritual. Investigating the ritual for patient harm or illegality is ethnocentric; the nurse's first thoughts should not be on the potential

Negative aspects of a deeply meaningful activity. The patient has not requested the chaplain; offering to call the chaplain shows ethnocentrism and lack of respect for the patient. While working to facilitate the ritual, the nurse will discover if any aspect of it might be problematic and can collaborate with the patient and shaman to resolve the situation (e.g., if lighted candles are needed but prohibited by policy and fire code).

DIF: Applying OBJ: 21.5 TOP: Implementation

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Culture

11. A new graduate nurse tells the manager that she does not believe she needs more in-service training on culturally congruent care because she already recognizes that there are significant differences among cultures to consider when providing care. What response by the manager is best?
- a. —You have done a great job becoming culturally competent.‖
 - b. —Providing culturally congruent care takes ongoing work and effort.‖
 - c. —That is a great start but be sure to sign up for the in-service.‖
 - d. —Cultural sensitivity and cultural competence are not the same.‖

ANS: B

Cultural sensitivity is the recognition that there are profound differences among cultures that can affect health care. But to provide culturally congruent care, the nurse must do more than just recognize these differences. This is an ongoing process. Option B is the only one that provides useful information to the nurse as to why she must continue to work on this aspect of her profession.

DIF: Applying OBJ: 21.4

TOP: Integrated Process: Communication and Documentation

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Culture

12. A nurse is working with a patient who has limited English proficiency. What action by the nurse is best?
- Use a qualified interpreter.
 - Ask family members to translate.
 - Use drawings and pictures.
 - Speak in simple sentences.

ANS: A

Qualified interpreters should be utilized when working with non- or limited-English speaking persons. Using a family member to interpret can upset the balance of power within the family, cause embarrassment, and lead to inaccuracies. Using drawings and pictures or speaking in simple sentences is not as effective as using an interpreter.

DIF: Applying OBJ: 21.4

TOP: Integrated Process: Communication and Documentation

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Culture

13. The nurse is working with a patient from an unfamiliar culture. After assessing the patient and the patient's cultural beliefs related to health care, what action by the nurse is best?
- Create a nursing plan of care for the patient.
 - Recheck cultural beliefs with the patient.
 - Use a standard plan of care for consistency.
 - Have an interpreter validate the information.

ANS: B

According to Leininger, the nurse should recheck assumptions and findings related to culture with the patient. This is an important step prior to creating a care plan. A standard plan will not be culturally congruent. The stem does not indicate that the patient has limited English, but if he did, using an interpreter would be important.

DIF: Applying OBJ: 21.6 TOP: Assessment

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Culture

14. A patient from an unfamiliar culture appears disinterested when the physician is telling her about options for treatment of a new diagnosis. After the physician leaves, the nurse attempts to talk to the patient and notices the same behavior. What action by the nurse is best?
- Give the patient the information in writing to read later.
 - Ask the patient about the meaning of the patient's behavior.
 - Investigate nonverbal communication patterns of this group.
 - Leave the patient alone to come to terms with the diagnosis.

ANS: C

Communication differences can lead to misunderstandings and possible medical errors. Many cultural groups have verbal and nonverbal communication patterns that differ from other groups. Variations can occur due to personal or social situations. The nurse should attempt to learn about the cultural group's communication patterns. Giving the patient written material and leaving the patient alone do not help solve this dilemma, and the patient may not have the literacy skills to understand the material. Asking the patient the meaning of behavior is unlikely to elicit useful information because the patient herself may not totally understand it or be able to articulate it. This may be a deeply seated cultural custom that is simply part of who the patient is.

DIF: Applying OBJ: 21.5 TOP: Assessment

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Culture

15. A patient in the emergency department needs an emergency operation. The patient refuses to consent and wants the nurse to call a respected elder in the community for consent. What action by the nurse is best?
- Explain that this violates privacy laws.
 - Call the elder to get consent for the operation.
 - Tell the woman she has the right to consent.
 - Arrange for admission without the operation.

ANS: B

In some cultures, decisions are made by men or community leaders. Although the patient may have the legal right to consent, if she comes from a culture in which gender and/or social roles do not permit decision making, she will likely refuse to consent. The best action is for the nurse to contact the elder and have him participate in the decision-making process per the patient's wishes. If the patient has given permission to share the information, doing so does not violate privacy laws. Admitting the patient without the operation does not help her medically.

DIF: Applying OBJ: 21.5

TOP: Integrated Process: Communication and Documentation

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Culture

16. A patient refuses to take his blood pressure medication because —I feel totally fine and don't need it. What action should the nurse take first?
- Assess the patient's time orientation.
 - Document the patient's noncompliance.
 - Educate the patient about the medication.
 - Warn the patient about possible complications.

ANS: A

People with a present time orientation typically live in the —here and now and may not see the benefit of adhering to medical regimens when they are not symptomatic. The nurse should assess the patient's time orientation. Documentation and education are both important but are not likely to secure the patient's cooperation.

DIF: Applying OBJ: 21.5 TOP: Assessment

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Culture

17. The nurse is caring for a patient from a different cultural background. What action by the nurse best demonstrates cultural maintenance?
- Assist the patient with a healing ritual.
 - Teach the patient a heart healthy diet.
 - Instruct the patient on monitoring blood glucose.
 - Discuss what self-care activities the patient is willing to do.

ANS: A

Cultural maintenance maintains and preserves relevant cultural care values pertaining to health care. Assisting the patient with a healing ritual important to him/her is an example. Teaching a heart-healthy diet and blood glucose monitoring falls into cultural care repatterning. Discussing what changes the patient is willing to accommodate is an example of cultural care accommodation.

DIF: Applying OBJ: 21.5 TOP: Integrated Process: Caring
MSC: NCLEX Client Needs Category: Psychosocial Integrity NOT: Concepts: Culture

18. A student nurse is caring for a patient who is a refugee. The patient will take his own blood glucose readings and will self-administer a set dose of insulin but will not follow a sliding scale regimen in which the patient has to choose what dose of insulin to give. What action by the student nurse is best?
- Ask the provider to prescribe only a set insulin regimen.
 - Instruct the patient on the benefits of sliding scale insulin.
 - Teach the patient that strict carbohydrate limits are needed.
 - Ask the patient to explain the meaning of making this decision.

ANS: D

The patient may have a more fatalistic worldview than is common in Western societies. The patient may follow —orders from an authority figure but may feel like it is not his place to determine his insulin dose, or the patient may not feel competent in making that decision. Many explanations are possible. The student needs to determine what the patient feels related to this type of decision making before doing anything else.

DIF: Applying OBJ: 21.5 TOP: Assessment
MSC: NCLEX Client Needs Category: Psychosocial Integrity NOT: Concepts: Culture

19. A home health care nurse is visiting the home of a patient whose culture is totally unfamiliar to the nurse. What action by the nurse is best?
- Perform nursing care with a high degree of professionalism.
 - Watch family interaction patterns closely and try to copy them.
 - Tell the family you need to learn about their culture.
 - Apologize after performing tasks that make the patient uncomfortable.

ANS: B

Nurses should observe family dynamics carefully, including communication, and try to copy them as much as possible. For instance, if the family does not make eye contact with the nurse, he/she should avoid trying to make direct eye contact with the family. The other options are reasonable, although telling the family you need to learn about their culture may place the burden of educating the nurse on them.

DIF: Applying OBJ: 21.4

TOP: Integrated Process: Communication and Documentation

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Culture

20. The nurse is caring for a patient from a different culture. After assessing the patient and formulating the care plan, what action by the nurse is best?
- Review the care plan for acceptance by the patient.
 - Delegate appropriate tasks to unlicensed assistive personnel.
 - Go over the care plan with the charge nurse.
 - Begin implementing the planned interventions.

ANS: A

Care plans, with their goals and interventions, should always be validated by the patient. This is especially true when the patient is from a different culture than the nurse. The charge nurse may or may not need to view the care plan, but after validation with the patient, the nurse can begin implementing the plan, including delegating appropriate tasks.

DIF: Applying OBJ: 21.6

TOP: Integrated Process: Communication and Documentation

MSC: NCLEX Client Needs Category: Psychosocial Integrity NOT: Concepts: Culture

21. A patient has hypertension and is on a very-low-sodium diet. However, the patient is going to celebrate an important religious holiday soon that includes many food items high in sodium. What action by the nurse is best?
- Tell the patient you are so sorry she can't have any of these foods.
 - Consult with the prescriber about increasing the blood pressure medications.
 - Collaborate with the patient and dietitian to include some of these foods.
 - Tell the patient eating these foods once won't hurt her condition.

ANS: C

Food has important meaning to many people, especially when they are part of celebrations, religious, or cultural activities. The nurse should collaborate with the patient and dietitian and try to find ways to incorporate some of these items. The nurse should not just tell the patient she can't have them. Increasing the medications or encouraging the patient to be nonadherent could lead to adverse outcomes.

DIF: Applying OBJ: 21.4

TOP: Integrated Process: Communication and Documentation

MSC: NCLEX Client Needs Category: Psychosocial Integrity NOT: Concepts: Culture

MULTIPLE RESPONSE

1. The student studying culture learns that which are characteristics of all cultures? (*Select all that apply.*)
- Integrated
 - Shared
 - Learned
 - Symbolic
 - Inherited

ANS: A, B, C, D

Cultures are learned, symbolic, shared, and integrated. Since culture refers to patterns of beliefs, actions, values, and ways of life that are taught, they are not inherited.

DIF: Remembering

OBJ: 21.1

TOP: Integrated Process: Teaching/Learning

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Culture

2. The nurse understands that which are important in the process of developing a cultural identity? (*Select all that apply.*)
- a. School
 - b. Church/religious institution
 - c. Family
 - d. History
 - e. Community

ANS: A, B, C, E

Many institutions and groups, both formal and informal, assist an individual in developing a cultural identity, including school, religious institutions, family, and community.

DIF: Remembering

OBJ: 21.3

TOP: Integrated Process: Teaching/Learning

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Culture

3. The nursing student learns that which are correct regarding acculturation and assimilation? (*Select all that apply.*)
- a. Assimilation is forced entry into a different culture.
 - b. Acculturation depends on first-hand contact between groups.
 - c. Acculturation results in changes to the minority culture only.
 - d. Assimilation can occur at the group or individual level.
 - e. Assimilation causes a minority group member to blend into the majority group.

ANS: B, D, E

Acculturation occurs from first-hand contact between a minority group and the majority cultural group and can result in changes to one or both cultures. Assimilation occurs when members of a minority group blend into the majority group and can occur at the group or individual level. Assimilation is not a forced change.

DIF: Remembering

OBJ: 21.3

TOP: Integrated Process: Teaching/Learning

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Culture

4. The nurse is using Giger and Davidhizar's Transcultural Assessment Model to gain information about a patient from an unfamiliar culture. What questions does the nurse ask that are relevant to this mode? (*Select all that apply.*)
- a. —Who would you like present to help answer questions?||
 - b. —What do you believe caused your current illness?||
 - c. —How important is planning for the future to you?||
 - d. —Why don't you want to shake my hand?||
 - e. —What activities would you do to control your health?||

ANS: A, B, C, E

The Giger and Davidhizar Transcultural Assessment Model looks at communication, space, social orientation, time, environmental control, and biological variation. The questions all address these factors; however, asking why the patient does not want to shake the nurse's hand sounds judgmental and —why? questions are a communication barrier.

DIF: Applying OBJ: 21.5 TOP: Assessment

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Culture

NURSINGTB.COM

Chapter 22: Spiritual Health**Yoost & Crawford: Fundamentals of Nursing: Active Learning for Collaborative Practice, 3rd Edition****MULTIPLE CHOICE**

1. The student nurse asks why he needs to assess a patient's spirituality when he can call the chaplain. What response by the nurse is best?
 - a. —This way you learn what is involved in a spiritual assessment.¶
 - b. —Students need to perform all aspects of patient care.¶
 - c. —Regulatory organizations list this as a required BSN competence.¶
 - d. —All patients should have a spirituality assessment.¶

ANS: C

Although there is some truth to all options, several regulatory groups list conducting a spiritual assessment as a vital skill for nurses, including the American Association of Colleges of Nursing, The Joint Commission, and the American Nurses Association.

DIF: Understanding

OBJ: 22.3

TOP: Assessment

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Culture

2. The nurse is caring for four patients. Which one should the nurse assess for spirituality needs as a priority?
 - a. New mother, older child at home.
 - b. Faces terminal diagnosis.
 - c. Needs to change medications.
 - d. Pleasant but quiet.

ANS: B

There are many cues to alert the nurse that a patient might have unmet spiritual needs, including facing a terminal illness. The nurse should conduct spiritual assessments on all patients, but this one is the priority.

DIF: Analyzing

OBJ: 22.3

TOP: Assessment

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Culture

3. A patient has the Nursing diagnosis Spiritual Distress. What assessment by the patient best indicates that an important goal has been met?
 - a. Observed praying quietly.
 - b. Indecisive about treatment.
 - c. Asks nurse if God exists.
 - d. Executes living will.

ANS: A

Patients may have spiritual distress when facing situations that threaten their meaning and purpose in life, such as in the face of a terminal diagnosis. Patients often express anger, frustration, neediness, or crying. The patient who has worked through this situation and is able to pray has best shown goal attainment. Indecision and questioning do not indicate the resolution of this diagnosis. Executing a living will may be an indication of pragmatism.

DIF: Evaluating OBJ: 22.5 TOP: Evaluation
 MSC: NCLEX Client Needs Category: Psychosocial Integrity NOT: Concepts: Culture

4. The nurse concerned about a patient's spiritual needs can best address this by which action?
- Leaving a note on the chart for other professionals
 - Calling the chaplain to come see the patient
 - Collaborating during interdisciplinary rounds
 - Informing the provider of the patient's needs

ANS: C

Spiritual care must be multidisciplinary to be most effective. The nurse best addresses patients' spiritual needs by discussing them during interdisciplinary rounds.

DIF: Applying OBJ: 22.6 TOP: Communication and Documentation MSC:
 NCLEX Client Needs Category: Psychosocial Integrity NOT: Concepts: Culture

5. A patient is hesitating to accept a blood transfusion as a course of treatment. What Nursing diagnosis is most appropriate for this patient?
- Spiritual distress
 - Anxiety
 - Moral distress
 - Decisional conflict

ANS: C

Moral distress is cultural conflict between medical treatment and religious beliefs, expressions of concern about rejection by religious community, hesitation in accepting blood transfusion. The other diagnoses are not related.

DIF: Applying OBJ: 22.5 TOP: planning
 MSC: NCLEX Client Needs Category: Psychosocial Needs NOT: Concepts: Culture

6. A patient is finding conflict when trying to maintain personal beliefs while making health care decisions. What Nursing diagnosis is a priority as the nurse plans care?
- Spiritual distress
 - Impaired religiosity
 - Moral distress
 - Decisional conflict

ANS: D

Decisional conflict is unclear personal beliefs, questioning of personal beliefs while making decisions, delayed decision making. The other diagnoses may exist as well, but they are not manifested by this conflict.

DIF: Analyzing OBJ: 22.5 TOP: Planning
 MSC: NCLEX Client Needs Category: Psychosocial Integrity NOT: Concepts: Culture

7. A patient asks the nurse to pray with him. The nurse is an atheist and uncomfortable with this request. What action by the nurse is best?
- Deny the request because of atheistic beliefs.
 - Offer to call the chaplain instead.
 - Agree to sit with the patient while he prays.
 - Ask the patient if he will meditate instead.

ANS: C

Although the nurse is uncomfortable with the request, the patient's needs (not the nurse's) come first. The nurse should attempt to honor the request while not imposing his/her ideas of religion and spirituality on the patient. The best option is to agree to sit with the patient while he prays himself. This is consistent with caring behaviors and fulfilling the patient's needs. Denying the request does nothing to address the patient's needs. The nurse can offer to call the chaplain in addition to sitting with the patient. Asking the patient to change his practices is unethical.

DIF: Applying

OBJ: 22.7

TOP: Integrated Process: Caring

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Culture

8. A nurse is concerned about not consistently meeting the spiritual needs of patients. What action by the nurse is best?
- Care for own spiritual needs.
 - Begin a meditation practice.
 - Consult the chaplain.
 - Read books on the subject.

ANS: A

To avoid burnout and a decreased ability to attend to the spiritual needs of patients, nurses must take care of their own spiritual needs first. This may include meditation, consultations, and reading, but other activities can guide the nurse into a reflective practice that will allow better spiritual care.

DIF: Applying

OBJ: 22.3

TOP: Integrated Process: Caring

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Culture

9. The student nurse asks why spirituality is important in health care. What response by the registered nurse is best?
- All people have a spiritual aspect to their beings.||
 - Spirituality affects behavior, which also affects health.||
 - Knowledge of it is needed to understand a patient holistically.||
 - People who are less spiritual have worse outcomes.||

ANS: B

Spirituality affects behavior, which has a direct impact on health. Spirituality is a universal concept, but all people may not recognize it in themselves. Holistic knowledge is indeed based in part on spirituality, but that does not give the student information on a concrete link. Less spiritual people may or may not have worse outcomes.

DIF: Understanding

OBJ: 22.1

TOP: Integrated Process: Teaching/Learning

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Culture

10. A patient who claims to be very involved in church is near death. What action by the nurse is best?
- Get permission to contact the religious leader.
 - Allow the family to stay at the patient's bedside.
 - Call the hospital chaplain to come to the bedside.
 - Ask if the patient and family want to pray.

ANS: A

Organized religions use rituals to mark important life events such as birth, marriage, and death. This patient would most likely want end-of-life rituals as practiced in his/her church. The nurse's best action is to contact the religious leader (with permission) of that church or institution. Allowing the family to remain at the bedside is important but not the best option to care for the patient's spirituality needs. The hospital chaplain is a valuable resource, but the patient's own religious leader would be better. Praying with the family is always acceptable, but it is best to let the family take the lead in prayer.

DIF: Applying OBJ: 22.2 TOP: Implementation

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Culture

11. A patient, who is an adherent Muslim, is in a burn unit with severe burns. The patient has high caloric requirements but is refusing to eat during Ramadan. What action by the nurse is best?
- Insert a feeding tube and provide enteral feedings.
 - Ask the provider about Total Peripheral Nutrition.
 - Call the patient's religious leader for advice.
 - Tell the patient he has to eat to get better.

ANS: C

With permission, the nurse should consult with the patient's religious leader on this situation. There may be exceptions to the rule to fast during Ramadan for medical conditions. The other options ignore the patient's religious preferences, and both the tube feeding and parenteral nutrition have potential serious side effects.

DIF: Applying OBJ: 22.2

TOP: Integrated Process: Communication and Documentation

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Culture

12. A patient in the hospital is an adherent Muslim. Which of the five pillars of Islam can the nurse assist the patient in meeting?
- Praying five times a day
 - Having privacy
 - Personal cleanliness
 - Giving alms
 - Maintaining modesty

ANS: A

The five pillars of Islam are: believe in one God, pray five times a day facing Mecca, giving alms to the less fortunate, fasting during Ramadan, and making a pilgrimage to Mecca. The nurse is best able to help the patient maintain the practice of praying five times a day while hospitalized.

DIF: Applying OBJ: 22.3 TOP: Integrated Process: Caring

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Culture

13. A nurse works in a pediatric oncology unit and is feeling depressed and discouraged. What initial action by the nurse is best?
- Apply for a job transfer to another unit.
 - Consult with the hospital chaplain.
 - Make an appointment with Employee Assistance.

- d. Ask other nurses how they deal with the stress.

ANS: B

Hospital chaplains are great resources for nurses experiencing burnout, moral distress, or spiritual distress. The nurse can take all options, but a consultation with the chaplain is the best place to start to see if the issue can be resolved. The chaplain has a wider range of perceptions and tools than do the other staff nurses.

DIF: Applying OBJ: 22.6

TOP: Integrated Process: Communication and Documentation

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Culture

14. A patient died suddenly in the emergency department. Which action by the nurse best provides the family connection with others?
- Offering the family written information on grief support groups.
 - Asking the family if there is someone the nurse can call for them.
 - Having the hospital social worker or chaplain sit with the family.
 - Offering to stay with the family during this difficult time.

ANS: B

Promoting connectedness means recognizing that family and friends are providing at least some of the patient's spiritual care. The nurse best assists when offering to call someone for the patient or family. The other options may be appropriate but are not directly related to connectedness.

DIF: Applying OBJ: 22.7

TOP: Integrated Process: Communication and Documentation

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Culture

15. The charge nurse overhears a new nurse telling a patient that he should no longer follow his vegetarian diet because his protein needs are so high and because —God made animals for us to eat. What action by the charge nurse is best?
- No action is necessary for the charge nurse to take.
 - Reinforce the nurse's teaching on proper diet.
 - Offer to call the dietitian to work with the patient.
 - Privately speak to the nurse about this conversation.

ANS: D

The nurse should not share opinions or religious edicts with patients when those beliefs contradict the patient's. The charge nurse should counsel the new nurse about this practice. The patient may hold deep convictions about being a vegetarian and may feel disapproval from the nurse, which will impact the nurse-patient relationship. The other options are not appropriate, although the charge nurse could suggest the new nurse collaborate with the dietitian and patient to determine high-protein foods the patient finds acceptable.

DIF: Applying OBJ: 22.6

TOP: Integrated Process: Communication and Documentation

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Culture

16. A home health care nurse has been working with a patient who has the Nursing diagnosis Spiritual Distress. After a few weeks of implementing the care plan, what method is best for the nurse to determine if goals have been met?

- a. Ask the patient to what extent he/she feels goals have been met.
- b. Ask the patient to rate the distress on a scale of 1 to 10.
- c. Assess for objective data to support goal attainment.
- d. Determine if the patient thinks the interventions are helpful.

ANS: A

For a diagnosis with a large subjective component, getting the patient's feedback on goal attainment is best. There may be no objective data the nurse can use to rate goal attainment. Using a scale can be a part of the evaluation, but the patient's determination is best.

DIF: Evaluating

OBJ: 22.7

TOP: Evaluation

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Culture

17. A patient is scheduled to have an MRI and has a metal religious icon pinned to his gown, which can't go in the scanner. What action by the nurse is best?
- a. Take the icon off the patient's gown until she returns.
 - b. Give the icon to the patient's family for safekeeping.
 - c. Pin the icon to the patient's pillow so it can go to radiology.
 - d. Explain the restriction and ask the patient's preference.

ANS: D

The religious icon has profound significance for the patient and should not be removed by the nurse. Since the icon cannot go into the MRI scanner itself, the nurse should explain the situation to the patient and get the patient's opinion of various options. All other options are possibilities, but it should be the patient's determination.

DIF: Applying

OBJ: 22.3

TOP: Integrated Process: Caring

MSC: NCLEX Client

Needs Category: Psychosocial Integrity

NOT: Concepts: Culture

MULTIPLE RESPONSE

1. The nursing student learns which facts about religion and spirituality? (*Select all that apply.*)
- a. Spirituality focuses on the meaning of life to people.
 - b. Religion and spirituality are mutually exclusive.
 - c. Religion implies an organized way of worship.
 - d. Religion provides the structure by which to understand spirituality.
 - e. Spirituality is an individual practice that does not include others.

ANS: A, C, D

Spirituality focuses on the meanings of life, death, and existence. Religion is an organized and structured method of practicing or expressing one's spirituality, so they are interconnected and not mutually exclusive. Religion provides the structure for expressing spirituality. Spirituality can be expressed through relationships with others.

DIF: Remembering

OBJ: 22.1

TOP: Integrated Process: Teaching/Learning

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Culture

2. The student nurse learns that spirituality consists of practices that lead to connection to which items? (*Select all that apply.*)
- a. Other people

- b. Nature
- c. Religious institutions
- d. Oneself
- e. Higher power

ANS: A, D, E

Spiritual practices generally promote three categories of activity: connection with oneself, with others, and with a higher power.

DIF: Remembering

OBJ: 22.1

TOP: Integrated Process: Teaching/Learning

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Culture

3. The nurse who is aware of spirituality practices of major religions knows that which religions view health and illness as a process of balance or imbalance? (*Select all that apply.*)
- a. Catholicism
 - b. Native American
 - c. Hinduism
 - d. Greek Orthodox
 - e. Buddhism

ANS: B, C, E

Native American, Hindu, and Buddhist practitioners believe that health and illness are a matter of balance or imbalance in the body.

DIF: Remembering

OBJ: 22.2

TOP: Integrated Process: Teaching/Learning

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Culture

4. Which actions by a nurse constitute spiritual care? (*Select all that apply.*)
- a. Baptizing a critically ill child per the parent's request
 - b. Leaving the room, giving the patient and family privacy for prayer
 - c. Considering developmental stage when planning care
 - d. Notifying the hospital chaplain of a patient's request
 - e. Praying with patients and families when requested

ANS: A, C, D, E

Many activities fall into the realm of spiritual nursing care, including baptizing an infant in an emergency, notifying the chaplain or other religious leader of patient requests for service, and praying with the patient and family. The nurse always considers the patient's developmental level when planning or providing any type of care. The patient and/or family may or may not want privacy for prayer; the nurse should assess the situation and not just leave.

DIF: Remembering

OBJ: 22.3

TOP: Integrated Process: Caring

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Culture

5. The student using the FICA Spiritual Health Assessment will consider which factors? (*Select all that apply.*)
- a. Faith and belief
 - b. Focused practices
 - c. Importance of faith
 - d. Faith community involvement

- e. Address spirituality in care

ANS: A, C, D, E

FICA stands for faith and belief, importance of faith, faith community involvement, and address spirituality in care.

DIF: Remembering

OBJ: 22.4

TOP: Assessment

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Culture

6. The nurse assessing a patient using the SPIRIT framework would ask which questions?

(Select all that apply.)

- a. —Do you follow a particular religion?||
- b. —How involved in your church are you?||
- c. —Are there any practices I can help you with?||
- d. —How will your religion affect your care?||
- e. —What gives you hope in bad situations?||

ANS: A, B, C, D

SPIRIT stands for Spiritual belief system, personal spirituality, integration and involvement in a spiritual community, ritualized practices and restrictions, implications for medical care, and terminal events planning. Hope is a good thing to assess but is more related to the HOPE framework.

DIF: Applying

OBJ: 22.4

TOP: Assessment

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Culture

7. When does the nurse assess patients' spirituality? (Select all that apply.)

- a. Upon admission
- b. New diagnosis
- c. Life-changing diagnosis
- d. When the chaplain makes rounds
- e. When facing treatment decisions

NURSINGTB.COM

ANS: A, B, C, E

There are many times at which a spiritual assessment is necessary. All patients should have their spirituality assessed upon admission at a minimum. Other assessments should be conducted at times when the patient is at risk for spiritual distress. Assessment should be done based on patient need, not when the chaplain is available.

DIF: Applying

OBJ: 22.4

TOP: Assessment

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Culture

8. The nurse who incorporates the HOPE framework assesses a Native American patient for which of the following? (Select all that apply.)

- a. Desire for shaman to be present
- b. Personal use of herbs and prayers
- c. Desire to create a living will
- d. Power of storytelling for healing
- e. Involvement in church activities

ANS: A, B, D

Native Americans often use shamans; prayers, songs, and dances; storytelling; and herbs in health care. The HOPE framework assesses sources of hope, meaning comfort, strength, peace, love, and connection; organized religion; personal spirituality and practice; and effects on medical care and end-of-life issues. The nurse who knows about both topics will assess this patient for the desire for a shaman to be present, the personal use of herbs and prayers, and storytelling. A living will is more accurately assessed with the SPIRIT framework.

Involvement in church activities can be best assessed using either the SPIRIT or FICA framework.

DIF: Analyzing OBJ: 22.2 | 22.4 TOP: Assessment

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Culture

NURSINGTB.COM

Chapter 23: Public Health, Community-Based, and Home Health Care**Yoost & Crawford: Fundamentals of Nursing: Active Learning for Collaborative Practice, 3rd Edition**

MULTIPLE CHOICE

1. The student learns that which is the best definition of a public health nurse?
 - a. Works with the public.
 - b. Works in public areas.
 - c. Works with the greater community.
 - d. Works with public funding.

ANS: C

A public health nurse works with communities as a larger whole and is concerned with specific target or vulnerable groups within that community. The other options are inaccurate.

DIF: Remembering

OBJ: 23.1

TOP: Integrated Process: Teaching/Learning

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:

Concepts: Health Promotion

2. A nurse is discharging a patient and is planning on what material to give the patient to take home. What action by the nurse is best?
 - a. Assess the patient's ability to read and understand.
 - b. Determine if the patient wants to take written material home.
 - c. Give the patient the same material as other patients get.
 - d. Ask the patient if he/she has a need for written material.

ANS: A

Health literacy is an important concept in health. If the patient cannot read or comprehend written material, it will be of limited use. The nurse first assesses the patient's ability to read and comprehend written material before choosing the material with which to send him/her home. Patients may or may not realize what they need for discharge, if anything. Giving the patient the same material other patients get does not acknowledge their need for holistic and individualized care.

DIF: Applying

OBJ: 23.8

TOP: Assessment

MSC: NCLEX Client Needs Category: Health Promotion NOT:

Concepts: Health Promotion, Patient Education

3. A nurse is planning primary prevention activities. Which activity would the nurse include in this plan?
 - a. Safer sex education for teens
 - b. Mammogram screening
 - c. Medication compliance
 - d. Annual physical exams

ANS: A

Primary prevention includes activities designed to prevent a disease or condition from occurring in the first place. Examples of primary prevention activities include vaccinations, wellness programs, good nutrition for health, and safer sex programs. Mammograms and physical exams are secondary prevention measures. Medication compliance would be tertiary prevention.

DIF: Applying OBJ: 23.2 TOP: Planning
 MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:
 Concepts: Health Promotion

4. A nurse wants to volunteer for a community group providing secondary prevention. What activity would the nurse attend?
- Stroke rehabilitation support group
 - Blood pressure screening at the mall
 - Bicycle safety class at the elementary school
 - Drop by nutrition station at the grocery store

ANS: B

Secondary prevention activities are aimed at early diagnosis and prompt intervention. Blood pressure screening events are a good example. Stroke rehabilitation is tertiary prevention. Bicycle safety classes and nutrition education are examples of primary prevention.

DIF: Applying OBJ: 23.2 TOP: Implementation
 MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:
 Concepts: Health Promotion

5. A nurse is orienting to a new job in a home health care agency and is told that most of her patients need tertiary prevention. What activity does the nurse plan to include in the daily routine?
- Household safety checks
 - Well-baby checkups
 - Antibiotic administration
 - Monthly blood pressure assessments

ANS: C

Tertiary care is aimed at people who are already experiencing a health alteration, such as those with an infection who need antibiotics. The other options are secondary prevention.

DIF: Applying OBJ: 23.2 TOP: Implementation
 MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:
 Concepts: Health Promotion

6. A nurse is interested in epidemiology. What work activity would best fit this role?
- Studying census data to determine common causes of death
 - Researching population variables that contribute to disease
 - Developing sanitary measures to prevent foodborne illness
 - Designing research to determine the connection between pollution and cancer

ANS: B

The epidemiologist works to develop programs to prevent the development and spread of disease. Studying census data, researching population variables, and designing studies do not fall in this field.

DIF: Applying OBJ: 23.1 TOP: Planning
 MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:
 Concepts: Health Promotion

7. The student studying community health nursing learns that vulnerable populations can be best assisted by which activity?
- Researching their genetic risk for health problems
 - Working with the community to decrease health risks
 - Studying vital statistics to determine their causes of death
 - Making sure the population maintains immunizations

ANS: B

Vulnerable populations have some characteristic that puts them at higher risk for identified health problems. The nurse can best assist vulnerable populations by identifying and working with them to decrease their risks. Researching genetic risks, studying vital statistics, and improving immunizations are all part of the solution, but the overarching priority action is to help the community decrease its risks.

DIF: Applying OBJ: 23.4 TOP: Implementation
 MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:
 Concepts: Health Promotion

8. A nurse is completing an OASIS assessment on a patient. What data would be most important for the nurse to assess?
- Presence of grocery stores nearby
 - Safety concerns within the home
 - Number and kind of pets
 - Proximity to a health care facility

ANS: B

OASIS (Outcomes and Assessment Information Set) is a data set of outcome measures for adult home health care clients that is used to track outcome-based quality improvement. Factors that could potentially affect patient safety in the home are particularly important. The other options are not included in this assessment.

DIF: Applying OBJ: 23.5 TOP: Assessment
 MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:
 Concepts: Health Promotion

9. A community was devastated by a tornado several months ago. What nursing diagnosis would be most appropriate for the nurse to consider?
- Social isolation
 - Deficient community resources
 - Ineffective community coping
 - Deficient community health

ANS: C

This diagnosis considers those in a community who may be feeling helpless, hopeless, or frustrated because of an extraordinary event. Financial and physical resources may not be available for rebuilding. Social isolation refers to unacceptable social behavior. Deficient community resources is not an approved diagnosis. Deficient community health may become a problem if sanitary conditions lead to an outbreak of disease.

DIF: Applying OBJ: 23.6 TOP: Diagnosis
 MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:
 Concepts: Health Promotion

10. When planning interventions for a community, what action by the nurse is best?
- Involve community leaders in planning.
 - Create a plan of action addressing priorities.
 - Determine what resources are available.
 - Attempt to find funding for the plan.

ANS: A

Stakeholders need to be involved in planning to ensure buy-in from the community. The stakeholders could be community or business leaders. The other actions are important, but if the community leaders are not committed to the plan, the plan is unlikely to work.

DIF: Applying OBJ: 23.8 TOP: Planning
 MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:
 Concepts: Health Promotion

11. A nurse has referred a patient to a community agency. When talking to the patient later, he states that he did not find the agency helpful. What action by the nurse is best?
- Determine what the patient would find helpful.
 - Review the agency's mission and scope.
 - Make another appointment with the agency.
 - Warn the patient that nonadherence affects payment.

ANS: B

One of the most important aspects of a community health nurse's role is to be familiar with referral agencies. Awareness of the scope of an agency's influence and services helps the community nurse to pinpoint which agencies are best able to address specific needs. The nurse may have sent this patient to an agency that did not meet his needs. The nurse should ask the patient's opinion about what services are needed. Making another appointment without ensuring that this is the right agency for the patient will not solve the problem. Telling the patient that payment might not be ensured for nonadherence is not therapeutic communication.

DIF: Applying OBJ: 23.8
 TOP: Integrated Process: Communication and Documentation
 MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:
 Concepts: Health Promotion

12. A nurse has assessed a community and has found many areas in which health can be improved. As a result, the nurse has multiple ideas for programming. What action by the nurse is best?
- Determine what the community thinks is most important.

- b. Use vital statistics to determine which is most important.
- c. See what other communities are focusing programming on.
- d. Choose the easiest problem to address first.

ANS: A

The nurse's priorities may be very different from the community's. For programming to be successful, there must be buy-in from members of the community. Unless programming addresses a need the community thinks is important, it is unlikely to be successful.

DIF: Applying OBJ: 23.8 TOP: Assessment
 MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:
 Concepts: Health Promotion

13. A home health care nurse is working with the family of a patient who has Alzheimer disease and requires 24-hour care. What assessment by the nurse indicates the family is meeting an important goal for caregiver role stress?
- a. Family eats dinner together every night.
 - b. Family uses respite care one night a week.
 - c. Family investigates research trials for patient.
 - d. Family verbalizes exhaustion from caregiving.

ANS: B

Caregiver role stress can occur when the caregiver(s) is unable to meet obligations or unable to take care of personal needs. Using a respite caregiver once a week gives the family a little time off to accomplish needed tasks. The other observations are not tied to this diagnosis.

DIF: Evaluating OBJ: 23.6 TOP: Evaluation
 MSC: NCLEX Client Needs Category: Psychosocial Integrity NOT: Concepts: Coping

14. The nurse has implemented a community-wide immunization program for seasonal influenza. Once the program has ended, what action by the nurse is best?
- a. Begin planning for next year's program.
 - b. Send mail surveys to participants.
 - c. Determine financial gains or losses.
 - d. Evaluate the program and outcomes.

ANS: D

The last step of the nursing process is evaluation. The nurse should evaluate the program to see if interventions had the desired effect. Evaluation could include surveys or looking at financial outcomes, but those are only limited aspects of the process. Planning for next year's event should not occur until after evaluation has been completed.

DIF: Applying OBJ: 23.8 TOP: Evaluation
 MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:
 Concepts: Health Promotion

15. A nurse is wondering if home health care nursing is a good fit. What characteristic or ability does the experienced home health care nurse suggest is most important?
- a. Clinical reasoning
 - b. Organization
 - c. Assessment skills
 - d. Time management

ANS: A

The role of the registered nurse in home health care is essentially autonomous in that the nurse must be highly proficient in health assessment (physical and psychosocial), be well versed in complex technical and clinical skills, possess strong critical-thinking and clinical reasoning abilities, and demonstrate excellent organizational skills.

All choices are important characteristics or abilities of home health care nurses. However, since the nurse working out in the community may not have the resources (personnel or materiel) available in an acute care facility and often must improvise, clinical reasoning would be the most important of the choices provided.

DIF: Understanding

OBJ: 23.1

TOP: Communication and Documentation

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:

Concepts: Health Promotion

16. A nurse is a case manager for a home health care agency. The nurse often orders supplies for patients seen by the agency. What action by the nurse is best?
- Negotiate for cheaper prices from suppliers.
 - Investigate what each patient's insurance will cover.
 - Refer the patient to the closest supply source.
 - Use the same supplier for all patients' needs.

ANS: B

The case manager in home health care must be a well-versed financial steward and understand what each patient's insurance will cover to maximize the patient's benefit. The home health care nurse serves as a case manager (coordinator) of client care, needed services, and needed supplies in the home setting. The nurse must be well versed as a financial resource manager, who needs to be aware of what is or is not covered on the client's insurance plan.

DIF: Applying

OBJ: 23.1

TOP: Assessment

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:

Concepts: Health Promotion

17. The public health nurse volunteers for a missionary group caring for Ebola patients in Africa. The nurse is reviewing the data using analytic epidemiology methods. What information does the nurse collect as the priority?
- Cultural norms in burial practices
 - Genetic variables in disease acquisition
 - Statistics related to incidence and prevalence
 - Autopsy data on direct cause of death

ANS: A

Analytic epidemiology hypothesizes why a disease is occurring in a community and looks at cultural practices, nutrition, and extrinsic factors such as the environment for links. Genetic variables and direct cause of death data are more related to epidemiology.

DIF: Applying

OBJ: 23.1

TOP: Assessment

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:

Concepts: Health Promotion

MULTIPLE RESPONSE

1. The student nurse learns the ANA's Scope and Standards of Practice for public health nursing include components? (*Select all that apply.*)
 - a. Team membership
 - b. Developing research
 - c. Ethical behavior
 - d. Responsible resource use
 - e. Advocacy

ANS: C, D, E

The ANA's Scope and Standards of Practice for public health nursing requires participation in research, responsible resource utilization, ethical behavior, leadership, and advocacy like the standards of practice for all nurses. Team membership and developing one's own research are not included.

DIF: Remembering

OBJ: 23.1

TOP: Integrated Process: Teaching/Learning

MSC: NCLEX Client Needs Category: Health Promotion

NOT: Concepts: Health Promotion

2. A nurse is assessing social determinants of health. Which does the nurse include in the assessment? (*Select all that apply.*)
 - a. Vaccination compliance
 - b. Family structure
 - c. Communication patterns
 - d. Roles for women
 - e. Education

ANS: A, B, D, E

UNRIG.BCMO

Income, education, health literacy, where people live or work, early childhood development, social exclusion, family structure, the status and role of women, and vaccination adherence are just some of the social determinants of health recognized worldwide. Communication patterns often are important to assess in culturally diverse individuals, families, and communities, but this is not considered a social determinant of health care.

DIF: Remembering

OBJ: 23.1

TOP: Assessment

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance

NOT: Concepts: Health Promotion

3. A nurse is studying intrinsic factors that influence the development of asthma in a community. What factors does the nurse assess? (*Select all that apply.*)
 - a. Socioeconomic status
 - b. Genetics
 - c. Pollution in the area
 - d. Water cleanliness
 - e. Immunization status

ANS: A, E

Host, or intrinsic factors are individual variables such as genetics, age, gender, ethnic group, immunization status, and human behavior that impact a person's health. The other options are all extrinsic factors, which pertain to environmental characteristics.

DIF: Understanding OBJ: 23.3 TOP: Assessment
 MSC: NCLEX Client Needs Category: Health Promotion and Maintenance
 NOT: Concepts: Health Promotion

4. The nurse is conducting a windshield survey. What items does the nurse assess? (*Select all that apply.*)
- a. Types of housing available
 - b. Cars seen in parking lots
 - c. Recreational facilities
 - d. Health care facilities
 - e. Places of worship

ANS: A, C, D, E

A windshield survey is a type of community health assessment. The nurse walks or drives through a neighborhood and notes the type of housing available, the presence and condition of recreational facilities, the presence of health care facilities, and places of worship among other items. Types of cars noted in the neighborhood are not one of the assessments.

DIF: Remembering OBJ: 23.5 TOP: Assessment
 MSC: NCLEX Client Needs Category: Health Promotion and Maintenance
 NOT: Concepts: Health Promotion

5. The community health nurse knows that which are standards of professional performance for home care nurses according to the ANA? (*Select all that apply.*)
- a. Collegiality
 - b. Performance appraisal
 - c. Ethical behavior
 - d. Outcome identification
 - e. Resource utilization

NURSINGTB.COM

ANS: C, E

The ANA's *Public Health Nursing: Scope and Standards of Practice* (2013) requires participation in research, responsible resource utilization, ethical behavior, leadership, and advocacy similar to the standards of practice for all nurses.

DIF: Remembering OBJ: 23.1
 TOP: Integrated Process: Teaching/Learning
 MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:
 Concepts: Health Promotion

6. A nurse wants to create a community action plan for health problems related to air pollution from a nearby factory. Which stakeholders does the nurse consult as the priority? (*Select all that apply.*)
- a. Factory owners
 - b. Stock shareholders
 - c. Community residents
 - d. Local health care providers
 - e. Factory employees

ANS: A, C, D

Stakeholders have a significant interest in a topic. The priority stakeholders the nurse would want to consult for this project include the factory owners, community residents, and health care providers. The stockholders would probably not be consulted. The employees could be a significant stakeholder if the action plan affected employment.

DIF: Applying OBJ: 23.8 TOP: Assessment
 MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:
 Concepts: Health Promotion

7. The nurse explains to the patient that which services will be covered under Medicare? (*Select all that apply.*)
- a. Infusion therapy
 - b. Ostomy management
 - c. Renal dialysis
 - d. Chemotherapy
 - e. Grocery shopping

ANS: A, B, C, D

Medicare will reimburse for professionally rendered services provided by a licensed health care provider. Grocery shopping would not be covered. If homemaker services are provided to a patient also receiving skilled care, then they too are reimbursed.

DIF: Understanding OBJ: 23.1
 TOP: Integrated Process: Teaching/Learning
 MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:
 Concepts: Health Promotion

8. The home health care nurse educates patients on which goals of hospice care? (*Select all that apply.*)
- a. Relieve suffering.
 - b. Support the patient and family.
 - c. Provide grief support.
 - d. Keep patients out of the hospital.
 - e. Lower medical expenses.

ANS: A, B, C

The goals of hospice care include relief of suffering, supporting the family and patient, and providing grief support after the patient dies. Goals do not include keeping patients out of the hospital or lowering medical costs.

DIF: Understanding OBJ: 23.7
 TOP: Integrated Process: Teaching/Learning
 MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:
 Concepts: Health Promotion

Chapter 24: Human Sexuality**Yoost & Crawford: Fundamentals of Nursing: Active Learning for Collaborative Practice, 3rd Edition****MULTIPLE CHOICE**

1. A parent confides to the nurse that the parent's 3-year-old son seems to be touching his genitals frequently. What response by the nurse is best?
 - a. —This is normal behavior at his age.¶
 - b. —Why do you think he is doing that?¶
 - c. —Does he complain of burning with urination?¶
 - d. —I'd ignore that behavior; it's attention-seeking.¶

ANS: A

Self-exploration of the body is a normal behavior at this age. The other responses are not necessary.

DIF: Understanding

OBJ: 24.1

TOP: Integrated Process: Teaching/Learning

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Sexuality

2. The nurse learns that spermatozoa are produced in which sexual organ?
 - a. Scrotum
 - b. Testes
 - c. Glans
 - d. Prostate

ANS: B

NURSINGTB.COM

The male testes produce spermatozoa and the male hormone testosterone.

DIF: Remembering

OBJ: 24.2

TOP: Integrated Process: Teaching/Learning

MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:

Concepts: Sexuality

3. The nursing student learns that the function of the hypothalamus is to do which of the following?
 - a. Cause lactation to begin.
 - b. Produce spermatozoa.
 - c. Release follicle-stimulating hormone.
 - d. Release gonadotropin-releasing hormone.

ANS: D

The menstrual cycle is under the influence of the hypothalamus and gonadotropin-releasing hormone.

DIF: Remembering

OBJ: 24.2

TOP: Integrated Process: Teaching/Learning

MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:

Concepts: Sexuality

4. A patient states, —I just don't conform to my gender role. What does the nurse understand about this statement?
- The patient is a homosexual.
 - The patient's behaviors are abnormal.
 - The patient's actions differ from what is expected.
 - The patient is having a gender crisis.

ANS: C

Gender roles are socially imposed —rules about the behavior appropriate for men and women. When someone does not conform to gender role expectations, his/her behaviors are at odds with those expected by society. The patient is not necessarily homosexual, abnormal, or having a crisis.

DIF: Understanding

OBJ: 24.3

TOP: Assessment

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Sexuality

5. A patient asks the nurse to recommend a nonprescription contraceptive. What options does the nurse discuss?
- Diaphragm
 - Cervical cap
 - Condom
 - Intrauterine device

ANS: C

The condom is the only option listed that is nonprescription.

DIF: Understanding

OBJ: 24.5

TOP: Integrated Process: Teaching/Learning

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance

NOT: Concepts: Sexuality

6. A patient has been diagnosed with a sexually transmitted disease (STD) and the patient's partner is angry, saying, —She must have cheated on me. What response by the nurse is most appropriate?
- This infection may have been present for a long time.
 - You need to be tested for this disease too.
 - Yes, you're right; if you don't have the STD, she cheated.
 - Now, now, getting angry will not help anything.

ANS: A

Some STD symptoms may go unnoticed for a long time. Telling the partner to get tested as well without further explanation is likely to cause defensiveness. The nurse has no way of knowing if the patient —cheated on the partner or not. Telling the partner not to get angry is dismissive of his/her concerns.

DIF: Applying

OBJ: 24.6

TOP: Integrated Process: Communication and Documentation

MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation

NOT: Concepts: Infection, Sexuality

7. A nurse is working with a patient using the PLISSIT model. In the LI phase, what is an appropriate activity?

- a. Educate the patient on water-based lubricants.
- b. Ask the patient for permission to discuss sexuality.
- c. Instruct the patient on positions acceptable after knee replacement.
- d. Refer the patient and partner to a licensed therapist.

ANS: A

The LI phase of the PLISSIT model stands for *limited information*, which would be information the patient needs to function sexually. Asking *permission* is P. Discussing specific concerns related to a specific medical condition is SS (*specific suggestions*). Referral to a therapist is IT (*intensive therapy*).

DIF: Applying REF: p. 451 OBJ: 24.10

TOP: Nursing Process: Assessment

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Sexuality

8. A nurse in the emergency department wants to screen a patient for domestic violence, but the woman's partner won't leave. What action by the nurse is best?
- a. Ask the questions anyway.
 - b. Tell the partner to leave.
 - c. Go with the patient to the bathroom.
 - d. Skip the abuse assessment.

ANS: C

Nurses are required to screen for domestic abuse. The nurse needs to provide complete privacy during this assessment. If the partner won't leave, the nurse can go with the patient to the bathroom under the guise of obtaining a urine sample and ask the questions there. Telling the partner to leave will most likely increase the partner's vigilance. Skipping the assessment is not an option.

NURSINGTB.COM

DIF: Applying OBJ: 24.13 TOP: Assessment

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Safety

9. A woman complains that her partner threatens her and berates her in front of the children. She denies being in an abusive relationship or being the victim of physical violence. What action by the nurse is best?
- a. Tell the woman to leave the abusive partner.
 - b. Educate the woman on forms of domestic abuse.
 - c. Help the woman work on a physical safety plan.
 - d. Insist the woman take written information.

ANS: B

This woman first needs to understand she is indeed in an abusive relationship. The nurse gently educates her on the type of abuse that is possible. Telling the woman what to do is likely to be met with resistance, plus the time of leaving is the most dangerous part of the relationship. The woman may not be accepting of a physical safety plan, since she states there is no physical violence. The woman may be fearful of taking written information because the abuser may find it.

DIF: Applying OBJ: 24.13 TOP: Integrated Process: Teaching/Learning

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Safety

10. The nurse is working with a patient who has a sexual dysfunction. What statement by the patient indicates progress toward an important goal?
- I am beginning to enjoy sex more these days.¶
 - I'm glad my partner is understanding of the lack of sex.¶
 - I wish I didn't need these pills but I know they are important.¶
 - I hope one day to have a sexual partner again.¶

ANS: A

According to NANDA, sexual dysfunction occurs when a person has a change in sexual function that the person finds —unsatisfying, unrewarding, or inadequate.¶ To show that a goal has been met, the patient would state that sexual activity is more satisfying, rewarding, or adequate. The patient stating that he/she is beginning to enjoy sex more shows progress toward that goal.

DIF: Evaluating OBJ: 24.12

TOP: Evaluation

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Sexuality

11. A nurse is working with a patient after the patient had a heart attack and is using the PLISSIT model to address sexuality needs. For the SS phase, what action by the nurse is best?
- Ask the patient if he wants to discuss sexuality.
 - Teach the patient positions that require less stress.
 - Offer the patient a referral to a sex therapist.
 - Direct the patient to speak with the doctor about sex.

ANS: B

The PLISSIT model is a framework for addressing sexuality. In the SS (specific suggestions) phase, the nurse provides information that allows the patient to proceed with sexual relations. Informing the patient about sexual positions that are less stressful on the heart is an example. P stands for permission. LI stands for basic, limited information, and IT stands for intensive therapy.

DIF: Applying OBJ: 24.10

TOP: Integrated Process: Teaching/Learning MSC:

NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Sexuality

12. A nurse wishes to incorporate an assessment of patient sexuality into all patient encounters but is concerned about appearing inappropriate. What action by the nurse is best?
- State, —I always ask my patients permission to discuss sexuality. Is this alright?¶
 - Wait for the patient to bring the subject of sexuality up to the nurse.
 - Give the patient written material on sexuality, then ask if he/she has questions.
 - Tell patients that if they have any sexual concerns, you would be happy to discuss them.

ANS: A

A matter-of-fact, organized approach to sexuality will decrease anxiety in both patient and nurse. Stating that all patients are asked about this topic makes it appear to be a normal part of the assessment. The other options put the responsibility for discussing sexuality on the patient when the nurse should be directing the process.

DIF: Applying OBJ: 24.10

TOP: Assessment

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Sexuality

13. An emergency department (ED) manager wants to improve care for victims of sexual assault. What action by the manager is best?
- Designate a private area of the ED for examinations.
 - Establish a SART team for the department.
 - Ask nurses to volunteer to be advocates for these patients.
 - Have victims examined immediately, rather than waiting their turn.

ANS: B

Although all options would improve care for these patients, a SART (Sexual Abuse Response Team) approach offers comprehensive, evidence-based practice for these patients.

DIF: Applying OBJ: 24.13 TOP: Implementation
 MSC: NCLEX Client Needs Category: Psychosocial Integrity NOT:
 Concepts: Collaboration, Sexuality

14. A nurse is caring for a victim of domestic violence. What charting by the nurse is most appropriate?
- Patient allegedly beat up by her boyfriend.
 - Patient has several bruises on the legs.
 - Patient states, —My boyfriend hit me with a hammer.‡
 - Patient claims she was assaulted last night.

ANS: C

Good charting is objective and detailed. Using the patient's own words, in quotation marks, is the most accurate example of documentation. The nurse should not use words like —allegedly‡ or —claims‡ because they seem to cast doubt on the patient's story. The bruises on the legs need to be measured and described more fully.

DIF: Applying OBJ: 24.13 TOP: Integrated Process: Communication and Documentation
 MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
 Concepts: Professionalism, Communication

15. A patient is recovering from colostomy surgery and states, —I guess I'll never be able to have sex again ... who would want me?‡ What Nursing diagnosis is most important for this patient?
- Sexual dysfunction
 - Ineffective sexuality pattern
 - Knowledge deficit
 - Ineffective coping

ANS: B

Ineffective sexuality patterns refer to a patient who expresses concern about his/her own sexuality. This patient is concerned about the effect of this surgery on his/her attractiveness and desirability. Sexual dysfunction relates more to the physical problems. The patient may have a knowledge deficit or ineffective coping, but these are not apparent from the question.

DIF: Analyzing OBJ: 24.11 TOP: Diagnosis
 MSC: NCLEX Client Needs Category: Psychosocial Integrity NOT: Concepts: Sexuality

16. A male patient takes a medication known to cause erectile dysfunction. What action by the nurse is best?

- a. State, —If this medication has bad side effects, talk to your doctor.¶
- b. Ask, —Are you having any sexual problems in your life right now?¶
- c. Give the patient written information on the side effects of the drug.
- d. State, —Many men have erectile dysfunction on this drug.¶

ANS: D

Giving the patient factual information is best. The nurse can follow up on this statement by asking the patient if the medication is affecting his sexuality. —Bad side effects¶ is vague, as is —any sexual problems.¶ Written information may be helpful, but the patient may not be literate and this does not allow the nurse to be engaged with the patient.

DIF: Applying OBJ: 24.8
 TOP: Integrated Process: Communication and Documentation
 MSC: NCLEX Client Needs Category: Psychosocial Integrity NOT:
 Concepts: Sexuality, Communication

17. A school nurse is planning a sex education activity. What information from research does the nurse apply to this education?
- a. Sex education should be taught in high school.
 - b. The school nurse should be the primary source of sex education.
 - c. The method of birth control that should be presented is abstinence.
 - d. Parents' open communication regarding sex education has a positive impact on their children.

ANS: D

According to research, parents should be encouraged to have open communication with their children regarding sex educations. The study noted sex education initiated in the sixth grade had an overall positive effect in delaying sexual activity. The school-based program is secondary to parental input and supports that input by providing knowledge and skill building. Open communication would encourage discussion of a variety of birth control methods.

DIF: Understanding OBJ: 24.9
 TOP: Integrated Process: Teaching/Learning
 MSC: NCLEX Client Needs Category: Psychosocial Integrity NOT: Concepts: Sexuality

MULTIPLE RESPONSE

1. A nurse is teaching patients about their medications and implications for sexuality. Which combinations are correct? (*Select all that apply.*)
- a. Antipsychotics: erectile dysfunction
 - b. Phenytoin: decreased desire
 - c. Antihistamines: increased vaginal lubrication
 - d. SSRIs: prolonged orgasm
 - e. Marijuana: chronic use—reduced inhibitions

ANS: A, B

Antipsychotics can lead to erectile dysfunction. Phenytoin can lead to decreased desire and function. Antihistamines can cause decreased vaginal lubrication. SSRIs can lead to absent or delayed orgasm. Chronic marijuana used can lead to decreased desire.

DIF: Understanding OBJ: 24.8

TOP: Integrated Process: Teaching/Learning

MSC: NCLEX Client Needs Category: Physiological Integrity: Pharmacological and Parenteral Therapies

NOT: Concepts: Sexuality

2. A nurse is planning sexuality education programs. Which topics are important to each age-group? (*Select all that apply.*)
- Adolescents: contraception
 - Adolescents: infertility
 - Young adults: conception
 - Middle adulthood: sexual dysfunction
 - Old age: decreased sexuality

ANS: A, C, D

Adolescents need education on contraception and avoidance of unwanted pregnancy, STDs, HIV infection, sexual abuse, sexual orientation, and good decision making. Young adults particularly need information on conception and infertility. Middle adults need education on emotional and physical changes that occur during this age span including the empty nest syndrome and sexual dysfunction. Older adults need education on physical changes brought by age and encouragement that sexuality normally continues throughout life.

DIF: Remembering

OBJ: 24.1

TOP: Planning

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Sexuality

3. The student learns that which are cycles in the female sexual response cycle? (*Select all that apply.*)
- Excitement
 - Orgasm
 - Resolution
 - Detumescence
 - Plateau

NURSINGTB.COM

ANS: A, B, C, E

The female sexual response cycle includes the phases of excitement, plateau, orgasm, and resolution. Detumescence is when the penis is no longer erect after orgasm.

DIF: Remembering

OBJ: 24.4

TOP: Integrated Process: Teaching/Learning

MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT: Concepts: Sexuality

4. A nurse is uncomfortable with a patient's comments, which are sexual in nature. Which actions by the nurse are most appropriate? (*Select all that apply.*)
- Tell the patient to stop making sexual comments.
 - Try joking with the patient to establish rapport.
 - Tell the patient you are leaving and will return in a few minutes.
 - Inform the patient he/she can be sued for this behavior.
 - Explain to the patient how you feel about the comments.

ANS: A, C, E

There are several steps a nurse can take when patients are displaying inappropriate sexual behavior. First, the nurse should tell the patient to stop what he/she is doing. The nurse can also tell the patient that he/she is leaving for a few minutes and will return when the patient can control this behavior. The nurse can also tell the patient how he/she feels about the comments or actions. The nurse should not joke with the patient or in any way participate in this behavior. The nurse should not threaten a patient with a lawsuit.

DIF: Applying OBJ: 24.13
 TOP: Integrated Process: Communication and Documentation
 MSC: NCLEX Client Needs Category: Psychosocial Integrity NOT:
 Concepts: Communication, Sexuality

5. The nurse has assessed a patient and determined that the patient has a sexual issue that needs to be addressed. What actions by the nurse are most appropriate? (*Select all that apply.*)
- Use information from multiple sources to help plan care.
 - Collaborate with other health professionals to develop the plan.
 - Involve the patient and significant other in the process.
 - Use standard care plans to limit patient embarrassment.
 - Examine one's own biases before implementing the plan.

ANS: A, B, C, E

A good care plan uses information from multiple sources, involves other disciplines as appropriate, and involves the patient and significant other. In matters related to sexuality, the nurse also must examine his/her own biases so they do not limit the ability of the nurse to work with the patient. Using standard care plans does not address the patient's unique needs.

DIF: Applying OBJ: 24.12 TOP: Planning
 MSC: NCLEX Client Needs Category: Psychosocial Integrity NOT:
 Concepts: Sexuality, Collaboration

6. A nurse is planning an educational event on safer sex. What topics does the nurse include? (*Select all that apply.*)
- Proper use of condoms
 - Avoidance of risky behaviors
 - Need for routine examinations
 - Avoidance of homosexual activity
 - Symptoms of common STDs

ANS: A, B, C, E

Safe sex education includes proper use of condoms, avoidance of risky behaviors (and what those are), the need for routine examinations, and symptoms of common STDs. The nurse should not include judgmental comments about sexual practices; all people need information on safer sex practices.

DIF: Understanding OBJ: 24.12
 TOP: Integrated Process: Teaching/Learning
 MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:
 Concepts: Sexuality, Patient Education

7. A nurse understands that which characteristics of family dynamics impact a patient's sexuality? (*Select all that apply.*)

- a. Religion
- b. Age
- c. Ethnicity
- d. Culture
- e. Geographic location

ANS: A, B, C, D

Religion, values, age, ethnicity, and culture all impact family dynamics, which in turn affect expressions of sexuality. Although geographic location can influence culture, it is not a specific family dynamic important to sexuality.

DIF: Understanding

OBJ: 24.9

TOP: Integrated Process: Caring

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Sexuality

8. The nurse is assessing factors that affect sexual function in patients with chronic diseases.

What topics does the nurse include in the assessment? (*Select all that apply.*)

- a. Fatigue
- b. Medications
- c. Pain
- d. Occupation
- e. Physical impairment

ANS: A, B, C, E

Fatigue, medications, pain, and impairments all can have direct effects on sexuality. Lifestyle is another factor, but occupation does not in itself influence sexuality.

DIF: Remembering

OBJ: 24.8

TOP: Assessment

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Sexuality

Chapter 25: Safety**Yoost & Crawford: Fundamentals of Nursing: Active Learning for Collaborative Practice, 3rd Edition****MULTIPLE CHOICE**

1. The nursing instructor asks the student nurse to identify what Robert Wood Johnson Foundation funded project that focuses on nurses' increased attention to patient safety?
 - a. OSHA (Occupational Safety and Health Agency)
 - b. MSDS (material safety data sheets)
 - c. QSEN (Quality and Safety Education for Nurses)
 - d. ADA (Americans with Disability Act)

ANS: C

QSEN, or the Quality and Safety Education for Nurses, was funded by the RWJ to focus on preparing nurses of the future with the knowledge, skills, and attitudes to advance quality and safety on the job. MSDS are material safety data sheets, OSHA is the Occupational Safety and Health Agency, and ADA is the Americans with Disability Act.

DIF: Remembering

OBJ: 25.1

TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care

NOT: Concepts: Safety

2. The nurse knows changes in which body system affect overall mobility increasing the propensity of falling?
 - a. Neurologic
 - b. Hepatic
 - c. Cardiopulmonary
 - d. Musculoskeletal

NURSINGTB.COM

ANS: D

Impairments in the musculoskeletal system can impact mobility through restrictions of range of motion and strength, increasing the chances of falling. Changes to the neurologic system can impair cognitive functioning, changes to the hepatic system can affect mental status, and changes to the cardiopulmonary system can affect activity tolerance.

DIF: Understanding

OBJ: 25.2

TOP: Assessment MSC:

NCLEX Client Needs Category: Safe and Effective Care

NOT: Concepts: Safety

3. The nurse is visiting a patient with cardiac disease who has been experiencing increased episodes of shortness of breath when exercise is attempted. The nurse is concerned that the patient's decrease in activity may lead to which outcome?
 - a. Orthostatic hypotension
 - b. Increase risk of heart disease
 - c. Loss of short-term memory
 - d. Worsening shortness of breath

ANS: A

Inactivity in patients with cardiopulmonary disease can lead to an unsafe drop in blood pressure with position changes, or orthostatic hypotension. The patient already has heart disease. Loss of short-term memory is not related to the shortness of breath. The lack of activity is not likely to worsen the shortness of breath; improving activity level may help things eventually.

DIF: Applying OBJ: 25.2 TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care

NOT: Concepts: Safety

4. The nurse recognizes conversations about safe sexual practices, including the consequences of unprotected sex such as pregnancy and sexually transmitted infections, are important to begin in what patient population?
- Adults
 - School-aged children
 - Adolescents
 - Older adults

ANS: C

Sexual curiosity and experimentation occur in the adolescent patient population. Conversations about safe sexual practices, including the consequences of unprotected sex, such as pregnancy and sexually transmitted infections, are important. These conversations are also important for adults and older adults but are handled differently in context with their age-related needs. School-aged children may be too young depending on their age and their environment. The nurse must use judgment on when to have the conversation.

DIF: Understanding

OBJ: 25.4

TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care

NOT: Concepts: Safety

NURSINGTB.COM

5. The nurse manager is developing a training guide and identifies which organization that is the best for resources to help develop guidelines to prevent exposure to hazardous situations and decrease the risk of injury in the workplace?
- OSHA (Occupational Safety and Health Administration)
 - CDC (Centers for Disease Control and Prevention)
 - QSEN (Quality and Safety Education for Nurses)
 - NIOSH (National Institute for Occupational Safety and Health)

ANS: A

Occupational Safety and Health Administration (OSHA) was established in 1970 to provide employers with guidelines for preventing exposure to hazardous chemicals and hazardous situations and reducing the risk of injury in the workplace. The CDC is the Centers for Disease Control and Prevention and provides information to address exposure to infectious diseases. QSEN, or the Quality and Safety Education for Nurses, was funded by the RWJ to focus on preparing nurses of the future with the knowledge, skills, and attitudes to advance quality and safety on the job. NIOSH, or the National Institute for Occupational Safety and Health, is a federal agency within the CDC that was established to conduct research and recommend interventions for the prevention of work-related injury and illness.

DIF: Understanding

OBJ: 25.7

TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care

NOT: Concepts: Health Care Organizations

6. The nurse is educating parents about firearm safety. Which parent statement indicates to the nurse a need for further education?
- I should make sure I obtain the proper permits.¶
 - It is okay to store firearms with ammunition loaded.¶
 - I should store all firearms without ammunition.¶
 - I should make sure all firearms have trigger locks in place.¶

ANS: B

Firearms should be stored in a secure location with trigger locks in place. Ammunition should be stored in a separate location also locked. Proper permits should be obtained as appropriate. Loaded firearms should never be stored where children can access them.

DIF: Applying OBJ: 25.4 TOP: Implementation
MSC: NCLEX Client Needs Category: Safe and Effective Care NOT: Concepts: Safety

7. The nurse recognizes that a patient is using a portable generator in the house as a power source. What source of poisoning does the nurse appropriately identify?
- Lead
 - Carbon monoxide
 - Antifreeze
 - Pesticide

ANS: B

Sources of carbon monoxide include automobiles, stoves, gas ranges, portable generators, lanterns, the burning of charcoal and wood, and heating systems. Lead is found in lead-based paints in toys, buildings, and ceramic dishes; sources of lead include water from lead pipes or pipes soldered with lead, gasoline or soil contaminated by gasoline, and household dust that may contain paint chips or soil. Antifreeze and pesticides are liquids.

DIF: Applying OBJ: 25.4 TOP: Assessment
MSC: NCLEX Client Needs Category: Safe and Effective Care NOT: Concepts: Safety

8. The nurse is educating the patient about the proper disposal of medications in the home. Which statement by the patient indicates a good understanding of the information?
- Remove the label from the bottle and throw in the trash.¶
 - Flush the medication down the disposal.¶
 - Mix the medications with kitty litter, place the mixture in a jar, and put the jar in the trash.¶
 - Dissolve the medication in water and pour down the drain.¶

ANS: C

Flushing or pouring the medication down the drain can contaminate the water system. Throwing the medication in the trash poses potential for someone to remove the medication and use it. This can be avoided by mixing it with an undesirable substance like kitty litter or coffee grounds.

DIF: Applying OBJ: 25.7 TOP: Intervention
MSC: NCLEX Client Needs Category: Safe and Effective Care NOT: Concepts: Safety

9. The nurse knows that which patient has a teaching need based on statements by the patient's parents?
- My 6-month-old daughter only sleeps with me when she's ill.¶

- b. —I do not put pillows in the bed with my 3-month-old son.¶
- c. —I do not feed popcorn to my 2-year-old.¶
- d. —I have discussed the risks of the ‘choking game’ with my 16-year-old.¶

ANS: A

Small children should never sleep in the bed with others because of the risk of suffocation. The rest of the statements are appropriate. Pillows do present a hazard to a 3-month-old, and popcorn is a choking risk for a 2-year-old. The choking game is a risk to any adolescent.

DIF: Applying OBJ: 25.4 TOP: Intervention

MSC: NCLEX Client Needs Category: Safe and Effective Care

NOT: Concepts: Patient Education

10. The nurse is teaching a student nurse about restraint use in patients. Which statement by the student nurse indicates a learning need regarding restraints?
- a. —Having all four side rails up on the bed is considered a restraint.¶
 - b. —The use of restraints has been shown to decrease fall-related injuries.¶
 - c. —Death has been associated with the use of restraints.¶
 - d. —Medications administered to control behavior are considered a chemical restraint.¶

ANS: B

Restraints may be physical or chemical. A physical restraint is a mechanical or physical device, such as material or equipment attached or adjacent to the patient's body, used to restrict movement. Examples of physical restraints are wrist or ankle restraints, a jacket or vest, and side rails. A medication that is administered to a patient to control behavior is a chemical restraint. The use of restraints has been associated with patient injury including death and does not prevent patient falls.

DIF: Applying OBJ: 25.4 TOP: Intervention

MSC: NCLEX Client Needs Category: Safe and Effective Care

NOT: Concepts: Safety

11. The nurse displays an understanding of high-risk populations for MRSA when identifying which group as the lowest risk?
- a. Prison inmates
 - b. College dorm residents
 - c. Team athletes
 - d. Food service workers

ANS: D

High-risk populations for MRSA include those living in close quarters or those who have frequent skin-to-skin contact, including prison inmates, college dorm residents, and team athletes. Food service workers work together but do not generally live in close quarters or have skin-to-skin contact frequently.

DIF: Understanding

OBJ: 25.1

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care NOT: Concepts: Infection

12. The nurse knows that which assessment tool is not used to assess fall risk?
- a. Glasgow Falls Scale
 - b. Johns Hopkins Hospital Fall Assessment Tool
 - c. Morse Fall Scale
 - d. Hendrich II Fall Risk Model

ANS: A

The Glasgow is a coma scale used to measure level of consciousness, not falls. The rest are scales used to assess the risk for falls in patients.

DIF: Remembering

OBJ: 25.4

TOP: Assessment MSC:

NCLEX Client Needs Category: Safe and Effective Care

NOT: Concepts: Safety

13. The patient has a nursing diagnosis of Risk for Falls. The nurse identifies which goal to be most important?
- Patient will ambulate twice a day.
 - Patient will have no symptoms of infection.
 - Patient will perform activities of daily living.
 - Patient will have no injuries during hospital stay.

ANS: D

All the goals except lack of infection are appropriate for a patient with a Risk for Falls diagnosis; however, the most important goal is for the patient to have no injuries during the hospitalization.

DIF: Applying

OBJ: 25.5

TOP: Planning

MSC: NCLEX Client Needs Category: Safe and Effective Care

NOT: Concepts: Safety

14. Which collaborative team member would be most effective in assisting the nurse to identify medication alternatives that are less likely to cause drowsiness and dizziness to reduce the risk of falls in the elderly patient?
- Nursing case manager
 - Charge nurse
 - Physical therapist
 - Pharmacist

NURSINGTB.COM

ANS: D

The nurse collaborates with the pharmacist and health care provider to identify and implement safe medication alternatives for older adults to minimize side effects such as drowsiness, dizziness, and orthostatic hypotension, which can increase fall risk. Although case managers and charge nurses might have some experience in this area, pharmacists are educated to focus on medication. Physical therapists evaluate the patient's ability to perform and maintain balance during routine activities such as sitting, standing, and walking.

DIF: Applying

OBJ: 25.6

TOP: Planning

MSC: NCLEX Client Needs Category: Safe and Effective Care

NOT: Concepts: Safety

15. The nurse is concerned about helping the patient find resources to obtain assistive equipment to be used in the home. Which team member should the nurse contact first?
- Occupational therapist
 - Physical therapist
 - Health care provider
 - Social worker

ANS: D

The nurse should collaborate with the social worker to identify community resources for obtaining assistive equipment. The social worker facilitates contact with insurance companies or other agencies to assist with the financing of recommended therapeutic assistive and specialty devices. Occupational therapists evaluate the patient for safe performance of activities of daily living (ADLs) such as bathing, dressing, and grooming, and they make recommendations to enhance safe performance of these activities, such as the use of specialty equipment (e.g., grippers for pants, oversized shoehorns). Physical therapists evaluate the patient's ability to perform and maintain balance during routine activities such as sitting, standing, and walking. They make recommendations for assistive devices such as canes and walkers to promote safe performance of these activities. Health care providers order the equipment.

DIF: Understanding

OBJ: 25.6

TOP: Planning

MSC: NCLEX Client Needs Category: Safe and Effective Care

NOT: Concepts: Collaboration

16. Which statement by the patient indicates to the nurse a teaching need regarding safety in the home?
- I will put a night-light in every room.||
 - I will not use an extension cord to plug in multiple items.||
 - I will wash my throw rugs in the bathroom regularly.||
 - I will keep all cleaning supplies out of reach of children.||

ANS: C

Throw rugs present a fall or tripping hazard. Night-lights help light halls to prevent falls, extension cords can present a trip hazard, and cleaning supplies can contain poisonous materials.

DIF: Applying

OBJ: 25.6

TOP: Planning

MSC: NCLEX Client Needs Category: Safe and Effective Care

NOT: Concepts: Safety

17. The ER nurse is triaging a patient with suspected poisoning. Who should the nurse anticipate contacting first?
- Family services
 - Radiology
 - Poison Control Center
 - Respiratory

ANS: C

If poisoning is suspected, the National Poison Control Center should be contacted immediately. This information will be needed to determine treatment. Respiratory may be needed, and radiology and family services may also be needed, but that will be determined after the treatment plan is determined.

DIF: Applying

OBJ: 25.3

TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care

NOT: Concepts: Caring Behaviors

18. The staff nurse knows that many health care facilities use the fire emergency response defined by which acronym?
- RACE
 - PASS
 - PACE

d. QSEN

ANS: A

RACE stands for rescue, alarm, contain, and extinguish. QSEN is the Quality and Safety Education for Nurses. PASS is pull, aim, squeeze, and sweep for fire extinguishers. PACE is not a health care acronym.

DIF: Understanding

OBJ: 25.4

TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care

NOT: Concepts: Safety

19. The nurse is ambulating a patient back from the bathroom when the patient begins to have a seizure. Which action should the nurse do first?
- Lower the patient to the floor if standing.
 - Move sharp or hard objects away from the patient.
 - Turn the patient's head to the side to prevent aspiration.
 - Attempt to place a tongue blade to prevent choking.

ANS: A

During a seizure, a patient should be protected from injury by first lowering the patient to the ground if standing. The nurse should then place the head on a soft surface and turn it to the side to prevent aspiration and move sharp or hard objects out of the way. The nurse should never attempt to force any object into a seizing patient's mouth.

DIF: Applying

OBJ: 25.7

TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care

NOT: Concepts: Safety

20. The nurse is caring for a confused, combative patient. Which action would be considered last by the nurse to control behavior of the client?
- Orient the patient frequently.
 - Apply restraints.
 - Move the patient to a room close to the nurse's station.
 - Encourage the family to spend time with the patient.

ANS: B

All alternatives to physical restraints should be considered prior to their use.

DIF: Applying

OBJ: 25.7

TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care

NOT: Concepts: Safety

21. The nurse knows which method to be an appropriate way to tie restraints?
- Knot tied to the bed frame
 - Quick-release knot tied to the side rail
 - Bow tied to the bed rail
 - Quick-release ties attached to the bed frame

ANS: D

Restraints should never be tied in a knot because the knot may prohibit a quick exit in the event of an emergency requiring evacuation. Instead, use quick-release ties or mechanisms such as buckles. Restraints are never be tied to side rails because injuries may result when they are raised or lowered. They should be tied to a stable part of the bed such as the frame.

DIF: Understanding

OBJ: 25.7

TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care

NOT: Concepts: Safety

22. Which statement by the nurse correctly identifies the UAP role in patient restraint use?
- The UAP can perform initial assessment.¶
 - The UAP can apply a restraint.¶
 - The UAP can assist with applying and monitoring of a physical restraint.¶
 - The UAP can contact the health care provider and request an order for restraints.¶

ANS: C

The UAP cannot perform the initial assessment, and most facilities require that a registered nurse or licensed practical nurse. Applying a restraint. The health care provider should be contacted by the nurse, not the UAP. The UAP can assist with applying the restraint and can perform monitoring checks under the direction of a Registered Nursing.

DIF: Applying

OBJ: 25.7

TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care

NOT: Concepts: Safety

MULTIPLE RESPONSE

1. The nurse is explaining the National Patient Safety Goals (NPSG) to the student nurse. Which answers indicate that the student has a good understanding of these goals? (*Select all that apply.*)
- The NPSG's focus on treating chronic infections quickly
 - The NPSG's focus on improving staff communication
 - The NPSG's focus on using medications safely
 - The NPSG's focus on identifying patients correctly

ANS: B, C, D

NURSINGTB.COM

The NPSG focus on specific goals each year. The goals for 2018 included: identify patients correctly, improve staff communication, improve the safety of using medications, reduce harm associated with clinical alarm systems, reduce risk of health care-associated infections and the hospital identifies safety risks inherent in its patient population. Although treating chronic infections quickly is important, it is not an NPSG.

DIF: Applying

OBJ: 25.1

TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care

NOT: Concepts: Safety

2. The nurse is providing education to a cardiac patient who has multiple life stressors that are impacting the patient's health. Which statements by the patient indicate a good understanding of actions that can be taken to reduce stressors? (*Select all that apply.*)
- I should change my job.¶
 - I should plan some downtime.¶
 - I should meet with a financial counselor.¶
 - I should talk with my family about my situation.¶
 - I should make my family go to counseling with me.¶

ANS: B, C, D

In adulthood, life stressors such as financial concerns, work-related demands, and efforts to balance work with family life are common challenges that can take a physical toll on the body. Individuals should plan relaxation periods or vacations. Meeting with financial counselors and talking with family can help to achieve that balance. Changing jobs may be beneficial but could also create more stress and forcing family to go to counseling may also not be a wise choice.

DIF: Applying OBJ: 25.7 TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care

NOT: Concepts: Patient Education

3. The nurse is providing education to a community group on environmental safety. Which safety measures are effective in improving their environmental safety? (*Select all that apply.*)
- a. Use of night-lights throughout the home
 - b. Illumination of stairwells and pathways
 - c. Installation of motion-activated lighting on the exterior of the home
 - d. Application of wax to all floors to increase shine
 - e. Staying indoors when air pollution is high

ANS: A, B, C, E

Inadequate lighting presents safety concerns in home, work, community, and health care environments. For an individual to safely and successfully navigate pathways and perform various activities while avoiding potential obstacles and hazards, the environment must be well illuminated. Well-lit, glare-free halls, stairways, rooms, and work spaces help to reduce the risk of tripping, slipping, and falling. Night-lights reduce the risk of injuries to children, guests, and older adults. Lighting the exterior of the house will also reduce the risk of falling.

Staying indoors during episodes when air pollution is high can help prevent chronic lung disease.

Waxed floors are slippery.

DIF: Applying OBJ: 25.7 TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care

NOT: Concepts: Patient Safety

Chapter 26: Asepsis and Infection Control**Yoost & Crawford: Fundamentals of Nursing: Active Learning for Collaborative Practice, 3rd Edition**

MULTIPLE CHOICE

1. The nurse recognizes which term to identify the second line of defense that leads to local capillary dilation and leukocyte infiltration?
- Normal flora
 - Inflammatory response
 - Immune response
 - Humoral immunity

ANS: B

The second line of defense is the inflammatory response. Inflammation is a local response to cellular injury or infection that includes capillary dilation and leukocyte infiltration. Normal flora is the body's first line of defense. The immune response is the body's attempt to protect itself from foreign and harmful substances. Humoral immunity is a defense system that involves white blood cells (B lymphocytes) that produce antibodies in response to antigens or pathogens circulating in the lymph and blood.

DIF: Understanding

OBJ: 26.1

TOP: Assessment MSC:

NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:

Concepts: Infection

2. The nurse knows that the antigen-antibody reaction is an example of what type of immunity?
- Humoral
 - Cellular
 - Innate
 - Passive

ANS: A

Humoral immunity is a defense system that involves antibodies and white blood cells that are produced to fight antigens. Cellular immunity involves defense by white blood cells against any microorganisms that the body does not recognize as its own. The innate (nonspecific) immune system provides immediate defense against foreign antigens. Passive immunity occurs when a person receives an antibody produced in another body.

DIF: Understanding

OBJ: 26.1

TOP: Assessment MSC:

NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:

Concepts: Infection

3. The nurse uses what term to identify a disease-causing organism?
- Pathogen
 - Normal flora
 - Germ
 - Microorganism

ANS: A

Infectious agents include any disease-causing agent and are called pathogens. They include bacteria, fungi, viruses, and parasites. Normal flora is a group of non-disease-causing microorganisms that live in or on the body. Germ is a term used for microorganism. A microorganism is bacteria, fungi, or protozoa.

DIF: Understanding

OBJ: 26.1

TOP: Assessment MSC:

NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:

Concepts: Infection

4. The nurse is explaining to the patient why antibiotics are being administered. The answer would be correct if the nurse stated antibiotics are effective against which microorganism?
- Viruses
 - Fungi
 - Parasites
 - Bacteria

ANS: D

Antibiotics are effective against bacteria, and exact antibiotic sensitivity is tested so that appropriate antibiotics are prescribed. Infections that are caused by fungi are treated with antifungal medications. Certain antiviral medications are used to manage the symptoms of a viral infection. These medications, if given during the early phases of illness, can decrease the amount of time that the patient has viral symptoms. Treatment for parasitic infections varies depending on type of parasite.

DIF: Understanding

OBJ: 26.2

TOP: Teaching/Learning

MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:

Concepts: Infection

NURSINGTB.COM

5. The nurse anticipates correctly that what medication category would be ordered to treat athlete's foot?
- Antiviral
 - Antibiotic
 - Antihelminth
 - Antifungal

ANS: D

The nurse would expect to treat athlete's foot with an antifungal because it is a fungal infection. An antibiotic treats bacterial infections, antivirals treat viral infections, and antihelminth treats parasitic worms.

DIF: Applying

OBJ: 26.2

TOP: Implementation

MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:

Concepts: Infection

6. The nurse recognizes that the stethoscope most correctly represents which possible link in the chain of infection?
- Source
 - Portal of exit
 - Portal of entry
 - Mode of transmission

ANS: D

The stethoscope would be a means for the pathogen to travel from source to host. The source is the reservoir or host. The portal of exit is where the pathogen escapes from the reservoir of infection, and the portal of entry is where the microorganism enters the susceptible host.

DIF: Understanding

OBJ: 26.2

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Safety and Infection Control

NOT: Concepts: Infection

7. The nurse is teaching a group of patients about diseases that are transmitted by ticks. Which term would the nurse use when identifying the function of a tick in spreading disease?
- Vectors
 - Bacteria
 - Viruses
 - Fungi

ANS: A

Vectors carry pathogens from one host to another. Bacteria are single-cell organisms. Viruses are the smallest organisms. Fungi are single-cell organisms that can cause infection.

DIF: Understanding

OBJ: 26.2

TOP: Teaching/Learning

MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation

NOT: Concepts: Infection

8. What response would the nurse provide to correctly identify the most effective method to prevent hospital-acquired infections?
- Use of sterile technique
 - Isolation protocols
 - Antibiotic use
 - Handwashing

NURSINGTB.COM

ANS: D

Handwashing is the most effective method to prevent hospital-acquired infections. Sterile technique is only used for certain procedures and isolation protocols are used for patients already infected or for protective isolation in immune-compromised patients and are not used for every patient. Antibiotics are used to treat infections.

DIF: Understanding

OBJ: 26.2

TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Safety and Infection Control

NOT: Concepts: Infection

9. The nurse correctly identifies which patient as having the greatest risk for infection?
- An 80-year-old male with an enlarged prostate
 - A 24-year-old female long-distance runner
 - A 50-year-old obese male
 - A 40-year-old sexually active female

ANS: A

The 80-year-old male has more risk factors because he is elderly and has increased risk of urinary tract infection related to prostate enlargement, so he has two risk factors. A 24-year-old female runner is likely healthy with no additional risk factors. The 50-year-old obese male has one additional risk factor. The 40-year-old sexually active female may not have additional risk factors if she is using protection and does not have multiple partners.

DIF: Analyzing OBJ: 26.2 TOP: Assessment

MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:
Concepts: Infection

10. The nurse understands that which set of vital signs most likely indicates infection?
- T: 98.6 °F (37.0 °C), P: 75 beats/min, R: 18 breaths/min, BP 120/80 mm Hg
 - T: 99 °F (37.2 °C), P: 80 beats/min, R: 18 breaths/min, BP: 110/70 mm Hg
 - T: 100.5 °F (38 °C), P: 96 beats/min, R: 22 breaths/min, BP: 150/100 mm Hg
 - T: 98.9 °F (37.1 °C), P: 66 beats/min, R: 18 breaths/min, BP: 98/62 mm Hg

ANS: C

With infection, temperature will rise and blood pressure will increase along with pulse and respiratory rate.

DIF: Analyzing OBJ: 26.3 TOP: Assessment

MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:
Concepts: Infection

11. A patient admitted after abdominal surgery has a Nursing diagnosis of risk for infection. The nurse identifies which goal to be most appropriate?
- Patient will ambulate length of hallway this shift.
 - Patient will consume 20% of meals by the end of the week.
 - Patient's incision will be without signs or symptoms of infection at discharge.
 - Patient will verbalize need to stop antibiotics medication when symptom free.

ANS: C

Maintaining skin integrity is an appropriate goal for this patient to ensure the patient does not develop a wound infection. Ambulating will assist in preventing skin breakdown by getting the patient out of bed, but it is not the priority goal for a patient with an incision. Consuming only 20% of meals will not ensure adequate nutrition and verbalizing the end of antibiotic administration to be when symptoms end is inappropriate. Antibiotics should be taken until the prescription is complete.

DIF: Analyzing OBJ: 26.4 TOP: Planning

MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:
Concepts: Infection

12. The nurse is caring for a patient who is comatose. When performing oral hygiene, which interval is most appropriate?
- Every shift
 - Twice daily
 - Every 4 hours
 - Daily

ANS: C

Oral care should be performed every 4 hours to prevent the colonization of bacteria. Less often than every 4 hours is not effective.

DIF: Applying OBJ: 26.6 TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Safety and Infection Control
NOT: Concepts: Infection

13. The nurse knows which skill does not require the use of sterile technique?
- NG tube insertion
 - Foley catheterization
 - Tracheostomy care
 - PICC line insertion

ANS: A

NG tube insertion requires a clean, not sterile, technique as the gastrointestinal tract is not sterile. Use strict aseptic technique when inserting an intravenous (IV) or Foley catheter and when performing suctioning of the lower airway.

DIF: Applying OBJ: 26.6 TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Safety and Infection Control

NOT: Concepts: Infection

14. The nurse recognizes which situation to be inappropriate to use alcohol-based hand sanitizer?
- Patient with pneumonia
 - Patient with *Clostridium difficile*
 - Status post-appendectomy
 - Patient with HIV

ANS: B

Soap and water must be used to thoroughly clean hands if there is any visible soiling or dirt and with certain infections such as *Clostridium difficile* and vancomycin-resistant enterococci when preparing for a sterile or surgical procedure, before and after eating, and after using the restroom. In the other situations, a hand sanitizer is as effective as soap and water.

DIF: Applying OBJ: 26.6 TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Safety and Infection Control

NOT: Concepts: Infection

15. The nurse is preparing to perform suctioning on a new tracheostomy with the potential for forceful expulsion of secretions and identifies what PPE (personal protective equipment) should be worn?
- Gloves and eyewear
 - Gloves, gown, and mask
 - Eyewear and gown
 - Eyewear, mask, gown, and gloves

ANS: D

Use gloves routinely when blood or body fluid might be present. If splashing is possible, use your nursing judgment about what other PPE might be necessary. Forceful expulsion of secretions would require all PPE—gown, mask, eyewear, and gloves—to provide adequate protection.

DIF: Applying OBJ: 26.6 TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Safety and Infection Control

NOT: Concepts: Infection

16. Which isolation precaution should the nurse implement for the patient who has been diagnosed with hepatitis A?

- a. Airborne
- b. Contact
- c. Droplet
- d. Protective

ANS: B

Contact precautions are used when a known or suspected contagious disease may be present and is transmitted through direct contact with the patient or indirect contact with items in the patient's environment. Airborne precautions are used when known or suspected contagious diseases can be transmitted by means of small droplets or particles that can remain suspended in the air for prolonged periods. Droplet precautions are used when known or suspected contagious diseases can be transmitted through large droplets suspended in the air. Protective isolation is used for patients who have compromised immune systems.

DIF: Applying OBJ: 26.6 TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Safety and Infection Control

NOT: Concepts: Infection

17. When the patient is diagnosed with pertussis, which isolation precaution should the nurse implement?
- a. Droplet
 - b. Airborne
 - c. Contact
 - d. Protective

ANS: A

Droplet precautions are used when known or suspected contagious diseases can be transmitted through large droplets suspended in the air. Contact precautions are used when a known or suspected contagious disease may be present and is transmitted through direct contact with the patient or indirect contact with items in the patient's environment. Airborne precautions are used when known or suspected contagious diseases can be transmitted by means of small droplets or particles that can remain suspended in the air for prolonged periods.

DIF: Applying OBJ: 26.6 TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Safety and Infection Control

NOT: Concepts: Infection

18. When teaching a student nurse about removing PPE, the nurse would include which correct order of equipment removal?
- a. Gloves, eyewear, gown, and mask
 - b. Mask, eyewear, gown, and gloves
 - c. Gown, mask, eyewear, and gloves
 - d. Gloves, gown, mask, and eyewear

ANS: A

When removing PPE, gloves, which are contaminated, are removed first to prevent contamination of the face and eyes during removal of the mask and to prevent spread of microorganisms. Eyewear should then be removed, followed by the gown and finally the mask.

DIF: Applying OBJ: 26.6 TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Safety and Infection

Control

NOT: Concepts: Infection

19. When the nurse is wearing sterile gloves, which action would result in the gloves becoming nonsterile?
- Fold gloved hands until procedure begins.
 - Change a dressing using aseptic technique.
 - Place sterile gloved hands below waist.
 - Use correct protocol when donning sterile gloves.

ANS: C

Once the hands have been placed below the waist, they can longer be considered sterile or free from organisms. Asepsis refers to freedom from disease-causing contamination. All other choices maintain asepsis.

DIF: Applying

OBJ: 26.6

TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Safety and Infection Control

NOT: Concepts: Infection

MULTIPLE RESPONSE

1. The nurse is planning care for an elderly patient. The nurse recognizes the patient is at risk for respiratory infections based on which factors? (*Select all that apply.*)
- Decreased cough reflex
 - Decreased lung elasticity
 - Increased activity of the cilia
 - Abnormal swallowing reflex
 - Increased sputum production

ANS: A, B, D

The elderly are at an increased risk for respiratory infections because of decreased cough reflex, decreased elastic recoil of the lungs, decreased activity of the cilia, and abnormal swallowing reflex. They do not generally have increased sputum production.

DIF: Applying

OBJ: 26.2

TOP: Assessment

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Safety and Infection Control

NOT: Concepts: Infection

2. The nurse is providing education to a patient who is being discharged home on antibiotic therapy. Which statement(s) by the patient indicates further education is needed? (*Select all that apply.*)
- I should take antibiotics every time I am sick.||
 - I should take all antibiotics as prescribed.||
 - I should save all unused antibiotics.||
 - I should stop taking antibiotics when I feel better.||
 - If I develop a rash while taking these I will call the provider.||

ANS: A, C, D

The overuse of antibiotics and inappropriate use, such as not completing prescriptions and sharing antibiotics, has led to increased resistance. Taking antibiotics as prescribed helps to ensure the infection will be treated correctly. A rash may indicate an allergic reaction and the patient needs to report this to the provider.

DIF: Evaluating OBJ: 26.2 TOP: Evaluation
 MSC: NCLEX Client Needs Category: Physiological Integrity: Pharmacological and Parenteral Therapies
 NOT: Concepts: Infection

3. The nurse recognizes which statements by the student nurse regarding handwashing indicate a need for further education? (*Select all that apply.*)
- a. Wash hands first, then wrists.
 - b. Rinse from fingertips to wrists.
 - c. Dry using a scrubbing motion.
 - d. Turn off faucet with clean, dry paper towel.
 - e. Dry the hands in the same order as washing them.

ANS: A, B, C

When washing hands, first wet the wrists and hands; with fingers pointing downward, first wash the wrists and then the hands below the wrists. Then apply soap, lather, and rub using a circular motion for 15 to 20 seconds. When rinsing, rinse from wrist to fingertips, keeping hands with fingers pointing downward. Using clean paper towels, dry thoroughly in the same order (from wrists to fingers) using a patting motion. Turn off the faucet with a clean, dry paper towel.

DIF: Evaluating OBJ: 26.6 TOP: Evaluation
 MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Safety and Infection Control
 NOT: Concepts: Infection

4. The nurse knows that standard precautions are indicated for which group(s) of patients? (*Select all that apply.*)
- a. All patients
 - b. Patients with HIV
 - c. Patients with MRSA
 - d. Patients with tuberculosis
 - e. Patients who are bleeding

NURSINGTB.COM

ANS: A, E

The nurse can take steps at any link in the chain to halt the spread of infection. Standard precautions are used with all patients to limit direct exposure to blood and body fluids. The other choices are additional precautions such as airborne precautions are used with patients who have diseases such as tuberculosis and contact precautions with patients who have MRSA.

DIF: Applying OBJ: 26.6 TOP: Nursing Process: Implementation
 MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Safety and Infection Control
 NOT: Concepts: Infection

5. The patient is on protective precautions. The nurse knows which statements are true regarding these precautions? (*Select all that apply.*)
- a. A positive-pressure room with a HEPA filtration system is required.
 - b. Special respirator masks should be available and one size fits all.
 - c. No live plants are allowed in the room.
 - d. The patient may eat any foods desired.
 - e. Everyone entering the room wears a mask.

ANS: C, E

Protective precautions may require a positive-pressure room. No live plants, fresh flowers, fresh raw fruit or vegetables, sushi, or blue cheese may be brought into the room because they may harbor bacteria and fungi. The patient cannot eat just any foods because some are restricted. A mask is required for anyone entering the room and for the patient if leaving the room.

DIF: Applying OBJ: 26.6 TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Safety and Infection Control

NOT: Concepts: Infection

NURSINGTB.COM

Chapter 27: Hygiene and Personal Care**Yoost & Crawford: Fundamentals of Nursing: Active Learning for Collaborative Practice, 3rd Edition****MULTIPLE CHOICE**

1. The nurse knows that which statement is true regarding the importance of hygiene?
 - a. The nurse can assess other body systems during the bath.
 - b. UAPs perform hygiene because there is no benefit of nurses doing this care.
 - c. The mucous membranes of the lips, nostrils, anus, vagina, and urethra are not a part of the integumentary system when providing hygiene.
 - d. The main purpose of bathing is to decrease the patient's body odor.

ANS: A

The bath is an excellent opportunity for the nurse to assess multiple body systems. Although the UAP can perform hygiene, there is benefit to the nurse doing it because of the ability to assess the patient. The mucous membranes are a part of the integumentary system, and bathing cleanses the skin, reduces odor, provides comfort, and contributes to the patient's health and well-being.

DIF: Applying OBJ: 27.1 TOP: Implementation

MSC: NCLEX Client Needs Category: Physiological Integrity: Basic Care and Comfort NOT:

Concepts: Caregiving

2. Excessively dry skin can lead to cracks and openings in the integumentary system. Based on this, what is the most applicable Nursing diagnosis for a patient with excessively dry skin?
 - a. Impaired Health Maintenance
 - b. Risk for Injury
 - c. Risk for infection
 - d. Acute pain

ANS: C

Any interruption in the skin, which is the body's first line of defense, can potentially lead to infection. Impaired health maintenance could have dry skin as a symptom. Acute pain and risk for injury are not appropriate.

DIF: Evaluating OBJ: 27.4 TOP: Diagnosis

MSC: NCLEX Client Needs Category: Physiological Integrity: Basic Care and Comfort NOT:

Concepts: Caregiving

3. The nurse correctly identifies which patient as having the highest risk for injury related to temperature of water when bathing?
 - a. Patient with asthma
 - b. Patient with attention deficit hyperactivity disorder
 - c. Patient with a stroke
 - d. Patient with diabetes

ANS: D

Patients with neurologic deficits such as peripheral neuropathy resulting from diabetes may not be able to identify extremes of hot and cold. Patients with attention deficit hyperactivity disorder and asthma are not likely to be injured by temperature extremes. Patients with a stroke may have some alteration in sensation on one side of their body but can compensate by using the other side, and they are at less risk than a patient with diabetes.

DIF: Analyzing OBJ: 27.2 TOP: Assessment
 MSC: NCLEX Client Needs Category: Physiological Integrity: Basic Care and Comfort NOT:
 Concepts: Caregiving

4. Which tool is used by the nurse to determine risk for impaired skin integrity?
- Braden scale
 - Glasgow scale
 - Vanderbilt scale
 - MMSE scale

ANS: A

The Braden scale is used to determine risk for impaired skin integrity: The Glasgow is a coma scale, the Vanderbilt is a behavior scale, and the MMSE is the mini-mental exam to determine cognitive status.

DIF: Understanding OBJ: 27.3 TOP: Assessment
 MSC: NCLEX Client Needs Category: Health Promotion and Maintenance
 NOT: Concepts: Caregiving

5. The nurse is providing care to a post-stroke patient on the rehabilitation floor with a nursing diagnosis of Impaired health maintenance. Which goal is most appropriate on day one?
- Patient will ambulate independently twice a day.
 - Patient will perform all own ADLs.
 - Patient will consume 75% of all meals.
 - Patient will begin to perform 25% of own ADLs.

ANS: D

The patient needs to work toward achieving as much independence in self-care as possible; starting with 25% in a post-stroke patient on day one is more achievable than 100%.

Ambulating and eating meals are not goals for a problem with self-care.

DIF: Analyzing OBJ: 27.5 TOP: Planning
 MSC: NCLEX Client Needs Category: Physiological Integrity: Basic Care and Comfort NOT:
 Concepts: Caregiving

6. The nurse is preparing to give a patient a complete bed bath. What area of the body should be bathed first?
- Hands
 - Eyes
 - Face
 - Arms

ANS: B

The nurse should start washing the patient's eye area, using a washcloth without soap, followed by the patient's face, hands, and arms.

DIF: Understanding

OBJ: 27.6

TOP: Implementation

MSC: NCLEX Client Needs Category: Physiological Integrity: Basic Care and Comfort NOT:

Concepts: Caregiving

7. The UAP asks why the arms are washed from distal to proximal. Which response by the nurse is appropriate?
- To promote circulation
 - To maintain asepsis
 - To maintain comfort
 - To maintain tradition

ANS: A

Washing from distal to proximal promotes circulation and blood return. Asepsis is the state of being free from disease-causing contaminants. There is no difference in comfort. Tradition is a custom.

DIF: Understanding

OBJ: 27.6

TOP: Implementation

MSC: NCLEX Client Needs Category: Physiological Integrity: Basic Care and Comfort NOT:

Concepts: Caregiving

8. The nurse has assisted the patient to wash the hands, face, axillae, and perineal area. What type of bath does the nurse chart?
- Sink bath
 - Complete bed bath
 - Partial bed bath
 - Shower

ANS: C

A partial bed bath is performed when only part of the body is washed. A complete bed bath is for patients who are completely bedridden or are totally dependent on others for care. A shower is usually for patients who are strong enough to shower independently. A sink bath is when the patient washes while standing or sitting in front of a bath basin or sink.

DIF: Understanding

OBJ: 27.6

TOP: Implementation

MSC: NCLEX Client Needs Category: Physiological Integrity: Basic Care and Comfort NOT:

Concepts: Caregiving

9. The nurse is performing perineal care for the uncircumcised patient. Which action does the nurse take?
- Does not move the foreskin.
 - Retracts the foreskin, pulling it away from the body.
 - Leaves the foreskin retracted, allowing it to return to position naturally after care.
 - Retracts the foreskin and returns it to its natural position after cleaning, rinsing, and drying.

ANS: D

The foreskin must be returned to its normal position after cleaning to prevent contraction and swelling. It is okay to move the foreskin to clean the penis. To retract the foreskin, gently push it toward the body. It should be returned to its position by the nurse, not left to return on its own.

DIF: Applying

OBJ: 27.6

TOP: Implementation

MSC: NCLEX Client Needs Category: Physiological Integrity: Basic Care and Comfort NOT:
Concepts: Caregiving

10. Which member of the collaborative team is most appropriate to cut the toenails of a diabetic patient?
- Nurse
 - Physical therapist
 - Occupational therapist
 - Podiatrist

ANS: D

Patients with diabetes are usually seen by a podiatrist or diabetic specialist for foot care needs. Nurses can trim toenails of patients not at risk for infection. Physical therapists provide services that restore function and mobility. Occupational therapists use treatments to maintain or restore daily living and work skills.

DIF: Remembering OBJ: 27.6 TOP: Implementation
MSC: NCLEX Client Needs Category: Physiological Integrity: Basic Care and Comfort NOT:
Concepts: Caregiving

11. When providing the patient with routine hygienic care, which action would the nurse omit?
- Massage the back with lotion
 - Oral care with a toothbrush
 - Shaving with a disposable razor
 - Ear hygiene with cotton-tipped applicators

ANS: D

Cotton-tipped swabs or applicators should not be used in the ears for cleaning because this can push wax farther into the ears. A back massage may be given as part of a complete bed bath. Oral care is an essential nursing intervention that provides patient comfort, removes plaque and bacteria, reduces the risk of tooth decay, and decreases halitosis. Oral care includes brushing the teeth and tongue, flossing, rinsing the mouth, and cleaning dentures. Shaving a patient may be part of hygienic care and can be done with a disposable or electric razor.

DIF: Applying OBJ: 27.6 TOP: Implementation
MSC: NCLEX Client Needs Category: Physiological Integrity: Basic Care and Comfort NOT:
Concepts: Caregiving

12. The nurse is caring for a patient with swallowing concerns and decreased level of consciousness. The nurse knows to put the patient in what position for oral care?
- High Fowler's
 - Prone
 - Side-lying
 - Low Fowler's

ANS: C

The side-lying position should be used to prevent aspiration. The high Fowler's, low Fowler's, and prone position will not prevent aspiration.

DIF: Applying OBJ: 27.6 TOP: Implementation
MSC: NCLEX Client Needs Category: Physiological Integrity: Basic Care and Comfort NOT:
Concepts: Caregiving

13. What action by the nurse is inappropriate regarding denture care?
- Carrying the dentures to the sink wrapped in a paper towel
 - Placing a towel in the sink and brushing the dentures over the towel
 - Brushing the dentures as the nurse would the teeth of a conscious patient
 - Applying adhesive, then inserting upper and then lower dentures

ANS: A

Dentures should not be wrapped in a paper towel; they should be placed in the denture cup to carry them to the sink. The towel prevents the dentures from being damaged if the teeth are dropped. The nurse can brush the dentures as she would the teeth of a conscious patient. Apply denture adhesive (if used) and insert the dentures, inserting first the upper and then the lower plates, using 4 x 4 inch gauze.

DIF: Understanding

OBJ: 27.6

TOP: Implementation

MSC: NCLEX Client Needs Category: Physiological Integrity: Basic Care and Comfort NOT:

Concepts: Caregiving

14. What statement by the nurse is true regarding oral care of patients on anticoagulants?
- Use an electric toothbrush daily.
 - Avoid oral care.
 - Use mouthwash only.
 - Use a soft-bristled toothbrush.

ANS: D

Oral care is important regardless of medication, but a soft-bristled toothbrush should be used related to increased risk of bleeding for any patient on an anticoagulant. An electric toothbrush is too aggressive, and mouthwash is not adequate.

DIF: Understanding

OBJ: 27.6

TOP: Implementation

MSC: NCLEX Client Needs Category: Physiological Integrity: Basic Care and Comfort NOT:

Concepts: Caregiving

15. The nurse is assisting a patient to insert contacts and a contact is dropped. What action should occur next?
- Moisten the finger with lens solution and gently touch it to pick it up.
 - Moisten the contact lens with tap water and pick it up.
 - Pick it up and insert the contact lens.
 - Discard the contact lens.

ANS: A

If a lens is dropped, do the following: (1) moisten a finger with the lens solution, and then gently touch the lens with the moistened finger to pick it up. (2) Clean, rinse, and disinfect the lens to avoid a potential eye infection from any microorganisms that might have adhered to the lens. The contact lens does not need to be discarded and tap water should not be used for contact lens.

DIF: Understanding

OBJ: 27.6

TOP: Implementation

MSC: NCLEX Client Needs Category: Physiological Integrity: Basic Care and Comfort NOT:

Concepts: Caregiving

16. The nurse recognizes which statement by the patient indicates a teaching need?

- a. —I use bobby pins to remove excessive ear wax.||
- b. —I use soap and a warm cloth to clean the outside of my ear.||
- c. —My doctor sometimes gives me oil drops for my ears.||
- d. —I never use Q-Tips.||

ANS: A

Washing the ear with a washcloth and soap is sufficient in most patients. If the patient has a buildup of wax, or cerumen, the health care provider may order special oil drops to soften the wax before irrigating the ear canal. Do not try to remove the wax using a cotton-tipped applicator because this can push the wax farther into the ear canal. Caution patients to never insert anything sharp into the ear, such as bobby pins. Sharp objects can rupture the tympanic membrane.

DIF: Understanding OBJ: 27.6 TOP: Implementation
 MSC: NCLEX Client Needs Category: Physiological Integrity: Basic Care and Comfort NOT:
 Concepts: Caregiving

17. The nurse is asked to shave a patient who is taking warfarin (Coumadin). What is the most appropriate action?
- a. Refuse to shave the patient because he is on an anticoagulant.
 - b. Shave as usual with a safety razor.
 - c. Offer to wax rather than shave the patient.
 - d. Use an electric razor.

ANS: D

Patients on anticoagulants should use an electric razor for shaving to avoid bleeding complications. Patients should have the option of shaving if they would like to shave. Waxing may not be an option.

NURSINGTB.COM

DIF: Applying OBJ: 27.6 TOP: Implementation
 MSC: NCLEX Client Needs Category: Physiological Integrity: Basic Care and Comfort NOT:
 Concepts: Caregiving

18. The nurse and UAP are making an occupied bed together. Which action by the nurse is incorrect?
- a. The nurse asks and assists the patient to turn toward the UAP and loosens the fitted sheet and rolls it in toward the patient.
 - b. The nurse rolls dirty linens to the side then places the linens on the floor while finishing.
 - c. The nurse tucks the clean bottom sheet under the cleaner underside of the dirty linens.
 - d. The nurse wears gloves to remove dirty linens.

ANS: B

Bed linens should be placed in the linen hamper, not on the floor, after they are removed from the bed. The patient turns to each side while the bed linens are changed, and the nurse wears gloves.

DIF: Understanding OBJ: 27.6 TOP: Implementation
 MSC: NCLEX Client Needs Category: Physiological Integrity: Basic Care and Comfort NOT:
 Concepts: Caregiving

MULTIPLE RESPONSE

1. The nurse knows that which areas of the patient's body are at increased risk of excoriation?

(Select all that apply.)

- a. Exposed areas such as the face
- b. Areas exposed to stool
- c. Skin on skin areas
- d. Area under pendulous breasts
- e. Under an abdominal fold

ANS: B, C, D, E

Excoriation (red, scaly areas with surface loss of skin tissue) occurs in patients whose skin is exposed to bodily fluids such as stool, urine, or gastric juices. Excoriation also occurs in areas where skin rests on skin, such as in the axilla (armpit); under large, pendulous breasts; or in abdominal folds. Exposed areas are more likely to become sun burned or wind burned.

DIF: Remembering

OBJ: 27.2

TOP: Assessment

MSC: NCLEX Client Needs Category: Physiological Integrity: Basic Care and Comfort NOT:

Concepts: Caregiving

2. The nurse is demonstrating cultural sensitivity in performing perineal care when carrying out which actions? (Select all that apply.)

- a. The male nurse delegates perineal care of a female patient to the female UAP.
- b. The male nurse asks a female patient if she would prefer a female to perform care.
- c. The nurse approaches the care in a sensitive, professional manner.
- d. The nurse assesses cultural preferences of the patient prior to care.
- e. The nurse provides care quickly and in a matter of fact manner.

ANS: B, C, D

The nurse assesses patient backgrounds and provides hygienic care in a manner that is sensitive to the differences in habits and customs. This includes asking the patient about their preferences and not assuming what their preferences will be. A female patient may be comfortable with a male nurse performing perineal care. The nurse should not perform the care without asking first and should not perform the task quickly.

DIF: Applying

OBJ: 27.2

TOP: Implementation

MSC: NCLEX Client Needs Category: Physiological Integrity: Basic Care and Comfort NOT:

Concepts: Caregiving

3. When the nurse is assisting patients with hygiene care, which tasks should be included?

(Select all that apply.)

- a. Bathing
- b. Oral care
- c. Perineal care
- d. Foot care
- e. Patient communication

ANS: A, B, C, D, E

Hygienic practices include bathing, oral care, perineal care (cleansing of the genital area, urinary meatus, and anus), foot care, and shaving. During hygiene care the nurse communicates with the patient, assesses the skin, and observes for any abnormalities.

DIF: Applying OBJ: 27.3 TOP: Implementation
 MSC: NCLEX Client Needs Category: Physiological Integrity: Basic Care and Comfort NOT:
 Concepts: Caregiving

4. The nurse is bathing a patient and notes reddened skin above the coccyx. Which actions by the nurse are appropriate? (*Select all that apply.*)
- a. Apply a barrier cream and massage the area.
 - b. Document the findings.
 - c. Position the patient to relieve pressure on coccyx.
 - d. Report the area to the charge nurse.
 - e. Report the new finding to the provider.

ANS: B, C, D, E

Gently wash any reddened or swollen areas and pat them dry. Use clean, nonsterile gloves as needed to comply with standard precautions. Document the findings from the assessment and report them to the provider, charge nurse, or other appropriate personnel per agency policies. Avoid massaging reddened areas on the skin during the bath. Further tissue breakdown can occur if reddened areas are massaged.

DIF: Applying OBJ: 27.6 TOP: Implementation
 MSC: NCLEX Client Needs Category: Physiological Integrity: Basic Care and Comfort NOT:
 Concepts: Caregiving

5. Regarding perineal care, which nursing actions are appropriate? (*Select all that apply.*)
- a. The nurse applies gloves prior to performing perineal care.
 - b. The nurse ignores the erection of a male patient during perineal care.
 - c. The nurse documents the perineal care.
 - d. The nurse only completes perineal care with daily bathing.
 - e. The nurse can delegate perineal care.

ANS: A, B, C, E

The nurse uses standard precautions (gloves) whenever contact with body fluids is expected. A male patient may have an erection during care, which is a normal response with tactile stimulation. The care provider can ignore the erection and continue with the procedure or return later to complete the care, depending on the comfort level and the situation. Documentation is part of hygienic care. Note any redness, drainage, odor, edema, or skin changes. Perineal care is provided during a bath or shower but may be necessary more frequently, especially in incontinent patients. Perineal care can be delegated.

DIF: Applying OBJ: 27.6 TOP: Implementation
 MSC: NCLEX Client Needs Category: Physiological Integrity: Basic Care and Comfort NOT:
 Concepts: Caregiving

6. The nurse should avoid soaking the feet of which patient population? (*Select all that apply.*)
- a. Patients with peripheral vascular disease
 - b. Patients with a stroke
 - c. Patients with diabetes

- d. Patients with arthritis
- e. Patients who are malnourished

ANS: A, B, C

Soaking the feet of patients with peripheral vascular disease, cardiovascular disease such as strokes and diabetes are contraindicated because it may cause skin breakdown or infection. Patient with arthritis or malnourished have no contraindications to having their feet soaked.

DIF: Applying OBJ: 27.6

TOP: Implementation

MSC: NCLEX Client Needs Category: Physiological Integrity: Basic Care and Comfort NOT:

Concepts: Caregiving

7. The nurse notes that a trauma patient has multiple tangles in the hair. Which actions taken by the nurse are appropriate? (*Select all that apply.*)
- a. Work the tangles to the ends of the hair, then trim with scissors.
 - b. Apply warm water and conditioner.
 - c. Apply detangler as available.
 - d. Use a comb or fingers to work through tangles.
 - e. Cut the tangles out if working on them agitates the patient.

ANS: B, C, D

Apply warm water and a conditioner or a detangler, if available, to release tangles and avoid injury to the scalp. Use a comb and/or fingers to work through the tangles individually before shampooing. The nurse avoids cutting the patient's hair unless first asking the patient's permission.

DIF: Applying

OBJ: 27.6

TOP: Implementation

MSC: NCLEX Client Needs Category: Physiological Integrity: Basic Care and Comfort NOT:

Concepts: Caregiving

Chapter 28: Activity, Immobility, and Safe Movement**Yoost & Crawford: Fundamentals of Nursing: Active Learning for Collaborative Practice, 3rd Edition****MULTIPLE CHOICE**

1. What response would the nurse give the patient when questioned about the effect of rheumatoid arthritis on the musculoskeletal system?
 - a. Muscle weakness
 - b. Muscle wasting
 - c. Joint inflammation
 - d. Joint spasticity

ANS: C

Rheumatoid arthritis and osteoarthritis cause inflammation of joints, resulting in pain and limited joint mobility, not muscle mobility. Genetic disorders such as muscular dystrophy result in muscle weakness and gradual muscle wasting. Spasticity (increased muscle tone) occurs in developmental disorders, such as cerebral palsy, and results in reduced range of motion (ROM) and abnormal movement patterns.

DIF: Understanding

OBJ: 28.2

TOP: Assessment MSC:

NCLEX Client Needs Category: Physiological Integrity NOT: Concepts: Mobility

2. The nurse is implementing generalized falls precautions for patients who are at risk for falls. Which intervention indicates a lack of understanding of these precautions?
 - a. The bed is placed in the low position.
 - b. The patient is wearing socks.
 - c. The patient's cell phone is by the bedside.
 - d. The patient's call light is within reach.

ANS: B

If the patient is ambulatory, require the use of nonskid footwear. Socks can be slippery unless they have a grip surface on them. Keep patient belongings (e.g., tissues, water, urinals, personal items) within the patient's reach. Keep the call light in reach and remind the patient to use it and keep the bed in the low position.

DIF: Understanding

OBJ: 28.6

TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment

NOT: Concepts: Mobility

3. The nurse is educating the family of a patient on falls risk precautions. Which statement by the family indicates a need for further education?
 - a. —I should keep the wheelchair locked unless using it to move Mom.¶
 - b. —I should leave the bathroom light on as she does at her home.¶

ANS: C

- c. —I should leave her slippers by the wheelchair.¶
- d. —I should keep her cell phone close to her bed.¶

ANS: C

Leave lights on or off at night, depending on the patient's cognitive status and personal preference. Keep the wheels of any wheeled device (e.g., bed, wheelchair) in the locked position. Keep patient belongings (e.g., tissues, water, urinals, personal items) within the patient's reach. If the patient is ambulatory, require the use of nonskid footwear (socks or shoes).

DIF: Applying OBJ: 28.6 TOP: Implementation
 MSC: NCLEX Client Needs Category: Safe and Effective Care Environment NOT:
 Concepts: Mobility

4. The nurse is performing passive range-of-motion exercises on a patient when the patient begins to complain of pain. What is the first thing the nurse should do?
- Notify the health care provider.
 - Hyperextend the joint.
 - Stop the range of motion.
 - Switch to active range of motion.

ANS: C

Stop range-of-motion exercises if the patient begins to complain of pain or if resistance to movement is experienced. Never hyperextend or flex a patient's joints beyond the position of comfort. Active range of motion is when the patient moves the joint. Notifying the health care provider would happen later.

DIF: Understanding OBJ: 28.3 TOP: Implementation
 MSC: NCLEX Client Needs Category: Physiological Integrity NOT: Concepts: Mobility

5. The nurse recognizes which goal to be appropriate for the patient who is postoperative day one from a hip fracture with the nursing diagnosis Impaired mobility?
- Patient will interact with others.
 - Patient will ambulate to the bathroom with assistance.
 - Patient will have no skin breakdown.
 - Patient will have a physical therapy consult.

ANS: B

Patients with a diagnosis of Impaired mobility should have a goal aimed at improving their mobility. Although immobility can impact social isolation and skin breakdown, those goals are not appropriate for this diagnosis. Have a physical therapy consult is not a goal but an intervention.

DIF: Applying OBJ: 28.5 TOP: Diagnosis
 MSC: NCLEX Client Needs Category: Physiological Integrity NOT: Concepts: Mobility

6. The nurse identifies which goal to be appropriate for the patient who is postoperative day one from abdominal surgery and on bed rest with the nursing diagnosis impaired skin integrity?
- Patient will ambulate twice a day.
 - Patient will eat 50% of meals.
 - Patient will have no further skin breakdown.
 - Patient will interact with others.

ANS: C

The patient already has a wound, so the goal is focused on no further skin breakdown as a result of the bed rest and immobility. Although nutrition is important to wound healing, it is not the focus of this Nursing diagnosis. Ambulating and interacting with others are not goals for this diagnosis.

DIF: Analyzing OBJ: 28.5 TOP: Diagnosis
MSC: NCLEX Client Needs Category: Physiological Integrity NOT: Concepts: Mobility

7. The nurse is providing education to the patient about isometric exercises. Which statement by the patient indicates a good understanding of these exercises?
- An example of this type of exercise is walking.¶
 - An example of this type of exercise is running.¶
 - An example of this type of exercise is Kegels.¶
 - An example of this type of exercise is weight lifting.¶

ANS: C

Isometric exercise requires tension and relaxation of muscles without joint movement. An example is tension and relaxation of pelvic floor muscles (i.e., Kegel exercise). Isotonic exercise involves active movement with constant muscle contraction, such as walking, turning in bed, and self-feeding. Aerobic exercise requires oxygen metabolism to produce energy.

Patients may engage in rigorous walking or repeated stair climbing to achieve the positive effects of aerobic exercise. Anaerobic exercise builds power and body mass. Without oxygen to produce energy for activity, anaerobic exercise takes place, such as heavy weight lifting.

DIF: Applying OBJ: 28.6 TOP: Implementation
MSC: NCLEX Client Needs Category: Physiological Integrity NOT: Concepts: Mobility

8. The nurse is preparing to assist the patient to walk to the bathroom after medicating the patient with a narcotic for pain management. What possible adverse effect should the nurse be immediately aware of?
- Constipation
 - Depression
 - Dizziness
 - Pain relief

ANS: C

Potential adverse side effects of narcotics include respiratory depression, hypotension, confusion, sedation, constipation, and dizziness. The nurse should be immediately aware of dizziness during ambulation because of the safety risks. Pain relief is expected. Depression is not an immediate adverse side effect. Constipation will not impact the nurse's ability to safely ambulate the patient.

DIF: Applying OBJ: 28.6 TOP: Implementation
MSC: NCLEX Client Needs Category: Safe and Effective Care Environment NOT: Concepts: Mobility

9. The nurse correctly selects which intervention to avoid causing shear or friction when moving a patient in bed?
- Using an airflow bed
 - Using a slide board
 - Using a trochanter roll

- d. Using a gel mattress

ANS: B

A transfer or slide board is made of plastic-like material that reduces friction. Linens easily slide over the board, facilitating bed linen changes. Patients can be repositioned or transferred with a minimum of force required. A trochanter roll prevents outward rolling of the hip when a patient is lying on his/her back. An air-fluidized bed uses airflow to move silicone particles in the bed, creating a watery, fluid-like movement and resulting in lower pressure to avoid or alleviate decubitus ulcers. A foam or gel combination mattress reduces pressure.

DIF: Applying OBJ: 28.6 TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment NOT:

Concepts: Mobility

10. Which explanation by the nurse best describes active assistive range of motion?
- The patient independently moves all joints.
 - The patient partially moves all joints.
 - The caregiver must move the patient's joints.
 - The patient performs isotonic exercises.

ANS: B

Active assistive range of motion occurs when the caregiver minimally assists the patient, or the patient minimally assists himself/herself in the movement of joints through a full motion. Active range of motion occurs when the patient has full independent movement of all joints; this is also known as isotonic exercise. Passive range of motion occurs when the caregiver moves the patient's joints through a full motion. This exercise does not maintain or improve strength but maintains flexibility and prevents contractures and atrophy.

NURSINGTB.COM

DIF: Understanding

OBJ: 28.6

TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment

NOT: Concepts: Mobility

11. The nurse identifies which instruction to be appropriate to delegate to the UAP (Unlicensed assistive personnel)?
- Assess the patient's skin during a bath.
 - Reposition the patient using the trapeze.
 - Assess the patient's ability to perform range-of-motion exercises.
 - Notify the health care provider of any changes.

ANS: B

Repositioning a patient can be delegated to unlicensed assistive personnel (UAP); the nurse should provide proper instruction regarding specific positioning techniques, individualized patient concerns, and circumstances that require notifying a nurse. UAP may not perform assessments or evaluations but should notify the nurse about any skin or musculoskeletal issues (not the health care provider).

DIF: Understanding

OBJ: 28.6

TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment

NOT: Concepts: Mobility

12. The nurse knows that manual lifting should only be done in which situation?
- Patients who are less than 150 lb

- b. Life-threatening situations
- c. Postsurgical patients
- d. Patients who are less than 200 lb

ANS: B

Many manual patient handling tasks are unsafe, because the weights lifted and movements required are beyond the ability of most caregivers. The key is to identify the task to be accomplished, and then use the required equipment and personnel so that the task fits the capabilities of the staff (U.S. Department of Veterans Affairs, 2016). The patient's level of cooperation is taken into consideration when using the safe patient-handling and mobility (SPHM) algorithms to decide the best method of moving the patient. The patient's weight, medical conditions, and ability to assist are also considered (U.S. Department of Veterans Affairs, 2016). Postsurgical patients as well as patients less than 150 or 200 lb may not fit the criteria.

DIF: Understanding

OBJ: 28.6

TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment

NOT: Concepts: Mobility

13. The nurse is preparing to reposition the patient in bed. What is the first step in this process?
- a. Position the patient's arms across his/her chest.
 - b. Lower the side rails.
 - c. Grasp the draw sheet.
 - d. Raise the bed to a working height.

ANS: D

Raising the bed to a working height is the first step before beginning the procedure. Proper positioning of equipment prevents provider discomfort and reduces the chance of possible injury. Then lower the side rails as appropriate and safe and ensure that the bed wheels are locked. Then you can have the patient position his/her arms and/or grasp the draw sheet.

DIF: Applying

OBJ: 28.6

TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment

NOT: Concepts: Mobility

14. The nurse has delegated to the UAP to assist a patient with ambulating in the hallway with a cane. Which statement by the UAP indicates a need for further education?
- a. —I should report any complaints of soreness to the nurse.||
 - b. —I should watch for indications that the patient has difficulties using the cane.||
 - c. —I should let the nurse or PT know if the cane doesn't seem to fit correctly.||
 - d. —I should teach the patient how to walk with the cane.||

ANS: D

Educating patients on how to walk with assistive devices may not be delegated to unlicensed assistive personnel (UAP). UAP should report any of the following: noticeable incorrect usage or fit of assistive devices, complaints of soreness or weakness, difficulties involving balance or strength, or difficulties in performing the procedure or other concerns verbalized by the patient.

DIF: Applying

OBJ: 28.6

TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment

NOT: Concepts: Mobility

15. The nurse correctly teaches the patient to rise from a chair using crutches when which intervention is used?
- Patient starts from the back of the chair.
 - The weak leg is closest to the chair.
 - The hand on the strong side holds the hand bar of the crutch.
 - The strong leg is closest to the chair.

ANS: D

The patient's strongest leg needs to be closest to the chair. The patient's hand on the weak side holds the hand bar of the crutches, and the hand on the patient's strong side holds onto the armrest of the chair. The patient moves to the front edge of the chair.

DIF: Applying OBJ: 28.6 TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment NOT:

Concepts: Mobility

MULTIPLE RESPONSE

1. The nurse is teaching a patient about ways to decrease risk of bone fractures. Which statements by the patient indicate a good understanding of decreasing this risk? (*Select all that apply.*)
- I should do weight-bearing exercises.
 - I should get adequate intake of calcium and vitamin D.
 - I should exercise regularly.
 - I need to do yoga exercises.
 - I wish I could reduce my risk but I can't do anything.

ANS: A, B, C

Inadequate dietary intake of calcium and vitamin D or impaired calcium metabolism may result in osteoporosis, which increases bone fragility and may lead to fractures. Decreased physical exercise and lack of weight-bearing exercise also contribute to bone fragility, deterioration, and loss of strength. Any type of exercise will help; it does not need to be yoga, but it does need to include weight-bearing exercise.

DIF: Applying OBJ: 28.2 TOP: Implementation

MSC: NCLEX Client Needs Category: Physiological Integrity NOT: Concepts: Mobility

2. The nurse knows that a patient with a compromised cardiopulmonary system has a diminished capacity for exercise because of which conditions? (*Select all that apply.*)
- Decreased tissue perfusion
 - Loss of sensation
 - Hemiparesis
 - Diminished respiratory capacity
 - Muscle weakness

ANS: A, D

Compromised cardiac function, decreased tissue perfusion, and diminished respiratory capacity directly affect a person's ability to perform activities of daily living (ADLs) and exercise. Hemiparesis and loss of sensation are associated with nervous system disorders. Muscle weakness can be from a number of causes.

DIF: Understanding

OBJ: 28.2

TOP: Assessment

MSC: NCLEX Client Needs Category: Physiological Integrity

NOT: Concepts: Mobility

3. The nurse is educating the patient about the effects of immobility on the body. Which statements by the patient indicate a need for further education? (*Select all that apply.*)
- a. —I can become very weak. ||
 - b. —I will gain weight. ||
 - c. —I will lose muscle tone. ||
 - d. —I can get bed sores. ||
 - e. —I won't have any lung problems. ||

ANS: B, E

Immobility may cause weakness, instability, anorexia, elimination alterations, decreased muscle tone, circulatory stasis, DVTs, pulmonary embolism, and skin breakdown. Knowing the effects of immobility on various body systems allows the nurse to quickly assess a patient's risk and recognize signs of impending complications.

DIF: Applying

OBJ: 28.3

TOP: Implementation

MSC: NCLEX Client Needs Category: Physiological Integrity

NOT: Concepts: Mobility

4. The nurse knows which items are included in the documentation for a patient on fall precautions? (*Select all that apply.*)
- a. History of any falls
 - b. Falls risk assessment scores
 - c. Patient and family education
 - d. Use of assist devices
 - e. Any fall or reported fall

NURSINGTB.COM

ANS: A, B, C, D, E

The nurse should document the general assessment, include the patient's medical history, subjective and objective data, medication review, musculoskeletal status, and history of falls. Falls assessment and reassessment, patient family education and use of assist devices are also documented. Thoroughly document a fall or reported fall.

DIF: Applying

OBJ: 28.2

TOP: Implementation

MSC: NCLEX Client Needs Category: Physiological Integrity

NOT: Concepts: Mobility

5. The nurse knows which findings indicate orthostatic hypotension? (*Select all that apply.*)
- a. A decrease in systolic blood pressure by 30 mm Hg
 - b. A decrease in diastolic blood pressure by 10 mm Hg
 - c. An increase in heart rate by 30 beats/min
 - d. An increase in systolic blood pressure by 20 mm Hg
 - e. A decrease in heart rate by 20 beats/min

ANS: B, D

A drop in systolic blood pressure of 20 mm Hg, an increase in heart rate of 20 beats/min, or a drop of diastolic blood pressure of 10 mm Hg when a patient stands is classified as orthostatic hypotension.

DIF: Understanding

OBJ: 28.3

TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment

NOT: Concepts: Mobility

6. The nurse appropriately delegates care of the unit's patients to the properly trained UAP when that UAP is assigned which tasks? (*Select all that apply.*)
- UAP assigned to reposition the patient.
 - UAP assigned to complete the MORSE falls risk scale.
 - UAP assigned to provide range-of-motion exercises.
 - UAP assigned to ambulate the patient in the hallway.
 - UAP assigned to time the patient on a TUG test.

ANS: A, C, D

UAPs provide hands-on care for immobilized patients under the direct supervision of registered nurses. Turning and positioning of patients, range-of-motion exercises, transfers, and assistance with ambulation may be delegated to properly trained UAP. UAPs may not assess patients because that is a nursing responsibility. The MORSE falls risk scale is a risk assessment as is the Timed Up and Go (TUG) test.

DIF: Applying OBJ: 28.6 TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment NOT:

Concepts: Mobility

7. The nurse is correctly demonstrating the use of a transfer belt when engaging in which actions? (*Select all that apply.*)
- The belt is placed around the patient's hips.
 - The belt is secure, leaving only enough room for the nurse to grasp the belt.
 - The nurse stands on the weaker side.
 - The nurse holds the belt on the side of the patient.
 - The nurse stands behind the patient while ambulating.

ANS: B, C

Transfer belts are used for patients with an unsteady gait or generalized weakness. Canvas transfer or gait belts are applied snugly around the patient's waist, leaving only enough room for the nurse to grasp the belt firmly during ambulation. Some belts may have handles. If the patient has a weaker side, the nurse should stand on that side and hold the gait belt firmly at the back of the patient's waist while ambulating.

DIF: Applying OBJ: 28.6 TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment NOT:

Concepts: Mobility

8. The nurse is correctly assisting the patient in using a cane when the patient demonstrates which activities? (*Select all that apply.*)
- The top of the cane is level with the patient's bent elbow.
 - The patient holds the cane on his/her weaker side.
 - The patient moves the cane forward first.
 - The patient's arm is comfortably bent when walking.
 - The patient moves the strong leg forward first.

ANS: C, D

The top of the cane should be level with the hip joint, and the patient's arm should be comfortably bent when the patient is walking. The patient should hold the cane on his/her stronger side and move the cane forward first, followed by the weaker leg and then the stronger leg. This ensures that another point of support is always on the ground when the weaker leg is bearing weight and gives the patient a wide base of support. A patient using a cane should be encouraged to stand up straight and look forward. Leaning to one side or looking down can jeopardize safety and cause poor posture.

DIF: Evaluating OBJ: 28.6 TOP: Evaluation
 MSC: NCLEX Client Needs Category: Safe and Effective Care Environment NOT:
 Concepts: Mobility

9. The nurse is providing discharge education for the patient who is going home with a walker. Which statements by the patient indicate a good level of understanding of safety in the home? (*Select all that apply.*)
- a. —I need to remove the throw rugs.||
 - b. —I should make sure I only take a bath.||
 - c. —I cannot use the stairs.||
 - d. —I need to place a nonskid mat in front of the kitchen sink.||
 - e. —I wish I had two ways of leaving the house.||

ANS: A, D

To ensure patients do not have hazards that can cause falls at home, the nurse should evaluate where the living quarters are. If the patient has stairs, they need to be able to safely learn how to use the stairs. They need to remove throw rugs that are a trip hazard and place nonskid mats in front of sinks, tubs, and showers. They can shower with a bench or chair in the shower for sitting. Patients need a clear exit so they can get out of the house quickly in case of an emergency, but do not specifically need two different exits because of the walker.

DIF: Evaluating OBJ: 28.6 TOP: Evaluation
 MSC: NCLEX Client Needs Category: Safe and Effective Care Environment NOT:
 Concepts: Mobility

10. The nurse identifies that knee-high SCD (Sequential Compression Device) sleeves are correctly placed on the patient when which conditions are met? (*Select all that apply.*)
- a. Both sleeves are connected to the SCD device.
 - b. Two fingers fit inside when the SCDs are inflated.
 - c. There are no kinks in the tubing.
 - d. The ankle pressure is 55 to 65 mm Hg.
 - e. The cooling control is on.

ANS: A, C, E

Proper positioning of the SCD sleeve allows proper fit and application, which decreases the risk of constricting the blood flow or diminishing optimal outcomes. Wrap the sleeve around the leg and fasten it with Velcro straps. Verify that two fingers fit between the leg and the sleeve when the sleeve is not inflated. Connect the sleeves to the device, ensure that there are no kinks in the tubing, and turn on the cooling and set it to 35 to 55 mm Hg.

DIF: Understanding OBJ: 28.6 TOP: Assessment
 MSC: NCLEX Client Needs Category: Safe and Effective Care Environment
 NOT: Concepts: Mobility

Chapter 29: Skin Integrity and Wound Care**Yoost & Crawford: Fundamentals of Nursing: Active Learning for Collaborative Practice, 3rd Edition**

MULTIPLE CHOICE

1. The nurse knows which description would be classified as a closed wound?
 - a. A large bruise on the side of the face
 - b. A surgical incision that is sutured closed
 - c. A puncture wound that is healing
 - d. An abrasion on the leg

ANS: A

In a closed wound, as seen with bruising, the skin is still intact. An open wound is characterized by an actual break in the skin's surface. For example, an abrasion, a puncture wound, and a surgical incision are types of open wounds.

DIF: Applying OBJ: 29.3 TOP: Assessment
MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:
Concepts: Tissue Integrity

2. The nurse is educating the patient about the signs and symptoms of a wound infection. Which statement indicates a need for further education?
 - a. —The wound will be red. ||
 - b. —The wound will have pus. ||
 - c. —The wound will be warm. ||
 - d. —The wound will need to be treated. ||

ANS: B

An infected wound shows clinical signs of infection, including redness, warmth, and increased drainage that may or may not be purulent (contain pus), and has a bacterial count in the tissue of at least 10^5 /g of tissue sampled when cultured. The wound will need to be treated for the infection.

DIF: Applying OBJ: 29.3 TOP: Implementation
MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:
Concepts: Tissue Integrity

3. The nurse identifies which type of wounds heal by tertiary intention?
 - a. An acute wound in which the patient has sutures placed when it happened.
 - b. A pressure ulcer that was treated with dressing changes and is healed.
 - c. An acute wound in which surgical glue was used to close the wound.
 - d. A wound that was left open initially and closed later with sutures.

ANS: D

When a delay occurs between injury and closure, the wound healing is said to happen by tertiary intention. Wounds such as surgical incisions or traumatic wounds in which the edges of the wound can be approximated (brought together) to heal are examples of acute wounds. This type of wound is said to heal by primary intention. When a wound heals by secondary intention, new tissue must fill in from the bottom and sides of the wound until the wound bed is filled with new tissue such as a pressure ulcer.

DIF: Applying OBJ: 29.3 TOP: Implementation
 MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:
 Concepts: Tissue Integrity

4. The nurse is caring for a patient who is postoperative day one from an abdominal surgery. When the patient complains of a —popping sensation and a wetness in the dressing, the nurse immediately suspects which complication?
- A wound infection
 - The stitches came loose
 - Wound dehiscence
 - Wound crepitus

ANS: C

Wound dehiscence, which usually occurs in connection with surgical incisions, is the partial or complete separation of the tissue layers during the healing process. This is an emergency situation. Stitches can come loose, but there is no popping sensation. Wound infections are characterized by redness, warmth, and drainage, and crepitus is air trapped under the skin.

DIF: Analyzing OBJ: 29.3 TOP: Evaluation
 MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:
 Concepts: Tissue Integrity

5. The nurse is caring for a postoperative patient who has had abdominal surgery and whose wound has completely eviscerated when the nurse walks into the room. In addition to notifying the surgeon, what should the nurse do?
- Cover the wound with a sterile gauze pad.
 - Cover the wound with a transparent dressing.
 - Put pressure on the wound with a sterile gauze pad.
 - Cover the wound with gauze soaked with normal saline.

ANS: D

If dehiscence or evisceration occurs, cover the wound with gauze moistened with a sterile normal saline, and notify the surgeon immediately. Putting pressure on the wound could cause further complications. Transparent films are used for autolytic debridement. A gauze pad will allow the wound to become dry and cause further complications.

DIF: Analyzing OBJ: 29.6 TOP: Implementation
 MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:
 Concepts: Tissue Integrity

6. The nurse identifies what goal to be the most appropriate goal for a patient with a stage 3 pressure ulcer who has a Nursing diagnosis of Impaired skin integrity?
- Wound will be completely healed in 72 hours.
 - Wound will show signs of healing within 2 weeks.

- c. Patient will develop no new pressure ulcers.
- d. Patient will ambulate twice a day.

ANS: B

A stage 3 pressure ulcer is a more extensive wound and will take time to heal, so the most appropriate goal will be to show signs of healing in 2 weeks. It will not heal in 72 hours. The goal of no new pressure ulcers is good, but not the most appropriate, and ambulating twice a day is more of an intervention.

DIF: Applying OBJ: 29.5 TOP: Diagnosis
 MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:
 Concepts: Tissue Integrity

7. A new nurse is delegating care of a chronic, nonsterile wound to a UAP. What action by the new nurse causes the preceptor to intervene?
- a. The nurse asks the UAP to assess the wound.
 - b. The nurse asks the UAP to report increased wound drainage.
 - c. The nurse asks the UAP to observe changes in dietary intake.
 - d. The nurse asks the UAP to change the dressing.

ANS: A

Assessment and evaluation of a patient's skin and wounds, and the effectiveness of the treatment plan, are a nurse's responsibility and cannot be delegated to unlicensed assistive personnel (UAP). UAP should report to the nurse any changes in skin condition or integrity; elevation in temperature; complaints of pain; increased wound drainage or incontinence; and observed changes in dietary intake. Some dressing changes can be performed by UAP in some situations.

NURSINGTB.COM

DIF: Applying OBJ: 29.6 TOP: Implementation
 MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:
 Concepts: Tissue Integrity

8. The nurse is repositioning the patient in the side-lying position. To avoid putting the patient at risk for pressure ulcers, the nurse should place the head of the bed in which position?
- a. Flat
 - b. 90 degrees
 - c. 30 degrees
 - d. 45 degrees

ANS: C

When side-lying, patients should be positioned at 30 degrees, as opposed to 90 degrees, to avoid positioning the patient directly on bony prominences such as the head of the trochanter.

DIF: Applying OBJ: 29.6 TOP: Implementation
 MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:
 Concepts: Tissue Integrity

9. The nurse recognizes which intervention is not a form of mechanical debridement?
- a. Wet to dry dressings
 - b. Whirlpool baths
 - c. Wet to damp dressing
 - d. Enzymatic dressing

ANS: D

Enzymatic debridement is achieved through the application of topical agents containing enzymes that work by breaking down the fibrin, collagen, or elastin present in devitalized tissue, thus allowing for its removal. Mechanical debridement is a nonselective form of debridement because it not only removes the necrotic tissue, but also can remove or disturb exposed viable tissue that may be in the wound. The main forms of mechanical debridement are wet/damp-to-dry dressings and whirlpools.

DIF: Understanding

OBJ: 29.6

TOP: Implementation

MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:

Concepts: Tissue Integrity

10. The nurse is explaining the purpose of occlusive dressings to the student nurse. Which statement by the student nurse indicates a lack of understanding?
- Occlusive dressings are used for autolytic debridement.||
 - Hydrocolloids are a type of occlusive dressing.||
 - Occlusive dressings can be used on infected wounds.||
 - Occlusive dressings support the most comfortable form of debridement.||

ANS: C

Occlusive dressings such as hydrocolloids and transparent films are used for autolytic debridement and are contraindicated in infected wounds. It is the most comfortable form of debridement for the patient.

DIF: Applying

OBJ: 29.6

TOP: Implementation

MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:

Concepts: Tissue Integrity

11. The nurse knows that a hydrocolloid dressing is appropriate for use on which type of wound?
- A wound with a large amount of drainage
 - A wound that is tunneling
 - A postsurgical incision with staples
 - A wound with a moderate amount of drainage

ANS: D

Hydrocolloids are occlusive, adhesive dressings composed of gelling agents and carboxymethylcellulose. They absorb a small to moderate amount of drainage over a 3- to 7-day period, forming a gel as drainage is absorbed. A wound with a large amount of drainage would require a foam or alginate dressing, a postsurgical incision with staples could use Steri-Strips or gauze, and a wound that is tunneling may require packing.

DIF: Applying

OBJ: 29.6

TOP: Implementation

MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:

Concepts: Tissue Integrity

12. When the nurse is caring for a patient with a Penrose drain, what care needs to be carried out?
- The drain must be compressed after emptying to work properly.
 - The drain must be connected to suction if ordered.
 - The drain is not sutured in place so care is taken to not dislodge it.
 - The suction pulls drainage away from the wound as it re-expands.

ANS: C

The Penrose drain, an open drain that is a flexible piece of tubing, is usually not sutured into place and is not connected to suction. Closed drains are compressed or connected to suction if ordered and pull drainage away as they expand.

DIF: Applying OBJ: 29.6 TOP: Implementation

MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:

Concepts: Tissue Integrity

13. The nurse is educating the patient about the use of heat/cold therapy at home. Which statement by the patient indicates the need for further education?
- I should fill my ice bag 2/3 full of ice.¶
 - I should use distilled water in my Aqua-K pad.¶
 - I can warm up my hot pack in the microwave.¶
 - I should check the order for how long to leave the compress on.¶

ANS: C

Warm compresses and water for soaks should not be heated in the microwave unless the product and microwave are specifically designed for this type of heating. Ice bags are filled two-thirds full, distilled water is used in Aqua-K pads, and application time for heat is as stated in the PCP order (for cold, it is a maximum of 20 to 30 minutes).

DIF: Applying OBJ: 29.6 TOP: Implementation

MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:

Concepts: Tissue Integrity

14. The nurse identifies which syringe to use when irrigating a patient's deep wound?
- 5-mL syringe
 - 10-mL syringe
 - 3-mL syringe
 - 30-mL syringe

NURSINGTB.COM

ANS: D

A deep wound is irrigated with a 30- to 50-mL piston syringe with an 18-gauge angiocath. Unlike the 1 pound per square inch (psi) of pressure or less that is delivered by a standard bulb syringe, the use of a 30- to 50-mL syringe and 18-gauge catheter has been shown to achieve an irrigation force that falls within the recommended 4 to 15 psi.

DIF: Understanding OBJ: 29.6 TOP: Implementation

MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:

Concepts: Tissue Integrity

15. The nurse understands which rationale to be appropriate for drying a wound after irrigation?
- Ensure the new dressing adheres to the wound.
 - Ensure the new dressing remains occlusive.
 - Prevent skin breakdown from moisture.
 - Prevent infection from irrigate solution.

ANS: C

Proper drying prevents further skin breakdown from moisture. Patting (rather than rubbing) prevents healthy tissue from being removed and reduces trauma to the wound. The type of dressing will determine how it lays in the wound and whether it is occlusive. The drying does not prevent infection.

DIF: Understanding OBJ: 29.6 TOP: Implementation
 MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:
 Concepts: Tissue Integrity

16. The nurse is performing a wet/damp to dry dressing change when the patient begins to complain of severe pain. What does the nurse do first?
- Notify the provider.
 - Notify the wound care nurse.
 - Stop the procedure.
 - Give the patient pain medication.

ANS: C

If the patient is complaining of severe pain, the nurse should first stop the procedure and then determine if the pain is new or preexisting. Then the nurse can determine what to do next based on the patient's response.

DIF: Analyzing OBJ: 29.6 TOP: Implementation
 MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:
 Concepts: Tissue Integrity

17. The nurse knows what goal to be appropriate for a patient with a stage 3 pressure ulcer with the nursing diagnosis impaired physical mobility?
- Patient will remain free of wound infections during the hospitalization.
 - Patient will report pain management strategies and reduce pain to a tolerable level.
 - Patient will be able to assist with position changes using over bed trapeze within 1 week.
 - Patient will consume adequate nutrition to meet nutritional requirements within 1 week.

ANS: C

Patient will be able to assist with position changes using over bed trapeze within 1 week is an appropriate goal for impaired mobility. The patient remaining free of wound infections during the hospitalization is an appropriate goal for impaired tissue integrity. The patient reporting pain management strategies to reduce pain to a tolerable level is an appropriate goal for acute pain. The patient consuming adequate nutrition to meet nutritional requirements within 1 week is an appropriate goal for Impaired nutritional status.

DIF: Applying OBJ: 29.5 TOP: Diagnosis
 MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:
 Concepts: Tissue Integrity

18. When discussing stage 3 pressure ulcers with the student nurse, which description would the staff nurse include?
- A pressure ulcer that involves exposure of bone and connective tissue.
 - A pressure ulcer that does not extend through the fascia.
 - A pressure ulcer that does not include tunneling.

- d. A partial-thick wound that involves the epidermis.

ANS: B

Stage 3 pressure ulcers are full-thickness wounds that extend into the subcutaneous tissue but do not extend through the fascia to muscle, bone, or connective tissue. There may be undermining or tunneling present in the wound. Stage 4 pressure ulcers involve exposure of muscle, bone, or connective tissue such as tendons or cartilage. Stage 2 pressure ulcers are partial-thickness wounds that involve the epidermis and/or dermis.

DIF: Understanding

OBJ: 29.3

TOP: Assessment MSC:

NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:

Concepts: Tissue Integrity

19. The nurse identifies which skin layer that delivers the blood supply to the dermis, provides insulation, and has a cushioning effect?
- Stratum germinativum
 - Epidermis
 - Subcutaneous layer
 - Stratum corneum

ANS: C

The subcutaneous layer delivers the blood supply to the dermis, provides insulation, and has a cushioning effect. The stratum germinativum constantly produces new cells that are pushed upward through the other layers of the epidermis toward the stratum corneum, where they flatten, die, and are eventually sloughed off and replaced by new cells. The epidermis is the outermost layer of the skin and the thinnest of the layers. The stratum corneum is made up of flattened dead cells.

DIF: Understanding

NURSINGTB.COM

OBJ: 29.1

TOP: Assessment

MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation

NOT: Concepts: Tissue Integrity

MULTIPLE RESPONSE

1. The nurse knows which factors contribute to the development of wounds and lead to delays in wound healing? (*Select all that apply.*)
- A patient who has diabetes.
 - A patient with COPD.
 - A patient with on bed rest who is repositioned.
 - A patient who is obese and sweats excessively.
 - A patient on long-term steroid therapy.

ANS: A, B, C, D, E

Factors that contribute to the development of wounds and lead to delays in wound healing include comorbidities such as vascular disease, which impacts the skin's ability to obtain required oxygen and nutrients, or diabetes, which affects not only the microvasculature, but also the skin's normally acidic pH; malnutrition involving inadequate proteins, cholesterol and fatty acids, and vitamins and minerals; medications such as steroids, nonsteroidal, anti-inflammatories, and anticoagulants; excessive moisture from sweating; and external forces such as pressure, shear, and friction that occur when turning and repositioning the patient in bed.

DIF: Applying OBJ: 29.2 TOP: Assessment
 MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:
 Concepts: Tissue Integrity

2. The nurse recognizes that the cause of pressure ulcers includes which factors? (*Select all that apply.*)
- a. Intensity of the pressure
 - b. Duration of the pressure
 - c. Tissue's ability to tolerate the pressure
 - d. Person's age
 - e. Person's nutritional status

ANS: A, B, C, D, E

The primary cause of pressure ulcers is, as the name suggests, pressure. However, it is more than just pressure; it is the intensity of the pressure, the length of time that the tissue is subjected to the pressure, and intrinsic and extrinsic factors that affect the tissue's ability to withstand or tolerate that pressure. Intrinsic and extrinsic factors can include nutrition status and age.

DIF: Applying OBJ: 29.2 TOP: Assessment
 MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:
 Concepts: Tissue Integrity

3. When the nurse is performing a focused wound assessment on a patient, what information should be included in the documentation? (*Select all that apply.*)
- a. Location and size
 - b. Characteristics of the wound bed
 - c. Patient's response to wound treatment
 - d. Patient's pain level
 - e. Presence of drainage

ANS: A, B, C, E

A focused wound assessment includes an evaluation of the wound's location, size, and color; presence of drainage; condition of the wound edges; characteristics of the wound bed; and patient's response to the wound or wound treatment. The patient's pain level would be documented with his/her pain assessment.

DIF: Applying OBJ: 29.3 TOP: Assessment
 MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:
 Concepts: Tissue Integrity

4. The nurse is using the Braden scale to assess the patient's risk for a pressure ulcer. Which risk categories are associated with the Braden scale? (*Select all that apply.*)
- a. Activity
 - b. Friction and shear
 - c. Moisture
 - d. Sensory perception
 - e. Cognition

ANS: A, B, C, D

The Braden scale ranks the patient on the risk categories of sensory perception, moisture, activity, mobility, nutrition, and friction and shear. The scale does not include cognition.

DIF: Understanding

OBJ: 29.3

TOP: Assessment MSC:

NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:

Concepts: Tissue Integrity

5. The nurse is caring for a postoperative orthopedic patient who has two Hemovac drains in place. Which interventions will the nurse perform? (*Select all that apply.*)
- a. Measure the amount of drainage in the device prior to emptying.
 - b. Label each drain and record them separately.
 - c. Recompress the device after emptying.
 - d. Secure the device to the patient's gown above the level of the wound.
 - e. Check for kinks in the tubing.

ANS: B, C, E

Use a marked, graduated measuring device to collect the drainage when emptying the reservoir to facilitate accurate measurement of the drainage. After emptying, recompress the device to maintain suction. Secure the container(s) to the patient's hospital gown below the level of the wound, avoiding tension on the tubing and making sure there are no kinks. If there are multiple drains, label them and document observations by the drain label.

DIF: Applying

OBJ: 29.6

TOP: Implementation

MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:

Concepts: Tissue Integrity

6. The nurse recognizes that cold therapy is contraindicated in which conditions? (*Select all that apply.*)
- a. Edema
 - b. Shivering
 - c. Bleeding
 - d. Circulatory problems
 - e. Advanced age

NURSINGTB.COM

ANS: A, B, D

Cold should not be used if any of the following is present: edema (cold application slows reabsorption of the fluid), circulatory pathophysiology (cold application causes vasoconstriction, further reducing circulation to the area), and shivering (this is a comfort concern). Bleeding is contraindicated in heat therapy. Advanced age would require frequent observation due to thin skin.

DIF: Understanding

OBJ: 29.6

TOP: Implementation

MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:

Concepts: Tissue Integrity

Chapter 30: Nutrition**Yoost & Crawford: Fundamentals of Nursing: Active Learning for Collaborative Practice, 3rd Edition**

MULTIPLE CHOICE

1. The nurse is providing education to a patient about the difference between simple and complex carbohydrates. Which statement by the patient indicates a need for further education?
 - a. —Simple carbohydrates give me quick energy.¶
 - b. —Complex carbohydrates come from fruit.¶
 - c. —Complex carbohydrates take longer to break down.¶
 - d. —Simple carbohydrates come from milk products.¶

ANS: B

Simple carbohydrates are broken down and absorbed quickly, providing a quick source of energy. Examples are sugars such as those derived from fruit (fructose), table sugar (sucrose), milk products (lactose), and blood sugar (glucose). Complex carbohydrates are composed of starches, glycogen, and fiber. They take longer to break down prior to absorption and utilization by the body's cells.

DIF: Evaluating

OBJ: 30.1

TOP: Evaluation

MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:

Concepts: Nutrition

2. The nurse teaches the family member to provide the patient with how much dietary fiber per day?
 - a. 25 to 35 g
 - b. 20 to 35 g
 - c. 25 to 40 g
 - d. 20 to 40 g

NURSINGTB.COM

ANS: B

Older children, adolescents, and adults should consume 20 to 35 g of fiber a day. Food sources include whole grains, wheat bran, cereals, fresh fruits, vegetables, and legumes.

DIF: Understanding

OBJ: 30.1

TOP: Planning

MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:

Concepts: Nutrition

3. The nurse is providing education to an older adult around a healthy diet to support the challenges related to aging. Which statement indicates a need for further education?
 - a. —I should choose foods that are nutrient dense.¶
 - b. —High-fiber foods minimize the risk of constipation.¶
 - c. —I should eat more calories to avoid malnutrition.¶
 - d. —I can add spices to enhance the taste of food.¶

ANS: C

Calorie needs change with aging because of more body fat and less lean muscle. Less activity further decreases calorie requirements. Eating whole-grain foods and a variety of fruits and vegetables and drinking water may minimize the risk of constipation. The challenge for older adults is to choose foods that are nutrient dense; these foods are high in nutrients in relation to their calories. Older adults may experience a decreased sense of smell or taste, so the addition of spices and herbs may enhance the taste of foods.

DIF: Applying OBJ: 30.1 TOP: Implementation
 MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:
 Concepts: Nutrition

4. When caring for an adolescent patient with anorexia nervosa, the nurse knows what would be the best treatment option for this patient?
- Hospitalization with skill nursing care
 - Compulsory tube feedings
 - Individually determined by a collaborative team
 - Outpatient treatment

ANS: C

Ultimately, the decision on how best to ethically treat an adolescent suffering from an eating disorder needs to be one of collaboration among the child's physician, nurse, counselor, spiritual adviser, parents, and other concerned adults. Highly skilled nursing care with hospitalization is preferred prior to a drop in BMI levels below 13 kg/m². Compulsory tube feedings are not always the best option. Although most adolescents with eating disorders can be treated on an outpatient basis, those who exhibit severe depression, extreme physical complications resulting from electrolyte imbalances, or suicidal tendencies may require

extensive inpatient treatment.

DIF: Applying OBJ: 30.2 TOP: Implementation
 MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:
 Concepts: Nutrition

5. A new UAP is measuring a patient's height. Which step of the procedure indicates a need for the registered nurse to provide further education on this skill?
- The UAP instructs the patient to remove shoes.
 - The UAP measures from the top of the patient's head to the bottom of the patient's foot arch.
 - The UAP positions the head against the headboard or measuring device.
 - The UAP makes sure the patient is standing erect.

ANS: B

Height is measured from the top of the head to the bottom of the heel. The patient is instructed to remove shoes, stand erect, and position the top of the patient's head against the headboard or measuring device for accuracy.

DIF: Applying OBJ: 30.3 TOP: Implementation
 MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:
 Concepts: Nutrition

6. The nurse is performing an oral examination on a patient and notices a beefy-red tongue. The nurse identifies this as a characteristic finding for what condition?

- a. Anorexia nervosa
- b. Malnutrition
- c. Bulimia
- d. Pernicious anemia

ANS: D

In conditions such as pernicious anemia, a characteristic finding is a sore, smooth-surfaced, beefy-red tongue, which may interfere with the person's ability to chew certain foods. Anorexia nervosa and bulimia are eating disorders. In malnutrition the oral mucosa may be a darker red than normal with oral lesions and/or the tongue may reveal white irregular areas.

DIF: Remembering OBJ: 30.3 TOP: Assessment MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT: Concepts: Nutrition

7. The nurse has delegated the feeding of a patient who has recently had a stroke to the UAP. Which procedure that the UAP performs would demonstrate a need for further education?
- a. Uses thickened liquids.
 - b. Puts the bed at 25 degrees.
 - c. Encourages slow eating.
 - d. Has the patient alternate between food and sips of fluid.

ANS: B

During feeding, the head of the bed needs to be elevated at 30 to 45 degrees or higher. Liquids are thickened, and patients are encouraged to use slow-eating habits and to alternate between bites of food and sips of fluids to facilitate swallowing.

DIF: Evaluating OBJ: 30.6 TOP: Evaluation MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Safety and Infection Control NOT: Concepts: Nutrition

8. The nurse recognizes which outcome statement to be appropriate for the nursing diagnosis Impaired swallowing?
- a. Patient will consume 50% of each meal.
 - b. Patient will gain 2 lb a week.
 - c. Patient will not show any signs of aspiration during meals.
 - d. Patient will demonstrate using an assistive device to feed self.

ANS: C

An appropriate goal statement for impaired swallowing is that the patient will not exhibit any signs or symptoms of aspiration during this hospitalization (e.g., lungs clear, respiratory rate within normal range for patient). Consuming 50% of meals and gaining weight are appropriate goals for Impaired nutritional intake. Using assistive devices is an appropriate goal for Impaired self-feeding.

DIF: Applying OBJ: 30.5 TOP: Diagnosis MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Safety and Infection Control NOT: Concepts: Nutrition

9. The nurse is explaining to the UAP that the patient is on a full-liquid diet. Which statement by the UAP indicates a need for reorientation?
- a. —I can give the patient orange juice.¶

- b. —I can give the patient yogurt.¶
- c. —I can give the patient oatmeal.¶
- d. —I can give the patient milk.¶

ANS: C

Full-liquid diets consist of foods that are or may become liquid at room or body temperature. Full-liquid diets include juices with and without pulp, milk and milk products, yogurt, strained cream soups, and liquid dietary supplements. Oatmeal is not considered part of a full-liquid diet.

DIF: Applying OBJ: 30.6 TOP: Implementation
 MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:
 Concepts: Nutrition

10. The nurse is educating a patient about a renal diet. Which statement by the patient indicates a need for further education?
- a. —I need to eat a low-sodium diet.¶
 - b. —I can have limited amounts of meat.¶
 - c. —I can drink unlimited cola if it is diet.¶
 - d. —I should avoid or limit bananas.¶

ANS: C

Renal diets restrict potassium, sodium, protein, and phosphorous intake. Fresh fruits (except bananas) and vegetables are excellent dietary choices for individuals on a renal diet. Meats, processed foods, peanut butter, cheese, nuts, caramels, ice cream, and colas are typically allowed in limited quantities or contraindicated.

DIF: Evaluating OBJ: 30.6 TOP: Evaluation
 MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation
 NOT: Concepts: Nutrition

11. The nurse knows that initial verification of a nasogastric placement is important. Which method is considered the only reliable method to determine enteral tube placement?
- a. Auscultation of air bolus
 - b. Measurement of pH of the aspirate
 - c. Radiographic image
 - d. Aspirate contents to visually inspect appearance

ANS: C

Studies support the use of radiographic confirmation as the only reliable method to date of confirming enteral tube placement. Using only pH and the appearance of aspirate from the newly inserted tube is not a safe method of verifying proper gastric tube placement, especially in patients receiving antacid medications. Auscultation of an air bolus to assess tube placement is no longer recognized as a reliable source in determining gastric tube placement.

DIF: Applying OBJ: 30.6 TOP: Implementation
 MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:
 Concepts: Nutrition

12. The nurse is attempting to open an occluded PEG tube. Which intervention by the nurse requires re-education?
- a. Flushes the tube with a small amount of air.

- b. Flushes the tube using a 50- to 60-mL syringe and warm water.
- c. Reinserts the stylet to break up the clot.
- d. Flushes the tube with a special enzyme solution.

ANS: C

Once the stylet is removed, it is never reinserted because it can puncture the intestine. If the tube becomes occluded, flush it with a small amount of air. If this is unsuccessful in removing the occlusion, flush the tube using a 30 to 60 mL syringe and warm water. If flushing the tube with water is ineffective, research now suggests using special enzyme solutions or declogging devices rather than carbonated beverages or juices.

DIF: Applying OBJ: 30.6 TOP: Implementation
 MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Safety and Infection Control
 NOT: Concepts: Nutrition

13. When the nurse is caring for a patient who is receiving total parenteral nutrition (TPN), the nurse will change the tubing at which interval?
- a. Every 72 hours
 - b. Every 48 hours
 - c. Every 24 hours
 - d. Every 12 hours

ANS: C

Tubing should be changed every 24 hours, with aseptic technique used to minimize the risk of contamination, and the dressing over the site should be changed every 48 hours, with assessment for signs and symptoms of infection (redness, swelling, or drainage).

DIF: Understanding OBJ: 30.6 TOP: Implementation
 MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Safety and Infection Control
 NOT: Concepts: Nutrition

14. The nurse is preparing to insert a nasogastric (NG) tube in a patient. Which step in the process indicates a need for further education?
- a. The nurse lubricates 4 inches of the tube prior to insertion.
 - b. The nurse marks the length of the tube with a marker for insertion.
 - c. The nurse measures the length of tube needed using the nose-earlobe-xiphoid process.
 - d. The nurse applies clean gloves for the procedure.

ANS: B

Document the length of the tube to be used if the tube has a preprinted measurement scale. For any tube (with or without a preprinted scale), mark the measurement on the tube using a small piece of tape to ensure proper placement of the tube; fold the ends of the tape for easy removal. Do *not* use a permanent marker to mark the tube at this point of the procedure. When placement of the tube is confirmed as correct, then remove the measurement marking tape, mark the exit location on the tube with permanent marker, and proceed with the ordered treatment. Lubricate 4 inches of the tube tip with a water-soluble lubricant. For an NG tube, measure the length of tube needed for the patient by placing the tip of the tube at the tip of the patient's nose and extending it to the patient's earlobe and then to the patient's xiphoid process. Clean gloves are used.

DIF: Applying OBJ: 30.6 TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Safety and Infection Control
NOT: Concepts: Nutrition

15. The nurse has received an order from the health care provider to discontinue the nasogastric tube. Which action by the nurse indicates a need for further education?
- The nurse clears the tube with air prior to discontinuing.
 - The nurse stops the tube feeding.
 - The nurse instructs the patient to cough while pulling out the tube.
 - The nurse clamps the tube while pulling it out.

ANS: C

To remove the tube, instruct the patient to take a deep breath and hold it; pinch the tube, and pull it out smoothly and quickly. The nurse should stop any feedings, and suction and flush the tube with water and/or air as appropriate. The nurse should not ask the patient to cough while pulling out the tube. Coughing during tube insertion may indicate the tube is entering the patient's lungs.

DIF: Applying OBJ: 30.6 TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Safety and Infection Control
NOT: Concepts: Nutrition

MULTIPLE RESPONSE

1. Based on research on aging, the nurse knows that improper nutrition may result in the onset of which specific diseases? *(Select all that apply.)*
- Type 2 diabetes
 - Atherosclerosis
 - Osteoporosis
 - Rheumatoid arthritis
 - Chronic asthma

NURSINGTB.COM

ANS: A, B, C

Improper nutrition may result in the onset of specific diseases of the endocrine, cardiovascular, gastrointestinal, and musculoskeletal systems, such as diabetes type 2, atherosclerosis, diverticulosis, osteoporosis, and some cancers. Rheumatoid arthritis is an inflammatory autoimmune disorder. Asthma is a respiratory disorder not related to poor nutrition.

DIF: Understanding OBJ: 30.2 TOP: Implementation
MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT: Concepts: Nutrition

2. The nurse is helping a patient understand the difference between macronutrients and vitamins and minerals. The nurse identifies which items that should be included in the list of macronutrients? *(Select all that apply.)*
- Water
 - Potassium
 - Starches
 - Fiber
 - Riboflavin

ANS: A, C, D

The major nutrients, often referred to as macronutrients (nutrients that are needed in large amounts), include carbohydrates (sugar, starches, and dietary fiber). Water is also a macronutrient. Potassium is a mineral, and riboflavin is vitamin B₂; these are micronutrients.

DIF: Understanding

OBJ: 30.1

TOP: Implementation

MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:

Concepts: Nutrition

3. The nurse is providing dietary education to the patient to assist with inclusion of more complex carbohydrates in the diet. The nurse knows which foods would be beneficial to include? (*Select all that apply.*)
- a. Green peas
 - b. Bananas
 - c. Beans
 - d. Potatoes
 - e. Apples

ANS: A, C, D

Complex carbohydrates provide the body with vitamins and minerals. Food sources include bread; rice; pasta; legumes such as dried beans, peas, and lentils; and starchy vegetables such as corn, pumpkin, green peas, and potatoes. Bananas and apples are fruits, which are simple carbohydrates.

DIF: Evaluating

OBJ: 30.1

TOP: Evaluation

MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:

Concepts: Nutrition

NURSINGTB.COM

4. The nurse is educating a patient about including more omega-3 fatty acids in the diet and knows which food sources should be included? (*Select all that apply.*)
- a. Salmon
 - b. Flaxseed
 - c. Mackerel
 - d. Steak
 - e. Crayfish

ANS: A, B, C

Dietary sources of omega-3 include fatty fish, such as salmon, tuna, mackerel, and lake trout, as well as nuts, seeds, and oils; flaxseed oil contains the highest amount of total omega-3 fatty acids. Steak and crayfish do not contain omega-3 fatty acids.

DIF: Understanding

OBJ: 30.1

TOP: Implementation

MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:

Concepts: Nutrition

5. The nurse is planning dietary education for the patient. What food labeling considerations should the nurse be aware of when planning that education? (*Select all that apply.*)
- a. Ask patient if food labels are read routinely.
 - b. Assess patient's level of understanding of food labels.
 - c. Encourage patient to read the food labels.
 - d. Explain to patient all food labels are different.

- e. Assess patient's understanding of recommended daily allowance

ANS: A, B, C, E

Evidence indicates a consistent link between eating healthier foods and reading nutrition labels. Patients should be asked if they read food labels when shopping for groceries or food products. Evaluate their understanding of the main elements of a nutrient label (i.e., calories, fats, carbohydrates, sugar, and serving size). Assess patient understanding of the percentages of recommended daily allowances of fats, proteins, and carbohydrates listed on food labels. Uniform nutrition labeling for packaged food was introduced in the United States in 1994, as part of the Nutrition Labeling and Education Act (NLEA), to increase consumer awareness about the nutritional content of food and improve dietary practices; therefore, all labels are the same.

DIF: Applying OBJ: 30.1 TOP: Planning

MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:

Concepts: Nutrition

6. The nurse knows that a deficiency in vitamin C can result in which conditions? (*Select all that apply.*)

- a. Stiff joints
- b. Osteopenia
- c. Petechiae
- d. Loose teeth
- e. Bleeding gums

ANS: A, C, D, E

Deficiencies of vitamin C interfere with normal tissue synthesis and may result in gingivitis, which produces swollen and bleeding gums with loosened teeth, and painful, stiff joints. Other problems associated with malabsorption include anemia (a deficiency of red blood cells), excessive bleeding, petechiae (bleeding under the skin), poor wound healing, and neural tube defects. Osteopenia results from poor absorption of calcium.

DIF: Remembering

OBJ: 30.2

TOP: Assessment MSC:

NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:

Concepts: Nutrition

7. The nurse is teaching a patient about the impact of obesity and a high body mass index (BMI). The nurse identifies that as the BMI increases, so does the risk for which conditions? (*Select all that apply.*)

- a. Increase in blood pressure
- b. Increase in HDL
- c. Increase in total cholesterol
- d. Development of atherosclerosis
- e. Decrease in triglycerides

ANS: A, C, D

As BMI levels rise, blood pressure and cholesterol levels also rise and the average high-density lipoprotein (HDL), or good, cholesterol levels decrease. Hyperlipidemia (elevation of plasma cholesterol, triglycerides, or both) or low HDL levels contribute to the development of atherosclerosis (the buildup of fat deposits on arterial vessel walls). Obesity contributes to higher triglycerides.

DIF: Understanding

OBJ: 30.2

TOP: Implementation

MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:

Concepts: Nutrition

8. The nurse is completing a nutrition assessment on a patient. What are some important considerations? (*Select all that apply.*)
- a. The nurse should include the patient's cultural influences in the assessment.
 - b. The food diary accuracy is better for a 24-hour recall than a 3 to 5 day food journal.
 - c. The nurse should be nonjudgmental in the nutritional review.
 - d. A consultation with a registered dietitian may be indicated.
 - e. A gathering of anthropometric measurements may be necessary.

ANS: A, C, D, E

When collecting data, the nurse should take into consideration the patient's culture and ethnicity. Recognizing these influences on the patient's nutritional intake allows the nurse to make informed decisions. The data analysis may reveal the need to refer the patient to a registered dietitian for further evaluation of nutritional status. The 24-hour recall is dependent on the ability of the patient to remember consumption of foods and their quantities from the previous day. It is vital to remember that the patient's recall may not be factual and the intake may not be that of a typical day. The other means of assessing a patient's usual dietary pattern is to have the patient keep a written journal of food intake for a certain amount of time. The food diary should encompass entries for 3 to 5 days and includes dietary intake for a typical weekend. Anthropometric measurements may be needed for a full assessment.

DIF: Understanding

OBJ: 30.3

TOP: Assessment

MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:

Concepts: Nutrition

9. The nurse is educating the patient about the risk of heart disease from metabolic syndrome and describes a cluster of which symptoms? (*Select all that apply.*)
- a. Elevated blood glucose
 - b. High waist circumference
 - c. History of smoking
 - d. Hypertension
 - e. Elevation serum cholesterol

ANS: A, B, D, E

Metabolic syndrome is a cluster of medical conditions characterized by insulin resistance and the presence of obesity, abdominal fat, elevated blood glucose, triglycerides, serum cholesterol, and hypertension. Smoking is not part of the syndrome.

DIF: Understanding

OBJ: 30.2

TOP: Implementation

MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:

Concepts: Nutrition

10. The nurse is completing documentation after feeding a patient with aspiration precautions. Which items should the nurse document? (*Select all that apply.*)
- a. Episodes of coughing or gagging
 - b. Hesitation or fear of eating

- c. Amount eaten
- d. Aspiration protocol used
- e. Respiratory status

ANS: A, B, C, D, E

It is important to document thoroughly the patient's experience during the feeding so the other nursing staff will be aware of patient's needs including any episodes of coughing, gagging, or choking; respiratory status; hesitancy or fear of eating; and occurrences of nausea, vomiting, regurgitation, and/or reflux symptoms. The nurse would document the amount the patient ate, but this is not part of the required documentation for aspiration precautions.

DIF: Applying OBJ: 30.3 TOP: Assessment

MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:

Concepts: Nutrition

11. When the nurse is caring for a patient receiving enteral feedings, which tasks can that nurse delegate to the UAP? (*Select all that apply.*)
- a. Verify tube placement
 - b. Perform oral care
 - c. Administer tube feeding
 - d. Obtain vital signs and report results
 - e. Measure oxygen saturation

ANS: B, C, D, E

Administering an enteral feeding may be delegated, at the nurse's discretion, to UAP in accordance with state regulations and facility policies and procedures. The nurse should verify tube placement and assess the patient prior to delegating this procedure. The UAP can perform oral care and

obtain vital signs, including oxygen saturation, and report results.

DIF: Applying OBJ: 30.6 TOP: Assessment

MSC: NCLEX Client Needs Category: Physiological Integrity | NCLEX Client Needs Category: Safe and

Effective Care Environment: Safety and Infection Control

NOT: Concepts: Nutrition

Chapter 31: Cognitive and Sensory Alterations**Yoost & Crawford: Fundamentals of Nursing: Active Learning for Collaborative Practice, 3rd Edition****MULTIPLE CHOICE**

1. A nurse is caring for a patient with a stroke that has altered her ability to see. The nurse knows which area of the brain was likely impacted by the stroke that is responsible for visual function?
 - a. Parietal lobes
 - b. Frontal lobes
 - c. Occipital lobes
 - d. Temporal lobes

ANS: C

The occipital lobes process visual information. The frontal lobes of the cerebrum are the areas of the brain responsible for voluntary motor function, concentration, communication, decision making, and personality. The parietal lobes are responsible for the sense of touch, distinguishing the shape and texture of objects. The temporal lobes are concerned with the senses of hearing and smell.

DIF: Applying

OBJ: 31.1

TOP: Implementation

MSC: NCLEX Client Needs Category: Physiological Integrity

NOT: Concepts: Cognition

2. The family of a patient who was in a motor vehicle accident tells the nurse —I'm just not the person I was before the crash. The nurse recognizes this is likely because of the injury to what area of brain?
 - a. Parietal lobes
 - b. Frontal lobes
 - c. Occipital lobes
 - d. Temporal lobes

ANS: B

The frontal lobes of the cerebrum are the areas of the brain responsible for voluntary motor function, concentration, communication, decision making, and personality. The parietal lobes are responsible for the sense of touch, distinguishing the shape and texture of objects. The temporal lobes are concerned with the senses of hearing and smell. The occipital lobes process visual information.

DIF: Understanding

OBJ: 31.1

TOP: Implementation MSC:

NCLEX Client Needs Category: Physiological Integrity

NOT: Concepts: Cognition

3. The nurse is educating the family of a patient in the intensive care unit about the patient's cognitive status, including the current problem of delirium. Which statement by the family indicates a need for further education?
 - a. —The delirium can be caused by sensory overload.¶
 - b. —The delirium is reversible.¶
 - c. —The delirium is a mood disorder.¶
 - d. —The delirium is a state of confusion.¶

ANS: C

Delirium is a reversible state of acute confusion. It is characterized by a disturbance in consciousness or a change in cognition that develops over 1 to 2 days and is caused by a medical condition. Delirium may occur in intensive care patients as a result of sensory overload. It is not a mood disorder.

DIF: Evaluating OBJ: 31.2 TOP: Implementation

MSC: NCLEX Client Needs Category: Physiological Integrity NOT: Concepts: Cognition

4. The nurse is caring for a patient with depression. Which statement by the patient indicates a need for further education?
- Depression can be caused by chemical changes in the brain.||
 - Depression is always treated with medication.||
 - Depression is a mood disorder.||
 - Depression can have a rapid onset.||

ANS: B

Depression is usually reversible with treatment either by eliminating the underlying cause, providing counseling, or prescribing antidepressive agents. Depression is a mood disorder and is believed to be caused by chemical changes in the brain. Depression usually has a rapid onset, and the patient's mood is constant.

DIF: Evaluating OBJ: 31.2 TOP: Evaluation

MSC: NCLEX Client Needs Category: Psychosocial Integrity NOT: Concepts: Cognition

5. The nurse is caring for a patient who is complaining of tingling in the hands and fingers. The nurse knows this is a sign of what electrolyte imbalance?
- Hyponatremia
 - Hypernatremia
 - Hypocalcemia
 - Hypocalcemia

NURSINGTB.COM

ANS: C

Tactile disturbances, such as tingling and numbness around the mouth and in the fingers, are signs of hypocalcemia. Mental changes are associated with both hypercalcemia and hypocalcemia. Both hypernatremia and hyponatremia have symptoms of central nervous system disorder.

DIF: Understanding OBJ: 31.3

NCLEX Client Needs Category: Physiological Integrity

TOP: Implementation MSC:

NOT: Concepts: Cognition

6. The nurse is providing discharge instructions to an older adult who is being discharged with orthostatic hypotension. Which response by the patient indicates a need for further education?
- I should take my blood pressure once a day at home.||
 - I should get up quickly to avoid my blood pressure dropping.||
 - I should drink plenty of water during the day.||
 - I should get up slowly and carefully.||

ANS: B

In orthostatic hypotension, dizziness and loss of consciousness may occur if a patient changes position too quickly. Instead they should change positions slowly. A patient can take their blood pressure at home to monitor it. Drinking water will keep them hydrated.

DIF: Evaluating OBJ: 31.2 TOP: Evaluation
 MSC: NCLEX Client Needs Category: Physiological Integrity NOT:
 Concepts: Sensory Perception

7. The nurse is assessing the patient's ability to hear and knows which is the correct procedure for the doing this?
- The nurse whispers to the patient while standing on each side of the patient.
 - The nurse speaks in a normal voice while standing on each side of the patient.
 - The nurse speaks in a normal voice while standing directly in front of the patient.
 - The nurse speaks in a normal voice while standing slightly behind the patient.

ANS: D

Hearing ability can be determined by observing the patient's conversation and responses and by talking with the patient in a normal conversational tone while standing slightly behind the patient. If the patient does not respond appropriately, a hearing impairment may exist. Standing in front of the patient allows the patient to read your lips and will not detect a hearing loss. A whispered voice will also give a false reading.

DIF: Applying OBJ: 31.3 TOP: Assessment
 MSC: NCLEX Client Needs Category: Physiological Integrity NOT:
 Concepts: Sensory Perception

8. The nurse notices her 50-year-old patient is holding the lunch menu at arm's length while trying to read the choices. The nurse knows this is an indication of which condition?
- Retinopathy
 - Presbyopia
 - Cataracts
 - Macular degeneration

NURSINGTB.COM

ANS: B

The patient demonstrates presbyopia by holding reading materials at a distance or by being unable to read normal-sized or small print. Retinopathy is damage to the retina and occurs in diabetics. Cataracts are a clouding of the lens. Macular degeneration is a chronic condition that causes loss of vision in the center of your field of vision.

DIF: Applying OBJ: 31.3 TOP: Assessment
 MSC: NCLEX Client Needs Category: Physiological Integrity NOT:
 Concepts: Sensory Perception

9. The nurse is providing discharge education to the patient with diabetes regarding foot care. Which statement by the patient indicates a need for further education?
- I can go barefoot outside only in the summer.¶
 - I should wear good fitting shoes.¶
 - I cannot soak my feet in a hot tub.¶
 - I can use lotion on my feet.¶

ANS: A

Diabetic patients should not go barefoot outside even in the summer as they often have neuropathy, which decreases the patient's ability to discern touch, especially in the lower extremities. This can lead to foot injuries that can become infected and are slow to heal. The patient should wear good fitting shoes, should avoid extreme temperatures, and can use lotion to keep their skin moist to avoid overly dry skin.

DIF: Applying OBJ: 31.3 TOP: Assessment
 MSC: NCLEX Client Needs Category: Physiological Integrity NOT:
 Concepts: Sensory Perception

10. The nurse identifies which goal to be most appropriate for the Nursing diagnosis of acute confusion?
- The patient will use the call light before getting out of bed within 48 hours.
 - The patient will use a calendar to remember the date within 48 hours.
 - The patient will respond appropriately to questions about place within 48 hours.
 - The patient will remain within the unit while in long-term care.

ANS: C

The patient has acute confusion and therefore an appropriate early goal as the confusion resolves is to remember where they are. Remembering to use a call light would be appropriate for risk for falls. Using a calendar is appropriate for impaired memory and remaining in the unit is appropriate for chronic confusion.

DIF: Applying OBJ: 31.5 TOP: Assessment
 MSC: NCLEX Client Needs Category: Physiological Integrity NOT:
 Concepts: Cognition and Sensory Perception

11. The nurse recognizes which goal to be appropriate for the patient with a Nursing diagnosis of social isolation?
- The patient will participate in cognitive exercises.
 - The patient will interact with other residents during activities.
 - The patient will communicate basic needs through use of photos.
 - The patient will remain within the unit while in long-term care.

ANS: B

Interacting with others during activities is an appropriate goal to help the patient not feel so alone. Cognitive exercise is a goal for a patient with disturbed thought processes. Communication of basic needs through the use of photos is a goal for a patient with a diagnosis of impaired verbal communication and remaining in the unit is appropriate for chronic confusion.

DIF: Applying OBJ: 31.5 TOP: Assessment
 MSC: NCLEX Client Needs Category: Physiological Integrity NOT:
 Concepts: Cognition and Sensory Perception

12. The nurse is educating the family to care for a patient at home with cognitive alterations. Which statement by the family indicates a need for further education?
- I should keep the home free of scissors.||
 - I should minimize the number of visitors.||
 - I should use push-button door locks.||
 - 24-hour supervision may become necessary.||

ANS: C

Use of door locks that require a key may be necessary if the patient wanders. Keep the environment free of hazards such as sharp objects and minimize distractions. If the patient is not safe to be left alone, 24-hour supervision may be necessary.

DIF: Evaluating OBJ: 31.6

TOP: Evaluation

MSC: NCLEX Client Needs Category: Physiological Integrity

NOT: Concepts: Cognition

13. The nurse is delegating care to an unlicensed assistive personnel (UAP) to a patient who has sensory overload. Which statement by the UAP indicates a need for further orientation?
- I should keep the noise levels low.¶
 - I should schedule all the care together.¶
 - I should keep the room well lit.¶
 - I should allow the family to visit.¶

ANS: C

To prevent or alleviate overload, the nurse reduces sensory stimuli, dimming unnecessary lights and turning down the sound on alarms if possible. Nursing care is planned so that the patient is not constantly disturbed. Visitation by family provides reality orientation and a soothing, recognizable presence for some patients experiencing overload.

DIF: Applying OBJ: 31.6

TOP: Assessment

MSC: NCLEX Client Needs Category: Physiological Integrity NOT:

Concepts: Sensory Perception

14. The nurse is providing discharge instructions to a patient with visual alterations. Which statement by the patient indicates a need for further education?
- I should make sure the passageways are wide.¶
 - I should remove all the throw rugs.¶
 - I should keep the lights dim.¶
 - I can use a cane to feel for objects in front of me.¶

ANS: C

Bright lighting in hallways and stairways prevents falls by the patient who has limited vision. Furniture is placed to allow wide passageways. Throw rugs, which are a tripping hazard, are removed. If vision is severely limited, use of a cane or walking stick held slightly in front helps the patient feel objects in his/her path.

DIF: Applying OBJ: 31.6

TOP: Assessment

MSC: NCLEX Client Needs Category: Physiological Integrity NOT:

Concepts: Sensory Perception

MULTIPLE RESPONSE

- The nurse is completing an assessment of an older adult and notices some cognitive impairment not normally associated with aging. Which of these alterations would prompt further follow-up? (*Select all that apply.*)
 - The patient does not remember where her son lives.
 - The patient is unable to balance her checkbook.
 - The patient got lost in a city she never traveled to before.
 - The patient often has difficulty remembering words.

- e. The patient got lost going to her usual grocery store.

ANS: A, B, D, E

Symptoms of cognitive impairment include disorientation, loss of language skills, loss of the ability to calculate, poor judgment, and memory loss. If a patient exhibits these symptoms, further investigation is needed. Some decline in cognitive function occurs with aging, such as the ability to navigate easily in new areas.

DIF: Applying OBJ: 31.2 TOP: Implementation

MSC: NCLEX Client Needs Category: Physiological Integrity NOT: Concepts: Cognition

2. The nurse is providing education to the family of a patient being discharged with dementia. Which statement by the family indicates an appropriate level of understanding of dementia? (*Select all that apply.*)
- a. —The condition is permanent and has an acute onset.¶
 - b. —Alzheimer is the most common type of dementia.¶
 - c. —The condition worsens over time.¶
 - d. —I should observe for wandering behavior.¶
 - e. —Agitation can be worse in the evening.¶

ANS: B, C, D, E

Dementia, which is a permanent decline in mental function, has a subtle onset. The most common type of dementia is Alzheimer disease. Dementia is not reversible and worsens over time. Behavioral problems that arise in dementia patients include wandering, agitation, repetitive behaviors, and sundowning, or worsening of agitation and confusion in the evening.

DIF: Evaluating OBJ: 31.2 TOP: Evaluation

MSC: NCLEX Client Needs Category: Physiological Integrity NOT: Concepts: Cognition

3. The nurse is caring for a patient who suffered a stroke on the right side of the brain. The nurse is careful to implement what safety measures? (*Select all that apply.*)
- a. Puts a picture board in the room to communicate with the patient.
 - b. Places the call light on the patient's left side.
 - c. Leaves a light on in the bathroom at night for good visibility.
 - d. Places the call light on the patient's right side.
 - e. Makes sure there are no trip hazards in the patient's room.

ANS: C, D, E

If the damage is on the right side of the brain, there is loss of sensation and motor function in the extremities on the left side of the body and visual-spatial problems occur. Therefore, placing the call light on the side where the patient is likely to be strong is important. The patient will not necessarily have communication problems but might have visual problems, so the bathroom light is helpful. If the damage is on the left side of the brain, there is loss of sensation and motor function in the extremities on the right side of the body and problems with speech occur.

DIF: Applying OBJ: 31.2 TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment NOT: Concepts: Cognition and Sensory Perception

4. The nurse is performing a health history to determine the patient's cognitive status. Which questions will be best suited to elicit the information needed? (*Select all that apply.*)

- a. —Are you able to drive to the store or do errands?||
- b. —Do you have any pain?||
- c. —Is your vision blurry?||
- d. —Are you able to smell different foods?||
- e. —Have you noticed any difficulty adding up numbers?||

ANS: A, E

Driving and adding numbers relates to cognitive ability. The remaining three options have a sensory focus.

DIF: Applying OBJ: 31.3 TOP: Assessment
 MSC: NCLEX Client Needs Category: Physiological Integrity NOT:
 Concepts: Cognition and Sensory Perception

5. The nurse is performing a health history to determine the patient's sensory status. Which questions will be best suited to elicit the information needed? (*Select all that apply.*)
- a. —Do you ever lose your balance?||
 - b. —Do you wear glasses?||
 - c. —Do you like to read the newspaper?||
 - d. —Can you feel the difference between hot and cold water?||
 - e. —Do you wear a hearing aid?||

ANS: A, B, D, E

Balance, eyesight, hearing, and sensation are all sensory function. Asking if the patient likes the newspaper does not specifically address vision.

DIF: Understanding OBJ: 31.3 TOP: Assessment
 MSC: NCLEX Client Needs Category: Physiological Integrity
 NOT: Concepts: Cognition and Sensory Perception

6. The nurse is caring for a diabetic patient who has had a long history of poor glucose control. For what complications is the patient at risk? (*Select all that apply.*)
- a. Sudden loss of consciousness
 - b. Diabetic retinopathy
 - c. Stroke
 - d. Peripheral neuropathy
 - e. Memory loss

ANS: B, C, D, E

Long-term complications of hyperglycemia may contribute to cognitive and sensory deficits such as memory loss. They also can lead to diabetic retinopathy, peripheral neuropathy, and stroke. Loss of consciousness is usually seen with hypoglycemia in diabetics.

DIF: Understanding OBJ: 31.3 TOP: Assessment
 MSC: NCLEX Client Needs Category: Physiological Integrity
 NOT: Concepts: Cognition and Sensory Perception

7. The nurse is caring for a patient who is hospitalized with cognitive impairment and recognizes which interventions will assist the patient in orientation? (*Select all that apply.*)
- a. Keep a photo of the family in the room.
 - b. Use a clock on the wall.
 - c. Make sure the room is kept bright and well lit.

- d. Avoid moving the patient from room to room.
- e. Have each nurse introduce himself or herself to the patient.

ANS: A, B, D, E

The hospitalized patient with cognitive alterations is oriented by use of a clock, a calendar, and statements about the name of the location or name of the hospital. Orientation to person, place, and time is ongoing. Staff members are always identified by name, both verbally and nonverbally (with a name tag). The patient's environment is kept as constant as possible and moving the patient from room to room is avoided. Some familiar objects, such as a family photo, are placed near the patient if the hospital stay is longer than a few days. The environment is kept free of distractions such as loud noises and bright lights. Natural lighting to provide the patient with orientation to time of day can be accomplished by opening blinds or curtains during the day and darkening the room at night.

DIF: Applying OBJ: 31.6 TOP: Implementation

MSC: NCLEX Client Needs Category: Physiological Integrity NOT: Concepts: Cognition

8. The nurse is caring for a patient with expressive aphasia. Which interventions will assist the nurse in communicating with the patient? (*Select all that apply.*)
- a. Use simple phrases.
 - b. Speak loudly.
 - c. Use yes/no questions.
 - d. Use a picture board.
 - e. Be patient and unrushed.

ANS: C, D, E

If a patient has expressive aphasia, he or she understands language but is unable to answer questions, name common objects, or state simple ideas. The patient can answer yes/no questions by shaking the head. The patient might be able to point to pictures to express needs. For any type of aphasia, being patient and not rushing will make communication less stressful.

DIF: Applying OBJ: 31.6 TOP: Implementation

MSC: NCLEX Client Needs Category: Physiological Integrity NOT: Concepts: Cognition and Sensory Perception

9. The nurse is caring for a patient with receptive aphasia. Which interventions will assist the nurse in communicating with the patient? (*Select all that apply.*)
- a. Use simple phrases.
 - b. Speak louder than usual.
 - c. Stand in front of the patient.
 - d. Use a picture board.
 - e. Be patient and unrushed.

ANS: A, C, D, E

A patient with receptive aphasia cannot understand written or spoken language. Using simple phrases and talking either softly or loudly will not assist that patient. The sensory pathways are intact, but the words do not make sense. A picture board could be used by the nurse when assessing needs. As the patient participates in speech therapy, the ability to understand simple phrases may develop. Standing in front of the patient when talking may give non-verbal clues to the message.

DIF: Applying OBJ: 31.6 TOP: Implementation
 MSC: NCLEX Client Needs Category: Physiological Integrity NOT: Concepts: Cognition

10. The nurse is preparing discharge instructions for a patient who has tactile alterations in his legs. Which instructions would be included? (*Select all that apply.*)
- Verify bath water temperature is approximately 39.5 °C.
 - Do not use hot or cold therapy on any extremity.
 - Use sturdy shoes when walking outside or on hard surfaces.
 - Report any changes in skin color on your legs to your health care provider.
 - Set your water heater so that scalding is not possible.

ANS: C, D, E

Bath water temperature should be approximately 37.8 °C (100 °F), so 39.5 (103.1 °F) is too hot. Hot and cold therapy should not be used on the affected extremities, although it can be used on other areas of the body. Sturdy shoes can prevent foot injuries when there is decreased sensation in the lower extremities. Any decrease in sensation, change in the color of the skin, or wounds are reported to the health care provider. Water heaters are set so that scalding is not possible.

DIF: Applying OBJ: 31.6 TOP: Implementation
 MSC: NCLEX Client Needs Category: Physiological Integrity NOT:
 Concepts: Sensory Perception

11. The nurse is preparing discharge instructions for a patient who has equilibrium alterations. Which instructions will the nurse include? (*Select all that apply.*)
- Use grab bars in the tub and/or shower at home.
 - Keep rooms well-lit and focus ahead when walking.
 - Change positions quickly to avoid dizziness.
 - Use a cane or walker for stability.
 - Ride in the back seat of the car and look ahead.

ANS: A, B, D

The patient experiencing dizziness or vertigo exercises caution when changing positions. The patient suffering from motion sickness needs to ride in the front seat of the car and look far ahead through the car windshield. Keeping rooms well-lit and focusing ahead when walking, using grab bars in the shower and/or tub, and using canes or walkers are all good safety measures. Changing positions quickly may lead to dizziness.

DIF: Understanding OBJ: 31.6 TOP: Implementation
 MSC: NCLEX Client Needs Category: Physiological Integrity
 NOT: Concepts: Sensory Perception

Chapter 32: Stress and Coping**Yoost & Crawford: Fundamentals of Nursing: Active Learning for Collaborative Practice, 3rd Edition****MULTIPLE CHOICE**

1. The nurse knows the one theory explaining the variation in response to stress among individuals is identified by which term?
 - a. Stress appraisal
 - b. Sense of coherence
 - c. Allostasis
 - d. Homeostasis

ANS: B

Sense of coherence (SOC) is a characteristic of personality that references one's perception of the world as comprehensible, meaningful, and manageable. Stress appraisal is the automatic, often unconscious assessment of a demand or stressor. Allostasis is an alternative term for the stress response. Homeostasis is the tendency of the body to seek and maintain a condition of balance or equilibrium.

DIF: Understanding

OBJ: 32.1

TOP: Assessment MSC:

NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Stress

2. The nurse is caring for a patient with a new diagnosis of diabetes type 2. Which statement indicates a negative coping response?
 - a. —I will look up information on the Internet about diabetes.¶
 - b. —I will join a support group.¶
 - c. —I will only focus on learning to manage my medication first.¶
 - d. —I will make changes slowly so I can adapt to each change.¶

ANS: C

When the patient puts limits on learning by stating he/she will only learn about medication, he/she is using avoidance strategies to alleviate stress. Using strategies such as information gathering (seeking information about diabetes) is positive. Joining support groups and making changes slowly to adapt is also taking direct action by moving forward.

DIF: Understanding

OBJ: 32.1

TOP: Assessment

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Coping

3. The nurse is caring for a patient who is undergoing a major cardiac procedure. When the patient complains of a racing heart and nausea, the nurse recognizes these complaints as part of what hormone response?
 - a. Sense of coherence
 - b. Stress appraisal
 - c. Fight or flight
 - d. Sympathoadrenal response

ANS: C

In the —fight or flight response, the corticotropin-releasing hormone (CRH) released by the hypothalamus stimulates the pituitary to release adrenocorticotrophic hormone (ACTH). These hormones increase the heart rate, resulting in increased cardiac output, and the motility of the digestive tract is decreased, slowing digestive processes that could result in abdominal distress. Sense of coherence (SOC) is a characteristic of personality that references one's perception of the world as comprehensible, meaningful, and manageable. Stress appraisal is the automatic, often unconscious, assessment of a demand, or stressor. The sympathoadrenal response is a consequence of hypothalamic activation in sympathetic stimulation, which triggers epinephrine and norepinephrine release from the adrenal medulla.

DIF: Applying OBJ: 32.2 TOP: Assessment
 MSC: NCLEX Client Needs Category: Psychosocial Integrity NOT: Concepts: Coping

4. When the nurse measures the patient's blood glucose levels after an acute myocardial infarction (MI), the nurse knows this action is based on which rationale?
- Damaged muscle tissue releases glucose.
 - Corticosteroids increase glucose.
 - Myocardial infarctions are often seen in diabetics.
 - All patients should have their blood glucose checked.

ANS: B

The endocrine system responds to stress on the body such as what happens during an acute MI. Corticosteroids are important in the stress response because they increase serum glucose levels and inhibit the inflammatory response. Although MIs can be seen in diabetics, there is nothing to indicate this patient is diabetic. All patients do not routinely have their blood glucose checked regularly.

DIF: Applying OBJ: 32.2 TOP: Implementation
 MSC: NCLEX Client Needs Category: Psychosocial Integrity NOT: Concepts: Coping

5. The nurse is teaching a patient about the difference between mild anxiety and moderate anxiety. Which statement by the patient indicates a need for further education?
- Mild anxiety can help me remember things.
 - Moderate anxiety will narrow my focus.
 - Mild anxiety will help me be creative.
 - Moderate anxiety will increase my perception.

ANS: D

Moderate anxiety narrows a person's focus, dulls perception, and may challenge a person to pay attention or use appropriate problem-solving skills. Mild anxiety can be motivational, foster creativity, and increase a person's ability to think clearly.

DIF: Analyzing OBJ: 32.2 TOP: Implementation
 MSC: NCLEX Client Needs Category: Psychosocial Integrity NOT: Concepts: Stress and Coping

6. The nurse is providing discharge instructions for a patient with multiple sclerosis. Which discharge instruction is aimed at preventing a future exacerbation?
- Engage in some form of exercise as tolerated.
 - Avoid highly stressful situations.
 - Check your skin regularly for pressure sores.
 - Eat a diet with lots of fiber.

ANS: B

High stress levels are known to exacerbate multiple sclerosis and other autoimmune diseases. Exercise helps keep muscles loose and helps with balance. Assessing skin for pressure sores and eating a diet with high fiber prevents complications from multiple sclerosis.

DIF: Analyzing OBJ: 32.3 TOP: Implementation

MSC: NCLEX Client Needs Category: Psychosocial Integrity NOT: Concepts: Stress and Coping

7. The nurse is assessing level of stress in a patient from another culture. Which question is the most appropriate in helping the nurse understand the impact of the patient's belief system?
- Do you engage in prayer to help you during times of stress?||
 - Do you go to church or other form of organized worship?||
 - Do you have certain beliefs that are helpful during times of stress?||
 - Do you want spiritual counseling while you are here?||

ANS: C

The nurse needs to obtain a knowledge base of the patient's culture as well as identify health beliefs and cultural values from the patient's worldview. Asking the patient specific questions about prayer or church or spiritual counseling is inappropriate until the nurse first understands what the patient's own beliefs and practices are.

DIF: Analyzing OBJ: 32.4 TOP: Assessment

MSC: NCLEX Client Needs Category: Psychosocial Integrity NOT: Concepts: Stress and Coping

8. The nurse is performing a physical assessment of patient who is undergoing a bone marrow biopsy. What finding by the nurse indicates the patient is experiencing stress?
- Blood pressure of 120/84
 - Temperature of 99.5 °F (37.5 °C)
 - Heart rate of 110 beats/min
 - Respiratory rate of 10 breaths/min

ANS: C

The release of hormones increases the heart rate, resulting in increased cardiac output and elevated blood pressure. A reading of 120/84 is a normal blood pressure, and temperature is elevated is indicative of an infection. The respiratory rate increases in stress not decreases.

DIF: Analyzing OBJ: 32.4 TOP: Assessment

MSC: NCLEX Client Needs Category: Psychosocial Integrity NOT: Concepts: Stress and Coping

9. The nurse is assessing the patient's use of coping skills in response to stressful situations. The nurse identifies which question to be the most useful?
- Have you been evaluated for stress?||
 - Do you have someone you can go to for help when you are stressed?||
 - How have you managed stressful situations in the past?||
 - Does stress cause you to experience muscle tension or headaches?||

ANS: C

The use of open-ended questions assists in obtaining accurate information regarding the patient's stressors and coping skills. Questions that elicit yes/no answers will not allow the patient to provide as much information. Asking the patient about headaches and tension is asking about physical symptoms, not coping skills.

DIF: Applying OBJ: 32.4 TOP: Assessment

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Stress and Coping

10. The nurse is caring for a patient on a medical-surgical inpatient unit when the patient tells the nurse he is very sad and is considering suicide. What is the first thing the nurse should do?
- Notify the health care provider.
 - Make a referral to psychiatric services.
 - Implement one-on-one observations.
 - Document in the electronic medical record.

ANS: C

Verbalization of suicidal ideation or a suicide plan must be taken seriously. In the case of a hospitalized patient, one-on-one observation should be implemented to ensure patient safety. Once the patient is under observation, the health care provider is notified to put in the referral; nurses generally do not put in the referral. Documentation is always done after the patient's safety is ensured.

DIF: Analyzing OBJ: 32.4 TOP: Implementation

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Stress and Coping

11. The nurse identifies which goal to be appropriate for the nursing diagnosis of Difficulty coping?
- The patient will report an ability to remember discharge instructions.
 - The patient's family will understand how to access respite care services.
 - The patient will discuss possible coping strategies during weekly counseling sessions.
 - The patient will attend an online support group weekly.

ANS: C

An appropriate goal for Difficulty coping would be to discuss coping strategies. Remembering discharge instructions is an appropriate goal for Anxiety. Understanding how to access respite care services is an appropriate goal for Caregiver stress. Attending a support group is an appropriate goal for Difficulty coping.

DIF: Applying OBJ: 32.6 TOP: Diagnosis

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Stress and Coping

12. The nurse recognizes which goal to be appropriate for the nursing diagnosis of Anxiety?
- The patient will attend a weekly support group.
 - The patient will discuss possible coping strategies during weekly office visits.
 - The patient will report increased ability to concentrate on care instructions before discharge.
 - The patient's family will use respite care once a week for the next month.

ANS: C

Attending a weekly support group is an appropriate goal for Difficulty coping. An appropriate goal for Ineffective coping would be to discuss possible coping strategies during weekly visits. Using respite care once a week for the next month is an appropriate goal for Caregiver stress.

DIF: Applying OBJ: 32.6 TOP: Diagnosis

MSC: NCLEX Client Needs Category: Psychosocial Integrity NOT: Concepts: Stress and Coping

13. The nurse knows which goal to be appropriate for the nursing diagnosis of Caregiver stress?
- The patient will report an ability to focus on discharge instructions.
 - The caregiver will attend a coping skills class on a weekly basis.
 - Caregiver will use respite care for the family loved one once a week for the next month.
 - The patient will discuss strategies for coping with relationship violence within 24 hours.

ANS: C

The patient will discuss possible coping strategies during weekly office visits is an appropriate goal for Difficulty coping. The patient will report an ability to focus on discharge instructions is an appropriate goal for Anxiety. Relationship violence is not related.

DIF: Understanding OBJ: 32.6 TOP: Diagnosis
MSC: NCLEX Client Needs Category: Psychosocial Integrity NOT: Concepts: Stress and Coping

14. The nurse knows that when coordination between multiple health care disciplines is needed, which role should be utilized?
- Pastoral care
 - Case manager
 - Social worker
 - Dietitian

ANS: B

If coordination of care between multiple health care disciplines is needed, a case manager is

used. Pastoral care plays a significant role in addressing stress and anxiety issues when the patient has a preferred religion or strong faith background. A social worker identifies appropriate services and resources. A dietitian can provide education regarding dietary needs and food choices.

DIF: Understanding OBJ: 32.6 TOP: Implementation
MSC: NCLEX Client Needs Category: Psychosocial Integrity NOT: Concepts: Stress and Coping

15. The nurse is providing education to a patient around anger management strategies. Which statement indicates a need for further education by the patient?
- Exercise can help me deal with the anger.||
 - I can use humor.||
 - I can punch things.||
 - I can take a time-out.||

ANS: C

Strategies should focus on nonviolent methods. Some anger management interventions include expressing feelings in a calm, non-confrontational manner; exercising; identifying potential solutions; taking a time-out; forgiving; diffusing the situation with humor; owning one's feelings; and breathing deeply.

DIF: Understanding OBJ: 32.7 TOP: Implementation
MSC: NCLEX Client Needs Category: Psychosocial Integrity NOT: Concepts: Stress and Coping

16. The nurse is educating the patient about alternative therapies. Which statement by the patient indicates a need for more information?
- Alternative therapies can include relaxation techniques.
 - Alternative therapies are used in conjunction with medical therapies.
 - Alternative therapies can be used when patients are experiencing stress.
 - Some alternative therapists require certification.

ANS: B

Alternative therapies are used in place of medical treatment. These types of interventions are useful when patients are experiencing physiologic and psychological responses to stress. Some complementary and alternative therapies such as therapeutic touch, Reiki, biofeedback, and massage therapy require additional certification and training, whereas muscle relaxation and guided imagery do not.

DIF: Understanding

OBJ: 32.7

TOP: Implementation

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Stress and Coping

17. The nurse is educating the patient on the use of relaxation therapy. Which statement by the patient indicates a need for further education?
- I should relax my muscles from head to toe.‖
 - I visual the relaxed muscle.‖
 - I should do this three times a week.‖
 - I focus on muscles that are tense.‖

ANS: C

This technique should be done daily. Typically, relaxation progresses from head to toe. With practice, the patient visualizes an image of the relaxed muscles and will be able to relax muscles from the mental image. Progressive relaxation is implemented by having patient's focus on muscles that are tensed and then intentionally relax those muscle groups.

DIF: Applying

OBJ: 32.7

TOP: Implementation

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Stress and Coping

18. The nurse is seeing a patient during a follow-up visit after discharge in which the patient had a nursing diagnosis of Difficulty coping. Which statement by the patient would be a cause for concern?
- I am sleeping better most nights.‖
 - I feel less anxious.‖
 - I do not need to do the relaxation exercises anymore.‖
 - I am continuing my exercises every day.‖

ANS: C

Patients need to continue using the stress-reduction techniques to maintain a feeling of well-being. Once stress decreases, patients typically report feeling better, sleeping more soundly, and feeling less anxious. Continuing their positive activities such as exercising is good.

DIF: Applying

OBJ: 32.5

TOP: Implementation

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Stress and Coping

MULTIPLE RESPONSE

1. The nurse knows that when patients are experiencing stress, which physiologic changes can be seen in their signs and symptoms? (*Select all that apply.*)
 - a. Increase in heart rate
 - b. Flaccid muscles
 - c. Pupil dilation
 - d. Decrease in blood pressure
 - e. Increase in respiratory rate

ANS: A, C, E

The physiologic response to stress, whether physical or psychological, is activation of the autonomic nervous system, resulting in an increase in heart rate, blood pressure, and respirations along with pupil dilation and muscle tension and decreased blood flow to the skin.

DIF: Applying OBJ: 32.1 TOP: Assessment

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Stress

2. The nurse knows when the body responds to the release of hormones during —fight or flight, that response includes which physiological signs? (*Select all that apply.*)
 - a. Decreased respiratory rate
 - b. Slowing of the digestive process
 - c. Glucose being mobilized from the liver
 - d. Pupils dilating
 - e. Smooth muscles in the bronchi constricting

ANS: B, C, D

The release of hormones increases the heart rate, resulting in increased cardiac output, and elevated blood pressure. There is an increase in the flow of blood to muscles at the expense of the digestive and other systems not immediately needed in the fight-or-flight response.

Smooth muscles in the bronchi relax and dilate the bronchi and smaller airways, and the respiratory rate increases, allowing for an enhanced flow of well-oxygenated blood to muscles and other organs. The motility of the digestive tract is decreased, slowing digestive processes, but glucose and fatty acids are mobilized from the liver and other stores to support increased mental activities (alertness) and skeletal muscle function. Pupillary dilation produces a larger visual field.

DIF: Understanding

OBJ: 32.2

TOP: Assessment MSC:

NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Stress

3. The nurse recognizes which personality factors that have been shown to buffer the impact of stress? (*Select all that apply.*)
 - a. Resilience
 - b. Sense of coherence
 - c. Gender
 - d. Hardiness
 - e. Coping style

ANS: A, B, D

Personality factors such as resilience, hardiness, and sense of coherence can buffer the impact of stress, reducing the negative consequences. Gender is not a personality factor. Coping style refers to a pattern of measures taken to relieve stress but is not a personality factor.

DIF: Understanding

OBJ: 32.2

TOP: Assessment

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Stress and Coping

4. The nurse identifies which factors that center on the childhood stress related to school experiences? (*Select all that apply.*)
- a. Goal achievement
 - b. Family dissolution
 - c. Life changes
 - d. Test anxiety
 - e. Competition

ANS: A, D, E

Childhood stress related to the school experience centers on competition, goal achievement, and test anxiety. Family dissolution and life changes are not related to the school experience.

DIF: Understanding

OBJ: 32.3

TOP: Assessment MSC:

NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Stress

5. The nurse working with older adults wants to support healthy coping strategies. What actions by the nurse are most appropriate? (*Select all that apply.*)
- a. Installing boxing equipment in the recreation room
 - b. Provide reminiscing sessions for the adults to share personal stories
 - c. Arrange for gentle yoga to be provided at the senior center
 - d. Create activities designed to distract them from their losses
 - e. Encourage the adults to eat frequent, healthy snacks

ANS: B, C, E

To promote health coping in older adults, the nurse would provide reminiscing sessions, yoga, and would encourage small healthy snacks as this population frequently loses their appetite when stressed. Boxing equipment might cause the adults to focus on anger. Distraction can be a negative or positive coping mechanism.

DIF: Applying

OBJ: 32.7

TOP: Implementation

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Stress and Coping

6. The nurse manager of a busy oncology unit is concerned about compassion fatigue among the nursing staff. Which signs and symptoms would alert the nurse to this problem? (*Select all that apply.*)
- a. Nurses become very emotionally upset without an apparent cause.
 - b. Nurses start to avoid caring for certain patients.
 - c. Nurses start to call in sick more often.
 - d. Nurses begin working more overtime.
 - e. Nurses have difficulty showing empathy for patients.

ANS: A, B, C, E

Compassion fatigue occurs when deeply caring and empathetic nurses become overwhelmed by the constant needs of patients and families. Symptoms include mood swings, avoidance of working with some patients, frequent sick days, irritability, reduced memory, poor concentration, and a decreased ability to show empathy.

DIF: Applying

OBJ: 32.8

TOP: Assessment

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Stress and Coping

7. The nurse manager of the unit is implementing a program to assist the nursing staff in managing compassion fatigue. Which interventions will be the most successful? (*Select all that apply.*)
- a. Support group that nurses can participate in that meets on the unit
 - b. Exercise competitions to encourage nurse to exercise and log their time
 - c. Organized break times so nurses can get off the unit for breaks and lunches
 - d. Quiet area on the unit where the nurses can go during break
 - e. Promotion of work-life balance

ANS: A, B, C, D, E

To care most effectively for others, nurses must first take time to care for themselves. Many of the stress reduction interventions incorporated into patient care plans can be effective in addressing the stressors faced by nurses. Exercise, balanced nutrition, and mindfulness therapy have been shown to help health care professionals in coping with the demands of patient care. Interventions designed specifically to prevent nurse burnout and address compassion fatigue include mentoring programs, quiet areas on a nursing unit for relaxation, availability of pastoral care, the sharing of feelings with trusted colleagues, and promotion of work-life balance.

DIF: Applying OBJ: 32.8 TOP: Implementation

MSC: NCLEX Client Needs Category: Psychosocial Integrity

NOT: Concepts: Stress and Coping

NURSINGTB.COM

Chapter 33: Sleep**Yoost & Crawford: Fundamentals of Nursing: Active Learning for Collaborative Practice, 3rd Edition****MULTIPLE CHOICE**

1. The nurse identifies what physiological response occurs with the onset of darkness and in preparation for sleep?
 - a. Cortisol levels peak
 - b. Cortisol levels increase
 - c. Core body temperature increases
 - d. Melatonin levels increase

ANS: D

Melatonin levels increase and core temperature and cortisol levels decrease with the onset of darkness. Cortisol levels peak at 6 a.m.

DIF: Understanding

OBJ: 33.1

TOP: Assessment

MSC: NCLEX Client Needs Category: Physiological Integrity

NOT: Concepts: Caring Interventions

2. The nurse recognizes what function of the reticular activating system (RAS)?
 - a. Records brain waves and other variables.
 - b. Relays motor impulse to the hypothalamus.
 - c. Influences patterns of biological functioning.
 - d. Is affected by the light-dark cycle.

ANS: B

The RAS receives sensory impulses from the spinal cord and relays motor impulses to the thalamus and all parts of the cerebral cortex. Polysomnography is the recording of brain waves and other physiologic variables, such as muscle activity and eye movements, during sleep. The circadian rhythms influence patterns of biological and behavioral functions, and the sleep-wake circadian rhythm is affected by the light-dark cycle.

DIF: Understanding

OBJ: 33.1

TOP: Assessment

MSC: NCLEX Client Needs Category: Physiological Integrity

NOT: Concepts: Caring Interventions

3. The nurse is educating a patient about taking measures to help avoid disruption to the circadian rhythm. Which statement by the patient indicates a need for further education?
 - a. —I know the circadian rhythm influences biological functions.||
 - b. —I know the circadian rhythm exists only in humans.||
 - c. —I know the sleep-wake circadian rhythm is impacted by the light-dark cycle.||
 - d. —The most familiar circadian rhythm is the day-night 24-hour cycle.||

ANS: B

Biological rhythms exist in plants, animals, and humans. In humans, these biorhythms, along with internal and external factors, affect sleep. The most familiar rhythm is the day-night, 24-hour circadian rhythm cycle. Circadian rhythms influence patterns of biological and behavioral functions. Some creatures are diurnal, or primarily active during the day, whereas others are nocturnal, with most of their activity during the night.

DIF: Applying OBJ: 33.1 TOP: Implementation
 MSC: NCLEX Client Needs Category: Physiological Integrity NOT:
 Concepts: Caring Interventions

4. The nurse teaches the patient what information about polysomnography?
- This is the recording of brain waves and other variables.
 - This is the relay of motor impulse to the hypothalamus.
 - This is the patterns of biological functioning.
 - This is the recording of seizure activity in the brain.

ANS: A

Polysomnography is the recording of brain waves and other physiologic variables, such as muscle activity and eye movements, during sleep. The reticular activating system (RAS) receives sensory impulses from the spinal cord and relays motor impulses to the thalamus. The circadian rhythms influence patterns of biological and behavioral functions. An electroencephalogram is used to record seizure activity in the brain.

DIF: Understanding OBJ: 33.1 TOP: Implementation
 MSC: NCLEX Client Needs Category: Physiological Integrity
 NOT: Concepts: Caring Interventions

5. The nurse identifies which sequence to be the usual progression of sleep?
- NREM 1-3 then REM, then back through NREM 1 and 2
 - REM then NREM 1-4, then back through NREM 2 and 3
 - NREM 1-3 then back through NREM 3 and 2 then REM
 - REM then NREM 1-4 then back through NREM 3

ANS: C

The usual sleep sequence for a person is a fairly rapid progression through NREM 1 through 3, back through NREM 3 and 2, and then into REM sleep. There is no NREM stage 4.

DIF: Understanding OBJ: 33.1 TOP: Assessment
 MSC: NCLEX Client Needs Category: Physiological Integrity
 NOT: Concepts: Caring Interventions

6. The nurse is providing discharge instructions to the parents of a toddler about sleeping habits. Which statement indicates further education is needed?
- Sleep needs may change during growth spurts.¶
 - Children sleep 12 hours a day.¶
 - Toddlers will often resist going to bed.¶
 - The bedtime routine can vary.¶

ANS: D

The regular bedtime routine should be consistently followed. Children need 11 to 14 hours of sleep a day, and toddlers and preschoolers may exhibit resistance to going to bed. Sleep needs fluctuate with growth spurts.

DIF: Applying OBJ: 33.1 TOP: Implementation
 MSC: NCLEX Client Needs Category: Physiological Integrity NOT:
 Concepts: Caring Interventions

7. When the nurse is explaining cataplexy to the patient, which description should be included?
- It is an uncontrolled desire to sleep.
 - It is falling asleep for several minutes.
 - It is loss of voluntary muscle tone.
 - It is a sleep cycle that begins with NREM.

ANS: C

Cataplexy is characterized by the sudden loss of voluntary muscle tone; vivid hallucinations during sleep onset or on awakening; and brief episodes of total paralysis at the beginning or end of sleep. An uncontrolled desire to sleep and falling asleep for several minutes define narcolepsy. Narcolepsy begins with REM sleep.

DIF: Understanding OBJ: 33.2 TOP: Implementation
 MSC: NCLEX Client Needs Category: Physiological Integrity
 NOT: Concepts: Caring Interventions

8. The nurse is providing discharge instructions to a patient who has had sleep alterations. Which statement by the patient indicates further education is needed?
- I should avoid drinking caffeine too close to bedtime.¶
 - I should not eat anything too close to bedtime.¶
 - I should exercise regularly to help with sleeping.¶
 - I can gain weight if I don't sleep enough.¶

ANS: B

NURSINGTB.COM

Going to bed hungry or eating a large, heavy, or spicy meal just before going to bed can interfere with sleep. Bedtime snacks that contain complex carbohydrates are recommended to promote calmness and relaxation. Highly caffeinated substances such as coffee, cola, and chocolate are central nervous system stimulants that can disrupt the sleep cycle. Exercise can assist with weight-loss efforts and promote fatigue and relaxation, but excessive exercise, especially in the evening, interferes with sleep. Both children and adults who sleep less than the recommended number of hours each night are more likely to be overweight.

DIF: Applying OBJ: 33.2 TOP: Implementation
 MSC: NCLEX Client Needs Category: Physiological Integrity NOT:
 Concepts: Caring Interventions

9. The nurse is performing an assessment of the patient's sleep patterns. Which question by the nurse will elicit the best response?
- Do you feel rested when you awaken?¶
 - What is your normal eating pattern?¶
 - Do you awaken during the night?¶
 - Do you drink beverages with caffeine?¶

ANS: C

When gathering information about the patient's sleep patterns, the best questions will be to ask if they awaken during the night. Asking the patient if they feel rested evaluates quality of sleep. Normal eating pattern and drinking beverages with caffeine establishes if they have any habits that might interfere with sleep.

DIF: Applying OBJ: 33.3 TOP: Assessment
 MSC: NCLEX Client Needs Category: Physiological Integrity NOT:
 Concepts: Caring Interventions

10. A patient has Sleep deprivation. What finding by the nurse best indicates goal achievement?
- Remains asleep for 6 to 7 hours consistently for 1 week.
 - Falls asleep within 15 minutes of going to bed.
 - Reports an ability to concentrate on tasks.
 - Verbalizes understanding of medication instructions.

ANS: A

The patient remaining asleep for 6 to 7 hours consistently for 1 week is an appropriate goal for Sleep deprivation. The patient falling asleep within 15 minutes of going to bed is a goal for Insomnia. The patient reporting an ability to concentrate on tasks is a goal for Anxiety. The patient repeating medication instructions is an appropriate goal for any patient.

DIF: Applying OBJ: 33.5 TOP: Diagnosis
 MSC: NCLEX Client Needs Category: Physiological Integrity NOT:
 Concepts: Caring Interventions

11. The nurse is working with a patient who has been advised to take 2 20-minute naps during the day for fatigue. After a week, the patient states the naps have not helped. What response by the nurse is best?
- NURSINGTB.COM**
- Maybe that is too much sleep for you during the day.¶
 - Why don't you try one 40-minute nap instead?¶
 - Let's explore some sleeping medications for you to try.¶
 - It often takes a few weeks for napping to help.¶

ANS: D

Taking 2 20-minute naps is an appropriate action for the patient with fatigue to implement. Many non-pharmacological interventions take weeks to become effective, so the nurse would explain that fact and encourage the patient to continue the napping.

DIF: Applying OBJ: 33.5 TOP: Implementation
 MSC: NCLEX Client Needs Category: Physiological Integrity NOT:
 Concepts: Caring Interventions

12. The nurse wants to help a hospitalized patient get more sleep. What intervention will be most helpful?
- Allow the patient an hour nap during the day shift.
 - Administer sleeping medication if the patient can't go to sleep after an hour.
 - Place a —do not disturb sign on the door for the duration of the night shift.
 - Cluster cares so the patient gets at least 90 minutes of uninterrupted sleep at night.

ANS: D

Being able to sleep for at least 90 minutes without interruption will best help this patient as the normal sleep cycle is approximately 90 minutes long. An hour nap is probably too long even in the hospital. Administering sleeping pills is an alternative if the patient truly can't sleep while hospitalized, but this is not the preferred solution. Not disturbing a hospitalized patient for an entire night shift is unrealistic.

DIF: Applying OBJ: 33.6 TOP: Implementation
 MSC: NCLEX Client Needs Category: Physiological Integrity NOT:
 Concepts: Caring Interventions

13. The nurse is providing discharge education for a patient with narcolepsy. Which statement by the patient indicates a need for further education?
- Daytime naps are helpful. ||
 - Taking the medication will cure it. ||
 - High protein meals are helpful. ||
 - I need to avoid alcohol. ||

ANS: B

There is no cure currently for narcolepsy. Treatment includes a regular exercise routine, a regular sleep routine, daytime naps if possible, light meals high in protein to maintain alertness, and vitamins. Avoiding alcohol, heavy meals, long-distance driving, and long periods of sitting is helpful.

DIF: Applying OBJ: 33.6 TOP: Implementation
 MSC: NCLEX Client Needs Category: Physiological Integrity NOT:
 Concepts: Caring Interventions

14. The nurse is providing discharge education for a patient with restless leg syndrome. Which statement by the patient indicates a need for further instruction?
- I should avoid all caffeine. ||
 - I can do leg massage and knee bends. ||
 - Taking magnesium supplements may be helpful. ||
 - Taking a walk regularly may be helpful. ||

ANS: A

Decreasing caffeine is helpful but does not need to be eliminated. Massaging the legs, walking, or doing deep knee bends may temporarily relieve symptoms. Supplements to correct deficiencies in iron, folate, and magnesium may be helpful.

DIF: Applying OBJ: 33.6 TOP: Implementation
 MSC: NCLEX Client Needs Category: Physiological Integrity NOT:
 Concepts: Caring Interventions

15. Which task is inappropriate for the nurse to delegate to the unlicensed assistive personnel (UAP)?
- Providing oral care
 - Evaluating sleep patterns
 - Providing bedtime routines
 - Documenting sleep hours

ANS: B

The nurse may not delegate assessment activities such as evaluating sleeping patterns. The nurse may delegate activities to help the patient prepare for sleep such as oral care and other bedtime routines. The UAP may document the hours slept.

DIF: Applying OBJ: 33.6 TOP: Implementation
 MSC: NCLEX Client Needs Category: Physiological Integrity NOT:
 Concepts: Caring Interventions

16. The nurse is providing discharge instructions for the patient with sleep pattern disturbances. Which statement by the patient indicates a need for further education?
- It is a good idea to have a bedtime routine.¶
 - My bedtime routine can include watching TV in bed until I fall asleep.¶
 - I will keep my regular sleep pattern on the weekend.¶
 - If I can't fall asleep, I will get out of bed and do something relaxing.¶

ANS: B

Use the bedroom for sleep and sex only; do not watch television, read, study, or eat in there. A consistent bedtime routine signals to the mind and body that it is time to go to sleep.

Maintaining a regular sleep pattern helps maintain circadian rhythms. Continue the regular sleep pattern on weekends and on holidays. Getting out of bed if unable to sleep trains the mind to sleep when in bed. If unable to fall asleep within 20 minutes, get out of bed and engage in a relaxing activity such as reading or listening to music until sleepy.

DIF: Applying OBJ: 33.6 TOP: Implementation
 MSC: NCLEX Client Needs Category: Physiological Integrity NOT:
 Concepts: Caring Interventions

MULTIPLE RESPONSE

NURSINGTB.COM

1. The nurse knows that during rapid eye movement (REM) sleep, which activities occur? (*Select all that apply.*)
- Memories are stored
 - Increase in cerebral blood flow
 - Slow rhythmic scanning eye movements
 - Release of epinephrine
 - Repair of brain cells

ANS: A, B, D

Rapid eye movement (REM) sleep occurs during deep sleep and is manifested by quick scanning movements of the eyes that are associated with dreaming. REM sleep is associated with memory storage, learning, increased cerebral blood flow, and epinephrine release. Repair of brain cells occurs during non-REM sleep.

DIF: Understanding OBJ: 33.1 TOP: Assessment
 MSC: NCLEX Client Needs Category: Physiological Integrity
 NOT: Concepts: Caring Interventions

2. The student nurse learns that during non-rapid eye movement (NREM) sleep, which activities occur? (*Select all that apply.*)
- Brain waves slow
 - Slow rhythmic scanning eye movements

- c. Dreaming
- d. Drop in blood pressure
- e. Conservation of energy

ANS: A, D, E

During non-rapid eye movement (NREM) sleep, in which REM does not occur, physiological activity is reduced, brain waves, breathing and heart rate slow, and blood pressure drops. Slow scanning eye movements do not occur in either REM or NREM. Dreaming occurs in REM.

DIF: Understanding

OBJ: 33.1

TOP: Assessment

MSC: NCLEX Client Needs Category: Physiological Integrity

NOT: Concepts: Caring Interventions

3. The nurse recognizes which changes in sleep patterns occur in the older adult? (*Select all that apply.*)
- a. Sleep increases to approximately 8 to 10 hours a night.
 - b. REM sleep is shorter.
 - c. Stage 4 NREM is decreased.
 - d. The use of medication may interfere with sleep.
 - e. Older adults awaken more at night.

ANS: C, D, E

Older adults sleep approximately 7 to 8 hours a night. The first REM stage is longer. There is no stage 4 REM. Deeper stages of sleep are shortened, resulting in less restorative sleep. A decline in health or the use of medications may interfere with sleep. Older adults awaken more at night and take longer to go back to sleep.

DIF: Understanding

N R I C B . C
U S N M
OBJ: 33.1

TOP: Assessment

MSC: NCLEX Client Needs Category: Physiological Integrity

NOT: Concepts: Caring Interventions

4. The nurse is providing community education on sudden infant death syndrome (SIDS). What information does the nurse include? (*Select all that apply.*)
- a. SIDS is the second most common cause of death among infants (1 to 12 months).
 - b. The etiology remains largely unknown.
 - c. The most modifiable risk factor is sleeping supine.
 - d. Risk factors include being exposed to cigarette smoke.
 - e. It is defined as sudden unexpected death.

ANS: B, D, E

SIDS is the leading cause of death among infants 1 to 12 months of age. The etiology remains largely unknown. The most important modifiable SIDS risk factor appears to be prone sleeping. Risk factors include the infant's being exposed to cigarette smoke. Sudden infant death syndrome (SIDS) is defined as the sudden unexpected death of an infant younger than 1 year of age that remains unexplained after a thorough postmortem investigation.

DIF: Understanding

OBJ: 33.2

TOP: Assessment

MSC: NCLEX Client Needs Category: Physiological Integrity

NOT: Concepts: Caring Interventions

5. The nurse recognizes which sleeping conditions are identified as dyssomnias? (*Select all that apply.*)

- a. Difficulty getting to sleep
- b. Nocturnal enuresis
- c. Inability staying asleep
- d. Being excessively sleepy
- e. Falling asleep during the day

ANS: A, C, D, E

Dyssomnias are disorders associated with getting to sleep, staying asleep, or being excessively sleepy. Nocturnal enuresis is a parasomnia.

DIF: Understanding

OBJ: 33.2

TOP: Assessment

MSC: NCLEX Client Needs Category: Physiological Integrity

NOT: Concepts: Caring Interventions

6. The nurse manager is concerned about nursing staff who are working the night shift. What interventions can the manager suggest to help the nurses overcome shift-related sleep disturbances? (*Select all that apply.*)
- a. Power nap before leaving for the first night shift.
 - b. Get a minimum of 4 hours of sleep.
 - c. Wear dark glasses when driving home from work.
 - d. Seek exposure to bright light when waking.
 - e. Maintain a regular sleeping schedule when working and on nights off.

ANS: A, C, D, E

Obtain a minimum of 6 hours of sleep. Maintain a regular sleep schedule when working and on nights off. Wear dark glasses that block blue light when driving home after night work. Seek exposure to bright light (sunlight is best) as soon as possible after waking. Before the first night shift, power-nap 30 to 90 minutes before leaving for work.

DIF: Understanding

OBJ: 33.2

TOP: Assessment

MSC: NCLEX Client Needs Category: Physiological Integrity

NOT: Concepts: Caring Interventions

7. The nurse conducting a sleep workshop in the community would identify which patients to be at risk for obstructive sleep apnea (OSA)? (*Select all that apply.*)
- a. Deviated septum
 - b. Recessed chin
 - c. Alcohol use
 - d. Large neck
 - e. Recent tonsillectomy

ANS: A, B, C, D

Risk factors for OSA include obesity, large neck circumference, smoking, alcohol use, and a family history of OSA. Structural abnormalities such as a recessed chin, abnormal upper-airway structures, deviated septum, nasal polyps, or enlarged tonsils can predispose a person to OSA.

DIF: Understanding

OBJ: 33.2

TOP: Assessment

MSC: NCLEX Client Needs Category: Physiological Integrity

NOT: Concepts: Caring Interventions

8. The nurse is admitting a patient to the general medical–surgical unit. What should the nurse assess as part of a routine sleep assessment? (*Select all that apply.*)
- a. Usual sleeping and waking times
 - b. Bedtime routines
 - c. Sleeping environment preferences
 - d. Medications used for sleep
 - e. Any current life events

ANS: A, B, C, D, E

A sleep assessment should be completed when a patient is admitted to a health facility. The nurse assesses the patient's usual sleeping and waking times, medications, illnesses, bedtime routines, and sleeping environment preferences and incorporates the information into the plan of care when possible. The nurse should also assess current life events and emotional status.

DIF: Applying OBJ: 33.3

TOP: Assessment

MSC: NCLEX Client Needs Category: Physiological Integrity NOT:

Concepts: Caring Interventions

9. The nurse will include which interventions to help improve sleep quality during hospitalization on all patients' care plans? (*Select all that apply.*)
- a. Maintaining sleep routines
 - b. Minimizing disruptions
 - c. Providing light snacks
 - d. Using sleep medications
 - e. Using relaxation measures

ANS: A, B, C, E

Medications would be used carefully and do not always improve sleep. Addressing the sleep environment, maintaining sleep routines, providing light snacks if allowed, and instituting relaxation measures will all improve sleep.

DIF: Applying OBJ: 33.6

TOP: Implementation

MSC: NCLEX Client Needs Category: Physiological Integrity NOT:

Concepts: Caring Interventions

Chapter 34: Diagnostic Testing**Yoost & Crawford: Fundamentals of Nursing: Active Learning for Collaborative Practice, 3rd Edition****MULTIPLE CHOICE**

1. The nurse is caring for a patient who has diabetes. The patient reports compliance with the medical regime. Which test result indicates to the nurse that the patient has not been compliant with the treatment plan?
 - a. Hemoglobin A_{1c} 16%
 - b. Random blood sugar (RBS) 112 mg/dL
 - c. Lactate dehydrogenase (LDH) 55 units/L
 - d. Erythrocyte sedimentation rate (ESR) 14 mm/hr

ANS: A

Hemoglobin A_{1c} (Hgb A_{1c}), or glycosylated hemoglobin, testing evaluates blood sugar levels over a period of 2 to 3 months. This blood test is performed to provide the primary care provider (PCP) with information about long-term blood sugar control. The normal value of Hgb A_{1c} in patients without diabetes is 4% to 5.9%. The American Diabetes Association (2016) states that diabetes is diagnosed for Hgb A_{1c} levels greater than 6.5%. A higher level indicates that the patient has had poor blood glucose control during the past few weeks, and increases the patient's risk of long-term complications from hyperglycemia. The other tests are not related to long-term diabetes control.

DIF: Applying OBJ: 34.2 TOP: Assessment
 MSC: NCLEX Client Needs Category: Physiological Adaptation NOT:
 Concepts: Glucose Regulation

2. The nurse is caring for an elderly patient with dementia. Which laboratory finding indicates to the nurse that that patient is often forgetting to eat meals?
 - a. Serum bilirubin 0.4 mg/dL
 - b. PLT (platelet count) 425,000/mm³
 - c. Serum cholesterol 175 mg/dL
 - d. Albumin 1.4 g/dL

ANS: D

Albumin level is an indicator of the patient's protein intake and nutritional status. Normal albumin level is 3.3 to 5 g/dL. It is an essential component of fluid balance, responsible for maintaining colloidal oncotic pressure in the vascular and extravascular spaces. Low levels of albumin may indicate malnutrition.

DIF: Applying OBJ: 34.2 TOP: Assessment
 MSC: NCLEX Client Needs Category: Management of Care NOT: Concepts: Nutrition

3. The nurse is caring for a patient who has a deep leg wound that is badly infected. Which laboratory test results will the nurse expect to find in the patient's chart?
 - a. Elevated C-reactive protein (CRP) 6.5 mg/dL
 - b. Decreased serum creatinine 0.8 mg/dL
 - c. Elevated serum bilirubin 0.5 mg/dL
 - d. Prothrombin time (PT) 11.5 sec

ANS: A

C-reactive protein (CRP) is produced by the liver in response to inflammation, tissue damage, and infection. Blood levels of CRP have been used as a marker for inflammatory and autoimmune disorders. The nurse would expect to see an elevated CRP in a patient with an infected wound. Creatinine is an indicator of kidney function, and bilirubin is an indicator of liver function. Prothrombin time indicates clotting ability of the blood, particularly when the patient is taking warfarin (Coumadin).

DIF: Applying OBJ: 34.2 TOP: Assessment
 MSC: NCLEX Client Needs Category: Physiological Adaptation NOT:
 Concepts: Infection

4. The nurse is caring for a patient who has a bleeding gastric ulcer. How will the nurse expect the patient's stool to appear?
- Soft and formed with bright red streaks
 - Watery with particles of undigested food
 - Sticky and black
 - Hard lumps that are difficult to pass

ANS: C

Bleeding anywhere along the GI tract results in blood in the stool. Bleeding that occurs in the upper GI tract produces stools that are black and tarry in appearance. Bleeding within the lower GI tract presents with soft stools that have bright red streaks. Watery stool with particles of food is indicative of gastroenteritis. Hard lumps that are difficult to pass indicate constipation, often from medications or lack of fiber in the diet.

DIF: Applying OBJ: 34.3 TOP: Assessment MSC:
 NCLEX Client Needs Category: Physiological Adaptation
 NOT: Concepts: Elimination

5. The nurse is caring for a patient who is to have a noncontrast MRI scan performed. Which assessment finding leads the nurse to report that the patient may not be able to have the test?
- The patient has an implanted insulin pump.
 - The patient is breastfeeding her newborn infant.
 - The patient is severely allergic to iodine and latex.
 - The patient has profound hearing loss.

ANS: A

Any metal implants are a contraindication for an MRI scan because the scan uses powerful magnets. Insulin pumps often contain metal that can react with the strong magnets in the MRI machine. Breastfeeding is not a contraindication to MRI because there is no radiation exposure. No latex or iodine is used during MRI testing. Profound hearing loss will not be a problem, although MRI scanning is very loud.

DIF: Understanding OBJ: 34.5 TOP: Assessment
 MSC: NCLEX Client Needs Category: Reduction of Risk Potential
 NOT: Concepts: Safety

6. The nurse is caring for a patient who has had severe acid reflux. Which test will allow the physician to directly check for damage to the esophagus?
- Esophagogastroduodenoscopy (EGD)

- b. MRI scan with contrast
- c. Abdominal ultrasound
- d. Positron emission tomography (PET) scan

ANS: A

EGD is performed using a lighted tube that allows for direct visualization of the esophagus, stomach, and upper duodenum. MRI, ultrasound, and PET scanning do not allow physicians to see the esophagus directly.

DIF: Understanding

OBJ: 34.4

TOP: Assessment

MSC: NCLEX Client Needs Category: Reduction of Risk Potential

NOT: Concepts: Tissue Integrity

7. The nurse is caring for a woman who has a cyst in her breast that was found at her recent mammogram. The physician wants to make sure that the cyst is not malignant. Which test will be used to determine this?
- a. Needle aspiration with biopsy
 - b. Paracentesis
 - c. Thoracentesis
 - d. Fiberoptic endoscopy

ANS: A

Needle aspirations are procedures that are used to remove fluid and tissue for testing. A biopsy involves removing a larger collection of cells, as in a tumor or mass, and may be used to detect cancer in the skin, breast, or liver. Paracentesis is drainage of fluid from the abdomen, and thoracentesis is drainage of fluid from the pleural cavity. Fiberoptic endoscopy allows the physician to see inside the upper and/or lower GI tract.

NURSINGTB.COM

DIF: Understanding

OBJ: 34.4

TOP: Assessment

MSC: NCLEX Client Needs Category: Reduction of Risk Potential

NOT: Concepts: Technology

8. The nurse is caring for a patient who is scheduled for a needle aspiration and biopsy to rule out cancer. Which Nursing diagnosis is appropriate and important for this patient?
- a. Anxiety related to potential for cancer diagnosis depending on biopsy results
 - b. Impaired health maintenance related to delayed insurance coverage for procedure
 - c. Powerlessness related to lengthy wait for diagnosis
 - d. Ineffective coping related to patient stated she is a little nervous about the test results

ANS: A

Fear is an emotion commonly experienced by patients waiting for diagnostic tests and biopsy results. Impaired health maintenance related to delayed insurance coverage is not a priority diagnosis for this patient at this time. Powerlessness is about the patient's ability to control an outcome and is not related to the wait for test results. The patient statement of feeling a little nervous about the test results is not indicative of ineffective coping.

DIF: Applying

OBJ: 34.6

TOP: Diagnosis

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:

Concepts: Coping

9. The nurse is caring for a patient who will be undergoing flexible sigmoidoscopy testing to screen for colon cancer. What goal will the nurse include in the patient's plan of care?
- Patient will verbalize understanding of preprocedure preparation to be completed at home the day before the test.
 - Patient will feel comfortable about the upcoming test and have trust in the health care providers.
 - Patient will learn common side effects of the medications used to prepare the GI tract for endoscopy testing.
 - Patient will realize how important regular sigmoidoscopy testing is in the prevention of colon cancer.

ANS: A

The patient will need to complete colon preparation prior to the sigmoidoscopy testing. The nurse must determine that the patient understands how and when to complete the prep. Having the patient verbalize understanding of the prep procedure is an objective goal so that the nurse can readily determine whether or not it has been met. The other goals are not objective or measurable, so the nurse cannot determine whether or not they have been met.

DIF: Applying OBJ: 34.7 TOP: Planning
 MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:
 Concepts: Patient Education

10. The nurse is caring for a patient who will be receiving iodine-based contrast medium for a CT scan. Which allergy should be reported to the technician and radiologist before the test is performed?
- Gluten and lactose
 - Strawberries and blueberries
 - Peanuts and cashews
 - Shrimp and scallops

ANS: D

If the patient is undergoing an examination that involves an iodine contrast medium, check for a history of adverse reactions or allergies to iodine-containing food (e.g., shellfish, cabbage, kale, iodized salt). The other allergies are not related.

DIF: Remembering OBJ: 34.5 TOP: Assessment
 MSC: NCLEX Client Needs Category: Reduction of Risk Potential
 NOT: Concepts: Safety

11. The nurse is caring for a patient who has just undergone bronchoscopy. The patient requests a drink of water. What is the nurse's best action?
- Provide ice chips.
 - Check the patient for a gag reflex.
 - Provide a small cup of ice water with a straw.
 - Keep the patient NPO.

ANS: B

Numbing medication is applied to the back of the throat just before bronchoscopy. This may lead to swallowing difficulty and risk for aspiration until the gag reflex returns. The nurse should keep patient NPO until swallow, gag, and cough reflexes have returned.

The nurse does not need to keep the patient NPO after the gag reflex returns so it should be checked in order to allow the patient to have fluids as soon as possible to relieve thirst.

DIF: Applying OBJ: 34.8 TOP: Assessment MSC:
NCLEX Client Needs Category: Reduction of Risk Potential NOT:
Concepts: Safety

12. The nurse is caring for a patient who will be undergoing bone marrow biopsy. Which statement by the patient indicates that additional teaching is needed?
- I will count the ceiling tiles when the doctor inserts the numbing medicine.||
 - I will take acetaminophen later today if the site becomes uncomfortable.||
 - I will squeeze your hand to help calm my fears about the test.||
 - I will keep the biopsy site clean and dry for the next 24 hours.||

ANS: A

The patient will be positioned in the prone or lateral position for the test, so the patient will not be able to count ceiling tiles as a distraction during the numbing step of the test. The patient may take acetaminophen as needed for discomfort afterward. The biopsy site must be kept clean and dry for 24 hours after the biopsy to prevent infection. Holding the nurse's hand will help calm the patient before and during the procedure.

DIF: Applying OBJ: 34.8 TOP: Assessment MSC:
NCLEX Client Needs Category: Reduction of Risk Potential NOT:
Concepts: Patient Education

13. The nurse is caring for a patient who is sedated following a colonoscopy. Which is the priority action of the nurse?
- Provide a quiet, dark environment so that the patient can rest comfortably.
 - Monitor the patient's pulse oximetry and respirations closely.
 - Inform the patient that the procedure has been completed.
 - Assess the patient's bowel sounds and passage of flatus.

ANS: B

The priority intervention for sedated patients is to monitor pulse oximetry and respirations closely because sedation may suppress the respiratory drive. The nurse should monitor vital signs until the patient is fully awake and observe stools for visible blood. The nurse should also instruct the patient to report any abdominal pain as these assessment findings are alerts for possible perforation of bowel, hypotension, and hemorrhage. Providing a quiet environment is nice for the patient, but dim lighting may impair the nurse's ability to assess the patient. Informing the patient that the procedure has been completed is not a priority. Assessing the patient's bowel sounds and passage of flatus is not as important as careful respiratory monitoring.

DIF: Understanding OBJ: 34.8 TOP: Assessment
MSC: NCLEX Client Needs Category: Reduction of Risk Potential
NOT: Concepts: Gas Exchange

14. The nurse is caring for a patient who recently had a liver biopsy. To whom must the nurse give the results?
- The patient
 - The patient's health care provider

- c. The patient's insurance provider
- d. The patient's spouse

ANS: B

HIPAA protects the patient by requiring that testing results be shared only with health care professionals who need the information to provide treatment and with individuals designated in writing by the patient. The patient's provider will need the biopsy results to determine the patient's plan of care. The nurse does not give test results to the insurance company. The nurse may share the results with the patient or spouse, but it is not required, unless designated by the patient in writing.

DIF: Understanding

OBJ: 34.8

TOP: Implementation

MSC: NCLEX Client Needs Category: Management of Care

NOT: Concepts: Health Care Law

15. The nurse is caring for a patient who is to collect a 24-hour urine specimen. Which statement by the patient indicates that additional teaching is required?
- a. —I will keep the urine container on ice to keep it chilled until I bring it to the lab.¶
 - b. —I will start the test over if I forget and urinate into the toilet during the testing time.¶
 - c. —I will start the test tomorrow after I urinate first thing in the morning.¶
 - d. —I will drink extra fluids so that the lab will have a large specimen to test.¶

ANS: D

Drinking extra fluids so that the lab will have an extra-large specimen to test is not done as part of 24-hour urine collection, and it may skew the test results. The specimen should be kept chilled on ice or in a refrigerator until it is brought to the lab. If the patient accidentally urinates in the toilet, the test must be started over again. Urine collection is started after the patient's first void of the morning into the toilet.

DIF: Applying

OBJ: 34.3

TOP: Assessment MSC:

NCLEX Client Needs Category: Reduction of Risk Potential NOT:

Concepts: Patient Teaching

16. The nurse is caring for a patient whose immune system is destroying red blood cells at a very rapid rate. Which test result will the nurse expect to see in the patient's chart as a result?
- a. Bilirubin level 4 mg/dL
 - b. Platelet count 450,000/mm³
 - c. Serum uric acid level 1.7 mg/dL
 - d. Partial thromboplastin time 45 seconds

ANS: A

An elevated bilirubin level is the result of increased red blood cell destruction. Normal bilirubin levels are 0.3 to 1.0 mg/dL. Increased platelet count, decreased serum acid level, and decreased partial thromboplastin times are not indicative of increased red blood cell destruction.

DIF: Applying

OBJ: 34.2

TOP: Assessment

MSC: NCLEX Client Needs Category: Physiological Adaptation NOT:

Concepts: Gas Exchange

17. The nurse is caring for a patient with a urinary tract infection. Which test will indicate which antibiotics will be effective to treat the infection?

- a. Complete blood count (CBC)
- b. Culture and sensitivity (C&S)
- c. Renal scan and angiography
- d. Radioreceptor assay for HCG

ANS: B

Culture and sensitivity are performed on specimens to determine which bacteria are causing the infection and which antibiotics will be effective treatment. CBC, renal scan, and radioreceptor assay for HCG will not indicate which antibiotics may be used to treat an infection.

DIF: Remembering

OBJ: 34.4

TOP: Assessment

MSC: NCLEX Client Needs Category: Reduction of Risk Potential

NOT: Concepts: Infection

18. The nurse is caring for a patient who has just undergone paracentesis. For which complication will the nurse carefully monitor?
- a. Collapse of the lung with shortness of breath
 - b. Fecal impaction from retained barium in the colon
 - c. Cerebrospinal fluid leak resulting in severe headache
 - d. Perforation of the bowel resulting in abdominal infection

ANS: D

Paracentesis is drainage of fluid from the abdominal cavity. Since the needle is near the intestines, bowel perforation can occur, manifested by abdominal pain and fever as infection (peritonitis) sets in. Possible complications do not include lung collapse, CSF leak, or impaction.

NURSINGTB.COM

DIF: Applying

OBJ: 34.8

TOP: Assessment

MSC: NCLEX Client Needs Category: Reduction of Risk Potential

NOT: Concepts: Safety

19. The nurse is caring for a patient who is having blood drawn as part of preoperative testing. Which step is the most important to ensure the safety of the patient and the nurse?
- a. Ensuring that the tourniquet is not left in place for too long
 - b. Using the smallest possible needle for venipuncture
 - c. Properly disposing of the needle after the specimen is obtained
 - d. Making sure that all of the collection tubes are filled completely

ANS: C

Proper disposal of needles and sharps after procedures is essential for safe nursing practice to ensure the safety of staff as well as patients. Ensuring that the tourniquet is not left on too long, using the smallest needle possible, and making sure that all of the vials are filled are important steps in venipuncture, but only proper sharps disposal will help ensure the safety of the patient and the nurse.

DIF: Applying

OBJ: 34.8

TOP: Implementation

MSC: NCLEX Client Needs Category: Safety and Infection Control

NOT: Concepts: Safety

20. The nurse is caring for a patient with diabetes who will be doing fingerstick blood glucose testing at home. What is the best way for the nurse to ensure that the patient can perform the procedure correctly?
- Quiz the patient on the steps of the procedure.
 - Have the patient perform the procedure in front of the nurse.
 - Ask the patient if he has any questions about the test.
 - Use terminology that the patient can easily understand.

ANS: B

Having the patient successfully perform the procedure in front of the nurse is an excellent way for the nurse to ensure that the patient knows how to do it correctly. Quizzing the patient about the procedure, asking the patient if he/she has questions, and using understandable terminology are fine, but only a return demonstration will assess the patient's ability to perform the procedure successfully and correctly.

DIF: Applying OBJ: 34.8 TOP: Assessment

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:

Concepts: Patient Teaching

MULTIPLE RESPONSE

1. The nurse is caring for a patient who is anemic. Which CBC test results demonstrate that the patient's treatment plan is effective and the anemia is resolving? (*Select all that apply.*)
- Red blood cell count (RBC) 5.8 million/mm³
 - Hematocrit (HCT) 25%
 - Hemoglobin (HGB) 14 g/dL
 - White blood cell count (WBC) 4500/mm³
 - Platelet count (PLT) 255,000/mm³

ANS: A, C

Red blood cell count of 5.8 million and hemoglobin value of 14 g/dL are both normal. Hematocrit level of 25% is very low and indicative of ongoing anemia. White blood cell and platelet counts are not checked for anemia.

DIF: Applying OBJ: 34.2 TOP: Assessment

MSC: NCLEX Client Needs Category: Physiological Adaptation NOT:

Concepts: Perfusion

2. The nurse is caring for a patient who has been having abdominal pain. The doctor suspects that the patient may have an abdominal aortic aneurysm. Which tests would confirm the doctor's suspicion? (*Select all that apply.*)
- Magnetic resonance imaging (MRI) scan
 - Needle aspiration with biopsy
 - Fiberoptic endoscopy
 - Computed tomography (CT) scan
 - Flexible sigmoidoscopy
 - Thoracentesis

ANS: A, D

5. The nurse is caring for a patient who is taking medication that is toxic to the liver. Which laboratory test results will be reviewed by the nurse to ensure that the patient's liver is tolerating the medication without damage to the organ? (*Select all that apply.*)
- a. Alanine aminotransferase (ALT)
 - b. Alkaline phosphatase (ALP)
 - c. Blood urea nitrogen (BUN)
 - d. Anti-nuclear antibody (ANA)
 - e. Erythrocyte sedimentation rate (ESR)
 - f. Fibrin degradation products (FDP)

ANS: A, B

Alanine aminotransferase (ALT) and alkaline phosphatase (ALP) are indicators of liver function, and increased levels indicate liver damage from a variety of causes. BUN, ANA, ESR, and FDP are not indicators of liver function.

DIF: Applying OBJ: 34.2 TOP: Assessment MSC:

NCLEX Client Needs Category: Reduction of Risk Potential NOT:

Concepts: Safety

NURSINGTB.COM

Chapter 35: Medication Administration**Yoost & Crawford: Fundamentals of Nursing: Active Learning for Collaborative Practice, 3rd Edition****MULTIPLE CHOICE**

1. The nurse identifies which medication that has the highest potential for abuse?
 - a. Methylphenidate (Ritalin)—schedule II
 - b. Alprazolam (Xanax)—schedule IV
 - c. Acetaminophen & codeine (Tylenol #3)—schedule III
 - d. Diphenoxylate & atropine (Lomotil)—schedule V

ANS: A

According to the Controlled Substances Act, drugs that have the potential for abuse/dependency are classified as schedule I-V. Schedule I drugs have no approved medical applications in the United States. Schedule II drugs have high potential for abuse/dependency and have multiple restrictions for prescriptions. Schedule III, IV, and V have lower risks of dependency/abuse and fewer restrictions for prescriptions. Methylphenidate has the highest risk of abuse in this selection.

DIF: Applying OBJ: 35.1 TOP: Assessment

MSC: NCLEX Client Needs Category: Physiological Integrity: Pharmacological and Parenteral Therapies
NOT: Concepts: Addiction

2. The nurse is caring for a patient who will self-administer medication injections at home after discharge. How can the nurse best determine that the patient understands the technique and can administer the injections correctly?
 - a. Provide written instructions about how to administer the injections.
 - b. Watch the patient self-administer an injection.
 - c. Call the patient the next day to ask if there is any difficulty with administering the injections.
 - d. Ask the patient to express understanding as to how to administer the injections.

ANS: B

The nurse should watch the patient self-administer an injection to make sure that the patient is doing it correctly. This will give the nurse an opportunity to point out and correct any mistakes and offer the patient reassurance about the technique.

DIF: Remembering OBJ: 35.9 TOP: Assessment

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance

NOT: Concepts: Patient Education

3. The nurse is caring for a patient who is in agonizing pain. All the following options are listed on the patient's medication order sheet to relieve pain. The nurse knows which option that will provide the most rapid pain relief for the patient?
 - a. Morphine (MSContin) 10 mg PO
 - b. Hydromorphone (Dilaudid) 1 mg IV push
 - c. Meperidine (Demerol) 75 mg IM
 - d. Fentanyl (Duragesic) 50 mcg transdermal patch

ANS: B

IV administration has the most rapid onset of action and will provide the patient with the quickest pain relief.

DIF: Remembering OBJ: 35.4 TOP: Implementation MSC: NCLEX Client Needs Category: Physiological Integrity: Pharmacological and Parenteral Therapies
NOT: Concepts: Pain

4. The nurse administers a medication to a patient. Shortly afterward, the patient develops an itchy rash over the entire body and reports feeling very unwell. What is the priority action of the nurse?
- Leave the patient to notify the provider and the pharmacist.
 - Determine if the patient is having any difficulty breathing.
 - Document the reaction in the patient's chart.
 - Obtain an order for hydrocortisone cream to relieve the itching.

ANS: B

The nurse must first determine if the patient is having any difficulty breathing, since the patient may be starting to have an anaphylactic reaction to the medication, which can lead to shortness of breath and airway swelling. After assuring that the patient is stable, the nurse can notify the appropriate personnel and request any treatments for the reaction.

DIF: Applying OBJ: 35.6 TOP: Assessment
MSC: NCLEX Client Needs Category: Physiological Integrity: Pharmacological and Parenteral Therapies
NOT: Concepts: Clinical Judgment

5. The nurse identifies which medication order to be administered PRN?
- Zolpidem (Ambien) 10 mg PO tonight if the patient cannot sleep
 - Prednisone 10 mg PO today, then taper down 1 mg each day for the next 10 days
 - Humulin R 10 units subcutaneously before each meal and at bedtime
 - Kefzol (Ancef) 1 g IVPB 30 minutes prior to surgery

ANS: A

The nurse is to give the zolpidine (Ambien) if the patient cannot sleep. Therefore, this is the PRN (as needed) medication order. The other orders have specific time frames.

DIF: Applying OBJ: 35.9 TOP: Implementation
MSC: NCLEX Client Needs Category: Physiological Integrity: Pharmacological and Parenteral Therapies
NOT: Concepts: Caregiving

6. After administering an antibiotic to the patient, the nurse notes the patient complaining of feeling ill, is scratching and has hives. The patient soon starts having difficulty breathing and is hypotensive. What is the nurse's assessment of the situation?
- The patient is having a mild allergic reaction and an antihistamine will make the patient feel better.
 - The patient is having an anaphylactic reaction and epinephrine should be administered right away.
 - The patient's infection is worsening and progressing to septic shock so blood cultures should be drawn.
 - The patient has developed toxic shock syndrome and the antibiotic orders must be changed right away.

ANS: B

The patient's symptoms are indicative of anaphylaxis: a severe, life-threatening allergic reaction. The airways close, the throat swells closed, and the blood pressure drops dangerously low. The patient may go into shock and die. This is a medical emergency. Anaphylaxis can occur immediately after the administration of medication and can be fatal. Treatment includes immediate discontinuation of the drug and administration of epinephrine (an antagonist), intravenous (IV) fluids, steroids, and antihistamines while providing respiratory support. Patients may have very mild allergic reactions to medications and experience a rash or itching. This patient is not developing septic shock or toxic shock syndrome.

DIF: Applying

OBJ: 35.6

TOP: Assessment

MSC: NCLEX Client Needs Category: Physiological Integrity: Pharmacological and Parenteral Therapies

NOT: Concepts: Clinical Judgment

7. The nurse makes a medication error. Which action will the nurse take first?
- Prepare an incident report.
 - Explain to the patient that a medication error has occurred.
 - Assess the patient for any adverse reactions.
 - Document the medication given, the response, and corrective actions taken.

ANS: C

When a medication error occurs, the nurse's priorities are to determine the effect on the patient and intervene to offset any adverse effects of the error. Actions include immediate and ongoing assessment, notification of the prescribing health care provider, initiation of interventions as prescribed to offset any adverse effects, and documentation related to the event. Error reporting is an essential component of patient safety and should be completed as soon as the patient is assessed and stable. The nurse should follow facility guidelines for medication error reporting.

DIF: Remembering

OBJ: 35.9

TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Safety and Infection Control

NOT: Concepts: Safety

8. The nurse prepares to administer the following medication to the patient. Which instruction will the nurse be sure to give before the patient takes the medication?

MS Contin
Morphine sulfate
Extended release tablets, USP
15 mg
CII  only

- Be sure to swallow the pill whole.
- Crush the medication and place the powder in applesauce.
- Place the pill under your tongue.
- Let the pill slowly dissolve in your mouth.

ANS: A

Extended release medications must always be swallowed whole without crushing or dissolving the tablet. They are not given sublingually or allowed to dissolve in the mouth.

DIF: Remembering OBJ: 35.4 TOP: Implementation MSC:
 NCLEX Client Needs Category: Physiological Integrity: Pharmacological and Parenteral Therapies
 NOT: Concepts: Patient Education

9. The nurse begins a shift on a busy medical–surgical unit and will be caring for multiple patients. Which patient does the nurse assess first?
- A patient who would like some acetaminophen (Tylenol) for a mild headache.
 - A patient who has a question about her daily medications.
 - A patient who needs discharge teaching about an antibiotic.
 - A patient who just received nitroglycerin for chest pain.

ANS: D

The nurse's first priority is always: ABCs—Airway, Breathing, and Circulation. This includes any patients who are having chest pain and/or difficulty breathing. The nurse needs to see this patient first to determine if the chest pain has been relieved or not and to determine if the patient is now stable or if additional interventions need to be done. The other patients' needs are less critical and can be met after this patient is assessed.

DIF: Applying OBJ: 35.9 TOP: Assessment
 MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
 Concepts: Clinical Judgment

10. The nurse carefully reviews the patient's medication list. Which observation about the list indicates the highest risk for serious drug–drug interactions?
- The patient has been taking the same medications for a long time.
 - The patient is taking a large number of medications.
 - Most of the drugs on the list are prescribed in high doses.
 - The patient takes oral, injected, and inhaled medications.

ANS: B

The risk of drug–drug interactions increases when a patient takes many drugs. One of the most important ways to prevent adverse drug interactions is to minimize the number of drugs that the patient is taking. The other options do not show a high likelihood of drug–drug interactions.

DIF: Applying OBJ: 35.6 TOP: Assessment
 MSC: NCLEX Client Needs Category: Physiological Integrity: Pharmacological and Parenteral Therapies
 NOT: Concepts: Safety

11. The nurse is caring for a patient who is taking many prescription medications for various health problems. Which direction from the nurse will help the patient avoid dangerous drug interactions?
- Only take over-the-counter medications.
 - Have all of the prescriptions filled at the same pharmacy.
 - Avoid taking generic preparations of prescribed medications.
 - Only take the medications that the patient feels are necessary.

ANS: B

The patient's risk for dangerous drug interactions is increased when many medications are taken. Filling all the prescriptions at the same pharmacy will allow the pharmacist to check for possible interactions.

DIF: Applying OBJ: 35.9 TOP: Assessment
 MSC: NCLEX Client Needs Category: Physiological Integrity: Pharmacological and Parenteral Therapies
 NOT: Concepts: Safety

12. During discharge teaching, the nurse is to give the patient a signed, dated, and timed prescription from the physician for medications to be taken at home. Which prescription drug order needs to be corrected before it is given to the patient?
- Warfarin (Coumadin) 5 mg PO daily before dinner
 - Methotrexate (Trexall) 8 tablets PO once weekly on Saturdays
 - Levothyroxine (Synthroid) 137 mcg PO daily before breakfast
 - Zolpidem (Ambien) 5 mg PO at bedtime as needed for sleep

ANS: B

All prescriptions must have the name of the drug to be administered along with dosage, route, and frequency. The methotrexate order does not contain a dosage for the drug, just the number of pills to be taken. The other orders are complete.

DIF: Applying OBJ: 35.5 TOP: Implementation
 MSC: NCLEX Client Needs Category: Physiological Integrity: Pharmacological and Parenteral Therapies
 NOT: Concepts: Care Coordination

13. The nurse administers a medication to the patient. Which symptoms indicate to the nurse that the patient is having an allergic reaction rather than a side effect?
- Hair loss and sweaty skin
 - Nausea and constipation
 - Heartburn and nasty taste in the mouth
 - Itchy rash and difficulty breathing

ANS: D

Itchy rash and difficulty breathing are indicative of an allergic reaction to a medication. The other symptoms are common side effects of medications.

DIF: Applying OBJ: 35.6 TOP: Assessment
 MSC: NCLEX Client Needs Category: Physiological Integrity: Pharmacological and Parenteral Therapies
 NOT: Concepts: Clinical Judgment

14. The nurse suspects that the patient is experiencing a drug toxicity rather than a side effect. Which question will the nurse ask to help confirm this suspicion?
- When did you take your last dose of the medication?||
 - Have you been taking extra doses of the medication?||
 - Are you taking any other medications?||
 - Have you ever taken this medication in the past?||

ANS: B

Asking if the patient has been taking extra doses of the medication will allow the nurse to determine if the patient has been taking too much of the drug or more than was prescribed. Toxicity occurs when the patient receives/takes excessive amounts of the drug.

DIF: Applying OBJ: 35.6 TOP: Assessment
 MSC: NCLEX Client Needs Category: Physiological Integrity: Pharmacological and Parenteral Therapies
 NOT: Concepts: Safety

15. The nurse is caring for a patient who is receiving vancomycin (Vancocin) to treat a severe infection. The next dose is due to be administered at 10:00 a.m. What time will the nurse draw the vancomycin serum trough level?
- 7:30 a.m.
 - 9:30 a.m.
 - 11:30 a.m.
 - 1:30 p.m.

ANS: B

The trough is the lowest serum level of the medication. Serum trough levels are to be drawn just prior to the administration of the medication.

DIF: Applying OBJ: 35.5 TOP: Implementation
 MSC: NCLEX Client Needs Category: Physiological Integrity: Pharmacological and Parenteral Therapies
 NOT: Concepts: Care Coordination

16. When administering phenytoin (Dilantin) through the patient's IV line, the nurse carefully flushes the IV with normal saline before and afterward to avoid crystal formation of the medication that occurs when it mixes with dextrose in water (D₅W) solution. Which type of drug interaction is the nurse being careful to avoid?
- Antagonism
 - Potentiation
 - Synergism
 - Incompatibility

ANS: D

When medications combine to form crystals or adverse chemical reactions, the result is a drug incompatibility. Compatibility must be assessed prior to medication preparation and administration.

DIF: Remembering OBJ: 35.5 TOP: Implementation MSC:
 NCLEX Client Needs Category: Physiological Integrity: Pharmacological and Parenteral Therapies
 NOT: Concepts: Safety

17. The nurse is noting an order for a medication to be given TID. Which times will the nurse plan to administer the medication to the patient?
- 9 a.m., 1 p.m., 5 p.m., and 10 p.m.
 - 9 a.m. and 9 p.m.
 - 9 a.m., 1 p.m., and 5 p.m.
 - Nightly before the patient goes to sleep

ANS: C

TID indicates that the medication is to be administered three times daily. Common times for TID medications are 9 a.m., 1 p.m., and 5 p.m.

DIF: Applying OBJ: 35.5 TOP: Implementation
 MSC: NCLEX Client Needs Category: Physiological Integrity: Pharmacological and Parenteral Therapies
 NOT: Concepts: Care Coordination

18. The nurse is caring for a patient who was just made NPO. The nurse is to administer carvedilol (Coreg) 25 mg PO to the patient for control of high blood pressure. What is the best action of the nurse?
- Crush the medication and administer it to the patient mixed with applesauce.
 - Administer the medication to the patient with a small sip of water.
 - Contact the patient's provider to clarify the order.
 - Administer the equivalent medication dose through the patient's IV.

ANS: C

The nurse should contact the patient's provider to clarify the order. Oral medications should never be administered to NPO patients without specific orders to do so from the provider. Not all medications can be administered intravenously.

DIF: Applying

OBJ: 35.9

TOP: Implementation

MSC: NCLEX Client Needs Category: Medication Administration Physiological Integrity: Pharmacological and Parenteral Therapies

NOT: Concepts: Clinical Judgment

19. The nurse is to administer 1 mL of prochlorperazine (Compazine) 10 mg IM to an adult patient. Which syringe will the nurse select to administer the medication?
- 1 mL tuberculin syringe with 27 gauge, 1/2 inch needle
 - 3 mL syringe with 23 gauge, 1 1/2 inch needle
 - 1 mL syringe with 27 gauge, 5/8 inch needle
 - 3 mL syringe with 18 gauge, 1 inch needle

ANS: B

Intramuscular injections for adults are usually administered with a 3 mL syringe and a 1 to 3 inch, 19 to 25 gauge needle. Tuberculin syringes are typically used for subcutaneous injections. The inch needles are too short for intramuscular injections into adults. The 18 and 27 gauge needles are too small for adult intramuscular injections.

DIF: Remembering

OBJ: 35.9

TOP: Implementation MSC:

NCLEX Client Needs Category: Physiological Integrity: Pharmacological and Parenteral Therapies

NOT: Concepts: Safety

20. The nurse is to administer 15 mg of morphine liquid to the patient. How much morphine liquid will the nurse draw up to administer to the patient?

Morphine sulfate oral solution
(CONCENTRATE)
100 mg/5 mL
(20 mg/mL)
CII  only

- 0.5 mL
- 0.75 mL
- 1.3 mL
- 1.5 mL

ANS: B

$$\frac{15 \text{ mg}}{1} \times \frac{1 \text{ mL}}{20 \text{ mg}} = \frac{15}{20} = 0.75 \text{ mL}$$

DIF: Applying OBJ: 35.5 TOP: Implementation
 MSC: NCLEX Client Needs Category: Physiological Integrity: Pharmacological and Parenteral Therapies
 NOT: Concepts: Safety

21. The nurse is caring for a patient with multiple chronic illnesses who is having difficulty remembering to take multiple medications at the correct times. Which is the appropriate Nursing diagnosis for this patient?
- Activity intolerance related to inability to take medications on time
 - Impaired health maintenance related to complexity of medication schedule
 - Risk for aspiration related to need to swallow many pills during day
 - Powerlessness related to inability to figure out medication dose times

ANS: B

The patient is not able to manage the prescribed medication regimen because of the complexity of the schedule, so Impaired health maintenance is an appropriate diagnosis. Activity intolerance does not relate to the ability to take multiple medications at once and manage medication times. The patient does not state any difficulty swallowing pills, so risk for aspiration is not applicable. Inability to figure out medication dose times does not constitute powerlessness.

DIF: Applying OBJ: 35.7 TOP: Diagnosis
 MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:
 Concepts: Adherence Behavior

22. The nurse is caring for a patient who takes 6 tablets of methotrexate once every week on Fridays. How many mg of methotrexate does the patient take per dose?

NURSINGTB.COM

Trexall Methotrexate tablets, USP 2.5 mg tablets Ⓡ only
--

- 10 mg
- 15 mg
- 20 mg
- 25 mg

ANS: B

2.5 mg tablets 6 = 15 mg.

DIF: Applying OBJ: 35.5 TOP: Assessment
 MSC: NCLEX Client Needs Category: Physiological Integrity: Pharmacological and Parenteral Therapies
 NOT: Concepts: Assessment

23. The nurse is to administer 45 mg of phenobarbital to the patient. How many tablets will the patient receive?

Phenobarbital tablets, USP 15 mg CIV Ⓡ only

- a. 1 tablet
- b. 2 tablets
- c. 3 tablets
- d. 4 tablets

ANS: C

$$\frac{45 \text{ mg}}{1} \times \frac{1 \text{ tablet}}{15 \text{ mg}} = \frac{45}{15} = 3 \text{ tablets.}$$

DIF: Applying OBJ: 35.5 TOP: Implementation
 MSC: NCLEX Client Needs Category: Physiological Integrity: Pharmacological and Parenteral Therapies
 NOT: Concepts: Safety

MULTIPLE RESPONSE

1. The nurse is caring for a patient who is NPO with a new PEG (percutaneous endoscopic gastrostomy) tube. Which of the patient's medications can the nurse administer through the tube? (*Select all that apply.*)
 - a. Zolpidem tartrate (Edluar) sublingual tablet 5 mg nightly at bedtime
 - b. Ondansetron (Zofran) oral disintegrating tablet 8 mg q 8 hours PRN nausea
 - c. Cefaclor (Ceclor) for oral suspension 250 mg q 6 hours
 - d. Oxymorphone hydrochloride extended release (Opana ER) 40 mg q 12 hours
 - e. Phenytoin (Dilantin) chewable tablet 100 mg q 12 hours
 - f. Potassium chloride oral solution 20 mEq daily

ANS: C, E, F

Extended-release, oral disintegrating, and sublingual tablets may not be administered through feeding tubes. Suspensions and oral solutions are ideal for feeding tube administration. Chewable tablets may be crushed and dissolved in liquid for administration through feeding tubes.

DIF: Applying OBJ: 35.4 TOP: Implementation
 MSC: NCLEX Client Needs Category: Physiological Integrity: Pharmacological and Parenteral Therapies
 NOT: Concepts: Safety

2. The nurse identifies which medications that are to be administered via parenteral routes? (*Select all that apply.*)
 - a. Bisacodyl (Dulcolax) 10 mg suppository daily PRN constipation
 - b. Prochlorperazine (Compazine) 10 mg IM q 6 hours PRN nausea
 - c. Brimonidine (Alphagan) 0.1% solution 2 drops to each eye daily
 - d. Proventil (Ventolin) inhaler 2 puffs as needed for shortness of breath
 - e. Fentanyl (Duragesic) 50 mcg transdermal patch apply every 72 hours
 - f. Insulin lispro (Humalog) insulin 15 units subcutaneously ac meals

ANS: B, F

Parenteral medications are administered by injection into tissue, muscle, or a vein rather than through the gastrointestinal or respiratory route.

DIF: Applying OBJ: 35.5 TOP: Implementation
 MSC: NCLEX Client Needs Category: Physiological Integrity: Pharmacological and Parenteral

Therapies

NOT: Concepts: Safety

NURSINGTB.COM

Chapter 36: Pain Management**Yoost & Crawford: Fundamentals of Nursing: Active Learning for Collaborative Practice, 3rd Edition****MULTIPLE CHOICE**

1. The nurse is caring for a patient recovering from knee replacement surgery. The patient complains of severe pain in the knee after receiving hydrocodone with acetaminophen (Vicodin) 2 hours previously. What is the nurse's best action?
 - a. Administer another dose of the medication.
 - b. Apply ice packs to the knee.
 - c. Apply heat packs to the knee.
 - d. Perform gentle range of motion.

ANS: B

Application of cold decreases swelling and pain, produces local analgesia, and slows nerve conduction, which improves functioning. Examples of cold therapy are ice bags and cold compresses. The nurse should not administer another dose of medication without an order from the provider. Heat will increase blood flow to the area rather than reduce swelling.

Gentle ROM will increase pain if done at this time.

DIF: Applying OBJ: 36.8 TOP: Implementation
 MSC: NCLEX Client Needs Category: Physiological Integrity: Basic Care and Comfort NOT:
 Concepts: Pain

2. The nurse is checking on the patient after administering pain medication 30 minutes previously. Which assessment finding best indicates to the nurse that the pain medication was effective?
 - a. The patient is sleeping quietly.
 - b. The patient states a reduction of the pain.
 - c. The patient's respirations are slow and regular.
 - d. The patient's blood pressure has returned to baseline.

ANS: B

The best way for the nurse to determine that the pain medication was effective is for the patient to state a reduction of the pain. The other assessment findings cannot definitively determine whether the patient is still in pain.

DIF: Remembering OBJ: 35.5 TOP: Evaluation
 MSC: NCLEX Client Needs Category: Physiological Integrity: Pharmacological and Parenteral Therapies NOT: Concepts: Pain

3. The nurse is caring for a patient who has severe abdominal pain caused by acute cholecystitis. The nurse recognizes which type of pain is this patient experiencing?
 - a. Visceral pain
 - b. Somatic pain
 - c. Radiating pain
 - d. Referred pain

ANS: A

Visceral pain arises from the organs of the body and occurs when inflammation and tissue damage occur, such as with cholecystitis. Somatic pain occurs when there is tissue damage to skin, muscle, joints, and bones. Referred pain occurs when the discomfort is felt at a location other than the origin of the pain. Radiating pain extends to another area of the body.

DIF: Remembering OBJ: 36.1 TOP: Assessment MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT: Concepts: Pain

4. The nurse knows which is the best pain medication option for a patient to manage severe long-term cancer pain at home?
 - a. Fentanyl (Duragesic) 50 mcg transdermal patch q 72 hours
 - b. Meperidine (Demerol) 50 mg IM q 6 hours
 - c. Hydromorphone (Dilaudid) 0.2 mg q 10 minutes IV via PCA pump
 - d. Hydromorphone (Dilaudid) 0.08 mg/hour infusion through epidural catheter

ANS: A

An opioid transdermal patch is the best pain management option for home use with patients who have long-term, severe cancer pain as no injections are required and the opioid is slowly released over 72 hours. Epidurals and PCA pumps are intended for hospital use. Frequent IM injections require nursing administration, are not comfortable for the patient and are not optimal for chronic long-term pain.

DIF: Applying OBJ: 36.8 TOP: Implementation MSC: NCLEX Client Needs Category: Physiological Integrity: Pharmacological and Parenteral Therapies NOT: Concepts: Pain

5. The nurse is caring for a patient with severe chronic pain and applied the first 50 mcg transdermal fentanyl (Duragesic) patch 2 hours ago. The patient states that the pain is presently rated at 9 on a 1 to 10 scale. What is the nurse's best action?
 - a. Instruct the patient that the fentanyl patch will start to work soon.
 - b. Check the provider's orders for a short-acting narcotic medication to administer for breakthrough pain.
 - c. Give the patient a gentle back rub and encourage guided imagery.
 - d. Apply a second 25-mcg transdermal fentanyl patch now.

ANS: B

Transdermal administration of medication does not become effective for 12 to 16 hours after application. Short-acting narcotic medication should be given in the meantime to make the patient comfortable.

DIF: Remembering OBJ: 36.8 TOP: Implementation MSC: NCLEX Client Needs Category: Physiological Integrity: Pharmacological and Parenteral Therapies NOT: Concepts: Pain

6. The nurse is caring for a patient who has been taking ibuprofen (Motrin) 800 mg TID for the last several months to relieve knee pain from arthritis. Which assessment finding must be reported by the nurse to the provider promptly?
 - a. The patient has abdominal pain and pale skin.
 - b. The patient has constipation and takes stool softeners daily.
 - c. The patient enjoys a glass of wine every Friday and Saturday evening.

- d. The patient has gained 15 lb in the last 3 months.

ANS: A

A side effect of ibuprofen and other NSAIDs is the risk of gastrointestinal bleeding, especially with long-term use. Abdominal pain with pale skin in this patient may be indicative of a bleeding ulcer and should be reported to the provider promptly.

DIF: Applying OBJ: 36.5 TOP: Assessment

MSC: NCLEX Client Needs Category: Physiological Integrity: Pharmacological and Parenteral Therapies NOT: Concepts: Pain

7. The nurse is caring for a patient who just underwent laparoscopic appendectomy. The patient complains of severe postoperative pain between the shoulder blades. Which term best describes the pain that this patient is having?
- Referred pain
 - Phantom pain
 - Neuropathic pain
 - Psychogenic pain

ANS: A

Referred pain is pain that occurs when discomfort is felt in a different area than the source of the pain. Phantom pain occurs in amputees when pain is felt in the missing limb. Neuropathic pain occurs in the nervous system and often feels like burning or tingling. Psychogenic pain is discomfort felt by the patient that has no physical cause.

DIF: Remembering OBJ: 36.1 TOP: Assessment MSC:

NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:

Concepts: Pain

NURSINGTB.COM

8. The nurse administered 100 mcg sublingual fentanyl spray (Subsys) at 10:00 a.m. to a patient experiencing severe breakthrough pain. At what time will the nurse ask the patient if pain relief was obtained?
- 10:30 a.m.
 - 11:00 a.m.
 - 11:30 a.m.
 - 12:00 noon

ANS: A

Sublingual pain medications should be working well 15 to 30 minutes after administration, so the nurse should reassess the patient's pain at 10:30 a.m.

DIF: Remembering OBJ: 36.8 TOP: Evaluation

MSC: NCLEX Client Needs Category: Physiological Integrity: Pharmacological and Parenteral Therapies NOT: Concepts: Pain

9. The nurse is caring for a patient who will be using a hydromorphone (Dilaudid) PCA analgesia pump following surgery. Which intervention is the highest priority for the nurse to include in the patient's care plan related to this pump?
- Assess the patient's respiratory status frequently after PCA pump started.
 - Review patient's medication profile to check for interactions with hydromorphone.
 - Teach the patient how to use PCA pump when the pain level is still tolerable.
 - Keep naloxone (Narcan) available at the bedside in case of respiratory depression.

ANS: C

For patient safety, the nurse would check the patient's respirations frequently after the pump has been initiated due to possible respiratory depression. Reviewing the medication profile would occur prior to initiating the pump. Teaching the patient how to use the pump is important, but not the priority. Naloxone should be close by to treat respiratory depression but monitoring the respirations frequently would hopefully prevent depression.

DIF: Applying OBJ: 36.7 TOP: Implementation

MSC: NCLEX Client Needs Category: Physiological Integrity: Pharmacological and Parenteral Therapies
NOT: Concepts: Patient Education

10. The nurse is caring for a 6-month-old infant who has just undergone surgery. The infant's facial muscles are tight with a furrowed brow and the infant's respirations are shallow and irregular. The infant is mildly fussy and softly crying without muscular rigidity in the arms and legs. What score will the nurse give to the infant on the Neonatal Infant Pain Scale?
- 2
 - 3
 - 4
 - 5

ANS: C

Tight muscles and furrowed brow = 1 point. Softly crying = 1 point. Shallow, irregular respirations = 1 point. Relaxed arms and legs = 0 points. Mild fussiness = 1 point. Total = 4 points.

DIF: Applying OBJ: 36.5 TOP: Assessment

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance
NOT: Concepts: Pain

11. The nurse is caring for a patient who is recovering from thoracotomy surgery and notes that the patient's pain is rated 9/10 and is unable to focus on anything. Which intervention by the nurse is the highest priority?
- Administer prescribed IV pain medication and evaluate impact in 30 minutes.
 - Ask the patient to describe prior pain experiences and methods used to manage pain.
 - Explain that comfort is a priority goal of nursing care in the postoperative period.
 - Assist the patient to minimize the effects of pain on interpersonal relationships with family members.

ANS: A

The highest priority intervention for a patient in acute pain is to provide pain relief. The other interventions do not address acute pain relief.

DIF: Applying OBJ: 36.8 TOP: Implementation

MSC: NCLEX Client Needs Category: Physiological Integrity: Reduction of Risk Potential
NOT: Concepts: Pain

12. The nurse is caring for a patient with rheumatoid arthritis who is in constant severe pain. Which nursing diagnosis is the highest priority for this patient?
- Impaired mobility r/t patient's need to use a cane or walker with ambulation
 - Impaired health maintenance r/t sedentary lifestyle and poor physical condition

- c. Anxiety r/t mistrust of health care personnel
- d. Chronic pain r/t ongoing inflammatory tissue damage and joint destruction

ANS: D

Chronic pain is the highest priority diagnosis for this patient because it is severe. The other diagnoses may be addressed once the patient's pain is controlled.

DIF: Applying OBJ: 36.6 TOP: Diagnosis
 MSC: NCLEX Client Needs Category: Physiological Integrity: Pharmacological and Parenteral Therapies
 NOT: Concepts: Pain

13. The nurse is caring for a cancer patient with ongoing pain from widespread metastasis to the bones. The nurse notes that the patient's morphine dosage had to be increased to sufficiently manage the discomfort. What is the nurse's interpretation of this assessment finding?
- a. The patient became tolerant to the previous morphine dosage.
 - b. The patient is becoming addicted to the pain medication.
 - c. The patient has been abusing the prescribed pain medications.
 - d. The patient may be seeking to end life with an overdose of morphine.

ANS: A

Drug tolerance is an adaptation to the medication, which eventually leads to less effective pain relief. The patient is requiring higher doses of narcotic pain medication because of this tolerance. This is common when patients require long-term pain medication. Since the patient is taking morphine to control ongoing pain, the patient is not addicted to it. Need for increased morphine dosage is not indicative of drug abuse or a wish to die.

DIF: Applying OBJ: 36.5 TOP: Assessment
 MSC: NCLEX Client Needs Category: Physiological Integrity: Pharmacological and Parenteral Therapies
 NOT: Concepts: Pain

14. The nurse identifies which patient to be best suited for PCA analgesia?
- a. A patient who is confused after a head injury
 - b. A patient recovering from total hysterectomy surgery
 - c. A patient who has severe psychogenic pain
 - d. A patient with arthritis who is unable to push the nurse call button

ANS: B

Patients recuperating from surgery are often good candidates for PCA analgesia. Confusion, inability to push the PCA button, and psychogenic pain are all contraindications for PCA analgesia.

DIF: Applying OBJ: 36.8 TOP: Assessment
 MSC: NCLEX Client Needs Category: Physiological Integrity: Pharmacological and Parenteral Therapies
 NOT: Concepts: Pain

15. What is the priority nursing assessment for a patient who is receiving postoperative epidural analgesia with hydromorphone (Dilaudid)?
- a. Respiratory rate, depth, and pattern
 - b. Skin underneath the epidural dressing
 - c. Bladder scanning to check for urinary retention
 - d. Itching on the trunk and/or extremities

ANS: A

The respiratory system is the priority nursing assessment for patients receiving narcotic pain medication via any route. This is because narcotics can cause respiratory suppression. The other assessments are a lower priority and may be done after a respiratory assessment is completed.

DIF: Remembering

OBJ: 36.5

TOP: Assessment

MSC: NCLEX Client Needs Category: Physiological Integrity: Pharmacological and Parenteral Therapies
NOT: Concepts: Pain

16. The nurse is caring for a diabetic patient who has painful foot neuropathy. The patient asks why the nurse is administering gabapentin (Neurontin) when there is no history of seizure disorder. What is the nurse's best response?
- Gabapentin will help you sleep at night so you can deal with the pain more effectively.¶
 - Long-term diabetes can put patients at risk for certain type of seizures.¶
 - This medication can help relieve your anxiety from being admitted to the hospital.¶
 - Gabapentin works on the nervous system to help relieve the burning pain in your feet.¶

ANS: D

Anticonvulsant medication like gabapentin and tricyclic antidepressants are often used to relieve neuropathic pain as they work directly on the nervous system. The other statements do not correctly indicate why the patient is receiving this medication.

DIF: Remembering

OBJ: 36.8

TOP: Patient Teaching-Learning

MSC: NCLEX Client Needs Category: Physiological Integrity: Pharmacological and Parenteral Therapies
NOT: Concepts: Pain

17. The nurse is caring for a patient who has a PCA pump following total hysterectomy surgery. The nurse sees the visitor push the PCA button while the patient is sleeping quietly. What is the best response of the nurse?
- Thank you for pushing the button for her to help keep her comfortable after surgery.¶
 - Please do not push the button for the patient—she could receive more medication than she needs.¶
 - You can push the button for her now, but please have her do it herself when she awakens.¶
 - PCA pumps are great because she doesn't have to wait for me to administer her pain medication.¶

ANS: B

Only the patient should operate the PCA and push the administration button. Family members and visitors should never activate the PCA pump for the patient because too much medication could be delivered, resulting in overdose and respiratory suppression.

DIF: Applying

OBJ: 36.8

TOP: Patient Teaching-Learning

MSC: NCLEX Client Needs Category: Physiological Integrity: Pharmacological and Parenteral Therapies
NOT: Concepts: Pain

18. Which assessment question helps the nurse determine the character of the patient's pain?
- What does the pain feel like?||
 - When did the pain first start?||
 - What interventions make the pain better?||
 - Is there any pattern to when the pain occurs?||

ANS: A

Pain character should be assessed using questions to learn more about what the pain feels like. Examples like stabbing, aching, burning may be used so that patients can understand what the nurse is requesting. Onset is determined by asking when the pain started.

Exacerbating/relieving factors are determined by asking which interventions make the pain better. Time course is determined by asking if there is a pattern to when the pain occurs.

DIF: Remembering

OBJ: 36.5

TOP: Assessment

MSC: NCLEX Client Needs Category: Physiological Integrity: Pharmacological and Parenteral Therapies

NOT: Concepts: Pain

19. The nurse is caring for a patient who only speaks a foreign language. What is the best method for the nurse to assess the patient's pain level?
- Perform a pain assessment using a translator.
 - Check the patient's vital signs and pulse oximetry.
 - Check the patient's respiratory rate, depth, and rhythm.
 - Look to see if the patient appears to be resting comfortably.

ANS: A

The best method to determine pain in a patient who speaks only a foreign language is to use an interpreter. The Universal Pain Assessment Tool is available with foreign-language phrases that may also be used. Measuring vital signs and seeing if the patient is resting comfortably are not accurate pain assessment techniques.

DIF: Applying

OBJ: 36.5

TOP: Assessment

MSC: NCLEX Client Needs Category: Physiological Integrity: Pharmacological and Parenteral Therapies

NOT: Concepts: Communication

20. The nurse is caring for a trauma patient with the Nursing diagnosis of acute pain r/t fracture and muscle spasms. Which is an appropriate goal for this Nursing diagnosis?
- The patient will experience less pain when participating in physical therapy.
 - The patient will describe meditation techniques that can be used to cope with pain.
 - Nursing staff will explain the ordered pain management approach to the patient.
 - The patient will feel less pain each day when range-of-motion therapy is performed.

ANS: B

Goals must be measurable and objective so that nursing staff can determine when each of the goals has been met. Having the patient describe meditation techniques is measurable because the nursing staff can determine whether he can actually describe them. Goals are achieved by the patient, not nursing staff. The nursing staff cannot accurately measure whether the patient is experiencing or feeling less pain. The goal statements —The patient will report less pain ... or state that he has less pain ...|| are not measurable and appropriate.

DIF: Applying

OBJ: 36.7

TOP: Planning

MSC: NCLEX Client Needs Category: Physiological Integrity: Pharmacological and Parenteral

Therapies

NOT: Concepts: Pain

MULTIPLE RESPONSE

1. The nurse is caring for a patient who has pain following abdominal surgery. Which actions are independent nursing interventions that can be used to make the patient more comfortable? (*Select all that apply.*)
 - a. Encourage the patient to relax and imagine resting on a tropical beach.
 - b. Provide headphones so that the patient can listen to favorite music.
 - c. Increase pain medication dosage if prescribed regimen is ineffective to manage pain.
 - d. Teach the patient to take pain medication before discomfort becomes severe.
 - e. Switch the patient from IV to oral pain medication when bowel sounds return.
 - f. Demonstrate the use of relaxation breathing before painful procedures.

ANS: A, B, D, F

Independent nursing interventions may be carried out without an order from the provider. Changing medication orders must be done by the provider; increasing pain medication dosage and switching the patient to PO pain medications are not independent nursing interventions.

DIF: Applying

OBJ: 36.8

TOP: Implementation

MSC: NCLEX Client Needs Category: Physiological Integrity: Basic Care and Comfort NOT:

Concepts: Pain

2. The nurse is caring for a patient who has severe burning pain in the right arm caused by a compressed nerve in the neck. Which medications can be used along with a narcotic pain reliever to relieve the patient's pain until surgery can be performed to release the nerve? (*Select all that apply.*)
 - a. Diphenhydramine (Benadryl) 50 mg PO daily
 - b. Amitriptyline (Elavil) 50 mg PO BID
 - c. Ondansetron (Zofran) 8 mg PO q 4 hours PRN
 - d. Gabapentin (Neurontin) 400 mg PO BID
 - e. Senna (Senokot) 8.6 mg PO daily
 - f. Naloxone (Narcan) 0.4 mg IV now, may repeat in 1 hour PRN

ANS: B, D

Tricyclic antidepressants like amitriptyline and anticonvulsants like gabapentin are often used to treat neuropathic pain because they work directly on the nervous system. They may be given along with narcotic pain medication to make the patient comfortable. Senna will relieve constipation and diphenhydramine will relieve itching. Ondansetron is used to relieve nausea and vomiting, whereas naloxone will reverse narcotic-induced respiratory suppression.

DIF: Remembering

OBJ: 36.8

TOP: Implementation MSC:

NCLEX Client Needs Category: Physiological Integrity: Pharmacological and Parenteral Therapies

NOT: Concepts: Pain

3. The nurse is caring for a patient who just had knee replacement surgery. Which factors will affect how the patient experiences pain after this surgery? (*Select all that apply.*)
 - a. The patient has had rheumatoid arthritis for the last 16 years.
 - b. The patient is allergic to aspirin and strawberries.
 - c. The patient owns a business and is self-insured.

- d. The patient has been a vegetarian for the last 8 years.
- e. The patient had the other knee replaced 2 years ago.
- f. The patient was a marathon runner in high school and college.

ANS: A, E, F

The patient's history of rheumatoid arthritis, previous knee replacement surgery, and marathon running indicate that the patient has had significant experience dealing with pain, which will affect how he or she experiences pain after this surgery. The other factors will not affect how the patient experiences pain.

DIF: Understanding

OBJ: 36.4

TOP: Assessment MSC:

NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:

Concepts: Pain

NURSINGTB.COM

Chapter 37: Perioperative Nursing Care**Yoost & Crawford: Fundamentals of Nursing: Active Learning for Collaborative Practice, 3rd Edition****MULTIPLE CHOICE**

1. The nurse is caring for a patient who is about to have surgery. Which intervention will be included in the patient's care to meet the goals for risk for perioperative positioning injury related to immobilization during surgical procedure?
 - a. Use adequate assistance to move patient onto the OR table.
 - b. Watch for early signs of hypovolemia caused by patient's NPO status since midnight.
 - c. Use therapeutic touch and guided imagery to allay patient's fears of surgery.
 - d. Pad all bony prominences and avoid hyperextension of extremities.

ANS: D

Risk for perioperative positioning injury is addressed by ensuring that the patient's skin and bony prominences are well padded during the surgery. In addition, hyperextension of extremities may lead to joint damage, so this should be avoided as well. The other interventions are appropriate for perioperative care but do not relate directly to the Nursing diagnosis of positioning injury potential.

DIF: Applying OBJ: 37.7 TOP: Planning

MSC: NCLEX Client Needs Category: Reduction of Risk Potential: Potential for Complications from Surgical Procedures NOT: Concepts: Safety

2. The nurse is caring for a male patient who is having open heart surgery. The patient's chest is covered with thick hair, so the surgical technician begins to shave the patient's skin near the operative site. Which action by the technician requires intervention by the nurse to correct the technique?
 - a. A straight safety razor and antibiotic foam is used.
 - b. Disposable electric trimmers are used to trim the hair.
 - c. Antibacterial soap is used prior to hair removal.
 - d. Only the hair directly around the surgical site is removed.

ANS: A

Disposable electric trimmers should be used to remove excess hair from operative sites. Antibacterial soap is commonly used to clean the skin before surgical procedures. Only the hair around the surgical site is removed. A straight safety razor would never be used because small nicks in the skin can occur, increasing infection risk.

DIF: Remembering OBJ: 37.3 TOP: Implementation

MSC: NCLEX Client Needs Category: Safety and Infection Control: Safe Use of Equipment NOT: Concepts: Infection

3. The nurse is caring for a patient who has just been brought to the postoperative unit following major surgery and notes that the patient has many tubes and monitors in place. Which will the nurse assess first?
 - a. The patient's intravenous lines
 - b. The patient's urinary catheter

- c. The patient's nasogastric tube
- d. The patient's endotracheal tube

ANS: D

Airway maintenance and protection is the highest priority for this patient, so the nurse should assess the endotracheal tube first to ensure that it is patent and positioned correctly. The other tubes may be assessed afterward.

DIF: Applying OBJ: 37.5 TOP: Assessment
 MSC: NCLEX Client Needs Category: Management of Care: Establishing Priorities NOT:
 Concepts: Clinical Judgment

4. The nurse is caring for a patient who has a family history of reactions to general anesthesia. Which medication will the nurse anesthetist have ready as a precautionary measure before the patient's surgery is started?
- a. Protamine sulfate
 - b. Dantrolene sodium (Dantrium)
 - c. Activated charcoal with sorbitol
 - d. Folinic acid (Leucovorin)

ANS: B

Malignant hyperthermia is a dangerous anesthesia reaction caused by a genetic defect that may be passed down via family history. Knowing this, the anesthesiologist would have dantrolene ready as a precaution because it is a mainstay of treatment for malignant hyperthermia. The other medications are not related.

DIF: Applying OBJ: 37.4 TOP: Implementation
 MSC: NCLEX Client Needs Category: Reduction of Risk Potential: Potential for Complications from Surgical Procedures NOT: Concepts: Clinical Judgment

5. Which action by the nurse best demonstrates accountability in the operating room?
- a. Applying warm blankets when the patient reports feeling chilly
 - b. Holding the patient's hand to allay anxiety before anesthesia is administered
 - c. Double-checking that the surgical site is clearly marked and visible after draping
 - d. Using calming speech with a reassuring tone of voice when speaking with the patient

ANS: C

Accountability is accomplished by ensuring that proper precautions are taken to prevent errors from happening. The nurse can prevent wrong-site surgery by making sure that the surgical site is clearly marked and visible after the draping is completed. The surgeon could inadvertently operate on the incorrect site if the markings are covered by the surgical drapes. The other actions of the nurse are appropriate but do not demonstrate accountability and error prevention.

DIF: Applying OBJ: 37.3 TOP: Implementation
 MSC: NCLEX Client Needs Category: Safety and Infection Control: Error Prevention NOT:
 Concepts: Safety

6. The nurse is caring for a patient who will be having surgery. The patient has just signed the consent form for the operation. What does the patient's signature indicate?
- a. The patient agrees with the doctor's diagnosis.

- b. The patient gives permission for the surgery to be performed.
- c. The patient has agreed to pay for any costs not covered by insurance.
- d. The patient has been told of all the available treatment options.

ANS: B

The patient's signature on the consent form indicates that the patient gives permission for the surgery to be performed. It does not indicate that the patient agrees with the physician's diagnosis, agrees to pay for costs not covered by insurance, or has been informed of all the possible treatment options.

DIF: Remembering

OBJ: 37.3

TOP: Communication and Documentation

MSC: NCLEX Client Needs Category: Management of Care: Informed Consent NOT:

Concepts: Health Care Law

7. The nurse is caring for a postoperative patient who is very sleepy following general anesthesia and administration of pain medication. The nurse notes that the patient is making snoring sounds and his pulse oximetry has dropped to 88%. What is the best action of the nurse?
- a. Assess the airway and administer oxygen.
 - b. Call for anesthesia to immediately reintubate the patient.
 - c. Remove the pillow from behind the patient's head.
 - d. Elevate the head of the patient's bed.

ANS: A

The snoring sounds made by the patient and low oximetry levels indicate that the patient's airway is partially occluded because of anesthesia and pain medication. Patency of the airway should be assessed and oxygen administered to maintain the airway and oxygenation.

Reintubation should be performed if the patient does not maintain the airway and improve oxygenation. Removing the pillow from the head of the patient's bed or elevating the head of the patient's bed will not be sufficient to open the patient's airway.

DIF: Applying

OBJ: 37.8

TOP: Implementation

MSC: NCLEX Client Needs Category: Reduction of Risk Potential: Changes & Abnormalities in Vital

Signs

NOT: Concepts: Gas Exchange

8. The nurse is caring for a postoperative patient on his first day after surgery. The nurse informs the patient that the plan is to sit in the chair and ambulate in the hallway. The patient states that he is in pain and has no intention of getting out of bed. What is the nurse's best response?
- a. —It's important to move around so you don't get a blood clot in your leg.||
 - b. —Your doctor ordered that you are to get out of bed at least twice every day.||
 - c. —I understand. You can rest in bed until tomorrow when the pain is better.||
 - d. —I will call the doctor and let him know that you do not want to get up.||

ANS: A

The nurse should teach the patient why it is important to ambulate after surgery. Early ambulation helps to prevent many complications postoperatively, including constipation, deep vein thrombosis, atelectasis, pneumonia, and urinary stasis. Many patients who experience abdominal distention and gas pain obtain some relief from ambulating. Simply telling the patient that the physician ordered ambulation is not sufficient. Allowing the patient to stay in bed will increase the risk of DVT.

DIF: Understanding

OBJ: 37.8

TOP: Teaching/Learning

MSC: NCLEX Client Needs Category: Reduction of Risk Potential: Potential for Complications from Surgical Procedures NOT: Concepts: Patient Education

9. The nurse is walking a postoperative patient in the hallway when she notices a large red stain of fresh blood on the patient's gown over the abdominal incision. The patient states, —I felt something just ripped open. What is the priority action of the nurse?
- Lift up the patient's gown and assess the incision.
 - Assist the patient to the floor and call for assistance.
 - Return the patient to bed and irrigate the wound with sterile saline.
 - Check the patient's vital signs and pulse oximetry.

ANS: B

The large red blood stain over the incision and feeling of ripping open most likely indicates that the patient's wound has dehisced or eviscerated. The nurse should immediately lower the patient to the floor to reduce tension on the wound. Patient modesty and privacy should be maintained, so the incision should be assessed once the patient is transported back to his room. Checking the patient's vital signs and pulse oximetry can be performed once the patient has been lowered to the floor.

DIF: Applying OBJ: 37.4 TOP: Implementation

MSC: NCLEX Client Needs Category: Physiological Adaptation: Medical Emergencies NOT: Concepts: Safety

10. The nurse is caring for a patient with advanced colon cancer. The patient is to have surgery to relieve a bowel obstruction that has been causing unrelenting vomiting and abdominal pain. What type of surgery will this patient undergo?
- Palliative
 - Reconstructive
 - Diagnostic
 - Ablative

NURSINGTB.COM

ANS: A

Palliative surgery is performed to alleviate distressing symptoms such as unrelenting pain, nausea, and vomiting. Palliative surgery will not attempt to cure the underlying disease process but will make the patient more comfortable. Reconstructive surgery restores function or appearance of traumatized tissue. A diagnostic surgical procedure establishes or confirms a diagnosis. Ablative surgery removes diseased tissue.

DIF: Applying OBJ: 37.2 TOP: Planning

MSC: NCLEX Client Needs Category: Management of Care: Establishing Priorities NOT: Concepts: Palliation

11. After general anesthesia is administered, the patient is carefully placed in the prone position. What is the primary consideration of the nursing staff as the patient is positioned?
- Making sure that the patient's endotracheal tube does not become kinked
 - Ensuring that the patient's head is positioned to prevent cervical nerve injury
 - Carefully taping the patient's eyes shut to avoid corneal abrasions
 - Padding the operating table carefully and keeping linens free of wrinkles

ANS: A

Ensuring that the patient's respiratory status is not compromised is the priority of all staff during repositioning after general anesthesia has been administered. Positioning and protection of eyes is important but less than protection of the endotracheal tube patency.

DIF: Applying OBJ: 37.3 TOP: Implementation

MSC: NCLEX Client Needs Category: Reduction of Risk Potential: Potential for Complications from Surgical Procedures NOT: Concepts: Gas Exchange

12. The nurse is caring for a preoperative patient who has just received sedation prior to general anesthesia in the OR. What is the priority action of the nurse?
- Check to make sure that the consent form was signed.
 - Turn off the lights and provide a quiet environment.
 - Raise the side rails on the patient's stretcher.
 - Indicate the surgical site with an indelible marker.

ANS: C

Safety of the preoperative patient is a priority after sedation has been administered. Raising the side rails of the stretcher will help prevent the patient from falling. Turning off the lights is nice but is not a priority. Marking the surgical site and signing the consent form must be performed prior to administration of sedation.

DIF: Applying OBJ: 37.3 TOP: Implementation

MSC: NCLEX Client Needs Category: Safety and Infection Control: Injury Prevention NOT: Concepts: Safety

13. The nurse is caring for a postoperative patient who is recovering from abdominal surgery. The nurse notes that the patient's breath sounds are clear but diminished, shallow, and slightly labored. The patient's pulse oximetry is 96% on room air. What is the priority action of the nurse?
- Administer a dose of the prescribed pain medication.
 - Administer 2 L of oxygen via nasal cannula.
 - Obtain an order from the physician for a chest x-ray.
 - Ensure that the patient is using the spirometer 10 times every hour.

ANS: D

The assessment findings indicate that the patient most likely has atelectasis, so the nurse should ensure that the patient is using the incentive spirometer 10 times every hour to facilitate expansion and reinflation of alveoli. Administering prescribed pain medication is not the priority and may further suppress the patient's respiratory drive. The patient's pulse oximetry is 96% on room air so supplemental oxygen is not needed. A chest x-ray may be ordered if the patient's condition worsens or does not improve with regular spirometer use.

DIF: Applying OBJ: 37.5 TOP: Implementation

MSC: NCLEX Client Needs Category: Physiological Adaptation: Alterations in Body Systems NOT: Concepts: Gas Exchange

14. The nurse will be caring for a patient who has just arrived on the medical-surgical unit following surgical repair of his fractured right ankle. Which is the priority action of the nurse when the patient arrives on the unit?
- Instruct the patient how to call for assistance using the call light.
 - Assess the color and warmth of the toes on the patient's right foot.

- c. Determine when the patient's next pain medication is due.
- d. Check pulse oximetry and obtain a full set of vital signs.

ANS: D

Checking pulse oximetry and vital signs is the priority action when the patient first arrives on the medical-surgical unit from the postoperative area. The other actions can wait until the vital signs have been obtained.

DIF: Applying OBJ: 37.5 TOP: Implementation
 MSC: NCLEX Client Needs Category: Management of Care: Establishing Priorities NOT:
 Concepts: Clinical Judgment

15. The nurse is caring for a patient who is recovering from chest surgery. Which action by the patient indicates that additional teaching is needed about how to use the ordered incentive spirometer correctly?
- a. The patient breathes into the spirometer so that the marker rises slowly.
 - b. The patient uses the spirometer 5 to 12 times every 1 to 2 hours while awake.
 - c. The patient seals his lips tightly around the spirometer mouthpiece.
 - d. The patient should hold each inhaled breath 3 to 5 seconds.

ANS: A

The patient must take in a deep breath while holding the spirometer to the mouth so that the device can indicate how much air is being inhaled into the lungs. The remaining responses are correct components of the procedure.

DIF: Remembering OBJ: 37.8 TOP: Teaching/Learning
 MSC: NCLEX Client Needs Category: Reduction of Risk Potential: Therapeutic Procedures NOT:
 Concepts: Patient Education

16. The nurse is caring for a patient who is headed to the operating room for abdominal surgery. Which goal is appropriate for the Nursing diagnosis risk for perioperative positioning injury?
- a. Patient will deny numbness or tingling in extremities after surgical procedure.
 - b. Patient will maintain urine output of at least 30 mL/hr during and after surgery.
 - c. Patient will maintain elastic skin turgor as well as moist tongue and mucus membranes.
 - d. Patient will have no emesis and deny nausea following arousal from general anesthesia.

ANS: A

Numbness and tingling of extremities may be indicative of injury following improper positioning for surgery. The other goals do not relate to positioning injury but relate to Nursing diagnoses of fluid balance and nausea.

DIF: Applying OBJ: 37.7 TOP: Planning
 MSC: NCLEX Client Needs Category: Safety and Infection Control: Injury Prevention NOT:
 Concepts: Safety

17. The nurse is caring for a patient who requires emergency surgery for injuries sustained in a motor vehicle accident. The patient was on his way back to work after having lunch with colleagues when the accident happened. What is the highest priority Nursing diagnosis for this patient?
- a. Risk for imbalanced body temperature

- b. Risk for aspiration
- c. Risk for perioperative positioning injury
- d. Risk for delayed surgical recovery

ANS: B

Risk for aspiration is the highest priority because the patient has not been NPO and his stomach is filled with food after lunching with his colleagues. The patient may easily aspirate stomach contents into the airway when general anesthesia is administered, so precautions must be taken to prevent this from happening. The other Nursing diagnoses certainly apply but are not as important as risk for aspiration.

DIF: Applying OBJ: 37.6 TOP: Diagnosis
 MSC: NCLEX Client Needs Category: Management of Care: Establishing Priorities NOT:
 Concepts: Clinical Judgment

18. The nurse is caring for a postoperative patient who has a history of COPD. What is the priority Nursing diagnosis for this patient?
- a. Ineffective airway clearance
 - b. Readiness for enhanced knowledge
 - c. Risk for delayed surgical recovery
 - d. Activity intolerance

ANS: A

Ineffective airway clearance is the priority diagnosis for the COPD patient undergoing surgery because the patient is at high risk for bronchoconstriction, increased mucus, and ineffective cough, which may easily become worse after the patient has received general anesthesia. The other Nursing diagnoses are applicable to the patient but are not as high priority.

NURSINGTB.COM

DIF: Applying OBJ: 37.6 TOP: Diagnosis
 MSC: NCLEX Client Needs Category: Management of Care: Establishing Priorities NOT:
 Concepts: Gas Exchange

19. The nurse is assigned to care for several patients on the surgical unit. Which patient need will the nurse address first?
- a. A patient who is waiting for discharge teaching before going home.
 - b. A patient who needs to be ambulated for the first time postoperatively.
 - c. A patient who has not voided since the catheter was removed 8 hours ago.
 - d. A patient who requires a daily dressing change to the surgical incision.

ANS: C

Urinary retention is common after removal of urinary catheters and must be addressed promptly. The nurse should address the patient with urinary retention first because it is the highest priority. The other patients may wait until later.

DIF: Applying OBJ: 37.3 TOP: Planning
 MSC: NCLEX Client Needs Category: Management of Care: Establishing Priorities NOT:
 Concepts: Clinical Judgment

MULTIPLE RESPONSE

1. The nurse is obtaining preoperative information for a patient who will be having emergency surgery shortly for a ruptured appendix. Which information is crucial for the nurse to assess? (*Select all that apply.*)
 - a. All medications that the patient is taking
 - b. Use of tobacco, alcohol, or recreational drugs
 - c. Allergies to medications, foods, or other substances
 - d. Date of last tetanus shot and flu vaccination
 - e. Insurance coverage and preauthorization requirements
 - f. Possibility of pregnancy

ANS: A, B, C, F

Priority assessment must be completed prior to emergency surgery, including use of medications, alcohol, tobacco, or recreational drugs because these may interact with anesthesia medications. Allergies must be identified to prevent reactions in the operating room. Special precautions may be taken if the patient is pregnant, so this must also be determined preoperatively. Asking the patient about vaccinations or insurance coverage is not a priority prior to surgery.

DIF: Applying OBJ: 37.5 TOP: Assessment

MSC: NCLEX Client Needs Category: Reduction of Risk Potential: Potential for Complications from Surgical Procedures NOT: Concepts: Clinical Judgment

2. The nurse is working with a nursing assistant to care for several postoperative patients. Which interventions can the nurse delegate to the assistant for completion? (*Select all that apply.*)
 - a. Assess patients' comfort levels and need for pain medication.
 - b. Empty urinary catheter bags and record urine output.
 - c. Teach patients how to use incentive spirometers hourly.
 - d. Provide ice chips and juice to patients who are no longer NPO.
 - e. Monitor incisions for signs of infection.
 - f. Apply TED hose and assist with oral care.

ANS: B, D, F

Basic patient care tasks that do not require assessment or critical thinking may be assigned to the nursing assistant for completion. These include emptying drainage bags, providing ice chips to patients who are allowed oral intake, and applying TED hose. Teaching, monitoring, and assessing patients are done by the nurse.

DIF: Understanding OBJ: 37.8 TOP: Implementation

MSC: NCLEX Client Needs Category: Management of Care: Delegation

NOT: Concepts: Collaboration

3. Which patients would benefit from preoperative teaching about splinting of incisions to minimize discomfort? (*Select all that apply.*)
 - a. Patient having coronary bypass graft surgery
 - b. Patient having open breast biopsy
 - c. Patient having total hip replacement surgery
 - d. Patient having lumbar spine decompression surgery
 - e. Patient having surgery to repair retinal detachment
 - f. Patient having total abdominal hysterectomy

ANS: A, F

Postoperative splinting is done by supporting the abdominal and chest muscles to minimize the pain of coughing and deep breathing after surgery. Patients who have just had heart or abdominal surgery will benefit from splinting. The other surgical procedures do not affect the chest or abdomen, and these patients would not benefit from teaching about splinting.

DIF: Understanding OBJ: 37.2 TOP: Teaching/Learning MSC:
NCLEX Client Needs Category: Basic Care and Comfort: Non-Pharmacological Comfort Interventions
NOT: Concepts: Patient Education

4. The nurse is caring for a patient who is recovering from bowel resection surgery. Which assessment findings indicate to the nurse that the patient no longer needs to remain NPO and may progress to oral intake of food and fluids? (*Select all that apply.*)
- The patient passed flatus while ambulating this morning.
 - The patient's abdomen is soft with active bowel sounds 4 quadrants.
 - The patient denies nausea or vomiting and states that he feels hungry.
 - The patient's abdominal incision is clean, dry, and intact with staples.
 - The patient ambulated in the hallway with a slow, steady gait.
 - The patient's urinary catheter is patent with clear, yellow urine.

ANS: A, B, C

The patient may indicate readiness for oral intake when passing flatus and relating feelings of hunger. The absence of nausea and vomiting along with active bowel sounds in a soft abdomen also indicate that the patient's GI tract is ready for oral feedings.

DIF: Applying OBJ: 37.5 TOP: Assessment
MSC: NCLEX Client Needs Category: Physiological Adaptation: Alterations in Body Systems NOT:
Concepts: Clinical Judgment

NURSINGTB.COM

5. The nurse is caring for a patient who underwent abdominal surgery the previous day. Which assessment findings indicate to the nurse that the patient may be experiencing serious internal bleeding? (*Select all that apply.*)
- The patient's urinary output increased to 40 mL/hr.
 - The patient's pulse has risen from 76 to 112 beats/min.
 - The patient states that his abdominal pain is worse than yesterday.
 - The patient complains of generalized itching.
 - The patient's hematocrit dropped from 14.6 to 11.0 g/dL.
 - The patient has not been able to have a bowel movement since before surgery.

ANS: B, C, E

Signs of internal bleeding include tachycardia, increased abdominal pain and a drop in hematocrit/hemoglobin. Urinary output would decrease with internal bleeding because the kidneys work to conserve fluids. Itching and constipation are not signs of internal bleeding.

DIF: Applying OBJ: 37.5 TOP: Assessment
MSC: NCLEX Client Needs Category: Physiological Adaptation: Fluid and Electrolyte Imbalances NOT:
Concepts: Fluid and Electrolyte Balance

Chapter 38: Oxygenation and Tissue Perfusion**Yoost & Crawford: Fundamentals of Nursing: Active Learning for Collaborative Practice, 3rd Edition****MULTIPLE CHOICE**

1. The nurse finds the patient in cardiopulmonary arrest with no pulse or respirations. Which oxygen delivery device will the nurse use for this patient?
 - a. Non-rebreather mask
 - b. Bag-valve-mask unit
 - c. Continuous positive airway pressure (CPAP)
 - d. High-flow nasal cannula

ANS: B

The priority of the nurse is to ventilate the patient manually using a bag-valve-mask unit (also called by the proprietary name Ambu bag). This allows air to be forced into the patient's lungs when there are no spontaneous respirations. The non-rebreather mask and nasal cannula require the patient to breathe on his or her own. CPAP is used for patients who are awake, oriented, and in respiratory failure.

DIF: Understanding

OBJ: 38.6

TOP: Implementation

MSC: NCLEX Client Needs Category: Physiological Adaptation: Physiological Adaptation NOT:

Concepts: Gas Exchange

2. The nurse is caring for a patient who is slow to awaken following general anesthesia. The patient is breathing spontaneously but is minimally responsive and having difficulty maintaining a patent airway. Which intervention is the most appropriate for the patient to improve oxygenation?
 - a. Insert an oral airway.
 - b. Lower the head of the bed.
 - c. Turn the patient's head to the side.
 - d. Monitor the patient's pulse oximetry.

ANS: A

An oral airway will prevent the patient's tongue from falling back and occluding the airway. Lowering the head of the bed will only increase airway occlusion and risk of aspiration.

Turning the patient's head to the side will not clear the back of the patient's tongue from the airway. Monitoring the patient's pulse oximetry will not improve oxygenation or clear the airway.

DIF: Applying

OBJ: 38.6

TOP: Implementation

MSC: NCLEX Client Needs Category: Physiological Integrity: Reduction of Risk Potential NOT:

Concepts: Gas Exchange

3. The nurse is caring for a patient with a history of left-sided congestive heart failure who is acutely short of breath. The nurse hears fine crackles throughout both lung fields and notes that the patient's pulse oximetry is only 88% on 4 L of oxygen. What is the priority intervention of the nurse?
 - a. Administer the ordered intravenous diuretic.
 - b. Prepare for insertion of a chest tube.

- c. Suction secretions from the patient's respiratory tract.
- d. Have the patient use the ordered incentive spirometer.

ANS: A

The patient's respiratory distress is due to pulmonary edema and fluid overload from left-sided congestive heart failure. A diuretic will pull the excess fluid out of the body through the urine and relieve the patient's distress. A chest tube is not needed as the fluid is within the alveoli rather than between the lung and chest wall. Suctioning and use of an incentive spirometer will not address fluid overload or improve the patient's symptoms.

DIF: Understanding

OBJ: 38.6

TOP: Implementation

MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:

Concepts: Fluid and Electrolyte Balance

4. The nurse is caring for a patient who has been intubated with an oral endotracheal tube for several weeks. The physicians predict that the patient will need to remain on a ventilator for at least several more weeks before he will be able to maintain his airway and breathe on his own. What procedure does the nurse anticipate will be planned for the patient to facilitate recovery?
- a. Placement of a tracheostomy tube
 - b. Diagnostic thoracentesis
 - c. Pulmonary angiogram
 - d. Lung transplantation surgery

ANS: A

Placement of a tracheostomy tube will secure the patient's airway directly through the trachea, eliminating the need for the endotracheal tube. This will make the patient more comfortable and may allow eating while minimizing damage to the oropharynx from the endotracheal tube.

NURSINGTB.COM

DIF: Understanding

OBJ: 38.6

TOP: Planning

MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:

Concepts: Gas Exchange

5. The nurse is caring for a patient with a chest tube who was transported to radiology for testing. When the patient returns to the nursing unit, the transporter shows the nurse the patient's chest tube collection device, which was badly damaged after being caught in the elevator door. What is the priority action of the nurse?
- a. Clamp the chest tube until the collection device is replaced.
 - b. Cover the insertion site with a new occlusive dressing.
 - c. Ensure that there is gentle bubbling in the water seal chamber.
 - d. Check the patient's lung sounds and pulse oximetry.

ANS: A

The broken collection device may no longer be used to collect chest tube drainage. Clamping the chest tube until the collection device is replaced will prevent air from entering the lung space until the new collection device is attached.

DIF: Applying

OBJ: 38.6

TOP: Implementation

MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:

Concepts: Gas Exchange

6. The nurse is caring for a patient who is hospitalized for pneumonia. Which Nursing diagnosis has the highest priority?
- Activity intolerance r/t generalized weakness and hypoxemia
 - Impaired nutritional intake r/t poor appetite and increased metabolic needs
 - Impaired airway clearance r/t thick secretions in trachea and bronchi
 - Lack of knowledge r/t use of nebulizer and inhaled bronchodilators

ANS: C

Airway maintenance and patency is the highest priority for all patients, especially patients with respiratory disorders. Oxygenation is the most important human need. The other diagnoses can apply once the patient's airway is kept patent.

DIF: Applying OBJ: 38.4 TOP: Diagnosis
 MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
 Concepts: Gas Exchange

7. The nurse is caring for a patient who developed a pulmonary embolism after surgery. Which goal statement is the highest priority for the nurse to include in the patient's care plan for the diagnosis impaired gas exchange r/t impaired pulmonary blood flow from embolus?
- The patient will maintain pulse oximetry values of at least 95% on room air.
 - The patient will verbalize understanding of ordered anticoagulants.
 - The patient will report chest pain of no greater than 3 on a 1 to 10 scale.
 - The patient will ambulate 50 feet in hallway without shortness of breath.

ANS: A

Oxygenation is the most important human need, so adequate oxygenation of tissues as evidenced by pulse oximetry values of at least 95% on room air is the highest priority goal. The other goals may be addressed once the oxygenation goal has been met.

DIF: Applying OBJ: 38.5 TOP: Planning
 MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
 Concepts: Gas Exchange

8. The nurse is caring for a patient with severe COPD who is becoming increasingly confused and disoriented. What is the priority action of the nurse?
- Obtain an arterial blood gas to check for carbon dioxide retention.
 - Increase the patient's oxygen until the pulse oximetry is greater than 98%.
 - Lower the head of the patient's bed and insert a nasal airway.
 - Administer a mild sedative and reorient the patient as needed.

ANS: A

Confusion and disorientation in a patient with severe COPD may likely be due to carbon dioxide retention. An arterial blood gas should be drawn to determine if this is the case. COPD patients should be kept on low oxygen flow rates whenever possible to avoid impeding the drive to breathe. Lowering the head of the bed will increase the difficulty of breathing as the abdominal contents press on the diaphragm. A sedative will cause respiratory depression and should be avoided.

DIF: Applying OBJ: 38.6 TOP: Implementation
 MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
 Concepts: Gas Exchange

9. The nurse is caring for a patient who has been prescribed warfarin (Coumadin) therapy after being diagnosed with atrial fibrillation. The patient asks the nurse what could happen if the prescription doesn't get filled. What is the nurse's best response?
- You could have a stroke.
 - Your kidneys could fail.
 - You could develop heart failure.
 - You could go into respiratory failure.

ANS: A

A major complication of chronic atrial fibrillation is formation of blood clots within the atria due to sluggish blood flow. Anticoagulation therapy is common to prevent blood clot formation that could travel to the brain, causing a stroke.

DIF: Understanding

OBJ: 38.6

TOP: Teaching/Learning

MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:

Concepts: Patient Education

10. The preceptor is working with a new nurse to provide care for a patient with a chest tube to relieve a pneumothorax. Which action by the new nurse indicates need for additional teaching about chest tube care?
- The suction is discontinued when the patient is ambulated to the bathroom.
 - The collection device is emptied at the end of the shift and output recorded in the chart.
 - The patient's bed is placed in the semi-Fowler's position to facilitate lung reexpansion.
 - The patient is encouraged to use his incentive spirometer at least 10 times every hour.

ANS: B

NURSINGTB.COM

The chest tube collection device is not emptied at the end of the shift. Instead, the amount of drainage present at the end of the shift (or specified time) is marked on the collection device and the amount of drainage is documented in the patient's chart.

DIF: Understanding

OBJ: 38.6

TOP: Implementation

MSC: NCLEX Client Needs Category: Physiological Integrity: Reduction of Risk Potential NOT:

Concepts: Safety

11. The nurse is caring for a postoperative patient who has just been diagnosed with a deep vein thrombosis (DVT) in the right leg. Which focused assessment question has the highest priority for this patient?
- Do you have a headache or any dizziness?
 - Do you have any chest pain or shortness of breath?
 - When did you first notice the swelling and redness in your leg?
 - Do you have any cramping or muscle spasms in your leg?

ANS: B

The highest risk of a DVT is the potential for the clot to break free and travel through the bloodstream to cause a pulmonary embolus (PE). The nurse should ask the patient about chest pain or shortness of breath to assess if a PE may have occurred.

DIF: Applying

OBJ: 38.3

TOP: Assessment

MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation

NOT: Concepts: Clotting

12. The nurse identifies which patient who would benefit from postural drainage?
- A patient with a heart murmur and jugular venous distention
 - A patient with asthma and audible wheezing
 - A patient with right-sided heart failure and pitting edema
 - A patient with chronic bronchitis and congested cough

ANS: D

Postural drainage is used for patients who have difficulty removing thick secretions from the airway. A patient with chronic bronchitis and a congested, productive cough would benefit from postural drainage because it would help clear the airway.

DIF: Understanding

OBJ: 38.6

TOP: Assessment MSC:

NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:

Concepts: Gas Exchange

13. The nurse is caring for a patient who has a history of congestive heart failure with generalized pitting edema. Which laboratory results will the nurse expect to find in the patient's chart?
- Glycosylated hemoglobin 12%
 - Platelet count 450,000/mm³
 - Hematocrit 32%
 - Prothrombin time 8.8 seconds

ANS: C

Hemodilution is a common finding when patients are in fluid overload caused by congestive heart failure. A normal hematocrit result is 42% to 52% for a male and 37% to 47% for a female, so the patient's 32% hematocrit level is markedly low. The other laboratory results are not expected due to congestive heart failure or fluid overload.

DIF: Applying

OBJ: 38.6

TOP: Assessment

MSC: NCLEX Client Needs Category: Physiological Integrity: Reduction of Risk Potential NOT:

Concepts: Fluid and Electrolyte Balance

14. The nurse is caring for a patient with high cholesterol who has been prescribed atorvastatin (Lipitor). Which laboratory result indicates that the patient has been taking the medication as ordered and following the physician's dietary recommendations?
- Serum triglyceride level 325 mg/dL
 - High-density lipoproteins (HDL) 56 mg/dL
 - Low-density lipoproteins (LDL) 155 mg/dL
 - Total cholesterol level 185 mg/dL

ANS: D

Total cholesterol levels should be less than 200 mg/dL, so a cholesterol level of 185 mg/dL indicates that the patient has been compliant with the prescribed therapy. The other laboratory results are abnormal and would not indicate compliance.

DIF: Applying

OBJ: 38.6

TOP: Assessment

MSC: NCLEX Client Needs Category: Physiological Integrity: Reduction of Risk Potential NOT:

Concepts: Nutrition

15. The nurse is caring for a patient who has presented to the ER with chest pain. Which diagnostic test will best indicate if there is significant blockage of important blood vessels that provide oxygen to the heart muscle?
- Cardiac catheterization
 - Chest x-ray
 - Echocardiogram
 - Electrocardiogram

ANS: A

Cardiac catheterization includes the use of contrast dye to visualize the coronary arteries and determine blood flow to cardiac muscle. The other tests will not allow the physician to determine which (if any) coronary arteries are occluded.

DIF: Understanding

OBJ: 38.6

TOP: Assessment MSC:

NCLEX Client Needs Category: Physiological Integrity: Reduction of Risk Potential NOT:

Concepts: Perfusion

16. The nurse hears a loud murmur when listening to the patient's heart. Which diagnostic test will best display the condition of the valves and structures within the patient's heart that could be causing the murmur?
- Chest x-ray
 - Cardiac catheterization
 - Echocardiogram
 - Electrocardiogram

ANS: C

Echocardiograms allow for ultrasound visualization of the structures of the heart along with function of the heart valves and cardiac musculature.

DIF: Understanding

OBJ: 38.2

TOP: Assessment MSC:

NCLEX Client Needs Category: Physiological Integrity: Reduction of Risk Potential NOT:

Concepts: Perfusion

17. The nurse is caring for a patient who will be returning to the nursing unit following a cardiac catheterization via the right femoral artery. Which assessment is the highest priority for the nurse to perform when the patient arrives on the unit?
- Checking the patient's right pedal pulse and warmth of the right leg
 - Checking pulse oximetry and listening to the patient's lung sounds
 - Checking bilateral radial pulses to check for a pulse deficit
 - Estimating the patient's jugular venous pressure

ANS: A

Cardiac catheterization includes the insertion of a large IV needle into the patient's femoral artery. Occlusion of the femoral artery may develop after the procedure leading to faint or absent pedal pulses and loss of warmth to the right leg. The nurse should check the patient's right pedal pulses and leg warmth to ensure that the femoral artery has not become occluded. The other assessments may be performed once the patient's right leg is found to be warm with strong pulses.

DIF: Applying

OBJ: 38.3

TOP: Assessment

MSC: NCLEX Client Needs Category: Physiological Integrity: Reduction of Risk Potential NOT:

Concepts: Perfusion

18. The home care nurse is caring for a patient who has severe COPD and home oxygen therapy. The patient tells the nurse that she feels much better after increasing the oxygen flowmeter from 2 L to 5 L/min. The patient's pulse oximetry is 98%. What is the priority action of the nurse?
- Reduce the oxygen flow rate until the patient's pulse oximetry value is more than 90%.
 - Inform the patient's physician and obtain an order for oxygen at 5 L/min.
 - Document the intervention and findings in the patient's medical record.
 - Listen to the patient's lung fields and reinforce pursed-lip breathing techniques.

ANS: A

The goal of long-term therapy for the patient with COPD is usually to have an oxygen saturation level of more than 90%, which represents adequate delivery of oxygen to the tissues. Oxygen saturation may decrease during exercise, sleep, or deterioration of the respiratory status. For the patient with COPD, use low-flow oxygen delivery only (2 L/min) unless a higher level of oxygen administration is indicated by low oxygen saturation levels. High-flow oxygen may lead to respiratory suppression caused by loss of the patient's drive to breathe. The nurse should reduce the oxygen flow rate until the patient's pulse oximetry is more than 90% and educate the patient about oxygen therapy for COPD.

DIF: Analyzing OBJ: 38.6

TOP: Implementation

MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:

Concepts: Gas Exchange

MULTIPLE RESPONSE

1. **NURSINGTB.COM**

The nurse is caring for a patient with advanced COPD who reports feeling short of breath. The nurse notes that the patient's lung sounds are diminished bilaterally and the patient's pulse oximetry is 91% on 2 L/min oxygen via nasal cannula. What actions will the nurse take to make the patient more comfortable? (*Select all that apply.*)

- Increase the patient's oxygen to 4 L/min via nasal cannula.
- Suction the patient's airway using sterile technique.
- Maintain eye contact and provide calm reassurance.
- Turn the patient onto the side for postural drainage.
- Administer the ordered nebulized bronchodilator.
- Elevate the head of the patient's bed to fully upright.

ANS: C, E, F

Patients who are acutely short of breath due to advanced COPD will benefit from nebulized bronchodilator medication to open the airways. Elevating the head of the bed will prevent pressure on the diaphragm from the abdominal contents. A caring demeanor with eye contact will help the patient remain calm until the medication begins to work and the shortness of breath is eased. Patients with COPD should be kept on low-flow oxygen to maintain pulse oximetry of more than 90%.

DIF: Applying

OBJ: 38.6

TOP: Implementation

MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:

Concepts: Gas Exchange

2. The nurse is performing a respiratory assessment on a patient. Which assessment findings indicate to the nurse that the patient has a history of long-standing chronic respiratory disease? (*Select all that apply.*)
- All the patient's fingernails are noticeably clubbed.
 - The patient needs to sleep on at least four to five pillows at night.
 - The patient's chest has equal antero-posterior and transverse diameters.
 - The patient's lower legs have large areas of brownish spotted discoloration.
 - The patient reports puffiness of both feet when standing for long periods.
 - The patient's forced vital capacity test result is 3.8 L of air.

ANS: A, B, C

Clubbing of fingernails, the need to sleep in an upright position, and a barrel chest are all indicative of long-standing chronic respiratory disease like COPD. Brownish spotted discoloration is indicative of venous insufficiency. Edema can be seen in renal and heart failure. Forced vital capacity of almost 4 L is found in patients with good respiratory function.

DIF: Applying

OBJ: 38.3

TOP: Assessment

MSC: NCLEX Client Needs Category: Physiological Integrity: Reduction of Risk Potential NOT:

Concepts: Gas Exchange

3. The nurse notes the following findings when assessing a patient with COPD. Which require prompt nursing intervention? (*Select all that apply.*)
- The patient is unable to speak without gasping.
 - The patient's fingernails are noticeably clubbed.
 - The patient's sputum has turned from yellow to greenish-brown.
 - The patient has stridor with wheezes heard in all lung fields.
 - The patient's forced vital capacity has increased from 2.8 to 3.4 L.
 - The patient has become confused and mildly disoriented.

ANS: A, C, D, F

A patient who is unable to speak without gasping is indicative of poor airflow through the airways, which must be addressed promptly. Greenish-brown sputum may indicate pneumonia requiring antibiotic treatment. Stridor and wheezes are indicative of an acute asthma attack.

Confusion and disorientation in a patient with COPD may indicate retention of carbon dioxide.

Clubbed fingernails are indicative of a chronic respiratory condition. Increased forced vital capacity is a positive sign.

DIF: Applying

OBJ: 38.3

TOP: Assessment

MSC: NCLEX Client Needs Category: Physiological Integrity: Reduction of Risk Potential NOT:

Concepts: Gas Exchange

4. The nurse is working with a nursing assistant to care for a patient with a new tracheostomy. Which tasks may the nurse delegate to the assistant? (*Select all that apply.*)
- Obtaining masks, gloves, and suction supplies from the utility room
 - Helping to reassure the patient before, during, and after suctioning
 - Changing the Velcro or twill ties used to secure the tracheostomy
 - Transporting sputum specimens to the lab for culture and sensitivity testing
 - Assessing need for suctioning of the oropharynx or tracheostomy
 - Teaching the patient how to remove and clean the inner cannula

ANS: A, B, D

Care of a new tracheostomy may not be delegated to a nursing assistant. Obtaining supplies needed for care, helping to reassure the patient, and bringing specimens to the lab are tasks that may be assigned to the assistant.

DIF: Applying OBJ: 38.6 TOP: Implementation
 MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:
 Concepts: Care Coordination

5. The preceptor is working with a new nurse to provide care for a patient with a new tracheostomy. Which actions by the new nurse indicate need for additional teaching about the procedure? (*Select all that apply.*)
- The outer cannula is cleaned with the brush and half-strength H₂O₂.
 - The new tracheostomy holder is secured before the old soiled one is removed.
 - A Yankauer suction catheter is used to remove secretions from the patient's mouth.
 - Sterile gloves are applied before the soiled dressing is removed from the tracheostomy.
 - Half-strength H₂O₂ is used to remove crusted secretions around the tracheostomy site.
 - Pain medication is administered to the patient prior to suctioning.

ANS: A, D, E

Only the inner cannula of the tracheostomy is removed for cleaning. The outer cannula stays in the trachea to maintain airway patency. Clean gloves are applied before the soiled dressing is removed. Normal sterile saline is used to remove secretions that have built up on the inner cannula and also is used to clean the patient's skin as needed.

DIF: Applying OBJ: 38.6 TOP: Implementation

MSC: NCLEX Client Needs Category: Physiological Integrity: Reduction of Risk Potential NOT:
 Concepts: Gas Exchange

6. The preceptor is working with a new nurse to suction a patient through a new tracheostomy. Which actions by the new nurse indicate need for additional teaching about the procedure? (*Select all that apply.*)
- The suction is not applied to the catheter until it is being withdrawn.
 - The patient is placed in the supine position prior to suctioning.
 - The suction catheter is twirled side to side as it is being withdrawn.
 - Suction is applied continuously as the catheter is withdrawn.
 - The patient's oxygen is reapplied between suction attempts.
 - Water-soluble lubricant is applied to the suction catheter before insertion.

ANS: B, D, F

The head of the patient's bed should be elevated prior to suctioning to facilitate coughing out secretions. Suction is always applied intermittently as the catheter is withdrawn. Water-soluble lubricant is used when suctioning the nares but not a tracheostomy because the secretions negate the need for additional lubrication.

DIF: Understanding OBJ: 38.6 TOP: Implementation
 MSC: NCLEX Client Needs Category: Physiological Integrity: Reduction of Risk Potential NOT:
 Concepts: Gas Exchange

Chapter 39: Fluid, Electrolytes, and Acid-Base Balance**Yoost & Crawford: Fundamentals of Nursing: Active Learning for Collaborative Practice, 3rd Edition****MULTIPLE CHOICE**

1. The nurse will be caring for a patient who is severely malnourished. Laboratory test results show that the patient's albumin level is critically low. What assessment finding will the nurse expect to note when assessing the patient?
 - a. The patient has generalized 3+ pitting edema.
 - b. The patient is confused and disoriented.
 - c. The patient's urine is dark and very concentrated.
 - d. The patient lung sounds are very diminished.

ANS: A

The patient's low albumin level will lead to generalized pitting edema because there isn't enough protein in the blood to keep water within the bloodstream. Lack of oncotic pressure from low serum albumin leads to edema. The other findings are not related to malnutrition.

DIF: Understanding

OBJ: 39.3

TOP: Assessment MSC:

NCLEX Client Needs Category: Physiological Integrity: Reduction of Risk Potential NOT:

Concepts: Fluid and Electrolyte Balance

2. The nurse is reviewing the patient's laboratory results. Which result must be communicated to the physician immediately?
 - a. Serum chloride level 85 mEq/L
 - b. Serum sodium level 134 mEq/L
 - c. Serum potassium level 6.8 mEq/L
 - d. Serum magnesium level 2.3 mEq/L

ANS: C

Normal serum potassium level is 3.5 to 5.0 mEq/L. A serum potassium level of 6.8 mEq/L is very high and puts the patient at risk for cardiac arrhythmias. The potassium level should be reported to the physician immediately. The chlorine and sodium levels are slightly low and the magnesium level is slightly elevated.

DIF: Understanding

OBJ: 39.2

TOP: Implementation

MSC: NCLEX Client Needs Category: Physiological Integrity: Reduction of Risk Potential NOT:

Concepts: Fluid and Electrolyte Balance

3. The nurse is caring for a patient who is at risk for fluid overload due to a history of congestive heart failure. Which intervention will the nurse teach the patient to perform at home to monitor fluid balance?
 - a. —Check to make sure that your urine is a bright yellow color.¶
 - b. —Weigh yourself every morning before breakfast.¶
 - c. —Count your heart rate every evening before you go to bed.¶
 - d. —Drink plain water rather than soda, coffee, or fruit juice.¶

ANS: B

Checking the weight every morning before breakfast is a sensitive indicator of the patient's fluid volume status. Weight gain of 2 kg in 3 days generally indicates fluid retention and should be reported to the physician.

DIF: Understanding

OBJ: 39.6

TOP: Teaching/Learning

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance

NOT: Concepts: Patient Education

4. The nurse is caring for a patient who is admitted to the hospital with diabetic ketoacidosis. Which assessment finding indicates an attempt made by the patient's body to correct the pH?
- The patient's respirations are very deep and rapid.
 - The patient's urine is dark and concentrated.
 - The patient's skin is pale, cool, and diaphoretic.
 - The patient is sleepy and difficult to arouse.

ANS: A

The patient with diabetic ketoacidosis is in a state of metabolic acidosis. The body will attempt to compensate for the acidosis by blowing off extra amounts of carbon dioxide through deep, rapid respirations. Since carbon dioxide is converted to carbonic acid, removal of carbon dioxide will help shift the body's pH to a less acidotic state.

DIF: Applying

OBJ: 39.3

TOP: Assessment

MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation

NOT: Concepts: Acid-Base Balance

5. The nurse is caring for a patient who takes furosemide (Lasix) daily to treat congestive heart failure. The nurse will watch for which electrolyte imbalance that may occur due to this therapy?
- Hypocalcemia
 - Hypernatremia
 - Hypokalemia
 - Hyperphosphatemia

NURSINGTB.COM

ANS: C

Furosemide is a loop diuretic that causes loss of potassium through the urine. Patients taking this medication are at risk for hypokalemia, so the nurse should check the patient's electrolyte values closely, particularly the serum potassium level.

DIF: Understanding

OBJ: 39.2

TOP: Assessment

MSC: NCLEX Client Needs Category: Physiological Integrity: Pharmacological and Parenteral

Therapies

NOT: Concepts: Fluid and Electrolyte Balance

6. The nurse is caring for a patient who was brought to the ED after overdosing on narcotic pain medication. The patient was found unresponsive with no respirations. Arterial blood gases were drawn shortly after the patient's arrival to the hospital. Which results will the nurse expect to see?
- pH 7.56, PaCO₂ 32 mm Hg, HCO₃ 32 mEq/L, PaO₂ 90 mm Hg
 - pH 7.35, PaCO₂ 45 mm Hg, HCO₃ 26 mEq/L, PaO₂ 70 mm Hg
 - pH 7.45, PaCO₂ 38 mm Hg, HCO₃ 28 mEq/L, PaO₂ 80 mm Hg
 - pH 7.27, PaCO₂ 58 mm Hg, HCO₃ 24 mEq/L, PaO₂ 60 mm Hg

ANS: D

The patient who overdosed on narcotic pain medication will be in respiratory acidosis due to respiratory suppression. Low pH of 7.27 and elevated PaCO₂ are consistent with respiratory acidosis as insufficient carbon dioxide is removed from the blood. The low 60 mm Hg PaO₂ is due to insufficient oxygen intake.

DIF: Applying OBJ: 39.2 TOP: Assessment
 MSC: NCLEX Client Needs Category: Physiological Integrity: Reduction of Risk Potential NOT:
 Concepts: Acid-Base Balance

7. The nurse is caring for a patient who is admitted to the hospital with dehydration and gastroenteritis. The patient attempted to walk to the bathroom and fainted right after getting out of bed. The nurse knows which condition to be the most likely cause of the patient's collapse?
- Orthostatic hypotension
 - Circulatory overload
 - Hemolytic reaction
 - Catheter embolism

ANS: A

The patient with dehydration is at risk for orthostatic hypotension or falling of the blood pressure when the patient rises to a standing position. When the blood pressure falls sufficiently, fainting may occur. The patient should be assisted to rise slowly from a supine to a sitting position first before slowly getting to his feet. Circulatory overload, hemolytic anemia, and catheter embolism are unlikely to be causative factors.

DIF: Understanding OBJ: 39.2 TOP: Assessment MSC:
 NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:
 Concepts: Fluid and Electrolyte Balance

8. The nurse is caring for a patient whose ABG results reveal the following: pH 7.56, PaCO₂ 32 mm Hg, HCO₃ 42 mEq/L, PaO₂ 90 mm Hg. Which condition will the nurse expect to see in the patient's chart as the underlying cause of these results?
- Gastroenteritis with severe nausea, vomiting, and diarrhea
 - Widespread tissue ischemia caused by cardiogenic shock
 - Respiratory failure caused by pneumonia with pleural effusions
 - Hyperventilation after a panic attack

ANS: A

Gastroenteritis with nausea, vomiting, and diarrhea will lead to a metabolic alkalosis resulting from loss of electrolytes and acids through emesis and loose stools. Metabolic alkalosis features the elevated pH of 7.56, elevated HCO₃ 42 mEq/L, and normal PaCO₂ of 32 mm Hg. Widespread tissue ischemia would lead to metabolic acidosis with low pH resulting from release of lactic acid from the tissues. Respiratory failure leads to a respiratory acidosis with a low pH and elevated PaCO₂ level. Hyperventilation leads to respiratory alkalosis with an elevated pH and elevated HCO₃ level.

DIF: Applying OBJ: 39.2 TOP: Assessment
 MSC: NCLEX Client Needs Category: Physiological Integrity: Reduction of Risk Potential NOT:
 Concepts: Acid-Base Balance

9. The nurse is caring for a patient who has a 1200 mL daily fluid restriction. The patient has consumed 250 mL with each of the three meals and had another 150 mL with medication administration. The patient has received 150 mL of IV fluids during the day. How many mL of fluid may the patient still consume to stay within the prescribed fluid restriction?
- 100 mL
 - 150 mL
 - 250 mL
 - 300 mL

ANS: B

The patient has had an oral fluid intake of 900 mL and an IV fluid intake of 150 mL, giving a total of 1050 mL. This leaves 150 mL that the patient may consume for the rest of the evening to stay within the prescribed fluid restriction.

DIF: Applying OBJ: 39.2 TOP: Assessment

MSC: NCLEX Client Needs Category: Physiological Integrity: Reduction of Risk Potential NOT:

Concepts: Fluid and Electrolyte Balance

10. The nurse is caring for a patient who has a history of congestive heart failure. The nurse includes the diagnosis hypervolemia in the patient's care plan. Which goal statement has the highest priority for the patient and nurse?
- The patient's lung sounds will remain clear.
 - The patient will have urine output of at least 30 mL/hr.
 - The patient will verbalize understanding of fluid restrictions.
 - The patient's pitting pedal edema will resolve within 72 hours.

ANS: A

Oxygenation is the highest priority for the patient with congestive heart failure and hypervolemia. Keeping the patient's lungs clear is the most important goal for the nurse to consider when caring for this patient.

DIF: Applying OBJ: 39.5 TOP: Planning

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Fluid and Electrolyte Balance

11. The nurse is caring for a patient with syndrome of inappropriate antidiuretic hormone secretion (SIADH) who has a serum sodium level of 118 mEq/dL and symptoms of fluid overload. Which IV fluid will the nurse expect to administer to this patient to correct the patient's fluid imbalance?
- 0.33% normal saline
 - 0.45% normal saline
 - 0.9% normal saline
 - 3% normal saline

ANS: D

A hypertonic 3% saline solution will be used to correct the patient's hyponatremia and fluid overload that have developed due to SIADH. A 0.9% normal saline solution can be used once the serum sodium level has been raised nearer to normal range. A 0.45% or 0.33% normal saline solution is hypotonic and will only worsen the patient's fluid overload and hyponatremia.

DIF: Applying OBJ: 39.6 TOP: Implementation

MSC: NCLEX Client Needs Category: Physiological Integrity: Pharmacological and Parenteral Therapies
NOT: Concepts: Fluid and Electrolyte Balance

12. The nurse is caring for a patient with congestive heart failure who requires intermittent IV bolus doses of furosemide (Lasix) to correct fluid volume overload. No continuous IV fluids are ordered. Which type of IV will the nurse insert to administer the patient's medication?
- Peripherally inserted central catheter
 - Midline inside-the-needle catheter
 - Central venous catheter
 - Over-the-needle catheter

ANS: D

Intermittent doses of IV diuretics are best administered via an over-the-needle angiocatheter that is connected to a saline lock. The other IV catheter options are used when the patient requires a vesicant drug that could cause significant damage to tissues or when the patient requires weeks of IV therapy.

DIF: Understanding OBJ: 39.6 TOP: Implementation MSC:
NCLEX Client Needs Category: Physiological Integrity: Pharmacological and Parenteral Therapies
NOT: Concepts: Fluid and Electrolyte Balance

13. The nurse is caring for a patient who has a central venous catheter (CVC). Which nursing intervention is the most important for the nurse to include in the patient's plan of care?
- Carefully document all assessments of the catheter site.
 - Use strict sterile procedure when performing dressing changes.
 - Label each new dressing with the date, time, and nurse's initials.
 - Ensure that the CVC is discontinued as soon as possible.

ANS: B

Strict sterile procedure is mandatory when changing CVC dressings because of the high risk of septicemia and/or sepsis. The other actions are appropriate, but not of the highest priority.

DIF: Understanding OBJ: 39.6 TOP: Implementation
MSC: NCLEX Client Needs Category: Physiological Integrity: Reduction of Risk Potential NOT:
Concepts: Infection

14. The nurse is caring for a patient with a peripheral IV who tells the nurse that the IV site is painful and puffy. What is the nurse's best action?
- Discontinue the IV and start another line in the other arm.
 - Aspirate to check for blood return and flush the IV with sterile saline.
 - Clean the IV site with chlorhexidine and apply a new sterile dressing.
 - Change the IV tubing and administer prescribed pain medication.

ANS: A

An IV site that is puffy and painful should be discontinued promptly because the fluid has infiltrated outside the vein and is causing localized irritation. The IV should be restarted in the other arm if possible. The other actions are inappropriate.

DIF: Understanding OBJ: 39.6 TOP: Implementation MSC:
NCLEX Client Needs Category: Physiological Integrity: Pharmacological and Parenteral Therapies
NOT: Concepts: Fluid and Electrolyte Balance

15. The nurse is caring for a patient who is to receive dopamine (Intropin) through the IV line. Which intervention has the highest priority when administering this medication?
- Check for IV blood return prior to administration.
 - Use a new IV tubing set each time the medication is administered.
 - Document the date, time, and nurse's initials after each dose is administered.
 - Use sterile gloves when drawing up and administering the medication.

ANS: A

Dopamine is a vesicant and can cause significant irritation to blood vessels and tissues when administered via IV. For this reason, the nurse must ensure that the IV catheter is located correctly in the vein by checking for a blood return prior to administration.

DIF: Applying OBJ: 39.6 TOP: Implementation
 MSC: NCLEX Client Needs Category: Physiological Integrity: Pharmacological and Parenteral Therapies
 NOT: Concepts: Caregiving

16. The nurse is caring for a patient who is to receive a transfusion of packed red blood cells. The patient has a 22-gauge IV in his arm with 0.9% normal saline infusing. What intervention will the nurse perform before obtaining the packed red blood cells from the blood bank?
- Identify the blood group, type, and expiration date with another nurse.
 - Assess the patency of the current IV site for the administration of the packed RBCs.
 - Program the IV infusion pump so that the transfusion will complete within 4 hours.
 - Obtain a new microdrip tubing and extension tubing from the clean utility room.

ANS: B

Before obtaining the blood from the blood bank, the nurse ensures the patient's IV is patent. Research confirms blood can be safely infused as small as a 24 gauge IV. Then the nurse obtains the blood and double checks the blood group, type, expiration date, and patient ID with a second nurse. Next the nurse sets up the IV and IV pump. Blood is not run through a microdrip tubing set.

DIF: Applying OBJ: 39.6 TOP: Implementation
 MSC: NCLEX Client Needs Category: Physiological Integrity: Pharmacological and Parenteral Therapies
 NOT: Concepts: Fluid and Electrolyte Balance

17. The nurse is caring for a patient who is receiving a blood transfusion. Fifteen minutes into the transfusion, the patient's blood pressure decreases significantly, and the patient complains of a severe headache. What is the priority action of the nurse?
- Check the patient's temperature and administer acetaminophen (Tylenol) if higher than 101 °F.
 - Recheck the patient's blood pressure in 15 minutes after administering pain medication.
 - Stop the blood transfusion and administer 0.9% normal saline through new IV tubing.
 - Double-check that the transfusion blood type is an exact match to the patient.

ANS: C

A significant drop in blood pressure and a severe headache are signs that the patient may be experiencing a transfusion reaction. Also, most reactions to a transfusion occur within the first 15 minutes of initiation. Therefore the nurse should remain with the patient at the bedside during this time to observe for signs of a reaction. The transfusion should be stopped and 0.9% normal saline should be administered through new IV tubing to prevent infusion of additional blood through the tubing used for the transfusion. The physician should be notified immediately to evaluate the patient. Ensuring that the transfusion blood type is an exact match to the patient is done before the transfusion is begun.

DIF: Applying OBJ: 39.6 TOP: Implementation
 MSC: NCLEX Client Needs Category: Physiological Integrity: Pharmacological and Parenteral Therapies
 NOT: Concepts: Fluid and Electrolyte Balance

18. The nurse is caring for a patient who is very dehydrated. Which goal best indicates that dehydration has been corrected and that the patient's fluid balance has been restored?
- The patient had 1300 mL of light yellow urine in the last 24 hours.
 - The patient's lung sounds are clear bilaterally.
 - The patient has no jugular venous distention.
 - The patient verbalizes need for adequate daily fluid intake.

ANS: A

The goal that best indicates that the patient's dehydration has been corrected is output of 1300 mL of clear yellow urine in the last 24 hours. Dark concentrated urine is a symptom of dehydration. Jugular venous distention and presence of crackles in the lungs are both indicative of fluid volume overload.

DIF: Applying OBJ: 39.5 TOP: Assessment
 MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation
 NOT: Concepts: Fluid and Electrolyte Balance

19. The nurse is caring for a patient who is admitted with a serum sodium level of 120 mEq/L. Which is the most important intervention for the nurse to perform?
- Perform regular neurologic checks and institute seizure precautions.
 - Encourage the patient to eat foods that are high in sodium.
 - Administer hypotonic IV solutions as ordered by the physician.
 - Assess for signs and symptoms of digoxin (Lanoxin) toxicity.

ANS: A

A serum sodium level of 124 mEq/L is dangerously low and may cause neurologic problems including seizures, confusion, and weakness. Regular neurologic checks should be performed and the patient should be placed on seizure precautions until the sodium level is corrected. Encouraging the patient to eat high-sodium foods is fine, but it is not as important as the patient's safety. A hypotonic saline solution will further lower the patient's sodium level. Digoxin toxicity is seen with hypokalemia rather than hyponatremia.

DIF: Understanding OBJ: 39.6 TOP: Implementation
 MSC: NCLEX Client Needs Category: Physiological Integrity: Reduction of Risk Potential
 NOT: Concepts: Fluid and Electrolyte Balance

20. The nurse is caring for a patient who has a history of congestive heart failure and takes once-daily furosemide (Lasix) to prevent fluid overload and pulmonary edema. The patient admits to stopping the medication due to nocturia. What is the nurse's best response?
- You should ask your doctor to decrease the dose.||
 - Take the diuretic early in the morning before breakfast.||
 - Eat foods high in potassium and limit your salt intake.||
 - Restrict your fluid intake after dinner and in the evening.||

ANS: B

The patient should be instructed to take the diuretic early in the morning so that the effects will wear off before the patient goes to bed at night. Decreasing the dose could lead to fluid overload and pulmonary edema.

DIF: Applying

OBJ: 39.6

TOP: Teaching/Learning

MSC: NCLEX Client Needs Category: Physiological Integrity: Pharmacological and Parenteral Therapies

NOT: Concepts: Patient Education

21. The nurse is caring for a patient with renal failure who has a serum potassium level of 7.1 mEq/L and serum magnesium level of 3.5 mEq/L. The nurse prepares to administer 10 units of insulin and an ampule of 50% dextrose to the patient. The patient asks why he will be receiving insulin when he is not diabetic. What is the nurse's best answer?
- The doctor has prescribed these medications for you to help heal your kidneys.||
 - These medications will lower your potassium level and prevent an irregular heart rate.||
 - These medications will prevent you from having a seizure from too little magnesium.||
 - These medications will increase your urine output until your kidneys recover.||

ANS: B

Serum potassium levels above 7.0 mEq/L can lead to dangerous cardiac arrhythmias, so the potassium level must be lowered promptly. Administration of IV insulin with 50% dextrose will push potassium into the cells to avoid hyperkalemia symptoms.

DIF: Understanding

OBJ: 39.6

TOP: Teaching/Learning

MSC: NCLEX Client Needs Category: Physiological Integrity: Reduction of Risk Potential NOT: Concepts: Fluid and Electrolyte Balance

22. The nurse is caring for a patient with a history of hyperparathyroidism who presents with a serum calcium level of 14.5 mg/dL. What is the highest priority Nursing diagnosis for this patient?
- Risk for injury related to weakened bones that may easily fracture
 - Lack of knowledge related to need for supplemental calcium in diet
 - Risk for constipation caused by decreased gastrointestinal motility
 - Activity intolerance related to muscle cramping and spasms

ANS: A

Chronic hypercalcemia can lead to weakened bones as strengthening calcium is removed over time. Pathologic fractures can easily result, so risk for injury is a high priority Nursing diagnosis for this patient. The other Nursing diagnoses apply but are less important than the safety of the patient.

DIF: Applying

OBJ: 39.4

TOP: Diagnosis

MSC: NCLEX Client Needs Category: Physiological Integrity: Reduction of Risk Potential NOT:
Concepts: Fluid and Electrolyte Balance

23. The nurse is caring for a patient who has a serum magnesium level of 0.8 mEq/L. Which is the highest priority goal to include in the patient's plan of care?
- The patient will maintain urine output of at least 30 mL/hr.
 - The patient will verbalize the importance of sufficient dietary intake of magnesium.
 - The patient's oral mucous membranes will remain free of ulceration and pain.
 - The patient will remain alert and oriented 3 with no confusion or seizure activity.

ANS: D

A patient with low serum magnesium is at risk for neurologic symptoms including confusion, disorientation, and seizures. The highest priority goal for this patient is to avoid neurologic problems that could lead to injury. The other goals are applicable to the patient with low magnesium but are less important.

DIF: Applying OBJ: 39.5 TOP: Planning

MSC: NCLEX Client Needs Category: Physiological Integrity: Reduction of Risk Potential NOT:
Concepts: Fluid and Electrolyte Balance

24. The nurse is caring for a hospitalized patient with hyperparathyroid disease and a serum calcium level of 14.2 mg/dL. What is the priority intervention of the nurse?
- Instruct the patient to always call for assistance before getting out of bed.
 - Assist the patient to change into dry clothing after episodes of diaphoresis.
 - Teach stress-relieving techniques, including progressive muscle relaxation.
 - Notify the provider if urine output is less than 30 mL/hr.

ANS: A

The patient with hypercalcemia should always call for assistance before getting out of bed because of the risk of falling due to muscle weakness, soft bones, and lethargy. Diaphoresis and decreased urine output are not common symptoms of hypercalcemia. Teaching stress-relieving techniques is not a priority.

DIF: Applying OBJ: 39.6 TOP: Implementation

MSC: NCLEX Client Needs Category: Physiological Integrity: Reduction of Risk Potential NOT:
Concepts: Fluid and Electrolyte Balance

MULTIPLE RESPONSE

1. The nurse is caring for a patient who has B-positive blood. The patient is severely anemic and requires a blood transfusion. The nurse knows which types of blood can the patient receive? (Select all that apply.)
- AB positive
 - AB negative
 - B negative
 - B positive
 - O positive
 - O negative

ANS: C, D, E, F

If a person produces the B antigen, the blood type is classified as B. Type O blood is classified as universal donors because their blood cells contain no antigens. Rh positive (Rh⁺) blood which means the person has the Rh factor on the surface of the red blood cells. Those who do not have the Rh factor are considered Rh negative (Rh⁻). A person who is B positive can receive B or O blood, and it can be positive or negative Rh factor.

DIF: Understanding

OBJ: 39.1

TOP: Implementation MSC:

NCLEX Client Needs Category: Physiological Integrity: Pharmacological and Parenteral Therapies

NOT: Concepts: Perfusion

NURSINGTB.COM

Chapter 40: Bowel Elimination**Yoost & Crawford: Fundamentals of Nursing: Active Learning for Collaborative Practice, 3rd Edition**

MULTIPLE CHOICE

1. The nurse is caring for a patient who periodically has small streaks of fresh red blood in the stool. The patient denies abdominal pain or loss of appetite. The nurse identifies what to be the most likely cause of this patient's bleeding?
- Hemorrhoids
 - Bleeding gastric ulcer
 - Colon polyps
 - Perforated colon

ANS: A

Bleeding hemorrhoids can lead to small streaks of fresh red blood in the stool. Bleeding gastric ulcer would lead to black, tarry stools as the blood is digested. Colon polyps do not cause bleeding.

DIF: Understanding

OBJ: 40.2

TOP: Assessment MSC:

NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:

Concepts: Elimination

2. The nurse is caring for a patient who has diarrhea and identifies which priority nursing diagnosis for this patient?
- Lack of knowledge related to prescribed diet modifications
 - Impaired nutritional intake related to poor appetite
 - Diarrhea related to excessive loss of fluid through stool
 - Anxiety related to incontinence with loose stools and need for clothing change

ANS: C

Dehydration is the priority nursing problem for this patient, so diarrhea is the most important Nursing diagnosis. Impaired nutritional intake, lack of knowledge, and anxiety can be addressed once fluid balance is restored.

DIF: Applying

OBJ: 40.4

TOP: Diagnosis

MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:

Concepts: Elimination

3. The nurse is caring for a patient who is prescribed diphenoxylate-atropine (Lomotil). Which assessment finding by the nurse indicates a need to contact the prescriber and question the order?
- The patient has skin breakdown from loose stools.
 - The patient is constipated with last BM 3 days ago.
 - The patient is on a low-fiber, gluten-free diet.
 - The patient has painful bleeding hemorrhoids.

ANS: B

Diphenoxylate-atropine is an antidiarrheal medication. It should not be given to patients who are constipated until the patient is checked for impaction. The other assessment findings are not contraindications.

DIF: Understanding OBJ: 40.3 TOP: Assessment
 MSC: NCLEX Client Needs Category: Physiological Integrity: Pharmacological and Parenteral Therapies
 NOT: Concepts: Elimination

4. The nurse is caring for an immobile patient who has abdominal pain and frequent small, liquid stools. The patient vomited his breakfast and is still nauseated. Which action by the nurse is the highest priority?
- Provide oral care after each episode of emesis.
 - Apply a skin barrier to the patient's perineal area.
 - Check the patient for a fecal impaction.
 - Administer antiemetic medication with a sip of water.

ANS: C

The patient who has abdominal pain and frequent small liquid stools should be checked for fecal impaction, especially since the patient is vomiting. Immobility is a risk factor for the development of fecal impaction. The other actions can be performed once fecal impaction is ruled out.

DIF: Applying OBJ: 40.6 TOP: Implementation
 MSC: NCLEX Client Needs Category: Physiological Integrity: Basic Care and Comfort NOT:
 Concepts: Elimination

5. The nurse is caring for a patient who is recovering from bowel surgery. Which assessment finding best indicates that the bowels are starting to resume function and the patient will be able to resume oral intake soon?
- The patient has bowel sounds x 4 quadrants and is passing gas.
 - The patient has no nausea, and abdominal pain is minimal.
 - The patient feels hungry for chicken soup and hot tea.
 - The patient's nasogastric tube was discontinued the previous day.

ANS: A

The presence of bowel sounds and passage of flatus indicate that the patient's bowels are starting to resume function and the patient will be able to resume oral intake soon. Hunger, discontinuation of the NG tube, or absence of nausea are not definite indicators of readiness to resume oral feedings.

DIF: Applying OBJ: 40.3 TOP: Assessment
 MSC: NCLEX Client Needs Category: Physiological Integrity: Reduction of Risk Potential NOT:
 Concepts: Nutrition

6. The nurse is caring for a patient who has an ileostomy. Which Nursing diagnosis has the highest priority for the patient?
- Impaired skin integrity r/t localized skin irritation from liquid stool
 - Social isolation r/t potential leakage of stool from ostomy appliance
 - Lack of knowledge r/t care and maintenance of ostomy appliance
 - Disturbed body image r/t presence of stoma and altered elimination

ANS: A

The highest priority Nursing diagnosis for this patient is impaired skin integrity because the liquid stool from the ileostomy quickly leads to breakdown when in contact with the skin. Open sores can lead to bacterial infection and significant discomfort for the patient. In addition, ostomy appliances do not adhere well to open wounds, increasing the risk for continuing skin breakdown. The other nursing diagnoses are appropriate for this patient but are not the highest priority.

DIF: Applying OBJ: 40.4 TOP: Diagnosis
 MSC: NCLEX Client Needs Category: Physiological Integrity: Reduction of Risk Potential NOT:
 Concepts: Elimination

7. The nurse is caring for a patient who is taking narcotic pain medication after surgery. Which breakfast choices will help prevent constipation and promote return to regular bowel function?
- Raisin bran with skim milk, fresh fruit, and wheat toast
 - Pancakes with maple syrup, bacon, and coffee with cream
 - Omelet with cheddar cheese, green pepper, and onions
 - Bagel with cream cheese, and strawberry nonfat yogurt

ANS: A

The postoperative patient taking narcotic pain medications is at risk for developing constipation. A high-fiber diet with plenty of liquids will help prevent this from occurring. Raisin bran, fruit, and wheat bread are all good sources of fiber.

DIF: Applying OBJ: 40.6 TOP: Teaching/Learning
 MSC: NCLEX Client Needs Category: Physiological Integrity: Basic Care and Comfort NOT:
 Concepts: Nutrition

8. The nurse is caring for a patient who has not had a bowel movement for 2 days. Which is the priority nursing intervention for this patient?
- Obtain an order to administer a soap suds cleansing enema.
 - Teach the patient how to use the Valsalva maneuver.
 - Discontinue medications that can cause constipation.
 - Assess the patient's usual pattern of bowel movements.

ANS: D

The nurse should assess the patient's usual pattern of bowel movements to determine if it is normal for the patient to have a bowel movement every 2 to 3 days. Patients should be taught not to use the Valsalva maneuver because it can lead to bradycardia or death. Medications are not independently discontinued by the nurse and this would require a conversation with the provider.

DIF: Applying OBJ: 40.6 TOP: Implementation
 MSC: NCLEX Client Needs Category: Physiological Integrity: Basic Care and Comfort NOT:
 Concepts: Elimination

9. The nurse is caring for a patient who will be undergoing upper GI series testing the next day. Which instruction will the nurse provide to the patient about the upcoming exam?
- The back of your throat will be sprayed with numbing medicine.¶
 - You will need to have a clear liquid diet and take a laxative tonight.¶
 - You will be given a milky liquid to drink shortly before the test starts.¶
 - You should not take your dose of warfarin (Coumadin) tonight.¶

ANS: C

The patient is given a milky barium liquid to drink as part of the upper GI series, so the patient should be informed of this. The back of the throat is numbed for upper GI endoscopy, not an upper GI series. Warfarin is not contraindicated prior to an upper GI series, and no bowel prep is required.

DIF: Understanding

OBJ: 40.6

TOP: Teaching/Learning

MSC: NCLEX Client Needs Category: Physiological Integrity: Reduction of Risk Potential NOT:

Concepts: Patient Education

10. The nurse is caring for a patient who will undergo colonoscopy testing. Which intervention will the nurse include in the patient's plan of care for the day before the test?
- Provide the patient with zinc oxide skin barrier cream for the perineal area.
 - Obtain an order for a gentle laxative to be given once the test is completed.
 - Carefully assess the patient's ability to swallow liquids through a straw.
 - Check the patient for allergies to shellfish and iodine-based contrast dyes.

ANS: A

Complete bowel evacuation is required prior to colonoscopy so that the physician can visualize the interior of the large intestine. The patient will have multiple soft-liquid bowel movements as part of the bowel prep for the test, so skin barrier cream will be helpful to prevent perineal irritation. Laxatives will not be needed after the colonoscopy, and no contrast dyes are used.

DIF: Applying

OBJ: 40.5

TOP: Implementation

MSC: NCLEX Client Needs Category: Physiological Integrity: Reduction of Risk Potential NOT:

Concepts: Elimination

11. The nurse is caring for a patient who is to have testing for fecal occult blood. What step will the nurse perform during this testing?
- Keep the patient on a clear liquid diet for 72 hours.
 - Send the samples to the laboratory while they are still warm.
 - Inform the patient that several stool samples will be needed.
 - Use a sterile container when collecting the stool samples.

ANS: C

Three stool samples are required for fecal occult testing to avoid missing blood that appears intermittently. A sterile container is not required, and the patient does not need to be on a clear liquid diet for the test. Stool samples for culture and sensitivity should be sent to the laboratory when they are fresh and warm.

DIF: Understanding

OBJ: 40.6

TOP: Implementation

MSC: NCLEX Client Needs Category: Physiological Integrity: Reduction of Risk Potential NOT:

Concepts: Elimination

12. The nurse is caring for a patient who is to have a cleansing enema. Which assessment finding b the nurse indicates a need to contact the prescriber and question the order?
- The patient is recovering from a traumatic brain injury.
 - The patient has not had a bowel movement for 3 days.
 - The patient is to have a lower GI series the following morning.
 - The patient had an upper GI series performed the previous day.

ANS: A

Patients with a traumatic brain injury often have increased intracranial pressure, which can be worsened with enema administration, thus putting the patient at risk for additional neurologic damage. The provider should be contacted and the order should be questioned. Constipation, preparation for a lower GI series, and removal of barium from the colon after upper GI series are all indications for a cleansing enema.

DIF: Applying OBJ: 40.3 TOP: Assessment

MSC: NCLEX Client Needs Category: Physiological Integrity: Reduction of Risk Potential NOT:

Concepts: Elimination

13. The nurse is caring for a postoperative patient who underwent bowel resection surgery that morning. The nurse assesses the patient's abdomen and notes that there are hypoactive bowel sounds. The patient is resting quietly without nausea or vomiting. What is the appropriate action of the nurse?
- Keep the patient NPO and document the findings in the chart.
 - Administer a laxative suppository to stimulate peristalsis.
 - Insert a Salem sump nasogastric tube to low continuous suction.
 - Notify the surgeon and prepare the patient to return to surgery.

ANS: A

The presence of hypoactive bowel sounds is an expected finding for the first hours after abdominal surgery. The patient should be kept NPO to prevent nausea and vomiting. A laxative should not be administered. A nasogastric tube is not needed unless the patient starts vomiting or a paralytic ileus develops.

DIF: Understanding

OBJ: 40.6

TOP: Implementation

MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:

Concepts: Elimination

14. The nurse is caring for a patient who is constipated and has not had a bowel movement for 3 days. The nurse performs a rectal examination and finds hard dry stool in the rectum. What is the best option to help the patient have a bowel movement?
- Glass of warmed prune juice
 - Loperamide (Imodium)
 - Oral fiber supplement
 - An oil retention enema

ANS: D

The patient with hard, dry stool in the rectum will benefit from an oil retention enema because it will soften the stool and make it easier to pass. Imodium is an antidiarrheal that will worsen the constipation. An oral fiber supplement and prune juice should be given after the patient has a bowel movement to prevent constipation from recurring.

DIF: Applying OBJ: 40.6 TOP: Implementation

MSC: NCLEX Client Needs Category: Physiological Integrity: Basic Care and Comfort NOT:

Concepts: Elimination

15. The nurse is caring for a patient who has just completed 2 weeks of IV antibiotics for a severe infection. The patient now has frequent loose watery stools and a low-grade temperature. What is the most likely cause of the patient's new symptoms?

- a. *Clostridium difficile* infection
- b. Paralytic ileus
- c. Fecal impaction
- d. *Salmonella* food poisoning

ANS: A

Diarrhea, abdominal pain, and low-grade temperature after completing IV antibiotics are often caused by *C. difficile* infection.

DIF: Understanding

OBJ: 40.2

TOP: Assessment MSC:

NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:

Concepts: Elimination

16. The nurse is caring for a patient who had a colonoscopy earlier that day. The patient states that he still feels very bloated after the procedure. What is the best action of the nurse?
- a. Assist the patient to ambulate in the hall.
 - b. Insert a rectal tube to remove retained flatus.
 - c. Administer an enema to stimulate peristalsis.
 - d. Encourage oral intake of fluids and high-fiber foods.

ANS: A

Ambulation is a good way to promote peristalsis and relieve bloating. An enema should not be used after colonoscopy. A rectal tube is not needed. Eating high-fiber foods soon after colonoscopy may increase gas and bloating.

DIF: Understanding

OBJ: 40.6

TOP: Implementation

MSC: NCLEX Client Needs Category: Physiological Integrity: Basic Care and Comfort NOT:

Concepts: Elimination **NURSINGTB.COM**

17. The nurse is caring for a patient with a history of dementia who is incontinent of stool because of the inability to communicate the need to defecate. What is the priority action of the nurse?
- a. Administer a daily laxative and take the patient to the toilet afterward.
 - b. Digitally remove stool from the patient's rectum every other day.
 - c. Insert a rectal tube to facilitate drainage of soft or liquid stool.
 - d. Begin a prompted toileting program to facilitate bowel continence.

ANS: D

Patients who cannot communicate the need to use the toilet often benefit from a prompted toileting program in which the patient is brought to the toilet at the same times each day to promote urinary and bowel continence. A rectal tube should not be used. Digital removal of the impaction should be avoided whenever possible. Laxatives should be used only when necessary because continued use will lead to dependence.

DIF: Applying

OBJ: 40.6

TOP: Implementation

MSC: NCLEX Client Needs Category: Physiological Integrity: Basic Care and Comfort NOT:

Concepts: Elimination

18. The nurse is caring for a patient who is recovering after hip surgery. The patient requires assistance to use the bathroom because no weight bearing is allowed on the right leg. Which goal is most important for the nurse to include for the diagnosis Impaired self-toileting?
- a. The patient will demonstrate safe transfer technique between wheelchair and toilet.
 - b. The call light will be answered promptly when the patient needs to use the toilet.

- c. Toileting will be scheduled in the morning when the patient needs to defecate.
- d. Toilet paper and handwashing items will be kept within easy reach of the patient.

ANS: A

The highest priority goal for this patient is the demonstration of safe transfer technique between the chair and the toilet. The other statements are interventions performed by staff rather than goals that will be accomplished by the patient.

DIF: Applying OBJ: 40.5 TOP: Planning

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Safety and Infection Control

NOT: Concepts: Elimination

19. The nurse is caring for a patient who is recovering from gastroenteritis. The nurse teaches the patient about dietary recommendations as the digestive system recovers. Which menu selection by the patient indicates that additional teaching is needed?
- a. Applesauce
 - b. Orange Popsicle
 - c. White toast
 - d. Coffee with cream

ANS: D

Coffee with cream should be avoided by patients recovering from gastroenteritis because milk proteins are difficult for the digestive system and caffeine increases peristalsis. Caffeine is also a diuretic, which can lead to continued dehydration.

DIF: Evaluating OBJ: 40.6 TOP: Teaching/Learning

MSC: NCLEX Client Needs Category: Physiological Integrity: Basic Care and Comfort NOT:

Concepts: Nutrition

NURSINGTB.COM

20. The nurse is caring for a patient who has had a severe stroke and requires assistance to use the toilet. Which goal is the highest priority for this patient?
- a. The patient will remain continent with no perineal skin breakdown.
 - b. The patient will state satisfaction with use of gait belt for toilet transfers.
 - c. The patient will regain ability to pull up clothing after using the toilet.
 - d. The patient will have privacy once properly positioned on the toilet.

ANS: A

The highest priority goal for this patient is continence with no perineal skin breakdown to maintain skin integrity and self-esteem. Patient statements of satisfaction and the ability to pull up clothing are important but not the priority over preventing skin breakdown. Privacy is an intervention to be performed by the staff rather than a goal for the patient.

DIF: Applying OBJ: 40.5 TOP: Planning

MSC: NCLEX Client Needs Category: Physiological Integrity: Basic Care and Comfort NOT:

Concepts: Elimination

21. A student nurse is working with a preceptor to administer an enema to the patient. Which action by the student prompts intervention and redirection by the preceptor?
- a. Water-soluble lubricant is applied to the end of the enema tubing.
 - b. The enema tubing is primed with solution that has been warmed.
 - c. The patient is positioned comfortably in the right side-lying Sims position.
 - d. The patient's bedpan is put at the bedside in preparation for use.

ANS: C

The patient should be placed in the left side-lying Sims position prior to enema administration so that the enema fluid will readily flow through the colon without having to go uphill. The other actions demonstrate correct enema administration steps.

DIF: Remembering

OBJ: 40.6

TOP: Implementation

MSC: NCLEX Client Needs Category: Physiological Integrity: Reduction of Risk Potential NOT:

Concepts: Elimination

MULTIPLE RESPONSE

1. The nurse is caring for a postoperative patient who had a colostomy placed 2 days ago. The appliance needs to be changed for the first time. Which ostomy care actions can the nurse delegate to the nursing assistant? (*Select all that apply.*)
 - a. Gently cleaning the stoma with warm water and a washcloth
 - b. Assessing the stoma and incision for signs of infection or ischemia
 - c. Obtaining needed supplies from the clean utility room
 - d. Teaching the patient how to care for the ostomy after discharge
 - e. Determining which type of ostomy appliance to use
 - f. Application of skin protectant to the area surrounding the stoma

ANS: A, C, F

The nursing assistant can gently clean the stoma with warm water and a washcloth, obtain needed supplies, and apply skin protectant. The nurse is responsible for assessment, teaching, and determining which ostomy appliance to use.

DIF: Applying

OBJ: 40.6

TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care

NOT: Concepts: Elimination

2. The nurse is caring for a patient who will be having a colonoscopy the following morning. Which items must be removed from the patient's dinner tray since they are not allowed prior to the test? (*Select all that apply.*)
 - a. Cherry-flavored gelatin
 - b. Cream of chicken soup
 - c. Glass of apple juice
 - d. Coffee with cream and sugar
 - e. Lemon-flavored Italian ice
 - f. Can of ginger ale

ANS: A, B, D

Patients who will undergo colonoscopy testing should have a clear liquid diet the day before the exam, so cream of chicken soup and coffee creamer should not be consumed. Foods with red food coloring should also be avoided prior to colonoscopy.

DIF: Applying

OBJ: 40.6

TOP: Implementation

MSC: NCLEX Client Needs Category: Physiological Integrity: Reduction of Risk Potential NOT:

Concepts: Nutrition

Chapter 41: Urinary Elimination**Yoost & Crawford: Fundamentals of Nursing: Active Learning for Collaborative Practice, 3rd Edition****MULTIPLE CHOICE**

1. The nurse is caring for a patient who is recovering from septic shock. While in the ICU, the patient developed renal failure. The nurse recognizes which type of renal failure the patient most likely developed?
 - a. Prerenal
 - b. Renal
 - c. Postrenal
 - d. Mixed

ANS: A

Prerenal failure occurs as a result of reduction in blood flow to the kidneys, which would occur with septic shock. Causes of prerenal failure include dehydration, vascular collapse, and low cardiac output. Structural issues with the kidneys, from primary glomerular diseases or vascular lesions, result in renal failure. Postrenal failure is related to a mechanical or functional obstruction of the flow of urine.

DIF: Applying OBJ: 41.2 TOP: Assessment
 MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:
 Concepts: Elimination

2. The nurse is caring for a patient with a neurological condition that causes constant severe thirst, drinking fluids continuously, and voiding 3 to 4 L of clear yellow urine daily. Which term will the nurse use in the record to describe this patient's urinary output?
 - a. Anuria
 - b. Oliguria
 - c. Polyuria
 - d. Enuresis

ANS: C

Urinary output greater than 2500 mL/day is polyuria. Insufficient urine output is oliguria, whereas absence of urine is anuria. Enuresis is commonly known as —bedwetting— at night.

DIF: Understanding OBJ: 41.2 TOP: Documentation
 MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:
 Concepts: Elimination

3. The nurse is caring for a patient who is experiencing stress incontinence. The nurse identifies which goal to be the most important for this patient?
 - a. The patient will carefully complete a voiding diary for the duration of 2 weeks.
 - b. The patient will not experience involuntary urination during coughing or sneezing.
 - c. The patient will be able to recognize and effectively manage perineal dermatitis.
 - d. The patient will demonstrate how to appropriately use urinary incontinence products.

ANS: B

The patient with stress incontinence experiences loss of urine when coughing, sneezing, laughing, or exercising. The highest priority goal for this patient is to not experience incontinence at all and remain continent through all daily activities. If the patient remains continent, perineal dermatitis will not be a problem and urinary incontinence products will not be needed.

DIF: Understanding OBJ: 41.5 TOP: Planning MSC:
 NCLEX Client Needs Category: Physiological Integrity: Basic Care and Comfort NOT:
 Concepts: Elimination

4. The nurse is caring for a postoperative patient whose urinary catheter was removed 8 hours previously. The patient has not been able to void since the catheter was removed and now reports suprapubic pain. What is the priority action of the nurse?
- Encourage oral fluid intake and administer a diuretic.
 - Obtain a urine sample to test for culture and sensitivity.
 - Calculate the patient's daily intake and output.
 - Obtain an order to straight-catheterize the patient.

ANS: D

The patient who has not voided for 6 to 8 hours after urinary catheter removal and is complaining of suprapubic pain has acute urinary retention. The physician should be notified to obtain an order for straight catheterization to drain the bladder. A urine sample for culture and sensitivity is not ordered. Encouraging fluid intake and administering a diuretic will increase the amount of urine in the bladder and make the patient even more uncomfortable.

DIF: Applying OBJ: 41.6 TOP: Implementation
 MSC: NCLEX Client Needs Category: Physiological Integrity: Reduction of Risk Potential NOT:
 Concepts: Elimination [NURSINGTB.COM](https://www.nursingtb.com)

5. The nurse is caring for a patient who recently underwent ileal conduit surgery. Which nursing diagnosis is the highest priority for this patient?
- Impaired sexual function related to changed body structure
 - Social isolation related to potential for accidental leakage of urine
 - Lack of knowledge related to care and maintenance of ostomy appliance
 - Disturbed body image related to presence of stoma and appliance

ANS: C

The patient with a new ileal conduit needs to learn how to care for the urinary stoma and appliance prior to discharge from the hospital. If the appliance is not used and applied correctly, the patient may experience urinary leakage and significant skin breakdown from exposure to urine. The other diagnoses are less important than the patient's lack of knowledge about ostomy care.

DIF: Understanding OBJ: 41.4 TOP: Diagnosis
 MSC: NCLEX Client Needs Category: Health Promotion and Maintenance
 NOT: Concepts: Patient Education

6. The nurse is caring for a patient who reports an urgent need to urinate but is unable to pass more than a few drops of urine in the toilet. Which is the priority assessment to be performed by the nurse?
- Bladder scan to determine the amount of urine in the bladder

- b. Auscultation to assess circulation through the right and left renal arteries
- c. Bimanual palpation to assess for possible enlargement of the kidneys
- d. Calculate the patient's intake and output to check for fluid volume deficit

ANS: A

The patient with suspected urinary retention should have a bladder scan performed to determine the amount of urine in the bladder. If a significant amount of urine is found in the bladder, the provider may be notified to obtain an order for straight catheterization.

DIF: Applying

OBJ: 41.3

TOP: Assessment

MSC: NCLEX Client Needs Category: Physiological Integrity: Reduction of Risk Potential NOT:

Concepts: Elimination

7. The nurse is caring for a seriously ill patient whose laboratory results show a serum creatinine level of 3.5 mg/dL and a serum BUN of 35 mg/dL. Which conclusion can the nurse draw from these test results?
- a. The patient is severely dehydrated.
 - b. The patient's kidneys have been damaged.
 - c. The patient has a urinary tract infection.
 - d. The patient has developed a renal calculus.

ANS: B

Elevated BUN and creatinine are found in laboratory test results when the kidneys have been damaged and are unable to sufficiently clear metabolic wastes from the bloodstream. A dehydrated patient may have an elevated BUN, but the serum creatinine should be normal.

Urinary tract infection and kidney stone (renal calculus) would not cause elevated BUN and creatinine levels.

DIF: Understanding

NURSINGTB.COM

OBJ: 41.2

TOP: Assessment

MSC: NCLEX Client Needs Category: Physiological Integrity: Reduction of Risk Potential

NOT: Concepts: Elimination

8. The nurse is caring for a patient who has developed kidney failure. Which test finding leads the nurse to contact the nephrologist and arrange for emergency hemodialysis?
- a. Potassium level 6.8 mmol/L
 - b. Serum creatinine level of 2.8 mg/dL
 - c. Large amounts of protein in the urine
 - d. 1500 mL of retained urine in the bladder

ANS: A

Patients in renal failure often require dialysis to reduce serum potassium levels to less than 5.5 mmol/L. Critically high serum potassium levels can lead to lethal arrhythmias and must be corrected promptly. Patients with advanced renal failure may require emergency hemodialysis if the potassium level does not lower with other methods (insulin and 50% dextrose, kayexalate). An elevated creatinine is consistent with kidney dysfunction. Large amounts of protein in the urine occurs in some diseases. 1500 mL of retained urine requires straight catheterization.

DIF: Understanding

OBJ: 41.2

TOP: Assessment MSC:

NCLEX Client Needs Category: Physiological Integrity: Reduction of Risk Potential NOT:

Concepts: Elimination

9. The nurse is caring for a patient who will undergo ultrasound testing of the bladder and kidneys the next morning. Which instruction will the nurse provide to the patient about the test?
- A small IV will be inserted into your arm to inject the contrast dye.||
 - You will need to drink lots of water but not use the toilet.||
 - You should not have anything to eat or drink after midnight.||
 - You will receive a cleansing enema before you have the test.||

ANS: B

No preparation is needed for kidney and bladder ultrasound other than having the patient drink lots of fluid beforehand. The patient is instructed not to use the toilet so that the bladder will be filled and easy to visualize. No contrast dye, enemas, or fasting is required.

DIF: Understanding

OBJ: 41.6

TOP: Teaching/Learning

MSC: NCLEX Client Needs Category: Physiological Integrity: Reduction of Risk Potential NOT:

Concepts: Patient Education

10. The nurse is caring for a patient who has urinary retention resulting from benign prostatic hyperplasia (BPH). The patient requires catheterization in order to drain the urine from his bladder. Which action will the nurse take to facilitate this procedure?
- Obtain a Coudé catheter for insertion.
 - Attach a leg bag to the catheter prior to insertion.
 - Trim the pubic hair before cleaning the perineal area.
 - Wait until the bladder is full to perform catheterization.

ANS: A

A Coudé catheter is used when there is narrowing or constriction of the urethra, making insertion of a regular indwelling catheter difficult. The Coudé catheter has a special tip on the end that is designed to facilitate insertion of the catheter through the narrowed urethra caused by BPH. Coudé catheters may need to be placed using a metal wire introducer. Placement using an introducer typically is performed by a provider or the patient's urologist, to avoid damaging urethral tissue. Trimming the pubic hair will not facilitate catheterization. Attaching a leg bag to the catheter prior to insertion is not needed because a bedside collection bag will usually be used at first.

DIF: Applying

OBJ: 41.6

TOP: Implementation

MSC: NCLEX Client Needs Category: Physiological Integrity: Reduction of Risk Potential NOT:

Concepts: Elimination

11. The nurse is caring for an incontinent male patient who has a deep decubitus ulcer on his sacrum. Which intervention will best manage the patient's urinary incontinence and facilitate healing of the ulcer?
- Use of disposable absorbable incontinence briefs
 - Daily application of perineal barrier cream containing zinc oxide
 - Careful perineal care and application of a condom catheter
 - Insertion of a single-lumen straight urinary catheter

ANS: C

Condom catheters allow for collection of urine in the incontinent patient without the infection risks of an indwelling catheter. The condom catheter is applied to the outside of the penis like a condom instead of being inserted into the urethra. Careful perineal care is performed prior to application of the condom catheter and regularly thereafter. Use of disposable briefs or perineal barrier cream will not facilitate healing of the sacral ulcer. A single-lumen straight urinary catheter is used to drain the bladder to relieve urinary retention or to obtain a urine sample for testing. A straight catheter is not used for management of incontinence.

DIF: Applying OBJ: 41.6 TOP: Implementation
 MSC: NCLEX Client Needs Category: Physiological Integrity: Reduction of Risk Potential NOT:
 Concepts: Elimination

12. The nurse is caring for a patient who has urinary frequency. Which nursing diagnosis is the highest priority for this patient?
- Impaired urination r/t occasional incontinence
 - Anxiety r/t living alone at home with nocturia
 - Risk for infection r/t urine contact with perineal area skin
 - Risk for fall-related injury r/t hurried trips to the bathroom during the day and night

ANS: D

Risk for falls is the highest priority diagnosis for this patient because rushing to the bathroom can lead to loss of balance and serious injury. Walking to the bathroom at night is even more dangerous because of low lighting conditions and sleepiness. The other nursing diagnosis may be appropriate but not higher than the injury risk.

DIF: Applying OBJ: 41.4 TOP: Diagnosis
 MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Safety and Infection Control
 NOT: Concepts: Safety

13. The nurse is caring for a patient who has just had an intravenous pyelography (IVP) completed. Which assessment is the nurse's highest priority after the patient returns from the test?
- Calculate the patient's intake and output.
 - Monitor for discoloration of the patient's urine.
 - Assess for possible iodine or shellfish allergies.
 - Inquire if the patient has burning or pain with urination.

ANS: A

The nurse must carefully monitor the patient's intake and output after IVP testing to ensure that the patient's kidneys were not damaged by the contrast dye. PO fluid intake should be encouraged to facilitate excretion of the contrast dye. Urine is not discolored from the IVP. Burning or pain with urination should not occur after IVP testing because there is no instrumentation of the urinary tract. Assessment of allergies must be done before the IVP is done because iodine-based contrast is used.

DIF: Applying OBJ: 41.3 TOP: Assessment
 MSC: NCLEX Client Needs Category: Physiological Integrity: Reduction of Risk Potential NOT:
 Concepts: Elimination

14. The nurse is caring for a patient with benign prostatic hypertrophy who states that he feels a constant urge to urinate but cannot pass more than 30 to 60 mL of urine at a time. The nurse performs a bladder scan and finds that there are 1100 mL of urine in the patient's bladder. What is the priority nursing diagnosis for this patient?
- Anxiety r/t continual urge to urinate
 - Reflex incontinence of urine r/t over-distention of the bladder
 - Impaired urination r/t obstruction of urinary bladder outlet
 - Impaired self-toileting r/t inability to pass urine into the toilet

ANS: C

The patient has acute urinary retention with overflow as evidenced by 1100 mL of urine in the bladder and frequent passage of small amounts of urine. The priority nursing diagnosis is thus Impaired urination r/t obstruction of urinary bladder outlet. Urinary retention is the cause of the patient's discomfort and drainage of the bladder will result in relief of the patient's symptoms. The patient is able to get himself on and off the toilet so toileting self-care deficit is not a problem. Reflex incontinence of urine r/t over-distention of the bladder is not as specific to this scenario as the nursing diagnosis of impaired urination.

DIF: Applying OBJ: 41.4 TOP: Diagnosis
 MSC: NCLEX Client Needs Category: Physiological Integrity: Physiological Adaptation NOT:
 Concepts: Elimination

15. The nurse is caring for a patient who had prostate surgery the previous day. The patient has had significantly decreased urine output over the last shift despite ample oral and IV fluid intake. The patient's urine from the indwelling catheter is cherry red with occasional small clots. What is the appropriate action of the nurse?
- Remove the urinary catheter and replace it with a new one.
 - Gently irrigate the catheter using warmed sterile normal saline.
 - Send a sample of the patient's urine to the laboratory for analysis.
 - Call the provider and obtain an order for kidney and bladder ultrasound.

ANS: B

The patient most likely has decreased urine output caused by clot formation that is blocking urine from draining through the catheter. The catheter should be gently irrigated using sterile technique and warmed sterile saline to loosen clots and facilitate urinary drainage. The catheter should not be removed. Ultrasound and urinalysis are not necessary.

DIF: Applying OBJ: 41.6 TOP: Implementation
 MSC: NCLEX Client Needs Category: Physiological Integrity: Reduction of Risk Potential NOT:
 Concepts: Elimination

16. The nurse is caring for a patient with the nursing diagnosis of Urge incontinence of urine related to urinary tract infection. Which statement is appropriate for the —as evidenced by portion of the patient's diagnosis?
- Sudden leakage of urine when patient is unable to get to the toilet in time
 - Continuous urine flow from the bladder regardless of attempts to use the toilet
 - Leakage of urine from the bladder when the patient coughs, sneezes, or laughs
 - Leakage of urine because the patient is unable to indicate need to use the toilet

ANS: A

Urge incontinence of urine occurs when the patient has a sudden need to urinate but cannot get to the toilet in time. Continuous flow of urine is deemed total urinary incontinence. Leakage of urine when sneezing or coughing is stress incontinence. Functional incontinence occurs when the patient cannot indicate need to use the toilet.

DIF: Applying OBJ: 41.4 TOP: Diagnosis
 MSC: NCLEX Client Needs Category: Physiological Integrity: Basic Care and Comfort NOT:
 Concepts: Elimination

17. The nurse is caring for a patient with an indwelling urinary catheter caused by severe prostate enlargement. Which is the priority nursing diagnosis for this patient?
- Risk for infection r/t indwelling urinary catheter
 - Disturbed body image r/t presence of catheter
 - Risk for contamination r/t potential leakage of urine on clothing
 - Impaired urination r/t blockage of bladder outlet

ANS: A

The presence of an indwelling urinary catheter puts the patient at high risk for urinary tract infection, and this is the highest priority diagnosis for the patient. Disturbed body image is not as important as the risk of infection. Risk for contamination is not a nursing diagnosis.

Impaired urination was corrected by placement of the urinary catheter.

DIF: Applying OBJ: 41.4 TOP: Diagnosis
 MSC: NCLEX Client Needs Category: Physiological Integrity: Reduction of Risk Potential NOT:
 Concepts: Elimination

18. The preceptor is watching a nursing student care for a male patient who requires a condom catheter. Which action by the nursing student indicates that the procedure is performed correctly?
- Sterile gloves are donned before touching the catheter.
 - Adhesive tape is applied securely around the base of the penis.
 - Water-soluble lubricant is applied to the end of the catheter.
 - The foreskin is returned to its natural position before the catheter is applied.

ANS: D

The patient's penis should be cleaned with soap and water with the foreskin retracted prior to condom catheter application. The foreskin should then be returned to its natural position before the catheter is applied. Adhesive tape should never be applied around the base of the penis because circulation may be compromised. Sterile gloves and lubricant are not needed.

DIF: Understanding OBJ: 41.6 TOP: Implementation
 MSC: NCLEX Client Needs Category: Physiological Integrity: Basic Care and Comfort NOT:
 Concepts: Elimination

19. The nurse is caring for a patient with a history of type 1 diabetes. Which assessment finding indicates to the nurse that the patient may not be compliant with the diabetic treatment regimen?
- The patient is always thirsty and frequently voids very large amounts of urine.
 - The patient's urine is very concentrated with a dark amber color.
 - The patient complains of throbbing flank pain and burning with urination.
 - The patient has urinary hesitancy and difficulty initiating a stream of urine.

ANS: A

A noncompliant diabetic patient will have elevated blood sugars that cause thirst and polyuria. Concentrated urine indicates dehydration. Throbbing flank pain and burning with urination are indicative of urinary tract infection. Urinary hesitancy and difficulty initiating urine stream are not indicative of elevated blood sugar levels.

DIF: Applying OBJ: 41.3 TOP: Assessment

MSC: NCLEX Client Needs Category: Physiological Integrity: Reduction of Risk Potential NOT:

Concepts: Elimination

20. The nurse is caring for a patient who is to undergo computed tomography (CT) of the kidneys and ureters. Which assessment finding by the nurse must be reported to the provider and radiologist before the patient has the procedure?
- The patient is allergic to bananas and latex.
 - The patient thinks that she might be pregnant.
 - The patient has a family history of bladder cancer.
 - The patient currently has a urinary tract infection.

ANS: B

CT requires exposure to radiation similar to an x-ray, so the patient's provider and radiologist should be notified promptly of the possibility of pregnancy. The other conditions do not preclude CT scan examination for the patient.

DIF: Understanding OBJ: 41.3 TOP: Assessment MSC:

NCLEX Client Needs Category: Physiological Integrity: Reduction of Risk Potential NOT:

Concepts: Safety

21. The nurse is caring for an elderly patient with a history of arthritis, urinary incontinence and poor perineal hygiene practices. The patient has had four urinary tract infections in the past year. Which is the priority goal for the nursing diagnosis Impaired health maintenance for this patient?
- The patient will be provided with educational materials about risks of urosepsis.
 - The patient will allow family members to assist with daily bathing and perineal care.
 - The patient will discuss the possible consequences of frequent UTIs.
 - Regular home care nursing visits and follow-up telephone contact will be arranged.

ANS: B

The priority for this patient is to improve personal hygiene and perineal care in order to reduce the risk of future urinary tract infections. The patient's agreement to allow family members to assist with bathing and perineal care will greatly reduce this risk. Providing educational materials about the risk of urosepsis, discussion of UTI consequences, and regular follow-up care are interventions rather than patient goals.

DIF: Applying OBJ: 41.5 TOP: Planning

MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT:

Concepts: Caregiving

MULTIPLE RESPONSE

1. The nurse is caring for an elderly patient whose dementia has become worse over the last 24 hours. The nurse suspects that the patient may have developed a urinary tract infection and obtains a urine sample. Which assessment findings prompt the nurse to contact the provider to obtain an order for urine culture and sensitivity testing? (*Select all that apply.*)
- Urinary dipstick testing is positive for nitrates.
 - The urine appears cloudy with a foul odor.
 - The urine is concentrated and dark amber in color.
 - The urine smells faintly like sweet fruit.
 - The patient is urinating more frequently than usual.
 - The patient is normally continent but has been incontinent twice.

ANS: A, B, E, F

Concentrated dark urine indicates dehydration rather than infection of the urinary tract. Urine that smells of sweet fruit contains ketones from high blood sugar. Urine that is cloudy with a foul odor and positive for nitrites is most likely due to urinary tract infection. Frequent urination and incontinence are signs of urinary tract infection in the elderly.

DIF: Understanding OBJ: 41.3 TOP: Assessment MSC: NCLEX Client Needs Category: Physiological Integrity: Reduction of Risk Potential NOT: Concepts: Elimination

2. The nurse is caring for a male patient who will be performing intermittent self-catheterization at home. Which actions by the patient indicate the need for additional teaching about this procedure? (*Select all that apply.*)
- Patency of the balloon is tested prior to insertion of the catheter.
 - The catheter is inserted another 2 inches after urine is seen in the tubing.
 - The catheter is carefully secured to the leg to prevent accidental removal.
 - The foreskin is returned to its natural position after the catheter is removed.
 - Catheterization is performed regularly before the bladder becomes distended.
 - Water-soluble lubricant is generously applied along the length of the catheter.

ANS: A, C, F

Only 5 to 8 inches of the catheter tip are covered with water-soluble lubricant. Patency of the balloon is only checked when indwelling catheters are inserted. Intermittent catheters need not be secured to the patient's leg because they will be removed after the bladder is drained. The other actions are correct.

DIF: Understanding OBJ: 41.6 TOP: Teaching/Learning MSC: NCLEX Client Needs Category: Health Promotion and Maintenance NOT: Concepts: Elimination

3. The nurse is working with a new nursing assistant who is providing care to patients with urinary difficulties. Which actions by the nursing assistant indicates that additional teaching is required? (*Select all that apply.*)
- The length of the urinary catheter is cleaned up to the patient's perineum.
 - A urine sample is obtained from the drainage bag immediately after catheter insertion.
 - A fresh condom catheter is applied every other day following careful perineal care.
 - Zinc oxide barrier cream is applied liberally to the perineal area for incontinent patients.
 - The catheter drainage bag is disconnected in order to put pants on the patient.

- f. Clean technique is used to obtain a urine specimen for culture and sensitivity from the catheter.

ANS: A, C, E, F

The urinary catheter must be cleaned from the urinary meatus down toward the drainage bag rather than up toward the perineum. A fresh condom catheter must be applied daily. The catheter drainage bag should not be disconnected to put pants on the patient. The drainage bag can be threaded through the pants leg before putting pants on the patient. Sterile technique should be used to obtain samples from the catheter.

DIF: Understanding

OBJ: 41.6

TOP: Teaching/Learning

MSC: NCLEX Client Needs Category: Physiological Integrity: Reduction of Risk Potential NOT:

Concepts: Elimination

4. The nurse is caring for a patient who is to complete a 24-hour urine collection to measure creatinine clearance. Which tasks related to this test may be delegated to the nursing assistant? *(Select all that apply.)*
- a. Teaching the patient about sterile specimen collection
 - b. Keeping the urine collection container cool on ice
 - c. Dumping the urine from the patient's first void
 - d. Restricting the patient's oral fluid intake during the test
 - e. Transporting the specimen to the laboratory for testing
 - f. Reminding the patient not to put toilet paper in the urine

ANS: B, C, E, F

The nurse assistant can help the nurse by keeping the urine collection container cool on ice, dumping the urine from the patient's first void, and reminding the patient not to put toilet tissue in the urine specimen. The nurse assistant can also transport the specimen to the laboratory after the urine has been collected for 24 hours. Fluid intake should be encouraged during the test. Teaching the patient about the testing procedure is done by the nurse, although creatinine clearance testing does not require sterile technique.

DIF: Applying

OBJ: 41.6

TOP: Implementation

MSC: NCLEX Client Needs Category: Safe and Effective Care Environment: Management of Care NOT:

Concepts: Elimination

Chapter 42: Death and Loss**Yoost & Crawford: Fundamentals of Nursing: Active Learning for Collaborative Practice, 3rd Edition****MULTIPLE CHOICE**

1. The hospice nurse is caring for a terminally ill patient. The patient's son is distraught because the patient will probably die within the next few days and there is nothing he can do about it. What is the most appropriate nursing diagnosis for the patient's son currently?
 - a. Chronic grief related to impending death of mother
 - b. Death anxiety related to feeling powerless over situation
 - c. Powerlessness related to progression of mother's terminal illness
 - d. Complicated grieving related to desired avoidance of mourning

ANS: B

The patient's son is experiencing death anxiety because he is unable to change the outcome of his mother's imminent death. The son makes no mention of religious beliefs, so impaired religiosity is not appropriate. Complicated grieving is applicable to individuals who have recently experienced a loss. Chronic grief is grief that continues for a long period of time.

DIF: Applying OBJ: 42.5 TOP: Diagnosis
 MSC: NCLEX Client Needs Category: Psychosocial Integrity: Grief and Loss NOT:
 Concepts: Coping

2. The nurse is caring for a terminally ill patient whose children have come home to be with their mother during her last few days. They spend time looking through picture albums, watching old home movies, and remembering fun times spent together. The nurse identifies which term that best describes the activity of the patient's children?
 - a. Anticipatory grieving
 - b. Bereavement
 - c. Caregiver role strain
 - d. Death anxiety

ANS: A

The patient and her children are experiencing anticipatory grief as they prepare for the expected death of the patient. Reminiscence and life review are used to assist those experiencing anticipatory grief with the realization that death is approaching.

DIF: Applying OBJ: 42.1 TOP: Assessment
 MSC: NCLEX Client Needs Category: Psychosocial Integrity: Grief and Loss NOT:
 Concepts: Family Dynamics

3. The nurse is caring for a female patient who died a few minutes previously. The patient's family comes in to the room and immediately starts to wash the body in preparation for burial. What is the most appropriate action of the nurse currently?
 - a. Inform the patient's family that the body must be transported to the morgue.
 - b. Instruct the patient's family that hospital staff will provide postmortem care.
 - c. Obtain needed signatures for organ donation and autopsy.
 - d. Offer to provide any needed supplies and provide privacy for the family.

ANS: D

The most appropriate action of the nurse currently is to allow the family to wash the patient's body in accordance with their wishes and cultural values. The family may wish to participate in this procedure or may complete this procedure in private. Health care personnel should abide by their wishes as much as possible. Signatures may be obtained from the next of kin when washing is complete. The patient's body may be transported to the morgue or funeral home after washing is completed.

DIF: Applying OBJ: 42.7 TOP: Implementation
 MSC: NCLEX Client Needs Category: Psychosocial Integrity: End of Life Care NOT:
 Concepts: Family Dynamics

4. The nurse is caring for an emergency room patient who died because of a mishap with a loaded gun. The patient's body will be transported to the coroner's office for an autopsy. Which items will the nursing staff remove from the body before it leaves the hospital?
- Endotracheal tube
 - Foley catheter and IV line
 - Dentures
 - Necklace and watch

ANS: D

Medical devices and tubes are not removed from the body if an autopsy is to be performed. The patient's necklace and watch may be removed and given to the patient's family members before the body is transported to the coroner's office for autopsy. Dentures should be left in the patient's mouth.

DIF: Understanding OBJ: 42.7 TOP: Implementation
 MSC: NCLEX Client Needs Category: Psychosocial Integrity: End of Life Care
 NOT: Concepts: Health Care Law

5. The nurse is caring for a patient who suffered a miscarriage at 24 weeks of pregnancy. The patient is devastated by the loss but her husband minimizes her grief by stating, —Quit crying. It's not like you lost a real baby. What term best describes the anguish felt by the patient?
- Disenfranchised grief
 - Delayed grief
 - Moral distress
 - Masked grief

ANS: A

The patient is experiencing disenfranchised grief because she cannot share the pain of her loss with her husband. The husband is not willing to support his wife as she mourns the loss of her pregnancy or recognize the grief that she is going through. Delayed grief is suppression of the grief process. Moral distress occurs when people cannot act according to their moral values. Masked grief occurs when a person's bereavement behaviors interfere with his or her life, but the person does not notice this.

DIF: Applying OBJ: 42.2 TOP: Assessment
 MSC: NCLEX Client Needs Category: Psychosocial Integrity: Grief and Loss NOT:
 Concepts: Coping

6. A patient has recently been given a terminal diagnosis. When family members offer to help, the patient snaps and yells at them, but then angrily accuses them of not helping. The patient's spouse is frustrated and asks the hospice nurse what to do about this situation. What response by the nurse is best?
- Don't worry. Your spouse will get over this phase soon.¶
 - Anger is an expected part of the grieving process.¶
 - Would your spouse be open to professional counseling?¶
 - This diagnosis is difficult to handle; just be patient.¶

ANS: B

Anger is one of the stages of grief as identified by Elizabeth Kubler-Ross. The nurse would first explain this to the spouse. Telling the spouse the patient will get over the phase soon or that the diagnosis is difficult to handle is false reassurance and dismissive of the concerns. It is too early to consider counseling although the patient may need it later. This is also a yes/no question which is not therapeutic.

DIF: Applying OBJ: 42.3 TOP: Implementation
 MSC: NCLEX Client Needs Category: Psychosocial Integrity: Grief and Loss NOT:
 Concepts: Coping

7. The nurse is caring for a terminally ill patient who appears to be calmly having a conversation with someone even though there is nobody else in the room. The patient reaches out and appears to take something out of thin air and hold it close. Which is the appropriate action of the nurse?
- Reorient the patient and reassure that nobody else is in the room.
 - Be present but quiet and let the patient continue the conversation.
 - Carefully assess the patient's mental status and level of attention.
 - Obtain a set of vital signs and check the patient's pulse oximetry.

ANS: B

Patients who are near death sometimes have a special communication with loved ones who have already died. It is important to recognize that these experiences can be comforting to the dying patient, and nurses would not contradict or argue with the person. It is imperative to simply be present with the person, listen, and be open to any attempts to communicate. It is acceptable to ask gentle questions such as —What are you seeing?¶ or —How does that make you feel?¶ Having an open discussion with the family while describing what is occurring may provide further insight to the nurse as the health care provider, as well as promoting a sense of understanding and acceptance for the family. As long as the patient is calm and content, the best action of the nurse is to be present but let the patient continue the conversation undisturbed.

DIF: Applying OBJ: 42.7 TOP: Implementation
 MSC: NCLEX Client Needs Category: Psychosocial Integrity: End of Life Care NOT:
 Concepts: Palliation

8. The nurse is caring for a patient who died a few minutes ago. The patient's family is at the bedside and very demonstrative in their grief, weeping loudly and holding on to the patient's body. What is the most appropriate action of the nurse?
- Inform the family that the patient's body must be taken to the morgue shortly.
 - Ask the family members to step outside while postmortem care is provided.
 - Obtain required signatures for the body to be taken to the funeral home.

- d. Provide privacy and allow the patient's family to grieve over the body.

ANS: D

The nurse should allow the patient's family to grieve in private over the loss of their loved one. Some cultures favor free expression of emotions after death, and the nurse should respect this. Signatures can be obtained, postmortem care can be provided, and the body brought to the morgue after an appropriate time of grieving has been provided to the family.

DIF: Applying OBJ: 42.7 TOP: Implementation

MSC: NCLEX Client Needs Category: Psychosocial Integrity: Grief and Loss NOT:

Concepts: Family Dynamics

9. The nurse is caring for a patient who lost her husband 1 year ago after 55 years of marriage. The patient no longer takes care of herself or cooks and rarely eats, stating she has no appetite. The nurse determines that the Nursing diagnosis of complicated grieving applies to the patient. Which is the priority goal for the patient?
- The patient will shower every other day and eat at least two meals a day.
 - The patient will identify personal strengths that will increase coping ability.
 - The patient will discuss the meaning of her loss with a family member or friend.
 - The patient will be provided with phone numbers for local community resources.

ANS: A

The highest priority goal of this patient is self-care including showering and eating in order to protect her health and safety. The other goals are lower priority after the patient's necessary activities of daily living are addressed. Goals should also reflect what the patient accomplishes; so the goal of being provided with phone numbers is actually something for the nurse to do.

DIF: Applying OBJ: 42.6 TOP: Planning

MSC: NCLEX Client Needs Category: Psychosocial Integrity: Grief and Loss NOT:

Concepts: Coping

10. The hospice nurse is caring for a several adult children shortly after the death of a parent. They have various reactions as they deal with their loss. The nurse recognizes which reactions to be in the cognitive domain?
- They let the house get filthy because they can't be bothered to clean it.
 - They are tossing and turning all night and are unable to get a good night's sleep.
 - They are easily distracted and often lose train of thought during conversation.
 - They have lost their appetites and have no desire to eat anything.

ANS: C

Cognitive deficits include the inability to concentrate and follow a conversation. Letting the house get filthy is a sign of apathy, which is in the behavioral domain. Insomnia falls within the behavioral and physical domains. Loss of appetite is within the physical domain.

DIF: Applying OBJ: 42.1 TOP: Assessment

MSC: NCLEX Client Needs Category: Psychosocial Integrity: Grief and Loss NOT:

Concepts: Coping

11. The hospice nurse is caring for a patient who is terminally ill. The patient's spouse is the primary caregiver, providing constant care and spending all his or her time meeting the patient's needs. The spouse says to the nurse —After my spouse dies, I will finally get that colonoscopy my provider has been bugging me about. What does the nurse understand about this statement?
- The spouse is looking forward to being freed from the caretaker role.
 - The spouse has neglected his or her own physical needs for too long.
 - The spouse is making some realistic plans for life after the death.
 - The spouse is in denial that the patient is dying and the important role of caregiver will end.

ANS: C

Often caregivers neglect their own needs while in the caregiver role. The spouse understands the patient will die soon and is being realistic in understanding his or her own physical needs have been neglected. This shows healthy coping.

DIF: Understanding

OBJ: 42.3

TOP: Assessment

MSC: NCLEX Client Needs Category: Psychosocial Integrity: End of Life Care NOT:

Concepts: Coping

12. The nurse is caring for a patient who has just died in a motor vehicle accident. What is the priority action of the nurse before the patient's family arrives to see the patient's body?
- Gently wash the body and provide perineal care.
 - Remove the patient's dentures and jewelry.
 - Ensure that the death certificate has been signed.
 - Determine which funeral home will pick up the body.

ANS: A

NURSINGTB.COM

Release of bowel and bladder contents often occur at the time of death, and the perineal care is a priority before the family arrives. The body should be gently cleaned to remove blood and debris from the accident. The patient's dentures and jewelry should not be removed from the body. The death certificate does not need to be signed before the family arrives. The family can decide which funeral home will be used and notify the nurse after their arrival.

DIF: Applying

OBJ: 42.7

TOP: Implementation

MSC: NCLEX Client Needs Category: Psychosocial Integrity: End of Life Care NOT:

Concepts: Caregiving

13. The nurse is caring for a patient who is terminally ill with metastatic bone cancer. The patient tells the nurse that he is not afraid of death but does not want to be in pain and suffer before he dies. Which intervention by the nurse will be most appropriate to meet this patient's wishes?
- Establish around-the-clock dosing for pain medications with additional doses for breakthrough pain.
 - Assist the patient to reminisce and review his life, spending as much time as possible with loved ones.
 - Use therapeutic touch, guided imagery, and soft music to put the patient at ease and relieve anxiety.
 - Encourage the patient to participate in prayer and meditation along with preferred religious practices.

ANS: A

The patient's primary wish is to die without pain, and the best intervention to meet this goal is administration of pain medication around the clock with extra doses for breakthrough pain. The other interventions may make the patient more comfortable but will not address his primary desire for adequate pain management.

DIF: Applying OBJ: 42.7 TOP: Implementation
 MSC: NCLEX Client Needs Category: Psychosocial Integrity: End of Life Care NOT:
 Concepts: Pain

14. The home care nurse is caring for a terminally ill patient who states that he wants to set up a scholarship in his name at the local university before he dies. What is the best action by the nurse?
- Suggest that the patient think it over and wait a few days before contacting the school.
 - Direct the patient to ask his family about the possibility of starting a scholarship.
 - Assess the patient's mental status to ensure that he is competent to make the decision.
 - Assist the patient to find the necessary information about endowed scholarships.

ANS: D

As the patient's advocate, the nurse should help provide the necessary information for the patient to set up a scholarship if that is his decision. The patient does not need to discuss the subject with his family first, and assessment of the patient's mental status is not needed. The patient may not have the time to wait a few days before contacting the university.

DIF: Applying OBJ: 42.7 TOP: Implementation
 MSC: NCLEX Client Needs Category: Psychosocial Integrity: End of Life Care NOT:
 Concepts: Palliation

NURSINGTB.COM

15. Which statement by the patient indicates to the nurse that it may be an appropriate time to consider hospice care rather than further aggressive measures to treat his terminal illness?
- I am praying every day that this last round of chemotherapy will work.||
 - I want to spend what time I have left at home with my grandchildren.||
 - I need to meet with my financial planner to make sure my life insurance is all set.||
 - I am concerned that my wife won't be able to live on her own after my death.||

ANS: B

Hospice care is provided to patients who are terminally ill and wish to have no further aggressive treatment in attempt to cure the disease. The patient's statement that she just wants to be home with her grandchildren indicates a readiness for hospice care.

DIF: Understanding OBJ: 42.7 TOP: Assessment
 MSC: NCLEX Client Needs Category: Psychosocial Integrity: End of Life Care NOT:
 Concepts: Palliation

16. The hospice nurse is caring for a terminally ill patient who will probably die within the next hour or two. The patient's daughter is keeping a vigil by the bedside and asks what she can do to help her father at this time. What is the appropriate response of the nurse?
- Just let him know you are here, talk to him, and let him know that you love him.||
 - You can try to feed him a few bites of ice cream to keep his mouth from getting

dry.¶

- c. —You can take this time to ensure that arrangements are set with the funeral home.¶
- d. —You should let me know when your father's breathing pattern changes.¶

ANS: A

The patient's daughter should be encouraged to spend the last moments of her father's life with him, reassuring him with her presence. The daughter should be encouraged to continue talking with him because the patient may still hear her even if his eyes are closed and he does not speak. The nurse is responsible for monitoring the patient for breathing changes. Oral intake will lead to nausea and/or aspiration. This is not the time to make arrangements with the funeral home.

DIF: Applying OBJ: 42.7 TOP: Caring
 MSC: NCLEX Client Needs Category: Psychosocial Integrity: End of Life Care NOT:
 Concepts: Palliation

17. The nurse is caring for an Islamic patient who has just died. The family is traveling from overseas. Which action is the priority for the nurse to take right after the patient dies?
- a. Arranging for embalming to preserve the body until burial
 - b. Rearrange the furniture so the bed can face Mecca
 - c. Arranging for transportation of the body to the crematorium
 - d. Bringing in fruit for the patient's journey to the other world

ANS: B

After death, a patient's body can be turned to face Mecca which is the holy site for Muslims. The nurse would need to find out which direction that is. The family will work with the funeral home to determine when and where burial will take place. Buddhists often bring fruit when someone dies.

NURSINGTB.COM

DIF: Applying OBJ: 42.7 TOP: Implementation
 MSC: NCLEX Client Needs Category: Psychosocial Integrity: End of Life Care NOT:
 Concepts: Caregiving

18. The nurse is caring for a patient whose mother recently passed away. The patient states that she has not been able to concentrate or sleep since the funeral and is consuming increasing amounts of alcohol to get through each day. The nurse knows which goal to be most appropriate for this patient?
- a. The patient will be referred to medical social services for evaluation and counseling.
 - b. The patient will be encouraged to describe previous stressors and coping mechanisms.
 - c. Nursing staff support patient's coping attempts and encourage verbalization of feelings.
 - d. The patient will use effective coping strategies with no alcohol consumption.

ANS: D

Goals are met by the patient rather than nursing or medical staff. The patient's use of effective coping strategies without drinking alcohol is an appropriate goal. Referring the patient for counseling and encouraging the patient to verbalize stressors are interventions rather than goals.

DIF: Applying OBJ: 42.6 TOP: Planning

MSC: NCLEX Client Needs Category: Psychosocial Integrity: Grief and Loss NOT:
Concepts: Coping

19. The nurse sees a young child in the clinic whose mother has only a few weeks to live. The child has been misbehaving at school recently and is suspended after picking fights with other students and defying teachers. The nurse identifies which stage of grieving that the patient is experiencing?
- Denial
 - Anger
 - Bargaining
 - Depression

ANS: B

The patient is angry over the impending death of the mother and is acting out this anger at school by picking fights and defying his teachers. Denial is a temporary defense while processing the information. Bargaining is negotiation to change the predicted outcome.

Depression includes crying and sadness.

DIF: Understanding OBJ: 42.1 TOP: Assessment
MSC: NCLEX Client Needs Category: Psychosocial Integrity: Grief and Loss NOT:
Concepts: Coping

20. The nurse is caring for a terminally ill patient whose family is insistent that additional chemotherapy be administered even though the patient will most likely die within the next few days. What is the best response of the nurse?
- The insurance company will not pay for chemotherapy at this stage.¶
 - The focus right now needs to be on keeping your loved one comfortable.¶
 - I will call the provider and relay your wishes.¶
 - The patient needs to get stronger first before chemotherapy can be administered.¶

ANS: B

Nurses advocate for patients to ensure that they are aware of their options for care that include interventions, treatments, anticipated outcomes, as well as risk and benefits of any decision made concerning medical care. The nurse must function as the patient's advocate and encourage what is in the best interest of the patient. Chemotherapy will not extend the patient's life when death is expected within the next few days and will only make the patient suffer needlessly when it is administered. The patient will not get stronger over the next few days, and this criterion for chemotherapy will never be met.

DIF: Applying OBJ: 42.7 TOP: Caring
MSC: NCLEX Client Needs Category: Psychosocial Integrity: End of Life Care NOT:
Concepts: Palliation

21. The nurse is caring for a patient who is having difficulty coping after being in a motor vehicle crash in which her brother was killed. The patient was driving the car and blames herself for the accident. What is the priority nursing intervention of the nurse?
- Check to make sure that the patient does not want to hurt or kill herself.
 - Educate the patient about available support systems for grief resolution.
 - Enhance the patient's coping skills to alleviate depression and anxiety.
 - Encourage the patient to meet with a spiritual leader for guidance.

ANS: A

The highest priority for the nurse is to ensure the safety of the patient, so assessment of potential suicidal tendencies is paramount. The other interventions can take place once the nurse is confident that the patient will not try to hurt or kill herself.

DIF: Applying OBJ: 42.7 TOP: Assessment
 MSC: NCLEX Client Needs Category: Psychosocial Integrity: Grief and Loss NOT:
 Concepts: Coping

22. The nurse is caring for a terminally ill patient who will probably die within the next 2 weeks. What is the priority nursing intervention?
- Encouraging the patient to limit fluid intake to minimize congestion
 - Limiting the use of pain medications so that the patient can visit with family
 - Helping the patient to identify and complete desired tasks and activities
 - Completing funeral arrangements with the patient's next of kin

ANS: C

The priority intervention for the nurse currently is to help the patient identify and complete desired tasks and activities while the patient is still able to do so. Pain management is a high priority at this time, so analgesics should never be limited unless requested by the patient. The patient can drink as much or as little fluid as desired.

DIF: Applying OBJ: 42.7 TOP: Implementation
 MSC: NCLEX Client Needs Category: Psychosocial Integrity: End of Life Care NOT:
 Concepts: Palliation

23. The nurse is caring for a terminally ill patient who is actively dying and refuses to eat anything other than a few bites of ice cream. The patient's family member approaches the nurse and requests that a feeding tube be inserted so that her loved one will not starve to death. What is the best response of the nurse?
- Loss of appetite is a natural part of the dying process. Tube feedings would be uncomfortable and cause nausea.
 - I will contact the provider to obtain an order to insert the tube and start tube feedings.
 - Intravenous fluids would be more comfortable for the patient than a tube feeding. I will call the doctor to get the order.
 - I will listen to the patient's abdomen to make sure that bowel sounds are present and try encouraging oral fluids.

ANS: A

Common physical symptoms at the end of life include anorexia and cachexia. Tube feedings will cause discomfort as the tube is inserted and nausea as the GI tract is given food that it cannot handle. Encouraging oral intake will lead to increased secretions and congestion as well as possible aspiration of fluids. Intravenous fluids will increase congestion and edema. The nurse would educate the family on this part of the dying process.

DIF: Applying OBJ: 42.7 TOP: Caring
 MSC: NCLEX Client Needs Category: Psychosocial Integrity: End of Life Care NOT:
 Concepts: Coping

MULTIPLE RESPONSE

1. The nurse is caring for a patient who just died after a lengthy illness. Which portions of postmortem care may be delegated by the nurse to the nursing assistant? (*Select all that apply.*)
 - a. Gently washing the body and closing the patient's eyes
 - b. Offering support and empathy to the patient's family members
 - c. Documenting the patient's time of death in the medical record
 - d. Notifying all of the patient's consulting providers of the patient's death
 - e. Removing the patient's hospital ID band, IV lines, and urinary catheter
 - f. Gathering the patient's belongings so they may be taken home by the family

ANS: A, B, F

The nurse assistant can gently wash the patient's body, close the patient's eyes, and gather the patient's belongings. Offering support and empathy to the patient's family members would be done by all of the involved members of the nursing staff. Documenting the time of death in the chart and notifying all of the patient's providers is performed by the nurse. The nurse assistant can remove the patient's IV lines and urinary catheter if allowed by policy, but the hospital ID band would be left in place.

DIF: Understanding

OBJ: 42.7

TOP: Implementation

MSC: NCLEX Client Needs Category: Psychosocial Integrity: End of Life Care

NOT: Concepts: Collaboration

2. The nurse is caring for a patient who has just died. Which assessment findings by the physician and nurse are used to confirm that death has occurred? (*Select all that apply.*)
 - a. The patient was incontinent of bowel and bladder.
 - b. The patient's pupils are fixed and dilated.
 - c. The provider does not hear a heartbeat.
 - d. The patient's extremities are cool and mottled.
 - e. The patient has no palpable peripheral pulses.
 - f. The patient's face is relaxed and the mouth is open.

ANS: A, B, C, E

Assessment findings that confirm death has occurred include lack of pulse/heartbeat and fixed dilated pupils. Cool, mottled extremities, relaxed muscles, and incontinence of bowel and/or stool are common assessment findings in patients who are dying.

DIF: Understanding

OBJ: 42.4

TOP: Assessment

MSC: NCLEX Client Needs Category: Psychosocial Integrity: End of Life Care NOT:

Concepts: Caregiving